What Is Affective Psychotherap/Counseling Theory?

While every counselor works with people's emotions, thoughts, and behaviors, some counselors focus primarily on emotions or a person's affect. **Affect** is the outward expression of a feeling or emotion. In this lesson, we will focus on **affective counseling theory**, the idea that a person can be healed of mental health ailments by focusing on their emotions and taking responsibility for their feelings.

Margaret, age 40, decides to see a counselor for low self-esteem. She has always envisioned herself as a successful hard-working professional woman, but she is now in a dead-end job with a husband who makes just enough money to cover the basic bills. Margaret's counselor works with her using affective counseling techniques, which include providing supportive listening to Margaret, helping her identify and communicate her feelings, and helping her cope with her negative feelings.

Key Principles of Affective Psychotherapy Theory

The key principles of affective counseling theory are essentially the defining features of this type of therapy. From placing priority on the counselor and client relationship to the counselor's optimistic view of the client, the affective counseling techniques center on emotions, empathy, letting the client lead the session, non-judgment, and helping the client reach his or her full potential.

If Margaret visited a counselor using the affective counseling theory, the counselor would:

- Place priority on establishing a good, trusting, and respectful relationship with her.
- Assist her with becoming aware of her feelings of disappointment and sadness.
- Assist Margaret in taking responsibility for her own feelings (instead of blaming her sadness on others).
- Facilitate Margaret's ability to communicate her feelings effectively (i.e. by using 'I statements').
- Help Margaret learn coping mechanisms to deal with her troubling emotions, such as exercise and breathing techniques.
- Have an optimistic view of Margaret, knowing that her difficult feelings, thoughts, and behaviors are a sign that something in her environment is awry and her defense mechanisms have taken over.
- Believe that Margaret's view of herself has shaped the world in which she lives, including her behavior and her reactions to certain situations.
- Believe that Margaret, like others, will naturally grow and move towards **self-actualization**, or realizing one's full potential, as it is an innate drive to do so.

INTRODUCTION TO BRIEF PSYCHOTHERAPY

Brief psychotherapy (also brief therapy, planned short-term therapy) is an umbrella term for a variety of approaches to short-term, solution-oriented psychotherapy.

Brief therapy differs from other schools of therapy in that it emphasizes

(1) a focus on a specific problem and

(2) direct intervention.

In brief therapy, the therapist takes responsibility for working more pro-actively with the client in order to treat clinical and subjective conditions faster. It also emphasizes precise observation, utilization of natural resources, and a temporary suspension of disbelief to consider new perspectives and multiple viewpoints.

Rather than the formal analysis of historical causes of distress, the primary approach of brief therapy is to help the client to view the present from a wider context and to utilize more functional understandings (not necessarily at a conscious level). By becoming aware of these new understandings, successful clients will *de facto* undergo spontaneous and generative change.

Brief therapy is often highly strategic, exploratory, and solution-based rather than problem-oriented. It is less concerned with how a problem arose than with the current factors sustaining it and preventing change. Brief therapists do not adhere to one "correct" approach, but rather accept that there being many paths, any of them may or may not, in combination, turn out to be ultimately beneficial.

HISTORY OF BRIEF PSYCHOTHERAPY

Toward the end of the nineteenth century, when Breuer and Freud invented psychoanalysis, hysterical symptoms defined the focus of the work. These early treatments were brief, the therapist was active, and, basically, desperate patients selected themselves for the fledgling venture. In time, free association, exploration of the transference, and dream analysis replaced hypnosis and direct suggestion. So, as psychoanalysis evolved, the duration of treatment increased and therapist activity decreased.

Alexander's (1971) manipulation of the interval and spacing of sessions (the therapeutic "frame") explored the impact of decreased frequency, irregular spacing, therapeutic holidays, and therapist-dictated (rather than patient- or symptom-dictated) treatment schedules. All these frame changes enhanced the external reality orientation of psychotherapy. Then World War II produced large

numbers of patients who needed treatment for “shell shock” and “battle fatigue.” Grinker and Spiegel’s (1944) treatment of soldiers ensconced *brevity* in the treatment armamentarium. Lindemann’s (1944) work with survivors of the Coconut Grove fire—with its *focus* on grief work—also occurred during this period.

The modern era started in the 1960s when Sifneos and Malan independently developed the first theoretically coherent, short-term psychotherapies. The increased activity of Ferenczi and Rank, Lindemann’s crisis work, Grinker and Spiegel’s push for brevity, Alexander and French’s flexible framework, and Balint’s finding and holding the focus were all technical innovations, but none constituted a whole new method. Malan and Sifneos each invented ways of working that were not a grab bag of techniques but each a whole new therapy, with a coherent body of theory out of which grew an organized, specified way of proceeding.

Over time, as therapies became briefer, they became more focused and the therapist became more active. But brevity, focus, and therapist activity are ways in which short-term therapies differ, not only from long-term therapy, but also typically from one another. Patient selection makes up a fourth “essence” in the description of the short-term psychotherapies.

MODERN BRIEF PSYCHOTHERAPIES

There are four general schools of brief psychotherapy:
(1) psychodynamic;
(2) cognitive-behavioural;
(3) interpersonal; and
(4) eclectic. Each has indications and contraindications, but it is worth acknowledging at the outset that there is no conclusive evidence that any one short-term psychotherapy is more efficacious than another.

Psychodynamic Short-term Therapies

The “interpretive” short-term therapies all feature brevity, narrow focus, and careful patient selection, but the common feature is the nature of therapist activity. Psychoanalytic interpretation of defenses and appearance of unconscious conflicts in the transference appear in other short-term therapies (and are often downplayed), but only in these methods are interpretation and insight the leading edge of the method and, as in psychoanalysis, the main “curative” agents.

Sifneos's anxiety-provoking therapy (1972, 1992) is an ideal example of a brief psychodynamic psychotherapy. This treatment runs 12 to 20 sessions and focuses narrowly on issues (such as the failure to grieve, fear of success, or "triangular," futile love relationships). The therapist serves as a detached, didactic figure who holds to the focus and who challenges the patient to relinquish both dependency and intellectualization, while confronting anxiety-producing conflicts. One can think of this method as a classical oedipal-level defense analysis with all of the lull periods removed. One limiting feature is that it serves only 2% to 10% of the population, the subgroup able to tolerate its unremitting anxiety without acting out.

An illustrative contrast, Sifneos's anxiety-*suppressive* therapy, serves less healthy patients who are able to hold a job and to recognize the psychological nature of their illness, but who are unable to tolerate the anxiety of deeper levels of psychotherapy. Anxiety-provoking psychotherapy is longer, less crisis oriented, and aimed at the production of anxiety—which then is used as a lever to get to transference material. (In psychoanalysis, transference emerges, but in short-term therapy it sometimes is elicited.)

Malan's method is similar, but the therapist discerns and holds the focus without explicitly defining it for the patient. (In the initial trial, if the therapist has in mind the correct focus, there will be a deepening of affect and an increase in associations as the therapist tests it.) A unique feature of this treatment is that Malan sets a date to stop once the goal is in sight and the patient demonstrates capacity to work on his or her own. A fixed date (rather than the customary set number of sessions) avoids the chore of keeping track if acting out causes missed sessions or scheduling errors.

Malan's work is reminiscent of the British object-relations school. Like Sifneos, he sees interpretation as curative, but he aims less at defenses than at the objects they relate to. In other words, the therapist will call attention to behaviour toward the therapist, but rather than asking what affect is being warded off, Malan wants to know more about the original object in the nuclear conflict who set up the transference in the first place. Malan's later work converges toward that of Davanloo's,[8,13,14] so that his and Malan's approaches are conceptually similar.

Davanloo's method can appear dramatically different from all others in terms of therapist activity. The therapist's relentless, graduated, calculated clarification, pressure, and challenge elicit the anger used to dig out the transference from behind "superego resistance." Davanloo begins by criticizing the patient's passivity, withdrawal, or vagueness, while pointing out the body language and facial expressions demonstrating them. Patients who do not decompensate or withdraw are then offered a trial interpretation: the patient's

need to fail and clumsiness in the interview disguise aggression toward the therapist; the patient's need to become "a cripple," to be "amputated" and "doomed," disguises rage.

As shown in Figure 11-1, the therapist works with this "triangle of conflict"—which goes from defense (D) to affect (A) to impulse (I), in relation to the "triangle of persons." This begins with a problematic current object (C), one mentioned in the first half of the initial interview. Then investigation moves to the therapist (T), who the patient has just been protecting from anger, and then to the parent (P) who taught such patterns in the first place. One or two circuits around the "D/A/I" (conflict) triangle in relation to three points of the "C/T/P" (persons) triangle constitute the "trial therapy." This is a more elaborate version of the trial interpretations Malan and Sifneos use to test motivation and psychological-mindedness. Davanloo's patients lack the ability to distinguish between points in the "conflict [**D**efense/**A**ffect/**I**mpulse] triangle" or to experience negative affects directly. By trolling the D/A/I triangle around the "persons [**C**urrent object/**T**herapist/**P**arent] triangle," Davanloo forces the frigid patient to feel and creates a mastery experience for the patient.

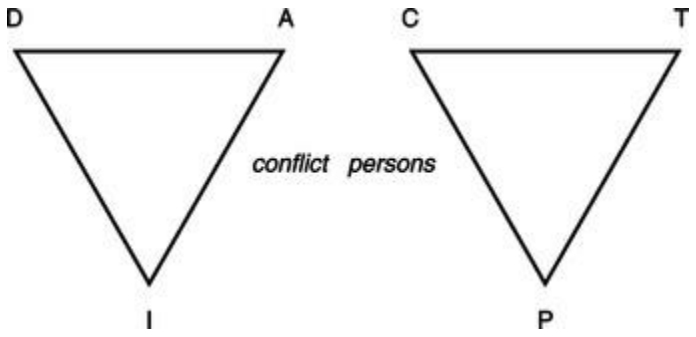


Figure 11-1 Davanloo's "trial therapy."

These are nonpsychotic, nonaddicted, nonorganic individuals who have a combination of retroflexed aggression and punitive superegos, but at the same time, enough observing ego to discount the apparent harshness. His patients find the seemingly harsh Davanloo method supportive; up to 35% of an average clinic's population are said to tolerate it, a range broader than either Sifneos or Malan claimed. Failures of the Davanloo "trial therapy" typically are referred to cognitive therapy.

The dynamic-existential method of James Mann relied on a strict limit of exactly 12 sessions. Time is not just a reality, a part of the framework, but time is an actual *tool* of treatment. Twelve sessions, which Mann chose somewhat arbitrarily, is sufficient time to do important work but short enough to put the patient under pressure. This set number with no reprieve is both enough time and

too little. It thrusts the patient and the therapist up against the existential reality they both tend to deny: Time is running out.

No other short-term therapy seems to require so much of the therapist. And even if this method does not appeal to all short-term therapists, almost every subsequent theorist in the field seems to have been influenced by Mann to some degree—even Budman and Gurman, whose use of time appears so unlike Mann's. It is described somewhat in detail here because the interaction of therapist activity with phases of treatment is so clearly highlighted.

Underlying the focus the patient brings, Mann posits a "central issue" (analogous to a core conflict) in relation to the all-important issue, "time itself." The therapist is a timekeeper who existentially stays with the patient through separations, helping master the developmental stages in which parents failed the patient. Mann's theoretical point of departure (which probably *followed* the empirical finding that 12 sessions was about right) is Winnicott's[18] notion that time sense is intimately connected to reality testing, a "capacity for concern," and unimpaired object relationships. A better sense of time and its limits is the soil for growing a better sense of objects.

During evaluation, the therapist begins to think about the "central issue" (such as problems with separation, unresolved grief, or failure to move from one developmental stage to another—delayed adolescence especially). This central issue is couched not in terms of drive or defense, but existentially, in terms of the patient's chronic suffering. If the patient is moved by this clarification, the therapist solicits the patient's agreement to work for a total of 12 sessions. The patient will at this point express some disbelief that 12 sessions is enough. But if the evaluation has been accurate and the method is suited to this patient, the therapist should look the patient in the eye and say that 12 sessions—and only 12—will be just enough.

The early sessions are marked by an outpouring of data and by the formation of a positive or idealizing transference. During this phase the therapist's job is to hold the focus on the central issue and allow the development of a sense of perfection. At about the fourth session, there is often an appearance of disillusionment and the return of a focus on symptoms. At this point the therapist makes the first interpretation that the patient is trying to avoid seeing that time is limited and avoiding feelings about separation. This sequence is repeated, and it deepens through the middle of therapy.

After the midpoint, session 6, overt resistance often occurs, perhaps lateness or absence on the part of the patient, and the emergence of negative transference. The therapist examines this in an empathic and welcoming way while inwardly examining countertransference issues that may impede the work. And finally, in the latter sessions comes a working through of the patient's pessimism and recollection of unconscious memories and previous bad separation events, along with an expectation of a repetition of the past. By the therapist's honest acceptance of the patient's anger and ambivalence over termination, the patient

moves from a state of neurotic fear of separation and its attendant depression to a point where the patient is ambivalent, sad, autonomous, and realistically optimistic.

Cognitive-Behavioural Brief Therapies

The behavioural therapies have a decades-long history and a good record of achievement. The cognitive therapies are the inheritors of this earlier track record. What the two have in common is that neither addresses "root causes" of disorders of mental life, but focus almost exclusively on the patient's outward manifestations. These "non-psychological" or "non–insight-oriented" styles of therapy are broadly applicable, both in terms of patients and problems.

The method of Aaron Beck aims at bringing the patient's "automatic" (preconscious) thoughts into awareness and demonstrating how these thoughts affect behaviour and feelings. The basic thrust is to challenge them consciously, and to practice new behaviours that change the picture of the world and the self in it. Beck says that an individual's interpretation of events in the world is encapsulated in these fleeting thoughts, which are often cognitions at the fringes of consciousness. These "automatic thoughts" mediate between an event and the affective and behavioural response. The patient labours under a set of slogans that, by their labelling function, ossify the worldview and inhibit experimenting with new behaviours.

The therapist actively schedules the patient's day-to-day activity, and the patient is asked to list in some detail actual daily activities and to rate the degree of "mastery" and "pleasure" in each. These allow the therapist to review the week with the patient and to sculpt behaviours. Cognitive rehearsals are also used to help the patient foresee obstacles in the coming week. The therapist repeatedly explains the major premise of the cognitive model, that an intermediating slogan lies between an event and the emotional reaction. This slogan may take a verbal or pictorial form. The therapist then, using Socratic questioning, elicits from the patient statements of fact that lead to a more accurate conceptualization of the problem, while at the same time *actively interfering*

Affective Change in Psychodynamic Psychotherapy: Theoretical Models and Clinical Approaches to Changing Emotions

Claudia Subic-Wrana[1], Leslie S. Greenberg[2], Richard D. Lane[3], Matthias Michal[1], Jörg Wiltink[1], Manfred E. Beutel[1]

Zusammenfassung

Veränderung des Emotionserlebens in der psychodynamischen Psychotherapie: eine Übersicht über theoretische Modelle und behandlungstechnische Empfehlungen

Fragestellung: Die Psychoanalyse betrachtet emotionale Veränderung als Indikator der Wirksamkeit therapeutischer Interventionen; psychoanalytische Autoren haben das durch die Neurowissenschaften vertiefte Verständnis der Emotionsregulation und -transformation zur Erweiterung der klinischen Theoriebildung genutzt. Wir fragen, ob sich diese Weiterentwicklung im behandlungstechnischen Umgang mit Emotionen in der psychodynamischen Psychotherapie reflektiert.
Methoden: Mit Bezug auf ein Modell therapeutischer Veränderung durch die Aktivierung emotionaler Erinnerung präsentieren wir eine Übersicht über psychodynamische Modelle und Behandlungsempfehlungen im Umgang mit maladaptiver Emotionsverarbeitung.
Ergebnisse: Mit Ausnahme der manualisierten Ansätze zur Behandlung von Persönlichkeitsstörungen, die spezifische Techniken zur Förderung der Emotionsregulation beschreiben, konzipiert die psychodynamische Behandlungstechnik Emotionen als einen psychischen Inhalt unter anderen; dies führt zu einem Mangel an spezifischen Behandlungstechniken zur Förderung und Vertiefung des Emotionserlebens.
Diskussion: Emotionsbezogene Erinnerungen müssen aktiviert werden, um verändert werden zu können. Deshalb sollte die psychodynamische Psychotherapie um Techniken ergänzt werden, die das Emotionserleben stimulieren.

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Keywords

Psychodynamic Psychotherapy – Emotional Memory – Emotional Activation – Emotion Focused Psychotherapy (EFT) – Levels of Emotional Awareness (LEA)

Summary

Objectives: Affective change has been considered the hallmark of therapeutic change in psychoanalysis. Psychoanalytic writers have begun to incorporate theoretically the advanced un-

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derstanding of emotional processing and transformation of the affective neurosciences. We ask if this theoretical advancement is reflected in treatment techniques addressing the processing of emotion.
Methods: We review psychoanalytic models and treatment recommendations of maladaptive affect processing in the light of a neuroscientifically informed model of achieving psychotherapeutic change by activation and reconsolidation of emotional memory.
Results: Emotions tend to be treated as other mental contents, resulting in a lack of specific psychodynamic techniques to work with emotions. Manualized technical modifications addressing affect regulation have been successfully tested in patients with personality pathology, but not for psychodynamic treatments of axis I disorders.
Conclusions: Emotional memories need to be activated in order to be modified, therefore, we propose to include techniques into psychodynamic therapy that stimulate emotional experience.

1. Introduction

The focus on affect and the expression of emotion has been identified as one of the key features distinguishing psychodynamic from cognitive behavioral psychotherapy in the programmatic paper by Hilsenroth (2007); in their meta-analysis of trials testing the effectiveness of psychodynamic psychotherapy, Diener et al. (2007) concluded that facilitation of affective experience and expression by the therapist contributes consistently and significantly to outcome. Its magnitude of r = .30 is in the range of the effect of the therapeutic alliance, the most consistent predictor of outcome to date. However, as Whelton (2004) stated, there has been a paucity of research on the facilitation of affective change in psychodynamic psychotherapy.

Although a recent review of neuroimaging studies could demonstrate that therapeutic improvement in psychodynamic psychotherapies was associated with changes in neuronal networks subserving emotional processes (e.g., the fear network in panic disorder; Abbass et al. 2014), empirical evidence linking emotion activating techniques and psychotherapy outcome has been collected mostly from researchers familiar with the Les Greenberg's Emotion Focused Therapy (EFT): Kramer et al. (2015) showed that the in-session experience of fundamentally adaptive grief contributed strongly to the reduction of depressive symptoms in short term psychotherapy. Missirlian and colleagues (2008) could demonstrate that the activation of emotional arousal in Emotion Focused Therapy (EFT) for depression was predictive for a positive outcome; Carryer & Greenberg (2010) found that emotional arousal in 25% of the episodes in which a patient was referring to an emotion was the best predictor of outcome, while emotional over-arousal as well as emotional under-arousal inhibited therapeutic change. Emotionally arousing exposure to difficult and fearful stimuli in a safe and supportive environment has been conducive to restructuring traumatic experiences (Paivio et al. 2001; Whelton et al. 2004).

In line with the lack of studies linking emotion focused techniques and outcome, in a recent review of the psychoanalytic literature, Lotterman (2012) found few references specifying how and when to address affects in the clinical situation, although

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psychoanalytic writers (e.g., Rice & Hoffman 2014) have taken up the burgeoning neuroscience literature on implicit and explicit processing of affective arousal theoretically. Lotterman's observation may help to explain our clinical experience from supervising videotaped treatments of three recent psychodynamic randomized controlled trials (Beutel et al. 2013, 2014; Leichsenring et al. 2013), psychodynamic psychotherapists often did not focus on salient emotions, missed to respond to the emotional arousal of the patient in a timely manner and to maintain the level of emotional arousal conducive to change, so that the change potential of critical emotional moments got lost (Stern 1998, 2004).

The paucity on empirical research on emotional change in psychodynamic psychotherapy and our observations in the videotaped session of the RCTs mentioned above have led us to wonder if we provide psychodynamic practitioners and candidates in psychodynamic training with "good enough" theories, technical guidelines and skills to assess and handle emotions, e.g., help patients gain access to, experience and symbolize their emotions. Based on a brief neurobiologically informed overview of psychoanalytic models of affect, this paper therefore reviews treatment techniques for emotional processing in psychodynamic psychotherapy and gives a short overview of the treatment rationale of Greenberg's EFT. Based on a case illustration it discusses the pros and cons of an integration of emotion-focused techniques into psychodynamic treatment.

2. Models of emotions in psychodynamic theory: implicit, explicit or repressed?

As a frame of reference for the review we use the Level of Emotional Awareness Theory, a timely and comprehensive attempt to integrate neurobiological and psychological findings on emotion processing which is compatible with psychoanalytic developmental and clinical theories, e.g., Bucci's (2002) concept of symbolic and subsymbolic processes and emotion schemes (Subic-Wrana et al. 2011). Integrating Gestalt psychology principles of sensory perception and Piaget's (1974) schemata of sensory-cognitive development, the LEA-model proposes a framework for the cognitive-emotional development. Level 1 of emotional awareness (Lane & Schwartz 1987) is characterized by the predominance of consciously unaware stimulus-reflex patterns; affect-related tension is experienced as a physical sensation. Level 2 refers to an action response to affective arousal (Lane & Schwartz 1987), which is experienced as positively or negatively valenced state of tension ("I feel good/bad") or as action tendencies ("I would like to run away"). At level 3, a certain mood or feeling state fills consciousness; however, different or contradictory feelings or moods cannot be experienced simultaneously at one time. At Level 4 (blends of emotion), it becomes possible to experience emotional ambivalence – different emotions can be experienced at one time. At Level 5, the world of feelings of another person can be experienced as separate from one's own. Emotional awareness progresses from implicit (levels 1 and 2) to explicit (3 to 5) levels with mental representation of feeling states that can be experienced consciously and verbally ex-

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pressed. At each level, there is also awareness of "the other" (e.g., Level 1 concerns reflexive empathy – one baby cries and another cries), and there is a progressive increase in self-other differentiation as one ascends levels. Lane has come to term Level 1 and Level 2 as *implicit levels* of emotional awareness, indicating that at these levels there is no mental representation of affective arousal as feeling states that can be experienced and verbally expressed. Levels 3 to 5 of emotional awareness are termed *explicit levels* of emotional awareness, indicating that starting with Level 3, bodily sensations, action tendencies, and tension states can be mentally represented as *feeling states* and can be put into words.

Starting from the exploration of the origins of hysteria, Freud initially followed two routes of explaining affective processing and representation. In the *trauma model*, the experience of being sexually abused leads to an overwhelming affective arousal disabling the individual's capacity to process the traumatic experience properly as a coherent narrative. Instead, the traumatic event and its emotional impact are stored and represented in the body, and the hysteric symptom represents the distorted narrative. With the technique of free association that equally welcomed every type of psychic contents the distorted narrative could be completed. The cathartic value of the accompanying reactivation of the overwhelming affective arousal, however, has been debated (Bushman et al. 2002). The reactivation of undifferentiated and intense affective arousal may not help to process arousal from implicit to explicit levels of emotional awareness, e.g., to enrich the primary visceral activation or action tendency (implicit level) with the experiencing of differentiated feeling states and the construction or re-construction of meaning.

The other route taken by Freud and his followers to understand the role of affect in mental disorders led to the model of *neurotic symptom formation*. When wishes derived from drives are incompatible with the interpersonal and environmental requirements of an individual, the wish (e.g., to kill a rival) and the accompanying emotional state (e.g., anger) are repressed. Rendered unconscious by defensive processes, emotions need to be recovered by suitable therapeutic intervention. From a current point of view, however, it must be questioned if entire, fully verbally symbolized and previously conscious emotions are repressed or if the defense mechanisms compromise the processing of affective arousal from implicit to explicit levels (Rice & Hoffmann 2014).

The *tripartite structural model* elaborated the dynamic processes set in motion by an inner conflict between drive-derived wishes and super-ego imposed objections against them. Anxiety was assumed to signal danger and motivate subconscious defensive processes. Moving the conception from drive to information (Lane & Garfield 2005) this model of affect is more compatible with the concept of implicit and explicit emotional processing (Lane & Schwartz 1987). Defensive processes activated by a subconsciously experienced anxiety signal – e.g., awareness of a negative tension state – could impede the processing of the affect activation from implicit to explicit awareness. E.g., a panic patient may seek the proximity to a significant other out of anxious tension and helplessness, but may be not become aware that feeling angry at this person motivated the proximity-seeking.

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In the Kleinian approach (Klein 1975a, 1975b) aggression as a derivate of the death drive adversely affects the relationship to one's objects and to one's self. The *schizoid* position – operating with primitive defenses like splitting or idealization – is characterized by the inability to experience ambivalence towards self and others. Therapeutic progress requires reaching the *depressive position*, the ability to endure ambivalence in memories, fantasies, wishes and feelings with regard to self and others (corresponding to LEA-levels 4 or 5). In a neo-Kleinian approach, Kernberg (1988) considered affect as the indicator or psychic representation of the instinctual needs that are met or neglected by the primary caregivers guaranteeing the physical and psychic survival of the infant. Cornerstones of the emerging psychic structures of the growing infant are introjections of child-caregiver interactions with negative or positive affective valence. Maturity is indicated by the simultaneous awareness or integration of positive and negative emotions with regard to self and others and an increasing ability to become aware of and to tolerate negative emotions.

The concept that the capacity for conscious emotional awareness results from a developmental process has been most clearly expressed by Bion (1984). He postulates that *beta-elements* – arousal states of the infant, communicated by bodily sensations and actions – have to be transformed into *alpha-elements* – symbols that permit to communicate one's needs and feelings and to reflect upon them. Influenced by Bion, mentalization theory (Fonagy et al. 2010) emphasizes the transformation from implicit to explicit emotional awareness in developing psychic structure. As delineated in Gergely and Watson's (1996) social biofeedback theory, the permanent marked mirroring of its affective states by the primary caregivers enables the child to identify and become aware of distinct affect states, which builds the basis for second order or psychic self-representation. Mark Solms (2013) neuroscientifically based considerations on the "conscious id" add a new meta-psychological perspective to the clinical theory of Fonagy and colleagues by stating that the primary affects generated in the limbic structures associated to the brain stem – to Solms the neurobiological substrate of the id – are the source of conscious self-awareness. From his point of view cortical based distinct sets of second order representations can only become consciously aware if a primary emotion (as defined by Panksepp 1998), linked to this set of psychic representations is activated. The notion of the "procedural organization of emotion" (Clayman 1991), another term for implicit emotional awareness – has been incorporated into recent psychoanalytic discourse. Implicit emotional procedures have been acquired in different contexts during development. Defining our fundamental ways of interacting with others, automatic procedures become resistant to change by repeated rehearsal. Facial, gestural, and behavioral displays of emotion serve regulatory functions, such as avoidance of embarrassment or shame. Based on automatic repertoires established in previous important relationships, these encoded emotional "ways of relating and responding" have been described as "model scenes" (Lichtenberg 1989) or "representations of interactions generalized"(RIGS) (Stern 1985) that shape the transference to the analyst. When the patient experiences the analyst as a new object who does not set off the old automatic emotional control procedures, the old emotional procedures can be replaced by a new set of more

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adaptive emotional procedures (Stern et al. 1998). Change (Sugarman 2006) may thus occur without insight, yet, subsequent comprehension of the relational procedures could consolidate the change "top-down" by consciously processed information. Thus, the therapeutic process involves transforming implicit into explicit emotion representations. Completely eliminating the old procedures rarely occurs as implied in the psychoanalytic model of regression and progression.

3. Emotional processing in psychodynamic treatment technique

In psychoanalytically informed treatment technique, an affective reaction is generally considered as an indicator for true and not merely intellectual insight (cf. Fenichel 1941; Spezzano 1993). Lotterman considers affects as highly informative ". . . to identify what in the patient's material is useful to explore and interpret" (p. 315). For him the dominant affect is ". . . a snapshot of the patient's current motivations and how he is doing in reaching his most important goals" (p. 316) and "a reliable marker of the immediate, living state of the self" (p. 317). But how do analysts and psychodynamic psychotherapists assess and work with emotions?

The basic tools of psychodynamic technique are clarification, confrontation and interpretation as means to structure the "stream of consciousness" (James 1890) which is elicited by the standard instruction given to the patient: to verbalize everything that comes to mind (free association). Correspondingly, the analyst is attuned in a way of free floating attentiveness. In concordance to clinical theory that regards the suppressed psychic contents as a blend of thoughts, wishes, fantasies, intentions and feelings – conceptualized in more recent approaches as model scenes consisting of wish/memory/affect-components related to introjected interactions with significant others – these three basic technical steps are meant to deal with all these components of psychic contents. After structuring the patient's associations with clarification and confrontation the interpretation reconstructs suppressed memories or creates new meaning by combining wishes, fantasies, intentions and feelings to a meaningful unit.

Striving to achieve therapeutic progress in a limited number of sessions psychodynamic short term psychotherapies rely on formulation of a treatment focus and a more active therapeutic stance working with the resistances of the patient toward the conscious experience of the repressed psychic contents. The focal theme helps the therapist to decide which of the patient's associations should be explored in more depth and which should be left aside (Balint et al. 1972; Luborsky & Crits-Christoph 1998). E.g., the core conflictual relationship theme method (CCRT; Luborsky & Crits-Christoph 1998) investigates the patients' narratives of interactions with regard to wishes (W) to significant others, the anticipated reaction of the others to their wish (RO) and their own reaction to this anticipated reaction (RS). But in the elaboration of a self-fulfilling maladaptive cycle of the experiences with others that lead to complex reinterpretations of the patient's experience and behavior underlying his symptoms, feeling states are only one component among others. Based on the triangles of persons (Menninger 1958) and conflict (Ezriel 1952) Malan (1976) conceptualized the focus as suppressed feeling or

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impulse, in the past related to the parents and now activated by a current relationship, causing anxiety which motivates defensive behavior. The interventions of the therapist aim at bringing defenses and hidden feelings into consciousness (e.g., "The anger you felt against your husband made you anxious; therefore you criticized yourself instead," example after Fredrickson 2013) – although it can be doubted if naming the suppressed feeling leads the patient to experiencing this feeling.

Davenloo's (1990) technique is a prominent example for an active stance in working with the resistances in short term approaches. Based on a conception of a cruel superego, the therapist aligns with the weakened self in order to facilitate a breakthrough of repressed impulses. The therapist reacts to nonverbal behaviour (such as avoidance of eye contact, tone and volume of voice, body movements) and verbal mannerisms (e.g., vague habits such as "sort of, maybe") indicating suppressed emotions and impulses by actively challenging and putting pressure on the patient to forego his defenses and communicate his true feelings. However, there is little technical advice how to process uncovered feelings. A full review of experiential short-term psychotherapy is beyond the scope of this paper (cf. Fosha 2000).

The interpretation of the transference and making use of emotions experienced in the countertransference (Heiman 1950) for gaining new insights into the relationships to significant others the patients are not able to symbolize are "core"-techniques of psychoanalysis and psychodynamic psychotherapy. This can be regarded as a means to bring repressed or not fully symbolized relational themes and related emotions into the "here and now" of the therapeutic session. Therefore, the use of interpretations derived from countertransference and the interpretation of the transference are in themselves emotion-provoking as far as the patient allows himself to be touched by them, e.g., is able to lower his resistances. In traditional Freudian psychoanalysis clarification and confrontation have been used as the technical means to prepare for transference interpretations. Transference themes were thought to emerge automatically (according to the theorem of psychic determinism) in the flow of the patient's associations after the resistances against transference wishes and the related emotions have been worked through sufficiently – thus, no special techniques to foster the conscious awareness of emotions were applied, as the suppressed emotions would "pop up" automatically.

Ferenczi's (1930) emphasis on emotional attunement between analyst and patient laid the groundwork for the concept of recommencement by Balint (1968) or corrective experience by Alexander and French (1980). These psychoanalytic scholars emphasized that "felt feelings" underlying a significant scene in the transference and their interpretation is a precondition for the achievement of change. The technical need to engage the patient emotionally as a prerequisite for change has also been suggested in Stern's (2004) concept of "now moments" as lived experiences subjectively shared by patient and analyst offering critical windows for psychoanalytic exploration. This peak moment of experience ensues when the two members of the dyad become aware of what the other is experiencing.

Additions to classical psychoanalytic technique were made by the clinical theories of Bion (1984) and his followers on the one hand and Kohut (2009) and the school of

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self-psychology on the other hand. In his clinical-theoretical contribution Bion pointed out that psychic representations of thoughts, feelings and desires are acquired by the introjection of the caregivers' words and actions that make sense out of the undifferentiated psycho-physiological utterances of the infant. It follows that traumatized or deprived patients do not suppress fully symbolized psychic contents related to relational conflictual themes but that they need the analyst to make sense of their utterances, actions or words. This requires an empathic identification allowing the analyst to suggest how the patient possibly experienced crucial scenes in the past and in the present. In a summary of basic conceptions of self-psychology, Ornstein (1991) pointed out that self-aspects and needs of the patient that were neglected in the primary relationships and thus not consciously available come into the awareness of the therapeutic dyad in how the psychoanalyst experiences or "feels" the patient and makes this available to the patient. The technique related to this clinical concept is also a kind of empathic listening, suggesting to the patient how he may feel or have felt in relation to himself and others. Lotterman (2012) also suggests empathic listening as a technique to gain access to the affect, when he suggests turning patients' experiences "to how it felt to have this emotion, rather than to focus on why this emotion occurred" (p. 326).

In line with Kleinian treatment technique, Kernberg's Transference Focused Psychotherapy (Clarkin et al. 2014) focusses on the interpretation of the transference, but adds a hierarchy of "dangers" (e.g., suicidal ideation, self-harm) that – if present – have to be given priority. These dangers are understood as indicators of negative affect, and they are interpreted in the context of the transference. Thus, strong but implicit activation of negative emotions is systematically related to the therapeutic relationship, "peaks" of implicit affective arousal are systematically related to feeling states that motivate the patients' behavior. Mentalization Based Treatment for Borderline Disorder (MBT) by Bateman and Fonagy (2006) advises against the immediate and frequent use of transference interpretation. Symptomatic behavior indicating a rupture in the therapeutic relationship has priority in the course of the session. The therapist explores the background of these states of non-mentalizing and tries to create meaning instead, using "basic mentalizing" techniques like suggestions of what the patient may have felt or challenges the patients' introspective skills and helps to differentiate them. Both treatments share special attention to peaks of affective over-arousal, indicated by symptomatic behaviour or acting out prompting specific techniques to identify and differentiate the emotional states that are embedded in them in order to give them meaning – from a LEA-point of view, both treatments systematically transform implicit to explicit affect; this may be a key-feature for their effectiveness.

To conclude, traditional psychoanalytic technique and established psychodynamic short-therapy approaches regard the efforts to pervade the defenses against the conscious awareness of conflictual relational themes – especially in the transference – as a powerful tool to achieve therapeutic change; the change of "felt feelings" or "experiencing" is regarded as an indicator for psychotherapeutic progress by psychodynamic treatment theory. The tools for working through the patient's resistance against becoming aware of his conflictual relational themes and the accompanying emotions are clarification, confrontation and interpretation. Guided by the concep-

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tion that the repressed contents including the emotions will be freely accessible after working through the resistance, no special technical means are suggested for accessing or processing emotions. Bionian technique as well as approaches informed by Kohut's self-psychology advocate empathic attunement to the patient in order to help him to symbolize previously non-symbolized feeling states that give meaning to the patient's experiences. But only recent manualized treatments for borderline disorder give *systematic* technical advice how to handle peaks of undifferentiated high tension – often indicated by symptomatic behavior – in order to facilitate the awareness of feeling states that are related to the situation that elicited the high tension state.

4. Toward an integrative model of emotion processing: a case illustration

In a seminal collaboration, Lane, Greenberg and colleagues (2015a) proposed a model of therapeutic change integrating evidence from cognitive neuroscience on implicit and explicit emotion and memory, emotion-memory interactions, memory reconsolidation, and the relationship between autobiographical and semantic memory. The integrative model examines how different psychotherapy approaches achieve therapeutic change by changing emotional memory, e.g., emotional memory may be accessed via episodic memories by psychodynamic psychotherapy. The model is based on a review of the neuroscientific literature in regard to the formation, destabilization and reconsolidation of semantic and episodic memory and on a review of papers in the field of psychotherapy research linking emotion activation through psychotherapy and outcome. The model holds that repetitive maladaptive reactions or vicious circles of affective arousal, its cognitive assessment and the resulting behavior, that underlie psychic symptom formation, are based on emotion-centered autobiographic memory and semantic structures. On a neurobiological level, the bidirectional activation circles between hippocampus and amygdala built the interface between emotion and memory, an emotional activation in the middle range is the precondition for the formation of a memory trace. Given there is a sufficiently high level of emotional activation, each recall of an autobiographical episode or semantic procedure leads to hippocampal activity – thus each memory activation can potentially change the episodic or semantic memory, in psychotherapy this neurobiological precondition helps to differentiate and deepen autobiographic memory, in symptom formation linked to semantic memory every anxiety attack may add new triggers for future anxiety attacks. From these neurobiological findings follows that change in psychotherapy can only be achieved when memory *and* emotional experiencing are activated simultaneously – the change of emotional memories requires reactivation of old or prior emotional memories. The experiencing of the emotions may start implicitly – via bodily sensations or action tendencies – with help of the therapist the emotional experience it will deepen and become explicit as feeling that can be verbally expressed, "known" and "owned" by the patient. This process is the precondition for the engagement in a new emotional experiences in the "here and now" in order to incorporate new experiences and create an "update" of the prior emotional

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memory. In a process of reconsolidation this new, integrative memory structure is reinforced by new ways of behaving and experiencing. In this model, emotional arousal plays a key role, as psychotherapeutically accomplished change needs the activation of primarily emotional memory.

In line with the model of therapeutically achieved change proposed by Lane et colleagues (2015a), a central assumption of Emotion Focused Therapy (EFT) is that dysfunction results from the *disowning* of primary experience and the resulting inability to integrate certain experiences into one's existing self-organization. What is unacceptable to the self, however, is not necessarily seen as being dealt with by expelling it from consciousness (repression), but rather by failing to experience it as belonging to one's self. Thus what leads to dysfunction is not having an embodied experience of one's anger, sadness, or fear as opposed to not knowing that this is what one feels.

Coming from a humanistic tradition of psychotherapy, Emotion Focused Therapy (EFT) integrates elements from client-centered and Gestalt therapy traditions. Emotion is seen as basic to the construction of the self and self-organization – an assumption that is fully compatible with psychodynamic thinking. EFT regards human emotion as organized by "emotion schematic memories"; the activation of these schemes produce experience that is idiosyncratic, based on the person's past emotional learning related to that emotion, and this influences all current processing. The basic mode of processing that is set in motion by scheme activation occurs out of awareness and leads to conscious processing. Thus the activation of a fear scheme will set a basic mode of processing for threat in motion and this conscious processing works in the service of the affective goal activated by the scheme – safety in the case of fear. Emotion schematic processes can include linguistic components but often consist largely or entirely of pre-verbal elements (including bodily sensations, visual images, and even smells); they are also oriented toward action to satisfy needs/goals/concerns. The development of schemes and the self-organizations formed by a synthesis of schemes are best understood as the development of neural networks. Emotion schemes and their activation ultimately are the targets of intervention in EFT.

The three overarching goals of EFT are to promote emotional awareness, emotion regulation and emotion transformation (Greenberg 2004). EFT consists of three levels of interventions for fostering and deepening emotional experiences. On the first level (empathic symbolization), the therapist *empathically explores* what the patient might have felt in a certain situation and puts these emotional experiences into words. In doing this the therapist takes care to put the interventions in a questioning stance so that the patient is free to accept or refuse the inferred feeling state. In *guiding attention* the therapist motivates the patient to intensify the awareness for bodily sensations or action tendencies that occur when he remembers events related to significant others or to give words to emotions that come to mind when he or she confronts himself with the memory. On the third level *emotional experiencing is actively stimulated*, e.g., with techniques derived from Gestalt therapy like two-chair or empty chair dialogues providing a highly emotional entry into self and object representations and bringing up salient emotional memories.

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These interventions are meant to deepen and differentiate an upcoming emotion – may it primarily be expressed implicitly (e.g., bodily sensations or action tendencies) or explicitly – and to help the patient to become aware of feeling states, to maintain awareness, to experience the unmet need in the painful emotion and – with the help of the therapist – feel deserving of having the need met. By focusing on what is felt, the patient may become aware that one emotion is accompanied by another emotion (e.g., sadness underlying anger), experiencing the core ingredient of psychic ambivalence, e.g., mixed feelings towards significant others as well as towards the self.

Our first experiences (M. B. & C. S. W.) with the integration of EFT-elements into psychodynamic psychotherapy are reflected in this case illustration: "Miss C., now a student in her mid-20s, has suffered from a severe eating disorder with bulimic and anorectic symptoms. After three years of treatment, she has her weight and her eating behavior under control, but especially when she has to learn for tests and examinations, she suffers from states of heightened inner tension that are accompanied by the urge to seek in the kitchen for something to eat. Although she presented these last remains of symptomatic behavior several times in the therapy sessions, its dynamic background has not been fully understood. Miss C. is a good student, although she usually starts late with learning, she always gets high marks. Several times she described an inner conflict between 'being sure' that she will be successful with her way to prepare for the tests and a 'bad conscience' that she does not learn as much as her fellow students; therefore the therapist invites her to enact these inner split, as this psychic state is termed in EFT, in a two-chair dialogue. When the patient enacts the 'bad conscience,' she becomes harsh, accusatory and predicts that one day her way to prepare for examinations will lead to catastrophic failure; when she then is asked to change in the seat, that represents her confidence that her way of learning will lead to success, the self confidence has melted away and she *experiences* fear and shame induced by the threat that 'someday' she will fail. In the next session the patient presents associations linking her formally idealized father with the harsh inner critic who questions that her line of learning will lead to success."

In psychoanalytic terms, introjected aspects of a demanding and pejorative father had contributed to an overly rigorous super-ego, that attacked the self-consciousness of the patient and led to states of heightened tension in conflict-specific situations like learning for examinations. Several times the therapist in her interpretations had tried to link the deep disappointment of the patient if her marks in examinations were not among the very best, although she simultaneously declared that she felt no sense in being always in the top ranks, with hidden expectations of the parents, especially of her beloved father, but the patient had stoically denied that the father did expect anything from her. As far as the patient had not (re)experienced in depth the shame and the fear that were related to the prospect of failing in an examination, memories and actual experiences of the humiliation she had to endure if she did not live up to her father's expectations were not consciously accessible. The *active stimulation* of the *experiencing of the emotions* that underlay the tension states that came up when she started to learn for her tests led out of a therapeutic impasse and allowed the working through of crucial aspects of super-ego pathology. It has to be noted that the emotional activation as pivotal point of the therapeutic interaction may

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change the relationship between patient and therapist; in sessions that integrate EFT-elements in psychodynamic psychotherapy the therapist does not only follow the associations of the patient, but does actively introduce emotion-provoking procedures. To reflect the meaning of this role-change for the interplay of transference and counter-transference will be crucial for an integration of techniques that foster emotional experiencing into psychodynamic psychotherapy.

5. Discussion

The renewed interest in the significance of emotion for process and outcome of psychotherapy has been spawned by new insights in the affective neurosciences and in psychotherapy research. Research has substantiated the crucial role of emotion as fundamentally adaptive processes enabling the organism to process complex information rapidly and automatically in order to evaluate the degree to which needs, values and goals are met in the interaction with the environment (Lane et al. 2015a). One of the basic change processes in psychotherapy refers to the updating of painful emotional memories which have been guiding maladaptive responses. Based on findings from memory research we must presume that psychotherapy changes emotional memories in a sequence of reactivating emotional memories, engaging in new experiences that are incorporated via reconsolidation and reinforcing the new memory structure (akin to working through in psychoanalysis; Lane et al. 2015a). Consistent with this model, mounting evidence from psychotherapy research has shown that a certain level of emotional arousal is required to achieve successful treatment outcome. Regardless of the therapeutic approach, in order to be modified, emotional memories need to be activated first – and this requires emotional experiencing. It is not sufficient to think or talk about adverse experience, but related fear, anger, shame, etc. need to be felt in the session.

Clinical lore in psychodynamic psychotherapy has considered affect an important indicator of therapeutic change. However, we found in videotaped treatments from recent treatment trials (Beutel et al. 2013, 2014; Leichsenring et al. 2013), that psychodynamic psychotherapists often find it difficult to focus on salient emotions, to respond in a timely manner and to maintain the level of emotional arousal conducive to change. This may have contributed to our finding in an RCT comparing the efficacy of psychodynamic and cognitive-behavioral short-term interventions for panic disorder; there we found across both treatment conditions that patients with low levels of emotional awareness benefitted less from the interventions, patients with low emotional awareness were predominate in the psychodynamic condition (Beutel et al. 2013).

Reviewing the psychoanalytic clinical and treatment models of emotions, we found that technical advice to assess, access and process emotion has been limited (we discussed some important exceptions notably in the treatment of personality pathology). In line with clinical theory that regards repressed psychic contents as blends of thoughts, wishes, fantasies, intentions and feelings, the three basic thera-

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peutic interventions in psychodynamic psychotherapy – clarification, confrontation, interpretation – are meant to deal equally with all components of psychic contents. Lotterman has made an important point in drawing the clinician's attention to the patient's emotional response which he considers a highly informative and reliable diagnostic indicator of his or her mental state (cf. Davanloo 1990; Johansson et al. 2014). However, his recommendation "simply to listen" (p. 324) once emotion has become conscious may be insufficient to deepen the experiencing of the upcoming emotion, both in regard to its quality (differentiation of "felt feelings") and to its quantity (maintaining a sufficient level of arousal to allow the activation of prior emotional memory as a precondition for its reconsolidation, enriched by new emotional experience in the here and now).

In line with the theoretical framework provided by Lane et al. (2015a) we want to stress that effective psychotherapeutic work with emotions requires the activation of emotional experiencing and of the cognitive processing of this experience by symbolizing and reflecting on the emotions in order to give them meaning. The recommendations of Lane et al. (2015a) on working with emotions seem to be paraphrasing "working through" from a psychodynamic point of view, but there is a marked difference. While these authors stress the significance of the fostering and working through emotional *experiences* for the achievement of psychotherapeutic change, in the working through of central conflicts related to the patient's psychopathology psychodynamic clinical theory and treatment technique regard emotion as one psychic content among others (e.g., thoughts and phantasies). However, we have to stress that we are referring to time-limited, conflict- and/or symptom centered psychodynamic psychotherapy. In psychoanalysis proper, the (re)activation of relational themes in the transference is strongly connected to emotional experiencing, in the working through of relational episodes, that activate both the patient (transference) and the analyst (countertransference) emotionally, central emotional memories can be enriched by new emotional experiences and reconsolidated.

But if it is not possible to offer a patient a high frequent and long-term treatment, establishing a therapeutic bond that makes micro-changes in the emotional climate, achieved by empathic listening and mirroring as in the Bionian and Kohutian approaches touching and meaningful, it must be questioned, if insight is a sufficient precondition for emotional change. This may be especially true for patients who have not reached the developmental level of explicit emotional awareness, but have rather been stuck experiencing affect activation as physical arousal or action tendencies; even if they gain insight in maladaptive patterns in relating to significant others (e.g., in the CCRT-method), this may not lead to the experiencing of the hurtful emotions that motivated their defensive ways to relate to others. However, specific techniques are needed in order to assess, access and work with emotions in order to change emotional memory by psychodynamic psychotherapy; in the light of the recent findings in psychotherapy research and the affective neurosciences these techniques should help patients to become aware of, experience, symbolize and transform emotions. In contrast to the sometimes eclectic modifications by the experiential approaches (e.g., Fosha 2000; McCullough et al. 2003), the model of Lane et al. (2015a)

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provides a framework of therapeutic change compatible with basic psychoanalytic assumptions.

Consistent with this model, helping patients to activate and reconsolidate emotional memory as a precondition of psychotherapeutic change requires specific psychotherapeutic concepts and techniques. Emotion Focused Therapy provides an empirically validated and stringent model of assessment and treatment strategies for accessing, deepening and symbolizing emotions (Greenberg 2014). EFT specifies diagnostic markers and procedures to work with emotions which can be taught and applied in the context of psychodynamic psychotherapies.

As our case vignette illustrates, we propose to include techniques into psychodynamic therapy that actively stimulate and deepen emotional experience, which may then be further analyzed and integrated based on biographical reconstruction. As in other psychodynamic short-term psychotherapies, the significance of the active role of the therapist needs to be reflected with regard to the transference. While we have no empirical data, yet, from the basic assumptions and clinical experience we would advocate pursuing a fruitful dialogue on integrating emotion focused strategies into psychodynamic psychotherapy.

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Humanistic Therapy

Humanistic therapy is a mental health approach that emphasizes the importance of being your true self in order to lead the most fulfilling life.

It's based on the principle that everyone has their own unique way of looking at the world. This view can impact your choices and actions.

Humanistic therapy also involves a core belief that people are good at heart and capable of making the right choices for themselves. If you don't hold yourself in high regard, it's harder to develop your full potential.

How does it work?

Humanistic therapy involves better understanding your world view and developing true self-acceptance.

This is accomplished partially through the development of unconditional positive regard, both from others and from yourself. When you believe that others only respect you if you act a certain way, it's easy to fall into the trap of constantly feeling like you aren't enough.

This feeling of worthlessness, in turn, can negatively impact how you view both yourself and the world around you. Remember, according to the underlying principles of humanistic therapy, how you view yourself and the world around you has a big impact on your thoughts and actions.

Humanistic therapy can help you to both develop self-acceptance and overcome criticism or disapproval from others by offering a safe space to work toward personal growth. There are ways of doing this, which we'll go over later.

How does it compare to other types of therapy?

Humanistic therapy differs from more traditional approaches, such as psychoanalysis or behavioral therapy.

To start, humanistic therapy tends to focus more on your current day-to-day life. This is very different from other approaches that tend to focus on your past experiences, including those you might not be aware of.

Similarly, humanistic therapy also focuses more on helping the individual as a whole, rather than treating a specific diagnosis. A humanistic therapist will often do this through active listening. This means that they'll listen carefully to your words, making sure they fully understand what you're saying. They may stop you to ask follow-up questions.

Humanistic therapists work from the idea that you are the expert in your difficulties. They'll support the direction you take each session, trusting you to know what you need to talk about in order to work through the things bringing you to therapy.

What are some examples of humanistic therapy?

Humanistic therapies include a number of approaches. Three of the most common are Gestalt therapy, client-centered therapy, and existential therapy.

Gestalt therapy

In Gestalt therapy, your personal experiences are key, along with describing what you're going through in your own words. It's based on an underlying theory that unresolved conflicts with others — including family members or romantic partners — lead to distress.

Gestalt therapy provides a state of "safe emergency" where you can explore, in the present moment, the things bothering you. For example, you might explore the belief that your opinions don't matter to your partner.

Therapists help create the "here and now" atmosphere by asking what you're currently aware of or how certain emotions make you feel. You might use a range of techniques to do this, including:

- role-playing
- exaggerating a behavior
- reenacting a scenario

For example, you might be asked to visualize a person you're having a conflict with sitting in an empty chair across from you. Then, you'll carry out a conversation as if the person were actually sitting there.

Client-centered therapy

Also known as person-centered therapy and Rogerian therapy, this approach is considered the main type of humanistic therapy.

It's based on the idea that absorbing criticism or disapproval from others can distort the way you see yourself. This blocks personal growth and prevents you from living a fulfilling life, which in turn leads to mental distress.

As the name suggests, it also places a lot of focus on developing a strong client-therapist relationship.

A client-centered therapist will unconditionally accept you, even if they disagree with some aspect of your behavior. Feeling accepted in therapy, no matter what you share, can help you avoid holding back out of fear of disapproval.

You'll guide the direction of therapy while your therapist listens without judgement.

Existential therapy

Existential therapy draws more from philosophy than most other approaches to mental health treatment. The goal of this approach is to help you understand how your existence — the concept of you as a whole person — affects your unique worldview.

Existential therapists help you understand and explore the meaning you give to things that happen in your life. With their guidance, you'll learn to accept responsibility for choices you make and realize the freedom you have to make changes that will give your life greater meaning.

Like other humanistic approaches, existential therapy is mainly concerned with the issues you currently face, rather than things from your past. But it does

consider how your thoughts — conscious or unconscious — impact your mental health and goals.

Who's a good candidate for humanistic therapy?

Humanistic therapy is worth a shot if you're looking for ways to make your life more fulfilling, regardless of whether you have an underlying mental health condition. It's also worth considering if you've previously had trouble building a rapport with therapists.

A 2002 review of 86 studies found that humanistic therapies were effective at helping people make lasting change over time. People in humanistic therapy showed more change than people in no therapy at all, according to the review.

People in other types of therapy showed similar amounts of change, suggesting it's more about finding a type of therapy that you enjoy and will commit to doing.

In addition, a 2013 review of existing research suggests that client-centered approaches can be helpful for:

- trauma
- relationship difficulties
- psychosis
- depression
- coping with chronic health issues

However, it wasn't quite as effective as cognitive behavioral therapy for addressing anxiety and panic disorder.

Whether a humanistic approach fits your needs can depend on what you want to get out of therapy. Humanistic therapies typically don't make diagnosis a priority and may not work toward specific treatment goals.

If you have specific symptoms or behaviors you'd like to address or are seeking therapy with a clear goal for diagnosis and treatment, you might find a different approach more helpful. On the other hand, it may be a good fit if you're simply feeling "stuck" or in a rut.

Keep in mind that other types of therapy often incorporate aspects of humanistic therapy, such as unconditional acceptance and active listening.

How do I find a humanistic therapist?

It's not always easy to find the right therapist. When looking for a humanistic therapist, start by considering what you'd like to work on. This can be a specific issue or a more abstract concept.

The bottom line

Humanistic therapy is a type of mental health treatment that centers around your unique experience and perspective. Humanistic therapists offer empathy, genuine concern for you and your experience, and unconditional positive regard.

While it might not be the best option for getting a concrete mental health diagnosis, it can be a good option if you're simply looking for ways to lead a more meaningful life.

What Is Humanistic Therapy?

Humanistic therapy is an umbrella term for a group of talk therapies based on the principles of humanism, which relates to human nature and potential. Individuals (not their challenges) are the primary focus. They're seen as highly valuable, unique, and capable of achieving their goals.[1] The therapist isn't there to diagnose, problem-solve, or lead; instead, they're an equal partner on a journey of meaningful growth.

The emphasis of humanistic therapy is on individual choice, freedom, values, and meaning to help people find fulfillment and purpose. By living meaningfully in the present (a concept known as mindfulness), people can break free from past experiences or concerns for the future. This allows them to act with purpose and passion for their own benefit and the benefit of society.[3]

A humanistic therapist sees people as being:

- Inherently good
- Deserving of unconditional positive regard (respect, empathy, and understanding for who they are rather than judgement)
- Motivated to meet their own needs and grow
- Capable of solving problems and making necessary changes to overcome obstacles
- Driven to self-actualize (to define personal meaning, take purposeful action, and use their unique strengths and talents to live to their full potential)
- Complete and whole: more than a collection of random experiences and problems

Types of talk therapy that fall under the category of humanistic therapy are:

- Person-centered therapy
- Existential therapy
- Logotherapy
- Gestalt therapy
- Constructivist therapy
- Narrative therapy
- Feminist therapy
- Transpersonal psychotherapy
- Transactional analysis

How Does Humanistic Therapy Compare to Other Therapy Types?

Unlike many types of therapy, humanistic counseling doesn't address problems directly. Instead, the emphasis is on the supportive relationship between counselor and client, allowing people to safely explore who they are, what is meaningful to them, and how to live to their fullest potential. Little emphasis is placed on someone's past, including on how it may contribute to current difficulties.

Humanistic therapy is unique in that it is considered non-directive. This means that the therapist doesn't lead the process, suggest goals, teach tools and techniques, or even

decide how many sessions a client needs. Instead, they guide clients to understand themselves and discover their own meaning and fulfillment

Humanistic therapy doesn't typically involve a diagnosis or formal assessments, as it is not a structured approach to problem-solving like cognitive behavioral therapy (CBT), dialectical behavior therapy (DBT), or solution-focused brief therapy. It does, however, share some commonalities with positive psychology in that it promotes optimistic ideals that see people as fundamentally good, strong, and capable.

What Can Humanistic Counseling Help With?

Because it is tailored to the unique needs of each person, humanistic counseling can help with a wide variety of difficulties. Regardless of how they're labeled, humanism believes that the challenges people face are rooted in existential anxieties such as freedom, meaninglessness, and the inevitability of death. Humanistic therapy can also help those dealing with existential depression, brought about by similar anxieties and dreads about the meaning of life.

Accordingly, a humanistic therapist can help with these issues:[2,3,6]

- Anxiety
- Panic
- Depression
- Obsessive-compulsive disorder (OCD)
- Post-traumatic stress disorder (PTSD)
- Substance use
- Personality disorders
- Relationship problems
- Low self-esteem

Whether humanistic therapy is the right approach for psychotic disorders such as schizophrenia remains controversial. Some studies indicate that it can be helpful while others, including some conducted by Carl Rogers, the founder of person-centered therapy, have found that it's not appropriate.[2,3] Humanistic therapy can likely help people who live with psychotic disorders find meaning and fulfillment, but it isn't meant for periods of active psychosis.

Is Humanistic Therapy Effective?

Because it is unstructured and focuses mainly on the supportive counselor-client relationship, humanistic therapy is difficult to research, but not impossible. A meta-analysis of 86 studies indicated that people receiving this type of therapy experienced more positive change than people who didn't receive therapy at all, and about as much change as people in other types of therapy. Further, gains made in humanistic therapy tended to last over time.[7]

Specific types of humanistic counseling, such as person-centered therapy, have been found to be effective for interpersonal and relationship issues, and as effective as CBT for depression, substance use, and chronic medical conditions. However, therapies that merely claim to be "non-directive" (just one element of humanistic therapy) are less effective.

Criticisms of Humanistic Therapy

Some mental health experts question the quality of the studies supporting humanistic therapy.[4] Random controlled trials that compare a research group to a control group are considered the gold standard in psychological research, but this type of study is nearly impossible to apply to humanistic therapy because its key elements are difficult to measure.[3]

Instead, much of the research into humanistic therapy involves quantitative investigations (gathering data via questionnaires, polls, and surveys and analyzing it numerically) and qualitative studies (case studies, conversations, video, and audio recordings) to understand how the process does or doesn't help.[4] These forms of research are considered less valid and reliable than randomized controlled trials.

Other criticisms relate to the nature of humanistic therapy itself.[3] Some people believe that humanistic counselors provide too little structure, direction, and guidance. It's been criticized for its inability (or unwillingness) to address practical problems that people face, such as prejudice and poverty. Some have even accused it of being elitist, tailored to people who are highly articulate and have unlimited time and money to spend on therapy.

Humanistic Therapy Examples

A humanistic approach to therapy is about the equal relationship between counselor and client, with the therapist seeking to listen deeply and practice genuine empathy. Conversations aren't about identifying and fixing problems but exploring the client as a whole person and helping them reach their potential. Accordingly, therapy is highly individualized.

Here are three examples of humanistic therapy:

1. Humanistic Counseling For Depression

Instead of seeking to uncover the cause of depression or cope with symptoms, the therapist and client will likely reflect on what might bring greater meaning to the client's life. The therapist might use what's called dereflection to help the client focus on something other than their depression. The client will explore how to meet their true potential by identifying their unique strengths and talents. They will also likely explore ways of experiencing wonder and awe to increase motivation, energy, and engagement with life.

2. Humanistic Therapy For Anxiety

Humanists see existential anxiety as an inevitable part of life that restricts potential, causing people to avoid seeking meaning and fulfilment due to fear of loss or failure. While it doesn't completely disappear, people are capable of choosing to live with courage and meaning in spite of anxiety. During therapy, the therapist might use paradoxical intention to exaggerate rather than suppress feelings of anxiety so the client can see that they're capable of facing their fears.

3. Humanistic Therapy For Guilt

Many humanists believe that a sense of guilt underlies the challenges that cause people to seek therapy. They also believe that this guilt may appear as a sense of general unwellness, discontent, regret, dread, or despair over things someone has or hasn't done.[3] Humanistic therapists view guilt as an opportunity for growth and positive action. Rather than dwelling on issues from the past, the therapist will focus on how the client can move out of a place of unease into a more meaningful life.

Cost of Humanistic Therapy

Humanistic therapy costs about the same as traditional talk therapy. Fees are set by each individual therapist based on their education, experience, and geographic location. As of 2018, the median cost of mental health therapy in the United States was $125 for a 45-minute session and $130 for a 60-minute session.[8]

Mental health therapy is increasingly covered by insurance; however, many policies require an official diagnosis and specific treatment. Because humanistic therapies don't tend to assign diagnoses, some insurance companies may not cover it. Call the number on the back of your insurance card or visit their website to learn what your policy covers.

How to Find a Humanistic Therapist

There are different ways to find a therapist, including asking your doctor for a referral, checking with trusted friends or family members, looking at local mental health organizations, or using an online directory. If you are searching for a humanistic therapist, look for key terms in their biography or on their website like "non-directive," "client-centered," "unconditional positive regard," "full-potential," and "self-actualization." When in doubt, call and ask if they take a humanistic approach.[2]

Many factors are involved in choosing a therapist who is right for you. Above all, make sure that you feel comfortable. It's important that you feel like you can trust them and that they value you as a person. All humanistic therapists strive for this, but you want to make sure that your personalities are a good fit.

What to Expect at Your First Appointment

You won't dive into problems at your first appointment, and you probably won't be given a mental health assessment.[3] Instead, you and your therapist will get to know each other.

They may ask you gentle questions about your hopes, goals, interests, beliefs, relationships, and passions.. Above all, your therapist will listen to you and treat you with empathy and respect.[2,3]

History of Humanistic Psychology

Humanism draws from the philosophies of ancient Greece and many Eastern cultures.[4] In 15th century Europe, with the onset of the protestant reformation and the Renaissance, humanism's focus on reason, rational thought, and exploration coupled with the belief that every individual person has worth was a reaction to the strict control of the Catholic Church and monarchies during Medieval times.[9]

Similarly, in the 1950s, psychologists began to break away from Freudian psychoanalysis and staunch behaviorism, considered the first and second forces of psychology, respectively.[9] Many professionals disagreed with the psychoanalysts' pessimistic view of human nature and belief that humans were driven by unconscious urges and desires. They were also unhappy with what they believed to be the behaviorists' emphasis on manipulating and controlling human behavior through punishment and reward.

Psychologists like Abraham Maslow, Carl Rogers, Rollo May, Irvin Yalom, and Viktor Frankl had different ideas about human nature and helping people thrive; these ideas were built on humanism. From their research, a third force in psychology arose, the approach known collectively as humanistic therapy.[2,5]

Final Thoughts on Humanistic Therapy

If you're experiencing any type of mental health challenge, working with a therapist can be invaluable. If you're not concerned about receiving a diagnosis and would like to focus on discovering who you are at your core so you can use your unique strengths and talents to feel fulfillment and meaning despite life's inevitable problems, humanistic therapy may be the right approach for you.

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in individual, couples, child, or family therapy. *Humanistic Psychotherapies: Handbook of Research and Practice, Second Edition* is unique in its review of the humanistic psychotherapy research literature from its inception. It illustrates clearly how to become a more evidence-based therapist while retaining one's core humanistic principles and values.

DEFINING HUMANISTIC PSYCHOTHERAPIES

The reader may fairly ask, "What defines therapy as humanistic?" The term *humanistic*, as used here, is understood to incorporate approaches variously defined as experiential, existential, relational, and phenomenological. Therapies that come under the humanistic umbrella share a number of core variables and emphases. They include

- an optimistic view of clients as resourceful and naturally inclined to grow and develop their potential.
- a belief that the quality of the relationship between therapist and client provides a safe sanctuary that supports the therapeutic process and that is growth inducing in and of itself. The primary endeavor of humanistic therapists, therefore, is the creation of an optimal therapeutic relationship that is individualized to adapt to clients' needs as they evolve throughout the course of therapy.
- the core role of therapist empathy in grasping the personal meaning of the client's experience, facilitating self-exploration and setting in motion a self-reflective process that promotes intrapersonal and interpersonal learning that leads to fresh perspectives and more effective ways of living.
- a phenomenological emphasis that focuses on clients' subjective world with the understanding that their current perception is reality and can best be understood by putting aside hypotheses, diagnoses, and preconceptions in an attempt to take in their experience as freshly as possible.
- a strong emphasis on the critical role emotion plays in both psychopathology and psychological health with an understanding of the adaptive role emotion plays in effective decision making, emotion regulation, and effective functioning.
- a focus on the self and self-concept of the client with the understanding that the way clients experience the self has a great influence on their behavior. Enhancing self-exploration, self-definition, reconstrual of the self, and the development of

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self-knowledge and self-efficacy are primary endeavors of the humanistic therapist.

- a view that meaning is not a given but is constructed from the raw data of experience, including one's culture, values, perspectives, and personal history. Humanistic therapists strive to enable their clients to grasp the larger meanings and purposes of their lives.
- a view that people are essentially free to choose the manner and course of their lives and their attitude toward events and that freedom, choice, and responsibility are interwoven and inescapable.
- an understanding that all people must confront existential givens and the challenges they present, including (a) life versus death; (b) freedom, choice, and responsibility; (c) isolation versus connection; (d) meaning versus meaninglessness; (e) their "thrownness" (forces or events beyond one's control); (f) capacity for awareness; (g) anxiety as an inevitable aspect of the human condition; and (h) the fact that they are essentially embodied.
- a holistic view of people as unique, indivisible organisms composed of interrelated systems that cannot be reduced to the sum of their parts. People are conceived as inseparable from their environment and are therefore best understood as beings-in-the-world.

RESEARCH SUPPORT FOR HUMANISTIC PSYCHOTHERAPIES

In the past few decades, the research on humanistic psychotherapies has continued to accumulate. The effectiveness of all major branches of humanistic psychotherapy continues to be validated by new research carried out since the first edition of *Humanistic Psychotherapies* was published. The research supporting person-centered therapies and child therapy remains strong, and the evidence for focusing-oriented therapy, Gestalt therapy, and existential therapies is growing at a more modest pace. The good news is that Gestalt and existential scholar–practitioners have begun to embrace and produce considerably more research than in previous decades.

More recently, the integrative movement in psychotherapy has gained momentum and influence. As noted above, many of the core concepts and styles of practice common to humanistic psychotherapies have increasingly been incorporated into mainstream psychotherapies. This is especially so in terms of other therapeutic approaches incorporating an emotion focus in their work, with an understanding that core beliefs are fundamentally embedded

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in emotion schemes. Furthermore, we are now beginning to understand that therapeutic learning is likely to be more integrated and lasting when it is experienced emotionally.

Other areas in which humanistic psychotherapies have made an impact include an emphasis on therapist presence, relational depth, a focus on the self and self-concept of the client, and more reliance on the client's resourcefulness for change. The positive psychology movement is clearly grounded in humanistic premises and values. Almost every major school of psychotherapy now understands and embraces the critical importance of Rogers' core therapist conditions of empathy, acceptance, and genuineness in facilitating client growth. Emphases on phenomenology and working in the subjective world of the client have also been embraced in mainstream psychotherapy.

Humanistic psychotherapists have also increasingly integrated concepts and methods from many therapeutic approaches. While remaining true to their core values and beliefs, humanistic therapies have incorporated aspects of other therapies in a manner that retains the integrity of the model used. This means that clients are viewed as capable partners with whom the therapist collaborates to make therapeutic adjustments and to create an optimal relationship and style of therapy that best fits their needs. This approach is guided by research of the past few decades that has clearly shown that client involvement is the best predictor of a good outcome.

Although it is likely that individual schools of psychotherapy will be maintained and taught in their pure forms, it is clear that most therapists will increasingly use concepts and methods from other sources to supplement their work and to meet the needs of clients who are not responding optimally to the core model. Emotion-focused therapy, while essentially grounded in person-centered therapy, has integrated aspects of experiential, Gestalt, and existential therapies. Therefore, it may be considered one of the first humanistic–integrative therapies. Similarly, integrated models of person-centered and existential–humanistic and Gestalt therapies have become more prominent in the past decade.

Developments in the field of humanistic psychotherapy have demonstrated that it has a renewed impact on the practice of psychotherapy that is far reaching. Kirk Schneider and Alfried Längle (2012) edited a special section of the journal *Psychotherapy* that made the case that

> the renewal of humanism is a viable and growing phenomenon among the leading areas of psychotherapy and that . . . humanism is (1) a foundational element of therapeutic effectiveness; (2) a pivotal (and needed) dimension of therapeutic training and (3) a critical contributor to social well-being. (p. 427)

Invited authors of this special section of *Psychotherapy* indicated that (a) humanistic psychotherapy has become widespread in Europe, (b) multiculturalism

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embraces many humanistic values, (c) humanistic values and practices have affected the practice of psychoanalytic and other schools of psychotherapy, (d) humanism serves as a common factor in effective psychotherapy, and (e) humanistic models of psychotherapy have increasingly been incorporated by the general field of psychotherapy. It can be fairly argued that many core elements of humanistic psychotherapy are vital aspects of all major approaches to psychotherapy.

OVERVIEW OF THIS BOOK

This second edition of *Humanistic Psychotherapies* gives us strong reasons for optimism. The facts are friendly. Seventy-five years of development in theory, practice, and research have demonstrated that humanistic approaches to psychotherapy are as effective or more effective than all other major approaches. Substantive growth in quantitative and qualitative research on humanistic psychotherapies has occurred over the past 25 years.

Each chapter in *Humanistic Psychotherapies* reviews research literature from 2000 to mid-2014 and also provides a brief summary of the research reviewed in the first edition. The research reviewed provides new evidence that supports the effectiveness of all major schools of humanistic therapies and humanistic variables for a wide range of problems.

Part I begins with a discussion of three core principles of humanistic psychotherapy that are key to creating positive change: relational connection and support, emotional expression and processing, and authenticity. Expanding on these fundamental concepts, Part II reviews the accumulating qualitative research that gives a glimpse into the inner world of psychotherapy as experienced by therapist and client while providing a close-to-the-bone sense of how therapy works. It also examines the continuing research support for empathy and working with clients' emotions as critical components of the psychotherapy process and future outcomes. Building on this research, Part III highlights how all major schools of humanistic psychotherapy (person-centered, Gestalt, focusing-oriented or experiential, existential, and emotion-focused therapy) have continued to evolve into more sophisticated and research-based forms of practice. Part IV demonstrates how various humanistic approaches for couples, families, and children, such as filial family therapy and child-centered play therapy, have been shown to be sufficiently effective to be considered empirically supported. Part V reviews the client and therapist variables that promote growth and the alleviation of psychopathology. Part VI ties everything together with a research-based synthesis of humanistic psychotherapies, compiling the best evidence we have into 20 proposed premises to promote the optimal practice of humanistic psychotherapy that will also benefit the larger field of psychotherapy.

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Regrettably, some areas of humanistic therapies could not be included in the second edition, primarily because of either space limitations or an inadequate research base. These include culture and diversity, the self and self-concept in therapy, group and transpersonal therapy, feminist therapies, and therapies for extreme forms of psychopathology. I hope that these areas will be given the coverage they deserve in future reviews of humanistic psychotherapy.

In a recent article (Angus, Watson, Elliott, Schneider, & Timulak, 2014) reviewing the humanistic psychotherapy literature over the past 25 years (1990–2015), the results are consistent with those reported in this book. The article makes a strong case that "qualitative and quantitative research findings, including meta-analyses, support the identification of [humanistic psychotherapy] approaches as evidence-based treatments for a variety of psychological conditions" (Angus et al., 2014, p. 1). They also indicate that many humanistic therapies meet the standard for evidence-based treatments for many problems. Therefore, the findings presented in *Humanistic Psychotherapies* are increasingly supported by other current reviews of the research literature. Fortunately, the rate of new studies is increasing and continues to provide useful evidence that guides and refines practice. Our future looks bright, and humanistic psychotherapists have good reason to feel confident that they are providing their clients with a high level of care that consistently leads to good outcomes.

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CHAPTER 5

Adlerian Therapy

Coauthored by Gerald Corey and James Robert Bitter

INTRODUCTION

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- Subjective Perception of Reality
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Alfred Adler



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ALFRED ADLER (1870–1937) grew up in a Vienna family of six boys and two girls. His younger brother died at a very young age in the bed next to Alfred. Adler's early childhood was not a happy time. He was sickly and very much aware of death. At age 4 he almost died of pneumonia. He heard the doctor tell his father that "Alfred is lost." Adler associated this time with his decision to become a physician. Because he was ill so much during the first few years of his life, Adler was pampered by his mother. He developed a trusting relationship with his father, but did not feel very close to his mother. He was extremely jealous of his older brother, Sigmund, which led to a strained relationship between the two during childhood and adolescence. When we consider Adler's strained relationship with Sigmund Freud, one cannot help but suspect that patterns from his early family constellation were repeated in this relationship with Freud.

Adler's early childhood experiences had an impact on the formation of his theory. Adler is an example of a person who shaped his own life as opposed to having it determined by fate. Adler was a poor student. His teacher advised his father to prepare Adler to be a shoemaker, but not much else. With determined effort Adler eventually rose to the top of his class. He went on to study medicine at the University of Vienna, entering private practice as an ophthalmologist, and then shifting to general medicine. He eventually specialized in neurology and psychiatry, and he had a keen interest in incurable childhood diseases.

Adler had a passionate concern for the common person and was outspoken about child-rearing practices, school reforms, and prejudices that resulted in conflict. He spoke and wrote in simple, nontechnical language so that the general population could understand and apply the principles of his approach in a practical way, which helped people meet the challenges of daily life. Adler's (1927/1959) *Understanding Human Nature* was the first major psychology book to sell hundreds of thousands of copies in the United States. After serving in World War I as a medical officer, Adler created 32 child guidance clinics in the Vienna public schools and began training teachers, social workers, physicians, and other professionals. He pioneered the practice of teaching professionals through live demonstrations with parents and children before large audiences, now called "open-forum" family counseling. The clinics he founded grew in number and in popularity, and he was indefatigable in lecturing and demonstrating his work.

Although Adler had an overcrowded work schedule most of his professional life, he still took some time to sing, enjoy music, and be with friends. In the mid-1920s he began lecturing in the United States, and he later made frequent visits and tours. He ignored the warning of his friends to slow down, and on May 28, 1937, while taking a walk before a scheduled lecture in Aberdeen, Scotland, Adler collapsed and died of heart failure.

If you have an interest in learning more about Adler's life, see Edward Hoffman's (1996) excellent biography, *The Drive for Self*.

INTRODUCTION

Along with Freud and Jung, Alfred Adler was a major contributor to the initial development of the psychodynamic approach to therapy. After 8 to 10 years of collaboration, Freud and Adler parted company, with Freud taking the position that Adler was a heretic who had deserted him. Adler resigned as president of the Vienna Psychoanalytic Society in 1911 and founded the Society for Individual Psychology in 1912. Freud then asserted that it was not possible to support Adlerian concepts and still remain in good standing as a psychoanalyst.

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Later, a number of other psychoanalysts deviated from Freud's orthodox position (see Chapter 4). These Freudian revisionists—including Karen Horney, Erich Fromm, and Harry Stack Sullivan—agreed that relational, social, and cultural factors were of great significance in shaping personality. Even though these three therapists are typically called neo-Freudians, it would be more appropriate, as Heinz Ansbacher (1979) has suggested, to refer to them as neo-Adlerians because they moved away from Freud's biological and deterministic point of view and toward Adler's social-psychological and teleological (or goal-oriented) view of human nature.

Adler stresses the unity of personality, contending that people can only be understood as integrated and complete beings. This view also espouses the purposeful nature of behavior, emphasizing that where we have come from is not as important as where we are striving to go. Adler saw humans as both the creators and the creations of their own lives; that is, people develop a unique style of living that is both a movement toward and an expression of their selected goals. In this sense, we create ourselves rather than merely being shaped by our childhood experiences.

After Adler's death in 1937, Rudolf Dreikurs was the most significant figure in bringing Adlerian psychology to the United States, especially as its principles applied to education, individual and group therapy, and family counseling. Dreikurs is credited with giving impetus to the idea of child guidance centers and to training professionals to work with a wide range of clients.



See the DVD program for Chapter 5, *DVD for Theory and Practice of Counseling and Psychotherapy: The Case of Stan and Lecturettes*. I suggest that you view the brief lecture for each chapter prior to reading the chapter.

KEY CONCEPTS

View of Human Nature

Adler abandoned Freud's basic theories because he believed Freud was excessively narrow in his emphasis on biological and instinctual determination. Adler believed that the individual begins to form an approach to life somewhere in the first 6 years of living. He focused on the person's past as perceived in the present and how an individual's interpretation of early events continued to influence that person's present behavior. According to Adler, humans are motivated primarily by social relatedness rather than by sexual urges; behavior is purposeful and goal-directed; and consciousness, more than unconsciousness, is the focus of therapy. Adler stressed choice and responsibility, meaning in life, and the striving for success, completion, and perfection. Adler and Freud created very different theories, even though both men grew up in the same city in the same era and were educated as physicians at the same university. Their individual and distinct childhood experiences, their personal struggles, and the populations with whom they worked were key factors in the development of their particular views of human nature (Schultz & Schultz, 2009).

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Adler's theory starts with a consideration of inferiority feelings, which he saw as a normal condition of all people and as a source of all human striving. Rather than being considered a sign of weakness or abnormality, **inferiority feelings** can be the wellspring of creativity. They motivate us to strive for mastery, success (superiority), and completion. We are driven to overcome our sense of inferiority and to strive for increasingly higher levels of development (Ansbacher & Ansbacher, 1956/1964). Indeed, at around 6 years of age our fictional vision of ourselves as perfect or complete begins to form into a life goal. The life goal unifies the personality and becomes the source of human motivation; every striving and every effort to overcome inferiority is now in line with this goal.

From the Adlerian perspective, human behavior is not determined solely by heredity and environment. Instead, we have the capacity to interpret, influence, and create events. Adler asserted that genetics and heredity are not as important as what we choose to do with the abilities and limitations we possess. Although Adlerians reject a deterministic stance, they do not go to the other extreme and maintain that individuals can become whatever they want to be. Adlerians recognize that biological and environmental conditions limit our capacity to choose and to create.

Adlerians put the focus on reeducating individuals and reshaping society. Adler was the forerunner of a subjective approach to psychology that focuses on internal determinants of behavior such as values, beliefs, attitudes, goals, interests, and the individual perception of reality. He was a pioneer of an approach that is holistic, social, goal oriented, systemic, and humanistic. Adler also was the first systemic therapist: he maintained that it is essential to understand people within the systems in which they live.

Subjective Perception of Reality

Adlerians attempt to view the world from the client's subjective frame of reference, an orientation described as **phenomenological**. Paying attention to the individual way in which people perceive their world, referred to as "subjective reality," includes the individual's perceptions, thoughts, feelings, values, beliefs, convictions, and conclusions. Behavior is understood from the vantage point of this subjective perspective. From the Adlerian perspective, objective reality is less important than how we interpret reality and the meanings we attach to what we experience.

As you will see in subsequent chapters, many contemporary theories have incorporated this notion of the client's subjective worldview as a basic factor explaining behavior, including existential therapy, person-centered therapy, Gestalt therapy, the cognitive behavioral therapies, reality therapy, feminist therapy, and the postmodern approaches.

Unity and Patterns of Human Personality

Adler chose the name **Individual Psychology** (from the Latin *individuum,* meaning indivisible) for his theoretical approach because he wanted to avoid reductionism. Adler emphasized the unity and indivisibility of the person and stressed understanding the whole person in the context of his or her life—how all dimensions of a person are interconnected components, and how all of these components

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are unified by the individual's movement toward a life goal. This **holistic concept** implies that we cannot be understood in parts; rather, all aspects of ourselves must be understood in relationship (Carlson & Englar-Carlson, 2008). The focus is on understanding whole persons within their socially embedded contexts of family, culture, school, and work. We are social, creative, decision-making beings who act with purpose and cannot be fully known outside the contexts that have meaning in our lives (Sherman & Dinkmeyer, 1987).

The human personality becomes unified through development of a life goal. An individual's thoughts, feelings, beliefs, convictions, attitudes, character, and actions are expressions of his or her uniqueness, and all reflect a plan of life that allows for movement toward a self-selected life goal. An implication of this *holistic view of personality* is that the client is an integral part of a social system. There is more emphasis on interpersonal relationships than on the individual's internal psychodynamics.

BEHAVIOR AS PURPOSEFUL AND GOAL ORIENTED Individual Psychology assumes that all human behavior has a purpose. The concept of the purposeful nature of behavior is perhaps the cornerstone of Adler's theory. Adler replaced deterministic explanations with teleological (purposive, goal-oriented) ones. A basic assumption of Individual Psychology is that we can only think, feel, and act in relation to our goal; we can be fully understood only in light of knowing the purposes and goals toward which we are striving. Although Adlerians are interested in the future, they do not minimize the importance of past influences. They assume that most decisions are based on the person's experiences, on the present situation, and on the direction in which the person is moving—with the latter being the most important. They look for continuity by paying attention to themes running through a person's life.

Adler was influenced by the philosopher Hans Vaihinger (1965), who noted that people often live by fictions (or views of how the world should be). Many Adlerians use the term **fictional finalism** to refer to an imagined life goal that guides a person's behavior. It should be noted, however, that Adler ceased using this term and replaced it with "guiding self-ideal" and "goal of perfection" to account for our striving toward superiority or perfection (Watts & Holden, 1994). Very early in life, we begin to envision what we might be like if we were successful, complete, whole, or perfect. Applied to human motivation, a guiding self-ideal might be expressed in this way: "Only when I am perfect can I be secure" or "Only when I am important can I be accepted." The guiding self-ideal represents an individual's image of a goal of perfection, for which he or she strives in any given situation. Because of our subjective final goal, we have the creative power to choose what we will accept as truth, how we will behave, and how we will interpret events.

STRIVING FOR SIGNIFICANCE AND SUPERIORITY Adler stressed that the recognition of inferiority feelings and the consequent striving for perfection or mastery are innate (Ansbacher & Ansbacher, 1979)—they are two sides of the same coin. To understand human behavior, it is essential to grasp the ideas of basic inferiority and compensation. From our earliest years, we recognize that we are helpless in many ways, which is characterized by feelings of inferiority.

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This inferiority is not a negative factor in life. According to Adler, the moment we experience inferiority we are pulled by the striving for superiority. He maintained that the goal of success pulls people forward toward mastery and enables them to overcome obstacles. The goal of superiority contributes to the development of human community. However, it is important to note that "superiority," as used by Adler, does not necessarily mean superiority over others. Rather, it means moving from a perceived lower (or minus) position to a perceived better (or plus) position. People cope with feelings of helplessness by striving for competence, mastery, and perfection. They can seek to change a weakness into a strength, for example, or strive to excel in one area to compensate for defects in other areas. The unique ways in which people develop a style of striving for competence is what constitutes individuality or lifestyle. The manner in which Adler reacted to his childhood and adolescent experiences is a living example of this aspect of his theory.

LIFESTYLE The movement from a felt minus to a desired plus results in the development of a life goal, which in turn unifies the personality and the individual's core beliefs and assumptions. These core beliefs and assumptions guide each person's movement through life and organize his or her reality, giving meaning to life events. Adler called this life movement the individual's "lifestyle." Synonyms for this term include "plan of life," "style of life," "strategy for living," and "road map of life." **Lifestyle** includes the connecting themes and rules of interaction that give meaning to our actions. Lifestyle is often described as our perceptions regarding self, others, and the world. It includes an individual's characteristic way of thinking, acting, feeling, living, and striving toward long-term goals (Mosak & Maniacci, 2011).

Adler saw us as actors, creators, and artists. Understanding one's lifestyle is somewhat like understanding the style of a composer: "We can begin wherever we choose: every expression will lead us in the same direction—toward the one motive, the one melody, around which the personality is built" (Adler, as cited in Ansbacher & Ansbacher, 1956/1964, p. 332).

People are viewed as adopting a proactive, rather than a reactive, approach to their social environment. Although events in the environment influence the development of personality, such events are not the causes of what people become; rather, it is our interpretation of these events that shape personality. Faulty interpretations may lead to mistaken notions in our private logic, which will significantly influence present behavior. Once we become aware of the patterns and continuity of our lives, we are in a position to modify those faulty assumptions and make basic changes. We can reframe childhood experiences and *consciously* create a new style of living.

Social Interest and Community Feeling

Social interest and community feeling (*Gemeinschaftsgefühl*) are probably Adler's most significant and distinctive concepts (Ansbacher, 1992). These terms refer to individuals' awareness of being part of the human community and to individuals' attitudes in dealing with the social world.

Social interest is the action line of one's community feeling, and it involves being as concerned about others as one is about onself. This concept involves the

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capacity to cooperate and contribute (Milliren & Clemmer, 2006). Social interest requires that we have enough contact with the present to make a move toward a meaningful future, that we are willing to give and to take, and that we develop our capacity for contributing to the welfare of others and striving for the betterment of humanity. The socialization process associated with social interest begins in childhood and involves helping children to find a place in society and acquire a sense of belonging, as well as the ability to make a contribution (Kefir, 1981). While Adler considered social interest to be innate, he also believed that it must be learned, developed, and used.

Adler equated social interest with a sense of identification and empathy with others: "to see with the eyes of another, to hear with the ears of another, to feel with the heart of another" (as cited in Ansbacher & Ansbacher, 1979, p. 42). Social interest is the central indicator of mental health. Those with social interest tend to direct the striving toward the healthy and socially useful side of life. From the Adlerian perspective, as social interest develops, feelings of inferiority and alienation diminish. People express social interest through shared activity and mutual respect.

Individual Psychology rests on a central belief that our happiness and success are largely related to this social connectedness. Because we are embedded in a society, and indeed in the whole of humanity, we cannot be understood in isolation from that social context. We are primarily motivated by a desire to belong. **Community feeling** embodies the feeling of being connected to all of humanity—past, present, and future—and to being involved in making the world a better place. Those who lack this community feeling become discouraged and end up on the useless side of life. We seek a place in the family and in society to fulfill basic needs for security, acceptance, and worthiness. Many of the problems we experience are related to the fear of not being accepted by the groups we value. If our sense of belonging is not fulfilled, anxiety is the result. Only when we feel united with others are we able to act with courage in facing and dealing with our problems (Adler, 1938/1964).

Adler taught that we must successfully master three universal life tasks: building friendships (social task), establishing intimacy (love–marriage task), and contributing to society (occupational task). All people need to address these tasks, regardless of age, gender, time in history, culture, or nationality. Each of these tasks requires the development of psychological capacities for *friendship* and *belonging*, for *contribution* and *self-worth*, and for *cooperation* (Bitter, 2006). These basic life tasks are so fundamental to human living that impairment in any one of them is often an indicator of a psychological disorder (American Psychiatric Association, 2000). More often than not, when people seek therapy, it is because they are struggling unsuccessfully to meet one or more of these life tasks. The aim of therapy is to assist clients in modifying their lifestyle so they can more effectively navigate each of these tasks (Carlson & Englar-Carlson, 2008).

Birth Order and Sibling Relationships

The Adlerian approach is unique in giving special attention to the relationships between siblings and the psychological birth position in one's family. Adler identified five psychological positions, or vantage points, from which children

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tend to view life: oldest, second of only two, middle, youngest, and only. **Birth order** is not a deterministic concept but does increase an individual's probability of having a certain set of experiences. Actual birth order is less important than the individual's interpretation of his or her place in the family. Because Adlerians view most human problems as social in nature, they emphasize relationships within the family as our earliest and, perhaps, our most influential social system.

Adler (1931/1958) observed that many people wonder why children in the same family often differ so widely, and he pointed out that it is a fallacy to assume that children of the same family are formed in the same environment. Although siblings share aspects in common in the family constellation, the psychological situation of each child is different from that of the others due to birth order. The following description of the influence of birth order is based on Ansbacher and Ansbacher (1964), Dreikurs (1953), and Adler (1931/1958).

1. The *oldest child* generally receives a good deal of attention, and during the time she is the only child, she is typically somewhat spoiled as the center of attention. She tends to be dependable and hard working and strives to keep ahead. When a new brother or sister arrives on the scene, however, she finds herself ousted from her favored position. She is no longer unique or special. She may readily believe that the newcomer (or intruder) will rob her of the love to which she is accustomed. Most often, she reasserts her position by becoming a model child, bossing younger children, and exhibiting a high achievement drive.

2. The *second child* of only two is in a different position. From the time she is born, she shares the attention with another child. The typical second child behaves as if she were in a race and is generally under full steam at all times. It is as though this second child were in training to surpass the older brother or sister. This competitive struggle between the first two children influences the later course of their lives. The younger child develops a knack for finding out the elder child's weak spots and proceeds to win praise from both parents and teachers by achieving successes where the older sibling has failed. If one is talented in a given area, the other strives for recognition by developing other abilities. The second-born is often opposite to the firstborn.

3. The *middle child* often feels squeezed out. This child may become convinced of the unfairness of life and feel cheated. This person may assume a "poor me" attitude and can become a problem child. However, especially in families characterized by conflict, the middle child may become the switchboard and the peacemaker, the person who holds things together. If there are four children in a family, the second child will often feel like a middle child and the third will be more easygoing, more social, and may align with the firstborn.

4. The *youngest child* is always the baby of the family and tends to be the most pampered one. Because of being pampered or spoiled, he may develop helplessness into an art form and become expert at putting others in his service. Youngest children tend to go their own way, often developing in ways no others in the family have attempted and may outshine everyone.

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5. The *only child* has a problem of her own. Although she shares some of the characteristics of the oldest child (for example, a high achievement drive), she may not learn to share or cooperate with other children. She will learn to deal with adults well, as they make up her original familial world. Often, the only child is pampered by her parents and may become dependently tied to one or both of them. She may want to have center stage all of the time, and if her position is challenged, she will feel it is unfair.

Birth order and the interpretation of one's position in the family have a great deal to do with how adults interact in the world. Individuals acquire a certain style of relating to others in childhood and form a definite picture of themselves that they carry into their adult interactions. In Adlerian therapy, working with family dynamics, especially relationships among siblings, assumes a key role. Although it is important to avoid stereotyping individuals, it does help to see how certain personality trends that began in childhood as a result of sibling rivalry influence individuals throughout life.

THE THERAPEUTIC PROCESS

Therapeutic Goals

Adlerian counseling rests on a collaborative arrangement between the client and the counselor. In general, the therapeutic process includes forming a relationship based on mutual respect; a holistic *psychological investigation* or lifestyle assessment; and disclosing *mistaken goals* and *faulty assumptions* within the person's style of living. This is followed by a reeducation or reorientation of the client toward the useful side of life. The main aim of therapy is to develop the client's sense of belonging and to assist in the adoption of behaviors and processes characterized by community feeling and social interest. This is accomplished by increasing the client's self-awareness and challenging and modifying his or her fundamental premises, life goals, and basic concepts (Dreikurs, 1967, 1997). Milliren, Evans, and Newbauer (2007), identify this goal of Adlerian therapy: "to assist clients to understand their unique lifestyles . . . and to act in such a way as to meet the tasks of life with courage and social interest" (p. 145).

Adlerians do not view clients as being "sick" and in need of being "cured." They favor the growth model of personality rather than the medical model. As Mosak and Maniacci (2011) put it: "The Adlerian is interested not in curing sick individuals or a sick society but in reeducating individuals and in reshaping society" (p. 78). Rather than being stuck in some kind of pathology, Adlerians contend that clients are often discouraged. The counseling process focuses on providing information, teaching, guiding, and offering encouragement to discouraged clients. Encouragement is the most powerful method available for changing a person's beliefs, for it helps clients build self-confidence and stimulates courage. Courage is the willingness to act *even when fearful* in ways that are consistent with social interest. Fear and courage go hand in hand; without fear, there would be no need for courage. The loss of courage, or discouragement, results in mistaken and dysfunctional behavior. Discouraged people do not act in line with social interest.

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Adlerian counselors provide clients with an opportunity to view things from a different perspective, yet it is up to the clients to decide whether to accept an alternative perspective. Adlerians work collaboratively with clients to help them reach their self-defined goals. Adlerians educate clients in new ways of looking at themselves, others, and life. Through the process of providing clients with a new "cognitive map," a fundamental understanding of the purpose of their behavior, counselors assist them in changing their perceptions. Mosak and Maniacci (2011) lists these goals for the educational process of therapy:

- Fostering social interest
- Helping clients overcome feelings of discouragement and inferiority
- Modifying clients' views and goals—that is, changing their lifestyle
- Changing faulty motivation
- Encouraging the individual to recognize equality among people
- Helping people to become contributing members of society

Therapist's Function and Role

Adlerian counselors realize that clients can become discouraged and function ineffectively because of mistaken beliefs, faulty values, and useless or self-absorbed goals. These therapists operate on the assumption that clients will feel and behave better once they discover and correct their basic mistakes. Therapists tend to look for major mistakes in thinking and valuing such as mistrust, selfishness, unrealistic ambitions, and lack of confidence.

Adlerians assume a nonpathological perspective and thus do not label clients with pathological diagnoses. One way of looking at the role of Adlerian therapists is that they assist clients in better understanding, challenging, and changing their life story. "When individuals develop a life story that they find limiting and problem saturated, the goal is to free them from that story in favor of a preferred and equally viable alternative story" (Disque & Bitter, 1998, p. 434).

A major function of the therapist is to make a comprehensive assessment of the client's functioning. Therapists often gather information about the individual's style of living by means of a questionnaire on the client's **family constellation,** which includes parents, siblings, and others living in the home, life tasks, and early recollections. When summarized and interpreted, this questionnaire gives a picture of the individual's early social world. From this information on the family constellation, the therapist is able to get a perspective on the client's major areas of success and failure and on the critical influences that have had a bearing on the role the client has assumed in the world.

The counselor also uses early recollections as an assessment procedure. **Early recollections** (ERs) are defined as "stories of events that a person *says* occurred [one time] before he or she was 10 years of age" (Mosak & Di Pietro, 2006, p. 1). ERs are *specific* incidents that clients recall, along with the feelings and thoughts that accompanied these childhood incidents. These recollections are quite useful in getting a better understanding of the client (Clark, 2002). After these early recollections are summarized and interpreted, the therapist identifies some of the major successes and mistakes in the client's life. The aim is to provide a point of

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departure for the therapeutic venture. ERs are particularly useful as a functional assessment device because they indicate what clients do and how they think in both adaptive and maladaptive ways (Mosak & Di Pietro, 2006). The process of gathering early memories is part of what is called a **lifestyle assessment,** which involves learning to understand the goals and motivations of the client. When this process is completed, the therapist and the client have targets for therapy.

Mosak and Maniacci (2011) consider dreams to be a useful part of the assessment process. Freud assumed that dreams were wish fulfillment, or, in some instances, an attempt at solving an old problem; Adler, on the other hand, viewed dreams as a rehearsal for possible future actions. Just as early recollections reflect a client's long-range goals, dreams suggest possible answers to a client's present problems. In interpreting dreams, the therapist considers their purposive function. Mosak and Maniacci (2011) assert that "dreams serve as weather vanes for treatment, bringing problems to the surface and pointing to the patient's movement" (p. 88).

Client's Experience in Therapy

How do clients maintain their lifestyle, and why do they resist changing it? A person's style of living serves the individual by staying stable and constant. In other words, it is predictable. It is, however, also resistant to change throughout most of one's life. Generally, people fail to change because they do not recognize the errors in their thinking or the purposes of their behaviors, do not know what to do differently, and are fearful of leaving old patterns for new and unpredictable outcomes. Thus, even though their ways of thinking and behaving are not successful, they tend to cling to familiar patterns (Sweeney, 2009). Clients in Adlerian counseling focus their work on desired outcomes and a resilient lifestyle that can provide a new blueprint for their actions.

In therapy, clients explore what Adlerians call **private logic,** the concepts about self, others, and life that constitute the philosophy on which an individual's lifestyle is based. Private logic involves our convictions and beliefs that get in the way of social interest and that do not facilitate useful, constructive belonging (Carlson, Watts, & Maniacci, 2006). Clients' problems arise because the conclusions based on their private logic often do not conform to the requirements of social living. The heart of therapy is helping clients to discover the purposes of behaviors or symptoms and the basic mistakes associated with their personal coping. Learning how to correct faulty assumptions and conclusions is central to therapy.

To provide a concrete example, think of a chronically depressed middle-aged man who begins therapy. After a lifestyle assessment is completed, these basic mistakes are identified:

- He has convinced himself that nobody could really care about him.
- He rejects people before they have a chance to reject him.
- He is harshly critical of himself, expecting perfection.
- He has expectations that things will rarely work out well.
- He burdens himself with guilt because he is convinced he is letting everyone down.

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Even though this man may have developed these mistaken ideas about himself and life when he was young, he is still clinging to them as rules for living. His expectations, most of which are pessimistic, tend to be fulfilled because on some level he is seeking to validate his beliefs. Indeed, his depression will eventually serve the purpose of helping him avoid contact with others, a life task at which he expects to fail. In therapy, this man will learn how to challenge the structure of his private logic. In his case the syllogism goes as follows:

- "I am basically unlovable."
- "The world is filled with people who are likely to be rejecting."
- "Therefore, I must keep to myself so I won't be hurt."

This person holds onto several basic mistakes, and his private logic offers a psychological focus for treatment. A central theme or convictions in this client's life might be: "I must control everything in my life." "I must be perfect in everything I do."

It is easy to see how depression might follow from this thinking, but Adlerians also know that the depression serves as an excuse for this man's retreat from life. It is important for the therapist to listen for the underlying purposes of this client's behavior. Adlerians see feelings as being aligned with thinking and as the fuel for behaving. First we think, then we feel, and then we act. Because emotions and cognitions serve a purpose, a good deal of therapy time is spent in discovering and understanding this purpose and in reorienting the client toward effective ways of being. Because the client is not perceived by the therapist to be mentally ill or emotionally disturbed, but as mainly discouraged, the therapist will offer the client encouragement so that change is possible. Through the therapeutic process, the client will discover that he or she has resources and options to draw on in dealing with significant life issues and life tasks.

Relationship Between Therapist and Client

Adlerians consider a good client–therapist relationship to be one between equals that is based on cooperation, mutual trust, respect, confidence, collaboration, and alignment of goals. They place special value on the counselor's modeling of communication and acting in good faith. From the beginning of therapy, the relationship is a collaborative one, characterized by two persons working equally toward specific, agreed-upon goals. Adlerian therapists strive to establish and maintain an egalitarian therapeutic alliance and a person-to-person relationship with their clients. Developing a strong therapeutic relationship is essential to successful outcomes (Carlson et al., 2006). Dinkmeyer and Sperry (2000) maintain that at the outset of counseling clients should begin to formulate a plan, or contract, detailing what they want, how they plan to get where they are heading, what is preventing them from successfully attaining their goals, how they can change nonproductive behavior into constructive behavior, and how they can make full use of their assets in achieving their purposes. This therapeutic contract sets forth the goals of the counseling process and specifies the responsibilities of both therapist and client. Developing a contract is not a requirement of Adlerian therapy, but a contract can bring a tight focus to therapy.

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APPLICATION: THERAPEUTIC TECHNIQUES AND PROCEDURES

Adlerian counseling is structured around four central objectives that correspond to the four phases of the therapeutic process (Dreikurs, 1967). These phases are not linear and do not progress in rigid steps; rather, they can best be understood as a weaving that leads to a tapestry. These phases are as follows:

1. Establish the proper therapeutic relationship.
2. Explore the psychological dynamics operating in the client (an assessment).
3. Encourage the development of self-understanding (insight into purpose).
4. Help the client make new choices (reorientation and reeducation).

Dreikurs (1997) incorporated these phases into what he called *minor psychotherapy* in the context and service of holistic medicine. His approach to therapy has been elaborated in what is now called **Adlerian brief therapy**, or ABT (Bitter, Christensen, Hawes, & Nicoll, 1998). This way of working is discussed in the following sections.

Phase 1: Establish the Relationship

The Adlerian practitioner works in a collaborative way with clients, and this relationship is based on a sense of interest that grows into caring, involvement, and friendship. Therapeutic progress is possible only when there is an alignment of clearly defined goals between therapist and client. The counseling process, to be effective, must deal with the personal issues the client recognizes as significant and is willing to explore and change. The therapeutic efficacy in the later phases of Adlerian therapy is predicated upon the development and continuation of a solid therapeutic relationship during this first phase of therapy (Watts, 2000; Watts & Pietrzak, 2000).

Adlerian therapists focus on making person-to-person contact with clients rather than starting with "the problem." Clients' concerns surface rather quickly in therapy, but the initial focus should be on the person, not the problem. One way to create effective contact is for counselors to help clients become aware of their assets and strengths rather than dealing continually with their deficits and liabilities. During the initial phase, a positive relationship is created by listening; responding; demonstrating respect for clients' capacity to understand purpose and seek change; and exhibiting faith, hope, and caring. When clients enter therapy, they typically have a diminished sense of self-worth and self-respect. They lack faith in their ability to cope with the tasks of life. Therapists provide support, which is an antidote to despair and discouragement. For some people, therapy may be one of the few times in which they have truly experienced a caring human relationship.

Adlerians pay more attention to the subjective experiences of the client than they do to using techniques. They fit their techniques to the needs of each client. During the initial phase of counseling, the main techniques are attending and listening with empathy, following the subjective experience of the client as closely as possible, identifying and clarifying goals, and suggesting initial hunches about purpose in client's symptoms, actions, and interactions. Adlerian counselors are

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generally active, especially during the initial sessions. They provide structure and assist clients to define personal goals, they conduct psychological assessments, and they offer interpretations (Carlson et al., 2006). Adlerians attempt to grasp both the verbal and nonverbal messages of the client; they want to access the core patterns in the client's life. If the client feels deeply understood and accepted, the client is likely to focus on what he or she wants from therapy and thus establish goals. At this stage the counselor's function is to provide a wide-angle perspective that will eventually help the client view his or her world differently.

Phase 2: Explore the Individual's Psychological Dynamics

The aim of the second phase of Adlerian counseling is to get a deeper understanding of an individual's lifestyle. During this assessment phase, the focus is on the individual's social and cultural context. Rather than attempting to fit clients into a preconceived model, Adlerian practitioners allow salient cultural identity concepts to emerge in the therapy process, and these issues are then addressed (Carlson & Englar-Carlson, 2008). This assessment phase proceeds from two interview forms: the *subjective interview* and the *objective interview* (Dreikurs, 1997). In the **subjective interview**, the counselor helps the client to tell his or her life story as completely as possible. This process is facilitated by a generous use of empathic listening and responding. Active listening, however, is not enough. The subjective interview must follow from a sense of wonder, fascination, and interest. What the client says will spark an interest in the counselor and lead, naturally, to the next most significant question or inquiry about the client and his or her life story. Indeed, the best subjective interviews treat clients as experts in their own lives, allowing clients to feel completely heard. Throughout the subjective interview, the Adlerian counselor is listening for clues to the purposive aspects of the client's coping and approaches to life. "The subjective interview should extract patterns in the person's life, develop hypotheses about what works for the person, and determine what accounts for the various concerns in the client's life" (Bitter et al., 1998, p. 98). Toward the end of this part of the interview, Adlerian brief therapists ask, "Is there anything else you think I should know to understand you and your concerns?"

An initial assessment of the purpose that symptoms, actions, or difficulties serve in a person's life can be gained from what Dreikurs (1997) calls "The Question." Adlerians often end a subjective interview with this question: "How would your life be different, and what would you be *doing* differently, if you did not have this symptom or problem?" Adlerians use this question to help with differential diagnosis. More often, the symptoms or problems experienced by the client help the client avoid something that is perceived as necessary but from which the person wishes to retreat, usually a life task: "If it weren't for my depression, I would get out more and see my friends." Such a statement betrays the client's concern about the possibility of being a good friend or being welcomed by his or her friends. "I need to get married, but how can I with these panic attacks?" indicates the person's worry about being a partner in a marriage. Depression can serve as the client's solution when faced with problems in relationships. If a client reports that nothing would be different, especially with physical symptoms, Adlerians suspect that the problem may be organic and require medical intervention.

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The **objective interview** seeks to discover information about (a) how problems in the client's life began; (b) any precipitating events; (c) a medical history, including current and past medications; (d) a social history; (e) the reasons the client chose therapy at this time; (f) the person's coping with life tasks; and (g) a lifestyle assessment. Mozdzierz and his colleagues (1986) describe the counselor as a "lifestyle investigator" during this phase of therapy. Based on interview approaches developed by Adler and Dreikurs, the lifestyle assessment starts with an investigation of the person's family constellation and early childhood history (Powers & Griffith, 1987; Shulman & Mosak, 1988). Counselors also interpret the person's early memories, seeking to understand the meaning that she or he has attached to life experiences. They operate on the assumption that it is the interpretations people develop about themselves, others, the world, and life that govern what they do. Lifestyle assessment seeks to develop a holistic narrative of the person's life, to make sense of the way the person copes with life tasks, and to uncover the private interpretations and logic involved in that coping. For example, if Jenny has lived most of her life in a critical environment, and now she believes she must be perfect to avoid even the appearance of failure, the assessment process will highlight the restricted living that flows from this perspective.

THE FAMILY CONSTELLATION Adler considered the family of origin as having a central impact on an individual's personality. Adler suggested that it was through the family constellation that each person forms his or her unique view of self, others, and life. Factors such as cultural and familial values, gender-role expectations, and the nature of interpersonal relationships are all influenced by a child's observation of the interactional patterns within the family. Adlerian assessment relies heavily on an exploration of the client's **family constellation**, including the client's evaluation of conditions that prevailed in the family when the person was a young child (family atmosphere), birth order, parental relationship and family values, and extended family and culture. Some of these questions are almost always explored:

- Who was the favorite child?
- What was your father's relationship with the children? Your mother's?
- Which child was most like your father? Your mother? In what respects?
- Who among the siblings was most different from you? In what ways?
- Who among the siblings was most like you? In what ways?
- What were you like as a child?
- How did your parents get along? In what did they both agree? How did they handle disagreements? How did they discipline the children?

An investigation of family constellation is far more comprehensive than these few questions, but these questions give an idea of the type of information the counselor is seeking. The questions are always tailored to the individual client with the goal of eliciting the client's perceptions of self and others, of development, and of the experiences that have affected that development.

EARLY RECOLLECTIONS As you will recall, another assessment procedure used by Adlerians is to ask the client to provide his or her earliest memories, including the age of the person at the time of the remembered events and the feelings or reactions

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associated with the recollections. Early recollections are one-time occurrences, usually before the age of 9, pictured by the client in clear detail. Adler reasoned that out of the millions of early memories we might have we select those special memories that project the essential convictions and even the basic mistakes of our lives. Early recollections are a series of small mysteries that can be woven together into a tapestry that leads to an understanding of how we view ourselves, how we see the world, what our life goals are, what motivates us, what we value and believe in, and what we anticipate for our future (Clark, 2002; Mosak & Di Pietro, 2006).

Early memories cast light on the "story of our life" because they represent metaphors for our current views. From the thousands of experiences we have before the age of 9, we tend to remember only 6 to 12 memories. By understanding why we retain these memories and what they tell us about how we see ourselves, others, and life in the present, it is possible to get a clear sense of our mistaken notions, present attitudes, social interests, and possible future behavior. Early recollections are specific instances that clients tell therapists, and they are very useful in understanding those who are sharing a story (Mosak & Di Pietro, 2006). Exploring early recollections involves discovering how mistaken notions based on faulty goals and values continue to create problems in a client's life.

To tap such recollections, the counselor might proceed as follows: "I would like to hear about your early memories. Think back to when you were very young, as early as you can remember (before the age of 10), and *tell me something that happened one time*." After receiving each memory, the counselor might also ask: "What part stands out to you? What was the most vivid part of your early memory? If you played the whole memory like a movie and stopped it at one frame, what would be happening? Putting yourself in that moment, what are you feeling? What's your reaction?" Three memories are usually considered a minimum to assess a pattern, and some counselors ask for as many as a dozen memories.

Adlerian therapists use early recollections as a projective technique (Clark, 2002; Hood & Johnson, 2007) and to (a) assess the client's convictions about self, others, life, and ethics; (b) assess the client's stance in relation to the counseling session and the counseling relationship; (c) verify the client's coping patterns; and (d) assess individual strengths, assets, and interfering ideas (Bitter et al., 1998, p. 99).

In interpreting these early recollections, Adlerians may consider questions such as these:

- What part does the client take in the memory? Is the client an observer or a participant?
- Who else is in the memory? What position do others take in relation to the client?
- What are the dominant themes and overall patterns of the memories?
- What feelings are expressed in the memories?
- Why does the client choose to remember this event? What is the client trying to convey?

INTEGRATION AND SUMMARY Once material has been gathered from both subjective and objective interviews with the client, integrated summaries of the data are developed. Different summaries are prepared for different clients, but

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common ones are a narrative summary of the person's subjective experience and life story; a summary of family constellation and developmental data; a summary of early recollections, personal strengths or assets, and interfering ideas; and a summary of coping strategies. The summaries are presented to the client and discussed in the session, with the client and the counselor together refining specific points. This information provides the client with the chance to discuss specific topics and to raise questions.

Mosak and Maniacci (2011) believe lifestyle can be conceived of as a personal mythology. People behave *as if* the myths were true because, for them, they *are true*. Mosak and Maniacci list five *basic mistakes* in what is essentially an integration of Adlerian psychology and cognitive behavioral theory: overgeneralizations, false or impossible goals of security, misperceptions of life and life's demands, minimization or denial of one's basic worth, and faulty values.

In addition to the concept of basic mistakes, Adlerian theory is useful in assisting clients to identify and examine some of their common fears. These fears include being imperfect, being vulnerable, being disapproved of, and suffering from past regrets (Carlson & Englar-Carlson, 2008).

The Student Manual that accompanies this textbook includes a concrete example of the lifestyle assessment as it is applied to the case of Stan. In *Case Approach to Counseling and Psychotherapy* (Corey, 2013, chap. 3), Drs. Jim Bitter and Bill Nicoll present a lifestyle assessment of another hypothetical client, Ruth.

Phase 3: Encourage Self-Understanding and Insight

During this third phase, Adlerian therapists interpret the findings of the assessment as an avenue for promoting self-understanding and insight. Mosak and Maniacci (2011) define **insight** as "understanding translated into constructive action" (p. 89). When Adlerians speak of insight, they are referring to an understanding of the motivations that operate in a client's life. Self-understanding is only possible when hidden purposes and goals of behavior are made conscious. Adlerians consider insight as a special form of awareness that facilitates a meaningful understanding within the therapeutic relationship and acts as a foundation for change. Insight is a means to an end, and not an end in itself. People can make rapid and significant changes without much insight.

Disclosure and well-timed interpretations are techniques that facilitate the process of gaining insight. **Interpretation** deals with clients' underlying motives for behaving the way they do in the here and now. Adlerian disclosures and interpretations are concerned with creating awareness of one's direction in life, one's goals and purposes, one's private logic and how it works, and one's current behavior.

Adlerian interpretations are suggestions presented tentatively in the form of open-ended questions that can be explored in the sessions. They are hunches or guesses, and they often begin with phrases such as "I could be wrong, but I am wondering if . . . ," "Could it be that . . . ," or "Is it possible that . . ." Because interpretations are presented in this manner, clients are not led to defend themselves, and they feel free to discuss and even argue with the counselor's hunches and impressions. Through this process, both counselor and client eventually come to understand the client's motivations, the ways in which these motivations are now

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contributing to the maintenance of the problem, and what the client can do to correct the situation. During this phase of therapy, the counselor helps the client understand the limitations of the style of life the client has chosen.

Phase 4: Reorientation and Reeducation

The final stage of the therapeutic process is the action-oriented phase known as reorientation and reeducation: putting insights into practice. This phase focuses on helping clients discover a new and more functional perspective. Clients are both encouraged and challenged to develop the courage to take risks and make changes in their life. During this phase, clients can choose to adopt a new style of life based on the insights they gained in the earlier phases of therapy.

Adlerians are interested in more than changes in behavior. **Reorientation** involves shifting rules of interaction, process, and motivation. These shifts are facilitated through changes in awareness, which often occur during the therapy session and which are transformed into action outside of the therapy office (Bitter & Nicoll, 2004). In addition, especially at this phase of therapy, Adlerians focus on reeducation (see the section on therapeutic goals).

In some cases, significant changes are needed if clients are to overcome discouragement and find a place for themselves in this life. More often, however, clients merely need to be reoriented toward the useful side of life. The useful side involves a sense of belonging and being valued, having an interest in others and their welfare, courage, the acceptance of imperfection, confidence, a sense of humor, a willingness to contribute, and an outgoing friendliness. The useless side of life is characterized by self-absorption, withdrawal from life tasks, self-protection, or acts against one's fellow human beings. People acting on the useless side of life become less functional and are more susceptible to psychopathology. Adlerian therapy stands in opposition to self-depreciation, isolation, and retreat, and it seeks to help clients gain courage and to connect to strengths within themselves, to others, and to life. Throughout this phase, no intervention is more important than encouragement.

THE ENCOURAGEMENT PROCESS Encouragement is the most distinctive Adlerian procedure, and it is central to all phases of counseling and therapy. It is especially important as people consider change in their lives. Encouragement literally means "to build courage." Courage develops when people become aware of their strengths, when they feel they belong and are not alone, and when they have a sense of hope and can see new possibilities for themselves and their daily living. **Encouragement** entails showing faith in people, expecting them to assume responsibility for their lives, and valuing them for who they are (Carlson et al., 2006). Carlson and Englar-Carlson (2008) note that encouragement involves acknowledging that life can be difficult, yet it is critical to instill a sense of faith in clients that they can make changes in life. Milliren, Evans, and Newbauer (2007) consider encouragement key in promoting and activating social interest. They add that encouragement is the universal therapeutic intervention for Adlerian counselors, that it is a fundamental attitude rather than a technique. Because clients often do not recognize or accept their positive qualities, strengths, or internal resources, one of the counselor's main tasks is to help them do so.

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Adlerians believe discouragement is the basic condition that prevents people from functioning, and they see encouragement as the antidote. As a part of the encouragement process, Adlerians use a variety of relational, cognitive, behavioral, emotional, and experiential techniques to help clients identify and challenge self-defeating cognitions, generate perceptional alternatives, and make use of assets, strengths, and resources (Ansbacher & Ansbacher, 1964; Dinkmeyer & Sperry, 2000; Watts & Pietrzak, 2000; Watts & Shulman, 2003).

Encouragement takes many forms, depending on the phase of the counseling process. In the relationship phase, encouragement results from the mutual respect the counselor seeks to engender. In the assessment phase, which is partially designed to illuminate personal strengths, clients are encouraged to recognize that they are in charge of their own lives and can make different choices based on new understandings. During reorientation, encouragement comes when new possibilities are generated and when clients are acknowledged and affirmed for taking positive steps to change their lives for the better.

CHANGE AND THE SEARCH FOR NEW POSSIBILITIES During the reorientation phase of counseling, clients make decisions and modify their goals. They are encouraged to act *as if* they were the people they want to be, which can serve to challenge self-limiting assumptions. Clients are asked to *catch themselves* in the process of repeating old patterns that have led to ineffective behavior. Commitment is an essential part of reorientation. If clients hope to change, they must be willing to set tasks for themselves in everyday life and do something specific about their problems. In this way, clients translate their new insights into concrete actions. Bitter and Nicoll (2004) emphasize that real change happens between sessions, and not in therapy itself. They state that arriving at a strategy for change is an important first step, and stress that it takes courage and encouragement for clients to apply what they have learned in therapy to daily living.

This action-oriented phase is a time for solving problems and making decisions. The counselor and the client consider possible alternatives and their consequences, evaluate how these alternatives will meet the client's goals, and decide on a specific course of action. The best alternatives and new possibilities are those generated by the client, and the counselor must offer the client a great deal of support and encouragement during this stage of the process.

MAKING A DIFFERENCE Adlerian counselors seek to make a difference in the lives of their clients. That difference may be manifested by a change in behavior or attitude or perception. Adlerians use many different techniques to promote change, some of which have become common interventions in other therapeutic models. Techniques that go by the names of immediacy, advice, humor, silence, paradoxical intention, acting as if, spitting in the client's soup, catching oneself, the push-button technique, externalization, reauthoring, avoiding the traps, confrontation, use of stories and fables, early recollection analysis, lifestyle assessment, encouraging, task setting and commitment, giving homework, and terminating and summarizing have all been used (Carlson & Slavik, 1997; Carlson et al., 2006; Dinkmeyer & Sperry, 2000; Disque & Bitter, 1998; Mosak & Maniacci, 2011; Mozdzierz, Peluso, & Lisiecki, 2009). Adlerian practitioners can creatively employ a wide range of other

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techniques, as long as these methods are philosophically consistent with the basic theoretical premises of Adlerian psychology (Milliren et al., 2007). Adlerians are pragmatic when it comes to using techniques that are appropriate for a given client. In general, however, Adlerian practitioners focus on motivation modification more than behavior change and encourage clients to make holistic changes on the useful side of living. All counseling is a cooperative effort, and making a difference depends on the counselor's ability to win the client's cooperation.

Areas of Application

Adler anticipated the future direction of the helping professions by calling upon therapists to become social activists and by addressing the prevention and remediation of social conditions that were contrary to social interest and resulted in human problems. Adler's pioneering efforts on prevention services in mental health led him to increasingly advocate for the role of Individual Psychology in schools and families. Because Individual Psychology is based on a growth model, not a medical model, it is applicable to such varied spheres of life as child guidance; parent–child counseling; couples counseling; family counseling and therapy; group counseling and therapy; individual counseling with children, adolescents, and adults; cultural conflicts; correctional and rehabilitation counseling; and mental health institutions. Adler's basic ideas have been incorporated into the practices of school psychology, school counseling, the community mental health movement, and parent education. Adlerian principles have been widely applied to substance abuse programs, social problems to combat poverty and crime, problems of the aged, school systems, religion, and business.

APPLICATION TO EDUCATION Adler (1930/1978) advocated training both teachers and parents in effective practices that foster the child's social interests and result in a sense of competence and self-worth. Adler had a keen interest in applying his ideas to education, especially in finding ways to remedy faulty lifestyles of schoolchildren. He initiated a process to work with students in groups and to educate parents and teachers. By providing teachers with ways to prevent and correct basic mistakes of children, he sought to promote social interest and mental health. Adler was ahead of his time in advocating for schools to take an active role in developing social skills and character education as well as teaching the basics. Many of the major teacher education models are based on principles of Adlerian psychology (see Albert, 1996). Besides Adler, the main proponent of Individual Psychology as a foundation for the teaching–learning process was Dreikurs (1968, 1971).

APPLICATION TO PARENT EDUCATION Parent education seeks to improve the relationship between parent and child by promoting greater understanding and acceptance. Parents are taught how to recognize the mistaken goals of children and to use logical and natural consequences to guide children toward more productive behavior. Adlerian parent education also stresses listening to children, helping children accept the consequences of their behavior, applying emotion coaching, holding family meetings, and using encouragement. Two of the leading parent education programs in the United States—STEP (Dinkmeyer & McKay, 1997) and Active Parenting (Popkin, 1993)—are based on Adlerian principles.

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APPLICATION TO COUPLES COUNSELING Adlerian therapy with couples is designed to assess a couple's beliefs and behaviors while educating them in more effective ways of meeting their relational goals. Clair Hawes has developed an approach to couples counseling within the Adlerian brief therapy model. In addition to addressing the compatibility of lifestyles, Hawes looks at the early recollections of the marriage and each partner's relationship to a broad set of life tasks, including occupation, social relationships, intimate relationships, spirituality, self-care, and self-worth (Bitter et al., 1998; Hawes, 1993; Hawes & Blanchard, 1993). Carlson, Watts, and Maniacci (2006) describe how Adlerians achieve the goals of brief couples therapy: they foster social interest, assist couples in decreasing feelings of inferiority and overcoming discouragement, help couples modify their views and goals, help couples to feel a sense of quality in their relationships, and provide skill-building opportunities. Therapists aim to create solutions for problems, increase choices of couples, and help clients discover and use their individual and collective resources.

The full range of techniques applicable to other forms of counseling can be used when working with couples. In couples counseling, couples are taught specific techniques that enhance communication and cooperation. Some of these techniques are listening, paraphrasing, giving feedback, having marriage conferences, listing expectations, doing homework, and enacting problem solving. Adlerians use psychoeducational methods and skills training in counseling couples. For useful books on this topic, see Carlson and Dinkmeyer (2003) and Sperry, Carlson, and Peluso (2006).

Adlerians will sometimes see clients as a couple, sometimes individually, and then alternately as a couple and as individuals. Rather than looking for who is at fault in the relationship, the therapist considers the lifestyles of the partners and the interaction of the two lifestyles. Emphasis is given to helping them decide if they want to maintain their relationship, and, if so, what changes they are willing to make.

APPLICATION TO FAMILY COUNSELING With its emphasis on the family constellation, holism, and the freedom of the therapist to improvise, Adler's approach contributed to the foundation of the family therapy perspective. Adlerians working with families focus on the family atmosphere, the family constellation, and the interactive goals of each member (Bitter, 2009). The family atmosphere is the climate characterizing the relationship between the parents and their attitudes toward life, gender roles, decision making, competition, cooperation, dealing with conflict, responsibility, and so forth. This atmosphere, including the role models the parents provide, influences the children as they grow up. The therapeutic process seeks to increase awareness of the interaction of the individuals within the family system. Those who practice Adlerian family therapy strive to understand the goals, beliefs, and behaviors of each family member and the family as an entity in its own right. Adler's and Dreikurs's influence on family therapy is covered in more depth in Chapter 14.

APPLICATION TO GROUP COUNSELING Adler and his coworkers used a group approach in their child guidance centers in Vienna as early as 1921 (Dreikurs, 1969). Dreikurs extended and popularized Adler's work with groups

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and used group psychotherapy in his private practice for more than 40 years. Although Dreikurs introduced group therapy into his psychiatric practice as a way to save time, he quickly discovered some unique characteristics of groups that made them an effective way of helping people change. Inferiority feelings can be challenged and counteracted effectively in groups, and the mistaken concepts and values that are at the root of social and emotional problems can be deeply influenced because the group is a value-forming agent (Sonstegard & Bitter, 2004).

The rationale for Adlerian group counseling is based on the premise that our problems are mainly of a social nature. The group provides the social context in which members can develop a sense of belonging, social connectedness, and community. Sonstegard and Bitter (2004) write that group participants come to see that many of their problems are interpersonal in nature, that their behavior has social meaning, and that their goals can best be understood in the framework of social purposes.

The use of early recollections is a unique feature of Adlerian group counseling. As mentioned earlier, from a series of early memories, individuals can get a clear sense of their mistaken notions, current attitudes, social interests, and possible future behavior. Through the mutual sharing of these early recollections, members develop a sense of connection with one another, and group cohesion is increased. The group becomes an agent of change because of the improved interpersonal relationships among members and the emergence of hope.

We particularly value the way Adlerian group counselors implement action strategies at each of the group sessions and especially during the reorientation stage when new decisions are made and goals are modified. To challenge self-limiting assumptions, members are encouraged to act *as if* they were the persons they want to be. They are asked to "catch themselves" in the process of repeating old patterns that have led to ineffective or self-defeating behavior. The members come to appreciate that if they hope to change, they need to set tasks for themselves, apply group lessons to daily life, and take steps in finding solutions to their problems. This final stage is characterized by group leaders and members working together to challenge erroneous beliefs about self, life, and others. During this stage, members are considering alternative beliefs, behaviors, and attitudes.

Adlerian group counseling can be considered a brief approach to treatment. The core characteristics associated with brief group therapy include rapid establishment of a strong therapeutic alliance, clear problem focus and goal alignment, rapid assessment, emphasis on active and directive therapeutic interventions, a focus on strengths and abilities of clients, an optimistic view of change, a focus on both the present and the future, and an emphasis on tailoring treatment to the unique needs of clients in the most time-efficient manner possible (Carlson et al., 2006).

Adlerian brief group therapy is addressed by Sonstegard, Bitter, Pelonis-Peneros, and Nicoll (2001). For more on the Adlerian approach to group counseling, refer to *Theory and Practice of Group Counseling* (Corey, 2012, chap. 7) and Sonstegard and Bitter (2004).

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ADLERIAN THERAPY FROM A MULTICULTURAL PERSPECTIVE

Strengths From a Diversity Perspective

Adlerian theory addressed social equality issues and social embeddedness of humans long before multiculturalism assumed central importance in the profession (Watts & Pietrzak, 2000). Adler introduced notions with implications toward multiculturalism that have as much or more relevance today as they did during Adler's time (Pedersen, as cited in Nystul, 1999b). Some of these ideas include (1) the importance of the cultural context, (2) the emphasis on health as opposed to pathology, (3) a holistic perspective on life, (4) the value of understanding individuals in terms of their core goals and purposes, (5) the ability to exercise freedom within the context of societal constraints, and (6) the focus on prevention and the development of a proactive approach in dealing with problems. Adler's holistic perspective is an articulate expression of what Pedersen calls a "culture-centered" or multicultural approach to counseling. Carlson and Englar-Carlson (2008) maintain that Adlerian theory is well suited to counseling diverse populations and doing social justice work. They assert: "Perhaps Adler's greatest contribution is that he developed a theory that recognizes and stresses the effects of social class, racism, sex, and gender on the behavior of individuals. His ideas, therefore, are well received by those living in today's global society" (p. 134).

Although the Adlerian approach is called Individual Psychology, its focus is on the person in a social context. Thus clients are encouraged to define themselves within their social environments. Adlerians allow broad concepts of age, ethnicity, lifestyle, sexual/affectional orientations, and gender differences to emerge in therapy. The therapeutic process is grounded within a client's culture and worldview rather than attempting to fit clients into preconceived models.

In their analysis of the various theoretical approaches to counseling, Arciniega and Newlon (2003) state that Adlerian theory holds a great deal of promise for addressing diversity issues. They note a number of characteristics of Adlerian theory that are congruent with the values of many racial, cultural, and ethnic groups, including the emphasis on understanding the individual in a familial and sociocultural context; the role of social interest and contributing to others; and the focus on belonging and the collective spirit. Cultures that stress the welfare of the social group and emphasize the role of the family will find the basic assumptions of Adlerian psychology to be consistent with their values.

Adlerian therapists tend to focus on cooperation and socially oriented values as opposed to competitive and individualistic values (Carlson & Carlson, 2000). Native American clients, for example, tend to value cooperation over competition. One such client told a story about a group of boys who were in a race. When one boy got ahead of the others, he would slow down and allow the others to catch up, and they all made it to the finish line at the same time. Although the coach tried to explain that the point of the race was for an individual to finish first, these boys were socialized to work together cooperatively as a group. Adlerian therapy is easily adaptable to cultural values that emphasize community.

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Not only is Adlerian theory congruent with the values of people from diverse cultural groups, but the approach offers flexibility in applying a range of cognitive and action-oriented techniques to helping clients explore their practical problems in a cultural context. Adlerian practitioners are not wedded to any particular set of procedures. Instead, they are conscious of the value of fitting their techniques to each client's situation. Although they utilize a diverse range of methods, most of them do conduct a lifestyle assessment. This assessment is heavily focused on the structure and dynamics within the client's family. Because of their cultural background, many clients have been conditioned to respect their family heritage and to appreciate the impact of their family on their own personal development. It is essential that counselors be sensitive to the conflicting feelings and struggles of their clients. If counselors demonstrate an understanding of these cultural values, it is likely that these clients will be receptive to an exploration of their lifestyle. Such an exploration will involve a detailed discussion of their own place within their family.

It should be noted that Adlerians investigate culture in much the same way that they approach birth order and family atmosphere. Culture is a vantage point from which life is experienced and interpreted; it is also a background of values, history, convictions, beliefs, customs, and expectations that must be addressed by the individual. Culture provides a way of grasping the subjective and experiential perspective of an individual. Although culture influences each person, it is expressed within each individual differently, according to the perception, evaluation, and interpretation of culture that the person holds. Contemporary Adlerians appreciate the role of spirituality and religion in the lives of clients because these factors are manifestations of social interest and responsibility to others (Carlson & Englar-Carlson, 2008).

Adler was one of the first psychologists at the turn of the century to advocate equality for women. He recognized that men and women were different in many ways, but he felt that the two genders were deserving of equal value and respect. This respect and appreciation for difference extends to culture as well as gender. Adlerians find in different cultures opportunities for viewing the self, others, and the world in multidimensional ways.

Shortcomings From a Diversity Perspective

As is true of most Western models, the Adlerian approach tends to focus on the self as the locus of change and responsibility. Because other cultures have different conceptions, this primary emphasis on changing the autonomous self may be problematic for many clients. Assumptions about the Western nuclear family are built into the Adlerian concepts of birth order and family constellation. For people brought up in extended family contexts, some of these ideas may be less relevant or at least may need to be reconfigured.

Adlerian theory has some potential drawbacks for clients from those cultures who are not interested in exploring past childhood experiences, early memories, family experiences, and dreams. This approach also has limited effectiveness with clients who do not understand the purpose of exploring the details of a lifestyle analysis when dealing with life's current problems (Arciniega & Newlon, 2003).

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In addition, the culture of some clients may contribute to their viewing the counselor as the "expert" and expecting that the counselor will provide them with solutions to their problems. For these clients, the role of the Adlerian therapist may pose problems because Adlerian therapists are not experts in solving other people's problems. Instead, they view it as their function to collaboratively teach people alternative methods of coping with life concerns.

Many clients who have pressing problems are likely to be hesitant to discuss areas of their lives that they may not see as connected to the struggles that bring them into therapy. Individuals may believe that it is inappropriate to reveal family information. On this point Carlson and Carlson (2000) suggest that a therapist's sensitivity and understanding of a client's culturally constructed beliefs about disclosing family information are critical. If the therapist is able to demonstrate an understanding of a client's cultural values, it is likely that this person will be more open to the assessment and treatment process.

Adlerian Therapy Applied to the Case of Stan



The basic aims of an Adlerian therapist working with Stan are fourfold and correspond to the four stages of counseling: (1) establishing and maintaining a good working relationship with Stan, (2) exploring Stan's dynamics, (3) encouraging Stan to develop insight and understanding, and (4) helping Stan see new alternatives and make new choices.

To develop mutual trust and respect, I pay close attention to Stan's subjective experience and attempt to get a sense of how he has reacted to the turning points in his life. During the initial session, Stan reacts to me as the expert who has the answers. He is convinced that when he makes decisions he generally ends up regretting the results. Stan approaches me out of desperation. Because I view counseling as a relationship between equals, I initially focus on his feeling of being unequal to most other people. A good place to begin is exploring his feelings of inferiority, which he says he feels in most situations. The goals of counseling are developed mutually, and I avoid deciding for Stan what his goals should be. I also resist giving Stan the simple formula he is requesting.

I prepare a lifestyle assessment based on a questionnaire that taps information about Stan's early years, especially his experiences in his family. (See the *Student Manual for Theory and Practice of Counseling and Psychotherapy* for a complete description of this lifestyle assessment form as it is applied to Stan.) This assessment includes a determination of whether he poses a danger to himself because Stan did mention suicidal ideation. During the assessment phase, which might take a few sessions, I explore with Stan his social relationships, his relationships with members of his family, his work responsibilities, his role as a man, and his feelings about himself. I place considerable emphasis on Stan's goals in life and his priorities. I do not pay a great deal of attention to his past, except to show him the consistency between his past and present as he moves toward the future.

As an Adlerian counselor, I place value on exploring early recollections as a source of understanding his goals, motivations, and values. I ask Stan to report his earliest memories. He replies as follows:

> *I was about 6. I went to school, and I was scared of the other kids and the teacher. When I came home, I cried and told my mother I didn't want to go back to school. She yelled at me and called me a baby. After that I felt horrible and even more scared.*

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Another of Stan's early recollections was at age 8:

> *My family was visiting my grandparents. I was playing outside, and some neighborhood kid hit me for no reason. We started fighting, and my mother came out and scolded me for being such a rough kid. She wouldn't believe me when I told her he started the fight. I felt angry and hurt that she didn't believe me.*

Based on these early recollections, I suggest that Stan sees life as frightening and unpredictably hostile and that he feels he cannot count on women; they are likely to be harsh, unbelieving, and uncaring.

Having gathered the data based on the lifestyle assessment about his family constellation and his early recollections, I assist Stan in the process of summarizing and interpreting this information. I give particular attention to identifying basic mistakes, which are faulty conclusions about life and self-defeating perceptions. Here are some of the mistaken conclusions Stan has reached:

- "I must not get close to people, because they will surely hurt me."
- "Because my own parents didn't want me and didn't love me, I'll never be desired or loved by anybody."
- "If only I could become perfect, maybe people would acknowledge and accept me."
- "Being a man means not showing emotions."

The information I summarize and interpret leads to insight and increased self-understanding on Stan's part. He gains increased awareness of his need to control his world so that he can keep painful feelings in check. He sees more clearly some of the ways he tries to gain control over his pain: through the use of alcohol, avoiding interpersonal situations that are threatening, and being unwilling to count on others for psychological support. Through continued emphasis on his beliefs, goals, and intentions, Stan comes to see how his private logic is inaccurate. In his case, a syllogism for his style of life can be explained in this way: (1) "I am unloved, insignificant, and do not count"; (2) "The world is a threatening place to be, and life is unfair"; (3) "Therefore, I must find ways to protect myself and be safe." During this phase of the process, I make interpretations centering on his lifestyle, his current direction, his goals and purposes, and how his private logic works. Of course, Stan is expected to carry out homework assignments that assist him in translating his insights into new behavior. In this way he is an active participant in his therapy.

In the reorientation phase of therapy, Stan and I work together to consider alternative attitudes, beliefs, and actions. By now Stan sees that he does not have to be locked into past patterns, feels encouraged, and realizes that he has the power to change his life. He accepts that he will not change merely by gaining insights and knows that he will have to make use of these insights by carrying out an action-oriented plan. Stan begins to feel that he can create a new life for himself and not remain the victim of circumstances.

Follow-Up: You Continue as Stan's Adlerian Therapist

Use these questions to help you think about how you would counsel Stan using an Adlerian approach:

- What are some ways you would attempt to establish a relationship with Stan based on trust and mutual respect? Can you imagine any difficulties in developing this relationship with him?
- What aspects of Stan's lifestyle particularly interest you? In counseling him, how would these be explored?
- The Adlerian therapist identified four of Stan's mistaken conclusions. Can you identify with any of these basic mistakes? If so, do you think this would help or hinder your therapeutic effectiveness with him?
- How might you assist Stan in discovering his social interest and going beyond a preoccupation with his own problems?
- What strengths and resources in Stan might you draw on to support his determination and commitment to change?



See the *DVD for Theory and Practice of Counseling and Psychotherapy: The Case of Stan and Lecturettes* (Session 3 on Adlerian therapy) for a demonstration of my approach to counseling Stan by focusing on his early recollections.

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SUMMARY AND EVALUATION

Summary

Adler was far ahead of his time, and most contemporary therapies have incorporated at least some of his ideas. Individual Psychology assumes that people are motivated by social factors; are responsible for their own thoughts, feelings, and actions; are the creators of their own lives, as opposed to being helpless victims; and are impelled by purposes and goals, looking more toward the future than back to the past.

The basic goal of the Adlerian approach is to help clients identify and change their mistaken beliefs about, self, others, and life and thus to participate more fully in a social world. Clients are not viewed as psychologically sick but as discouraged. The therapeutic process helps individuals become aware of their patterns and make some basic changes in their style of living, which lead to changes in the way they feel and behave. The role of the family in the development of the individual is emphasized. Therapy is a cooperative venture that challenges clients to translate their insights into action in the real world. Contemporary Adlerian theory is an integrative approach, combining cognitive, constructivist, existential, psychodynamic, and systems perspectives. Some of these common characteristics include an emphasis on establishing a respectful client–therapist relationship, an emphasis on clients' strengths and resources, and an optimistic and future orientation.

The Adlerian approach gives practitioners a great deal of freedom in working with clients. Major Adlerian contributions have been made in the following areas: elementary education, consultation groups with teachers, parent education groups, couples and family therapy, and group counseling.

Contributions of the Adlerian Approach

A strength of the Adlerian approach is its flexibility and its integrative nature. Adlerian therapists can be both theoretically integrative and technically eclectic (Watts & Shulman, 2003). This therapeutic approach allows for the use of a variety of relational, cognitive, behavioral, emotive, and experiential techniques. Adlerian therapists are resourceful and flexible in drawing on many methods, which can be applied to a diverse range of clients in a variety of settings and formats. Therapists are mainly concerned with doing what is in the best interests of clients rather than squeezing clients into one theoretical framework (Watts, 1999, 2000; Watts & Pietrzak, 2000; Watts & Shulman, 2003).

Another contribution of the Adlerian approach is that it is suited to brief, time-limited therapy. Adler was a proponent of time-limited therapy, and the techniques used by many contemporary brief therapeutic approaches are very similar to interventions created by or commonly used by Adlerian practitioners (Carlson et al., 2006). Adlerian therapy and contemporary brief therapy have in common a number of characteristics, including quickly establishing a strong therapeutic alliance, a clear problem focus and goal alignment, rapid assessment and application to treatment, an emphasis on active and directive intervention, a psychoeducational focus, a present and future orientation, a focus on clients' strengths and abilities

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and an optimistic expectation of change, and a time sensitivity that tailors treatment to the unique needs of the client (Carlson et al., 2006). According to Mosak and Di Pietro (2006), early recollections are a significant assessment intervention in brief therapy. They claim that early recollections are often useful in minimizing the number of therapy sessions. This procedure takes little time to administer and interpret and provides a direction for therapists to pursue.

Bitter and Nicoll (2000) identify five characteristics that form the basis for an integrative framework in brief therapy: time limitation, focus, counselor directiveness, symptoms as solutions, and the assignment of behavioral tasks. Bringing a time-limitation process to therapy conveys to clients the expectation that change will occur in a short period of time. When the number of sessions is specified, both client and therapist are motivated to stay focused on desired outcomes and to work as efficiently as possible. Because there is no assurance that a future session will occur, brief therapists tend to ask themselves this question: "If I had only one session to be useful in this person's life, what would I want to accomplish?" (p. 38).

The Adlerian concepts I (Jerry Corey) draw on most in my professional work are (1) the importance of looking to one's life goals, including assessing how these goals influence an individual; (2) the focus on the individual's interpretation of early experiences in the family, with special emphasis on their current impact; (3) the clinical use of early recollections in both assessment and treatment; (4) the use of dreams as rehearsals for future action; (5) the need to understand and confront basic mistakes; (6) the cognitive emphasis, which holds that emotions and behaviors are largely influenced by one's beliefs and thinking processes; (7) the idea of working out an action plan designed to help clients make changes; (8) the collaborative relationship, whereby the client and therapist work toward mutually agreed-upon goals; and (9) the emphasis given to encouragement during the entire counseling process. Several Adlerian concepts have implications for personal development. One of these notions that has helped me to understand the direction of my life is the assumption that feelings of inferiority are linked to a striving for superiority (Corey, 2010).

It is difficult to overestimate the contributions of Adler to contemporary therapeutic practice. Many of his ideas were revolutionary and far ahead of his time. His influence went beyond counseling individuals, extending into the community mental health movement (Ansbacher, 1974). Abraham Maslow, Viktor Frankl, Rollo May, Paul Watzlawick, Karen Horney, Erich Fromm, Aaron T. Beck, and Albert Ellis have all acknowledged their debt to Adler. Both Frankl and May see him as a forerunner of the existential movement because of his position that human beings are free to choose and are entirely responsible for what they make of themselves. This view also makes him a forerunner of the subjective approach to psychology, which focuses on the internal determinants of behavior: values, beliefs, attitudes, goals, interests, personal meanings, subjective perceptions of reality, and strivings toward self-realization. Bitter (2008) and his colleagues (Bitter, Robertson, Healey, & Cole, 2009) have drawn attention to the link between Adlerian thinking and feminist therapy approaches.

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One of Adler's most important contributions was his influence on other therapy systems. Many of his basic ideas have found their way into most of the other psychological schools, a few of which include existential therapy, cognitive behavior therapy, rational emotive behavior therapy, reality therapy, solution-focused therapy, and family therapy. In many respects, Adler seems to have paved the way for current developments in both the cognitive and constructivist therapies (Watts, 2003). Adlerians' basic premise is that if clients can change their thinking then they can change their feelings and behavior. A study of contemporary counseling theories reveals that many of Adler's notions have reappeared in these modern approaches with different nomenclature, and often without giving Adler the credit that is due to him (Watts, 1999; Watts & Pietrzak, 2000; Watts & Shulman, 2003). It is clear that there are significant linkages between Adlerian theory and most of the present-day theories, especially those that view the person as purposive, self-determining, and striving for growth. Carlson and Englar-Carlson (2008) assert that Adlerians face the challenge of continuing to develop their approach so that it meets the needs of contemporary global society: "Whereas Adlerian ideas are alive in other theoretical approaches, there is a question about whether Adlerian theory as a stand-alone approach is viable in the long term" (p. 133). These authors believe that for the Adlerian model to survive and thrive it will be necessary to find ways to strive for significance.

Limitations and Criticisms of the Adlerian Approach

Adler had to choose between devoting his time to formalizing his theory and teaching others the basic concepts of Individual Psychology. He placed practicing and teaching before organizing and presenting a well-defined and systematic theory. As a result, his written presentations are often difficult to follow, and many of them are transcripts of lectures he gave. Initially, many people considered his ideas somewhat loose and too simplistic.

Research supporting the effectiveness of Adlerian theory is limited but has improved over the last 25 years (Watts & Shulman, 2003). However, a large part of the theory still requires empirical testing and comparative analysis. This is especially true in the conceptual areas that Adlerians accept as axiomatic: for example, the development of lifestyle; the unity of the personality and an acceptance of a singular view of self; the rejection of the prominence of heredity in determining behavior, especially pathological behavior; and the usefulness of the multiple interventions used by various Adlerians.

WHERE TO GO FROM HERE

Refer to the *DVD for Integrative Counseling: The Case of Ruth and Lecturettes,* Session 6 ("Cognitive Focus in Counseling"), which illustrates Ruth's striving to live up to expectations and measure up to perfectionist standards. In this particular therapy session with Ruth, you will see how I draw upon cognitive concepts and apply them in practice.

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Free Podcasts for ACA Members

You can download ACA Podcasts (prerecorded interviews) by going to www.counseling.org and then clicking on the Resource button, and then the Podcast Series. For Chapter 5, look for Podcast 11, Adlerian Therapy, by Dr. Jon Carlson.

Other Resources

Videos from Psychotherapy.net demonstrate Adlerian therapy with adults, families, and children, and are available to students and professionals at their website, www.Psychotherapy.net. New articles, interviews, blogs, therapy cartoons, and videos are published monthly. For this chapter, see the following:

Carlson, J. (1997). *Adlerian Therapy* (Psychotherapy with the Experts Series)

Carlson, J. (2001). *Adlerian Parent Consultation* (Child Therapy with the Experts Series)

Kottman, T. (2001). *Adlerian Play Therapy* (Child Therapy with the Experts Series)

If your thinking is allied with the Adlerian approach, you might consider seeking training in Individual Psychology or becoming a member of the North American Society of Adlerian Psychology (NASAP). To obtain information on NASAP and a list of Adlerian organizations and institutes, contact:

North American Society of Adlerian Psychology (NASAP)
614 Old West Chocolate Avenue
Hershey, PA 17033
Telephone: (717) 579-8795
Fax: (717) 533-8616
E-mail: nasap@msn.com
Website: www.alfredadler.org

The society publishes a newsletter and a quarterly journal and maintains a list of institutes, training programs, and workshops in Adlerian psychology. *The Journal of Individual Psychology* presents current scholarly and professional research. Columns on counseling, education, and parent and family education are regular features. Information about subscriptions is available by contacting the society.

If you are interested in pursuing training, postgraduate study, continuing education, or a degree, contact NASAP for a list of Adlerian organizations and institutes. A few training institutes are listed here:

Adler School of Professional Psychology
65 East Wacker Place, Suite 2100
Chicago, IL 60601-7298
Telephone: (312) 201-5900
Fax: (312) 201-5917
E-mail: admissions@adler.edu
Website: www.adler.edu

Adler Graduate School
1550 East 78th Street
Richfield, MN 55423
Telephone: 612-861-7554
Fax: 612-861-7559
E-mail:info@alfredadler.edu

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Adler School of Professional Psychology, Vancouver Campus
595 Burrard Street, Suite 753
P.O. Box 49104
Vancouver, BC, Canada V7X 1G4
Telephone: (604) 482-5510
Fax: (604) 874-4634

Adlerian Training Institute, Inc.
Dr. Bill Nicoll, Coordinator
P.O. Box 881581
Port St. Lucie, FL 34988
Telephone/Fax: (772) 807-4141
Cell Phone: (954) 650-0637
E-mail: adleriantraining@aol.com
Website: www.adleriantraining.com

The Alfred Adler Institute of Northwestern Washington
2565 Mayflower Lane
Bellingham, WA 98226
Telephone: (360) 647-5670
E-mail: HTStein@att.net
Website: http://ourworld.compuserv.com/homepages/hstein/

Alfred Adler Institute of San Francisco
266 Bemis Street
San Francisco, CA 94131
Telephone: (415) 584-3833
E-mail: DPienkow@msn.com

The International Committee of Adlerian Summer Schools and Institutes
Michael Balla, ICASSI Administrator
257 Billings Avenue
Ottawa, Ontario, Canada K1H 5L1
Fax: (613) 733-0289
E-mail: mjballa@sympatico.ca
Website: www.icassi.net

Recommended Supplementary Readings

Adlerian Therapy: Theory and Practice (Carlson, Watts, & Maniacci, 2006) clearly presents a comprehensive overview of Adlerian therapy in contemporary practice. There are chapters on the therapeutic relationship, brief individual therapy, brief couples therapy, group therapy, play therapy, and consultation. This book lists Adlerian intervention videos that are available.

Adlerian Counseling and Psychotherapy: A Practitioner's Approach (Sweeney, 2009) is one of the most comprehensive books written on the wide range of Adlerian applications to therapy and wellness.

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Adlerian Psychotherapy: An Advanced Approach to Individual Psychology (Oberst & Stewart, 2003) is an up-to-date and in-depth presentation of Adlerian psychotherapeutic process, including chapters on family therapy and the relevance of this model to postmodern approaches.

Early Recollections: Interpretative Method and Application (Mosak & Di Pietro, 2006) is an extensive review of the use of early recollections as a way to understand an individual's dynamics and behavioral style. This book addresses the theory, research, and clinical applications of early recollections.

Understanding Life-Style: The Psycho-Clarity Process (Powers & Griffith, 1987) is a useful source of information for doing a lifestyle assessment. Separate chapters deal with interview techniques, lifestyle assessment, early recollections, the family constellation, and methods of summarizing and interpreting information.

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CHAPTER 6

Existential Therapy

INTRODUCTION

- Historical Background in Philosophy and Existentialism
- Key Figures in Contemporary Existential Psychotherapy

KEY CONCEPTS

- View of Human Nature
- Proposition 1: The Capacity for Self-Awareness
- Proposition 2: Freedom and Responsibility
- Proposition 3: Striving for Identity and Relationship to Others
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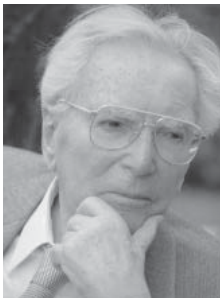
SUMMARY AND EVALUATION

- Summary
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WHERE TO GO FROM HERE

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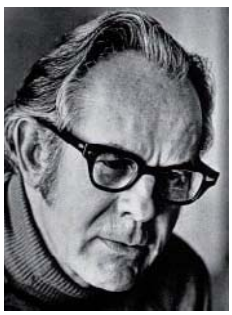
VIKTOR FRANKL (1905–1997) was born and educated in Vienna. He founded the Youth Advisement Centers there in 1928 and directed them until 1938. From 1942 to 1945 Frankl was a prisoner in the Nazi concentration camps at Auschwitz and Dachau, where his parents, brother, wife, and children died. He vividly remembered his horrible experiences in these camps, yet he was able to use them in a constructive way and did not allow them to dampen his love and enthusiasm for life. He traveled all around the world, giving lectures in Europe, Latin America, Southeast Asia, and the United States.

Frankl received his MD in 1930 and his PhD in philosophy in 1949, both from the University of Vienna. He became an associate professor at the University of Vienna and later was a distinguished speaker at the United States International University in San Diego. He was a visiting professor at Harvard, Stanford, and Southern Methodist universities. Frankl's works have been translated into more than 20 languages, and his ideas continue to have a major impact on the development of existential therapy. His compelling book *Man's Search for Meaning* (1963) has been a best-seller around the world.

Although Frankl had begun to develop an existential approach to clinical practice before his grim years in the Nazi death camps, his experiences there confirmed his views. Frankl (1963) observed and personally experienced the truths expressed by existential philosophers and writers who hold that we have choices in every situation. Even in terrible circumstances, he believed, we could preserve a vestige of spiritual freedom and independence of mind. He learned experientially that everything could be taken from a person except one thing: "the last of human freedoms—to choose one's attitude in any given set of circumstances, to choose one's own way" (p. 104).). Frankl believed that the essence of being human lies in searching for meaning and purpose. He believed that love is the highest goal to which humans can aspire and that our salvation is through love. We can discover this meaning through our actions and deeds, by experiencing a value (such as love or achievements through work), and by suffering.

Frankl knew and read Freud and attended some of the meetings of Freud's psychoanalytic group. Frankl acknowledged his indebtedness to Freud, although he disagreed with the rigidity of Freud's psychoanalytic system. Frankl often remarked that Freud was a *depth* psychologist and that he is a *height* psychologist who built on Freud's foundations. Reacting against most of Freud's deterministic notions, Frankl developed his own theory and practice of psychotherapy, which emphasized the concepts of freedom, responsibility, meaning, and the search for values. He established his international reputation as the founder of what has been called "The Third School of Viennese Psychoanalysis," the other two being Sigmund Freud's psychoanalysis and Alfred Adler's Individual Psychology.

I have selected Frankl as one of the key figures of the existential approach because of the dramatic way in which his theories were tested by the tragedies of his life. His life was an illustration of his theory, for he lived what his theory espouses.



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ROLLO MAY (1909–1994) first lived in Ohio and then moved to Michigan as a young child along with his five brothers and a sister. He remembered his home life as being unhappy, a situation that contributed to his interest in psychology and counseling. In his personal life May struggled with his own existential concerns and the failure of two marriages.

Despite his unhappy life experiences, he graduated from Oberlin College in 1930 and then went to Greece as a teacher. During his summers in Greece he traveled to Vienna to study with Alfred Adler. After receiving a degree in theology from Union Theological Seminary, May decided that the

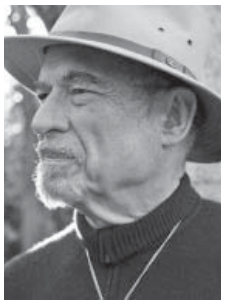
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best way to reach out and help people was through psychology instead of theology. He completed his doctorate in clinical psychology at Columbia University and started a private practice in New York; he also became a supervisory training analyst for the William Alanson Institute.

While May was pursuing his doctoral program, he came down with tuberculosis, which resulted in a 2-year stay in a sanitarium. During his recovery period, May spent much time learning firsthand about the nature of anxiety. He also spent time reading, and he studied the works of Søren Kierkegaard, which was the catalyst for his recognizing the existential dimensions of anxiety. This study resulted in his book *The Meaning of Anxiety* (1950). His popular book *Love and Will* (1969) reflects his own personal struggles with love and intimate relationships and mirrors Western society's questioning of its values pertaining to sex and marriage.

The greatest personal influence on Rollo May was the existential theologian Paul Tillich (author of *The Courage to Be,* 1952), who became his mentor and a personal friend. The two spent much time together discussing philosophical, religious, and psychological topics. Most of May's writings reflect a concern with the nature of human experience, such as recognizing and dealing with power, accepting freedom and responsibility, and discovering one's identity. He draws from his rich knowledge based on the classics and his existential perspective.

Rollo May was one of the main proponents of humanistic approaches to psychotherapy, and he was the principal American spokesperson of European existential thinking as it is applied to psychotherapy. He believed psychotherapy should be aimed at helping people discover the meaning of their lives and should be concerned with the problems of *being* rather than with problem solving. Questions of being include learning to deal with issues such as sex and intimacy, growing old, facing death, and taking action in the world. According to May, the real challenge is for people to be able to live in a world where they are alone and where they will eventually have to face death. He contends that our individualism should be balanced by what Adler refers to as social interest. It is the task of therapists to help individuals find ways to contribute to the betterment of the society in which they live.


Photo courtesy of Joseph Siroker.

IRVIN YALOM (b. 1931-) was born of parents who immigrated from Russia shortly after World War I. During his early childhood, Yalom lived in the inner city of Washington, D.C., in a poor neighborhood. Life on the streets was perilous, and Yalom took refuge indoors reading novels and other works. Twice a week he made the hazardous bicycle trek to the library to stock up on reading supplies. He found an alternative and satisfying world in reading fiction, which was a source of inspiration and wisdom to him. Early in his life he decided that writing a novel was the very finest thing a person could do, and subsequently he has written several teaching novels.

Irvin Yalom is professor emeritus of psychiatry at the Stanford University School of Medicine. A psychiatrist and author, Yalom has been a major figure in the field of group psychotherapy since publication in 1970 of his influential book *The Theory and Practice of Group Psychotherapy* (1970/2005), which has been translated into 12 languages and is currently in its fifth edition. His pioneering work, *Existential Psychotherapy,* written in 1980, is a classic and authoritative textbook on existential therapy. Drawing on his clinical experience and on empirical research, philosophy, and literature, Yalom developed an existential approach to psychotherapy that addresses four "givens of existence" or ultimate human concerns: freedom and responsibility, existential isolation, meaninglessness, and death. These existential themes deal with the client's existence, or being-in-the-world. Yalom urges all therapists, regardless of theoretical orientation, to develop a sensibility to existential issues because generally these issues emerge in all courses of therapy.

Psychotherapy has been endlessly intriguing for Yalom, who has approached all of his patients with a sense of wonderment at the stories they reveal. He believes that a different therapy must be

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designed for each client because each has a unique story. He advocates using the here and now of the therapeutic relationship to explore the client's interpersonal world, and he believes the therapist must be transparent, especially regarding his or her experience of the client. His basic philosophy is existential and interpersonal, which he applies to both individual and group therapy.

Irvin Yalom has authored many stories and novels related to psychotherapy, including *Love's Executioner* (1987), *When Nietzsche Wept* (1992), *Lying on the Couch* (1997), *Momma and the Meaning of Life* (2000), and *The Schopenhauer Cure* (2005a). His 2008 nonfiction book, *Staring at the Sun: Overcoming the Terror of Death,* is a treatise on the role of death anxiety in psychotherapy, illustrating how death and the meaning of life are foundational themes associated with in-depth therapeutic work. Yalom's works, translated into more than 20 languages, have been widely read by therapists and laypeople alike.

Yalom's wife, Marilyn, has a PhD in comparative literature and has had a successful career as a university professor and writer. His four children have chosen a variety of careers—medicine, photography, creative writing, theater directing, and clinical psychology. He has five grandchildren and is still counting.

INTRODUCTION

Existential therapy is more a way of thinking, or an attitude about psychotherapy, than a particular style of practicing psychotherapy. It is neither an independent nor separate school of therapy, nor is it a clearly defined model with specific techniques. Existential therapy can best be described as a *philosophical approach* that influences a counselor's therapeutic practice. Yalom and Josselson (2011) capture the essence of this approach:

> Existential psychotherapy is an attitude toward human suffering [that] has no manual. It asks deep questions about the nature of the human being and the nature of anxiety, despair, grief, loneliness, isolation, and anomie. It also deals centrally with the questions of meaning, creativity, and love. (p. 310)

Existential therapy focuses on exploring themes such as mortality, meaning, freedom, responsibility, anxiety, and aloneness as these relate to a person's current struggle. The goal of existential therapy is to assist clients in their exploration of the existential "givens of life," how these are sometimes ignored or denied, and how addressing them can ultimately lead to a deeper, more reflective and meaningful existence. Clients are invited to reflect on life, to recognize their range of alternatives, and to decide among them.

The existential approach rejects the deterministic view of human nature espoused by traditional psychoanalysis and radical behaviorism. Psychoanalysis sees the individual as primarily determined by unconscious forces, irrational drives, and past events; behaviorists see the individual as primarily determined by sociocultural conditioning. Although deterministic forces affect us and we sometimes cannot control external events thrust upon us, existential therapists believe we retain the freedom to choose how we respond to such events. Existential therapy is grounded on the assumption that we are free and therefore responsible for our choices and actions. We are the authors of our lives, and we design the pathways we follow. This chapter addresses some of the existential concepts and themes that have significant implications for the existentially oriented practitioner.

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A basic existential premise is that we are not victims of circumstance because, to a large extent, we are what we choose to be. Once clients begin the process of recognizing the ways in which they have passively accepted circumstances and surrendered control, they can start down a path of consciously shaping their own lives. The first step in the therapeutic journey is for clients to accept responsibility. As Yalom (2003) puts it, "Once individuals recognize their role in creating their own life predicament, they also realize that they, and only they, have the power to change that situation" (p. 141). One of the aims of existential therapy is to challenge people to stop deceiving themselves regarding their lack of responsibility for what is happening to them and their excessive demands on life (van Deurzen, 2002b).

Emmy van Deurzen (2002a), a key contributor to British existential psychology, writes that existential counseling is not designed to "cure" people of illness in the tradition of the medical model. She does not view clients as being sick, but as "sick of life or clumsy at living" (p. 18) and unable to live a productive life.



See the DVD program for Chapter 6, *DVD for Theory and Practice of Counseling and Psychotherapy: The Case of Stan and Lecturettes*. I suggest that you view the brief lecture for each chapter prior to reading the chapter.

Historical Background in Philosophy and Existentialism

The existential therapy movement was not founded by any particular person or group; many streams of thought contributed to it. Drawing from a major orientation in philosophy, existential therapy arose spontaneously in different parts of Europe and among different schools of psychology and psychiatry in the 1940s and 1950s. Many Europeans found that their lives had been devastated by World War II, and they struggled with existential issues including feelings of isolation, alienation, and meaninglessness. Early writers focused on the individual's experience of being alone in the world and facing the anxiety of this situation. The European existential perspective focused on human limitations and the tragic dimensions of life (Sharp & Bugental, 2001).

The thinking of existential psychologists and psychiatrists was influenced by a number of philosophers and writers during the 19th century. To understand the philosophical underpinnings of modern existential psychotherapy, one must have some awareness of such figures as Søren Kierkegaard, Friedrich Nietzsche, Martin Heidegger, Jean-Paul Sartre, and Martin Buber. These major figures of existentialism and existential phenomenology and their cultural, philosophical, and religious writings provided the basis for the formation of existential therapy. Ludwig Binswanger and Medard Boss are also included in this section because both were early existential psychoanalysts who contributed key ideas to existential psychotherapy.

SØREN KIERKEGAARD (1813–1855) A Danish philosopher and Christian theologian, Kierkegaard was particularly concerned with *angst*—a Danish and German word whose meaning lies between the English words *dread* and *anxiety*—and he addressed the role of anxiety and uncertainty in life. Existential anxiety is associated with making basic decisions about how we want to live, and it is not pathological. Kierkegaard believed that anxiety is the school in which we are educated to be a self. Without the experience of angst, we may go through life as

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sleepwalkers. But many of us, especially in adolescence, are awakened into real life by a terrible uneasiness. Life is one contingency after another, with no guarantees beyond the certainty of death. This is by no means a comfortable state, but it is necessary to our becoming human. Kierkegaard believed that "the sickness unto death" arises when we are not true to ourselves. What is needed is the willingness to risk a leap of faith in making choices. Becoming human is a *project,* and our task is not so much to discover who we are as to *create* ourselves.

FRIEDRICH NIETZSCHE (1844–1900) The German philosopher Nietzsche is the iconoclastic counterpart to Kierkegaard, expressing a revolutionary approach to the self, to ethics, and to society. Like Kierkegaard, he emphasized the importance of subjectivity. Nietzsche set out to prove that the ancient definition of humans as *rational* was entirely misleading. We are far more creatures of will than we are impersonal intellects. But where Kierkegaard emphasized the "subjective truth" of an intense concern with God, Nietzsche located values within the individual's "will to power." We give up an honest acknowledgment of this source of value when society invites us to rationalize powerlessness by advocating other worldly concerns. If, like sheep, we acquiesce in "herd morality," we will be nothing but mediocrities. But if we release ourselves by giving free rein to our will to power, we will tap our potentiality for creativity and originality. Kierkegaard and Nietzsche, with their pioneering analyses of anxiety, depression, subjectivity, and the authentic self, together are generally considered to be the originators of the existential perspective (Sharp & Bugental, 2001).

MARTIN HEIDEGGER (1889–1976) Heidegger's phenomenological existentialism reminds us that we exist "in the world" and should not try to think of ourselves as beings apart from the world into which we are thrown. The way we fill our everyday life with superficial conversation and routine shows that we often assume we are going to live forever and can afford to waste day after day. Our moods and feelings (including anxiety about death) are a way of understanding whether we are living authentically or whether we are inauthentically constructing our lives around the expectations of others. When we translate this wisdom from vague feeling to explicit awareness, we may develop a more positive resolve about how we want to be. Phenomenological existentialism, as presented by Heidegger, provides a view of human history that does not focus on past events but motivates individuals to look forward to "authentic experiences" that are yet to come.

MARTIN BUBER (1878–1965) Leaving Germany to live in the new state of Israel, Buber took a less individualistic stand than most of the other existentialists. He said that we humans live in a kind of *betweenness;* that is, there is never just an *I,* but always an *other*. The *I*, the person who is the agent, changes depending on whether the other is an *it* or a *Thou*. But sometimes we make the serious mistake of reducing another person to the status of a mere object, in which case the relationship becomes *I/it*. Although Buber recognizes that of necessity we must have many *I/it* interactions (in everyday life), we are seriously limited if we live only in the world of the *I/it*. Buber stresses the importance of *presence,* which has three functions: (1) it enables true I/Thou relationships; (2) it allows for meaning to exist

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in a situation; and (3) it enables an individual to be responsible in the here and now (Gould, 1993). In a famous dialogue with Carl Rogers, Buber argued that the therapist and client could never be on the same footing because the latter comes to the former for help. When the relationship is fully mutual, we have become "dialogic," a fully human condition.

LUDWIG BINSWANGER (1881–1966) An existential analyst, Binswanger proposed a holistic model of self that addresses the relationship between the person and his or her environment. He used a phenomenological approach to explore significant features of the self, including choice, freedom, and caring. He based his existential approach largely on the ideas of Heidegger and accepted Heidegger's notion that we are "thrown into the world." However, this "thrown-ness" does not release us from the responsibility of our choices and for planning for the future (Gould, 1993). **Existential analysis** (*dasein analyse*) emphasizes the subjective and spiritual dimensions of human existence. Binswanger (1975) contended that crises in therapy were typically major choice points for the client. Although he originally looked to psychoanalytic theory to shed light on psychosis, he moved toward an existential view of his patients. This perspective enabled him to understand the worldview and immediate experience of his patients, as well as the meaning of their behavior, as opposed to superimposing his view as a therapist on their experience and behavior.

MEDARD BOSS (1903–1991) Both Binswanger and Boss were early existential psychoanalysts and significant figures in the development of existential psychotherapy. They made reference to *dasein* or *being-in-the-world,* which pertains to our ability to reflect on life events and attribute meaning to these events. They believed that the therapist must enter the client's subjective world without presuppositions that would get in the way of this experiential understanding. Both Binswanger and Boss were significantly influenced by Heidegger's seminal work, *Being and Time* (1962), which provided a broad basis for understanding the individual (May, 1958). Boss was deeply influenced by Freudian psychoanalysis, but even more so by Heidegger. Boss's major professional interest was applying Heidegger's philosophical notions to therapeutic practice, and he was especially concerned with integrating Freud's methods with Heidegger's concepts, as described in his book *Daseinanalysis and Psychoanalysis* (1963).

JEAN-PAUL SARTRE (1905–1980) A philosopher and novelist, Sartre was convinced, in part by his years in the French Resistance in World War II, that humans are even more free than earlier existentialists had believed. The existence of a space—nothingness—between the whole of our past and the *now* frees us to choose what we will. Our values are what we choose. The failure to acknowledge our freedom and choices results in emotional problems. This freedom is hard to face, so we tend to invent an excuse by saying, "I can't change now because of my past conditioning." Sartre called excuses "bad faith." No matter what we have been, we can make choices now and become something quite different. We are condemned to be free. To choose is to become committed; this is the responsibility that is the other side of freedom. Sartre's view was that at every moment, by our actions, we are choosing who we are being. Our existence is never fixed or finished.

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Every one of our actions represents a fresh choice. When we attempt to pin down who we are, we engage in self-deception (Russell, 2007).

Key Figures in Contemporary Existential Psychotherapy

Viktor Frankl, Rollo May, Irvin Yalom, and James Bugental all developed their existential approaches to psychotherapy from strong backgrounds in both existential and humanistic psychology. Some of their contributions to psychotherapy are illustrated in the brief sketches that follow.

VIKTOR FRANKL (1905–1997) Viktor Frankl was a central figure in developing existential therapy in Europe and also in bringing it to the United States. As a youth, Frankl was deeply influenced by Freud, but he became a student of Adler. Later, he was influenced by the writings of existential philosophers, and he began developing his own existential philosophy and psychotherapy. He was fond of quoting Nietzsche: "He who has a *why* to live for can bear with almost any *how*" (as cited in Frankl, 1963, pp. 121, 164). Frankl contended that those words could be the motto for all psychotherapeutic practice. Another quotation from Nietzsche seems to capture the essence of his own experience and his writings: "That which does not kill me, makes me stronger" (as cited in Frankl, 1963, p. 130).

Frankl developed **logotherapy,** which means "therapy through meaning." Frankl's philosophical model sheds light on what it means to be fully alive. The central themes running through his works are *life has meaning,* under all circumstances; the central motivation for living is the *will to meaning*; we have the *freedom to find meaning* in all that we think; and we must *integrate body, mind, and spirit* to be fully alive. Frankl said that Freud viewed humans as motivated by the "will to pleasure" and that Adler focused on the "will to power." For Frankl, the most powerful motivation for humans is the "will to meaning." Frankl's writings reflect the theme that the modern person has the means to live, but often has no meaning to live for. The therapeutic process is aimed at challenging individuals to find meaning and purpose through, among other things, suffering, work, and love (Frankl, 1965). (For more background information on Viktor Frankl, see his biography at the beginning of this chapter).

ROLLO MAY (1909–1994) Along with Frankl, psychologist Rollo May was deeply influenced by the existential philosophers, by the concepts of Freudian psychology, and by many aspects of Alfred Adler's Individual Psychology. May was one of the key figures responsible for bringing existentialism from Europe to the United States and for translating key concepts into psychotherapeutic practice. His writings have had a significant impact on existentially oriented practitioners. Of primary importance in introducing existential therapy to the United States was the book *Existence: A New Dimension in Psychiatry and Psychology* (May, Angel, & Ellenberger, 1958). According to May, it takes courage to "be," and our choices determine the kind of person we become. There is a constant struggle within us. Although we want to grow toward maturity and independence, we realize that expansion is often a painful process. Hence, the struggle is between the security of dependence and the joys and pains of growth. (For more background information on Rollo May, see his biography at the beginning of this chapter).

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IRVIN YALOM (1931-) Irvin Yalom is a significant contemporary existential therapist in the United States. He acknowledges the contributions of both European and American psychologists and psychiatrists who have influenced the development of existential thinking and practice (Yalom, 1980). Yalom has developed his approach to individual and group psychotherapy based on the notion that existentialism deals with basic "givens of existence": isolation and relationship with others; death and living fully; and meaninglessness and meaning. Yalom believes the vast majority of experienced therapists, regardless of their theoretical orientation, employ many of the core existential themes. He also contends that how we address these existential themes has a good deal to do with the design and quality of our lives. Yalom recognizes Frankl as an eminently pragmatic thinker who has had an impact on his writing and practice. He also acknowledges the influence on his writings of several novelists and philosophers. More specifically, he draws on the following themes from those philosophers discussed earlier:

- From Kierkegaard: creative anxiety, despair, fear and dread, guilt, and nothingness
- From Nietzsche: death, suicide, and will
- From Heidegger: authentic being, caring, death, guilt, individual responsibility, and isolation
- From Sartre: meaninglessness, responsibility, and choice
- From Buber: interpersonal relationships, I/Thou perspective in therapy, and self-transcendence

(For more background information on Irvin Yalom, see his biography at the beginning of this chapter.)

JAMES BUGENTAL (1915–2008) James Bugental coined the term "existential-humanistic" psychotherapy, and he was a leading spokesman for this approach. His philosophical and therapeutic approach included a curiosity and focus that moved him away from the traditional therapeutic milieu of labeling and diagnosing clients. His work emphasized the cultivation of both client and therapist *presence*. He developed techniques to assist the client in deepening inner exploration, or *searching*. The therapist's primary task involved helping clients to make new discoveries about themselves in the living moment, as opposed to merely *talking about* themselves.

Central to Bugental's approach is his view of *resistance,* which from an existential-humanistic perspective is not resistance to therapy per se but rather to being fully present both during the therapy hour and in life. Resistance is seen as part of the *self-and-world construct*—how a person understands his or her being and relationship to the world at large. Forms of resistance include intellectualizing, being argumentative, always seeking to please, and any other life-limiting pattern. As resistance emerges in the therapy sessions, the therapist repeatedly notes, or "tags," the resistance so the client increases his or her awareness and ultimately has an increased range of choices.

Bugental's theory and practice emphasized the distinction between therapeutic process and content. He became known for being a masterful teacher and psychotherapist, primarily because he lived his work. He was an existentialist at heart,

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which made him a great model and mentor, not only for clients but also for students and professionals. In his workshops, he developed many exercises to help therapists refine and practice their skills. He frequently brought his techniques to life with live demonstrations, which emphasized therapeutic work taking place in the moment, impromptu *here-and-now* dialogue, and exploring in the context of self as client or therapist. Bugental's (1987) classic text, *The Art of the Psychotherapist,* is widely recognized for deconstructing the therapy process and moving beyond theory and generalizations to show what actually occurs moment-to-moment in the therapeutic encounter. *Psychotherapy Isn't What You Think* (Bugental, 1999) is the last book he wrote before he died in 2008, at the age of 93.

OTHER CONTRIBUTORS TO EXISTENTIAL THERAPY A more recent influential figure in contemporary existential psychology is Kirk Schneider, who with colleagues Orah Krug, David Elkins, and Ken Bradford are helping to extend existential principles to a new generation of practitioners. Schneider and his colleagues developed an existential-integrative therapy (see Schneider, 2008, 2011; Schneider & Krug, 2010) mainly because of the need to address today's ethnically and diagnostically diverse clinical populations. The existential-integrative approach emphasizes such areas as personal and interpersonal presence, the working through of resistance, the rediscovery of meaning and awe, and contemplative practices. These themes are increasingly being incorporated into the therapeutic mainstream.

Significant developments in the existential approach are also occurring in Britain, largely due to the efforts of Emmy van Deurzen, who is developing academic and training programs at the New School of Psychotherapy and Counselling (see Other Resources at the end of the chapter for details). In the past decades the existential approach has spread rapidly in Britain and is now an alternative to traditional methods (van Deurzen, 2002b). For a description of the historical context and development of existential therapy in Britain, see van Deurzen (2002b) and Cooper (2003); for an excellent overview of the theory and practice of existential therapy, see van Deurzen (2002a) and Schneider and Krug (2010).

KEY CONCEPTS

View of Human Nature

The crucial significance of the existential movement is that it reacts against the tendency to identify therapy with a set of techniques. Instead, it bases therapeutic practice on an understanding of what it means to be human. The existential movement stands for respect for the person, for exploring new aspects of human behavior, and for divergent methods of understanding people. It uses numerous approaches to therapy based on its assumptions about human nature.

The **existential tradition** seeks a balance between recognizing the limits and tragic dimensions of human existence on one hand and the possibilities and opportunities of human life on the other hand. It grew out of a desire to help people engage the dilemmas of contemporary life, such as isolation, alienation, and meaninglessness. The current focus of the existential approach is on the individual's experience of being in the world alone and facing the anxiety of this isolation.

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"No relationship can eliminate existential isolation, but aloneness can be shared in such a way that love compensates for its pain" (Yalom & Josselson, 2011, p. 326).

The existential view of human nature is captured, in part, by the notion that the significance of our existence is never fixed once and for all; rather, we continually recreate ourselves through our projects. Humans are in a constant state of transition, emerging, evolving, and becoming in response to the tensions, contradictions, and conflicts in our lives. Being a person implies that we are discovering and making sense of our existence. We continually question ourselves, others, and the world. Although the specific questions we raise vary in accordance with our developmental stage in life, the fundamental themes do not vary. We pose the same questions philosophers have pondered throughout Western history: "Who am I?" "What can I know?" "What ought I to do?" "What can I hope for?" "Where am I going?"

The basic dimensions of the human condition, according to the existential approach, include (1) the capacity for self-awareness; (2) freedom and responsibility; (3) creating one's identity and establishing meaningful relationships with others; (4) the search for meaning, purpose, values, and goals; (5) anxiety as a condition of living; and (6) awareness of death and nonbeing. I develop these propositions in the following sections by summarizing themes that emerge in the writings of existential philosophers and psychotherapists, and I also discuss the implications for counseling practice of each of these propositions.

Proposition 1: The Capacity for Self-Awareness

Freedom, choice, and responsibility constitute the foundation of self-awareness. The greater our awareness, the greater our possibilities for freedom (see Proposition 2). Schneider (2008) explains that the core existential position is that we are both *free* (willful, creative, and expressive) and *limited* (by environmental and social constraints). We increase our capacity to live fully as we expand our awareness in the following areas:

- We are finite and do not have unlimited time to do what we want in life.
- We have the potential to take action or not to act; inaction is a decision.
- We choose our actions, and therefore we can partially create our own destiny.
- Meaning is the product of discovering how we are "thrown" or situated in the world and then, through commitment, living creatively.
- As we increase our awareness of the choices available to us, we also increase our sense of responsibility for the consequences of these choices.
- We are subject to loneliness, meaninglessness, emptiness, guilt, and isolation.
- We are basically alone, yet we have an opportunity to relate to other beings.

We can choose either to expand or to restrict our consciousness. Because self-awareness is at the root of most other human capacities, the decision to expand it is fundamental to human growth. Here are some areas of emerging awareness that individuals may experience in the counseling process:

- They see how they are trading the security of dependence for the anxieties that accompany choosing for themselves.
- They begin to see that their identity is anchored in someone else's definition of them; that is, they are seeking approval and confirmation of their being in others instead of looking to themselves for affirmation.

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- They learn that in many ways they are keeping themselves prisoner by some of their past decisions, and they realize that they can make new decisions.
- They learn that although they cannot change certain events in their lives they can change the way they view and react to these events.
- They learn that they are not condemned to a future similar to the past, for they can learn from their past and thereby reshape their future.
- They realize that they are so preoccupied with suffering, death, and dying that they are not appreciating living.
- They are able to accept their limitations yet still feel worthwhile, for they understand that they do not need to be perfect to feel worthy.
- They come to realize that they are failing to live in the present moment because of preoccupation with the past, planning for the future, or trying to do too many things at once.

Increasing self-awareness—which includes awareness of alternatives, motivations, factors influencing the person, and personal goals—is an aim of all counseling. Clients need to learn that a price must be paid for increased awareness. As we become more aware, it is more difficult to "go home again." Ignorance of our condition may have brought contentment along with a feeling of partial deadness, but as we open the doors in our world, we can expect more turmoil as well as the potential for more fulfillment.

Proposition 2: Freedom and Responsibility

A characteristic existential theme is that people are free to choose among alternatives and therefore play a large role in shaping their own destiny. Schneider and Krug (2010) write that existential therapy embraces three values: (1) the *freedom to become* within the context of natural and self-imposed limitations; (2) the capacity to *reflect* on the meaning of our choices; and (3) the capacity to *act* on the choices we make. A central existential concept is that although we long for freedom, we often try to escape from our freedom by defining ourselves as a fixed or static entity (Russell, 2007). We have no choice about being thrust into the world, yet the manner in which we live and what we become are the result of our choices. Because of the reality of this freedom, our task is to accept responsibility for directing our lives. However, it is possible to avoid this reality by making excuses. In speaking about "bad faith," the existential philosopher Jean-Paul Sartre (1971) refers to the **inauthenticity** of not accepting personal responsibility. Here are two statements that reveal bad faith: "Since that's the way I'm made, I couldn't help what I did" or "Naturally I'm this way, because I grew up in a dysfunctional family." An inauthentic mode of existence consists of lacking awareness of personal responsibility for our lives and passively assuming that our existence is largely controlled by external forces. Sartre claims we are constantly confronted with the choice of what kind of person we are becoming, and to exist is never to be finished with this kind of choosing.

Freedom implies that we are responsible for our lives, for our actions, and for our failures to take action. From Sartre's perspective people are condemned to freedom. He calls for a *commitment* to choosing for ourselves. **Existential guilt** is being aware of having evaded a commitment, or having chosen not to choose.

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This guilt is a condition that grows out of a sense of incompleteness, or a realization that we are not what we might have become. Guilt may be a sign that we have failed to rise to the challenge of our anxiety and that we have tried to evade it by not doing what we know is possible for us to do (van Deurzen, 2002a). This condition is not viewed as neurotic, nor is it seen as a symptom that needs to be cured. Instead, the existential therapist explores it to see what clients can learn about the ways in which they are living their life. This guilt also results from allowing others to define us or to make our choices for us. Sartre said, "We are our choices." **Authenticity** implies that we are living by being true to our own evaluation of what is a valuable existence for ourselves; it is the courage to be who we are.

For existentialists, then, being free and being human are identical. Freedom and responsibility go hand in hand. We are the authors of our lives in the sense that we create our destiny, our life situation, and our problems (Russell, 1978). Assuming responsibility is a basic condition for change. Clients who refuse to accept responsibility by persistently blaming others for their problems are not likely to profit from therapy.

Frankl (1978) also links freedom with responsibility. He suggested that the Statue of Liberty on the East Coast should be balanced with a Statue of Responsibility on the West Coast. His basic premise is that freedom is bound by certain limitations. We are not free from conditions, but we are free to take a stand against these restrictions. Ultimately, these conditions are subject to our decisions, which means we are responsible.

The therapist assists clients in discovering how they are avoiding freedom and encourages them to learn to risk using it. Not to do so is to cripple clients and make them dependent on the therapist. Therapists have the task of teaching clients that they can explicitly accept that they have choices, even though they may have devoted most of their life to evading them. Those who are in therapy often have mixed feelings when it comes to choice. As Russell (2007) puts it: "We resent it when we don't have choices, but we get anxious when we do! Existentialism is all about broadening the vision of our choices" (p. 111).

People often seek psychotherapy because they feel that they have lost control of how they are living. They may look to the counselor to direct them, give them advice, or produce magical cures. They may also need to be heard and understood. Two central tasks of the therapist are inviting clients to recognize how they have allowed others to decide for them and encouraging them to take steps toward choosing for themselves. In inviting clients to explore other ways of being that are more fulfilling than their present restricted existence, some existential counselors ask, "Although you have lived in a certain pattern, now that you recognize the price of some of your ways, are you willing to consider creating new patterns?" Others may have a vested interest in keeping the client in an old pattern, so the initiative for changing it will have to come from the client.

Cultural factors need to be taken into account in assisting clients in the process of examining their choices. A person who is struggling with feeling limited by her family situation can be invited to look at her part in this process and values that are a part of her culture. For example, Meta, a Norwegian American, is working to

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attain a professional identity as a social worker, but her family thinks she is being selfish and neglecting her primary duties. The family is likely to exert pressure on her to give up her personal interests in favor of what they feel is best for the welfare of the entire family. Meta may feel trapped in the situation and see no way out unless she rejects what her family wants. In cases such as this, it is useful to explore the client's underlying values and to help her determine whether her values are working for her and for her family. Clients such as Meta have the challenge of weighing values and balancing behaviors between two cultures. Ultimately, Meta must decide in what ways she might change her situation, and she needs to assess values based on her culture. The existential therapist will invite Meta to begin to explore what she *can* do and to realize that she can be authentic in spite of pressures on her by her situation. According to Vontress (2008), we can be authentic in any society, whether we are a part of an individualistic or collectivistic society.

It is essential to respect the purpose that people have in mind when they initiate therapy. If we pay careful attention to what our clients tell us about what they want, we can operate within an existential framework. We can encourage individuals to weigh the alternatives and to explore the consequences of what they are doing with their lives. Although oppressive forces may be severely limiting the quality of their lives, we can help people see that they are not solely the victims of circumstances beyond their control. Even though we sometimes cannot control things that happen to us, we have complete control over how we choose to perceive and handle them. Although our freedom *to act* is limited by external reality, our freedom *to be* relates to our internal reality. At the same time that people are learning how to change their external environment, they can be challenged to look within themselves to recognize their own contributions to their problems. Through the therapy experience, clients may be able to discover new courses of action that will lead to a change in their situation.

Proposition 3: Striving for Identity and Relationship to Others

People are concerned about preserving their uniqueness and centeredness, yet at the same time they have an interest in going outside of themselves to relate to other beings and to nature. Each of us would like to discover a self or, to put it more authentically, to create our personal identity. This is not an automatic process, and creating an identity takes courage. As relational beings, we also strive for connectedness with others. Many existential writers discuss loneliness, uprootedness, and alienation, which can be seen as the failure to develop ties with others and with nature.

The trouble with so many of us is that we have sought directions, answers, values, and beliefs from the important people in our world. Rather than trusting ourselves to search within and find our own answers to the conflicts in our life, we sell out by becoming what others expect of us. Our being becomes rooted in their expectations, and we become strangers to ourselves.

THE COURAGE TO BE Paul Tillich (1886–1965), a leading Protestant theologian of the 20th century, believes awareness of our finite nature gives us an appreciation of ultimate concerns. It takes courage to discover the true "ground of our

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being" and to use its power to transcend those aspects of nonbeing that would destroy us (Tillich, 1952). Courage entails the will to move forward in spite of anxiety-producing situations, such as facing our death (May, 1975). We struggle to discover, to create, and to maintain the core deep within our being. One of the greatest fears of clients is that they will discover that there is no core, no self, no substance, and that they are merely reflections of everyone's expectations of them. A client may say, "My fear is that I'll discover I'm nobody, that there really is nothing to me. I'll find out that I'm an empty shell, hollow inside, and nothing will exist if I shed my masks." If clients demonstrate the courage to confront these fears, they might well leave therapy with an increased tolerance for the uncertainty of life. By assisting clients in facing the fear that their lives or selves are empty and meaningless, therapists can help clients to *create* a self that has meaning and substance that *they have chosen*.

Existential therapists may begin by asking their clients to allow themselves to intensify the feeling that they are nothing more than the sum of others' expectations and that they are merely the introjects of parents and parent substitutes. How do they feel now? Are they condemned to stay this way forever? Is there a way out? Can they create a self if they find that they are without one? Where can they begin? Once clients have demonstrated the courage to recognize this fear, to put it into words and share it, it does not seem so overwhelming. I find that it is best to begin work by inviting clients to accept the ways in which they have lived outside themselves and to explore ways in which they are out of contact with themselves.

THE EXPERIENCE OF ALONENESS The existentialists postulate that part of the human condition is the experience of aloneness. But they add that we can derive strength from the experience of looking to ourselves and sensing our separation. The sense of isolation comes when we recognize that we cannot depend on anyone else for our own confirmation; that is, we alone must give a sense of meaning to life, and we alone must decide how we will live. If we are unable to tolerate ourselves when we are alone, how can we expect anyone else to be enriched by our company? Before we can have any solid relationship with another, we must have a relationship with ourselves. We are challenged to learn to listen to ourselves. We have to be able to stand alone before we can truly stand beside another.

There is a paradox in the proposition that humans are existentially both alone and related, but this very paradox describes the human condition. To think that we can cure the condition, or that it should be cured, is erroneous. Ultimately we are alone, yet our aloneness is set in the context of our inevitable relatedness to other people.

THE EXPERIENCE OF RELATEDNESS We humans depend on relationships with others. We want to be significant in another's world, and we want to feel that another's presence is important in our world. When we are able to stand alone and tap into our own strength, our relationships with others are based on our fulfillment, not our deprivation. If we feel personally deprived, however, we can expect little but a clinging and symbiotic relationship with someone else.

Perhaps one of the functions of therapy is to help clients distinguish between a neurotically dependent attachment to another and a life-affirming relationship in

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which both persons are enhanced. The therapist can challenge clients to examine what they get from their relationships, how they avoid intimate contact, how they prevent themselves from having equal relationships, and how they might create therapeutic, healthy, and mature human relationships. Existential therapists speak of intersubjectivity, which is the fact of our interrelatedness with others and the need for us to struggle with this in a creative way.

STRUGGLING WITH OUR IDENTITY Because of our fear of dealing with our aloneness, Farha (1994) points out that some of us get caught up in ritualistic behavior patterns that cement us to an image or identity we acquired in early childhood. He writes that some of us become trapped in a *doing mode* to avoid the experience of being.

Part of the therapeutic journey consists of the therapist challenging clients to begin to examine the ways in which they have lost touch with their identity, especially by letting others design their life for them. The therapy process itself is often frightening for clients when they realize that they have surrendered their freedom to others and that in the therapy relationship they will have to assume their freedom again. By refusing to give easy solutions or answers, existential therapists confront clients with the reality that they alone must find their own answers.

Proposition 4: The Search for Meaning

A distinctly human characteristic is the struggle for a sense of significance and purpose in life. In my experience the underlying conflicts that bring people into counseling and therapy are centered in these existential questions: "Why am I here?" "What do I want from life?" "What gives my life purpose?" "Where is the source of meaning for me in life?"

Existential therapy can provide the conceptual framework for helping clients challenge the meaning in their lives. Questions that the therapist might ask are, "Do you like the direction of your life?" "Are you pleased with what you now are and what you are becoming?" "If you are confused about who you are and what you want for yourself, what are you doing to get some clarity?"

THE PROBLEM OF DISCARDING OLD VALUES One of the problems in therapy is that clients may discard traditional (and imposed) values without creating other, suitable ones to replace them. What does the therapist do when clients no longer cling to values that they never really challenged or internalized and now experience a vacuum? Clients may report that they feel like a boat without a rudder. They seek new guidelines and values that are appropriate for the newly discovered facets of themselves, and yet for a time they are without them. One of the tasks of the therapeutic process is to help clients create a value system based on a way of living that is consistent with their way of being.

The therapist's job is to trust in the capacity of clients to eventually create an internally derived value system that provides the foundation for a meaningful life. They will no doubt flounder for a time and experience anxiety as a result of the absence of clear-cut values. The therapist's trust is important in helping clients trust their own capacity to create a new source of values.

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MEANINGLESSNESS When the world they live in seems meaningless, clients may wonder whether it is worth it to continue struggling or even living. Faced with the prospect of our mortality, we might ask, "Is there any point to what I do now, since I will eventually die? Will what I do be forgotten when I am gone? Given the fact of mortality, why should I busy myself with anything?" A man in one of my groups captured precisely the idea of personal significance when he said, "I feel like another page in a book that has been turned quickly, and nobody bothered to read the page." For Frankl (1978) such a feeling of meaninglessness is the major existential neurosis of modern life.

Meaninglessness in life can lead to emptiness and hollowness, or a condition that Frankl calls the **existential vacuum.** This condition is often experienced when people do not busy themselves with routine or with work. Because there is no preordained design for living, people are faced with the task of creating their own meaning. At times people who feel trapped by the emptiness of life withdraw from the struggle of creating a life with purpose. Experiencing meaninglessness and establishing values that are part of a meaningful life are issues that become the heart of counseling.

CREATING NEW MEANING Logotherapy is designed to help clients find meaning in life. The therapist's function is not to tell clients what their particular meaning in life should be but to point out that they can create meaning even in suffering (Frankl, 1978). This view holds that human suffering (the tragic and negative aspects of life) can be turned into human achievement by the stand an individual takes when faced with it. Frankl also contends that people who confront pain, guilt, despair, and death can effectively deal with their despair and thus triumph.

Yet meaning is not something that we can directly search for and obtain. Paradoxically, the more rationally we seek it, the more likely we are to miss it. Frankl (1978) and Yalom and Josselson (2011) are in basic agreement that, like pleasure, meaning must be pursued obliquely. Finding satisfaction and meaning in life is a by-product of engagement, which is a commitment to creating, loving, working, and building. Meaning is created out of an individual's engagement with what is valued, and this commitment provides the purpose that makes life worthwhile (van Deurzen, 2002a). I like the way Vontress (2008) captures the idea that meaning in life is an ongoing process we struggle with throughout our lives: "What provides meaning one day may not provide meaning the next, and what has been meaningful to a person throughout life may be meaningless when a person is on his or her deathbed" (p. 158).

Proposition 5: Anxiety as a Condition of Living

Anxiety arises from one's personal strivings to survive and to maintain and assert one's being, and the feelings anxiety generates are an inevitable aspect of the human condition. **Existential anxiety** is the unavoidable result of being confronted with the "givens of existence"—death, freedom, choice, isolation, and meaninglessness (Vontress, 2008; Yalom, 1980; Yalom & Josselson, 2011). Existential anxiety arises as we recognize the realities of our mortality, our confrontation with pain and suffering, our need to struggle for survival, and our basic fallibility. We experience

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this anxiety as we become increasingly aware of our freedom and the consequences of accepting or rejecting that freedom. In fact, when we make a decision that involves reconstruction of our life, the accompanying anxiety can be a signal that we are ready for personal change and can be a stimulus for growth. If we learn to listen to the subtle messages of anxiety, we can dare to take the steps necessary to change the direction of our lives.

Existential therapists differentiate between normal and neurotic anxiety, and they see anxiety as a potential source of growth. **Normal anxiety** is an appropriate response to an event being faced. Further, this kind of anxiety does not have to be repressed, and it can be used as a motivation to change. Because we could not survive without some anxiety, it is not a therapeutic goal to eliminate normal anxiety. Existential philosophers have argued that at the root of our normal (or *ontic*) anxiety, which is an appropriate anxiety that relates to concrete things in the world, is a more fundamental existential (or *ontological*) anxiety, which is based on our awareness of our own temporality and is present even when we do not have to face particularly difficult situations. **Neurotic anxiety,** in contrast, is anxiety about concrete things that is out of proportion to the situation. Neurotic anxiety is typically out of awareness, and it tends to immobilize the person. Being psychologically healthy entails living with as little neurotic anxiety as possible, while accepting and struggling with the unavoidable existential anxiety that is a part of living.

Many people who seek counseling want solutions that will enable them to eliminate anxiety. Attempts to avoid anxiety by creating the illusion that there is security in life may help us cope with the unknown, yet we really know on some level that we are deceiving ourselves when we think we have found fixed security. Facing existential anxiety involves viewing life as an adventure rather than hiding behind imagined securities that seem to offer protection. Opening up to new life means opening up to anxiety. We pay a steep price when we short-circuit anxiety.

People who have the courage to face themselves are, nonetheless, frightened. I am convinced that those who are willing to live with their anxiety for a time profit from personal therapy. Those who flee too quickly into comfortable patterns might experience temporary relief but in the long run seem to experience the frustration of being stuck in old ways.

The existential therapist can help clients recognize that learning how to tolerate ambiguity and uncertainty and how to live without props can be a necessary phase in the journey from dependence to autonomy. The therapist and client can explore the possibility that although breaking away from crippling patterns and building new ways of living will be fraught with anxiety for a while, anxiety will diminish as the client experiences more satisfaction with newer ways of being. When a client becomes more self-confident, the anxiety that results from an expectation of catastrophe is likely to decrease.

Proposition 6: Awareness of Death and Nonbeing

The existentialist does not view death negatively but holds that awareness of death as a basic human condition gives significance to living. A distinguishing human characteristic is the ability to grasp the reality of the future and the inevitability of death. It is necessary to think about death if we are to think significantly about life.

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Death should not be considered a threat; death provides the motivation for us to take advantage of appreciating the present moment. Instead of being frozen by the fear of death, death can be viewed as a positive force that enables us to live as fully as possible. Although the notion of death is a wake-up call, it is also something that we strive to avoid (Russell, 2007). If we defend ourselves against the reality of our eventual death, life becomes insipid and meaningless. But if we realize that we are mortal, we know that we do not have an eternity to complete our projects and that the present is crucial. Our awareness of death is the source of zest for life and creativity. Death and life are interdependent, and though physical death destroys us, the idea of death saves us (Yalom, 1980, 2003).

Yalom (2008) recommends that therapists talk directly to clients about the reality of death. He believes the fear of death percolates beneath the surface and haunts us throughout life. Death is a visitor in the therapeutic process, and Yalom believes that ignoring its presence sends the message that death is too overwhelming to explore. Confronting this fear can be the factor that helps us transform an inauthentic mode of living into a more authentic one. Accepting the reality of our personal death can result in a massive shift in the way we live in the world (Yalom & Josselson, 2011). We can turn our fear of death into a positive force when we accept the reality of our mortality. In *Staring at the Sun: Overcoming the Terror of Death*, Yalom (2008) develops the idea that confronting death enables us to live in a more compassionate way.

One focus in existential therapy is on exploring the degree to which clients are doing the things they value. Without being morbidly preoccupied by the ever-present threat of nonbeing, clients can develop a healthy awareness of death as a way to evaluate how well they are living and what changes they want to make in their lives. Those who fear death also fear life. When we emotionally accept the reality of our eventual death, we realize more clearly that our actions do count, that we do have choices, and that we must accept the ultimate responsibility for how well we are living (Corey & Corey, 2010).

THE THERAPEUTIC PROCESS

Therapeutic Goals

Existential therapy is best considered as an invitation to clients to recognize the ways in which they are not living fully authentic lives and to make choices that will lead to their becoming what they are capable of being. An aim of therapy is to assist clients in moving toward authenticity and learning to recognize when they are deceiving themselves (van Deurzen, 2002a). The existential orientation holds that there is no escape from freedom as we will always be held responsible. We can relinquish our freedom, however, which is the ultimate inauthenticity. Existential therapy aims at helping clients face anxiety and engage in action that is based on the authentic purpose of creating a worthy existence.

May (1981) contends that people come to therapy with the self-serving illusion that they are inwardly enslaved and that someone else (the therapist) can free them. Existential therapists are mainly concerned about helping people to reclaim and reown their lives. The task of existential therapy is to teach clients to listen to

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what they already know about themselves, even though they may not be attending to what they know. Schneider and Krug (2010) identify four essential aims of existential-humanistic therapy: (1) to help clients become more present to both themselves and others; (2) to assist clients in identifying ways they block themselves from fuller presence; (3) to challenge clients to assume responsibility for designing their present lives; and (4) to encourage clients to choose more expanded ways of being in their daily lives.

Increased awareness is the central goal of existential therapy, which allows clients to discover that alternative possibilities exist where none were recognized before. Clients come to realize that they are able to make changes in their way of being in the world.

Therapist's Function and Role

Existential therapists are primarily concerned with understanding the subjective world of clients to help them come to new understandings and options. Existential therapists are especially concerned about clients avoiding responsibility; they consistently invite clients to accept personal responsibility. When clients complain about the predicaments they are in and blame others, the therapist is likely to ask them how they contributed to their situation.

Therapists with an existential orientation usually deal with people who have what could be called a **restricted existence.** These clients have a limited awareness of themselves and are often vague about the nature of their problems. They may see few, if any, options for dealing with life situations, and they tend to feel trapped, helpless, and stuck. For Bugental (1997), a therapist's function is to assist clients in seeing the ways in which they constrict their awareness and the cost of such constrictions. The therapist may hold up a mirror, so to speak, so that clients can gradually engage in self-confrontation. In this way clients can see how they became the way they are and how they might enlarge the way they live. Once clients are aware of factors in their past and of stifling modes of their present existence, they can begin to accept responsibility for changing their future.

Existential practitioners may make use of techniques that originate from diverse theoretical orientations, yet no set of techniques is considered essential. The therapeutic journey is creative and uncertain and different for each client. Russell (2007) captures this notion well when he writes: "There is no one right way to do therapy, and certainly no rigid doctrine for existentially rooted techniques. What is crucial is that you create your own authentic way of being attuned to your clients" (p. 123). Existential therapists encourage experimentation not only within the therapy office but also outside of the therapy setting, based on the belief that life outside therapy is what counts. Practitioners often ask clients to reflect on or write about problematic events they encounter in daily life (Schneider, 2011).

Client's Experience in Therapy

Clients in existential therapy are clearly encouraged to assume responsibility for how they are currently choosing to be in their world. Effective therapy does not stop with this awareness itself, for the therapist encourages clients to take action on the

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basis of the insights they develop through the therapeutic process. Experimentation with new ways of behaving in the outside world is necessary if clients are to change. Further, clients must be active in the therapeutic process, for during the sessions they must decide what fears, guilt feelings, and anxieties they will explore.

Merely deciding to enter psychotherapy is itself a frightening prospect for most people. The experience of opening the doors to oneself can be frightening, exciting, joyful, depressing, or a combination of all of these. As clients wedge open the closed doors, they also begin to loosen the deterministic shackles that have kept them psychologically bound. Gradually, they become aware of what they have been and who they are now, and they are better able to decide what kind of future they want. Through the process of their therapy, individuals can explore alternatives for making their visions real.

When clients plead helplessness and attempt to convince themselves that they are powerless, May (1981) reminds them that their journey toward freedom began by putting one foot in front of the other to get to his office. As narrow as their range of freedom may be, individuals can begin building and augmenting that range by taking small steps. The therapeutic journey that opens up new horizons is poetically described by van Deurzen (2010):

> Embarking on our existential journey requires us to be prepared to be touched and shaken by what we find on the way and to not be afraid to discover our own limitations and weaknesses, uncertainties and doubts. It is only with such an attitude of openness and wonder that we can encounter the impenetrable everyday mysteries, which take us beyond our own preoccupations and sorrows and which by confronting us with death, make us rediscover life. (p. 5)

Another aspect of the experience of being a client in existential therapy is confronting ultimate concerns rather than coping with immediate problems. Rather than being solution-oriented, existential therapy is aimed toward removing roadblocks to meaningful living and helping clients assume responsibility for their actions (Yalom & Josselson, 2011). Existential therapists assist people in facing life with courage, hope, and a willingness to find meaning in life.

Relationship Between Therapist and Client

Existential therapists give central prominence to their relationship with the client. The relationship is important in itself because the quality of this person-to-person encounter in the therapeutic situation is the stimulus for positive change. Attention is given to the client's immediate, ongoing experience, especially what is going on in the interaction between the therapist and the client. Therapy is viewed as a social microcosm in the sense that the interpersonal and existential problems of the client will become apparent in the here and now of the therapy relationship (Yalom & Josselson, 2011).

Therapists with an existential orientation believe their basic attitudes toward the client and their own personal characteristics of honesty, integrity, and courage are what they have to offer. Therapy is a journey taken by therapist and client that delves deeply into the world as perceived and experienced by the client. But this type of quest demands that therapists also be in contact with their own phenomenological world. Existential therapy is a voyage into self-discovery and a journey of life-discovery for both client and therapist (van Deurzen, 2010; Yalom & Josselson, 2011).

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Buber's (1970) conception of the I/Thou relationship has significant implications here. His understanding of the self is based on two fundamental relationships: the "I/it" and the "I/Thou." The I/it is the relation to time and space, which is a necessary starting place for the self. The I/Thou is the relationship essential for connecting the self to the spirit and, in so doing, to achieve true dialogue. This form of relationship is the paradigm of the fully human self, the achievement of which is the goal of Buber's existential philosophy. Relating in an I/Thou fashion means that there is direct, mutual, and present interaction. Rather than prizing therapeutic objectivity and professional distance, existential therapists strive to create caring and intimate relationships with clients.

The core of the therapeutic relationship is respect, which implies faith in clients' potential to cope authentically with their troubles and in their ability to discover alternative ways of being. Existential therapists share their reactions to clients with genuine concern and empathy as one way of deepening the therapeutic relationship. Therapists invite clients to grow by modeling authentic behavior. If therapists keep themselves hidden during the therapeutic session or if they engage in inauthentic behavior, clients will also remain guarded and persist in their inauthentic ways.

Bugental (1987) emphasizes the crucial role the *presence* of the therapist plays in the therapeutic relationship. In his view many therapists and therapeutic systems overlook its fundamental importance. He contends that therapists are too often so concerned with the content of what is being said that they are not aware of the distance between themselves and their clients:

> The therapeutic alliance is the powerful joining of forces which energizes and supports the long, difficult, and frequently painful work of life-changing psychotherapy. The conception of the therapist here is not of a disinterested observer-technician but of a fully alive human companion for the client. (p. 49)

Schneider (2011) believes that the therapist's *presence* is both a condition and a goal of therapeutic change. Presence serves the dual functions of reconnecting people to their pain and to attuning them to the opportunities to transform their pain.

APPLICATION: THERAPEUTIC TECHNIQUES AND PROCEDURES

The existential approach is unlike most other therapies in that it is not technique-oriented. Although existentially oriented therapists may incorporate many techniques from other models, these interventions are made within the context of striving to understand the subjective world of the client. The interventions existential practitioners employ are based on philosophical views about the nature of human existence. These practitioners prefer description, understanding, and exploration of the client's subjective reality, as opposed to diagnosis, treatment, and prognosis (van Deurzen, 2002b). As Vontress (2008) puts it: "Existential therapists prefer to be thought of as philosophical companions, not as people who repair psyches" (p. 161). Yalom and Josselson (2011) assert that existential therapists are "fellow travelers" (p. 332), willing to make themselves known through appropriate

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self-disclosure. As mentioned earlier, existential therapists are free to draw from techniques that flow from many other orientations. However, they do not employ an array of unintegrated techniques; they have a set of assumptions and attitudes that guide their interventions with clients. See *Case Approach to Counseling and Psychotherapy* (Corey, 2013, chap. 4) for an illustration of how Dr. J. Michael Russell works in an existential way with some key themes in the case of Ruth.

Van Deurzen (2010) identifies as a primary ground rule of existential work the openness to the individual creativity of the therapist and the client. She maintains that existential therapists need to adapt their interventions to their own personality and style, as well as being sensitive to what each client requires. The main guideline is that the existential practitioner's interventions are responsive to the uniqueness of each client.

Van Deurzen (2002a, 2002b) believes that the starting point for existential work is for practitioners to clarify their views on life and living. She stresses the importance of therapists reaching sufficient depth and openness in their own lives to venture into clients' murky waters without getting lost. The nature of existential work is assisting people in the process of living with greater expertise and ease. Van Deurzen (2010) identifies how therapists make a difference with clients: "We help them to get better at reflecting on their situation, deal with their dilemma, face their predicament and think for themselves" (p. 236). Van Deurzen reminds us that existential therapy is a collaborative adventure in which both client and therapist will be transformed if they allow themselves to be touched by life. When the deepest self of the therapist meets the deepest part of the client, the counseling process is at its best. Therapy is a creative, evolving process of discovery that can be conceptualized in three general phases.

Phases of Existential Counseling

During the initial phase of counseling, therapists assist clients in identifying and clarifying their assumptions about the world. Clients are invited to define and question the ways in which they perceive and make sense of their existence. They examine their values, beliefs, and assumptions to determine their validity. This is a difficult task for many clients because they may initially present their problems as resulting almost entirely from external causes. They may focus on what other people "make them feel" or on how others are largely responsible for their actions or inaction. The counselor teaches them how to reflect on their own existence and to examine their role in creating their problems in living.

During the middle phase of existential counseling, clients are assisted in more fully examining the source and authority of their present value system. This process of self-exploration typically leads to new insights and some restructuring of values and attitudes. Individuals get a better idea of what kind of life they consider worthy to live and develop a clearer sense of their internal valuing process.

The final phase of existential counseling focuses on helping people take what they are learning about themselves and put it into action. Transformation is not limited to what takes place during the therapy hour. The therapeutic hour is a small contribution to a person's renewed engagement with life, or a rehearsal for life (van Deurzen, 2002b). The aim of therapy is to enable clients to discover

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ways of implementing their examined and internalized values in a concrete way between sessions and after therapy has terminated. Clients typically discover their strengths and find ways to put them to the service of living a purposeful existence.

Clients Appropriate for Existential Counseling

Existential practice has been applied in a variety of settings and with a diverse population of clients, including those with substance abuse issues, ethnic and racial minorities, gay and lesbian clients, and psychiatric inpatients (Schneider, 2011). A strength of the perspective is its focus on available choices and pathways toward personal growth. For people who are coping with developmental crises, experiencing grief and loss, confronting death, or facing a major life decision, existential therapy is especially appropriate. Some examples of these critical turning points that mark passages from one stage of life into another are the struggle for identity in adolescence, coping with possible disappointments in middle age, adjusting to children leaving home, coping with failures in marriage and work, and dealing with increased physical limitations as one ages. These developmental challenges involve both dangers and opportunities. Uncertainty, anxiety, and struggling with decisions are all part of this process.

Van Deurzen (2002b) suggests that this form of therapy is most appropriate for clients who are committed to dealing with their problems about living, for people who feel alienated from the current expectations of society, or for those who are searching for meaning in their lives. It tends to work well with people who are at a crossroads and who question the state of affairs in the world and are willing to challenge the status quo. It can be useful for people who are on the edge of existence, such as those who are dying or contemplating suicide, who are working through a developmental or situational crisis, who feel that they no longer belong in their surroundings, or who are starting a new phase of life.

Application to Brief Therapy

The existential approach can focus clients on significant areas such as assuming personal responsibility, making a commitment to deciding and acting, and expanding their awareness of their current situation. It is possible for a time-limited approach to serve as a catalyst for clients to become actively and fully involved in each of their therapy sessions. Strasser and Strasser (1997), who are connected to the British school of existential analysis, maintain that there are clear benefits to time-limited therapy, which mirrors the time-limited reality of human existence. Sharp and Bugental (2001) maintain that short-term applications of the existential approach require more structuring and clearly defined and less ambitious goals. At the termination of short-term therapy, it is important for individuals to evaluate what they have accomplished and what issues may need to be addressed later. It is essential that both therapist and client determine that short-term work is appropriate, and that beneficial outcomes are likely.

Application to Group Counseling

An existential group can be described as people making a commitment to a lifelong journey of self-exploration with these goals: (1) enabling members to become honest with themselves, (2) widening their perspectives on themselves and the world

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around them, and (3) clarifying what gives meaning to their present and future life (van Deurzen, 2002b). An open attitude toward life is essential, as is the willingness to explore unknown territory. Recurring universal themes evolve in many groups and challenge members to seriously explore existential concerns such as choice, freedom and anxiety, awareness of death, meaning in life, and living fully.

Yalom (1980) contends that the group provides the optimal conditions for therapeutic work on responsibility. The members are responsible for the way they behave in the group, and this provides a mirror for how they are likely to act in the world. A group represents a microcosm of the world in which participants live and function. Over time the interpersonal and existential problems of the participants become evident in the here-and-now interactions within the group (Yalom & Josselson, 2011). Through feedback, members learn to view themselves through others' eyes, and they learn the ways in which their behavior affects others. Building on what members learn about their interpersonal functioning in the group, they can take increased responsibility for making changes in everyday life. The group experience provides the opportunity to participants to relate to others in meaningful ways, to learn to be themselves in the company of other people, and to establish rewarding, nourishing relationships.

In existential group counseling, members come to terms with the paradoxes of existence: that life can be undone by death, that success is precarious, that we are determined to be free, that we are responsible for a world we did not choose, that we must make choices in the face of doubt and uncertainty. Members experience anxiety when they recognize the realities of the human condition, including pain and suffering, the need to struggle for survival, and their basic fallibility. Clients learn that there are no ultimate answers for ultimate concerns. Through the support that is within a group, participants are able to tap the strength needed to create an internally derived value system that is consistent with their way of being.

A group provides a powerful context to look at oneself, and to consider what choices might be more authentically one's own. Members can openly share their fears related to living in unfulfilling ways and come to recognize how they have compromised their integrity. Members can gradually discover ways in which they have lost their direction and can begin to be more true to themselves. Members learn that it is not in others that they find the answers to questions about significance and purpose in life. Existential group leaders help members live in authentic ways and refrain from prescribing simple solutions. For a more detailed discussion of the existential approach to group counseling, see Corey (2012, chap. 9).

EXISTENTIAL THERAPY FROM A MULTICULTURAL PERSPECTIVE

Strengths From a Diversity Perspective

Because the existential approach does not dictate a particular way of viewing or relating to reality, and because of its broad perspective, this approach is highly relevant in working in a multicultural context (van Deurzen, 2002a). Vontress and colleagues (1999) write about the existential foundation of cross-cultural counseling: "Existential

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counseling is probably the most useful approach to helping clients of all cultures find meaning and harmony in their lives, because it focuses on the sober issues each of us must inevitably face: love, anxiety, suffering, and death" (p. 32). These are the human experiences that transcend the boundaries that separate cultures.

Existential therapy is useful in working with culturally diverse clients because of its focus on universality, or the common ground that we all share. This approach emphasizes presence, the I/Thou relationship, and courage. As such, it can be effectively applied with diverse client populations with a range of specific problems and in a wide array of settings (Schneider, 2008, 2011; Schneider & Krug, 2010). Schneider's (2008) "existential-integrative" model of practice coordinates a variety of therapeutic modes within an overarching existential or experiential framework. Vontress (1996) points out that all people are multicultural in the sense that they are all products of many cultures. He encourages counselors-in-training to focus on the universal commonalities of clients first and secondarily on areas of differences. In working with cultural diversity, it is essential to recognize simultaneously the commonalities and differences of human beings: "Cross-cultural counseling, in short, does not intend to teach specific interventions for each culture, but to infuse the counselor with a cultural sensitivity and tolerant philosophical outlook that will befit all cultures" (p. 164).

The focus on subjective experience, or phenomenology, is a strength from a multicultural perspective. Another strength of the existential approach is that it enables clients to examine the degree to which their behavior is being influenced by social and cultural conditioning. Clients can be challenged to look at the price they are paying for the decisions they have made. Although it is true that some clients may not feel a sense of freedom, their freedom can be increased if they recognize the social limits they are facing. Their freedom can be hindered by institutions and limited by their family. In fact, it may be difficult to separate individual freedom from the context of their family structure.

There is wide-ranging international interest in the existential approach. Several Scandinavian societies, an East European society (encompassing Estonia, Latvia, Lithuania, Russia, Ukraine, and Belarus), and Mexican and South American societies are thriving. In addition, an Internet course, SEPTIMUS, is taught in Ireland, Iceland, Sweden, Poland, Czech Republic, Romania, Italy, Portugal, France, Belgium, the United Kingdom, Israel, and Australia. Most recently, the First International East-West Existential Psychology conference was held in Nanjing, China, with representatives from the United States, Korea, and Japan. These international developments, as well as the creation of the International Collaborative of Existential Counselors and Psychotherapists, which has members from all over the world, reveal that existential therapy has wide appeal to diverse populations in many parts of the world.

Shortcomings From a Diversity Perspective

For those who hold a systemic perspective, the existentialists can be criticized on the grounds that they are excessively individualistic and that they ignore the social factors that cause human problems. However, with the advent of the "existential-integrative" model of practice (Schneider, 2008), this situation is beginning to change. According to Schneider (2011), existential practitioners are not only concerned with facilitating

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individual change but with promoting an in-depth inquiry that has implications for social change: "One cannot simply heal individuals to the neglect of the social context within which they are thrust. To be a responsible practitioner, one must develop a vision of responsible social change alongside and in coordination with one's vision of individual transformation" (p. 281).

Some individuals who seek counseling may operate on the assumption that they have very little choice because environmental circumstances severely restrict their ability to influence the direction of their lives. Even if they change internally, they see little hope that the external realities of racism, discrimination, and oppression will change. They are likely to experience a deep sense of frustration and feelings of powerlessness when it comes to making changes outside of themselves. As you will see in Chapter 12, feminist therapists maintain that therapeutic practice will be effective only to the extent that therapists intervene with some form of social action to change those factors that are creating clients' problems. In working with people of color who come from the barrio or ghetto, for example, it is important to engage their survival issues. If a counselor too quickly puts across the message to these clients that they have a choice in making their lives better, they may feel patronized and misunderstood. These real-life concerns can provide a good focus for counseling, assuming the therapist is willing to deal with them.

A potential problem within existential theory is that it is highly focused on the philosophical assumption of self-determination, which may not take into account the complex factors that many people who have been oppressed must deal with. In many cultures it is not possible to talk about the self and self-determination apart from the context of the social network and environmental conditions. However, a case can be made for the existential approach being instrumental in enabling clients to make conscious choices when it comes to the values they live by. Existential therapists do not push autonomy apart from a client's culture. They do assist clients in critically evaluating the source of their values and making a choice rather than uncritically accepting the values of their culture and family.

Many clients expect a structured and problem-oriented approach to counseling that is not found in the conventional existential approach. Although clients may feel better if they have an opportunity to talk and to be understood, they are likely to expect the counselor to do something to bring about a change in their life situation. A major task for the counselor who practices from an existential perspective is to provide enough concrete direction for these clients without taking the responsibility away from them.

Existential Therapy Applied to the Case of Stan



As an existentially oriented therapist, I counsel Stan with the assumption that he has the capacity to increase his self-awareness and decide for himself the future direction of his life. I want him to realize more than anything else that he does not have to be the victim of his past conditioning but can be the architect in redesigning his future. He can free himself of his deterministic shackles and accept the responsibility that comes with directing his own life. This approach emphasizes the importance of my understanding of Stan's

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world, primarily by establishing an authentic relationship as a means to a fuller degree of self-understanding.

Stan is demonstrating what Sartre would call "bad faith" by not accepting personal responsibility. I confront Stan with the ways in which he is attempting to escape from his freedom through alcohol and drugs. Eventually, I challenge Stan's passivity. I reaffirm that he is now entirely responsible for his life, for his actions, and for his failure to take action. I do this in a supportive yet firm manner.

I do not see Stan's anxiety as something negative, but as a vital part of living with uncertainty and freedom. Because there are no guarantees and because the individual is ultimately alone, Stan can expect to experience some degree of healthy anxiety, aloneness, guilt, and even despair. These conditions are not neurotic in themselves, but the way in which Stan orients himself and copes with these conditions is critical.

Stan sometimes talks about his suicidal feelings. Certainly, I investigate further to determine if he poses an immediate threat to himself. In addition to this assessment to determine lethality, I view his thoughts of "being better off dead" as symbolic. Could it be that Stan feels he is dying as a person? Is Stan using his human potential? Is he choosing a way of merely existing instead of affirming life? Is Stan mainly trying to elicit sympathy from his family? I invite Stan to explore the meaning and purpose in his life. Is there any reason for him to want to continue living? What are some of the projects that enrich his life? What can he do to find a sense of purpose that will make him feel more significant and alive?

Stan needs to accept the reality that he may at times feel alone. Choosing for oneself and living from one's own center accentuates the experience of aloneness. He is not, however, condemned to a life of isolation, alienation from others, and loneliness. I hope to help Stan discover his own centeredness and live by the values he chooses and creates for himself. By doing so, Stan can become a more substantial person and come to appreciate himself more. When he does, the chances are lessened that he will have a need to secure approval from others, particularly his parents and parental substitutes. Instead of forming a dependent relationship, Stan could choose to relate to others out of his strength. Only then would there be the possibility of overcoming his feelings of separateness and isolation.

Follow-Up: You Continue as Stan's Existential Therapist

Use these questions to help you think about how you would counsel Stan using an existential approach:

- If Stan resisted your attempts to help him see that he is responsible for the direction of his life, how might you intervene?
- Stan experiences a great deal of anxiety. From an existential perspective, how do you view his anxiety? How might you work with his anxiety in helpful ways?
- If Stan talks with you about suicide as a response to despair and a life without meaning, how would you respond?



See the *DVD for Theory and Practice of Counseling and Psychotherapy: The Case of Stan and Lecturettes* (Session 4 on existential therapy) for a demonstration of my approach to counseling Stan from this perspective. This session focuses on the themes of death and the meaning of life.

SUMMARY AND EVALUATION

Summary

As humans, according to the existentialist view, we are capable of self-awareness, which is the distinctive capacity that allows us to reflect and to decide. With this awareness we become free beings who are responsible for choosing the way we live, and we influence our own destiny. This awareness of freedom and responsibility

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gives rise to existential anxiety, which is another basic human characteristic. Whether we like it or not, we are free, even though we may seek to avoid reflecting on this freedom. The knowledge that we must choose, even though the outcome is not certain, leads to anxiety. This anxiety is heightened when we reflect on the reality that we are mortal. Facing the inevitable prospect of eventual death gives the present moment significance, for we become aware that we do not have forever to accomplish our projects. Our task is to create a life that has meaning and purpose. As humans we are unique in that we strive toward fashioning purposes and values that give meaning to living. Whatever meaning our life has is developed through freedom and a commitment to make choices in the face of uncertainty.

Existential therapy places central prominence on the person-to-person relationship. It assumes that client growth occurs through this genuine encounter. It is not the techniques a therapist uses that make a therapeutic difference; rather, it is the quality of the client–therapist relationship that heals. It is essential that therapists reach sufficient depth and openness in their own lives to allow them to venture into their clients' subjective world without losing their own sense of identity. Presence is both a condition for therapy to occur and a goal of therapy. Existential therapists are fellow travelers, and as such they strive to be authentic and self-disclosing in their therapy work. Because this approach focuses on the goals of therapy, basic conditions of being human, and therapy as a shared journey, practitioners are not bound by specific techniques. Although existential therapists may apply techniques from other orientations, their interventions are guided by a philosophical framework about what it means to be human.

Contributions of the Existential Approach

The existential approach has helped bring the person back into central focus. It concentrates on the central facts of human existence: self-consciousness and our consequent freedom. To the existentialist goes the credit for providing a new view of death as a positive force, not a morbid prospect to fear, for death gives life meaning. Existentialists have contributed a new dimension to the understanding of anxiety, guilt, frustration, loneliness, and alienation.

I particularly appreciate the way van Deurzen (2002a) views the existential practitioner as a mentor and fellow traveler who encourages people to reflect upon the problems they encounter in living. What clients need is "some assistance in surveying the terrain and in deciding on the right route so that they can again find their way" (p. 18). According to van Deurzen, the existential approach encourages people to live life by their own standards and values: "The aim of existential work is to assist people in developing their talents in their own personal way, helping them in being true to what they value" (p. 21).

One of the major contributions of the existential approach is its emphasis on the human quality of the therapeutic relationship. This aspect lessens the chances of dehumanizing psychotherapy by making it a mechanical process. Existential counselors reject the notions of therapeutic objectivity and professional distance, viewing them as being unhelpful.

I very much value the existential emphasis on freedom and responsibility and the person's capacity to redesign his or her life by choosing with awareness.

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This perspective provides a sound philosophical base on which to build a personal and unique therapeutic style because it addresses itself to the core struggles of the contemporary person.

CONTRIBUTIONS TO THE INTEGRATION OF PSYCHOTHERAPIES

From my perspective, the key concepts of the existential approach can be integrated into most therapeutic schools. Regardless of a therapist's orientation, the foundation for practice can be based on existential themes. Existential psychotherapy continues to have an enduring impact on a variety of psychological practices. "Indeed, existential psychotherapy is in the ironic position of being one of the most widely influential yet least officially embraced orientations on the professional scene" (Schneider, 2008, p. 1). Bruce Wampold (2008), a leading researcher in the psychotherapy field, agrees "that an understanding of the principles of existential therapy is needed by all therapists, as it adds a perspective that might . . . form the basis for all effective treatments" (p. 6).

A key contribution is the possibility of a creative integration of the conceptual propositions of existential therapy with many other therapeutic orientations (Bugental & Bracke, 1992; Schneider, 2008, 2011; Schneider & Krug, 2010). One example of such a creative integration is provided by Dattilio (2002), who integrates cognitive behavioral techniques with the themes of an existential approach. As a cognitive behavior therapist and author, Dattilio maintains that he directs much of his efforts to "helping clients make a deep existential shift—to a new understanding of the world" (p. 75). He uses techniques such as restructuring of belief systems, relaxation methods, and a variety of cognitive and behavioral strategies, but he does so within an existential framework that can begin the process of real-life transformation. Many of his clients suffer from panic attacks or depression. Dattilio often explores with these people existential themes of meaning, guilt, hopelessness, anxiety—and at the same time he provides them with cognitive behavioral tools to cope with the problems of daily living. In short, he grounds symptomatic treatment in an existential approach.

Some people have argued that the new trend toward positive psychology is similar to the existential approach, but this rests on a superficial comparison of these two approaches. Existential therapists favor intensity and passionate experience, including that of happiness, but they equally value the darker side of human nature and would encourage clients to learn to value both sides of their experience (van Deurzen, 2009).

Limitations and Criticisms of the Existential Approach

A major criticism often aimed at this approach is that it lacks a systematic statement of the principles and practices of psychotherapy. Some practitioners have trouble with what they perceive as its mystical language and concepts. Some therapists who claim adherence to an existential orientation describe their therapeutic style in vague and global terms such as *self-actualization, dialogic encounter, authenticity,* and *being in the world.* This particular use of language causes confusion at times and makes it difficult to conduct research on the process or outcomes of existential therapy.

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Both beginning and advanced practitioners who are not of a philosophical turn of mind tend to find many of the existential concepts lofty and elusive. As we have seen, this approach places primary emphasis on a subjective understanding of the world of clients. It is assumed that techniques follow understanding. The fact that few techniques are generated by this approach makes it essential for practitioners to develop their own innovative procedures or to borrow from other schools of therapy. For counselors who believe they need a specific set of techniques to counsel effectively, this approach has limitations (Vontress, 2008).

Practitioners who prefer a counseling practice based on research contend that the concepts should be empirically sound, that definitions should be operational, that the hypotheses should be testable, and that therapeutic practice should be based on the results of research into both the process and outcomes of counseling. Certainly, the notions of manualized therapy and evidence-based practice are not part of the existential perspective because every psychotherapy experience is unique (Walsh & McElwain, 2002). According to Cooper (2003), existential practitioners generally reject the idea that the therapeutic process can be measured and evaluated in quantitative and empirical ways. Although existential practices are generally upheld by recent research on therapeutic effectiveness (see Elkins, 2009), few studies directly evaluate and examine the existential approach. To a large extent, existential therapy makes use of techniques from other theories, which makes it difficult to apply research to this approach to study its effectiveness (Sharf, 2012).

According to van Deurzen (2002b), the main limitation of this approach is that of the level of maturity, life experience, and intensive training required of practitioners. Existential therapists need to be wise and capable of profound and wide-ranging understanding of what it means to be human. Authenticity is a cardinal characteristic of a competent existential practitioner, which is certainly more involved than mastering a body of knowledge and acquiring technical skills. Russell (2007) puts this notion nicely: "Authenticity means being able to sign your own name on your work and your life. It means you will want to take responsibility for creating your own way of being a therapist" (p.123).

WHERE TO GO FROM HERE

Refer to the *DVD for Integrative Counseling: The Case of Ruth and Lecturettes,* Session 11 ("Understanding How the Past Influences the Present and the Future"), for a demonstration of ways I utilize existential notions in counseling Ruth. We engage in a role play where Ruth becomes the voice of her church and I take on a new role as Ruth—one in which I have been willing to challenge certain beliefs from church. This segment illustrates how I assist Ruth in finding new values. In Session 12 ("Working Toward Decisions and Behavioral Changes") I challenge Ruth to make new decisions, which is also an existential concept.

Free Podcasts for ACA Members

You can download ACA Podcasts (prerecorded interviews) at www.counseling.org; click on the Resource button and then the Podcast Series. For Chapter 6, Existential Therapy, look for Podcast 14 by Dr. Gerald Corey.

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Other Resources

The American Psychological Association offers a DVD by K. J. Schneider (2009) titled *Existential-Humanistic Therapy* in their Systems of Psychotherapy Video Series.

Psychotherapy.net is a comprehensive resource for students and professionals that offers videos and interviews on existential therapy featuring Irvin Yalom, James Bugental, and Rollo May. New video and editorial content is made available monthly. DVDs relevant to this chapter are available at www.psychotherapy.net and include the following:

Bugental, J.F.T. (1995). *Existential-Humanistic Psychotherapy in Action*
Bugental, J. (1997). *Existential-Humanistic Psychotherapy* (Psychotherapy with the Experts Series)
Bugental, J. (2008). *James Bugental: Live Case Consultation*
May, R. (2007). *Rollo May on Existential Psychotherapy*
Yalom, I. (2002) *The Gift of Therapy: A Conversation with Irvin Yalom, M.D.*
Yalom, I. (2006). *Irvin Yalom: Live Case Consultation*
Yalom, I. (2011) *Confronting Death and Other Existential Issues in Psychotherapy*

If you are interesting in further information on Irvin Yalom, check out his website at www.yalom.com.

The Existential-Humanistic Institute, Dr. Kirk Schneider, Orah Krug, and Nader Shabahangi

Website:www.ehinstitute.org

The Existential-Humanistic Institute's (EHI) primary focus is training; offering courses, and in conjunction with Saybrook University, a new certificate program in existential-humanistic therapy and theory. A secondary focus is community building. EHI was formed as a nonprofit organization under the auspices of the Pacific Institute in 1997 and provides a home for those mental health professionals, scholars, and students who seek in-depth training in existential-humanistic theory and practice. EHI's year-long certificate program offers graduate and postgraduate students an opportunity to gain a basic foundation in the theory and practice of existential-humanistic therapy. EHI offers courses on the principles of existential-humanistic practice and case seminars in existential-humanistic theory and practice. Most of EHI's instructors have studied extensively with such masters as James Bugental, Irvin Yalom, and Rollo May, and are, like Kirk Schneider and Orah Krug, acknowledged leaders of the existential-humanistic movement today.

Society for Existential Analysis

Website: www.existentialanalysis.co.uk/
Additional Information: www.dilemmas.org

The Society for Existential Analysis is a professional organization devoted to exploring issues pertaining to an existential/phenomenological approach to counseling and therapy. Membership is open to anyone interested in this approach and includes students, trainees, psychotherapists, philosophers, psychiatrists, counselors, and psychologists. Members receive a regular newsletter and an

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annual copy of the *Journal of the Society for Existential Analysis*. The society provides a list of existentially oriented psychotherapists for referral purposes. The School of Psychotherapy and Counselling at Regent's College in London offers an advanced diploma in existential psychotherapy as well as short courses in the field.

International Society for Existential Psychotherapy and Counselling
Website: www.existentialpsychotherapy.net

The International Society for Existential Psychotherapy and Counselling was created in London in July 2006 and was renamed International Collaborative of Existential Counselors and Psychotherapists soon after (www.icecap.org.uk). It brings together the existing national societies as well as providing a forum for the development and accreditation of the approach.

Psychotherapy Training on the Net: SEPTIMUS
Website: www.septimus.info
Additional Information: www.psychotherapytraining.net

SEPTIMUS is an Internet-based course taught in Ireland, Iceland, Sweden, Poland, Czech Republic, Romania, Italy, Portugal, Austria, Belgium, France, Israel, Australia, and the United Kingdom.

New School of Psychotherapy and Counselling
Royal Waterloo House
51-55 Waterloo Road
London, England SE1 8TX
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E-mail: Admin@nspc.org.uk
Website: www.nspc.org.uk

The New School of Psychotherapy and Counselling (NSPC) is set up especially for training existential therapists. It offers an MA and doctoral program in Existential Psychotherapy and Counseling together with Middlesex University, as well as an MSC and professional doctorate in Existential Counseling Psychology and Psychotherapy also jointly with Middlesex University. NSPC offers intensive courses for distance learners (worldwide student body), including e-learning.

Recommended Supplementary Readings

Everyday Mysteries: A Handbook of Existential Psychotherapy (van Deurzen, 2010) provides a framework for practicing counseling from an existential perspective. The author puts into clear perspective topics such as anxiety, authentic living, clarifying one's worldview, determining values, discovering meaning, and coming to terms with life.

Existential Therapies (Cooper, 2003) provides a useful and clear introduction to the existential therapies. There are separate chapters on logotherapy, the British school of existential analysis, the U.S. existential-humanistic approach, dimensions of existential therapeutic practice, and brief existential therapies.

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Existential Psychotherapy (Yalom, 1980) is a superb treatment of the ultimate human concerns of death, freedom, isolation, and meaninglessness as these issues relate to therapy. This book has depth and clarity, and it is rich with clinical examples that illustrate existential themes.

Existential-Humanistic Therapy (Schneider & Krug, 2010) is a clear presentation of the theory and practice of existential-humanistic therapy. This approach incorporates techniques from other contemporary therapeutic approaches.

Existential-Integrative Psychotherapy: Guideposts to the Core of Practice (Schneider, 2008) is an edited book that offers recent and future trends in existential-integrative therapy and case illustrations of this model.

I Never Knew I Had a Choice (Corey & Corey, 2010) is a self-help book written from an existential perspective. Topics include our struggle to achieve autonomy; the meaning of loneliness, death, and loss; and how we choose our values and philosophy of life.

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CHAPTER 7



Person-Centered Therapy

INTRODUCTION

- Four Periods of Development of the Approach
- Existentialism and Humanism
- Abraham Maslow's Contributions to Humanistic Psychology

KEY CONCEPTS

- View of Human Nature

THE THERAPEUTIC PROCESS

- Therapeutic Goals
- Therapist's Function and Role
- Client's Experience in Therapy
- Relationship Between Therapist and Client

APPLICATION: THERAPEUTIC TECHNIQUES AND PROCEDURES

- Early Emphasis on Reflection of Feelings
- Evolution of Person-Centered Methods
- The Role of Assessment
- Application of the Philosophy of the Person-Centered Approach
- Application to Crisis Intervention
- Application to Group Counseling

PERSON-CENTERED EXPRESSIVE ARTS THERAPY

- Principles of Expressive Arts Therapy
- Creativity and Offering Stimulating Experiences
- Contributions of Natalie Rogers

MOTIVATIONAL INTERVIEWING

- The MI Spirit
- The Basic Principles of Motivational Interviewing
- The Stages of Change

PERSON-CENTERED THERAPY FROM A MULTICULTURAL PERSPECTIVE

- Strengths From a Diversity Perspective
- Shortcomings From a Diversity Perspective

PERSON-CENTERED THERAPY APPLIED TO THE CASE OF STAN

SUMMARY AND EVALUATION

- Summary
- Contributions of the Person-Centered Approach
- Limitations and Criticisms of the Person-Centered Approach

WHERE TO GO FROM HERE

- Recommended Supplementary Readings
- References and Suggested Readings

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Carl Rogers



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CARL ROGERS (1902–1987), a major spokesperson for humanistic psychology, led a life that reflected the ideas he developed for half a century. He showed a questioning stance, a deep openness to change, and the courage to forge into unknown territory both as a person and as a professional. In writing about his early years, Rogers (1961) recalled his family atmosphere as characterized by close and warm relationships but also by strict religious standards. Play was discouraged, and the virtues of the Protestant ethic were extolled. His boyhood was somewhat lonely, and he pursued scholarly interests instead of social ones. Rogers was an introverted person, and he spent a lot of time reading and engaging in imaginative activity and reflection. During his college years his interests and academic major changed from agriculture to history, then to religion, and finally to clinical psychology.

Rogers held academic positions in various fields, including education, social work, counseling, psychotherapy, group therapy, peace, and interpersonal relations, and he earned recognition around the world for originating and developing the humanistic movement in psychotherapy. His foundational ideas, especially the central role of the client–therapist relationship as a means to growth and change, have been incorporated by many other theoretical approaches. Rogers was a pioneer whose groundbreaking discoveries continue to have far-reaching effects on the field of psychotherapy (Cain, 2010).

It is difficult to overestimate the significance of Rogers's contributions to clinical and counseling psychology. He was a courageous pioneer who "was about 50 years ahead of his time and has been waiting for us to catch up" (Elkins, 2009, p. 20). Often called the "father of psychotherapy research," Rogers was the first to study the counseling process in depth by analyzing the transcripts of actual therapy sessions; he was the first clinician to conduct major studies on psychotherapy using quantitative methods; he was the first to formulate a comprehensive theory of personality and psychotherapy grounded in empirical research; and he contributed to developing a theory of psychotherapy that de-emphasized pathology and focused on the strengths and resources of individuals. He was not afraid to take a strong position and challenged the status quo throughout his professional career.

During the last 15 years of his life, Rogers applied the person-centered approach to world peace by training policymakers, leaders, and groups in conflict. Perhaps his greatest passion was directed toward the reduction of interracial tensions and the effort to achieve world peace, for which he was nominated for the Nobel Peace Prize.

For a detailed video presentation of the life and works of Carl Rogers, see *Carl Rogers: A Daughter's Tribute* (CD ROM, 2002), which is described at the end of this chapter. See also *Carl Rogers: The Quiet Revolutionary* (Rogers & Russell, 2002) and *The Life and Work of Carl Rogers* (Kirschenbaum, 2009).

INTRODUCTION

Of all the pioneers who have founded a therapeutic approach, for me Carl Rogers stands out as one of the most influential figures in revolutionizing the direction of counseling theory and practice. Rogers has become known as a "quiet revolutionary" who both contributed to theory development and whose influence continues to shape counseling practice today (see Cain, 2010; Kirschenbaum, 2009; Rogers & Russell, 2002).

The person-centered approach shares many concepts and values with the existential perspective presented in Chapter 6. Rogers's basic assumptions are

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that people are essentially trustworthy, that they have a vast potential for understanding themselves and resolving their own problems without direct intervention on the therapist's part, and that they are capable of self-directed growth if they are involved in a specific kind of therapeutic relationship. From the beginning, Rogers emphasized the attitudes and personal characteristics of the therapist and the quality of the client–therapist relationship as the prime determinants of the outcome of the therapeutic process. He consistently relegated to a secondary position matters such as the therapist's knowledge of theory and techniques. This belief in the client's capacity for self-healing is in contrast with many theories that view the therapist's techniques as the most powerful agents that lead to change (Bohart & Tallman, 2010). Clearly, Rogers revolutionized the field of psychotherapy by proposing a theory that centered on the client as the primary agent for constructive self-change (Bohart & Tallman, 2010; Bozarth, Zimring, & Tausch, 2002).

Contemporary person-centered therapy is the result of an evolutionary process that continues to remain open to change and refinement (see Cain, 2010; Cain & Seeman, 2002). Rogers did not present the person-centered theory as a fixed and completed approach to therapy. He hoped that others would view his theory as a set of tentative principles relating to how the therapy process develops, not as dogma. Rogers expected his model to evolve and was open and receptive to change.



See the video program for Chapter 7, *DVD for Theory and Practice of Counseling and Psychotherapy: The Case of Stan and Lecturettes*. I suggest that you view the brief lecture for each chapter prior to reading the chapter.

Four Periods of Development of the Approach

In tracing the major turning points in Rogers's approach, Zimring and Raskin (1992) and Bozarth, Zimring, and Tausch (2002) have identified four periods of development. In the first period, during the 1940s, Rogers developed what was known as *nondirective counseling*, which provided a powerful and revolutionary alternative to the directive and interpretive approaches to therapy then being practiced. While he was a professor at Ohio State University, Rogers (1942) published *Counseling and Psychotherapy: Newer Concepts in Practice*, which described the philosophy and practice of nondirective counseling. Rogers's theory emphasized the counselor's creation of a permissive and nondirective climate. His theory took power away from the therapist and honored the inherent power of the client. When he challenged the basic assumption that "the counselor knows best," he realized this radical idea would affect the power dynamics and politics of the counseling profession, and indeed it caused a great furor (Elkins, 2009).

Rogers also challenged the validity of commonly accepted therapeutic procedures such as advice, suggestion, direction, persuasion, teaching, diagnosis, and interpretation. Based on his conviction that diagnostic concepts and procedures were inadequate, prejudicial, and often misused, Rogers omitted them from his approach. Nondirective counselors avoided sharing a great deal about themselves with clients and instead focused mainly on reflecting and clarifying the clients' verbal and nonverbal communications.

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In the second period, during the 1950s, Rogers (1951) wrote *Client-Centered Therapy* and renamed his approach *client-centered therapy,* to reflect its emphasis on the *client* rather than on nondirective methods. In addition, he started the Counseling Center at the University of Chicago. This period was characterized by a shift from clarification of feelings to a focus on the phenomenological world of the client. Rogers assumed that the best vantage point for understanding how people behave was from their own internal frame of reference. He focused more explicitly on the actualizing tendency as the basic motivational force that leads to client change.

The third period, which began in the late 1950s and extended into the 1970s, addressed the necessary and sufficient conditions of therapy. Rogers (1957) set forth a hypothesis that resulted in three decades of research. A significant publication was *On Becoming a Person* (Rogers, 1961), which addressed the nature of "becoming the self that one truly is," an idea he borrowed from Kierkegaard. Rogers published this work during the time that he held joint appointments in the departments of psychology and psychiatry at the University of Wisconsin. In this book he described the process of "becoming one's experience," which is characterized by an openness to experience, a trust in one's experience, an internal locus of evaluation, and the willingness to be in process. During the 1950s and 1960s, Rogers and his associates continued to test the underlying hypotheses of the client-centered approach by conducting extensive research on both the process and the outcomes of psychotherapy. He was interested in how people best progress in psychotherapy, and he studied the qualities of the client–therapist relationship as a catalyst leading to personality change. On the basis of this research the approach was further refined and expanded (Rogers, 1961). For example, client-centered philosophy was applied to education and was called *student-centered teaching* (Rogers & Freiberg, 1994). The approach was also applied to encounter groups (Rogers, 1970).

The fourth phase, during the 1980s and the 1990s, was marked by considerable expansion to education, couples and families, industry, groups, conflict resolution, politics, and the search for world peace. Because of Rogers's ever-widening scope of influence, including his interest in how people obtain, possess, share, or surrender power and control over others and themselves, his theory became known as the *person-centered approach*. This shift in terms reflected the broadening application of the approach. Although the person-centered approach has been applied mainly to individual and group counseling, important areas of further application include education, family life, leadership and administration, organizational development, health care, cross-cultural and interracial activity, and international relations. It was during the 1980s that Rogers directed his efforts toward applying the person-centered approach to politics, especially to efforts related to the achievement of world peace.

In a comprehensive review of the research on person-centered therapy over a period of 60 years, Bozarth, Zimring, and Tausch (2002) concluded the following:

- In the earliest years of the approach, the client rather than the therapist determined the direction and goals of therapy and the therapist's role was to help the client clarify feelings. This style of nondirective therapy was associated with increased understanding, greater self-exploration, and improved self-concepts.

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- Later a shift from clarification of feelings to a focus on the client's lived experiences took place. Many of Rogers's hypotheses were confirmed, and there was strong evidence for the value of the therapeutic relationship and the client's resources as the crux of successful therapy.
- As person-centered therapy developed further, research centered on the core conditions assumed to be both necessary and sufficient for successful therapy. The attitude of the therapist—an empathic understanding of the client's world and the ability to communicate a nonjudgmental stance to the client—along with the therapist's genuineness were found to be basic to a successful therapy outcome.
- The main source of successful psychotherapy is the client. The therapist's attention to the client's frame of reference fosters the client's utilization of inner and outer resources.

Existentialism and Humanism

In the 1960s and 1970s there was a growing interest among counselors in a "third force" in therapy as an alternative to the psychoanalytic and behavioral approaches. Under this heading fall existential therapy (Chapter 6), person-centered therapy (Chapter 7), Gestalt therapy (Chapter 8), and certain other experiential and relationship-oriented approaches.

Partly because of this historical connection and partly because representatives of existentialist thinking and humanistic thinking have not always clearly sorted out their views, the connections between the terms *existentialism* and *humanism* have tended to be confusing for students and theorists alike. The two viewpoints have much in common, yet there also are significant philosophical differences between them. They share a respect for the client's subjective experience, the uniqueness and individuality of each client, and a trust in the capacity of the client to make positive and constructive conscious choices. They have in common an emphasis on concepts such as freedom, choice, values, personal responsibility, autonomy, purpose, and meaning. Both approaches place little value on the role of techniques in the therapeutic process and emphasize instead the importance of genuine encounter. They differ in that existentialists take the position that we are faced with the anxiety of choosing to create an identity in a world that lacks intrinsic meaning. Existentialists tend to acknowledge the stark realities of human experience, and their writings often focus on death, anxiety, depression, and isolation. The humanists, in contrast, take the somewhat less anxiety-evoking position and more optimistic view that each of us has a natural potential that we can actualize and through which we can find meaning. Many contemporary existential therapists refer to themselves as *existential-humanistic* practitioners, indicating that their roots are in existential philosophy but that they have incorporated many aspects of North American humanistic psychotherapies (Cain, 2002a; Schneider & Krug, 2010).

As will become evident in this chapter, the existential and person-centered approaches have parallel concepts with regard to the client–therapist relationship at the core of therapy. The phenomenological emphasis that is basic to the existentialist approach is also fundamental to person-centered theory. Both approaches focus on the client's perceptions and call for the therapist to be fully present with the client

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so that it is possible to understand the client's subjective world, and they both emphasize the client's capacity for self-awareness and self-healing. The therapist aims to provide the client with a safe, responsive, and caring relationship to facilitate self-exploration, growth, and healing (Watson, Goldman, & Greenberg, 2011).

Abraham Maslow's Contributions to Humanistic Psychology

Abraham Maslow (1970) was a pioneer in the development of humanistic psychology and was influential in furthering the understanding of self-actualizing individuals. Many of Carl Rogers's ideas, especially on the positive aspects of being human and the fully functioning person, are built on Maslow's basic philosophy. Maslow criticized Freudian psychology for what he saw as its preoccupation with the sick and negative side of human nature. Maslow believed too much research was being conducted on anxiety, hostility, and neuroses and too little into joy, creativity, and self-fulfillment. Self-actualization was the central theme of the work of Abraham Maslow (1968, 1970, 1971). The *positive psychology* movement that recently has come into prominence shares many concepts on the healthy side of human existence with the humanistic approach.

Maslow studied what he called "self-actualizing people" and found that they differed in important ways from so-called normal individuals. The core characteristics of self-actualizing people are self-awareness, freedom, basic honesty and caring, and trust and autonomy. Other characteristics of self-actualizing individuals include a capacity to welcome uncertainty in their lives, acceptance of themselves and others, spontaneity and creativity, a need for privacy and solitude, autonomy, a capacity for deep and intense interpersonal relationships, a genuine caring for others, an inner-directedness (as opposed to the tendency to live by others' expectations), the absence of artificial dichotomies within themselves (such as work/play, love/hate, and weak/strong), and a sense of humor (Maslow, 1970). All of these personal characteristics have been identified by Rogers as being central to the person-centered philosophy.

Maslow postulated a *hierarchy of needs* as a source of motivation, with the most basic needs being physiological needs. If we are hungry and thirsty, our attention is riveted on meeting these basic needs. Next are the safety needs, which include a sense of security and stability. Once our physical and safety needs are fulfilled, we become concerned with meeting our needs for belonging and love, followed by working on our need for esteem, both from self and others. We are able to strive toward self-actualization only after these four basic needs are met. The key factor determining which need is dominant at a given time is the degree to which those below it are satisfied. We cannot strive toward self-actualization, for example, if our self-esteem is low.

THE VISION OF HUMANISTIC PHILOSOPHY The underlying vision of humanistic philosophy is captured by the metaphor of how an acorn, if provided with the appropriate conditions, will "automatically" grow in positive ways, pushed naturally toward its actualization as an oak. In contrast, for many existentialists there is nothing that we "are," no internal "nature" we can count on. We are faced

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at every moment with a choice about what to make of this condition. Maslow's emphasis on the healthy side of being human and the emphasis on joy, creativity, and self-fulfillment are part of the person-centered philosophy. The humanistic philosophy on which the person-centered approach rests is expressed in attitudes and behaviors that create a growth-producing climate. According to Rogers (1986b), when this philosophy is lived, it helps people develop their capacities and stimulates constructive change in others. Individuals are empowered, and they are able to use this power for personal and social transformation.

KEY CONCEPTS

View of Human Nature

A common theme originating in Rogers's early writing and continuing to permeate all of his works is a basic sense of trust in the client's ability to move forward in a constructive manner if conditions fostering growth are present. His professional experience taught him that if one is able to get to the core of an individual, one finds a trustworthy, positive center (Rogers, 1987a). In keeping with the philosophy of humanistic psychology, Rogers firmly maintained that people are trustworthy, resourceful, capable of self-understanding and self-direction, able to make constructive changes, and able to live effective and productive lives. When therapists are able to experience and communicate their realness, support, caring, and nonjudgmental understanding, significant changes in the client are most likely to occur.

Rogers maintained that three therapist attributes create a growth-promoting climate in which individuals can move forward and become what they are capable of becoming: (1) *congruence* (genuineness, or realness), (2) *unconditional positive regard* (acceptance and caring), and (3) *accurate empathic understanding* (an ability to deeply grasp the subjective world of another person). According to Rogers, if therapists communicate these attitudes, those being helped will become less defensive and more open to themselves and their world, and they will behave in prosocial and constructive ways.

Brodley (1999) writes about the **actualizing tendency,** a directional process of striving toward realization, fulfillment, autonomy, and self-determination. This natural inclination of humans is based on Maslow's (1970) studies of self-actualizing people. This growth force within us provides an internal source of healing, but it does not imply a movement away from relationships, interdependence, connection, or socialization. This humanistic view of human nature has significant implications for the practice of therapy. Because of the belief that the individual has an inherent capacity to move away from maladjustment and toward psychological health and growth, the therapist places the primary responsibility on the client. The person-centered approach rejects the role of the therapist as the authority who knows best and of the passive client who merely follows the beliefs of the therapist. Therapy is rooted in the client's capacity for awareness and self-directed change in attitudes and behavior.

In the person-centered approach the emphasis is on how clients act in their world with others, how they can move forward in constructive directions, and how

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they can successfully deal with obstacles (both from within themselves and outside of themselves) that are blocking their growth. By promoting self-awareness and self-reflection, clients learn to exercise choice. Humanistic therapists emphasize a discovery-oriented approach in which clients are the experts on their own inner experience (Watson, Goldman, & Greenberg, 2011), and they encourage clients to make changes that will lead to living fully and authentically, with the realization that this kind of existence demands a continuing struggle. Maslow taught us that becoming self-actualizing individuals is an ongoing process rather than a final destination.

THE THERAPEUTIC PROCESS

Therapeutic Goals

The person-centered approach aims toward the client achieving a greater degree of independence and integration. Its focus is on the person, not on the person's presenting problem. Rogers did not believe the goal of therapy was merely to solve problems. Rather, the goal is to assist clients in their growth process so clients can better cope with problems as they identify them.

Rogers (1961) wrote that people who enter psychotherapy often ask: "How can I discover my real self? How can I become what I deeply wish to become? How can I get behind my facades and become myself?" The underlying aim of therapy is to provide a climate conducive to helping the individual strive toward self-actualization. Before clients are able to work toward that goal, they must first get behind the masks they wear, which they develop through the process of socialization. Clients come to recognize that they have lost contact with themselves by using facades. In a climate of safety in the therapeutic session, they also come to realize that there are more authentic ways of being.

When the facades are put aside during the therapeutic process, what kind of person emerges from behind the pretenses? Rogers (1961) described people who are becoming increasingly actualized as having (1) an openness to experience, (2) a trust in themselves, (3) an internal source of evaluation, and (4) a willingness to continue growing. Encouraging these characteristics is the basic goal of person-centered therapy.

These four characteristics provide a general framework for understanding the direction of therapeutic movement. The therapist does not choose specific goals for the client. The cornerstone of person-centered theory is the view that clients in a relationship with a facilitating therapist have the capacity to define and clarify their own goals. Person-centered therapists are in agreement on the matter of not setting goals for *what* clients need to change, yet they differ on the matter of *how* to best help clients achieve their own goals and to find their own answers (Bohart & Watson, 2011).

Therapist's Function and Role

The role of person-centered therapists is rooted in their ways of being and attitudes, not in techniques designed to get the client to "do something." Research on person-centered therapy seems to indicate that the attitude of therapists, rather than

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their knowledge, theories, or techniques, facilitate personality change in the client (Rogers, 1961). Basically, therapists use themselves as an instrument of change. When they encounter the client on a person-to-person level, their "role" is to be without roles. They do not get lost in a professional role. It is the therapist's attitude and belief in the inner resources of the client that creates the therapeutic climate for growth (Bozarth et al., 2002).

Thorne (2002a) reinforces the importance of therapists encountering clients in a person-to-person way, as opposed to being overly reliant on a professional contract. He cautions about retreating into a stance of pseudo-professionalism characterized by presenting a detailed contract to clients, rigid observation of boundaries, and the commitment to empirically validated methods. He suggests that this overemphasis on professionalism is aimed at protecting therapists from overinvolvement with clients, which often results in underinvolvement with them.

Person-centered theory holds that the therapist's function is to be present and accessible to clients and to focus on their immediate experience. First and foremost, the therapist must be willing to be real in the relationship with clients. By being congruent, accepting, and empathic, the therapist is a catalyst for change. Instead of viewing clients in preconceived diagnostic categories, the therapist meets them on a moment-to-moment experiential basis and enters their world. Through the therapist's attitude of genuine caring, respect, acceptance, support, and understanding, clients are able to loosen their defenses and rigid perceptions and move to a higher level of personal functioning. When these therapist attitudes are present, clients then have the necessary freedom to explore areas of their life that were either denied to awareness or distorted.

Client's Experience in Therapy

Therapeutic change depends on clients' perceptions both of their own experience in therapy and of the counselor's basic attitudes. If the counselor creates a climate conducive to self-exploration, clients have the opportunity to explore the full range of their experience, which includes their feelings, beliefs, behavior, and worldview. What follows is a general sketch of clients' experiences in therapy.

Clients come to the counselor in a state of incongruence; that is, a discrepancy exists between their self-perception and their experience in reality. For example, Leon, a college student, may see himself as a future physician, yet his below-average grades could exclude him from medical school. The discrepancy between how Leon sees himself (self-concept) or how he would *like* to view himself (ideal self-concept) and the reality of his poor academic performance may result in anxiety and personal vulnerability, which can provide the necessary motivation to enter therapy. Leon must perceive that a problem exists or, at least, that he is uncomfortable enough with his present psychological adjustment to want to explore possibilities for change.

One reason clients seek therapy is a feeling of basic helplessness, powerlessness, and an inability to make decisions or effectively direct their own lives. They may hope to find "the way" through the guidance of the therapist. Within the person-centered framework, however, clients soon learn that they can be responsible

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for themselves in the relationship and that they can learn to be more free by using the relationship to gain greater self-understanding.

As counseling progresses, clients are able to explore a wider range of beliefs and feelings. They can express their fears, anxiety, guilt, shame, hatred, anger, and other emotions that they had deemed too negative to accept and incorporate into their self-structure. With therapy, people distort less and move to a greater acceptance and integration of conflicting and confusing feelings. They increasingly discover aspects within themselves that had been kept hidden. As clients feel understood and accepted, they become less defensive and become more open to their experience. Because they feel safer and are less vulnerable, they become more realistic, perceive others with greater accuracy, and become better able to understand and accept others. Individuals in therapy come to appreciate themselves more as they are, and their behavior shows more flexibility and creativity. They become less concerned about meeting others' expectations, and thus begin to behave in ways that are truer to themselves. These individuals direct their own lives instead of looking outside of themselves for answers. They move in the direction of being more in contact with what they are experiencing at the present moment, less bound by the past, less determined, freer to make decisions, and increasingly trusting in themselves to manage their own lives. In short, their experience in therapy is like throwing off the self-imposed shackles that had kept them in a psychological prison. With increased freedom they tend to become more mature psychologically and move toward increased self-actualization.

Person-centered therapy is grounded on the assumption that it is clients who heal themselves, who create their own self-growth, and who are active self-healers (Bohart & Tallman, 1999, 2010; Bohart & Watson, 2011). The therapy relationship provides a supportive structure within which clients' self-healing capacities are activated. What clients value most is being understood and accepted, which results in creating a safe place to explore feelings, thoughts, behaviors, and experiences; clients also value support for trying out new behaviors (Bohart & Tallman, 2010).

Relationship Between Therapist and Client

Rogers (1957) based his hypothesis of the "necessary and sufficient conditions for therapeutic personality change" on the quality of the relationship: "If I can provide a certain type of relationship, the other person will discover within himself or herself the capacity to use that relationship for growth and change, and personal development will occur" (Rogers, 1961, p. 33). Rogers (1967) hypothesized further that "significant positive personality change does not occur except in a relationship" (p. 73). Rogers's hypothesis was formulated on the basis of many years of his professional experience, and it remains basically unchanged to this day. This hypothesis (cited in Cain 2002a, p. 20) is stated thusly:

1. Two persons are in psychological contact.
2. The first, whom we shall term the client, is in a state of incongruence, being vulnerable or anxious.
3. The second person, whom we term the therapist, is congruent (real or genuine) in the relationship.
4. The therapist experiences unconditional positive regard for the client.

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5. The therapist experiences an empathic understanding of the client's internal frame of reference and endeavors to communicate this experience to the client.
6. The communication to the client of the therapist's empathic understanding and unconditional positive regard is to a minimal degree achieved.

Rogers hypothesized that no other conditions were necessary. If the **therapeutic core conditions** exist over some period of time, constructive personality change will occur. The core conditions do not vary according to client type. Further, they are both necessary *and* sufficient for therapeutic change to occur.

From Rogers's perspective the client–therapist relationship is characterized by equality. Therapists do not keep their knowledge a secret or attempt to mystify the therapeutic process. The process of change in the client depends to a large degree on the quality of this equal relationship. As clients experience the therapist listening in an accepting way to them, they gradually learn how to listen acceptingly to themselves. As they find the therapist caring for and valuing them (even the aspects that have been hidden and regarded as negative), clients begin to see worth and value in themselves. As they experience the realness of the therapist, clients drop many of their pretenses and are real with both themselves and the therapist.

This humanistic approach is perhaps best characterized as a *way of being* and as a *shared journey* in which therapist and client reveal their humanness and participate in a growth experience. The therapist can be a relational guide on this journey because he or she is usually more experienced and more psychologically experienced in this role than the client. Thorne (2002b) delivered this message: "Therapists cannot confidently invite their clients to travel further than they have journeyed themselves, but for person-centred therapists the quality, depth and continuity of their own experiencing becomes the very cornerstone of the competence they bring to their professional activity" (p. 144). Therapists are invested in broadening their own life experiences and are willing to do what it takes to deepen their self-knowledge.

Rogers admitted that his theory was striking provocative and radical. His formulation has generated considerable controversy, for he maintained that many conditions other therapists commonly regard as necessary for effective psychotherapy were nonessential. The core therapist conditions of congruence, unconditional positive regard, and accurate empathic understanding subsequently have been embraced by many therapeutic schools as essential in facilitating therapeutic change. These core qualities of therapists, along with the therapist's presence, work holistically to create a safe environment for learning (Cain, 2010). Regardless of theoretical orientation, most therapists strive to listen fully and empathically to clients, especially during the initial stages of therapy. We now turn to a detailed discussion of how these core conditions are an integral part of the therapeutic relationship.

CONGRUENCE, OR GENUINENESS implies that therapists are real; that is, they are genuine, integrated, and authentic during the therapy hour. They are without a false front, their inner experience and outer expression of that experience match, and they can openly express feelings, thoughts, reactions, and attitudes that are present in the relationship with the client. This communication is done with careful reflection and considered judgment on the therapist's part (Kolden, Klein, Wang, & Austin, 2011).

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Through authenticity the therapist serves as a model of a human being struggling toward greater realness. Being congruent might necessitate expressing a range of feelings including anger, frustration, liking, concern, and annoyance. This does not mean that therapists should impulsively share all their reactions, for self-disclosure must be appropriate, well timed, and have a constructive therapeutic intent. Counselors can try too hard to be genuine; sharing because one thinks it will be good for the client, without being genuinely moved to express something regarded as personal, can be incongruent. Person-centered therapy stresses that counseling will be inhibited if the counselor feels one way about the client but acts in a different way. For example, if the practitioner dislikes or disapproves of the client but feigns acceptance, therapy will be impaired. Cain (2010) stresses that therapists need to be attuned to the emerging needs of the client and to respond in ways that are in the best interests of the individual. If therapists keep this in mind, they are likely to make sound clinical decisions most of the time.

Rogers's concept of congruence does not imply that only a fully self-actualized therapist can be effective in counseling. Because therapists are human, they cannot be expected to be fully authentic. Congruence exists on a continuum from highly congruent to very incongruent. This is true of all three characteristics.

UNCONDITIONAL POSITIVE REGARD AND ACCEPTANCE The second attitude therapists need to communicate is deep and genuine caring for the client as a person, or a condition of **unconditional positive regard**, which can best be achieved through empathic identification with the client (Farber & Doolin, 2011). The caring is nonpossessive and is not contaminated by evaluation or judgment of the client's feelings, thoughts, and behavior as good or bad. Therapists value and warmly accept clients without placing stipulations on their acceptance. It is not an attitude of "I'll accept you when . . ."; rather, it is one of "I'll accept you as you are." Therapists communicate through their behavior that they value their clients as they are and that clients are free to have feelings and experiences. Acceptance is the recognition of clients' rights to have their own beliefs and feelings; it is not the approval of all behavior. All overt behavior need not be approved of or accepted.

According to Rogers's (1977) research, the greater the degree of caring, prizing, accepting, and valuing of the client in a nonpossessive way, the greater the chance that therapy will be successful. He also makes it clear that it is not possible for therapists to genuinely feel acceptance and unconditional caring at all times. However, if therapists have little respect for their clients, or an active dislike or disgust, it is not likely that the therapeutic work will be fruitful. If therapists' caring stems from their own need to be liked and appreciated, constructive change in the client is inhibited. This notion of positive regard has implications for all therapists, regardless of their theoretical orientation (Farber & Doolin, 2011).

ACCURATE EMPATHIC UNDERSTANDING One of the main tasks of the therapist is to understand clients' experience and feelings sensitively and accurately as they are revealed in the moment-to-moment interaction during the therapy session. The therapist strives to sense clients' subjective experience, particularly in the here and now. The aim is to encourage clients to get closer to themselves, to feel more deeply and intensely, and to recognize and resolve the incongruity that exists within them.

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Empathy is a deep and subjective understanding of the client *with* the client. Empathy is not sympathy, or feeling sorry for a client. Therapists are able to share the client's subjective world by drawing from their own experiences that may be similar to the client's feelings. Yet therapists must not lose their own separateness. Rogers asserts that when therapists can grasp the client's private world as the client sees and feels it—without losing the separateness of their own identity—constructive change is likely to occur. Empathy, particularly emotionally focused empathy, helps clients (1) pay attention to and value their experiencing, (2) process their experience both cognitively and bodily, (3) view prior experiences in new ways, and (4) increase their confidence in making choices and in pursuing a course of action (Cain, 2010).

Clark (2010) describes an integral model of empathy in the counseling process that is based on three ways of knowing: (1) *subjective empathy* enables practitioners to experience what it is like to be the client; (2) *interpersonal empathy* pertains to understanding a client's internal frame of reference and conveying a sense of the private meanings to the person; and (3) *objective empathy* relies on knowledge sources outside of a client's frame of reference. By using a multiple-perspective model of empathy, counselors have a broader way to understand clients.

Accurate empathy is the cornerstone of the person-centered approach, and it is a necessary ingredient of any effective therapy (Cain, 2010). **Accurate empathic understanding** implies that the therapist will sense clients' feelings *as if* they were his or her own without becoming lost in those feelings. It is a way for therapists to hear the meanings expressed by their clients that often lie at the edge of their awareness. A primary means of determining whether an individual experiences a therapist's empathy is to secure feedback from the client (Norcross, 2010). According to Watson (2002), full empathy entails understanding the meaning and feeling of a client's experiencing. Therapists need to understand clients on both emotional and cognitive levels. Empathy is an active ingredient of change that facilitates clients' cognitive processes and emotional self-regulation. Watson (2002) states that 60 years of research has consistently demonstrated that empathy is the most powerful determinant of client progress in therapy.

Clients' perceptions of feeling understood by their therapists relate favorably to outcome. Empathic therapists strive to discover the meaning of the client's experience, understand the overall goals of the client, and tailor their responses to the particular client. Effective empathy is grounded in authentic caring for the client (Elliott, Bohart, Watson, & Greenberg, 2011).

APPLICATION: THERAPEUTIC TECHNIQUES AND PROCEDURES

Early Emphasis on Reflection of Feelings

Rogers's original emphasis was on grasping the world of the client and reflecting this understanding. As his view of psychotherapy developed, however, his focus shifted away from a nondirective stance and emphasized the therapist's relationship with the client. Many followers of Rogers simply imitated his reflective style,

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and client-centered therapy has often been identified primarily with the technique of reflection despite Rogers's contention that the therapist's relational attitudes and fundamental ways of being with the client constitute the heart of the change process. Rogers and other contributors to the development of the person-centered approach have been critical of the stereotypic view that this approach is basically a simple restatement of what the client just said.

Evolution of Person-Centered Methods

Contemporary person-centered therapy is the result of an evolutionary process of more than 70 years, and it continues to remain open to change and refinement. One of Rogers's main contributions to the counseling field is the notion that the quality of the therapeutic relationship, as opposed to administering techniques, is the primary agent of growth in the client. The therapist's ability to establish a strong connection with clients is *the* critical factor determining successful counseling outcomes.

No techniques are basic to the practice of person-centered therapy; "being with" clients and entering imaginatively into their world of perceptions and feelings is sufficient for facilitating a process of change. Person-centered therapists are not prohibited from suggesting techniques, but *how* these suggestions are presented is crucial. Techniques may be suggested when doing so fosters the process of client and therapist being together in an empathic way. Techniques are not attempts at "doing anything" to a client (Bohart & Watson, 2011).

The person-centered philosophy is based on the assumption that clients have the resourcefulness for positive movement without the counselor assuming an active, directive, or problem-solving role. Traditional person-centered therapists would not tend to suggest a technique (Bohart & Watson, 2011). What is essential for clients' progress is the therapist's **presence**, being completely attentive to and immersed in the client as well as in the client's expressed concerns (Cain, 2010). This way of being is far more powerful than any technique a therapist might use to bring about change. Qualities and skills such as listening, accepting, respecting, understanding, and responding must be honest expressions by the therapist. As discussed in Chapter 2, counselors need to evolve as persons, not just acquire a repertoire of therapeutic strategies. Therapists mainly stay within the client's frame of reference and strive to understand and reflect the client's communication and experience.

Rogers expected person-centered therapy to continue to evolve and supported others in breaking new ground. One of the main ways in which person-centered therapy has evolved is the diversity, innovation, and individualization in practice. There is no longer one way of practicing person-centered therapy (Cain, 2010), and there has been increased latitude for therapists to share their reactions, to confront clients in a caring way, and to participate more actively and fully in the therapeutic process (Bozarth et al., 2002). **Immediacy**, or addressing what is going on between the client and therapist, is highly valued in this approach. This development encourages the use of a wider variety of methods and allows for considerable diversity in personal style among person-centered therapists. The shift toward genuineness

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enables person-centered therapists both to practice in more flexible and integrative ways that suit their personalities and to have greater flexibility in tailoring the counseling relationship to suit different clients (Bohart & Watson, 2011).

Cain (2008, 2010) believes it is essential for therapists to adapt their therapeutic style to accommodate the unique needs of each client. Person-centered therapists have the freedom to use a variety of responses and methods to assist their clients; a guiding question therapists need to ask is, "Does it fit?" Cain contends that, ideally, therapists will continually monitor whether what they are doing fits, especially whether their therapeutic style is compatible with their clients' way of viewing and understanding their problems. For an illustration of how Dr. David Cain works with the case of Ruth in a person-centered style, see *Case Approach to Counseling and Psychotherapy* (Corey, 2013, chap. 5).

Today, those who practice a person-centered approach work in diverse ways that reflect both advances in theory and practice and a plethora of personal styles. This is appropriate and fortunate, for none of us can emulate the style of Carl Rogers and still be true to ourselves. If we strive to model our style after Rogers, and if that style does not fit for us, we are not being ourselves and we are not being fully congruent.

The Role of Assessment

Assessment is frequently viewed as a prerequisite to the treatment process. Many mental health agencies use a variety of assessment procedures, including diagnostic screening, identification of clients' strengths and liabilities, and various tests. Person-centered therapists generally do not find traditional assessment and diagnosis to be useful because these procedures encourage an external and expert perspective on the client (Bohart & Watson, 2011). What matters is not how the counselor assesses the client but the client's self-assessment. From a person-centered perspective, the best source of knowledge about the client is the individual client.

In the early development of nondirective therapy, Rogers (1942) recommended caution in using psychometric measures or in taking a complete case history at the outset of counseling. If a counseling relationship began with a battery of psychological tests and a detailed case history, he believed clients could get the impression that the counselor would be providing the solutions to their problems. Assessment seems to be gaining in importance in short-term treatments in most counseling agencies, and it is imperative that clients be involved in a collaborative process in making decisions that are central to their therapy. Today it may not be a question of whether to incorporate assessment into therapeutic practice but of *how* to involve clients as fully as possible in their assessment and treatment process.

Application of the Philosophy of the Person-Centered Approach

The person-centered approach has been applied to working with individuals, groups, and families. Bozrath, Zimring, and Tausch (2002) cite studies done through the 1990s that revealed the effectiveness of person-centered therapy with a wide range of client problems including anxiety disorders, alcoholism, psychosomatic problems, agoraphobia, interpersonal difficulties, depression, cancer, and personality disorders.

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Person-centered therapy has been shown to be as viable as the more goal-oriented therapies. Furthermore, outcome research conducted in the 1990s revealed that effective therapy is based on the client–therapist relationship in combination with the inner and external resources of the client (Duncan, Miller, Wampold, & Hubble, 2010).

The person-centered approach has been applied extensively in training both professionals and paraprofessionals who work with people in a variety of settings. This approach emphasizes staying with clients as opposed to getting ahead of them with interpretations. People without advanced psychological education are able to benefit by translating the therapeutic conditions of genuineness, empathic understanding, and unconditional positive regard into both their personal and professional lives. Learning to listen with acceptance to oneself is a valuable life skill that enables individuals to be their own therapists. The basic concepts are straightforward and easy to comprehend, and they encourage locating power in the person rather than fostering an authoritarian structure in which control and power are denied to the person. These core skills also provide an essential foundation for virtually all of the other therapy systems covered in this book. If counselors are lacking in these relationship and communication skills, they will not be effective in carrying out a treatment program for their clients.

The person-centered approach demands a great deal of the therapist. An effective person-centered therapist must be grounded, centered, genuine, respectful, caring, present, a focused and astute listener, patient, and accepting in a way that involves maturity. Without a person-centered *way of being*, mere application of skills is likely to be hollow. Natalie Rogers (2011) points out that the person-centered approach is a way of being that is easy to understand intellectually but is very difficult to put into practice. She continues to find the core conditions of genuineness, positive regard, and empathy most important in developing trust, safety, and growth in a group.

Application to Crisis Intervention

The person-centered approach is especially applicable in crisis intervention such as an unwanted pregnancy, an illness, a disastrous event, or the loss of a loved one. People in the helping professions (nursing, medicine, education, the ministry) are often first on the scene in a variety of crises, and they can do much if the basic attitudes described in this chapter are present. When people are in crisis, one of the first steps is to give them an opportunity to fully express themselves. Sensitive listening, hearing, and understanding are essential at this point. Being heard and understood helps ground people in crises, helps to calm them in the midst of turmoil, and enables them to think more clearly and make better decisions. Although a person's crisis is not likely to be resolved by one or two contacts with a helper, such contacts can pave the way for being open to receiving help later. If the person in crisis does not feel understood and accepted, he or she may lose hope of "returning to normal" and may not seek help in the future. Genuine support, caring, and nonpossessive warmth can go a long way in building bridges that can motivate people to *do* something to work through and resolve a crisis. Communicating a deep sense of understanding should always precede other more problem-solving interventions.

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In crisis situations person-centered therapists may need to provide more structure and direction than would be the case for clients who are not experiencing a crisis. Suggestions, guidance, and even direction may be called for if clients are not able to function effectively. For example, in certain cases it may be necessary to take action to hospitalize a suicidal client to protect this person from self-harm.

Application to Group Counseling

The person-centered approach emphasizes the unique role of the group counselor as a facilitator rather than a leader. The primary function of the facilitator is to create a safe and healing climate—a place where the group members can interact in honest and meaningful ways. In this climate members become more appreciative and trusting of themselves as they are and are able to move toward self-direction and empowerment. Ultimately, group members make their own choices and bring about change for themselves. Yet with the presence of the facilitator and the support of other members, participants realize that they do not have to experience the struggles of change alone and that groups as collective entities have their own source of transformation. The facilitator's way of being can create a productive climate within a group:

> Facilitators cannot make participants trust the group process. Facilitators earn trust by being respectful, caring, and even loving. Being an effective group facilitator has much to do with one's "way of being." No method or technique can evoke trust unless the facilitator herself has a capacity to be fully present, considerate, caring, authentic, and responsive. This includes the ability to challenge people constructively (N. Rogers, 2011, p. 57)

Rogers (1970) clearly believed that groups tend to move forward if the facilitator exhibits a deep sense of trust in the members and refrains from using techniques or exercises to get a group moving. The core conditions of person-centered therapy apply to the process of a group. The role of the facilitator is to empathically understand what an individual is communicating within the group. Instead of leading the members toward specific goals, the group facilitator assists members in developing attitudes and behaviors of genuineness, acceptance, and empathy, which enables the members to interact with each other in therapeutic ways to find their own sense of direction as a group.

From Rogers's perspective, facilitators should avoid making interpretive comments or group process observations because such comments are apt to make the group self-conscious and slow the process down. Group process observations should come from members, a view that is consistent with Rogers's philosophy of placing the responsibility for the direction of the group on the members. According to Raskin, Rogers, and Witty (2008), groups are fully capable of articulating and pursuing their own goals. They assert, "when the therapeutic conditions are present in a group and when the group is trusted to find its own way of being, group members tend to develop processes that are right for them and to resolve conflicts within time constraints in the situation" (p. 143).

Regardless of a group leader's theoretical orientation, the core conditions that have been described here are highly applicable to any leader's style of group facilitation. Only when the leader is able to create a person-centered climate will movement take place within a group. All of the theories discussed in this book depend

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on the quality of the therapeutic relationship as a foundation. As you will see, the cognitive behavioral approaches to group work place emphasis on creating a working alliance and collaborative relationships. In this way, most effective approaches to group work share key elements of a person-centered philosophy. For a more detailed treatment of person-centered group counseling, see Corey (2012, chap. 10). Also see Natalie Rogers's (2011), *The Creative Connection for Groups: Person-Centered Expressive Arts for Healing and Social Change*.

PERSON-CENTERED EXPRESSIVE ARTS THERAPY*

Natalie Rogers (1993, 2011) expanded on her father, Carl Rogers's (1961), theory of creativity using the expressive arts to enhance personal growth for individuals and groups. Rogers's approach, known as **expressive arts therapy**, extends the person-centered approach to spontaneous creative expression, which symbolizes deep and sometimes inaccessible feelings and emotional states. Counselors trained in person-centered expressive arts offer their clients the opportunity to create movement, visual art, journal writing, sound, and music to express their feelings and gain insight from these activities.

Principles of Expressive Arts Therapy

Expressive arts therapy uses various artistic forms—movement, drawing, painting, sculpting, music, writing, and improvisation—toward the end of growth, healing, and self-discovery. This is a multimodal approach integrating mind, body, emotions, and inner spiritual resources. Methods of expressive arts therapy are based on humanistic principles but giving fuller form to Carl Rogers's notions of creativity. These principles include the following (N. Rogers, 1993):

- All people have an innate ability to be creative.
- The creative process is transformative and healing. The healing aspects involve activities such as meditation, movement, art, music, and journal writing.
- Personal growth and higher states of consciousness are achieved through self-awareness, self-understanding, and insight.
- Self-awareness, understanding, and insight are achieved by delving into our feelings of grief, anger, pain, fear, joy, and ecstasy.
- Our feelings and emotions are an energy source that can be channeled into the expressive arts to be released and transformed.
- The expressive arts lead us into the unconscious, thereby enabling us to express previously unknown facets of ourselves and bring to light new information and awareness.
- One art form stimulates and nurtures the other, bringing us to an inner core or essence that is our life energy.

*Much of the material in this section is based on key ideas that are more fully developed in two of Natalie Rogers's books, *The Creative Connection: Expressive Arts as Healing* (Rogers, 1993) and *The Creative Connection for Groups: Person-Centered Expressive Arts for Healing and Social Change* (Rogers, 2011). This section was written in close collaboration with Natalie Rogers.

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- A connection exists between our life force—our inner core, or soul—and the essence of all beings.
- As we journey inward to discover our essence or wholeness, we discover our relatedness to the outer world, and the inner and outer become one.

The various art modes interrelate in what Natalie Rogers calls the "creative connection." When we move, it affects how we write or paint. When we write or paint, it affects how we feel and think.

Natalie Rogers's approach is based on a person-centered theory of individual and group process. The same conditions that Carl Rogers and his colleagues found basic to fostering a facilitative client–counselor relationship also help support creativity. Personal growth takes place in a safe, supportive environment created by counselors or facilitators who are genuine, warm, empathic, open, honest, congruent, and caring—qualities that are best learned by first being experienced. Taking time to reflect on and evaluate these experiences allows for personal integration at many levels—intellectual, emotional, physical, and spiritual.

Creativity and Offering Stimulating Experiences

According to Natalie Rogers, this deep faith in the individual's innate drive to become fully oneself is basic to the work in person-centered expressive arts. Individuals have a tremendous capacity for self-healing through creativity if given the proper environment. When one feels appreciated, trusted, and given support to use individuality to develop a plan, create a project, write a paper, or to be authentic, the challenge is exciting, stimulating, and gives a sense of personal expansion. N. Rogers believes the tendency to actualize and become one's full potential, including innate creativity, is undervalued, discounted, and frequently squashed in our society. Traditional educational institutions tend to promote conformity rather than original thinking and the creative process.

Person-centered expressive arts therapy utilizes the arts for spontaneous creative expression that symbolizes deep and sometimes inaccessible feelings and emotional states. The conditions that foster creativity require acceptance of the individual, a nonjudgmental setting, empathy, psychological freedom, and availability of stimulating and challenging experiences. With this type of environment in place, the facilitative internal conditions of the client are encouraged and inspired: a nondefensive openness to experience and an internal locus of evaluation that receives but is not overly concerned with the reactions of others. N. Rogers (1993) believes that we cheat ourselves out of a fulfilling and joyous source of creativity if we cling to the idea that an artist is the only one who can enter the realm of creativity. Art is not only for the few who develop a talent or master a medium. We all can use various art forms to facilitate self-expression and personal growth.

Contributions of Natalie Rogers

As is clear from this brief section, Natalie Rogers has built upon a person-centered philosophy and incorporated expressive and creative arts as a basis for personal growth. Cain (2010) believes "Natalie Rogers's expressive arts therapy represents a major innovation in practice and helped open the way for other person-centered therapists to expand the variety and range of practice" (p. 60). Sommers-Flanagan

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(2007) notes that person-centered expressive arts therapy may be a solution for clients who are stuck in linear and rigid ways of being. He concludes: "Using her own love of creativity and art in combination with her father's renowned therapeutic approach, Natalie Rogers developed a form of therapy that extends person-centered counseling into a new and exciting domain" (p. 124). Rogers continues her active professional life, conducting workshops in the United States, Europe, Japan, Hong Kong, Latin America, and Russia. At the end of this chapter are some resources for those interested in training in the person-centered approach to expressive arts therapy.

MOTIVATIONAL INTERVIEWING

Motivational Interviewing (MI) is a humanistic, client-centered, psychosocial, directive counseling approach that was developed by William R. Miller and Stephen Rollnick in the early 1980s. The clinical and research applications of motivational interviewing have received increased attention in recent years, and MI has been shown to be effective as a relatively brief intervention (Levensky, Kersh, Cavasos, & Brooks, 2008). MI has been defined as "a directive, client centered counseling style for eliciting behavior change by helping clients to explore and resolve ambivalence" (Rollnick & Miller, 1995, p. 326). Motivational interviewing is based on humanistic principles, has some basic similarities with person-centered therapy, and expands the traditional person-centered approach.

Motivational interviewing was initially designed as a brief intervention for problem drinking, but more recently this approach has been applied to a wide range of clinical problems including substance abuse, compulsive gambling, eating disorders, anxiety disorders, depression, suicidality, chronic disease management, and health behavior change practices (Arkowitz & Miller, 2008; Arkowitz & Westra, 2009). MI stresses client self-responsibility and promotes an invitational style for working cooperatively with clients to generate alternative solutions to behavioral problems. MI therapists avoid arguing with clients, avoid assuming a confrontational stance, reframe resistance as a healthy response, express empathy, and listen reflectively. MI therapists do not view clients as opponents to be defeated but as allies who play a major role in their present and future success. Both MI and person-centered practitioners believe in the client's abilities, strengths, resources, and competencies. The underlying assumption is that clients want to be healthy and desire positive change.

The MI Spirit

MI is rooted in the philosophy of person-centered therapy, but with a "twist." Unlike the nondirective and unstructured person-centered approach, MI is deliberately directive and is aimed at reducing client ambivalence about change and increasing intrinsic motivation (Arkowitz & Miller, 2008). It is essential that therapists function within the spirit of MI, rather than simply applying the strategies of the approach (Levensky et al., 2008). The attitudes and skills in MI are based on a person-centered philosophy and include using open-ended questions, employing reflective listening, affirming and supporting the client, responding to resistance

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in a nonconfrontational manner, guiding a discussion of ambivalence, summarizing and linking at the end of sessions, and eliciting and reinforcing *change talk*. MI works by activating clients' own motivation for change and adherence to treatment. Practitioners assist clients in becoming their own advocates for change and the primary agents of change in their lives.

In both person-centered therapy and MI, the counselor provides the conditions for growth and change by communicating attitudes of accurate empathy and unconditional positive regard. In MI, the therapeutic relationship is as important in achieving successful outcomes as the specific theoretical model or school of psychotherapy from which the therapist operates (Miller & Rollnick, 2002). Both MI and person-centered therapy are based on the premise that individuals have within themselves the capacity to generate intrinsic motivation to change. Responsibility for change rests with clients, not with the counselor, and therapist and client share a sense of hope and optimism that change is possible. Once clients believe that they have the capacity to change and heal, new possibilities open up for them.

The Basic Principles of Motivational Interviewing

Miller and Rollnick (2002) formulated five basic principles of MI, which are summarized below:

1. Therapists practicing motivational interviewing strive to experience the world from the client's perspective without judgment or criticism. MI emphasizes reflective listening, which is a way for practitioners to better understand the subjective world of clients. Expressing empathy is foundational in creating a safe climate for clients to explore their ambivalence for change. When clients are slow to change, it may be assumed that they have compelling reasons to remain as they are as well as having reasons to change.

2. MI is designed to evoke and explore both discrepancies and ambivalence. Counselors using MI reflect discrepancies between the behaviors and values of clients to increase the motivation to change. Counselors pay particular attention to clients' arguments for changing compared to their arguments for not changing. Therapists elicit and reinforce *change talk* by employing specific strategies to strengthen discussion about change. MI therapists assume a directive stance by steering the conversation in the direction of considering change without persuading clients to change. Clinicians encourage clients to determine whether change will occur, and if so, what kinds of changes will occur and when.

3. Reluctance to change is viewed as a normal and expected part of the therapeutic process. Although individuals may see advantages to making life changes, they also may have many concerns and fears about changing. People who seek therapy are often ambivalent about change, and their motivation may ebb and flow during the course of therapy. A central goal of MI is to increase internal motivation to change based on the personal goals and values of clients (Arkowitz & Miller, 2008). MI therapists assume a respectful view of resistance and work therapeutically with any reluctance or caution on the part of clients.

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4. Practitioners operating from an MI orientation support clients' self-efficacy, mainly by encouraging them to use their own resources to take necessary actions that can lead to success in changing. MI clinicians strive to enhance client agency about change and emphasize the right and inherent ability of clients to formulate their own personal goals and to make their own decisions. MI focuses on present and future conditions and empowers clients to find ways to achieve their goals.

5. When clients show signs of readiness to change through decreased resistance to change and increased talk about change, a critical phase of MI begins. In this stage, clients may express a desire and ability to change, show an interest in questions about change, experiment with making changes between sessions, and envision a future picture of how their life will be different once the desired changes have been made. At this time therapists shift their focus toward strengthening clients' commitments to change and helping them implement a change plan.

The Stages of Change

The stages of change model assumes that people progress through a series of five identifiable stages in the counseling process. In the *precontemplation stage*, there is no intention of changing a behavior pattern in the near future. In the *contemplation stage*, people are aware of a problem and are considering overcoming it, but they have not yet made a commitment to take action to bring about the change. In the *preparation stage*, individuals intend to take action immediately and report some small behavioral changes. In the *action stage,* individuals are taking steps to modify their behavior to solve their problems. During the *maintenance stage,* people work to consolidate their gains and prevent relapse.

People do not pass neatly through these five stages in linear fashion, and a client's readiness can fluctuate throughout the change process. If change is initially unsuccessful, individuals may return to an earlier stage (Prochaska & Norcross, 2010). MI therapists strive to match specific interventions with whatever stage of change clients are experiencing. If there is a mismatch between process and stage, movement through the stage will be impeded and is likely to be manifested in reluctant behavior. When clients demonstrate any form of reluctance or resistance, this could be due to a therapist's misjudgment of a client's readiness to change. Certain behaviors on a therapist's part may lead to a client feeling invalidated or misunderstood, which is likely to result in what appears to be a client's resistive behavior (Levensky et al., 2008).

Working within the framework of the stages of change model has implications for the therapist's role at the different stages. Norcross, Krebs, and Prochaska (2011) describe the relational stances and roles taken by therapists during the course of therapy. With clients in the precontemplation stage, the role assumed is that of a *nurturing parent*. With clients in contemplation, therapists function as a *Socratic teacher* who encourages them to achieve their own insights. For clients who are in the preparation stage, therapists take the stance of an *experienced coach*. With clients who are progressing into action and maintenance, therapists function in the role of a *consultant*. As termination approaches, therapists are consulted less often as a way to foster client autonomy.

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Motivational interviewing is but one example of how therapeutic strategies have been developed based on the foundational principles and philosophy of the person-centered approach. Indeed, most of the therapeutic models illustrate how the core therapeutic conditions are necessary aspects leading to client change. Where many therapeutic approaches, including motivational interviewing, diverge from traditional person-centered therapy is the assumption that the therapeutic factors are both *necessary and sufficient* in bringing about change. Many other models employ specific intervention strategies to address specific concerns clients bring to therapy.

PERSON-CENTERED THERAPY FROM A MULTICULTURAL PERSPECTIVE



Strengths From a Diversity Perspective

One of the strengths of the person-centered approach is its impact on the field of human relations with diverse cultural groups. Person-centered philosophy and practice can now be studied in several European countries, South America, and Japan. Here are some examples of ways in which this approach has been incorporated in various countries and cultures:

- In several European countries person-centered concepts have had a significant impact on the practice of counseling as well as on education, cross-cultural communication, and reduction of racial and political tensions. In the 1980s Rogers (1987b) elaborated on a theory of reducing tension among antagonistic groups that he began developing in 1948.
- In the 1970s Rogers and his associates began conducting workshops promoting cross-cultural communication. Well into the 1980s he led large workshops in many parts of the world. International encounter groups have provided participants with multicultural experiences.
- Japan, Australia, South America, Mexico, and the United Kingdom have all been receptive to person-centered concepts and have adapted these practices to fit their cultures.
- Shortly before his death, Rogers conducted intensive workshops with professionals in the former Soviet Union.

Cain (1987c) sums up the reach of the person-centered approach to cultural diversity: "Our international family consists of millions of persons worldwide whose lives have been affected by Carl Rogers's writings and personal efforts as well as his many colleagues who have brought his and their own innovative thinking and programs to many corners of the earth" (p. 149).

There is no doubt that Carl Rogers has had a global impact. His work has reached more than 30 countries, and his writings have been translated into 12 languages. In addition to this global impact, the emphasis on the core conditions makes the person-centered approach useful in understanding diverse worldviews. The underlying philosophy of person-centered therapy is grounded on the importance of hearing the deeper messages of a client. Empathy, being present, and respecting the values of clients are essential attitudes and skills in counseling culturally diverse

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clients. Cain (2008, 2010) contends that although person-centered therapists are aware of diversity factors, they do not make initial assumptions about individuals. They realize that each client's journey is unique and take steps to tailor their methods to fit each individual.

Several writers consider person-centered therapy as being ideally suited to clients in a diverse world. Cain (2008, 2010) views this approach as being a potent way of working with individuals representing a wide range of cultural backgrounds because the core therapeutic conditions are qualities that are universal. Bohart and Watson (2011) claim that the person-centered philosophy is particularly appropriate for working with diverse client populations because the counselor does not assume the role of expert who is going to impose a "right way of being" on the client. Instead, the therapist is a "fellow explorer" who attempts to understand the client's phenomenological world in an interested, accepting, and open way and checks with the client to confirm that the therapist's perceptions are accurate. Motivational interviewing, which is based on the philosophy of person-centered therapy, is a culturally sensitive approach that can be effective across population domains, including gender, age, ethnicity, and sexual orientation (Levensky et al., 2008).

Shortcomings From a Diversity Perspective

Although the person-centered approach has made significant contributions to counseling people from diverse social, political, and cultural backgrounds, there are some shortcomings to practicing exclusively within this framework. Many clients who come to community mental health clinics or who are involved in outpatient treatment want more structure than this approach provides. Some clients seek professional help to deal with a crisis, to alleviate emotional problems, or to learn coping skills in dealing with everyday problems. Because of certain cultural messages, when these clients do seek professional help, it may be as a last resort. They often expect a directive counselor and can be put off by a professional who does not provide sufficient structure.

A second shortcoming of the person-centered approach is that it is difficult to translate the core therapeutic conditions into actual practice in certain cultures. Communication of these core conditions must be consistent with the client's cultural framework. Consider, for example, the expression of therapist congruence and empathy. Clients accustomed to indirect communication may not be comfortable with direct expressions of empathy or self-disclosure on the therapist's part. For some clients the most appropriate way to express empathy is for the therapist to demonstrate it indirectly through respecting their need for distance or through suggesting task-focused interventions (Bohart & Greenberg, 1997).

A third shortcoming in applying the person-centered approach with clients from diverse cultures pertains to the fact that this approach extols the value of an *internal* locus of evaluation. The humanistic foundation of person-centered therapy emphasizes dimensions such as self-awareness, freedom, autonomy, self-acceptance, inner-directedness, and self-actualization. In collectivist cultures, clients are likely to be highly influenced by societal expectations and not simply motivated by their own personal preferences. The focus on development of individual autonomy and personal growth may be viewed as being selfish in a culture that

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stresses the common good. Cain (2010) contends that "persons from collectivistic cultures are oriented less toward self-actualization and more toward intimacy, connection, and harmony with others and toward what is best for the community and the common good" (p. 143).

Consider Lupe, a Latina client who values the interests of her family over her self-interests. From a person-centered perspective she could be viewed as being in danger of "losing her own identity" by being primarily concerned with her role in taking care of others in the family. Rather than pushing her to make her personal wants a priority, the counselor will explore Lupe's cultural values and her level of commitment to these values in working with her. It would be inappropriate for the counselor to impose a vision of the kind of woman she should be. (This topic is discussed more extensively in Chapter 12.)

Although there may be particular shortcomings in practicing exclusively within a person-centered perspective, it should not be concluded that this approach is unsuitable for working with clients from diverse cultures. There is great diversity among any group of people, and therefore, there is room for a variety of therapeutic styles. According to Cain (2010), rigid insistence on a nondirective style of counseling for all clients, regardless of their cultural background or personal preference, may be perceived as an imposition that does not fit the client's interpersonal and therapeutic needs. Counseling a culturally different client may require more activity and structuring than is usually the case in a person-centered framework, but the potential positive impact of a counselor who responds empathically to a culturally different client cannot be overestimated.

Person-Centered Therapy Applied to the Case of Stan



Stan's autobiography indicates that he has a sense of what he wants for his life. As a person-centered therapist, I rely on his self-report of the way he views himself rather than on a formal assessment and diagnosis. My concern is with understanding him from his internal frame of reference. Stan has stated goals that are meaningful for him. He is motivated to change and seems to have sufficient anxiety to work toward these desired changes. I have faith in Stan's ability to find his own way, and I trust that he has the necessary resources for reaching his therapy goals. I encourage Stan to speak freely about the discrepancy between the person he sees himself as being and the person he would like to become; about his feelings of being a failure, being inadequate; about his fears and uncertainties; and about his hopelessness at times. I attempt to create an atmosphere of freedom and security that will encourage Stan to explore the threatening aspects of his self-concept.

Stan has a low evaluation of his self-worth. Although he finds it difficult to believe that others really like him, he wants to feel loved. He says, "I hope I can learn to love at least a few people, most of all, women." He wants to feel equal to others and not have to apologize for his existence, yet most of the time he feels inferior. By creating a supportive, trusting, and encouraging atmosphere, I can help Stan learn to be more accepting of himself, with both his strengths and limitations. He has the opportunity to openly express his fears of women, of not being able to work with people, and of feeling inadequate and stupid. He can explore how he feels judged by his parents and by

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authorities. He has an opportunity to express his guilt—that is, his feelings that he has not lived up to his parents' expectations and that he has let them and himself down. He can also relate his feelings of hurt over not having ever felt loved and wanted. He can express the loneliness and isolation that he so often feels, as well as the need to numb these feelings with alcohol or drugs.

Stan is no longer totally alone, for he is taking the risk of letting me into his private world of feelings. Stan gradually gets a sharper focus on his experiencing and is able to clarify his own feelings and attitudes. He sees that he has the capacity to make his own decisions. In short, our therapeutic relationship frees him from his self-defeating ways. Because of the caring and faith he experiences from me in our relationship, Stan is able to increase his own faith and confidence in himself.

My empathy assists Stan in hearing himself and accessing himself at a deeper level. Stan gradually becomes more sensitive to his own internal messages and less dependent on confirmation from others around him. As a result of the therapeutic venture, Stan discovers that there is someone in his life whom he can depend on—himself.

Follow-Up: You Continue as Stan's Person-Centered Therapist

Use these questions to help you think about how you would counsel Stan using a person-centered approach:

- How would you respond to Stan's deep feelings of self-doubt? Could you enter his frame of reference and respond in an empathic manner that lets Stan know you hear his pain and struggle without needing to give advice or suggestions?
- How would you describe Stan's deeper struggles? What sense do you have of his world?
- To what extent do you think that the relationship you would develop with Stan would help him move forward in a positive direction? What, if anything, might get in your way—either with him or in yourself—in establishing a therapeutic relationship?



See *DVD for Theory and Practice of Counseling and Psychotherapy: The Case of Stan and Lecturettes* (Session 5 on person-centered therapy) for a demonstration of my approach to counseling Stan from this perspective. This session focuses on exploring the immediacy of our relationship and assisting Stan in finding his own way.

SUMMARY AND EVALUATION

Summary

Person-centered therapy is based on a philosophy of human nature that postulates an innate striving for self-actualization. Further, Rogers's view of human nature is phenomenological; that is, we structure ourselves according to our perceptions of reality. We are motivated to actualize ourselves in the reality that we perceive.

Rogers's theory rests on the assumption that clients can understand the factors in their lives that are causing them to be unhappy. They also have the capacity for self-direction and constructive personal change. Change will occur if a congruent therapist makes psychological contact with a client in a state of anxiety or incongruence. It is essential for the therapist to establish a relationship the client perceives as genuine, accepting, and understanding. Therapeutic counseling is based on an I/Thou, or person-to-person, relationship in the safety and acceptance of which clients drop their defenses and come to accept and integrate aspects that they have denied or distorted. The person-centered approach emphasizes this

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personal relationship between client and therapist; the therapist's attitudes are more critical than are knowledge, theory, or techniques employed. In the context of this relationship, clients unleash their growth potential and become more of the person they are capable of becoming.

This approach places primary responsibility for the direction of therapy on the client. In the therapeutic context, individuals have the opportunity to decide for themselves and come to terms with their own personal power. The underlying assumption is that no one knows the client better than the client; in short, the client is viewed as an expert on his or her own life (Cain, 2010). The general goals of therapy are becoming more open to experience, achieving self-trust, developing an internal source of evaluation, and being willing to continue growing. Specific goals are not suggested for clients; rather, clients choose their own values and goals. Current applications of the theory emphasize more active participation by the therapist than was the case earlier. Counselors are now encouraged to be fully involved as persons in the therapeutic relationship. More latitude is allowed for therapists to express their reactions and feelings as they are appropriate to what is occurring in therapy. Person-centered practitioners are willing to be transparent about persistent feelings that exist in their relationships with clients (Watson et al., 2011). It is the therapist's job to adapt and accommodate in a manner that works best for each client, which means being flexible in the application of methods in the counseling process (Cain, 2010).

Contributions of the Person-Centered Approach

When Rogers founded nondirective counseling more than 70 years ago, there were very few other therapeutic models. The longevity of this approach is certainly a factor to consider in assessing its influence. Cain (2008) states: "An extensive body of research has been generated and provides support for the effectiveness of person-centered therapy with a wide range of clients and problems of all age groups" (p. 214).

Rogers had, and his theory continues to have, a major impact on the field of counseling and psychotherapy. When he introduced his revolutionary ideas in the 1940s, he provided a powerful and radical alternative to psychoanalysis and to the directive approaches then practiced. Rogers was a pioneer in shifting the therapeutic focus from an emphasis on technique and reliance on therapist authority to that of the power of the therapeutic relationship.

Kirschenbaum (2009) contends that the scope and influence of Rogers's work has continued well beyond his death; the person-centered approach is alive, well, and expanding. Today there is not one version of person-centered therapy, but a number of continuously evolving person-centered psychotherapies (Cain, 2010). Although few psychotherapists claim to have an exclusive person-centered theoretical orientation, the philosophy and principles of this approach permeate the practice of most therapists. Other schools of therapy are increasingly recognizing the centrality of the therapeutic relationship as a route to therapeutic change (Kirschenbaum, 2009).

Person-centered therapy is strongly represented in Europe, and there is continuing interest in this approach in both South America and the Far East. The

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person-centered approach has established a firm foothold in British universities. According to Natalie Rogers (2011), some of the most in-depth training of person-centered counselors is in the United Kingdom. In addition, British scholars including Fairhurst (1999), Keys (2003), Lago and Smith (2003), Mearns and Cooper (2005), Mearns and Thorne (2000, 2007), Merry (1999), Natiello (2001), Thorne (2002a, 2002b), and Watson (2003) continue to expand and refine this approach.

As we have seen, Natalie Rogers has made a significant contribution to the application of the person-centered approach by incorporating the expressive arts as a medium to facilitate healing and social change, primarily in a group setting. She has been instrumental in the evolution of the person-centered approach by using nonverbal methods to enable individuals to heal and to develop. Many individuals who have difficulty expressing themselves verbally can find new possibilities for self-expression through nonverbal channels and through the expressive arts (N. Rogers, 2011).

EMPHASIS ON RESEARCH One of Rogers's contributions to the field of psychotherapy was his willingness to state his concepts as testable hypotheses and to submit them to research. He literally opened the field to research. He was truly a pioneer in his insistence on subjecting the transcripts of therapy sessions to critical examination and applying research technology to counselor–client dialogues (Combs, 1988). Rogers's basic hypothesis gave rise to a great deal of research and debate in the field of psychotherapy, perhaps more than any other school of therapy (Cain, 2002a). According to Cain (2010), an enormous body of research, conducted over a period of 70 years, supports the effectiveness of the person-centered approach. This research is ongoing in many parts of the world and continues to expand and refine our understanding of what constitutes effective psychotherapy. Cain (2010) concludes, "person centered therapy is as vital and effective as it has ever been and continues to develop in ways that will make it increasingly so in the years to come" (p. 169).

Even his critics give Rogers credit for having conducted and inspired others to conduct extensive studies of counseling process and outcome. Rogers presented a challenge to psychology to design new models of scientific investigation capable of dealing with the inner, subjective experiences of the person. His theories of therapy and personality change have had a tremendous heuristic effect, and though much controversy surrounds this approach, his work has challenged practitioners and theoreticians to examine their own therapeutic styles and beliefs.

THE IMPORTANCE OF EMPATHY Among the major contributions of person-centered therapy are the implications of empathy for the practice of counseling. More than any other approach, person-centered therapy has demonstrated that therapist empathy plays a vital role in facilitating constructive change in the client. Watson's (2002) comprehensive review of the research literature on therapeutic empathy has consistently demonstrated that therapist empathy is the most potent predictor of client progress in therapy. Indeed, empathy is an essential component of successful therapy in every therapeutic modality.

Person-centered research has been conducted predominantly on the hypothesized necessary and sufficient conditions of therapeutic personality change (Cain,

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1986, 1987b). Most of the other counseling approaches covered in this book have incorporated the importance of the therapist's attitude and behavior in creating a therapeutic relationship that is conducive to the use of their techniques. For instance, the cognitive behavioral approaches have developed a wide range of strategies designed to help clients deal with specific problems, and they recognize that a trusting and accepting client–therapist relationship is necessary for successful application of these procedures. In contrast to the person-centered approach, however, cognitive behavioral practitioners contend that the working relationship is not sufficient to produce change. They contend that active procedures, in combination with a collaborative relationship, are needed to bring about change.

INNOVATIONS IN PERSON-CENTERED THERAPY One of the strengths of the person-centered approach is "the development of innovative and sophisticated methods to work with an increasingly difficult, diverse, and complex range of individuals, couples, families, and groups" (Cain, 2002b, p. xxii). A number of people have made significant advancements that are compatible with the essential values and concepts of person-centered therapy. Table 7.1 describes some of the innovators who have played a role in the evolution of person-centered therapy.

Rogers consistently opposed the institutionalization of a client-centered "school." Likewise, he reacted negatively to the idea of founding institutes, granting certificates, and setting standards for membership. He feared this institutionalization would lead to an increasingly narrow, rigid, and dogmatic perspective. If Rogers (1987a) were to give students-in-training advice it would be: "There is one *best* school of therapy. It is the school of therapy you develop for yourself based on a continuing critical examination of the effects of your way of being in the relationship" (p. 185).

EMOTION-FOCUSED THERAPY One of the developments associated with the person-centered approach is the emergence of emotion-focused therapy (EFT). Leslie Greenberg (2011) is a prominent figure in the development of this integrative approach. Emotion-focused therapy is rooted in a person-centered philosophy, but it is integrative in that it synthesizes aspects of Gestalt therapy and existential therapy. EFT is a therapeutic practice informed by an understanding of the role of emotion in psychotherapeutic change. A number of strategies in EFT are aimed at the goal of strengthening the self, regulating affect, and creating new meaning. Many traditional therapies emphasize conscious understanding and cognitive and behavioral change, yet they often neglect the foundational role of emotional change. EFT emphasizes the importance of awareness, acceptance, and understanding of emotion and the visceral experience of emotion. In EFT, clients are assisted to identify, experience, accept, explore, transform, and manage their emotions. A premise of EFT is that we can change only when we accept ourselves as we are. This approach has a good deal to offer with respect to teaching us about the role of emotion in personal change and how emotional change can be a primary pathway to cognitive and behavioral change.

Other therapeutic approaches are increasingly focusing on emotions. For example, both psychoanalytic and cognitive behavioral approaches are giving more attention to the role of emotions and are rapidly assimilating many aspects of EFT.

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TABLE 7.1 Therapists Who Contributed to the Evolution of Person-Centered Theory

INNOVATOR	CONTRIBUTION
Natalie Rogers (1993, 1995, 2011)	Expanded the theory by developing person-centered expressive arts therapy in groups.
Virginia Axline (1964, 1969)	Made significant contributions to client-centered therapy with children and play therapy.
Eugene Gendlin (1996)	Developed experiential techniques, such as focusing, as a way to enhance client experiencing.
Laura Rice (Rice & Greenberg, 1984)	Taught therapists to be more evocative in re-creating crucial experiences that continue to trouble the client.
Peggy Natiello (2001)	Works on collaborative power and gender issues.
Art Combs (1988, 1989, 1999)	Developed perceptual psychology.
Leslie Greenberg (2011); Greenberg and colleagues (Greenberg, Korman, & Paivio, 2002; Greenberg, Rice, & Elliott, 1993)	Contributed to the development of emotion-focused therapy. Focused on the importance of facilitating emotional change in therapy and advanced person-centered theory and methods. Demonstrated that the emotional route can be a key to changing cognitions and behavior.
David Rennie (1998)	Provided a glimpse at the inner workings of the therapeutic process.
Art Bohart (Bohart & Greenberg, 1997; Bohart & Tallman, 1999; 2010; Bohart & Watson, 2011)	Contributed to a deeper understanding of empathy in therapeutic practice and the active role of the client.
Jeanne Watson (2002)	Demonstrated that when empathy is operating on the cognitive, affective, and interpersonal levels it is one of the therapist's most powerful tools.
Dave Mearns and Brian Thorne (2000, 2007)	Contributed to understanding new frontiers in the theory and practice of the person-centered approach and have been significant figures in teaching and supervising in the United Kingdom.
C. H. Patterson (1995)	Showed that client-centered therapy is a universal system of psychotherapy.
Barry Duncan, Scott Miller, Bruce Wampold, & Mark Hubble (2010)	Demonstrated that the client-centered relationship and the core conditions are essential to all therapeutic approaches.

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A strength of EFT is that it is an evidence-based approach, an idea that is increasingly being emphasized in graduate programs (Greenberg, 2011).

Limitations and Criticisms of the Person-Centered Approach

Although I applaud person-centered therapists for their willingness to subject their hypotheses and procedures to empirical scrutiny, some researchers have been critical of the methodological errors contained in some of these studies. Accusations of scientific shortcomings involve using control subjects who are not candidates for therapy, failing to use an untreated control group, failing to account for placebo effects, relying on self-reports as a major way to assess the outcomes of therapy, and using inappropriate statistical procedures.

There is a similar limitation shared by both the person-centered and existential (experiential) approaches. Neither of these therapeutic modalities emphasizes the role of techniques aimed at bringing about change in clients' behavior. Proponents of psychotherapy manuals, or manualized treatment methods for specific disorders, find serious limitations in the experiential approaches due to their lack of attention to proven techniques and strategies. Those who call for accountability as defined by evidence-based practices within the field of mental health also are quite critical of the experiential approaches.

I do not believe manualized treatment methods can be considered the gold standard in psychotherapy, however. There is good research demonstrating that techniques account for only 15% of client outcome (see Duncan et al., 2010), whereas contextual factors have powerful effects on what happens in therapy (Elkins, 2009). Research points to relational and client factors as the main predictors of effective therapy. Furthermore, the evaluation of evidence-based practices should be broadened to include best available research, the expertise of the clinician, and client characteristics, culture, and preferences (see Norcross, Hogan, & Koocher, 2008).

A potential limitation of the person-centered approach is that some students-in-training and practitioners with this orientation may have a tendency to be very supportive of clients without being challenging. Out of their misunderstanding of the basic concepts of the approach, some have limited the range of their responses and counseling styles mainly to reflections and empathic listening. Although there is value in really hearing a client and in reflecting and communicating understanding, counseling entails more than this. I believe that the therapeutic core conditions are *necessary* for therapy to succeed, yet I do not see them as being *sufficient* conditions for change for all clients at all times. From my perspective, these basic attitudes are the foundation on which counselors must then build the *skills* of therapeutic intervention. Motivational interviewing rests on the therapeutic core conditions, for example, but MI employs a range of strategies that enable clients to develop action plans leading to change.

A related challenge for counselors using this approach is to truly support clients in finding their own way. Counselors sometimes experience difficulty in allowing clients to decide their own specific goals in therapy. It is easy to give lip service to the concept of clients' finding their own way, but it takes considerable respect for clients and faith on the therapist's part to encourage clients to listen to themselves and follow their own directions, particularly when they make choices that are not what the therapist hoped for.

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More than any other quality, the therapist's genuineness determines the power of the therapeutic relationship. If therapists submerge their unique identity and style in a passive and nondirective manner, they are not likely to affect clients in powerful ways. Therapist authenticity and congruence are so vital to this approach that those who practice within this framework must feel natural in doing so and must find a way to express their own reactions to clients. If not, a real possibility is that person-centered therapy will be reduced to a bland, safe, and ineffectual approach.

WHERE TO GO FROM HERE

In the *DVD for Integrative Counseling: The Case of Ruth and Lecturettes,* you will see a concrete illustration of how I view the therapeutic relationship as the foundation for our work together. Refer especially to Session 1 ("Beginning of Counseling"), Session 2 ("The Therapeutic Relationship"), and Session 3 ("Establishing Therapeutic Goals") for a demonstration of how I apply principles from the person-centered approach to my work with Ruth.

Free Podcasts for ACA Members

You can download ACA Podcasts (prerecorded interviews) by going to www.counseling.org; click on the Resource button and then select the Podcast Series. For Chapter 7, Carl Rogers and the Person-Centered Approach, look for Podcast 7 by by Dr. Howard Kirschenbaum.

Other Resources

The American Psychological Association offers the following DVDs in their Psychotherapy Video Series:

Greenberg, L. S. (2010). *Emotion-Focused Therapy Over Time*
Cain, D. J. (2010). *Person-Centered Therapy Over Time*

Psychotherapy.net is a comprehensive resource for students and professionals that offers videos and interviews featuring Natalie Rogers, Rollo May, and more. New articles, interviews, blogs, therapy cartoons, and videos are published monthly. DVDs relevant to this chapter are available at www.psychotherapy.net and include the following:

Rogers, N. (1997). *Person-Centered Expressive Arts Therapy*
May, R. (2007). *Rollo May on Existential Psychotherapy*

Association for the Development of the Person-Centered Approach, Inc. (ADPCA)
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The Association for the Development of the Person-Centered Approach (ADPCA) is an interdisciplinary and international organization that consists of a network of individuals who support the development and application of the person-centered approach. Membership includes a subscription to the *Person-Centered Journal,* the

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association's newsletter, a membership directory, and information about the annual meeting. ADPCA also provides information about continuing education and supervision and training in the person-centered approach. For information about the *Person-Centered Journal,* contact the editor (Jon Rose).

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The Association for Humanistic Psychology (AHP) is devoted to promoting personal integrity, creative learning, and active responsibility in embracing the challenges of being human in these times. Information about the *Journal of Humanistic Psychology* is available from the Association for Humanistic Psychology or at publisher's website.

Society for Humanistic Psychology
Website: http://www.societyforhumanisticpsychology.com/

Division 32 of APA, Society for Humanistic Psychology, represents a constellation of "humanistic psychologies" that includes the earlier Rogerian, transpersonal, and existential orientations as well as the more recently developing perspectives. Division 32 seeks to contribute to psychotherapy, education, theory, research, epistemological diversity, cultural diversity, organization, management, social responsibility, and change. The division has been at the forefront in the development of qualitative research methodologies. The Society for Humanistic Psychology offers journal access to *The Humanistic Psychologist*. Information about membership, conferences, and journals is available from the website of Division 32.

Carl Rogers: A Daughter's Tribute
Website: www.nrogers.com

The Carl Rogers CD-ROM is a visually beautiful and lasting archive of the life and works of the founder of humanistic psychology. It includes excerpts from his 16 books, over 120 photographs spanning his lifetime, and award-winning video footage of two encounter groups and Carl's early counseling sessions. It is an essential resource for students, teachers, libraries, and universities. It is a profound tribute to one of the most important thinkers, influential psychologists, and peace activists of the 20th century. Developed for Natalie Rogers, PhD, by Mindgarden Media, Inc.

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The Center for Studies of the Person (CSP) offers workshops, training seminars, experiential small groups, residential workshops, and sharing of learning in community meetings. The Distance Learning Project and the Carl Rogers Institute for Psychotherapy Training and Supervision provide experiential and didactic training and supervision for professionals interested in developing their own person-centered orientation.

Saybrook Graduate School
E-mail: admissions@saybrook.edu
Website: www.nrogers.com

For training in expressive art therapy, you could join Natalie Rogers, PhD, and Shellee Davis, MA, faculty of the certificate program at Saybrook Graduate School in their course, "Expressive Arts for Healing and Social Change: A Person-Centered Approach." A 16-unit certificate program includes 6 separate weeks spread over 2 years at a retreat center north of San Francisco. Rogers and Davis offer expressive arts within a person-centered counseling framework. They use counseling demonstrations, practice counseling sessions, readings, discussions, papers, and a creative project to teach experiential and theoretical methods.

Recommended Supplementary Readings

On Becoming a Person (Rogers, 1961) is one of the best primary sources for further reading on person-centered therapy. This is a collection of Rogers's articles on the process of psychotherapy, its outcomes, the therapeutic relationship, education, family life, communication, and the nature of the healthy person.

A Way of Being (Rogers, 1980) contains a series of writings on Rogers's personal experiences and perspectives, as well as chapters on the foundations and applications of the person-centered approach.

The Creative Connection: Expressive Arts as Healing (N. Rogers, 1993) is a practical, spirited book lavishly illustrated with color and action photos and filled with fresh ideas to stimulate creativity, self-expression, healing, and transformation. Natalie Rogers combines the philosophy of her father with the expressive arts to enhance communication between client and therapist.

The Life and Work of Carl Rogers (Kirschenbaum, 2009) is a definitive biography of Carl Rogers that follows his life from his early childhood through his death. This book illustrates the legacy of Carl Rogers and shows his enormous influence on the field of counseling and psychotherapy.

Person-Centered Psychotherapies (Cain, 2010) contains a clear discussion of person-centered theory, the therapeutic process, evaluation of the approach, and future developments.

Humanistic Psychology: A Clinical Manifesto (Elkins, 2009) offers an insightful critique of the medical model of psychotherapy and the myth of empirically

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supported treatments. The author calls for a relationship-based approach to psychotherapy that can provide both individual and social transformation.

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CHAPTER 8



Gestalt Therapy

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FREDERICK S. ("FRITZ") PERLS, MD, PhD (1893–1970), was the main originator and developer of Gestalt therapy. Born in Berlin, Germany, into a lower-middle-class Jewish family, he later identified himself as a source of much trouble for his parents. Although he failed the seventh grade twice and was expelled from school because of difficulties with the authorities, his brilliance was never quashed, and he returned—not only to complete high school but to earn his medical degree (MD) with a specialization in psychiatry. In 1916 he joined the German Army and served as a medic in World War I. His experiences with soldiers who were gassed on the front lines led to his interest in mental functioning, which led him to Gestalt psychology.

After the war Perls worked with Kurt Goldstein at the Goldstein Institute for Brain-Damaged Soldiers in Frankfurt. It was through this association that he came to see the importance of viewing humans as a whole rather than as a sum of discretely functioning parts. It was also through this association that he met his wife, Laura, who was earning her PhD with Goldstein. Later he moved to Vienna and began his psychoanalytic training. Perls was in analysis with Wilhelm Reich, a psychoanalyst who pioneered methods of self-understanding and personality change by working with the body.

Perls and several of his colleagues established the New York Institute for Gestalt Therapy in 1952. Eventually Fritz left New York and settled in Big Sur, California, where he conducted workshops and seminars at the Esalen Institute, carving out his reputation as an innovator in psychotherapy. Here he had a great impact on people, partly through his professional writings, but mainly through personal contact in his workshops.

Personally, Perls was both vital and perplexing. People typically either responded to him in awe or found him harshly confrontational and saw him as meeting his own needs through showmanship. Having a predilection for the theater since childhood, he loved being on stage and putting on a show. He was viewed variously as insightful, witty, bright, provocative, manipulative, hostile, demanding, and inspirational. Unfortunately, some of the people who attended his workshops went on to mimic the less attractive side of Perls's personality. Even though Perls was not happy with this, he did little to discourage it.

For a firsthand account of the life of Fritz Perls, I recommend his autobiography, *In and Out of the Garbage Pail* (1969b). For a well-researched chapter on the history of Gestalt therapy, see Bowman (2005).

LAURA POSNER PERLS, PhD (1905–1990), was born in Pforzheim, Germany, the daughter of well-to-do parents. She began playing the piano at the age of 5 and played with professional skill by the time she was 18. From the age of 8 she was involved in modern dance, and both music and modern dance remained vital parts of her adult life and were incorporated into her therapy with some clients. By the time Laura began her practice as a psychoanalyst she had prepared for a career as a concert pianist, had attended law school, achieved a doctoral degree in Gestalt psychology, and made an intensive study of existential philosophy with Paul Tillich and Martin Buber. Clearly, Laura already had a rich background when she met Fritz in 1926 and they began their collaboration, which resulted in the theoretical foundations of Gestalt therapy. Laura and Fritz were married in 1930 and had two children while living and practicing in South Africa. Laura continued to be the mainstay for the New York Institute for Gestalt Therapy after Fritz abandoned his family to become internationally famous as the traveling minstrel for Gestalt therapy. Laura also made significant contributions to the development and maintenance of the Gestalt therapy movement in the United States and throughout the world (although in very different ways) from the late 1940s until her death

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in 1990. Laura's own words make it clear that Fritz was a generator, not a developer or organizer. At the 25th anniversary of the New York Institute for Gestalt Therapy, Laura Perls (1990) stated, "Without the constant support from his friends, and from me, without the constant encouragement and collaboration, Fritz would never have written a line, nor founded anything" (p. 18).

Laura paid a great deal of attention to contact and support, which differed from Fritz's attention to intrapsychic phenomena and his focus on awareness. Her emphasis on contact underscored the role of the interpersonal and of being responsive at a time when the popular notion of Gestalt therapy was that it fostered responsibility only to oneself. She corrected some of the excesses committed in the name of Gestalt therapy and adhered to the basic principles of Gestalt therapy theory as written in *Gestalt Therapy: Excitement and Growth in the Human Personality* (Perls, Hefferline, & Goodman, 1951). She taught that every Gestalt therapist needs to develop his or her own therapeutic style. From her perspective, whatever is integrated in our personality becomes support for what we use technically (Humphrey, 1986).

INTRODUCTION

Gestalt therapy is an existential, phenomenological, and process-based approach created on the premise that individuals must be understood in the context of their ongoing relationship with the environment. Awareness, choice, and responsibility are cornerstones of practice. The initial goal is for clients to expand their *awareness* of what they are experiencing in the present moment. Through this awareness, change automatically occurs. The approach is phenomenological because it focuses on the client's perceptions of reality and existential because it is grounded in the notion that people are always in the process of becoming, remaking, and rediscovering themselves. As an existential approach, Gestalt therapy gives special attention to existence as individuals experience it and affirms the human capacity for growth and healing through interpersonal contact and insight (Yontef, 1995). In a nutshell, this approach focuses on the here and now, the what and how, and the I/Thou of relating (Brown, 2007; Yontef & Jacobs, 2011).

Contemporary Gestalt therapy, sometimes called **relational Gestalt therapy,** stresses dialogue and relationship between client and therapist. Following the lead of Laura Perls and the "Cleveland school," when Erving and Miriam Polster and Joseph Zinker were on the faculty in the 1960s and 1970s, this model includes more support and increased sensitivity and compassion in therapy as compared to the confrontational and dramatic style of Fritz Perls (Yontef, 1999). The majority of today's Gestalt therapists employ a style that is supportive, accepting, empathic, respectful, dialogical, as well as challenging. The emphasis is on the quality of the therapist–client relationship and empathic attunement while tapping the client's wisdom and resources (Cain, 2002).

Although Fritz Perls was influenced by psychoanalytic concepts, he took issue with Freud's theory on a number of grounds. Whereas Freud's view of human beings is basically mechanistic, Perls stressed a holistic approach to personality. Freud focused on repressed intrapsychic conflicts from early childhood, whereas Perls valued examining the present situation. The Gestalt approach focuses much

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more on process than on content. Therapists devise experiments designed to increase clients' awareness of *what* they are doing and *how* they are doing it. Perls asserted that how individuals behave in the present moment is far more crucial to self-understanding than why they behave as they do. Awareness usually involves insight and sometimes introspection, but Gestalt therapists consider it to be much more than either.

Self-acceptance, knowledge of the environment, responsibility for choices, and the ability to make contact with their **field** (a dynamic system of interrelationships) and the people in it are important awareness processes and goals, all of which are based on a here-and-now experiencing that is always changing. Clients are expected to do their own seeing, feeling, sensing, and interpreting, as opposed to waiting passively for the therapist to provide them with insights and answers.

Gestalt therapy is lively and promotes direct experiencing rather than the abstractness of talking about situations. The approach is experiential in that clients come to grips with what and how they are thinking, feeling, and doing as they interact with the therapist. Gestalt practitioners value being fully present during the therapeutic encounter with the belief that growth occurs out of genuine contact between client and therapist.

The experiential therapies focus on emotion as a route to both cognitive and behavioral change, and these therapies encourage clients to stay with their moment-by-moment experiencing. **Emotion-focused therapy** (EFT), developed by Leslie Greenberg (2011), is related to Gestalt therapy. EFT entails the practice of therapy being informed by understanding the role of emotion in psychotherapeutic change. Emotion-focused therapy blends the relational aspects of the person-centered approach with the active phenomenological awareness experiments of Gestalt therapy. EFT is an integrative model, drawing from the relationship-oriented approaches of existential therapy, person-centered therapy, and Gestalt therapy. EFT methodology is similar to Gestalt therapy but emphasizes empirically supported treatments. In-depth presentations of emotion-focused therapy can be found in *Learning Emotion-Focused Therapy: The Process-Experiential Approach to Change* (Elliott, Watson, Goldman, & Greenberg, 2004) and *Emotion-Focused Therapy* (Greenberg, 2011).



See the video program for Chapter 8, *DVD for Theory and Practice of Counseling and Psychotherapy: The Case of Stan and Lecturettes*. I suggest that you view the brief lecture for each chapter prior to reading the chapter.

KEY CONCEPTS

View of Human Nature

Fritz Perls (1969a) practiced Gestalt therapy paternalistically. Clients have to grow up, stand on their own two feet, and "deal with their life problems themselves" (p. 225). Perls's style of doing therapy involved two personal agendas: moving the client from environmental support to self-support and reintegrating the disowned parts of one's personality. His conception of human nature and these two agendas set the stage for a variety of techniques and for his confrontational style of

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conducting therapy. He was a master at intentionally frustrating clients to enhance their awareness.

The Gestalt view of human nature is rooted in existential philosophy, phenomenology, and field theory. Genuine knowledge is the product of what is immediately evident in the experience of the perceiver. Therapy aims at awareness and contact with the environment, which consists of both the external and internal worlds. The quality of contact with aspects of the external world (for example, other people) and the internal world (for example, parts of the self that are disowned) are monitored. The process of "reowning" parts of oneself that have been disowned and the unification process proceed step by step until clients can carry on with their own personal growth. By becoming aware, clients become able to make informed choices and thus to live a more meaningful existence.

A basic assumption of Gestalt therapy is that individuals have the capacity to self-regulate when they are aware of what is happening in and around them. Therapy provides the setting and opportunity for that awareness to be supported and restored. The therapist is attentive to the client's present experience and trusts in the process, thereby assisting the client in moving toward increased awareness, contact, and integration (Brown, 2007).

The Gestalt theory of change posits that the more we work at becoming who or what we are not, the more we remain the same. Fritz's good friend and psychiatrist colleague Arnie Beisser (1970) suggested that authentic change occurs more from being who we are than from trying to be who we are not. Beisser called this simple tenet the **paradoxical theory of change.** It is important for clients to "be" as fully as possible in their current condition, rather than striving to become what they "should be." Gestalt therapists focus on creating the conditions that promote client growth rather than relying on therapist-directed change (Yontef, 2005).

Some Principles of Gestalt Therapy Theory

Several basic principles underlying the theory of Gestalt therapy are briefly described in this section: holism, field theory, the figure-formation process, and organismic self-regulation. Other key concepts of Gestalt therapy are developed in more detail in the sections that follow.

HOLISM *Gestalt* is a German word meaning a whole or completion, or a form that cannot be separated into parts without losing its essence. All of nature is seen as a unified and coherent whole, and the whole is different from the sum of its parts. Because Gestalt therapists are interested in the whole person, they place no superior value on a particular aspect of the individual. Gestalt practice attends to a client's thoughts, feelings, behaviors, body, memories, and dreams. Emphasis may be on a **figure** (those aspects of the individual's experience that are most salient at any moment) or the **ground** (those aspects of the client's presentation that are often out of his or her awareness). Cues to this background can be found on the surface through physical gestures, tone of voice, demeanor, and other nonverbal content. This is often referred to by Gestalt therapists as "attending to the obvious," while paying attention to how the parts fit together, how the individual makes contact with the environment, and integration.

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FIELD THEORY Gestalt therapy is based on **field theory,** which, simply put, asserts that the organism must be seen in its environment, or in its context, as part of the constantly changing field. Gestalt therapy rests on the principle that everything is relational, in flux, interrelated, and in process. Gestalt therapists pay attention to and explore what is occurring at the boundary between the person and the environment.

THE FIGURE-FORMATION PROCESS Derived from the study of visual perception by a group of Gestalt psychologists, the figure-formation process describes how the individual organizes experience from moment to moment. In Gestalt therapy the field differentiates into a foreground (figure) and a background (ground). The **figure-formation process** tracks how some aspect of the environmental field emerges from the background and becomes the focal point of the individual's attention and interest. The dominant needs of an individual at a given moment influence this process (Frew, 1997).

ORGANISMIC SELF-REGULATION The figure-formation process is intertwined with the principle of **organismic self-regulation,** a process by which equilibrium is "disturbed" by the emergence of a need, a sensation, or an interest. Organisms will do their best to regulate themselves, given their own capabilities and the resources of their environment (Latner, 1986). Individuals can take actions and make contacts that will restore equilibrium or contribute to growth and change. What emerges in therapeutic work is associated with what is of interest or what the client needs to pursue a sense of equilibrium or change. Gestalt therapists direct the client's awareness to the figures that emerge from the background during a therapy session and use the figure-formation process as a guide for the focus of therapeutic work.

The Now

One of the main contributions of the Gestalt approach is its emphasis on learning to appreciate and fully experience the present moment. Focusing on the past and the future can be a way to avoid coming to terms with the present. Polster and Polster (1973) developed the thesis that "power is in the present." It is a common tendency for clients to invest their energies in bemoaning their past mistakes and ruminating about how life could and should have been different or engaging in endless resolutions and plans for the future. As clients direct their energy toward what was or what might have been or live in fantasy about the future, the power of the present diminishes.

Phenomenological inquiry involves paying attention to what is occurring now. To help the client make contact with the present moment, Gestalt therapists ask "what" and "how" questions, but rarely ask "why" questions. To promote "now" awareness, the therapist encourages a dialogue in the present tense by asking questions like these: "What is happening now?" "What is going on now?" "What are you experiencing as you sit there and attempt to talk?" "What is your awareness at this moment?" "How are you experiencing your fear?" "How are you attempting to withdraw at this moment?" Phenomenological inquiry also involves suspending any preconceived ideas, assumptions, or interpretations concerning the meaning of a client's experience.

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Most people can stay in the present for only a short time and are inclined to find ways of interrupting the flow of the present. Instead of experiencing their feelings in the here and now, clients often *talk about* their feelings, almost as if their feelings were detached from their present experiencing. One of the aims of Gestalt therapy is to help clients become aware of their present experience. For example, if Josephine begins to talk about sadness, pain, or confusion, the Gestalt therapist invites her to experience her sadness, pain, or confusion *now*. As she attends to the present experience, the therapist gauges how much anxiety or discomfort is present and chooses further interventions accordingly. The therapist might choose not to comment as Josephine moves away from the present moment, only to extend another invitation several minutes later. If a feeling emerges, the therapist might suggest an experiment that would help Josephine to increase her awareness of the feeling, such as exploring where and how she experiences it. Likewise, if a thought or idea emerges, introducing an experiment can help her delve into the thought, explore it more fully, and consider its effects and possible ramifications.

Gestalt therapists recognize that the past will make regular appearances in the present moment, usually because of some lack of completion of that past experience. When the past seems to have a significant bearing on clients' present attitudes or behavior, it is dealt with by bringing it into the present as much as possible. When clients speak about their past, the therapist may ask them to reenact it as though they were living it now. The therapist directs clients to "bring the fantasy here" or "tell me the dream as though you were having it now," striving to help them relive what they experienced earlier. For example, rather than talking about a past childhood trauma with her father, a client becomes the hurt child and talks directly to her father in fantasy, or by imagining him being present in the room in an empty chair.

Unfinished Business

When figures emerge from the background but are not completed and resolved, individuals are left with **unfinished business,** which can be manifested in unexpressed feelings such as resentment, rage, hatred, pain, anxiety, grief, guilt, and abandonment. Unacknowledged feelings create unnecessary emotional debris that clutters present-centered awareness. Because the feelings are not fully experienced in awareness, they linger in the background and are carried into present life in ways that interfere with effective contact with oneself and others: "These incomplete directions *do seek* completion and when they get powerful enough, the individual is beset with preoccupation, compulsive behavior, wariness, oppressive energy and much self-defeating behavior" (Polster & Polster, 1973, p. 36). Unfinished business persists until the individual faces and deals with the unexpressed feelings. The effects of unfinished business often show up in some blockage within the body. Gestalt therapists emphasize paying attention to the bodily experience on the assumption that if feelings are unexpressed they tend to result in some physical sensations or problems.

The **impasse,** or stuck point, is the time when external support is not available or the customary way of being does not work. The therapist's task is to accompany clients in experiencing the impasse without rescuing or frustrating them.

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The counselor assists clients by providing situations that encourage them to fully experience their condition of being stuck. By completely experiencing the impasse, they are able to get into contact with their frustrations and accept whatever is, rather than wishing they were different. Gestalt therapy is based on the notion that individuals have a striving toward actualization and growth and that if they accept all aspects of themselves without judging these dimensions they can begin to think, feel, and act differently.

Contact and Resistances to Contact

In Gestalt therapy, contact is necessary if change and growth are to occur. **Contact** is made by seeing, hearing, smelling, touching, and moving. Effective contact means interacting with nature and with other people without losing one's sense of individuality. Prerequisites for good contact are clear awareness, full energy, and the ability to express oneself (Zinker, 1978). Miriam Polster (1987) claimed that contact is the lifeblood of growth. It is the continually renewed creative adjustment of individuals to their environment. It entails zest, imagination, and creativity. There are only moments of this type of contact, so it is most accurate to think of levels of contact rather than a final state to achieve. After a contact experience, there is typically a withdrawal to integrate what has been learned. Gestalt therapists talk about the two functions of boundaries: to connect and to separate. Both contact and withdrawal are necessary and important to healthy functioning.

Gestalt therapists also focus on interruptions, disturbances, and resistances to contact, which were developed as coping processes but often end up preventing us from experiencing the present in a full and real way. Resistances are typically adopted out of our awareness, and, when they function in a chronic way, can contribute to dysfunctional behavior. Because resistances are developed as a means of coping with life situations, they possess positive qualities as well as problematic ones, and many contemporary Gestalt therapists refer to them as "contact boundary phenomena." Polster and Polster (1973) describe five different kinds of contact boundary disturbances: introjection, projection, retroflection, deflection, and confluence.

Introjection is the tendency to uncritically accept others' beliefs and standards without assimilating them to make them congruent with who we are. These introjects remain alien to us because we have not analyzed and restructured them. When we introject, we passively incorporate what the environment provides rather than clearly identifying what we want or need. If we remain in this stage, our energy is bound up in taking things as we find them and believing that authorities know what is best for us rather than working for things ourselves.

Projection is the reverse of introjection. In projection we disown certain aspects of ourselves by assigning them to the environment. Those attributes of our personality that are inconsistent with our self-image are disowned and put onto, assigned to, and seen in other people; thus, blaming others for lots of our problems. By seeing in others the very qualities that we refuse to acknowledge in ourselves, we avoid taking responsibility for our own feelings and the person who we are, and this keeps us powerless to initiate change. People who use projection as a pattern tend to feel that they are victims of circumstances, and they believe that people have hidden meanings behind what they say.

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Retroflection consists of turning back onto ourselves what we would like to do to someone else or doing to ourselves what we would like someone else to do to or for us. This process is principally an interruption of the action phase in the cycle of experience and typically involves a fair amount of anxiety. People who rely on retroflection tend to inhibit themselves from taking action out of fear of embarrassment, guilt, and resentment. People who self-mutilate or who injure themselves, for example, are often directing aggression inward out of fear of directing it toward others. Depression and psychosomatic complaints are often created by retroflecting. Typically, these maladaptive styles of functioning are adopted outside of our awareness; part of the process of Gestalt therapy is to help us discover a self-regulatory system so that we can deal realistically with the world.

Deflection is the process of distraction or veering off, so that it is difficult to maintain a sustained sense of contact. We attempt to diffuse or defuse contact through the overuse of humor, abstract generalizations, and questions rather than statements (Frew, 1986). When we deflect, we speak through and for others, beating around the bush rather than being direct and engaging the environment in an inconsistent and inconsequential basis, which results in emotional depletion.

Confluence involves blurring the differentiation between the self and the environment. As we strive to blend in and get along with everyone, there is no clear demarcation between internal experience and outer reality. Confluence in relationships involves the absence of conflicts, slowness to anger, and a belief that all parties experience the same feelings and thoughts we do. This style of contact is characteristic of clients who have a high need to be accepted and liked, thus finding enmeshment comfortable. This condition makes genuine contact extremely difficult. A therapist might assist clients who use this channel of resistance by asking questions such as: "What are you doing now?" "What are you experiencing at this moment?" "What do you want right now?"

Terms such as *interruptions in contact* or *boundary disturbance* refer to the characteristic styles people employ in their attempts to control their environment through one of these channels of resistance. The premise in Gestalt therapy is that contact is both normal and healthy, and clients are encouraged to become increasingly aware of their dominant style of blocking contact and their use of resistance. Today's Gestalt therapists readily attend to how clients interrupt contact, approaching the interruptive styles with respect and taking each style seriously, knowing that it has served an important function in the past. It is important to explore what the resistance does for clients: what it protects them from, and what it keeps them from experiencing.

Energy and Blocks to Energy

In Gestalt therapy special attention is given to where energy is located, how it is used, and how it can be blocked. Blocked energy is another form of defensive behavior. It can be manifested by tension in some part of the body, by posture, by keeping one's body tight and closed, by not breathing deeply, by looking away from people when speaking to avoid contact, by choking off sensations, by numbing feelings, and by speaking with a restricted voice, to mention only a few.

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Some of the therapeutic endeavor involves finding the focus of interrupted energy and bringing these sensations to the client's awareness. Clients may not be aware of their energy or where it is located, and they may experience it in a negative way. One of the tasks of the therapist is to help clients identify the ways in which they are blocking energy and transform this blocked energy into more adaptive behaviors. Clients can be encouraged to recognize how their resistance is being expressed in their body. Rather than trying to rid themselves of certain bodily symptoms, clients can be encouraged to delve fully into tension states. For example, by allowing themselves to exaggerate their tight mouth and shaking legs, they can discover for themselves how they are diverting energy and keeping themselves from a full expression of aliveness.

THE THERAPEUTIC PROCESS

Therapeutic Goals

Gestalt therapy does not ascribe to a "goal-oriented" methodology per se. However, as Melnick and Nevis (2005) aptly say, "Because of the complexity of therapeutic work, a well-grounded methodology is essential. . . . The six methodological components we consider vital or integral to Gestalt therapy are: (a) the continuum of experience, (b) the here and now, (c) the paradoxical theory of change, (d) the experiment, (e) the authentic encounter, and (f) process-oriented diagnosis" (pp. 102–103). Despite not being focused on predetermined goals for their clients, Gestalt therapists clearly attend to a basic goal—namely, assisting the client to attain greater awareness, and with it, greater choice. Awareness includes knowing the environment, knowing oneself, accepting oneself, and being able to make contact. Increased and enriched awareness, by itself, is seen as curative. Without awareness clients do not possess the tools for personality change. With awareness they have the capacity to face and accept denied parts as well as to fully experience their subjectivity. They can experience their unity and wholeness. When clients stay with their awareness, important unfinished business will emerge and can be dealt with in therapy. The Gestalt approach helps clients note their own awareness process so that they can be responsible and can selectively and discriminatingly make choices. Awareness emerges within the context of a genuine meeting between client and therapist.

The existential view (see Chapter 6) is that we are continually engaged in a process of remaking and discovering ourselves. We do not have a static identity, but discover new facets of our being as we face new challenges. Gestalt therapy is basically an existential encounter out of which clients tend to move in certain directions. Through a creative involvement in Gestalt process, Zinker (1978) expects clients will do the following:

- Move toward increased awareness of themselves
- Gradually assume ownership of their experience (as opposed to making others responsible for what they are thinking, feeling, and doing)
- Develop skills and acquire values that will allow them to satisfy their needs without violating the rights of others

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- Become more aware of all of their senses
- Learn to accept responsibility for what they do, including accepting the consequences of their actions
- Be able to ask for and get help from others and be able to give to others

Therapist's Function and Role

According to Perls, Hefferline, and Goodman (1951), the therapist's job is to invite clients into an active partnership where they can learn about themselves by adopting an experimental attitude toward life in which they try out new behaviors and notice what happens. Gestalt therapists use active methods and personal engagement with clients to increase their awareness, freedom, and self-direction rather than directing them toward preset goals (Yontef & Jacob, 2011).

Contemporary Gestalt practitioners view clients as the experts on their own experience and encourage them to attend to their sensory awareness in the present moment. Gestalt therapists value self-discovery and assume that clients can discover for themselves the ways in which they block or interrupt their awareness and experience (Watson, Goldman, & Greenberg, 2011). Yontef (1993) stresses that although the therapist functions as a guide and a catalyst, presents experiments, and shares observations, the basic work of therapy is done by the client. Yontef maintains that the therapist's task is to create a climate in which clients are likely to try out new ways of being and behaving. Gestalt therapists do not force change on clients through confrontation. Instead, they work within a context of I/Thou dialogue in a here-and-now framework.

An important function of Gestalt therapists is paying attention to clients' body language. These nonverbal cues provide rich information as they often represent feelings of which the client is unaware. The therapist needs to be alert for gaps in attention and awareness and for incongruities between verbalizations and what clients are doing with their bodies. Therapists might direct clients to speak for and become their gestures or body parts by asking, "What do your eyes say?" "If your hands could speak at this moment, what would they say?" "Can you carry on a conversation between your right and left hands?" Clients may verbally express anger and at the same time smile. Or they may say they are in pain and at the same time laugh. Therapists can ask clients to become aware of how they are using their laughter to mask feelings of anger or pain.

In addition to calling attention to clients' nonverbal language, the Gestalt counselor places emphasis on the relationship between language patterns and personality. Clients' speech patterns are often an expression of their feelings, thoughts, and attitudes. The Gestalt approach focuses on overt speaking habits as a way to increase clients' awareness of themselves, especially by asking them to notice whether their words are congruent with what they are experiencing or instead are distancing them from their emotions.

Language can both describe and conceal. By focusing on language, clients are able to increase their awareness of what they are experiencing in the present moment and of how they are avoiding coming into contact with this here-and-now experience. Here are some examples of the aspects of language that Gestalt therapists might focus on:

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- *"It" talk*. When clients say "it" instead of "I," they are using depersonalizing language. The counselor may ask them to substitute personal pronouns for impersonal ones so that they will assume an increased sense of responsibility. For example, if a client says, "It is difficult to make friends," he could be asked to restate this by making an "I" statement: "I have trouble making friends."

- *"You" talk*. Global and impersonal language tends to keep the person hidden. The therapist often points out generalized uses of "you" and asks the client to substitute "I" when this is what is meant.

- *Questions*. Questions have a tendency to keep the questioner hidden, safe, and unknown. Gestalt counselors often ask clients to change their questions into statements. In making personal statements, clients begin to assume responsibility for what they say. They may become aware of how they are keeping themselves mysterious through a barrage of questions and how this serves to prevent them from making declarations that express themselves.

- *Language that denies power*. Some clients have a tendency to deny their personal power by adding qualifiers or disclaimers to their statements. The therapist may also point out to clients how certain qualifiers subtract from their effectiveness. Experimenting with omitting qualifiers such as "maybe," "perhaps," "sort of," "I guess," "possibly," and "I suppose" can help clients change ambivalent messages into clear and direct statements. Likewise, when clients say "I can't," they are really implying "I won't." Asking clients to substitute "won't" for "can't" often assists them in owning and accepting their power by taking responsibility for their decisions. The counselor must be careful in intervening so that clients do not feel that everything they say is subject to scrutiny. Rather than fostering a morbid kind of introspection, the counselor hopes to foster awareness of what is really being expressed through words.

- *Listening to clients' metaphors*. In his workshops, Erv Polster (1995) emphasizes the importance of a therapist learning how to listen to the metaphors of clients. By tuning into metaphors, the therapist gets rich clues to clients' internal struggles. Examples of metaphors that can be amplified include client statements such as "It's hard for me to spill my guts in here." "At times I feel that I don't have a leg to stand on." "I feel like I have a hole in my soul." "I need to be prepared, in case someone blasts me." "I felt ripped to shreds after you confronted me last week." "After this session, I feel as though I've been put through a meat grinder." Beneath the metaphor may lie a suppressed internal dialogue that represents critical unfinished business or reactions to a present interaction. For example, to the client who says she feels that she has been put through a meat grinder, the therapist could ask: "What is your experience of being ground meat?" or "Who is doing the grinding?" It is essential to encourage this client to say more about what she is experiencing. The art of therapy consists of assisting clients in translating the meaning of their metaphors so that they can be dealt with in therapy.

- *Listening for language that uncovers a story*. Polster (1995) also teaches the value of what he calls "fleshing out a flash." He reports that clients often use language that is elusive yet gives significant clues to a story that illustrates their life struggles. Effective therapists learn to pick out a small part of what someone says and

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then to focus on and develop this element. Clients are likely to slide over pregnant phrases, but the alert therapist can ask questions that will help them flesh out their story line. It is essential for therapists to pay attention to what is fascinating about the person who is sitting before them and get that person to tell a story.

In a workshop I observed Erv Polster's magnificent style in challenging a person (Joe) who had volunteered for a demonstration of an individual session. Although Joe had a fascinating story to reveal about a particular facet of his life, he was presenting himself in a lifeless manner, and the energy was going flat. Eventually, Polster asked him, "Are you keeping my interest right now? Does it matter to you whether I am engaged with you?" Joe looked shocked, but he soon got the point. He accepted Polster's challenge to make sure that he not only kept the therapist interested but also presented himself in a way to keep those in the audience interested. It was clear that Polster was directing Joe's attention to a process of how he was expressing his feelings and life experiences rather than being concerned with what he was talking about.

Polster believes storytelling is not always a form of resistance. Instead, it can be the heart of the therapeutic process. He maintains that people are storytelling beings. The therapist's task is to assist clients in telling their story in a lively way. Polster (1987b) believes many people come to therapy to change the titles of their stories rather than to transform their life stories.

Client's Experience in Therapy

The general orientation of Gestalt therapy is toward dialogue. Whereas Fritz Perls would have said that clients must be confronted about how they avoid accepting responsibility, the dialogic attitude carried into Gestalt therapy originally by Laura Perls creates the ground for a meeting place between client and therapist. Other issues that can become the focal point of therapy include the client–therapist relationship and the similarities in the ways clients relate to the therapist and to others in their environment.

Gestalt therapists do not make interpretations that explain the dynamics of an individual's behavior or tell a client why he or she is acting in a certain way because they are not the experts on the client's experience. Clients in Gestalt therapy are active participants who make their own interpretations and meanings. It is they who increase awareness and decide what they will or will not do with their personal meaning.

Miriam Polster (1987) described a three-stage integration sequence that characterizes client growth in therapy. The first part of this sequence consists of *discovery*. Clients are likely to reach a new realization about themselves or to acquire a novel view of an old situation, or they may take a new look at some significant person in their lives. Such discoveries often come as a surprise to them.

The second stage of the integration sequence is *accommodation,* which involves clients' recognizing that they have a choice. Clients begin by trying out new behaviors in the supportive environment of the therapy office, and then they expand their awareness of the world. Making new choices is often done awkwardly, but with therapeutic support clients can gain skill in coping with difficult

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situations. Clients are likely to participate in out-of-office experiments, which can be discussed in the next therapy session.

The third stage of the integration sequence is *assimilation,* which involves clients' learning how to influence their environment. At this phase clients feel capable of dealing with the surprises they encounter in everyday living. They are now beginning to do more than passively accept the environment. Behavior at this stage may include taking a stand on a critical issue. Eventually, clients develop confidence in their ability to improve and improvise. Improvisation is the confidence that comes from knowledge and skills. Clients are able to make choices that will result in getting what they want. The therapist points out that something has been accomplished and acknowledges the changes that have taken place within the client. At this phase, clients have learned what they can do to maximize their chances of getting what is needed from their environment.

Relationship Between Therapist and Client

As an existential brand of therapy, Gestalt practice involves a person-to-person relationship between therapist and client. Therapists are responsible for the quality of their presence, for knowing themselves and the client, and for remaining open to the client. They are also responsible for establishing and maintaining a therapeutic atmosphere that will foster a spirit of work on the client's part. It is important that therapists allow themselves to be affected by their clients and that they actively share their own present perceptions and experiences as they encounter clients in the here and now.

Gestalt therapists not only allow their clients to be who they are but also remain themselves and do not get lost in a role. Therapists are expected to encounter clients with honest and immediate reactions, and therapists share their personal experience and stories in relevant and appropriate ways. Further, they give feedback that allows clients to develop an awareness of what they are actually doing. Brown (2007) suggests that therapists share their reactions with clients, yet she also stresses the importance of demonstrating an attitude of respect, acceptance, present-centeredness, and presence.

A number of writers have given central importance to the I/Thou relationship and the quality of the therapist's presence, as opposed to technical skills. They warn of the dangers of becoming technique-bound and losing sight of their own being as they engage the client. The therapist's attitudes and behavior and the relationship that is established are what really count (Brown, 2007; Frew, 2008; Lee, 2004; Melnick & Nevis, 2005; Parlett, 2005; E. Polster, 1987a, 1987b; M. Polster, 1987; Yontef & Jacobs, 2011). These writers point out that current Gestalt therapy has moved beyond earlier therapeutic practices.

Many contemporary Gestalt therapists place increasing emphasis on factors such as presence, authentic dialogue, gentleness, more direct self-expression by the therapist, decreased use of stereotypic exercises, and greater trust in the client's experiencing. Laura Perls (1976) stressed the notion that the person of the therapist is more important than using techniques. She says, "There are as many styles as there are therapists and clients who discover themselves and each other and together invent their relationship" (p. 223). Jacobs (1989) asserts that a current

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trend in Gestalt practice is toward greater emphasis on the client–therapist relationship rather than on techniques divorced from the context of this encounter. She believes therapists who operate from this orientation are able to establish a present-centered, nonjudgmental dialogue that allows clients to deepen their awareness and to make contact with another person.

Polster and Polster (1973) emphasized the importance of therapists knowing themselves and being therapeutic instruments. Like artists who need to be in touch with what they are painting, therapists are artistic participants in the creation of new life. The Polsters implore therapists to use their own experiences as essential ingredients in the therapy process. According to them, therapists are more than mere responders or catalysts. If they are to make effective contact with clients, therapists must be in tune with both their clients and themselves. Therapy is a two-way engagement that changes both the client and the therapist. If therapists are not sensitively tuned to their own qualities of tenderness, toughness, and compassion and to their reactions to the client, they become technicians. Experiments should be aimed at awareness, not at simple solutions to a client's problem.

APPLICATION: THERAPEUTIC TECHNIQUES AND PROCEDURES

The Experiment in Gestalt Therapy

Although the Gestalt approach is concerned with the obvious, its simplicity should not be taken to mean that the therapist's job is easy. Developing a variety of interventions is simple, but employing these methods in a mechanical fashion allows clients to continue inauthentic living. If clients are to become authentic, they need contact with an authentic therapist. Yontef and Jacobs (2011) point out that techniques in Gestalt therapy are considered experiments and clients hear the message, "Try this out and see what it is like for you." Gestalt therapy methodology is tailored to the needs of clients, and experiments are typically presented in an invitational manner. Dr. Jon Frew, a Gestalt therapist, demonstrates Gestalt interventions applied to the case of Ruth in *Case Approach to Counseling and Psychotherapy* (Corey, 2013, chap. 6).

Before discussing the variety of Gestalt methods you could include in your repertoire of counseling procedures, it is helpful to differentiate between exercises (or techniques) and experiments. **Exercises** are ready-made techniques that are sometimes used to make something happen in a therapy session or to achieve a goal. They can be catalysts for individual work or for promoting interaction among members of a therapy group. **Experiments**, in contrast, grow out of the interaction between client and therapist, and they emerge within this dialogic process. They can be considered the very cornerstone of experiential learning. Frew (2008) defines the experiment "as a method that shifts the focus of counseling from talking about a topic to an activity that will heighten the client's awareness and understanding through experience" (p. 253). According to Melnick and Nevis (2005), experiments have been confused with techniques: "A technique is a performed experiment with specific learning goals. . . . An experiment, on the other hand, flows

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directly from psychotherapy theory and is crafted to fit the individual as he or she exists in the here and now" (p. 108). The purpose of an experiment is to assist an individual in active self-exploration. Melnick and Nevis suggest using the Gestalt continuum of experience as a guide for custom designing experiments.

The experiment is fundamental to Gestalt therapy. Zinker (1978) sees therapy sessions as a series of experiments, which are the avenues for clients to learn experientially. What is learned from an experiment is a surprise to both the client and the therapist. Gestalt experiments are a creative adventure and a way in which clients can express themselves behaviorally. Experiments are spontaneous, one-of-a-kind, and relevant to a particular moment and a particular development of a figure-formation process. They are not designed to achieve a particular goal but occur in the context of a moment-to-moment contacting process between therapist and client. Polster (1995) indicates that experiments are designed by the therapist and evolve from the theme already developing through therapeutic engagement, such as the client's report of needs, dreams, fantasies, and body awareness. Gestalt therapists invite clients to engage in experiments that lead to fresh emotional experiencing and new insights (Strumpfel & Goldman, 2002). Experimentation is an attitude inherent in all Gestalt therapy; it is a collaborative process with full participation of the client. Clients test an experiment to determine what does and does not fit for them through their own awareness (Yontef, 1993, 1995).

Miriam Polster (1987) says that an experiment is a way to bring out some kind of internal conflict by making this struggle an actual process. It is aimed at facilitating a client's ability to work through the stuck points of his or her life. Experiments encourage spontaneity and inventiveness by bringing the possibilities for action directly into the therapy session. By dramatizing or playing out problem situations or relationships in the relative safety of the therapy context, clients increase their range of flexibility of behavior. According to M. Polster, Gestalt experiments can take many forms: imagining a threatening future encounter; setting up a dialogue between a client and some significant person in his or her life; dramatizing the memory of a painful event; reliving a particularly profound early experience in the present; assuming the identity of one's mother or father through role playing; focusing on gestures, posture, and other nonverbal signs of inner expression; or carrying on a dialogue between two conflicting aspects within the person. Through these experiments, clients may actually experience the feelings associated with their conflicts. Experiments bring struggles to life by inviting clients to enact them in the present. It is crucial that experiments be tailored to each individual and used in a timely and appropriate manner; they also need to be carried out in a context that offers a balance between support and risk. Sensitivity and careful attention on the therapist's part are essential so that clients are "neither blasted into experiences that are too threatening nor allowed to stay in safe but infertile territory" (Polster & Polster, 1990, p. 104).

Preparing Clients for Gestalt Experiments

If students-in-training limit their understanding of Gestalt therapy to simply reading about the approach, Gestalt methods are likely to seem abstract and the notion of experiments may seem strange. Asking clients to "become" an object in one

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of their dreams, for instance, may seem silly and pointless. It is important for counselors to *personally* experience the power of Gestalt experiments and to feel comfortable suggesting them to clients. In this regard, it can be most useful for trainees to personally experience Gestalt methods as a client.

It is also essential that counselors establish a relationship with their clients, so that the clients will feel trusting enough to participate in the learning that can result from Gestalt experiments. Clients will get more from Gestalt experiments if they are oriented and prepared for them. Through a trusting relationship with the therapist, clients are likely to recognize their resistance and allow themselves to participate in these experiments.

If clients are to cooperate, counselors must avoid directing them in a commanding fashion to carry out an experiment. Typically, I ask clients if they are willing to try out an experiment to see what they might learn from it. I also tell clients that they can stop when they choose to, so the power is with them. Clients at times say that they feel silly or self-conscious or that the task feels artificial or unreal. At such times I am likely to respond by asking, "Are you willing to give it a try and see what happens?"

I cannot overemphasize the power of the therapeutic relationship and the necessity for trust as the foundation for implementing any experiment. If I meet with hesitation, I tend to be interested in exploring the client's reluctance. It is helpful to know the reason the client is stopping. Hesitation in becoming emotionally involved often is a function of the client's cultural background. Some clients have been conditioned to work hard to maintain emotional control. They may have reservations about expressing intense feelings openly, even if they are in an emotional state. This can well be due to their socialization and to cultural norms they abide by. In some cultures it is considered rude to express emotions openly, and there are certain cultural injunctions against showing one's vulnerability or psychological pain. Gestalt experiments work best when the therapist is respectful of the client's cultural background and is in good contact with the person. Therapists need to understand that for clients with a long history of containing their feelings, they will most likely be reluctant to participate in experiments that are likely to bring their emotions to the surface. Of course, many men have been socialized not to express intense feelings. Their reluctance to allow themselves to be emotional should be dealt with in a respectful manner.

Other clients may resist becoming emotionally involved because of their fear, lack of trust, concern over losing control, or some other concern. The *way* in which clients resist doing an experiment reveals a great deal about their personality and their way of being in the world. Therefore, Gestalt therapists expect and respect the emergence of reluctance on a client's part. The therapist's aim is not to eliminate clients' defenses but to meet clients wherever they are.

The essence of current Gestalt therapy involves honoring and respecting reluctance or resistance and supporting clients to become more aware of their experience. If the therapist meets with hesitation, it is a good idea to explore its meaning for the client. Contemporary Gestalt therapy places much less emphasis on resistance than the early version of Gestalt therapy. Although it is possible to

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look at "resistance to awareness" and "resistance to contact," the idea of resistance is viewed as unnecessary by some Gestalt therapists. Frew (2008) argues that the notion of resistance is completely foreign to the theory and practice of Gestalt therapy and suggests that resistance is a term frequently used for clients who are not doing what the therapist wants them to do. Polster and Polster (1976) suggest that it is best for therapists to observe what is actually and presently happening rather than trying to make something happen. This gets away from the notion that clients are resisting and thus behaving wrongly. According to the Polsters, change occurs through contact and awareness—one does not have to try to change. Maurer (2005) writes about "appreciating resistance" as a creative adjustment to a situation rather than something to overcome. Maurer claims that we need to respect resistance, take it seriously, and view it as "the energy" and not "the enemy."

It is well to remember that Gestalt experiments are designed to expand clients' awareness and to help them try out new modes of behavior. Within the safety of the therapeutic situation, clients are given opportunities and encouraged to "try on" a new behavior. This heightens the awareness of a particular aspect of functioning, which leads to increased self-understanding (Breshgold, 1989; Yontef, 1995). Experiments are only means to the end of helping people become more aware and making changes they most desire.

The Role of Confrontation

Students are sometimes put off by their perception that a Gestalt counselor's style is direct and confrontational. I tell my students that it is a mistake to equate the practice of any theory with its founder. As has been mentioned, the contemporary practice of Gestalt therapy has progressed beyond the style exhibited by Fritz Perls. Yontef (1993) refers to the Perlsian style as a "boom-boom-boom therapy" characterized by theatrics, abrasive confrontation, and intense catharsis. He implies that the charismatic style of Perls probably met more of his own narcissistic needs than the needs of his clients. Yontef (1993, 1999) is critical of the anti-intellectual, individualistic, dramatic, and confrontational flavor that characterized Gestalt therapy in the "anything goes environment" of the 1960s and 1970s. According to Yontef (1999), the newer version of relational Gestalt therapy has evolved to include more support and increased kindness and compassion in therapy. This approach "combines sustained empathic inquiry with crisp, clear, and relevant awareness focusing" (p. 10). Perls practiced a highly confrontational approach as a way to deal with avoidance. However, this confrontational model is not representative of Gestalt therapy as it is currently being practiced (Bowman, 2005; Frew, 2008; Yontef & Jacobs, 2011).

Confrontation is used at times in the practice of Gestalt therapy, yet it does not have to be viewed as a harsh attack. **Confrontation** can be done in such a way that clients cooperate, especially when they are *invited* to examine their behaviors, attitudes, and thoughts. Therapists can encourage clients to look at certain incongruities, especially gaps between their verbal and nonverbal expression. Further, confrontation does not have to be aimed at weaknesses or negative traits; clients can be challenged to recognize how they are blocking their strengths.

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Counselors who care enough to make demands on their clients are telling them, in effect, that they could be in fuller contact with themselves and others. Ultimately, however, clients must decide for themselves if they want to accept this invitation to learn more about themselves. This caveat needs to be kept in mind with all of the experiments that are to be described.

Gestalt Therapy Interventions

Experiments can be useful tools to help the client gain fuller awareness, experience internal conflicts, resolve inconsistencies and dichotomies, and work through an impasse that is preventing completion of unfinished business. Exercises can be used to elicit emotion, produce action, or achieve a specific goal. When used at their best, the interventions described here fit the therapeutic situation and highlight whatever the client is experiencing. The following material is based on Levitsky and Perls (1970), with my own suggestions added for implementing these methods.

THE INTERNAL DIALOGUE EXERCISE One goal of Gestalt therapy is to bring about integrated functioning and acceptance of aspects of one's personality that have been disowned and denied. Gestalt therapists pay close attention to splits in personality function. A main division is between the "top dog" and the "underdog," and therapy often focuses on the war between the two.

The top dog is righteous, authoritarian, moralistic, demanding, bossy, and manipulative. This is the "critical parent" that badgers with "shoulds" and "oughts" and manipulates with threats of catastrophe. The underdog manipulates by playing the role of victim: by being defensive, apologetic, helpless, and weak and by feigning powerlessness. This is the passive side, the one without responsibility, and the one that finds excuses.

The top dog and the underdog are engaged in a constant struggle for control. The struggle helps to explain why one's resolutions and promises often go unfulfilled and why one's procrastination persists. The tyrannical top dog demands that one be thus-and-so, whereas the underdog defiantly plays the role of disobedient child. As a result of this struggle for control, the individual becomes fragmented into controller and controlled. The civil war between the two sides continues, with both sides fighting for their existence.

The conflict between the two opposing poles in the personality is rooted in the mechanism of introjection, which involves incorporating aspects of others, usually parents, into one's personality. It is essential that clients become aware of their introjects, especially the toxic introjects that poison the person and prevent personality integration.

The **empty-chair technique** is one way of getting the client to externalize the introject, a technique Perls used a great deal. Using two chairs, the therapist asks the client to sit in one chair and be fully the top dog and then shift to the other chair and become the underdog. The dialogue can continue between both sides of the client. Essentially, this is a role-playing technique in which all the parts are played by the client. In this way the introjects can surface, and the client can experience the conflict more fully. The conflict can be resolved by the client's acceptance and

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integration of both sides. This exercise helps clients get in touch with a feeling or a side of themselves that they may be denying; rather than merely talking about a conflicted feeling, they intensify the feeling and experience it fully. Further, by helping clients realize that the feeling is a very real part of themselves, the intervention discourages clients from disassociating the feeling.

The goal of this exercise is to promote a higher level of integration between the polarities and conflicts that exist in everyone. The aim is not to rid oneself of certain traits but to learn to accept and live with the polarities.

MAKING THE ROUNDS Making the rounds is a Gestalt exercise that involves asking a person in a group to go up to others in the group and either speak to or do something with each person. The purpose is to confront, to risk, to disclose the self, to experiment with new behavior, and to grow and change. I have experimented with "making the rounds" when I sensed that a participant needed to face each person in the group with some theme. For example, a group member might say: "I've been sitting here for a long time wanting to participate but holding back because I'm afraid of trusting people in here. And besides, I don't think I'm worth the time of the group anyway." I might counter with "Are you willing to do something right now to get yourself more invested and to begin to work on gaining trust and self-confidence?" If the person answers affirmatively, my suggestion could well be, "Go around to each person and finish this sentence: 'I don't trust you because . . .'." Any number of exercises could be invented to help individuals involve themselves and choose to work on the things that keep them frozen in fear.

Some other related illustrations and examples that I find appropriate for the making-the-rounds intervention are reflected in clients' comments such as these: "I would like to reach out to people more often." "Nobody in here seems to care very much." "I'd like to make contact with you, but I'm afraid of being rejected [or accepted]." "It's hard for me to accept compliments; I always discount good things people say to me."

THE REVERSAL EXERCISE Certain symptoms and behaviors often represent reversals of underlying or latent impulses. Thus, the therapist could ask a person who claims to suffer from severe inhibitions and excessive timidity to play the role of an exhibitionist. I remember a client in one of our therapy groups who had difficulty being anything but sugary sweet. I asked her to reverse her typical style and be as negative as she could be. The reversal worked well; soon she was playing her part with real gusto, and later she was able to recognize and accept her "negative side" as well as her "positive side."

The theory underlying the reversal technique is that clients take the plunge into the very thing that is fraught with anxiety and make contact with those parts of themselves that have been submerged and denied. This technique can help clients begin to accept certain personal attributes that they have tried to deny.

THE REHEARSAL EXERCISE Oftentimes we get stuck rehearsing silently to ourselves so that we will gain acceptance. When it comes to the performance, we experience stage fright, or anxiety, because we fear that we will not play our role well. Internal rehearsal consumes much energy and frequently inhibits our spontaneity

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and willingness to experiment with new behavior. When clients share their rehearsals out loud with a therapist, they become more aware of the many preparatory means they use in bolstering their social roles. They also become increasingly aware of how they try to meet the expectations of others, of the degree to which they want to be approved, accepted, and liked, and of the extent to which they go to attain acceptance.

THE EXAGGERATION EXERCISE One aim of Gestalt therapy is for clients to become more aware of the subtle signals and cues they are sending through body language. Movements, postures, and gestures may communicate significant meanings, yet the cues may be incomplete. In this exercise the person is asked to exaggerate the movement or gesture repeatedly, which usually intensifies the feeling attached to the behavior and makes the inner meaning clearer. Some examples of behaviors that lend themselves to the exaggeration technique are trembling (shaking hands, legs), slouched posture and bent shoulders, clenched fists, tight frowning, facial grimacing, crossed arms, and so forth. If a client reports that his or her legs are shaking, the therapist may ask the client to stand up and exaggerate the shaking. Then the therapist may ask the client to put words to the shaking limbs.

STAYING WITH THE FEELING Most clients desire to escape from fearful stimuli and to avoid unpleasant feelings. At key moments when clients refer to a feeling or a mood that is unpleasant and from which they have a great desire to flee, the therapist may urge clients to stay with their feeling and encourage them to go deeper into the feeling or behavior they wish to avoid. Facing and experiencing feelings not only takes courage but also is a mark of a willingness to endure the pain necessary for unblocking and making way for newer levels of growth.

THE GESTALT APPROACH TO DREAM WORK In psychoanalysis dreams are interpreted, intellectual insight is stressed, and free association is used to explore the unconscious meanings of dreams. The Gestalt approach does not interpret and analyze dreams. Instead, the intent is to bring dreams back to life and relive them as though they were happening now. The dream is acted out in the present, and the dreamer becomes a part of his or her dream. The suggested format for working with dreams includes making a list of all the details of the dream, remembering each person, event, and mood in it, and then becoming each of these parts by transforming oneself, acting as fully as possible and inventing dialogue. Each part of the dream is assumed to be a projection of the self, and the client creates scripts for encounters between the various characters or parts. All of the different parts of a dream are expressions of the client's own contradictory and inconsistent sides, and, by engaging in a dialogue between these opposing sides, the client gradually becomes more aware of the range of his or her own feelings.

Perls's concept of projection is central in his theory of dream formation; every person and every object in the dream represents a projected aspect of the dreamer. Perls (1969a) suggested that "we start with the impossible assumption that whatever we believe we see in another person or in the world is nothing but a projection" (p. 67). Recognizing the senses and understanding projections go hand in hand. Clients do not think about or analyze the dream but use it as a script and experiment with the dialogue among the various parts of the dream. Because clients can

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act out a fight between opposing sides, eventually they can appreciate and accept their inner differences and integrate the opposing forces. Freud called the dream the royal road to the unconscious, but to Perls dreams are the "royal road to integration" (p. 66).

According to Perls, the dream is the most spontaneous expression of the existence of the human being. It represents an unfinished situation, but every dream also contains an existential message regarding oneself and one's current struggle. Everything can be found in dreams if all the parts are understood and assimilated; dreams serve as an excellent way to discover personality voids by revealing missing parts and clients' methods of avoidance. Perls asserts that if dreams are properly worked with, the existential message becomes clearer. If people do not remember dreams, they may be refusing to face what is wrong with their life. At the very least, the Gestalt counselor asks clients to talk to their missing dreams. For example, as directed by her therapist, a client reported the following dream in the present tense, as though she were still dreaming:

> I have three monkeys in a cage. One big monkey and two little ones! I feel very attached to these monkeys, although they are creating a lot of chaos in a cage that is divided into three separate spaces. They are fighting with one another—the big monkey is fighting with the little monkey. They are getting out of the cage, and they are clinging onto me. I feel like pushing them away from me. I feel totally overwhelmed by the chaos that they are creating around me. I turn to my mother and tell her that I need help, that I can no longer handle these monkeys because they are driving me crazy. I feel very sad and very tired, and I feel discouraged. I am walking away from the cage, thinking that I really love these monkeys, yet I have to get rid of them. I am telling myself that I am like everybody else. I get pets, and then when things get rough, I want to get rid of them. I am trying very hard to find a solution to keeping these monkeys and not allowing them to have such a terrible effect on me. Before I wake up from my dream, I am making the decision to put each monkey in a separate cage, and maybe that is the way to keep them.

The therapist then asked his client, Brenda, to "become" different parts of her dream. Thus, she became the cage, and she became and had a dialogue with each monkey, and then she became her mother, and so forth. One of the most powerful aspects of this technique was Brenda's reporting her dream as though it were still happening. She quickly perceived that her dream expressed a struggle she was having with her husband and her two children. From her dialogue work, Brenda discovered that she both appreciated and resented her family. She learned that she needed to let them know about her feelings and that together they might work on improving an intensely difficult lifestyle. She did not need an interpretation from her therapist to understand the clear message of her dream.

Application to Group Counseling

As a therapeutic orientation based on field theory, Gestalt therapy is well suited for a group context. Gestalt therapy encourages direct experience and actions as opposed to merely *talking about* conflicts, problems, and feelings. If members have anxieties pertaining to some future event, they can enact these future concerns in the present. This here-and-now focus enlivens the group and assists members in vividly exploring their concerns. Moving from *talking about* to action is often done by the use of experiments in a group. Gestalt therapy employs a rich variety of

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interventions designed to intensify what group members are experiencing in the present moment for the purpose of leading to increased awareness.

When one member is the focus of work, other members can be used to enhance an individual's work. Through the skill of linking, the group leader can bring a number of members into the exploration of a problem. I prefer an interactive style of Gestalt group work and find that bringing in an interpersonal dimension maximizes the therapeutic potency within the group. I do not like to introduce a technique to promote something happening within a group; rather, I tend to invite members to try out different behavioral styles as a way to heighten what a given member might be experiencing at the moment. A group format provides a context for a great deal of creativity in using interventions and designing experiments. These experiments need to be tailored to each group member and used in a timely manner; they also need to be carried out in a context that offers a balance between support and risk. Experiments, at their best, evolve from what is going on within individual members and what is happening in the group at the moment.

Although Gestalt group leaders encourage members to heighten their awareness and attend to their interpersonal style of relating, leaders tend to take an active role in creating experiments to help members tap their resources. Gestalt leaders are actively engaged with the members, and leaders frequently engage in self-disclosure as a way to enhance relationships and create a sense of mutuality within the group. Gestalt leaders focus on awareness, contact, and experimentation (Yontef & Jacobs, 2011).

If members experience the group as being a safe place, they will be inclined to move into the unknown and challenge themselves. To increase the chances that members will benefit from Gestalt methods, group leaders need to communicate the general purpose of these interventions and create an experimental climate. Leaders are not trying to push an agenda; rather, members are free to try something new and determine for themselves the outcomes of an experiment.

In training workshops in group counseling that Marianne Schneider Corey and I conducted in Korea, the Gestalt approach was well accepted. Group members were very open and willing to share themselves emotionally once a climate of safety was created. Adopting the stance of phenomenological inquiry, we strive to avoid making assumptions about the members of a group, and we are careful not to impose our worldviews or values on them. Instead, we approach clients with respect, interest, compassion, and presence. We work collaboratively with our clients to discover how to best help them resolve the difficulties they experience internally, interpersonally, and in the context of their social environment. Although it is unrealistic to think you need to know everything about different cultures, it is essential to bring an attitude of respect and appreciation for differences to your work in diverse cultural environments around the world. With these attitudes we found that we were able to use many Gestalt interventions with Korean people in a group training context. In some ways this is not surprising because in Korea there is an emphasis on collectivistic values, and group work fits well into the Korean culture.

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For a more detailed account of Gestalt therapy in groups, see Feder and Frew (2008), Feder (2006), and Corey (2012, chap. 11).

GESTALT THERAPY FROM A MULTICULTURAL PERSPECTIVE



Strengths From a Diversity Perspective

There are opportunities to creatively use Gestalt methods with culturally diverse populations if interventions are timed appropriately and used flexibly. Frew (2008) has made the case that "contemporary Gestalt therapy has evolved as a culturally sensitive and diversity friendly orientation" (p. 267). He notes that Gestalt therapy can be a useful and effective approach with clients from diverse backgrounds because it takes the clients' context into account. One of the advantages of drawing on Gestalt experiments is that they can be tailored to fit the unique way in which an individual perceives and interprets his or her culture. Although most therapists have preconceptions, Gestalt therapists strive to approach each client in an open way. They do this by checking out their biases and views in dialogue with the client. This is particularly important in working with individuals from other cultures.

Fernbacher and Plummer (2005) stress the importance of assisting Gestalt therapy trainees in developing their own awareness: "To develop awareness of one's cultural identity, one must attend to its influence not only in training but also as part of ongoing development of a Gestalt practitioner" (p. 121). Fernbacher and Plummer contend that "to undertake work across cultures from a Gestalt perspective, it is essential that we explore our own cultural selves . . . to make contact and encourage contact in and with others, we need to know about ourselves" (p. 131).

Gestalt therapy is particularly effective in helping people integrate the polarities within themselves. Many bicultural clients experience an ongoing struggle to reconcile what appears to be diverse aspects of the two cultures in which they live. In one of my weeklong groups, a dynamic piece of work was done by a woman with European roots. Her struggle consisted of integrating her American side with her experiences in Germany as a child. I suggested that she "bring her family into this group" by talking to selected members in the group as though they were members of her family. I invited her to imagine that she was 8 years old and that she could now say to her parents and siblings things that she had never expressed. She was asked to speak in German (since this was her primary language as a child). The combined factors of her trust in the group, her willingness to re-create an early scene by reliving it in the present moment, and her symbolic work with fantasy helped her achieve a significant breakthrough. She was able to put a new ending to an old and unfinished situation through her participation in this Gestalt experiment.

There are many opportunities to apply Gestalt experiments in creative ways with diverse client populations. In cultures where indirect speech is the norm, nonverbal behaviors may emphasize the unspoken content of verbal communication. These clients may express themselves nonverbally more expressively than they do with words. Gestalt therapists typically ask clients to focus on their gestures, facial

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expressions, and what they are experiencing within their own bodies. They attempt to fully understand the background of their clients' culture. They are concerned about which aspects of this background become central or figural for their clients and what meaning clients place on these figures.

Shortcomings From a Diversity Perspective

To a greater extent than is true of most other approaches, there are some potential problems in too quickly utilizing some Gestalt experiments with some clients. Gestalt methods can lead to a high level of intense feelings. This focus on affect has some clear limitations with those clients who have been culturally conditioned to be emotionally reserved and to avoid openly expressing feelings. As mentioned earlier, some individuals believe expressing feelings openly is a sign of weakness and a display of one's vulnerability. Counselors who operate on the assumption that catharsis is necessary for any change to occur are likely to find certain clients becoming increasingly reluctant to participate in experiments, and such clients may prematurely terminate counseling. Other individuals have strong cultural injunctions prohibiting them from directly expressing their emotions to their parents (such as "Never show your parents that you are angry at them" or "Strive for peace and harmony, and avoid conflicts"). I recall a client from India who was asked by his counselor to "bring your father into the room." The client was very reluctant to even symbolically tell his father of his disappointment with their relationship. In his culture the accepted way to deal with his father was to use his uncle as a go-between, and it was considered highly inappropriate to express any negative feelings toward his father. The client later said that he would have felt very guilty if he had symbolically told his father what he sometimes thought and felt.

Gestalt therapists who have truly integrated their approach are sensitive enough to practice in a flexible way. They consider the client's cultural framework and are able to adapt methods that are likely to be well received. They strive to help clients experience themselves as fully as possible in the present, yet they are not rigidly bound by dictates, nor do they routinely intervene whenever clients stray from the present. Sensitively staying in contact with a client's flow of experiencing entails the ability to focus on the person and not on the mechanical use of techniques for a certain effect.

Gestalt Therapy Applied to the Case of Stan



Gestalt-oriented therapy focuses on the unfinished business Stan has with his parents, siblings, and ex-wife. It appears that this unfinished business consists mainly of feelings of resentment, and Stan turns this resentment on himself. His present life situation is spotlighted, but he may also need to reexperience past feelings that could be interfering with his present attempts to develop intimacy with others.

Although the focus is on Stan's present behavior, I guide him toward becoming aware of how he is carrying old baggage around and how it interferes with his life today. My task is to assist him in re-creating the context in which he made *creative adjustments* during his childhood years that no longer serving him well. One of his cardinal introjections was, "I'm stupid, and it would be better if I did not exist."

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Stan has been influenced by cultural messages that he has accepted. I am interested in exploring his cultural background, including his values and the values characteristic of his culture. With this focus, I assist Stan in identifying some of the following cultural introjections: "Don't talk about your family with strangers, and don't hang your dirty linen in public." "Don't confront your parents because they deserve respect." "Don't be too concerned about yourself." "Don't show your vulnerabilities; hide your feelings and weaknesses." I invite Stan to examine those introjections to assess their utility in his present circumstances. Although he can decide to retain those aspects of his culture that he prizes, he is in a position to modify or reject other cultural expectations. Of course, this will be done when these issues emerge in the foreground of his work.

I ask Stan to attend to what he becomes aware of as the session begins: "What are you experiencing as we are getting started today?" As I encourage Stan to tune in to his present experience and selectively make observations, a number of figures will emerge. The goal is to focus on a figure of interest, one that seems to hold the most energy or relevance for Stan. When a figure is identified, my task is to deepen Stan's awareness of this thought, feeling, body sensation, or insight through related experiments.

In typical Gestalt fashion, Stan deals with his present struggles within the context of our relationship and through experimentation. One possible experiment would involve Stan becoming some of those individuals who told him how to think, feel, and behave as a child. He can then become the child that he was and respond to them from the place where he feels the most confusion or pain. He experiences in new ways the feelings that accompany his beliefs about himself, and he comes to a deeper appreciation of how his feelings and thoughts influence what he is doing today.

Stan has learned to hide his emotions rather than to reveal them. Understanding this about him, we explore his objections and concerns about "getting into feelings." The figure of interest now is his hesitation to experience or express emotion. Although I have no agenda to get Stan to experience his feelings at this point, it is important for him to increase his awareness of his reluctance and to explore the meaning it holds for him.

If Stan decides that he wants to experience his emotions rather than deny them, I ask: "What are you aware of now having said what you did?" Stan says that he can't get his ex-wife out of his mind. He tells me about the pain he feels over that relationship and how he is frightened of getting involved again lest he be hurt again. I continue to ask him to focus inward and get a sense of what stands out for him at this very moment. Stan replies: "I'm hurt and angry over all the pain that I've allowed her to inflict on me." I ask him to imagine himself in earlier scenes with his ex-wife, as though the painful situation were occurring in the here and now. He symbolically relives and reexperiences the situation by talking "directly" to his wife. By expressing his resentments and hurts directly, Stan can begin to complete some unfinished business that is interfering with his current functioning. By participating in this experiment, Stan is attaining more awareness of what he is now doing and how he keeps himself locked into his past.

Follow-Up: You Continue as Stan's Gestalt Therapist

Use these questions to help you think about how to work with Stan using the Gestalt approach:

- How might you begin a session with Stan? Would you suggest a direction he should pursue? Would you wait for him to initiate work? Would you ask him to continue from where he left off in the previous session? Would you attend to whatever theme or issue becomes figural to him?
- What unfinished business can you identify in Stan's case? Does any of his experience of being stuck remind you of yourself? How might you work with Stan if he did bring up your own unfinished business?
- What kind of an experiment might you propose to assist Stan in learning more about his hesitation and reluctance to access and express his feelings?

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- Stan participated in an experiment to deal with pain, resentment, and hurt over situations with his ex-wife. How might you have worked with the material Stan brought up? What kind of experiment might you design? How would you decide what kind of experiment to create?
- How might you work with Stan's cultural messages? Would you be able to respect his cultural values and still encourage him to make an assessment of some of the ways in which his culture is affecting him today?



See *DVD for Theory and Practice of Counseling and Psychotherapy: The Case of Stan and Lecturettes* (Session 6 on Gestalt therapy) for a demonstration of my approach to counseling Stan from this perspective. This session consists of Stan exploring one of his dreams in Gestalt fashion.

SUMMARY AND EVALUATION

Summary

Gestalt therapy is an experiential approach that stresses present awareness and the quality of contact between the individual and the environment. The major focus is on assisting the client to become aware of how behaviors that were once part of creatively adjusting to past environments may be interfering with effective functioning and living in the present. The goal of the approach is, first and foremost, to gain awareness.

Another therapeutic aim is to assist clients in exploring how they make contact with elements of their environment. Change occurs through the heightened awareness of "what is." Because the Gestalt therapist has no agenda beyond assisting clients to increase their awareness, there is no need to label a client's behavior as "resistance." Instead, the therapist simply follows this new process as it emerges. The therapist has faith that self-regulation is a naturally unfolding process that does not have to be controlled (Breshgold, 1989). With expanded awareness, clients are able to reconcile polarities and dichotomies within themselves and proceed toward the reintegration of all aspects of themselves.

The therapist works with the client to identify the figures, or most salient aspects of the individual–environmental field, as they emerge from the background. The Gestalt therapist believes each client is capable of self-regulating if those figures are engaged and resolved so others can replace them. The role of the Gestalt therapist is to help clients identify the most pressing issues, needs, and interests and to design experiments that sharpen those figures or that explore resistances to contact and awareness. Gestalt therapists are encouraged to be appropriately self-disclosing, both about their here-and-now reactions in the therapy hour and about their personal experiences (Yontef & Jacobs, 2011; Zahm, 1998).

Contributions of Gestalt Therapy

One contribution of Gestalt therapy is the exciting way in which the past is dealt with in a lively manner by bringing relevant aspects into the present. Therapists challenge clients in creative ways to become aware of and work with issues that are obstructing current functioning. Further, paying attention to the obvious verbal and nonverbal leads provided by clients is a useful way to approach a counseling

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session. Through the skillful and sensitive use of Gestalt interventions, practitioners can assist clients in heightening their present-centered awareness of what they are thinking and feeling as well as what they are doing.

Gestalt methods bring conflicts and human struggles to life. Gestalt therapy is a creative approach that uses experiments to move clients from talk to action and experience. The creative and spontaneous use of active experiments is a pathway to experiential learning. The focus is on growth and enhancement rather than being a system of techniques to treat disorders, which reflects an early Gestalt motto, "You don't have to be sick to get better." Clients are provided with a wide range of tools—in the form of Gestalt experiments—for discovering new facets of themselves and making decisions about changing their course of living.

The Gestalt approach to working with dreams is a unique pathway for people to increase their awareness of key themes in their lives. By seeing each aspect of a dream as a projection of themselves, clients are able to bring the dream to life, to interpret its personal meaning, and to assume responsibility for it.

Gestalt therapy is a holistic approach that values each aspect of the individual's experience equally. Therapists allow the figure-formation process to guide them. They do not approach clients with a preconceived set of biases or a set agenda. Instead, they place emphasis on what occurs at the boundary between the individual and the environment. Therapists do not try to move the client anywhere. The main goal is to increase the client's awareness of "what is." Instead of trying to make something happen, the therapist's role is assisting the client to increase awareness that will allow reidentification with the part of the self from which he or she is alienated.

A key strength of Gestalt therapy is the attempt to integrate theory, practice, and research. Although Gestalt therapy was light on empirical research for several years, it has come more into vogue recently. Two books show potential for influencing future research: *Handbook for Theory, Research and Practice in Gestalt Therapy* (Brownell, 2008); and *Becoming a Practitioner Researcher: A Gestalt Approach to Holistic Inquiry* (Barber, 2006). Strumpfel and Goldman (2002) note that both process and outcome studies have advanced the theory and practice of Gestalt therapy, and they summarize a number of significant findings based on outcome research:

- Outcome studies have demonstrated Gestalt therapy to be equal to or greater than other therapies for various disorders.
- More recent studies have shown that Gestalt therapy has a beneficial impact with personality disturbances, psychosomatic problems, and substance addictions.
- The effects of Gestalt therapy tend to be stable in follow-up studies 1 to 3 years after the termination of treatment.
- Gestalt therapy has demonstrated effectiveness in treating a variety of psychological disorders.

Limitations and Criticisms of Gestalt Therapy

Most of my criticisms of Gestalt therapy pertain to the older version, or the style of Fritz Perls, which emphasized confrontation and de-emphasized the cognitive factors of personality. This style of Gestalt therapy placed more attention on using

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techniques to confront clients and getting them to experience their feelings. Contemporary Gestalt therapy has come a long way, and more attention is being given to theoretical instruction, theoretical exposition, and cognitive factors in general (Yontef, 1993, 1995).

In Gestalt therapy clients clarify their thinking, explore beliefs, and put meaning to experiences they are reliving in therapy. Clients assume an active role in participating in experiments, and they learn experientially. The emphasis is on *facilitating* the clients' own process of self-discovery and learning. This experiential and self-directed learning process is based on the fundamental belief in organismic self-regulation, which implies that clients arrive at their own truths through awareness and improved contact with the environment. It seems to me, however, that clients can engage in self-discovery and at the same time benefit from appropriate teaching by the therapist. In addition to the benefits of experiential learning, clients can profit from timely and useful information, and a psychoeducational focus can enhance the learning process.

Current Gestalt practice places a high value on the contact and dialogue between therapist and client. For Gestalt therapy to be effective, the therapist must have a high level of personal development. Being aware of one's own needs and seeing that they do not interfere with the client's process, being present in the moment, and being willing to be nondefensive and self-revealing all demand a lot of the therapist. There is a danger that therapists who are inadequately trained will be primarily concerned with impressing clients. Yontef and Jacobs (2011) maintain that the competent practice of Gestalt therapy requires a strong general clinical background and training, not only in the theory and practice of Gestalt theory but also in personality theory, psychopathology, and knowledge of psychodynamics. Competent practitioners need to have engaged in their own personal therapy and to have had advanced clinical training and supervised experience.

SOME CAUTIONS Typically, Gestalt therapists are highly active, and if they do not have the characteristics mentioned by Zinker (1978)—sensitivity, timing, inventiveness, empathy, and respect for the client—their experiments can easily boomerang. Some therapists who do not have a solid grounding in the theory and practice of Gestalt therapy have employed Fritz Perls's techniques, resulting in an abuse of power. Inept therapists may use powerful techniques to stir up feelings and open up problems clients have kept from full awareness, only to abandon the clients once they have managed to have a dramatic catharsis. Such a failure to stay with clients and help them work through what they have experienced and bring some closure to the experience can be detrimental and could be considered as unethical practice.

Ethical practice depends on adequate training and supervision of therapists, and the most immediate limitation of Gestalt or any other therapy is the skill, training, experience, and judgment of the therapist. Proper training in Gestalt therapy involves understanding the theory, hours of supervised practice, observing Gestalt therapists at work, and experiencing one's own personal therapy. Therapists who are trained in the theory and method of Gestalt therapy are likely to do effec-

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tive work. Such therapists have learned to blend a phenomenological and dialogic approach, which is inherently respectful to the client, with well-timed experiments.

WHERE TO GO FROM HERE

In the *DVD for Integrative Counseling: The Case of Ruth and Lecturettes,* Session 7 ("Emotive Focus in Counseling"), I demonstrate how I create experiments to heighten Ruth's awareness. In my version of Gestalt work with Ruth, I watch for cues from Ruth about what she is experiencing in the here and now. By attending to what she is expressing both verbally and nonverbally, I am able to suggest experiments during our sessions. In this particular session I employ a Gestalt experiment, asking Ruth to talk to me as if I were her husband, John. During this experiment, Ruth becomes quite emotional. You will see ways of exploring emotional material and integrating this work into a cognitive framework as well.

Other Resources

Psychotherapy.net is a comprehensive resource for students and professionals that offers videos demonstrating Gestalt therapy with adults and children. New articles, interviews, blogs, therapy cartoons, and videos are published monthly. DVDs relevant to this chapter are available at www.psychotherapy.net and include the following:

Oaklander, V. (2001). *Gestalt Therapy with Children* (Child Therapy with the Experts Series)
Polster, I. (1997). *Psychotherapy With the Unmotivated Patient*

If you are interested in furthering your knowledge and skill in the area of Gestalt therapy, you might consider pursuing Gestalt training, which would include attending workshops, seeking out personal therapy from a Gestalt therapist, and enrolling in a Gestalt training program that would involve reading, practice, and supervision. In 2007 there were approximately 120 active Gestalt institutes in the United States and another 180 in other countries throughout the world. In addition, there are numerous professional associations, and other resources available in nearly every country and language (Woldt, personal communication, January 15, 2007). A comprehensive list of these resources, along with their Websites, is available in the Appendixes of Woldt and Toman's textbook (2005). Some of the most prominent training programs and associations are listed here.

Gestalt Institute of Cleveland. Inc.
1588 Hazel Drive
Cleveland, OH 44106-1791
Telephone: (216) 421-0468
Fax: (216) 421-1729
E-mail: registrar@gestaltcleveland.org
Website: www.gestaltcleveland.org

Pacific Gestalt Institute
1626 Westwood Blvd., Suite 104

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Los Angeles, CA 90024
Telephone: (310) 446-9720
Fax: (310) 475-4704
E-mail: info@gestalttherapy.org
Website: www.gestalttherapy.org

Gestalt Center for Psychotherapy and Training

220 Fifth Avenue, Suite 802
New York, NY 10001
Telephone: (212) 387-9429
E-mail: info@gestaltnyc.org
Website: www.gestaltnyc.org

Gestalt International Study Center

1035 Cemetery Road
South Wellfleet, Cape Cod, MA 02667
Telephone: (508) 349-7900
E-mail: office@gisc.org
Website: www.GISC.org

Gestalt Therapy Training Center Northwest

757 SE 34th Avenue
Portland, OR 97214
Telephone: (503) 230-0900
E-mail: gttcnw@aol.com
Website: www.gttcnw.org

Gestalt Associates Training, Los Angeles

1460 Seventh Street, Suite 300
Santa Monica, CA 90401
Telephone/Fax: (310) 395-6844
E-mail: ritaresnick@gatla.org
Website: www.gatla.org

The most prominent professional associations for Gestalt therapy that hold international conferences follow.

Association for the Advancement of Gestalt Therapy (AAGT)

Website: www.AAGT.org

European Association for Gestalt Therapy (EAGT)

Website: www.EAGT.org

Gestalt Review

Website: www.gestaltreview.com

British Gestalt Journal

Website: www.britishgestaltjournal.com

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Recommended Supplementary Readings

Gestalt Therapy Verbatim (Perls, 1969a) provides a firsthand account of the way Fritz Perls worked. It contains many verbatim transcripts of workshop demonstrations.

Gestalt Therapy: History, Theory, and Practice (Woldt & Toman, 2005) introduces the historical underpinnings and key concepts of Gestalt therapy and features applications of those concepts to therapeutic practice. This is a significant recent publication in the field of Gestalt therapy that contains pedagogical learning activities and experiments, review questions, and photographs of all contributors.

Gestalt Therapy Integrated: Contours of Theory and Practice (Polster & Polster, 1973) is a classic in the field and an excellent source for those who want a more advanced and theoretical treatment of this model.

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*Books and articles marked with an asterisk are suggested for further study.

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TRANSACTIONAL ANALYSIS OF ERIC BERNE

Transactional Analysis of Eric Berne

Historical Background

The major proponent of Transaction Analysis, Eric Berne, was born on May 10, 1910 in Montreal Canada as Eric Leonard Bernstein, he son of Dr David Bernestein a general practitioner. His only sibling, Grace was born 5 years later. After the father's death at the age of 38, Eric's mother encouraged him to follow his father's footsteps and study medicine. He received an MD and Master of Surgery from McGill University Medical School in 1935. Berne interned in the United States at Englewood hospital in New Jersey. Sometimes around 1938, Berne became an American citizen and shortened his name from Eric Leonard Bernstein to Eric Berne. Dr Berne served in the US army medical corps from 1943-1964, rising from first Lieutenant to Major. It was while in he military service that he found group therapy and thereafter stared to develop his own approach, in the early 50's. He first began to use the idea regularly and by 1956, the need for the principles of Transactional Analysis had emerged with sufficient clarity to indicate a more systematized on-going therapeutic programme.

He was a prolific writer and has written many books including, The Mind in Action (1984) A laymen's Guide to psychiatry and psychoanalysis (1957). Transactional Analysis a Psychotherapy (1961). Structure and Dynamics of Organizations and Groups (1963). Games People Play (1964) Principles and Group Treatment (1966) Sex in Human Loving (1970) and what do you say after you say hello (1972).

After his death in 1970, the growth of Transactional Analysis continued. His work has been developed by the many adherents of Transactional Association. Including Claude Steiner, Carmen Kerr and Thomas Harris, among others.

Berne's views on human behaviour were influenced by the Freudian theory, Alfred Alder's notions of life style, Carl Jung's archetype's and wholeness, and Stack Sullivan and some other non-Freudians. However, these influences did not overshadow his originality, imagination and creativity in his conceptualization and descriptions of human behaviour and motivations.

Philosophy and Concept of Human Nature

The Transactional theory of personality is also a theory of life (Berne 1965). According to him, every individual is born with the capacity to develop his potentials to the best advantage of himself and society. To enjoy himself and to be able to work productively and creatively and to be free of psychological disabilities. Transactional Analysis believes that each individual is ultimately responsible for his own life decisions. It is also a basic assumption in Transactional Analysis that all clients need total cure instead of being helped.

Transactional Analysis views men as beings that develop mental attitudes that influence all their transactions.

His mental attitude he called life positions; which is about I and You being Ok or not Ok. However, Transactional Analysis constructs the following classification of life positions in respect of oneself and others.

1. I am not O.k; You are O.k.
2. I am not OK; You are not O.k.
3. I am O.k; You are not Ok,

4. I am O.k; You are O.k. (Harris, 1969).

1. I am not O.k; You are O.k.

The general observations about position are that by the end of the second year of life, or sometimes during the third year, he child has decided on the first life positions. The "I am not O.k; You are O.k.", is the first tentative decision based on the experiences of the first year of life. This is the most common life position, especially in those seeking counselling. It is a depreciating position that sees the self as inadequate and others as adequate. It is a depressive state, compelling people to be disintegrated and deferential to others to procure strokes. Suicide is popular with people in this position who feel so based about themselves in comparison to others that they seek to end their misery through suicide.

2. I am not OK; You are not O.k.

By the end of the second year of life, the first position is either confirmed and settled or it gives way to the second position. "I am not O.k., You are not O.k; This is the position adopted by children who have been rejected by their parents and think that there is something wrong with them, (I am not Ok.) This feeling of personal hopelessness is generalized to everyone. They receive no stroke from their parents and none from their peers, therefore no one is O.k. Depression and despair are common manifestations of this position. The child or individual may stay for the res of his or her life in this position, unless he or she later consciously changes it to third or fourth position.

3. I am O.k; You are not Ok

This position is adopted by self-centered, arrogant people who believed that in this world they are right and others are wrong. No one else can be trusted. It is a paranoid position that distorts the person's objectivity about both self and others. Less extreme form of this position is "do gooders", who see themselves as above others, but are willing to lower themselves to help them.'

4. I am O.k; You are O.k.

This is the healthiest position to hold Unfortunately, it is infrequently adopted. This position maintains basic acceptance of self and others and trusts their growth to be positive. It must be adopted early in life; it would be very difficult to adopt later because the individual's prior learning will blas their life position.(Harris, 1969).

Counselling Goals

According to Eric Berne, the ultimate goal of Transactional Analysis is structural adjustment and re-integration of the individual. Specifically it is meant to:

1. Help the client to identify and decontaminate any ego state that has been distorted.
2. Help the client to be able to use all ego states when appropriate. This means to either build effective ego boundaries or make rigid boundaries more flexible.
3. Help the client to free the Adult ego state towards reality principles that will enable him use reason and to grow productively.
4. Help the client to achieve complementarily with the therapist in the transactional-analysis.
5. Help the client to discard an inappropriate life script and replace it with a productive script.
6. Help the client to develop the capacity to use all their ego stales as situations warrant.
7. Help the client to evaluate and alter an appropriate life position and adopt "I am O.k; you are O.k".

Counselling Process

The knowledge of the theory of and the Language is a necessary perquisite for effective group or individual counselling. The client is thus taught the theory through readings, workshop and instructions.

The theory as a system consists of the behaviour of the client in terms of roles in the transactions with the Therapist as a role partner. The transactional approach exposes or evokes implicit roles and repetition of old adaptive patterns. This leads to complementarities in the client-therapy relationship, resulting in disequilibrium which makes re-equilibrium possible. The old receptive processes arrdisrupted and a new system established.

It is from behaviour with the therapeutic transaction that understanding and change arise, not from insight. Implicit rule are made explicit, by being brought into awareness and control. The behaviour in the two-person transaction underlines progressive changes in relationships with other people in other environment.

However the five stages of counselling are:

1. Structural Analysis.

This is the first stage of counselling. It help client to identify their ego states and to recognize the operation of each state. Work will deal with development of boundaries, contamination, and stabilization so that the adult can take his rightful places of maintaining control. Treatment may not need to go beyond this first step, if the ego is relatively well-develop, and the child and parents are able to respond to the adult, and yet be distinct from him other.

2. Transaction Analysis

The second step of counselling is to understand the transaction one make with others. Contaminated, crossed and complementary transactions are taught by analyzing the clients transaction and those of the group members, if counselling is be conducted in a group. This is a prerequisite for the third step. This is because transaction analysis believe, that anybody can study and learn about one's self can also understand other people in one's life and thus can try to improve on one's relationship with others.

3. Games Analysis.

Engagement in games is analyzed. The counselor must be able to help the client to determine the payoff gained from games. Games are emphasized since they are dishonest transactions and yield no lasting or profitable gain. The objective is to enable the client to stop playing games, and to take the appropriate risk to seek intimacy. Games can be defined as set of transaction undertaken with a concealed motive, which are self- defeating or basically dishonest to the disadvantage of the receiver. They are different from games associated with play and sports. Here games are used figuratively.

4. Script Analysis.

Scripts are series of games that are organized into a recurring theme, such as a series of games which makes the subject a victim and consequently helpless. They are indicated by one's life position. Unless interrupted, scripts will last for time and pass on to one's children. A script analysis serves analysis serves to identify one's script, to recognize its theme and its relationship to the chosen life position. This is a difficult step, since it requires clients to see their

5. Relationship Analysis.

This final step in counselling is used as a home work to help the client to distinguish and clarify the ego state and its transaction. The client may be asked to find another person with

whom complementary transaction may be achieved naturally in a real life situation. When the client become aware of the nature of his implicit role, he takes flight from the transaction. Here ego state consist of three part (the parent ego, the adult ego state and the child ego). We have parent ego, adult ego and child ego where parent ego represents parental figure to blame and praise the child; while adult ego refer to the social reality to the child is subjected; and the child ego refers to the child as he reacts to the external world. The parent the adult and the child do not refer to people but to psychological realities.

Counselling Techniques in Transactional Analysis

1. Interrogation: This is used to obtain information about the client and his problems. Questions are asked about the origin of the problem, the life style of the client and his relationship. Care should be taken not to ask for more information than is needed. Questions should be directed to the client's adult and as such it should be used when the counselor is confident that the client's adult will respond. (Harris, 1969).

Interrogation

1. Counsellor: How are you and your father getting along? Susan: I have been… I have been very good to my father Cousellor: How's your mother?

Susan: we have been closer than we have been, and I have been affectionate to both of them. I have been trying to be some kind of a daughter that they can like now because, I don't know, I am developing a guilt complex about…. I feel that I have been so rotten and everything.

2. **Specification**: This is used to direct the client to what she said and fix them in her mind so that she would not deny what she said or meant.

For example, an adaptation from Harris (1969) Specification.

Counsellor: Why don't you talk about your Parent-Adult-Child?

Susan: Ob, I can't today. I can't use it, I am not using it now. I am dealing with everything totally, Isn't that what you are saying?

Susan: When did I say that to you?

Counselor: Well, that's what you are implying.

3. **Confrontation**: The counsellor use information obtained and specified to point out inconsistencies. This expected to energize the uncontaminated part of the client's adult. If successful, the client responds insightfully. For example an adaptation from Harris (1969).

Confrontation:

Counsellor: So you want to play Den nervous today.

Susan: No, I don't want to play anything. I want a stronger tranquilizer.

Counsellor: You want a stronger tranquilizer to use your Parent Adult Child.

Susan: I've been using it, and I did try, but I am short-tempered and I.

4. **Explanation**: Here, the counsellor explains the inconsistencies to the client. The aim is to decontaminate or re-orient the adult.

5. Illustration: This is "an anecdote, simile, or comparison that follows a successful confrontation for the purpose of reinforcing the confrontation and softening its possible undesirable effects" (Berne, 1966): Illustration should be light, lively or humorous and should be intelligible to the child as well as to the Adult of the patient.

6. **Confirmation:** Here the Counsellor reinforces the material offered by the client to confirm his or her confrontation. The key verb here is reinforcing.
7. **Interpretation:** In Interpretation, the counsellor attempts to decode the past experiences of the client's child which are presented in coded form. He then goes ahead to rectify distortions, and help the client to reorganize his or experiences.

8. **Crystallization:** This is a statement of the client's position 'in factual and straightforward objective terms. The counsellor tells the client his or her potentialities and what he or she can do if he or she stops playing games. He makes it clear to the client that it is within his or her powers to function normally if he or she chooses to do so. The client may attempt to resist this approach to well-being but the resistance must be dealt with (possibly by further confrontation and explanation). However, the client should not be rushed so that, he or she does not become unwell psychologically and develop somatic symptoms.

Criticisms

Strength

Transactional Analysis literature is more interesting to read and easy to understand Teaching the concepts and labels openly to clients makes it easier for them to get better as they are able to rationalize their behaviour according to some system. Moreover, it makes the client a co-counsellor to the counsellor and 'increases his or her level of commitment to change.

With group treatment as its modality, it is relatively economical and saves time.

4. The use of contracting seems to make the client and counselor equals and makes the client assume , responsibility for the outcome of counselling.

5. The absence of rigid techniques gives the Transactional Analyst some measure of flexibility and encourages innovative combinations with other view points.

6. Its use of simple colloquial, vocabulary and labels makes it easy for the lay person to understand.
7. It is an attractive approach and it has broad appeal a; applicability.

Weaknesses

1. Its simplicity and use of popular technology, myths and metaphors open the way for abuses and misuse of this approach.

2. Its lack of attention to specification of techniques has also been seen by some critiques as a limitation.

3. Transactional Analysis has also been criticized for the vocabulary that it employs. These critiques see the vocabulary as agonistic and a hindrance to self-understanding.

Application To The Nigerian Situation

Transactional Analysis is a very practicable and relevant the therapeutic framework which can be of immense use for handling cases of every day experience in Nigeria. The

knowledge or understanding of the theory of Transactional Analysis may be necessary for all well- meaning Nigerians working either in schools or non-school settings.

For instance

1. Many of Nigerian school-going adolescents, have a very poor concept of themselves and others. In order to assist these adolescents, to see their life positions as "I am ok; you are ok, the relationship between teachers and students' students and students, should be always encouraged to be on a complimentary level position.

2. Moreso, the teachers should relate rationally with students in order to encourage the development of adult thinking or rational attitudes among them.

3. Transactional Analysis insists that individuals are responsible for their own basic life decisions, this is applicable for example, to the Igbo adage that says "when a man concurs, his God stands by him" (Onye kwe,chi ya ekwe).

4. Another applicability of the responsibility on the Nigerian individuals for his or her offence is in line with our penal laws which punish an individual for his or her offence. This encourages individuals to try and adopt or change to a productive life style. It is for this reason that Patterson (1973) observed that people feel better when they are able to rationalize their behaviours according to some system or theory.

5. Also this theory encourages the clients to read and understand the concepts and labels for purposes of treatment. An understanding of its concepts will be helpful in improving the parenting styles of Nigerian parents which use more of directive approach. Parents should relate with their children so that the children will view life in the area of I'm ok; you are ok.

Transactional Analysis is seen as a new way of treating emotional disorders without much focus on unconscious mental processes. It is also a theory of life which gives the responsibility for one's life position to the individual. Each individual personality consists of the Parent, the Adult and the Child Ego states. Behaviour is motivated by structure hunger, position hunger and stimulus hunger.

Maladaptive personality results from bad and unhealthy parenting characterized by conditional Strokes and negative and or mixed valence strokes. Bad parenting is also characterized by injunctions, discounts and absence of permission. As a result, an individual adopts any of the three maladaptive life positions and writes his life script to confirm the position. The individual may also develop Ego state problems.

Counselling is preceded by a clearly stated contract, and passes through stages. This approach is not known for its clear specification of techniques, but operational techniques are delineated. In spite of its limitations, it has some potentials for applicability in the Nigerian situation.

Introduction

The subject of this unit is transactional analysis and its use in the context of counselling and therapy. Transactional analysis is a psychoanalytically inspired approach that links problem behaviour to early experience. According to the theory, this early experience exerts considerable influence in the present and is discernible in the 'ego states' each person feels and exhibits at any given time. In the course of this unit, we shall define and discuss these ego states and consider a number of other important concepts that are central to the theory of transactional analysis.

The approach also highlights each person's aptitude and capacity for change. This means, for example, that through awareness and insight the events of early childhood can be redefined by the individual and self-defeating patterns of behaviour altered. These ideas are similar to those expressed in psychodynamic theory, where the emphasis is also on childhood experience and the possibility of change through therapeutic intervention in later life.

The goals of psychodynamic counselling and transactional analysis have much in common, since both aim to help clients bring unconscious material into consciousness. In transactional analysis terms this means helping clients to become more autonomous and 'script'-free. A similar view is implicit in the person-centred approach, where the individual's 'actualising tendency' is seen as a powerful force for improvement and change. However, transactional analysis differs greatly from both the psychodynamic and humanistic approaches, since it stresses the importance of social transactions – a dimension not emphasised by the other two.

KEY TERM

Transactional analysis: This refers to the analysis of a person's communications style or 'transactions', which is carried out with the aid of diagrams so that the specific ego states involved are identified.

Eric Berne (1910–1970)

Eric Berne, who was the founder of transactional analysis, completed his medical training in 1935 and later studied psychiatry at Yale University. In the 1940s he trained as a psychoanalyst at the New York Psychoanalytic Institute, and in 1956 applied for membership there. This request was refused, possibly because of Berne's divergence from, and criticism of, traditional Freudian teaching.

Berne was greatly influenced by his father (also a doctor), who died when Eric was 11 years old. This early bereavement had a lasting effect on Berne and, when he came to formulate his own method of helping the patients in his care, he dedicated his most important work to the memory of his father (Berne, 1961). It is worth mentioning this, because it serves to underline a degree of commitment to patients (or clients), which Berne shared with his father, a man who exemplified dedication to the poor and disadvantaged. Berne demonstrated his own concern for the patients in his care through the design and use of transactional analysis. As a method of communication and a psychotherapeutic tool, it was meant to translate complex ideas and concepts into more accessible and user-friendly language that could be understood by ordinary people. In addition to his work with patients, Eric Berne also wrote several very successful books. These include *Transactional Analysis in Psychotherapy* (1961), *Games People Play* (1964), *The Principles of Group Treatment* (1966) and *What Do You Say After You Say Hello?,* which was published in 1972, after his death. The principles of transactional analysis are expressed very clearly in these books, and should be first choice for anyone wishing to specialise in this approach.

Terminology

Already we can see that the terminology used in transactional analysis is quite different from that used in any other approach. This is both a strength and a weakness of the model, because although the language is memorable and accessible, it is also regarded by some critics as simplistic and superficial. Berne was concerned to demystify the esoteric language of psychotherapy so that it could be grasped by anyone. He was especially interested in helping his patients to understand the basic principles of the approach and the origins of the psychological problems they had come to experience. In this respect, Berne was certainly successful, not least because transactional analysis does appear to help clients and patients to participate more fully in their own therapy. Since its inception, however, Berne's model has been extended considerably by different schools and theorists, and new and more complex ideas have been added to it. While the fundamental principles of transactional analysis remain intact, the accessibility, which Berne valued so much, is often obscured.

Apart from these criticisms, however, transactional analysis continues to gain popularity, not only as a theoretical approach to counselling and therapy, but also as a communications skills model that is used extensively in education, management, industry, health care and many of the caring professions. The International Transactional Analysis Association (ITAA) was formed in 1964 as a training and accreditation body, while the European Association for Transactional Analysis (EATA) performs a similar function. Many of the people who become involved in transactional analysis do so because they are interested in it as a communications model. It is important to remember this fact, since it highlights the point that transactional analysis is first and foremost about the development of effective interpersonal skills.

Groupwork

Transactional analysis therapy usually takes place in a group setting. Berne believed that many problems could be addressed more readily in groups, since such a format lends itself to the identification and analysis of faulty social interactions and communication styles. He had gained a great deal of experience of groupwork in the 1940s, while working as a psychiatrist in the United States Army Corps, and he set up the first transactional analysis group in 1954. Invaluable information about the way people relate to each other is readily available when clients work together in groups. Each participant is afforded the opportunity to monitor and perhaps change the interactive styles they habitually use. As we shall see, Berne formulated his user-friendly theory of personality in a way that seems to owe something to Freud's concept of Id, Ego and Superego, although Berne did point out that his concept of personality was different in the sense that his ego states represent 'phenomological realities' (Berne, 1961: 31). In simple terms, this means that the three ego states are real and observable, whereas the parts of personality Freud described were not always clearly seen in this way. Group participants can, therefore, learn about personal styles of social interaction and any problems associated with these, while at the same time becoming aware of the intent behind their ways of relating to others.

Ego states

An idea fundamental to transactional analysis is that of ego states. Berne suggested that human personality is made up of three elements, which he referred to as Parent, Adult and Child. These familiar words were used by him to describe states of 'self' or states of mind, which he believed gave rise to their own individual patterns of behaviour. In transactional analysis, 'Parent', 'Adult' and 'Child' are always spelt with capital letters, in order to distinguish between their usual meaning and that which Berne assigned to them. Ego states are, of course, common to all of us and govern our thoughts, feelings and behaviour. In any given situation an individual will exhibit a certain pattern of behaviour that corresponds to the way that person is actually feeling at the time. As feelings change, so also do the patterns of behaviour the person displays. As a result of his experience with groups, Berne observed that these patterns included noticeable changes in voice, posture, vocabulary and all other aspects of behaviour (Berne, 1964: 23). The principles of transactional analysis are illustrated in diagrammatic form, the most basic of which is the structural diagram illustrated in Figure 7.1.

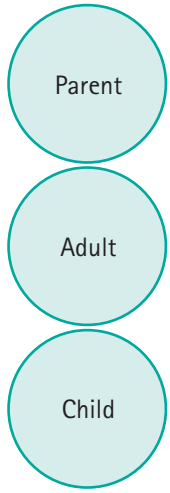


Figure 7.1 Structural diagram of personality

Each of these states, Parent, Adult and Child, is present from early childhood, and all are described by Berne as 'psychological realities' (Berne, 1964: 23). In Figure 7.1 the ego states are separated from each other because they are incompatible and differ considerably.

Parent ego state

The Parent ego state represents a set of thoughts, feelings and behaviour, which are derived from parental figures. According to Berne (1964) the Parent ego state is exhibited in both indirect and direct form. A person may, for example, respond as one of his parents actually responded in a given situation. When this occurs, the ego state is directly shown. When the parental influence is an indirect one, then the person is likely to respond as his parents would have wished him to respond. A distinction between these two forms is important, and explains how people:

- can sometimes 'become' one of their own parents when the Parent ego state is directly active.
- exhibit the kind of response which parents required in the past.

A client called Viv recounted the following experience.

CASE STUDY

Parent ego state

Sometimes I feel that the children are driving me crazy. Most of the time I cope quite well, and my husband is supportive and helpful. At other times, especially when I have just got in from work, I simply don't know where to start. Then I find myself getting into a panic and I start to shout at them. When this happened to me yesterday I suddenly realised that I sounded exactly like my mother. It wasn't just the tone of my voice . . . it was the words as well. I actually used the word 'weary' as she used to do, and later on I used another expression of hers as well. I'm sure I looked like her too, standing there with my hands on my hips glaring at the children. It pulled me up short and made me really think . . . do I want to become old before my time and end up hassled and bad tempered, the way she always seemed to be?

Comment: In the example just given, the client described a situation in which she felt she had 'become' her own mother in her response to her children. Her actions and her state of mind were the same as her mother's used to be in a similar situation. It is important to point out here that the word 'Parent' refers to parental substitutes as well as to actual parents, and could include, for example, teachers and others who exerted influence on a person's early life.

Do as I say: indirect Parent

The second category of Parent Berne identifies actually operates in conjunction with a person's Child ego state. This is, in effect, a dialogue between ego states, and when a person responds in this way, the parental 'influence' is evident. Another way of saying this is that the response shown is one which clearly stems from the instruction 'this is how one should behave' or 'do as I say'. This

adaptation to parental influence is evident in the 'adapted Child' ego state, which we shall consider later in this section.

Aspects of socialisation

Whether shown in direct or indirect form, the Parent ego state resembles a compendium of the entire socialisation process each person has received in early life. This early socialisation is passed from parents, teachers and other significant people in childhood, and is usually absorbed without question by the growing child. However, it is not just parental pronouncements and injunctions that are recorded in this way; the example given by parents and other significant people is also relevant here. Both negative and positive influences are recorded in the Parent ego state, and everything that is experienced by the child is internalised to become part of the personality. Once again Berne's psychoanalytic training is evident in his theory, since the idea of parental internalisation is common to all branches of psychodynamic teaching, including object relations theory.

There is an important point of difference, however, between Berne's theory of personality and those described by the various branches of psychodynamic theory. This difference concerns the 'paternal' influence, which, in Berne's formulation, is potentially just as influential as the 'maternal'. If we consider the theories discussed in Units 3 and 4, it is clear that such an equal emphasis is absent from many of the psychodynamic approaches. In transactional analysis, therefore, both parents are believed to impart information explicitly and implicitly to the developing child's Parent ego state.

CASE STUDY

Paternal influence

During bereavement counselling, a client called Kavita remembered her experiences in childhood and her relationship with her father, which she described as follows:

My mother was talkative and outgoing. She was the one who seemed to have an opinion about everything. I can see many aspects of my mother in myself, but I can identify characteristics of my father too. When he came to this country he concentrated on working hard, setting up a business and working virtually non-stop from early morning until late at night. I only really started to think about him after he died, and to appreciate all that he did for the family. Though I can't remember him directly influencing me when I was a child, I know now that his unspoken influence was immense. I have absorbed his attitude to work and to family, and I am very grateful now for the many positive messages he conveyed to me.

Information from the past

Information which is recorded in the Parent ego state enables us to cope with all aspects of living. It also provides all the data necessary to enable people to function as parents themselves and raise their own children.

Another significant aspect of the Parent ego state is that it enables people to respond automatically in many situations, so that many 'routine matters', as Berne describes them, can be dealt with without unnecessary expenditure of energy (Berne, 1964: 27). Problems arise, though, when the Parent ego state is inappropriate or counter-productive in a given situation. One example of this is the bank cashier who adopts a certain 'superior' stance in relation to the account holders she is meant to serve. Her critical parental attitude is likely to stem from a childhood in which attitudes to money (and especially those who borrow it) are fraught with ambiguity and moral disapproval. In the present situation, however, the bank cashier's Parent ego state is liable to cause problems, since it is sure to encourage a 'Child' response from the customer and an end to real, productive communication.

Berne (1972: 75) elaborated on his structural diagram of personality, so that the Parent ego state is now commonly divided into 'Nurturing Parent' and 'Critical' or 'Controlling' Parent – see Figure 7.2. This diagram also illustrates the subdivision of the Child ego state into 'Free' or 'Rebellious' Child, and 'Adapted' Child.

EXERCISE

Parent ego state

Working individually, try to identify as many situations as possible where you respond in a Parent ego state. How do you think, feel and act when you respond in this way? Think carefully about your tone of voice, your gestures, your facial expressions and any other observable features that are manifest in your Parent ego state.

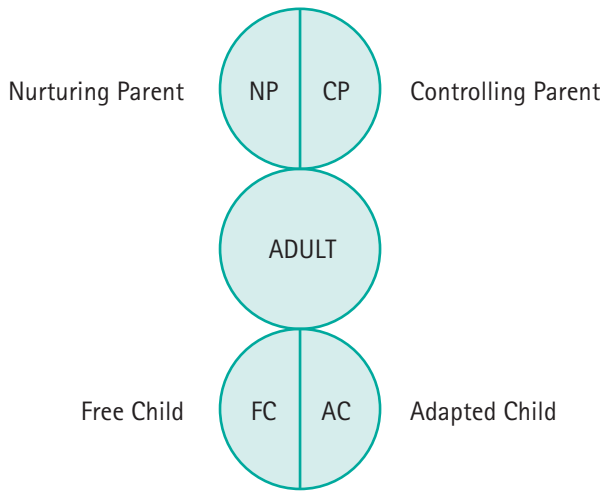


Figure 7.2 Descriptive diagram of personality

Nurturing Parent and Controlling Parent

The terms 'Nurturing Parent' and 'Controlling Parent' refer to two different sets of influence, which are absorbed in early childhood and are clearly distinguishable from each other when they are manifest. Berne originally used the word 'prejudicial' instead of 'controlling' to describe the second form, in which the Parent ego state is exhibited (Berne, 1961: 48). However, both these words, 'Controlling' and 'Prejudicial', are meant to denote arbitrary and prohibitive attitudes that are borrowed from parental figures and are usually exhibited automatically and without question in certain situations. The example of the bank cashier is a case in point here, and serves to illustrate the way in which some people respond to particular situations exactly as a parent or parents would have done. The 'Nurturing Parent' ego state is shown in a completely different way, and is usually manifest as sympathy or support for others in need or distress. It is important to remember that children are also capable of responding in all three ego states, and when they are in the 'Nurturing Parent' state it tends to be demonstrated as concern for someone or something smaller, weaker and more vulnerable, including toys and dolls.

EXERCISE

Nurturing Parent and Controlling Parent

Working individually, identify any situations where other people responded to you as either 'Controlling Parent' or 'Nurturing Parent'. How did these people sound and look when they responded to you in these two ego states? How did each style of communication affect you, and how did you respond to each?

Child ego state

While information is being recorded in the Parent ego state, another recording is taking place simultaneously. This second recording also derives from the past and represents a young child's actual reactions to what is going on in the environment. Later on, in adult life, the Child ego state is evident when the emotions that the original situation produced are felt once again. Harris (1973) refers to this response, making the point that a child has no vocabulary in the very early years, so experiences are recorded as feelings. These feelings can be evoked at any time in adult life, and often the individual concerned is unaware that the response is an archaic one. The following is an example of this.

CASE STUDY

Child ego state

Gwyneth, who was in her early 40s, received counselling because she was unable to cope at work. Her problems arose when she had to deal with complaints from staff or customers, or when any situation arose in which there was conflict. When things became difficult, Gwyneth felt angry and defensive, or extremely tearful. She was aware that her responses were inappropriate, but was unable to modify the strong feelings certain people and situations evoked. In the course of counselling, she revealed that her parents had been habitually critical so that she reached a point in her childhood where she despaired of ever pleasing them. Her transferential responses to staff and customers were an exact replica of the way she responded to parental criticism in the past. When she understood this, Gwyneth was in a position to alter her responses and access her Adult ego state when conflict or disagreement arose. This was not something she achieved immediately, however. Understanding and practice are necessary before long-established ways of responding are changed in this way. Gwyneth's counsellor taught her the basic principles of transactional analysis and showed her how 'complementary' and 'crossed' transactions work in practice (see also Figures 7.3, 7.4 and 7.5).

Adapted Child and Free Child

We have seen that the Child ego state is preserved from childhood so that a person functioning in this way behaves as she or he did at a very early age. Like the Parent ego state, the Child is demonstrated in speech, stance, demeanour and overall appearance. In addition, Steiner (1974) suggests that these outward signs are accompanied by feelings and thoughts that correspond to the visible indicators. In referring to the Child ego state, Berne stressed that it was not intended to mean 'childish' or 'immature' (Berne, 1964: 24). Berne was aware of the danger that his terminology could be misinterpreted and used in a disparaging way. In his early writings he uses the terms 'Adapted' and 'Natural' Child to describe the two broad manifestations of that particular ego state. The word 'Free' is now often substituted for 'Natural'.

Adapted Child

In the section entitled 'Do as I say' we looked briefly at how people adapt to parental influence, and the way in which this adaptation is revealed through the Child ego state in adult life.

The person who is functioning in Adapted Child will demonstrate behaviour meant to conform to the expectations of parents and parental figures. Different parents have different expectations, however, and behaviours that please one will not necessarily please the other. This means that people learn to respond or adapt to both parents; and in later life, these responses will be triggered in situations that are reminiscent of earlier ones. A young woman who has received poor grades in college assignments, for

example, may respond to a male lecturer by becoming silent and withdrawn, since that was the way she responded to her father's disapproval when she was a child. A woman teacher may, on the other hand, evoke a different, perhaps more combative response, which stems from the relationship the student had with her mother. The point to make here is that both these 'adapted' responses are outmoded and archaic, because in each case the student is communicating in the way that she would have done at a much earlier time with the important people in her life. The Adult ego state, which we shall consider later, represents a much more effective way of communicating.

Free Child

The Free Child ego state is the source of strong feelings and can be classified as either positive or negative. Spontaneous expressions of joy, anger, sorrow and a sense of fun are characteristic of Free Child. These are all feelings which have escaped the inhibiting influence of parents, but when they are expressed inappropriately these uncensored impulses can be problematic. On the other hand, positive Free Child ego states are often observed in social situations or contexts where such expressions are permitted. Steiner (1974) lists sports events and parties as examples of structured situations, in which the Free Child ego state is given unlimited opportunity for expression. Apart from aspects of behaviour that can be identified with Free Child, there are also words and phrases like 'super' and 'wow', for example, which are associated with it.

Adult ego state

The Adult ego state denotes the feelings, attitudes and behaviour patterns which belong exclusively to the individual, and which are in touch with current reality and relationships in the present. Once again, it is possible to detect similarities between Berne's description of personality and aspects of psychodynamic theory, since the Adult ego state appears to have much in common with the Freudian concept of the 'Ego'. Like the Ego, the Adult ego state represents reason and common sense. It is that part of the personality which can truly be described as autonomous and free from parental influence, or strong feelings emanating from the Child ego state. The Adult ego state is characterised by objective, logical thinking, and an ability to make independent judgments and decisions in any situation. Experiences gained throughout life are examined and used by the individual when in Adult, so that realistic choices can be freely made. Every so often the material stored in the Adult ego state is reassessed and updated, in order to keep in touch with changing circumstances and needs. Children, too, have an Adult ego state, which represents that part of personality which has absorbed and tested information passed to them from parents and other significant people. The central concern for the Adult ego state is the development of individual autonomy and the ability to make informed and realistic decisions on one's own.

EXERCISE

Looking at ego states

Working individually, look at the responses given to the situations described. Say whether you think these responses come from each person's Parent, Adult or Child ego states.

1. Helen has been asked by her manager to work extra hours at the weekend. She asks for some time to consider the situation. Next day, she replies that she has given it some thought and is willing to do the overtime.
2. Bruce has been admitted to hospital for an operation. He is approached by a doctor who wishes to carry out some diagnostic procedures which Bruce does not understand. Instead of asking, however, he keeps silent and worries for the rest of the day.
3. Catherine has just learned that she has not got the promotion she hoped for. She has a confrontation with her boss and finds she becomes angry and tearful.
4. Joanne and Lyn are friends and attend the same university. Joanne has a row with her boyfriend and Lyn responds in the following way:
   'He's not worth it. I never thought he was good enough for you. To be quite honest, I thought you were wasting your time.'
5. Phil and Simon are friends who share the same office at work. Phil's girlfriend breaks off their relationship and Simon responds in the following way:
   'Look, I know you must be feeling rotten. If you like, I'll clear up those files for you and you could leave a bit early.'
6. Karen's husband asks her to consider changing their next year's holiday plans. They had planned a sightseeing holiday, including visits to the local art galleries and museums. He suggests they need a more relaxing break. Karen responds by pointing out that they had both resolved to improve their knowledge of art and, anyway, she has better things to do than waste time on the beach.
7. Mrs Rae is an 80-year-old resident in a nursing home. One of the attendants asks her if she would like to go out for a walk, since the day is warm and sunny. Mrs Rae is enthusiastic about the suggestion, and adds that she wouldn't mind paddling at the local beach as well.
8. James, who is eight years old, has been told by his parents that he must not play in the sun without wearing sunscreen lotion. He replies that he knows not to do this, because he got burned on holiday last year.
9. Carolyn's mother has asked her to visit home more often. Carolyn, who is 20 and shares an apartment with some friends, replies that her mother needs to get out and about more so that she can have a life of her own.
10. The manager of a small firm is concerned that her secretary is repeatedly late for work. The manager, Mrs Burrows, waits until her irritation is under control, then makes an appointment to speak to her secretary. She expresses her concern and asks for an explanation.

EXERCISE Cont...

Answers

It should be stressed that it is not possible to be totally accurate about the correct answers to this questionnaire. This is because so much depends on the manner and tone of voice of the person responding in each case. However, from the evidence given, the answers are shown at the end of this unit.

Application to counselling

Berne's main objective in formulating transactional analysis was that it should provide a frame of reference for helping clients in therapy. He developed his theories as a result of working directly with patients, whose behavioural changes he observed and then identified in the ego state model. The behavioural changes he noted included alterations in voice tone, gesture, choice of words, facial expression, posture, body movement and even sentence construction. Berne was intrigued by this phenomenon, and observed that people seemed to be governed by different 'inner' personalities, whose influences were manifest at different times and according to circumstances. He also observed that people communicated in a variety of ways, depending on the ego state which dominated the personality at any given time. These 'transactions', as he called them, could be analysed in order to help people identify some of the problems they experienced in communicating with others (Berne, 1964: 28). Transactional analysis, therefore, is meant to help clients gain intellectual insight through analysis of the way they relate to other people. It is concerned with four major areas of analysis:

1. Structural analysis: the analysis of individual personality or ego states
2. Transactional analysis: the analysis of communication styles or social behaviour
3. Game analysis: analysis of the psychological games people play
4. Script analysis: analysis of a complex set of transactions people act out compulsively.

KEY TERMS

Script analysis: This is based on the idea that everyone has a life plan or 'script', which determines behaviour and life choices. Analysis of scripts brings these, largely unconscious, motivations into conscious awareness.

Structural analysis: This refers to a theory of personality based on the study of specific ego states. These ego states are: Adult, Parent and Child.

Transaction: This refers to communication between two or more people in any social situation. This communication can be verbal or non-verbal. Berne used the term 'strokes' to denote the exchanges that people engage in socially (Berne, 1964:14).

Structural analysis

We have considered Berne's structure of personality, which was shown in diagram form (see Figure 7.1). Berne believed that structural analysis should always come before transactional analysis, and can be taught to clients in either group or individual settings (Berne, 1961). It is important for clients to understand the ego state model before they can move on to consider the way they conduct transactions with other people.

Transactions

While structural analysis refers to the individual, analysis of transactions refers to social behaviour. The word 'stroking' is used by Berne to describe the social exchanges that take place between people. Stroking implies recognition of another person and is defined by Berne as 'the fundamental unit of social action' (Berne, 1964:15). Any exchange of strokes is called a 'transaction' and is the basic unit of social interaction. We exchange strokes through verbal greetings, non-verbal recognition, touching, kissing, hugging, and so on. According to Berne, strokes are a necessary and integral part of human interaction, and we all need them even when they are negative. Children will, for example, seek recognition from parental or other important figures; and, when positive recognition is not forthcoming, negative strokes are seen as preferable to none at all. When patterns like this are established in early life, it may take some time for people to change them. Transactional analysis counselling is one way in which clients can learn to change the way they communicate with, and seek approval and recognition from, other people.

Complementary transactions

People communicate with others from the Parent, Adult or Child ego state. The person making the response will also do so from one of the three ego states. A complementary transaction will take place when, for example, one person addresses another from the Adult ego state, and the second person replies from Adult. This is illustrated the following example. (See also Figure 7.3.)

STIMULUS: Have you seen my glasses?
(Adult to Adult)

RESPONSE: Yes, they're on the dining room table.
(Adult to Adult)

Berne illustrated his theory of transactional analysis in diagram form, in order to make it easier to understand. He also stressed the point that when transactions remain complementary, communication proceeds indefinitely. Complementary transactions don't just take place between Adult to Adult ego states. Berne (1972: 14) identifies nine possible types of complementary transactions, in which the responses given were those which the question or stimulus intended. Figure 7.4 is an example of one.

STIMULUS: Isn't it dreadful how long it takes to get through these checkouts?
(Parent to Parent)

RESPONSE: Yes, it's disgraceful. They should have more staff.
(Parent to Parent)

The following is another example of a complementary transaction, this time Parent to Child.

STIMULUS: That's twice this week you've kept me waiting.
(Parent to Child)

RESPONSE: I'm sorry to be such a nuisance.
(Child to Parent)

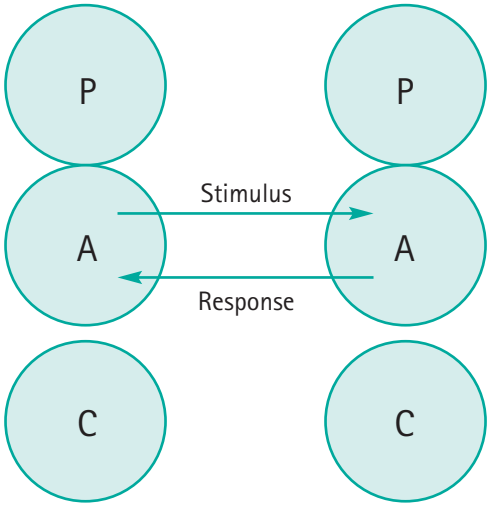


Figure 7.3 Complementary transaction

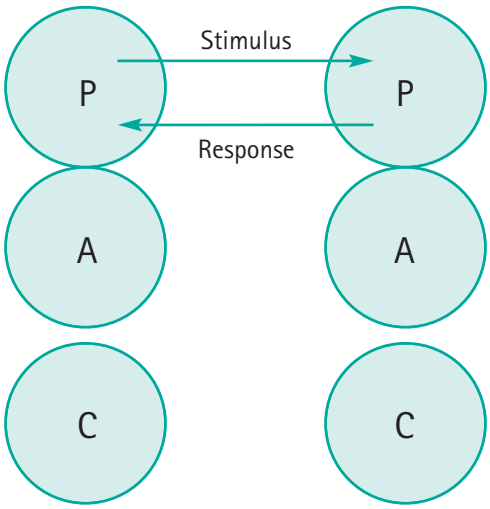


Figure 7.4 Complementary transaction

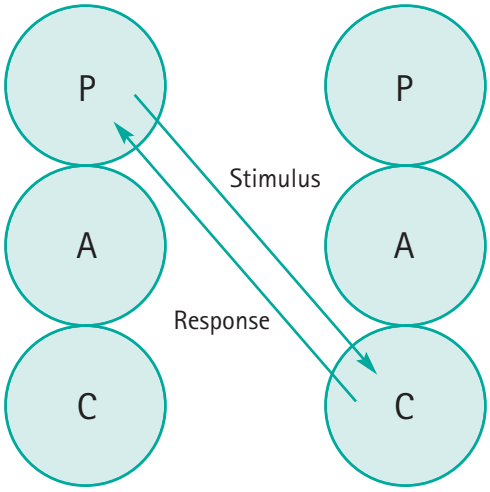


Figure 7.5 Complementary transaction

Crossed transactions

A crossed transaction occurs when the response given is not the one which the stimulus intended. When such a situation arises, communication tends to break down. Figure 7.6 illustrates this.

STIMULUS: Have you seen my glasses?
(Adult to Adult)

RESPONSE: Oh you haven't lost them again!
(Parent to Child)

The response to the question 'Have you seen my glasses' might have been given from a Child ego state as follows:

STIMULUS: Have you seen my glasses?
(Adult to Adult)

RESPONSE: You can't expect me to look after your things.
(Child to Parent)

Once again this is illustrated in diagrammatic form (see Figure 7.7).

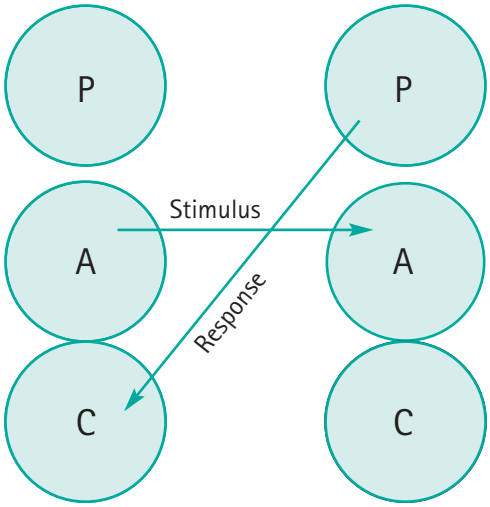


Figure 7.6 Crossed transaction

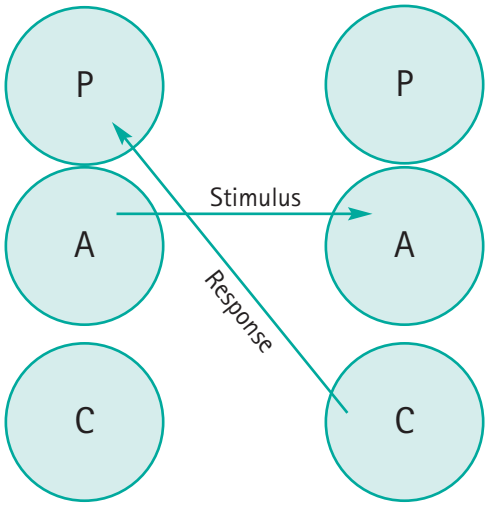


Figure 7.7 Crossed transaction

Ulterior transactions

Both complementary and crossed transactions are relatively simple, and operate on one level. However, Berne describes another, more complicated, transaction that operates on two levels and conveys a double set of messages. These are referred to as 'ulterior' transactions because they contain a hidden agenda. Another way of stating this is to say that one of the messages occurs at a psychological level, while the other takes place at a social level. The psychological aspect of the transaction is conveyed subtly, and is often picked up via non-verbal clues. It is by far the stronger of the two messages and dominates the social message, which is conveyed verbally. Ulterior transactions are frequently contained in exchanges between people who wish to become

more intimate and are wary of stating their intentions openly. However, they are also common in many other situations. The following is an example:

PATIENT: My back is less painful, although it does tend to play up when I'm sitting at my desk.
(Adult to Adult)

DOCTOR: Certain situations make it worse for you.
(Adult to Adult)

On the surface this is a straightforward transaction between two people who are discussing a medical condition. At a psychological level, however, the transaction is likely to be more complex, with a subtle testing of responses taking place on both sides. Depending on voice tone, body language and other non-verbal clues, the ulterior message may be as follows:

PATIENT: I'm not ready to go back to work and I need your help.
(Child to Parent)

DOCTOR: I'll take care of you and give you extra time.
(Parent to Child)

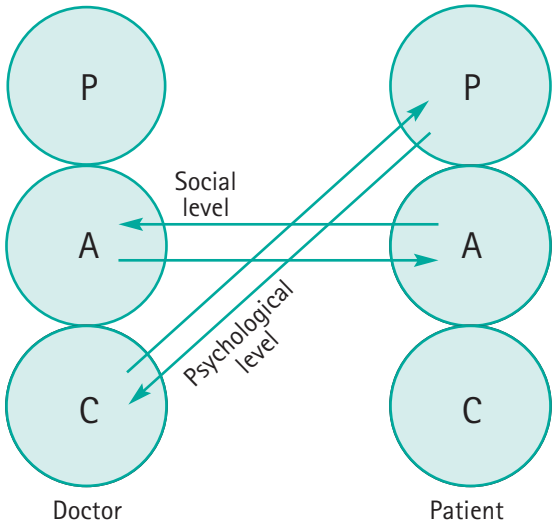


Figure 7.8 Ulterior transaction

EXERCISE

Ulterior transactions

Working individually, identify any ulterior transactions you have experienced either at work or at home. Illustrate these in diagram form and discuss the way they work with other members of the training group. Are there some transactions that are common to a number of people in the group?

Game analysis

Berne (1964) defines the psychological games people play as follows:

- They are extensions of ulterior transactions.
- They progress to an expected and definite outcome.
- Superficially, they appear to be straightforward transactions.
- What is said is not what is felt.
- All games have some common elements, including a hidden agenda and a negative payoff.

Berne also makes a useful distinction between an 'operation' – a simple transaction or transactions set in motion for a specific purpose – and a game – a dishonest 'manoeuvre' (Berne, 1964: 44). If someone asks for help and gets it, for example, then that is an operation. If, on the other hand, the person who gives the help is adversely affected in doing so, then that is a game. The following case study is an example of a game.

CASE STUDY

Mr Phillips

Mr Phillips was a 36-year-old man who had been out of work for several years. He had a number of minor health problems, which meant that he had visited his doctor on many occasions. However, his GP had some difficulty in persuading Mr Phillips that he had no serious health problems, an assessment confirmed by the many diagnostic procedures that had been carried out. Mr Phillips had also visited several alternative practitioners, all of whom he described as 'hopeless' and incapable of giving him the help he needed. At a later date, he requested counselling, and during his first sessions informed his counsellor that no one had been able to help him so far. The counsellor observed that Mr Phillips smiled as he described all the failed attempts to understand and assist him. Nevertheless, she worked with him over a period of ten weeks and, during that time, was able to help him identify the game he was playing over and over again, and the payoff he received on each occasion.

Comment: We can see that Mr Phillips was receiving a great deal of attention as a result of his repeated visits to helpers. Gaining attention in this way was a continuation of the strategy he had employed as a small child. However, the kind of attention this client received effectively prevented him from achieving any real intimacy in relation to other people. In transactional analysis terms, games are designed to confirm and support the original decisions people make in early life about the best way to relate to others. As far as Mr Phillips was concerned, real intimacy was fraught with danger. This was a lesson he had learned in childhood, at the hands of neglectful and often abusive parents. In response to these early events, he had devised his own method of gaining the 'strokes' or recognition he desperately needed. As a result, he was also able to experience the familiar, and what Berne (Harris, 1972: 137–139) referred to as 'racket', feelings of hurt and satisfaction associated with his own special game. The fact that he was sabotaging his own best interests did not occur to him. In the units dealing with psychodynamic approaches to counselling, we noted that Freud (and other Freudian theorists) believed that people frequently repeat early patterns of behaviour in an unconscious and repetitive way. This idea is taken up by Berne and is integral to his theory of games and life scripts.

Scripts

The theory of life scripts has a central place in transactional analysis. Berne refers to life scripts as 'complex operations' and goes on to describe a script as 'an extensive unconscious life plan' to which people construct their activities, lives and relationships (Berne, 1961: 23). According to this theory, fundamental decisions about an individual's entire lifespan are formulated at a very early age. Scripts are based on parental conditioning and influence, and they come under two headings: 'Winning' and 'Losing' (Berne, 1972: 203–5). These scripts are closely associated with another transactional analysis concept, the OK positions, described by Harris (1973). OK positions represent a variety of convictions people adopt and adhere to from childhood through life. There are four basic OK positions, from which games and scripts originate. These are as follows:

1 I'm not OK – You're OK.
2 I'm not OK – You're not OK.
3 I'm OK – You're not OK.
4 I'm OK – You're OK.

(Harris, 1973: 42)

What the positions mean

The fourth position is the most positive one; it is based on the conviction that everyone has worth and value. An infant who is loved and cared for is likely to imbibe feelings of goodness which, if the quality of care continues, will last a lifetime. Feelings of goodness about 'self' are then translated into feelings of goodness about other people too. Once again it is possible to detect echoes of psychodynamic theory – in particular, object relations theory – in Berne's ideas. Unfortunately, problems tend to arise in later childhood when the parents and others become more critical and demanding. When this happens, the I'm OK – You're OK position of an earlier era may be questioned, and the original conviction of worth may then be lost.

Some fortunate people retain the I'm OK – You're OK position throughout life, but the majority of others probably fall into the depressive I'm not OK – You're OK group. This is because so much learning has to be done in childhood, and so many obstacles have to be overcome. Parents often give negative feedback and, in extreme cases, they may even be abusive or neglectful.

When this last situation obtains, some children respond by adopting the I'm not OK – You're not OK 'futility' position. Another group of people, those who are arrogant and distrustful of others, will slot into the I'm OK – You're not OK 'paranoid' position. An important point to make in relation to OK positions is that it is possible for all of us to move from one to the other at different stages of life, depending on changing circumstances.

However, a fundamental I'm OK – You're OK position that is established in childhood is never totally obscured, even in the most adverse conditions of life.

Focus on counselling

Achieving the I'm OK – You're OK position often requires a conscious decision (and some effort) in adult life. People who do achieve it tend to respect themselves, and all people with whom they are in contact. In his writing, Berne (1961) makes it quite clear that personal change cannot simply be effected by external circumstances, but must come from within the individual.

An important objective in counselling is to help clients identify and use their own resources so that therapeutic change becomes possible. In transactional analysis people are viewed in a positive way; a basic principle of the approach is that people can be helped to locate their innate (though often neglected) coping abilities. Clients are encouraged to make new decisions about their lives, and alternative ways of behaving are explored in therapy. The OK positions adopted in early childhood can be challenged and changed, and negative life scripts can be reconstructed to become more positive.

Earlier in this unit, we noted that transactional analysis is frequently conducted in groups. The group is, in fact, the ideal medium for clients who wish to become more aware about their personal life scripts and how they originated. Understanding must always precede change, and in transactional analysis groupwork, clients receive feedback from other participants. This practice is of considerable help in increasing self-knowledge for individual clients. The following is a summary of objectives in transactional analysis therapy. Clients:

- should become more autonomous and 'script free'
- work towards the I'm OK – You're OK life position
- learn to state their needs and views clearly without game playing
- take responsibility for their own feelings.

EXERCISE

Life positions

Working either individually or as a group, look at the four life positions listed below and identify any situations in your own life when you experienced each of them. What were the circumstances in each case, and how did you resolve any difficulties you had?

1. I'm not OK – You're OK
2. I'm not OK – You're not OK
3. I'm OK – You're not OK
4. I'm OK – You're OK

Contracts and goals

In transactional analysis, client and counsellor establish a specific and detailed contract, which must be agreed by both. All administrative and business aspects of the counsellor–client relationship are included in a transactional analysis contract, but a much wider agreement on the goals of therapy, and the way these will be achieved is included too. Goals are always stated in positive terms and revised and updated when necessary. This means that attention is directed towards a wide view of personal development and away from a superficial problem-solving approach. However, this does not imply that therapy should always be lengthy. Clients state the specific beliefs and behaviours they wish to change and both client and counsellor work towards these objectives. This joint approach serves to strengthen the equality of the relationship, and encourages clients to experience themselves as active participants in their own therapy. In 1971 Franklin Ernst, a behavioural transactional analysis practitioner, suggested that contracts should be agreed at the end of therapy, as well as the beginning. This end of therapy was meant to solidify the clients' commitment to change. In fact, contracts can be renegotiated at any stage throughout transactional analysis counselling.

The relationship

The relationship itself is based on the concept of equality between the two people involved, and all information, including any notes taken, is shared openly. Clients in transactional analysis counselling are helped to achieve emotional and intellectual insight, but the primary focus of therapy is certainly cognitive. This refers to the understanding we have already mentioned, as a necessary component in the progress towards autonomy and script-free independence. However, creativity and flexibility are further attributes that transactional analysis counsellors need to have.

Clients are also encouraged to become more aware of all aspects of everyday life, and to live with spontaneity in the present. Spontaneous living in the here and now implies freedom from outdated scripts, as well as an ability to choose appropriate responses in personal and social situations. The ability to form relationships without resorting to games or subterfuge is another desired outcome in counselling. Communication between client and counsellor should be clear, and this clarity is facilitated when clients are familiar with the theoretical framework of transactional analysis and its terminology. Clients are encouraged to learn about transactional analysis and to attend courses, if possible. When therapy is taking place in a group context, its aims and objectives are discussed so that all participants are clear about purpose and progress.

Attitudes about the relationship between counsellor and client have changed considerably since Berne first introduced his transactional analysis model of helping. These changes are discussed later in this unit under the heading of 'How transactional analysis has evolved' on page 229.

Permission

In the transactional analysis model of counselling, the concept of 'permissions' has an important place, although, as Stewart (1996) points out, some transactional analysis counsellors now tend to de-emphasise this. The word 'permission' refers to the counsellor's role in encouraging clients to abandon unhealthy or destructive behaviours, in favour of more positive and life-affirming action. Giving permission may take the form of simply telling clients that they need not continue with certain types of behaviour that stem from childhood beliefs still operating at an unconscious level. Many of these beliefs have been passed on from parents and become absorbed into the client's own Parent ego state. The rationale underlining the concept of permission is that it is possible for the counsellor or therapist to switch off the parental recording that is responsible for the client's problems. However, there is another much more important reason for helping clients to become free of negative parental messages. Clients need to learn to trust their own judgment before they can experience true autonomy and freedom from outdated scripts. The following case study highlights the therapeutic effects of permission in counselling.

CASE STUDY

Shelly

Shelly was a middle-aged client, whose mother had died two years before she started counselling. She was now living alone and complained of an inability to enjoy herself, which she described in the following way:

I make plans to go out with Freda [her best friend] and I'm really looking forward to it. Then when I get to the cinema or the restaurant, a black cloud seems to descend on me and something inside me says that I should not enjoy myself. After that I start to feel guilty that I am out on the town, when so many other people can't afford to socialise in that way. I think something bad will happen to me if I enjoy myself.

Comment: The counsellor who helped this client agreed and established a specific contract with her. Background details about Shelly's lifestyle and general health were also discussed, and after this a treatment contract was devised. Shelly stated what it was she wished to achieve through counselling, and later on goals were agreed between client and counsellor. The treatment contract was outlined in positive terms, and Shelly stated that she wished to become more confident about going out. She also wished to be free of the guilt feelings that had plagued her for so long. Her parents had been very religious, and the idea that enjoyment was sinful was accepted in the family when Shelly was a child. These strong messages were stored, therefore, in her Parent ego state, and she was unable to dislodge them without someone to encourage her. The counsellor told Shelly that she need not feel guilty on the next social outing. In order to reinforce this permission, she asked Shelly to picture their present conversation each time pangs of guilt assailed her. This worked very well for the client, who then learned to give herself permission when she needed it. Eventually, even this conscious self-permission was no longer necessary, as Shelly started to feel more independent and free to think for herself.

EXERCISE

Inherited beliefs

Working in groups of three to four, identify any strongly held beliefs (either positive or negative) that you have inherited from your families of origin. Discuss how these beliefs affect you now, and identify any that you now consider to be outdated.

Potency and protection

Offering permission to clients, in the way just described, can only be done when the other important conditions of potency and protection are also present. The word 'potency' refers to the counsellor's strength or conviction, which must be felt by the client if parental injunctions and outdated rules are to be challenged. In other words, the counsellor must, as Berne describes it, feel sufficiently potent to deal with the client's 'Parent'. Furthermore, the client's 'Child' must be convinced that the counsellor is potent enough to give 'protection' against parental anger (Berne, 1972: 371–6). Protection can often take the form of assurance from the counsellor that she is available if the client should need her. This highlights an aspect of transactional analysis that sets it apart from many other models, for although there is an emphasis on client–counsellor equality, there is also recognition that clients frequently need strong support and protection at certain stages of therapy. This means that counsellors need to be aware of their own strength and protective position within the relationship, as well as the need to be directive when necessary. The client is there to receive help, and the counsellor is there to give it. However, clients are not encouraged to become dependent in transactional analysis therapy, and the emphasis throughout is on helping the client to get well and become more autonomous.

Transference

We have considered the concept of transference in the units dealing with psychodynamic approaches to counselling, and it is discussed in Unit 9, alongside other important issues in counselling. Individual approaches to therapy have their own views about transference, but all are aware of its existence, or potential existence, in the counselling relationship. Transactional analysis is no exception. Indeed, Berne (1972) believed that it was a common cause of many problems, both inside and outside therapy. In the context of transactional analysis, the most effective way of illustrating transference (and countertransference) is in diagram form. When clients transfer to counsellors their feelings and attitudes stemming from childhood, then the counsellor is cast in the role of a parent figure. An example is shown in Figure 7.9.

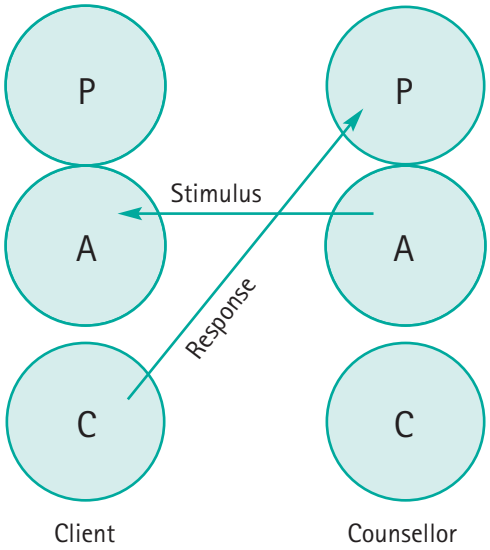


Figure 7.9 Transference (crossed transaction)

The conversation taking place between counsellor and client was as follows:

COUNSELLOR: Last week we talked about the changes you wanted to make . . .
(Adult to Adult)

CLIENT: I'm going to need a lot of help to get started.
(Child to Parent)

Transference dependence on the counsellor is certainly not encouraged in transactional analysis, and the equality of the relationship tends to work against its continuing development in any case.

Countertransference

Countertransference can also be illustrated in diagram form. Figure 7.10 shows the counsellor's Parent response to a statement the client has made.

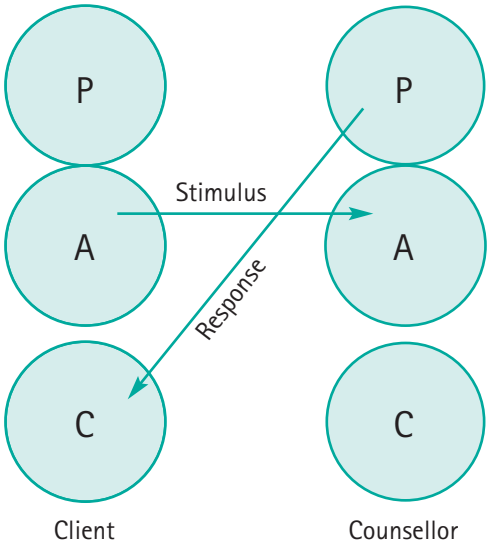


Figure 7.10 Countertransference

The conversation during this exchange was as follows:

CLIENT: I want to refer to one point in the contract we discussed last week.
(Adult to Adult)

COUNSELLOR: Is there something that worries you . . . something I can help you with?
(Parent to Child)

EXERCISE

Transference and countertransference

Using the ego state model described in this unit, illustrate in diagram form any examples of transference or countertransference you can identify from personal experience of helping, or being helped.

How transactional analysis has evolved

Since its inception in the 1960s, transactional analysis has evolved and changed in fundamental ways. Erskine, who refers to transactional analysis as a 'live culture' (Erskine, 2009), enumerates the many and varied developments

the approach has undergone, especially within the past ten years (Erskine, 2009: 15). These changes and developments are seen as essential by Erskine and proof to him that the theory and practice of transactional analysis is in a vibrant and healthy state.

In his analysis of the ways in which transactional analysis has absorbed and integrated new methods and theories, Erskine details his 40 years of professional involvement with it.

The first point he makes is that some practitioners of transactional analysis have always emphasised the 'centrality of the therapeutic relationship', but it is only within recent years that the quality of this relationship has become 'paramount' in training programmes and in its literature (Erskine, 2008: 16). Before going on to describe the centrality of the relationship as it exists now within transactional analysis, Erskine enumerates some of the significant developmental milestones that have preceded it. He highlights his own introduction to Berne's original theories, with its emphasis on ego states, analysis of transactions and theory of life scripts, but it was not until he started to treat clients with posttraumatic stress that he came to understand the true nature of games, for example. To Erskine, it was not that clients were simply involved in game playing. On the contrary, games were the clients' attempts to convey unconscious messages they could not convey in any other way. From this point onwards, he details the major advances in transactional analysis theory and practice. Some of these are as follows.

Major advances in transactional analysis theory and practice

Franklin Ernst

Franklin Ernst (1971), who was a behavioural transactional analyst and writer, emphasised the importance of behavioural change as a means of ending outdated script patterns. He introduced the idea of using contracts at the end of therapy and not just the beginning. This end of therapy innovation was meant to solidify the client's commitment to change. In addition, Ernst placed greater stress on the use of strokes with clients. This meant addressing them by name and focusing on areas of their lives that they were proud of. Ernst often worked with prisoners, and his approach with them was positive and affirming.

Behavioural change and short-term therapy

An emphasis on behavioural change began to emerge in transactional analysis, along with a focus on short-term therapy. This era also became synonymous with a more confrontational style. Aspects of these developments are indicated by Karpman (1968), whose illustration of the 'Drama Triangle' was meant to explain the dynamics of game playing, but often encouraged confrontation. Schiff (1975) developed the 'Cathexis' model of transactional analysis, which was designed to help patients in

residential care and used the concept of 'reparenting', in order to mend their original dysfunctional relationship with parents. The word 'Cathexis' is derived from psychoanalysis and refers to the psychic energy a person invests in another person, ideas or objects. In the Cathexis model of transactional analysis the Parent ego state develops constantly throughout the life span. If the Parent ego state is not fixed as Schiff suggests, then it follows that a reparenting relationship with a therapist is possible. The Cathexis model was meant to help disturbed clients repair a dysfunctional relationship with an actual parent. Schiff's techniques were also designed to encourage these clients to consider the effects of their behaviour on other people, but Schiff's approach tended to be confrontational as well.

The 'Redecision' school

The 'Redecision' school founded by Mary and Robert Goulding (1979) integrated the theory and practice of transactional analysis with Gestalt therapy and often used confrontation as a prerequisite to change or redecision. Their theory of redecision stresses the possibility of reversing early decisions made in childhood. In order to facilitate this change in clients, the Gouldings encouraged them to return to childhood and the time when inappropriate decisions were made. This approach meant that clients were prompted to go back to the original childhood scenes, and from a Child ego state new decisions could be made. Theoretically, it is then possible, over time and with practice, for clients to gain a new sense of confidence and freedom from outdated injunctions.

Egograms

The egogram is a diagram or graph designed by Jack Dusay (1977) to indicate the relative importance of each ego state within a person's personality. This is based on the premise that we all have different amounts of psychic energy invested in the three ego states. The egogram shows this distribution of energy. It should be possible for a person to chart their own egogram by drawing a graph of their feelings over a period of time, indicating the ego states operating as feelings and behaviour change. Aside from his work with egograms, Dusay's style was also confrontational at this time.

Decrease in confrontational methods in 1980s

The 1980s saw a decrease in confrontational methods within transactional analysis. Many practitioners challenged aspects of the approach to clients and patients in vogue at that time. Erskine (2009) refers to his work with people suffering from post-traumatic stress disorder; sensing their vulnerability he avoided any focus on behaviour change or confrontation. He did not use the transactional analysis methods of interviewing, interpretation or explanation, but focused instead on clients' experiences and feelings. Alongside this, Erskine came to believe that clients most needed attitudes of acceptance rather than confrontation.

Change in theory and practice continued to develop

Change in the theory and practice of transactional analysis continued to develop. At numerous workshops and seminars, practitioners discussed the relative merits of various approaches. Cognitive understanding was a central tenet of transactional analysis, but gradually its pre-eminence began to wane, though it certainly did not disappear. The emphasis on explanation through teaching and the use of diagrams as aids to client understanding were no longer the bedrock on which transactional analysis therapists built their approach to helping. However, these skills are still considered useful in transactional analysis, but they no longer dominate the approach.

Integration of diverse theoretical concepts

Today, transactional analysis integrates diverse theoretical concepts. Erskine (2009: 18) suggests that there may be a 'dozen' different schools of transactional analysis, including 'psychodynamic transactional analysis', 'constructivist transactional analysis', 'relational transactional analysis' and 'body-centred transactional analysis'. In addition to those listed by Erskine, there are now 'cognitive behavioural' transactional analysis practitioners and 'narrative' transactional analysis practitioners. The term 'narrative' used in this context refers to an approach that is client centred and collaborative and seeks to understand the client's story, with an emphasis on what is of interest to the client. Erskine attributes this proliferation of different approaches to the 'flexibility' of transactional analysis, as well as to its lack of strict or unbending dogma.

Quality of therapeutic relationship is central to therapy and healing

The quality of the therapeutic relationship is now regarded as central to the process of therapy and healing. Erskine (2009: 19) describes key aspects of current transactional analysis theory and practice. These include the ability of the therapist to be personally involved in relationship with the client. This does not imply any overlapping of professional boundaries, however. What it does mean is that the focus has moved away from the client's 'internal disturbance alone' to a newer, two-person 'relational psychotherapy', in which the therapist is engaged in an empathetic relationship with the client (Erskine, 2009: 20). The work of Kohut (1977), whose theories we have discussed in Unit 4, was a key influence on Erskine's decision to move from an interpretive approach to one that emphasised validation, presence, empathy and an acknowledgement of the client's unique, phenomenological experience. As we have already seen, this does not mean that the classical concepts of transactional analysis (described in this unit) have been abandoned; what it does mean is that new perspectives, including the focus on the integrative and relational are welcomed and absorbed. These new perspectives ensure that transactional analysis theory and practice continues to grow and develop.

Clients who benefit from this approach

Transactional analysis is a model of counselling that has potential benefit for most clients. Its strength lies in its accessibility as a communications skills model, and it is especially helpful to those clients who have relationship problems (whether at home or in the workplace). In either of these two situations, clients can be helped through the use of the structural and descriptive diagrams of personality, in order to identify the source of their difficulties in communicating effectively with others. Transactional analysis can also encourage clients to abandon outdated ways of relating to others through game playing. The unpleasant feelings like anger, hurt, fear and guilt, associated with games and referred to as 'rackets' by Berne (1972: 137), are highlighted in therapy too. When such feelings are identified in this way, clients are then able to make the changes necessary to become more open and spontaneous in relation to other people.

The concept of scripts is useful, too, since it serves to illuminate the early events and formative influence that tend to govern the course of our lives and behaviour. Clients who experience a compulsion to repeat certain patterns of behaviour, for example, can learn a great deal through transactional analysis, and those clients who are simply interested in personal growth can also benefit from it. The approach has many applications to everyday life, and, since it is often conducted in groups, it has special relevance for clients who experience problems in relation to groups. The group approach tends to make therapy cheaper for clients as well; and because transactional analysis training is well established and thorough, with an emphasis on professionalism and accreditation, clients are likely to get quality service from the counsellors who help them.

Because of its strong 'cognitive' orientation, transactional analysis tends to appeal to people who value a more intellectual approach. As we have already seen, this does not imply an absence of attention to feelings, but it does mean that a thinking and analytical engagement is more prominent in transactional analysis than in some other models. In addition, some people respond well to an approach that is questioning and educational. With its attention to contracts and goals, which can be renegotiated between counsellor and client when necessary, transactional analysis offers a structured and problem-focused psychotherapy that can be either short or long term, depending on the needs of clients. Corey (2009: 32) suggests that transactional analysis has much to offer when applied to multicultural counselling. This is because of its focus on 'family injunctions and the emphasis on early decisions'. Clients from different cultural backgrounds may have received injunctions that bind them to certain restricted work choices, for example, or to limited marital or other choices. Depending on their commitment to autonomy from family injunctions, these clients may welcome the descriptive clarity and structure the theory of transactional analysis offers.

Some limitations

At the beginning of this unit, we noted that the specialised terminology of transactional analysis is both its strength and its weakness. The language of

the approach is attractive to many people, but there are others for whom it seems simplistic or contrived. However, it is relevant to point out here that Berne deliberately designed a system that would simplify complex Freudian concepts, thereby making them comprehensible to everyone. The personality structure Berne delineated through his use of Parent, Adult and Child is readily understood by most people, including teenagers and even children. The traditional cognitive emphasis in transactional analysis may limit it for some people, especially those who place more value on the expression of emotion. On the other hand, most transactional analysis therapists are familiar with diverse approaches and integrate aspects of these in their work, where appropriate. In addition, the current emphasis on the client–counsellor relationship means that a more empathic approach to clients is likely within contemporary practice. It is possible that the language of transactional analysis may come to dominate exchanges between therapist and client during sessions, thereby potentially distancing both from true person-to-person contact. There are some situations, including crisis counselling, for example, for which the approach may not be immediately suitable. This is because people in severe crisis often lack the concentration necessary to engage with unfamiliar concepts such as those intrinsic to transactional analysis. On the other hand, clients post-crisis may benefit greatly from transactional analysis, especially if the crisis has led them to seek enlightenment about family and their experiences within it. Corey (2009), whom we quoted in the previous section, also points out that even though transactional analysis has definite strengths in relation to multicultural counselling, it has some limitations in this context too. He states, for example, that 'the terminology may seem foreign to some people', in spite of the fact that it is generally considered easy to understand (Corey, 2009: 34). In addition, there are clients who, because of their culture and background, would consider it anathema to question family traditions in the way that transactional analysis encourages people to do.

SUMMARY

In this unit, we looked at transactional analysis and considered its application to therapeutic counselling. The first part of the unit dealt with some aspects of the specialised terminology used in the approach, and its groupwork format was also discussed. A brief history of Eric Berne was provided, and his ego state model was described in some detail. The four major areas of transactional analysis (structural, transactional, game and script) were outlined. The four basic OK positions were also described, and the way in which games and scripts originate from these was discussed. We looked at the objectives in transactional analysis counselling, the importance of contracts and goals, and the therapeutic relationship between client and counsellor. Transference and countertransference were shown in diagram form, and the dialogue was included to illustrate these concepts. Early and more recent developments in transactional analysis were outlined, along with some discussion about the advantages and disadvantages of the approach in relation to clients.

CHAPTER 11



Reality Therapy

INTRODUCTION

KEY CONCEPTS

- View of Human Nature
- Choice Theory Explanation of Behavior
- Characteristics of Reality Therapy

THE THERAPEUTIC PROCESS

- Therapeutic Goals
- Therapist's Function and Role
- Client's Experience in Therapy
- Relationship Between Therapist and Client

APPLICATION: THERAPEUTIC TECHNIQUES AND PROCEDURES

- The Practice of Reality Therapy
- The Counseling Environment
- Procedures That Lead to Change
- The "WDEP" System
- Application to Group Counseling

REALITY THERAPY FROM A MULTICULTURAL PERSPECTIVE

- Strengths From a Diversity Perspective
- Shortcomings From a Diversity Perspective

REALITY THERAPY APPLIED TO THE CASE OF STAN

SUMMARY AND EVALUATION

- Summary
- Contributions of Reality Therapy
- Limitations and Criticisms of Reality Therapy

WHERE TO GO FROM HERE

- Recommended Supplementary Readings
- References and Suggested Readings

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William Glasser / Robert E. Wubbolding



Courtesy of The William Glasser Institute, Chatsworth, CA

WILLIAM GLASSER (b. 1925), currently retired, was educated at Case Western Reserve University in Cleveland, Ohio. Initially a chemical engineer, he turned to psychology (MA, Clinical Psychology, 1948) and then to psychiatry, attending medical school (MD, 1953) with the intention of becoming a psychiatrist. By 1957 he had completed his psychiatric training at the Veterans Administration and UCLA in Los Angeles and in 1961 was board certified in psychiatry.

Glasser was married to Naomi for 47 years, and she was very involved with the William Glasser Institute until her death in 1992. In 1995 Glasser married Carleen, who is an instructor at the institute. Glasser played tennis often until recently, and now, at age 86, he enjoys watching basketball on television.

Very early Glasser rejected the Freudian model, partly due to his observation of psychoanalytically trained therapists who did not seem to be implementing Freudian principles. Rather, they tended to hold people responsible for their behavior. Early in his career, Glasser was a psychiatrist at the Ventura School, a prison and school for girls operated by the California Youth Authority. He became convinced that his psychoanalytic training was of limited utility in counseling these young people. From these observations, Glasser thought it best to talk to the sane part of clients, not their disturbed side. Glasser was also influenced by G. L. Harrington, a psychiatrist and mentor. Harrington believed in getting his patients involved in projects in the real world, and by the end of his residency Glasser began to put together ideas that would later be known as reality therapy.

In 1962 Glasser began to present public lectures on "reality psychiatry," but few psychiatrists were in the audience. Most of those attending were educators, social workers, counselors, and correctional workers, so Glasser changed the name of his system to "reality therapy," which became the title of his groundbreaking book published in 1965. Educators found the principles of reality therapy helpful, and he was asked to apply it to the classroom and the school as an organization. As a result of this experience, he wrote *Schools Without Failure* in 1968, which had a major impact on the administration of schools, the training of teachers, and the way learning is conducted in schools. Glasser took the position that schools needed to be structured in ways to help students achieve a *success identity* as opposed to a *failure identity*. He advocated for a curriculum geared to the lives of learners. Glasser made significant contributions through in-service workshops for teachers and administrators. Since the late 1960s, reality therapy has been further applied to education and to virtually all other human relationships, especially intimate relationships.

Glasser became convinced that it was of paramount importance that clients accept personal responsibility for their behavior. By the early 1980s, Glasser was looking for a theory that could explain all his work. Glasser learned about *control theory* from William Powers, and he believed this theory had great potential. He spent the next 10 years expanding, revising, and clarifying what he was initially taught. By 1996 Glasser had become convinced that these revisions had so changed the theory that it was misleading to continue to call it control theory, and he changed the name to *choice theory* to reflect all that he had developed. The essence of reality therapy, now taught all over the world, is that we are all responsible for what we choose to do. We are internally motivated by current needs and wants, and we control our present behavioral choices.



Courtesy of Robert E. Wubbolding

ROBERT E. WUBBOLDING, EdD (b. 1936), born and raised in Cincinnati, Ohio, is the youngest of six children. He received his doctorate in counseling from the University of Cincinnati, is a member of several professional organizations, and has licenses as a counselor and as a psychologist. He taught high school history, worked as a high school and elementary school counselor, and served as a consultant to drug and alcohol abuse programs of the U.S. Army and Air Force. In addition, Bob Wubbolding began a career in the Catholic priesthood but later "left the clergy freely and honorably." He is married

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to Sandra Trifilio, a former French teacher, who shares his passion for his work and is administrator of the Center for Reality Therapy, as well as editor of his writings.

He is now the director of the Center for Reality Therapy in Cincinnati and professor emeritus of Xavier University, where he taught counselor education for 32 years. He loved teaching and viewed his students as being highly motivated, eager to learn, and experienced. One of his most meaningful experiences was teaching graduate students in the counseling department at Xavier University.

After completing his doctorate, Wubbolding attended training sessions representing a wide range of counseling approaches, yet he found reality therapy to be best suited to his interests. He attended many intensive training workshops conducted by William Glasser in Los Angeles, and in 1988 Glasser appointed him director of training for the William Glasser Institute.

Wubbolding served as visiting professor at the University of Southern California in their overseas programs in Japan, Korea, and Germany, thus fulfilling his lifelong desire to travel and to live in other countries. He has become an internationally known teacher, author, and practitioner of reality therapy and has introduced choice theory and reality therapy in Europe, Asia, and the Middle East. Among his specialties is adapting choice theory and reality therapy to various cultures and ethnic groups. He received the Gratitude Award (2009) for Initiating Reality Therapy in the United Kingdom and the Certificate of Reality Therapy Psychotherapist by the European Association for Psychotherapy (2009).

Wubbolding has extended the theory and practice of reality therapy with his conceptualization of the WDEP system, which is described later in the chapter. He has written 11 books and more than 145 articles, essays, and chapters in textbooks as well as preparing 12 videos/DVDs, some of which are referenced in this chapter. His religious commitment and his life of service to others are apparent in his work, and he continues his vocation of teacher, counselor, psychologist, and active member of his church.

INTRODUCTION

Reality therapists believe the underlying problem of most clients is the same: they are either involved in a present unsatisfying relationship or lack what could even be called a relationship. Many of the problems of clients are caused by their inability to connect, to get close to others, or to have a satisfying or successful relationship with at least one significant person in their lives. The therapist guides clients toward a satisfying relationship and teaches them more effective ways of behaving. The more clients are able to connect with people, the greater chance they have to experience happiness.

Few clients understand that their problem, which is unhappiness, results from the way they are choosing to behave. What they do know is that they feel a great deal of pain or that they are unhappy because they have been sent for counseling by someone with authority who is not satisfied with their behavior—typically a court official, a school administrator, an employer, a spouse, or a parent. Reality therapists recognize that clients choose their behaviors as a way to deal with the frustrations caused by unsatisfying relationships.

Glasser (2003) contends that clients should not be labeled with a diagnosis except when it is necessary for insurance purposes. From Glasser's perspective, diagnoses are descriptions of the behaviors people choose in their attempt to deal with the pain and frustration that is endemic to their unsatisfying present relationships. Labeling these ineffective behaviors as mental illness is inaccurate. Glasser

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believes mental illnesses are conditions such as Alzheimer's disease, epilepsy, head trauma, and brain infections—conditions associated with tangible brain damage. Because these people are suffering from a brain abnormality, he contends they should be treated primarily by neurologists.

Reality therapy is based on choice theory as it is explained in several of Glasser's (1998, 2001, 2003) books. (In this chapter, the discussion of Glasser's ideas pertains to these three books, unless otherwise specified.) **Choice theory** is the theoretical basis for reality therapy; it explains why and how we function. **Reality therapy** provides a delivery system for helping individuals take more effective control of their lives. If choice theory is the highway, reality therapy is the vehicle delivering the product (Wubbolding, 2011a). Therapy consists mainly of helping and sometimes teaching clients to make more effective choices as they deal with the people they need in their lives. Glasser maintains that it is essential for the therapist to establish a satisfying relationship with clients as a prerequisite for effective therapy. Once this relationship is developed, the skill of the therapist as a teacher assumes a central role.

Reality therapy has been used in a variety of settings. The approach is applicable to counseling, social work, education, crisis intervention, corrections and rehabilitation, institutional management, and community development. Reality therapy is popular in schools, general hospitals, state mental hospitals, halfway houses, and alcohol and drug abuse centers. Many of the military clinics that treat substance abusers use reality therapy as their preferred therapeutic approach.



See the video program for Chapter 11, *DVD for Theory and Practice of Counseling and Psychotherapy: The Case of Stan and Lecturettes*. I suggest that you view the brief lecture for each chapter prior to reading the chapter.

KEY CONCEPTS

View of Human Nature

Choice theory posits that we are not born blank slates waiting to be *externally motivated* by forces in the world around us. Rather, we are born with five genetically encoded needs that drive us all our lives: *survival,* or self-preservation*; love and belonging; power,* or inner control*; freedom,* or independence*;* and *fun,* or enjoyment. Each of us has all five needs, but they vary in strength. For example, we all have a need for love and belonging, but some of us need more love than others. Choice theory is based on the premise that because we are by nature social creatures we need to both receive and give love. Glasser (2001, 2005) believes the need *to love and to belong* is the primary need because we need people to satisfy the other needs. It is also the most difficult need to satisfy because we must have a cooperative person to help us meet it.

Our brain functions as a control system. It continually monitors our feelings to determine how well we are doing in our lifelong effort to satisfy these needs. Whenever we feel bad, one or more of these five needs is unsatisfied. Although we may not be aware of our needs, we know that we want to feel better. Driven by

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pain, we try to figure out how to feel better. Reality therapists teach clients choice theory, sometimes subtly and indirectly, so clients can identify unmet needs and try to satisfy them.

Choice theory teaches that we do not satisfy our needs directly. What we do, beginning shortly after birth and continuing all our lives, is to keep close track of anything we do that feels very good. We store information inside our minds and build a file of wants, called our **quality world**, which is at the core of our life. It is our personal Shangri-la—the world we would like to live in if we could. It is completely based on our wants and needs, but unlike the needs, which are general, it is very specific. The quality world consists of specific images of people, activities, events, beliefs, possessions, and situations that fulfill our needs (Wubbolding, 2000, 2011a). Our quality world is like a picture album. We develop an inner **picture album** of specific wants as well as precise ways to satisfy these wants. We are attempting to behave in a way that gives us the most effective control over our lives. Some pictures may be blurred, and the therapist's role is to help the client clarify them. Pictures exist in priority for most people, yet clients may have difficulty identifying their priorities. Part of the process of reality therapy is assisting clients in prioritizing their wants and uncovering what is most important to them (Wubbolding, 2011a).

People are the most important component of our quality world, and these are the people we most want to connect with. It contains the people we are closest to and most enjoy being with. Those who enter therapy generally have no one in their quality world or, more often, someone in their quality world that they are unable to relate to in a satisfying way. For therapy to have a chance of success, a therapist must be the kind of person that clients would consider putting in their quality world. Getting into the clients' quality world is the art of therapy. It is from this relationship with the therapist that clients begin to learn how to get close to the people they need.

Choice Theory Explanation of Behavior

Choice theory explains that all we ever do from birth to death is behave and, with rare exceptions, everything we do is chosen. Every total behavior is our best attempt to get what we want to satisfy our needs. **Total behavior** teaches that all behavior is made up of four inseparable but distinct components—*acting, thinking, feeling,* and *physiology*—that necessarily accompany all of our actions, thoughts, and feelings. Choice theory emphasizes thinking and acting, which makes this a general form of cognitive behavior therapy. The primary emphasis is on what the client is doing on and how the doing component influences the other aspects of total behavior. Behavior is purposeful because it is designed to close the gap between what we want and what we perceive we are getting. Specific behaviors are always generated from this discrepancy. Our behaviors come from the inside, and thus we choose our destiny.

Glasser says that to speak of being depressed, having a headache, being angry, or being anxious implies passivity and lack of personal responsibility, and it is inaccurate. It is more accurate to think of these as parts of total behaviors and to use

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the verb forms *depressing, headaching, angering,* and *anxietying* to describe them. It is more accurate to think of people depressing or angering themselves rather than being depressed or being angry. When people choose misery by developing a range of "paining" behaviors, it is because these are the best behaviors they are able to devise at the time, and these behaviors often get them what they want.

When a reality therapist starts teaching choice theory, the client will often protest and say, "I'm suffering; don't tell me I'm choosing to suffer like this." As painful as depressing is, the therapist explains that people do not choose pain and suffering directly; rather, it is an unchosen part of their total behavior. The behavior of the person is the best effort, ineffective as it is, to satisfy needs.

Robert Wubbolding (personal communication, September 7, 2010) has added a new idea to choice theory. He believes that behavior is a language, and that we send messages by what we are doing. The purpose of behavior is to influence the world to get what we want. Therapists ask clients what messages they are sending to the world by way of their actions: "What message do you want others to get?" "What message are others getting whether or not you intended to send them?" By considering the messages that clients send to others, counselors can help clients indirectly gain a greater appreciation of messages they unintentionally send to others.

Characteristics of Reality Therapy

Contemporary **reality therapy** focuses quickly on the unsatisfying relationship or the lack of a relationship, which is often the cause of clients' problems. Glasser has given increasing attention to the role of meaningful relationships in fostering emotional health. Clients may complain of a problem such as not being able to keep a job, not doing well in school, or not having a meaningful relationship. When clients complain about how other people are causing them pain, the therapist does not get involved with finding fault. Reality therapists ask clients to consider how effective their choices are, especially as these choices affect their relationships with significant people in their lives. Choice theory teaches that talking about what clients cannot control is of minimal value; the emphasis is on what clients *can* control in their relationships. The basic axiom of choice theory, which is crucial for clients to understand, is this: "The only person you can control is yourself." Two books devoted to how choice theory can help people deal with their relationship problems and enhance their relationships are *Getting Together and Staying Together* (Glasser & Glasser, 2000) and *Eight Lessons for a Happier Marriage* (Glasser & Glasser, 2007).

Reality therapists do not listen very long to complaining, blaming, and criticizing, for these are the most ineffective behaviors in our behavioral repertoire. Because reality therapists give little attention to these self-defeating *total behaviors,* Glasser maintains that they tend to disappear from therapy. What do reality therapists focus on? Here are some underlying characteristics of reality therapy.

EMPHASIZE CHOICE AND RESPONSIBILITY If we choose all we do, we must be responsible for what we choose. This does not mean we should be blamed or punished, unless we break the law, but it does mean the therapist should never lose sight of the fact that clients are responsible for what they do. Choice theory changes the focus of responsibility to choice and choosing.

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Reality therapists deal with people "as if" they have choices. Therapists focus on those areas where clients have choice, for doing so gets them closer to the people they need. For example, being involved in meaningful activities, such as work, is a good way to gain the respect of other people, and work can help clients fulfill their need for power. It is very difficult for adults to feel good about themselves if they don't engage in some form of meaningful activity. As clients begin to feel good about themselves, it is less necessary for them to continue to choose ineffective and self-destructive behaviors.

REJECT TRANSFERENCE Reality therapists strive to be themselves in their professional work. By being themselves, therapists can use the relationship to teach clients how to relate to others in their lives. Glasser contends that transference is a way that both therapist and client avoid being who they are and owning what they are doing right now. It is unrealistic for therapists to go along with the idea that they are anyone but themselves. Assume the client claims, "I see you as my father or mother and this is why I'm behaving the way I am." In such a situation a reality therapist is likely to say clearly and firmly, "I am not your mother, father, or anyone but myself."

KEEP THE THERAPY IN THE PRESENT Some clients come to counseling convinced that they must revisit the past if they are to be helped. Many therapeutic models teach that to function well in the present people must understand and revisit their past. Glasser (2001) disagrees with this assumption and contends that whatever mistakes were made in the past are not pertinent now. An axiom of choice theory is that the past may have contributed to a current problem but that the past is never the problem. To function effectively, people need to live and plan in the present and take steps to create a better future. We can only satisfy our needs in the present.

The reality therapist does not totally reject the past. If the client wants to talk about past successes or good relationships in the past, the therapist will listen because these may be repeated in the present. Reality therapists will devote only enough time to past failures to assure clients that they are not rejecting them. As soon as possible, therapists tell clients, "What has happened is over; it can't be changed. The more time we spend looking back, the more we avoid looking forward." Although the past has propelled us to the present, reality therapists contend that it does not have to determine our future. We are free to make choices, even though our external world limits our choices (Wubbolding, 2011b).

AVOID FOCUSING ON SYMPTOMS In traditional therapy a great deal of time is spent focusing on symptoms by asking clients how they feel and why they are obsessing. Focusing on the past "protects" clients from facing the reality of unsatisfying present relationships, and focusing on symptoms does the same thing. Glasser (2003) contends that people who have symptoms believe that if they could only be symptom-free they would find happiness. Whether people are depressing or paining, they tend to think that what they are experiencing is happening to them. They are reluctant to accept the reality that their suffering is due to the total behavior they are choosing. Their symptoms can be viewed as the body's way

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of warning them that the behavior they are choosing is not satisfying their basic needs. The reality therapist spends as little time as he or she can on the symptoms because they will last only as long as they are needed to deal with an unsatisfying relationship or the frustration of basic needs.

According to Glasser, if clients believe that the therapist wants to hear about their symptoms or spend time talking about the past, they are more than willing to comply. Engaging in long journeys into the past or exploring symptoms results in lengthy therapy. Glasser (2005) maintains that almost all symptoms are caused by a present unhappy relationship. By focusing on present problems, especially interpersonal concerns, therapy can generally be shortened considerably.

CHALLENGE TRADITIONAL VIEWS OF MENTAL ILLNESS Choice theory rejects the traditional notion that people with problematic physical and psychological symptoms are mentally ill. Glasser (2003) has warned people to be cautious of psychiatry, which can be hazardous to both one's physical and mental health. He criticizes the traditional psychiatric establishment for relying heavily on the *DSM-IV-TR* (American Psychiatric Association, 2000) for both diagnosis and treatment. Glasser (2003) challenges the traditionally accepted views of mental illness and treatment by the use of medication, especially the widespread use of psychiatric drugs that often result in negative side effects both physically and psychologically. Wubbolding (personal communication, September 7, 2010) emphasizes that reality therapy is a mental health system rather than a remediating system. He incorporates the Ericksonian principle that "people don't have problems, they have solutions that have not worked." By reframing diagnostic categories and negative behaviors, the counselor helps the client to perceive his or her behaviors in a very different light, which facilitates the search for more effective solutions and choices.

THE THERAPEUTIC PROCESS

Therapeutic Goals

A primary goal of contemporary reality therapy is to help clients get connected or reconnected with the people they have chosen to put in their quality world. In addition to fulfilling this need for love and belonging, a basic goal of reality therapy is to help clients learn better ways of fulfilling all of their needs, including achievement, power or inner control, freedom or independence, and fun. The basic human needs serve to focus treatment planning and setting both short- and long-term goals. Reality therapists assist clients in making more effective and responsible choices related to their wants and needs.

In many instances, clients come voluntarily for therapy, and these clients are the easiest to help. However, another goal entails working with an increasing number of involuntary clients who may actively resist the therapist and the therapy process. These individuals often engage in violent behavior, addictions, and other kinds of antisocial behaviors. It is essential for counselors to do whatever they can to get connected with involuntary clients. If the counselor is unable to make a connection, there is no possibility of providing significant help. If the counselor *can* make a connection, the goal of teaching the client how to fulfill his or her needs can slowly begin.

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Therapist's Function and Role

Therapy is often considered as a mentoring process in which the therapist is the teacher and the client is the student. Reality therapists teach clients how to engage in self-evaluation, which is done by raising the question, "Is what you are choosing to do getting you what you want and need?" Here are some other questions that therapists tend to ask clients:

- How would you most like to change your life?
- What do you want in your life that you are not getting?
- What would you have in your life if you were to change?
- What do you have to do now to make the changes happen?

The role of the reality therapist is not to make the evaluation for clients but to challenge clients to examine what they are doing. Reality therapists assist clients in evaluating their own behavioral direction, specific actions, wants, perceptions, level of commitment, possibilities for new directions, and action plans. Clients then decide what to change and formulate a plan to facilitate the desired changes. The outcome is better relationships, increased happiness, and a sense of inner control of their lives (Wubbolding, 2011b).

It is the job of therapists to convey the idea that no matter how bad things are there is hope. If therapists are able to instill this sense of hope, clients feel that they are no longer alone and that change is possible. The therapist functions as an advocate, or someone who is on the client's side. Together they can creatively address a range of concerns and options.

Client's Experience in Therapy

Clients are not expected to backtrack into the past or get sidetracked into talking about symptoms. Neither will much time be spent talking about feelings separate from the acting and thinking that are part of the total behaviors over which clients have direct control. The emphasis in on actions. When clients change what they are doing, they often change how they are feeling and thinking.

Therapists will gently, but firmly confront clients. Reality therapists often ask clients questions such as these: "Is what you are choosing to do bringing you closer to the people you want to be closer to right now?" "Is what you are doing getting you closer to a new person if you are presently disconnected from everyone?" These questions are part of the self-evaluation process, which is the cornerstone of reality therapy.

Clients can expect to experience some urgency in therapy. Time is important, as each session may be the last. Clients should be able to say to themselves, "I can begin to use what we talked about today in my life. I am able to bring my present experiences to therapy as my problems are in the present, and my therapist will not let me escape from that fact."

Relationship Between Therapist and Client

Reality therapy emphasizes an understanding and supportive relationship, or therapeutic alliance, which is the foundation for effective outcomes (Wubbolding & Brickell, 2005; Wubbolding, Robey, & Brickell, 2010). The therapist's skill in

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establishing a trusting relationship is critical. It is also important that the client perceives the therapist as being skilled and knowledgeable. Although the therapeutic relationship is paramount, it is not an end in itself, and it is not automatically curative or healing (Wubbolding, 2011a).

For involvement between the therapist and the client to occur, the counselor must have certain personal qualities, including warmth, sincerity, congruence, understanding, acceptance, concern, respect for the client, openness, and the willingness to be challenged by others. (For other personal characteristics, see Chapter 2.) Wubbolding (2010, 2011a, 2011b) identifies specific ways for counselors to create a climate that leads to involvement with clients. Some of these ways entail using attending behavior, listening to clients, suspending judgment, doing the unexpected, using humor appropriately, being oneself as a counselor, engaging in facilitative self-disclosure, listening for metaphors in the client's mode of self-expression, listening for themes, summarizing and focusing, allowing consequences, allowing silence, and being an ethical practitioner. The basis for therapeutic interventions to work effectively rests on a fair, firm, friendly, and trusting environment. Once involvement has been established, the counselor assists clients in gaining a deeper understanding of the consequences of their current behavior.

APPLICATION: THERAPEUTIC TECHNIQUES AND PROCEDURES

The Practice of Reality Therapy

The practice of reality therapy can best be conceptualized as the *cycle of counseling,* which consists of two major components: (1) creating the counseling environment and (2) implementing specific procedures that lead to changes in behavior. The art of counseling is to weave these components together in ways that lead clients to evaluate their lives and decide to move in more effective directions.

How do these components blend in the counseling process? The cycle of counseling begins with creating a working relationship with clients, which was described in the previous section. The process proceeds through an exploration of clients' wants, needs, and perceptions. Clients explore their total behavior and make their own evaluation of how effective they are in getting what they want. If clients decide to try new behavior, they make plans that will lead to change, and they commit themselves to those plans. The cycle of counseling includes following up on how well clients are doing and offering further consultation as needed.

It is important to keep in mind that although the concepts may seem simple as they are presented here, being able to translate them into actual therapeutic practice takes considerable skill and creativity. Although the principles will be the same when used by any counselor who is certified in reality therapy, the manner in which these principles are applied does vary depending on the counselor's style and personal characteristics. These principles are applied in a progressive manner, but they should not be thought of as discrete and rigid categories. The art of practicing reality therapy involves far more than following procedures in a step-by-step, cookbook fashion. Although these procedures are described in jargon-free language, they can

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be challenging to implement (Wubbolding, 2007, 2011b). Counseling is not a simplistic method that is applied in the same way with every client. With choice theory in the background of practice, the counselor tailors the counseling to what the client presents. Although the counselor is prepared to work in a way that is meaningful to the client, the move toward satisfying relationships remains in the foreground.

Robert Wubbolding is a reality therapist who has extended the practice of reality therapy (WDEP system) for both implementing and teaching reality therapy (Wubbolding, 2009). Over the years he has played a major role in the development of reality therapy. I especially value Wubbolding's contributions to teaching reality therapy and to conceptualizing therapeutic procedures. His ideas render choice theory practical and useable by counselors, and his system provides a basis for conceptualizing and applying the theory. Although reality therapists operate within the spirit of choice theory, they practice in their own unique ways and develop their own individual therapeutic style. This section is based on an integrated summary and adaptation of material from various sources (Glasser, 1992, 1998, 2001; Wubbolding, 1988, 1991, 2000, 2007, 2008, 2011a, 2011b; Wubbolding et al., 1998, 2004). The *Student Manual* that accompanies this textbook contains Wubbolding's (2010) chart, which highlights issues and tasks to be accomplished throughout the cycle of counseling.

The Counseling Environment

The practice of reality therapy rests on the assumption that a supportive and challenging environment allows clients to begin making life changes. The therapeutic relationship is the foundation for effective practice; if this is lacking, there is little hope that the system can be successfully implemented. Counselors who hope to create a therapeutic alliance strive to avoid behaviors such as arguing, attacking, accusing, demeaning, bossing, criticizing, finding fault, coercing, encouraging excuses, holding grudges, instilling fear, and giving up easily (Wubbolding, 2010, 2011a, 2011b). In a short period of time, clients generally begin to appreciate the caring, accepting, noncoercive choice theory environment. It is from this mildly confrontive yet always noncriticizing, nonblaming, noncomplaining, caring environment that clients learn to create the satisfying environment that leads to successful relationships. In this coercion-free atmosphere, clients feel free to be creative and to begin to try new behaviors.

Procedures That Lead to Change

Reality therapists operate on the assumption that we are motivated to change (1) when we are convinced that our present behavior is not meeting our needs and (2) when we believe we can choose other behaviors that will get us closer to what we want. Reality therapists begin by asking clients what they want from therapy. Therapists take the mystery and uncertainty out of the therapeutic process. They also inquire about the choices clients are making in their relationships. In most instances, there is a major unsatisfied relationship, and clients usually do not believe they have any choice in what is going on in this relationship. In the beginning the client may deny this is the case. For example, the client might say, "I'm depressed. My depression is the problem. Why are you talking about my relationships?" The

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client often does not want to talk about the real problem, which is the unsatisfying relationship or lack thereof.

In the first session a skilled therapist looks for and defines the wants of the client. The therapist also looks for a key unsatisfying present relationship—usually with a spouse, a child, a parent, or an employer. The therapist might ask, "Whose behavior can you control?" This question may need to be asked several times during the next few sessions to deal with the client's resistance to looking at his or her own behavior. The emphasis is on encouraging clients to focus on what they can control.

When clients begin to realize that they can control only their own behavior, therapy is under way. The rest of therapy focuses on how clients can make better choices. There are more choices available than clients realize, and the therapist explores these possible choices. Clients may be stuck in misery, blaming, and the past, but they can choose to change—even if the other person in the relationship does not change. Wubbolding (2011a) makes the point that clients can learn that they are not at the mercy of others, are not victims, are capable of gaining a sense of inner control, and have a range of choices open to them. In short, clients in reality therapy often acquire a sense of hope for a better future.

Reality therapists explore the tenets of choice theory with clients, helping clients identify basic needs, discovering clients' quality world, and finally, helping clients understand that they are choosing the total behaviors that are their symptoms. In every instance when clients make a change, it is their choice. With the therapist's help, clients learn to make better choices than they did when they were on their own. Through choice theory, clients can acquire and maintain successful relationships.

The "WDEP" System

Wubbolding (2000) uses the acronym WDEP to describe key procedures in the practice of reality therapy. The WDEP system of reality therapy can be described as "effective, practical, usable, theory-based, cross-cultural, and founded on universal human principles" (Wubbolding, 2007, p. 204). The WDEP system can be used to help clients explore their *wants*, possible things they can *do*, opportunities for *self-evaluation,* and design *plans* for improvement (Wubbolding, 2007, 2011a, 2011b). Grounded in choice theory, the WDEP system assists people in satisfying their basic needs. Each of the letters refers to a cluster of strategies: W = wants, needs, and perceptions; D = direction and doing; E = self-evaluation; and P = planning. These strategies are designed to promote change. Let's look at each one in more detail.

WANTS (EXPLORING WANTS, NEEDS, AND PERCEPTIONS) Reality therapists assist clients in discovering their wants and hopes. All wants are related to the five basic needs. The key question asked is, "What do you want?" Through the therapist's skillful questioning, clients are assisted in defining what they want from the counseling process and from the world around them. It is useful for clients to define what they expect and want from the counselor and from themselves. Part of counseling consists of exploring the "picture album," or **quality world**, of clients and how their behavior is aimed at moving their perception of the external world closer to their inner world of wants.

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Clients are given the opportunity to explore every facet of their lives, including what they want from their family, friends, and work. Furthermore, this exploration of wants, needs, and perceptions should continue throughout the counseling process as clients' pictures change.

Here are some useful questions to help clients pinpoint what they want:

- If you were the person that you wish you were, what kind of person would you be?
- What would your family be like if your wants and their wants matched?
- What would you be doing if you were living as you want to?
- Do you really want to change your life?
- What is it you want that you don't seem to be getting from life?
- What do you think stops you from making the changes you would like?

Wubbolding and Brickell (2009) now include questions focusing on perceptions: "How do you look at the situation?" "Where do you see your control?" (p. 51). People have a great deal more control than they often perceive, and these questions help clients move from a sense of external control to a sense of internal control. This line of questioning sets the stage for applying other procedures in reality therapy. It is an art for counselors to know *what* questions to ask, *how* to ask them, and *when* to ask them. Relevant questions help clients gain insights and arrive at plans and solutions. Although well-timed, open-ended questions can help clients identify their counseling goals, excessive questioning can result in resistance and defensiveness. Part of this phase of counseling involves eliciting a commitment to counseling. Personal growth will occur to the degree that clients are committed to making changes in their actions (Wubbolding, 2011b).

DIRECTION AND DOING The focus on the present is characterized by the key question asked by the reality therapist: "What are you doing?" Even though problems may be rooted in the past, clients need to learn how to deal with them in the present by learning better ways of getting what they want. Problems must be solved either in the present or through a plan for the future. The therapist's challenge is to help clients make more need-satisfying choices.

Early in counseling it is essential to discuss with clients the overall direction of their lives, including where they are going and where their behavior is taking them. This exploration is preliminary to the subsequent evaluation of whether it is a desirable direction. The therapist holds a mirror before the client and asks, "What do you see for yourself now and in the future?" It often takes some time for this reflection to become clearer to clients so they can verbally express their perceptions.

Reality therapy focuses on gaining awareness of and changing current total behavior. To accomplish this, reality therapists focus on questions like these: "What are you doing now?" "What did you actually do yesterday?" "What did you want to do differently this past week?" "What stopped you from doing what you said you wanted to do?" "What will you do tomorrow?"

Listening to clients talk about feelings can be productive, but only if it is linked to what they are doing. When an emergency light on the car dashboard lights up, the driver is alerted that something is wrong and that immediate action is necessary to remedy a problem. In a similar way, when clients talk about problematic feelings, most reality therapists affirm and acknowledge these feelings. Rather than

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focusing mainly on these feelings, however, reality therapists encourage clients to take action by changing what they are doing and thinking. It is easier to change what we are doing and thinking than to change our feelings. From a choice theory perspective, discussions centering on feelings, without strongly relating them to what people are doing and thinking, are counterproductive.

SELF-EVALUATION **Self-evaluation** is the cornerstone of reality therapy procedures. The core of reality therapy, as we have seen, is to ask clients to make the following self-evaluation: "Does your present behavior have a reasonable chance of getting you what you want now, and will it take you in the direction you want to go?" Specifically, evaluation involves the client examining behavioral direction, specific actions, wants, perceptions, new directions, and plans (Wubbolding, 2011b). According to Wubbolding (2007), clients often present a problem with a significant relationship, which is at the root of much of their dissatisfaction. The counselor can help clients evaluate their behavior by asking this question: "Is your current behavior bringing you closer to people important to you or is it driving you further apart?" Through skillful questioning, the counselor helps clients determine if what they are doing is helping them. Artful questioning assists clients in evaluating their present behavior and the direction this is taking them. Wubbolding (2000, 2011a) suggests questions like these:

- Is what you are doing helping or hurting you?
- Is what you are doing now what you want to be doing?
- Is your behavior working for you?
- Is there a healthy congruence between what you are doing and what you believe?
- Is what you are doing against the rules?
- Is what you want realistic or attainable?
- Does it help you to look at it that way?
- Is it really true that you have no control over your situation?
- How committed are you to the therapeutic process and to changing your life?
- After carefully examining what you want, does it appear to be in your best interests and in the best interest of others?

Asking clients to evaluate each component of their total behavior is a major task in reality therapy. It is the counselor's task to assist clients in evaluating the quality of their actions and to help them make responsible choices and devise effective plans. Individuals will not change until they first decide that a change would be more advantageous. Without an honest self-assessment, it is unlikely that clients will change. Reality therapists are relentless in their efforts to help clients conduct explicit self-evaluations of each behavioral component. When therapists ask a depressing client if this behavior is helping in the long run, they introduce the idea of choice to the client. The process of evaluation of the doing, thinking, feeling, and physiological components of total behavior is within the scope of the client's responsibility.

Reality therapists may be directive with certain clients at the beginning of treatment. This is done to help clients recognize that some behaviors are not effective. In working with clients who are in crisis, for example, it is sometimes

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necessary to suggest straightforwardly what will work and what will not. Other clients, such as alcoholics and children of alcoholics, need direction early in the course of treatment, for they often do not have the thinking behaviors in their control system to be able to make consistent evaluations of when their lives are seriously out of effective control. These clients are likely to have blurred pictures and, at times, to be unaware of what they want or whether their wants are realistic. As they grow and continue to interact with the counselor, they learn to make evaluations with less and less help from the counselor (Wubbolding, 2011a; Wubbolding & Brickell, 2005).

PLANNING AND ACTION Much of the significant work of the counseling process involves helping clients identify specific ways to fulfill their wants and needs. Once clients determine what they want to change, they are generally ready to explore other possible behaviors and formulate an action plan. The key question is, "What is your plan?" The process of creating and carrying out plans enables people to begin to gain effective control over their lives. If the plan does not work, for whatever reason, the counselor and client work together to devise a different plan. The plan gives the client a starting point, a toehold on life, but plans can be modified as needed. Throughout this planning phase, the counselor continually urges the client to be willing to accept the consequences for his or her own choices and actions. Not only are plans discussed in light of how they can help the client personally, but plans are also designed in terms of how they are likely to affect others in the client's life.

Wubbolding (2000, 2007, 2008, 2011a, 2011b) discusses the central role of planning and commitment. The culmination of the cycle of counseling rests with a plan of action. Although planning is important, it is effective only if the client has made a self-evaluation and determined that he or she wants to change a behavior. Wubbolding uses the acronym SAMIC to capture the essence of a good plan: simple, attainable, measurable, immediate, involved, controlled by the planner, committed to, and consistently done. Wubbolding contends that clients gain more effective control over their lives with plans that have the following characteristics:

- The plan is within the limits of the motivation and capacities of the client. Skillful counselors help clients identify plans that involve greater need-fulfilling payoffs. Clients may be asked, "What plans could you make now that would result in a more satisfying life?"
- Good plans are simple and easy to understand. They are realistically doable, positive rather than negative, dependent on the planner, specific, immediate, and repetitive. Although they need to be specific, concrete, and measurable, plans should be flexible and open to revision as clients gain a deeper understanding of the specific behaviors they want to change.
- The plan involves a positive course of action, and it is stated in terms of what the client is willing to do. Even small plans can help clients take significant steps toward their desired changes.
- Counselors encourage clients to develop plans that they can carry out independently of what others do. Plans that are contingent on others lead clients to sense that they are not steering their own ship but are at the mercy of the ocean.

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- Effective plans are repetitive and, ideally, are performed daily.
- Plans are carried out as soon as possible. Counselors can ask the question, "What are you willing to do today to begin to change your life?"
- Plans involve process-centered activities. For example, clients may plan to do any of the following: apply for a job, write a letter to a friend, take a yoga class, substitute nutritious food for junk food, devote 2 hours a week to volunteer work, or take a vacation that they have been wanting.
- Before clients carry out their plan, it is a good idea for them to evaluate it with their therapist to determine whether it is realistic and attainable and whether it relates to what they need and want. After the plan has been carried out in real life, it is useful to evaluate it again and make any revisions that may be necessary.
- To help clients commit themselves to their plan, it is useful for them to firm it up in writing.

Resolutions and plans are empty unless there is a commitment to carry them out. It is up to clients to determine how to take their plans outside the restricted world of therapy and into the everyday world. Effective therapy can be the catalyst that leads to self-directed, responsible living.

Asking clients to determine what they want for themselves, to make a self-evaluation, and to follow through with action plans includes assisting them in determining how intensely they are willing to work to attain the changes they desire. Commitment is not an all-or-nothing matter; it exists in degrees. Wubbolding (2007, 2011a, 2011b) maintains that it is important for a therapist to express concern about clients' level of commitment, or how much they are willing to work to bring about change. This communicates in an implicit way to clients that they have within them the power to take charge of their lives. It is essential that those clients who are reluctant to make a commitment be helped to express and explore their fears of failing. Clients are helped by a therapist who does not easily give up believing in their ability to make better choices, even if they are not always successful in completing their plans. In his workshops, Wubbolding often mentions this axiom of reality therapy: "To fail to plan is to plan to fail."

Application to Group Counseling

With the emphases on connection and interpersonal relationships, reality therapy is well suited for various kinds of group counseling. Groups provide members with many opportunities for exploring ways to meet their needs through the relationships formed within the group. In particular, the WDEP system can be applied to helping group members satisfy their basic needs. If members talk about their past experiences or make excuses for their current behavior, the group leader redirects them to what they are presently doing. From the very beginning of a group, the members can be asked to take an honest look at what they are doing and to clarify whether their behavior is getting them what they say they want. Once group members get a clearer picture of what they have in their lives

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now and what they want to be different, they can use the group as a place to explore an alternative course of behavior.

This model lends itself to expecting the members to carry out homework assignments between the group meetings. However, it is the members, with the help of the leader, who evaluate their own behavior and decide whether they want to change. Members also take the lead in deciding what kinds of homework tasks they will set for themselves as a way to achieve their goals. Group leaders often meet with resistance if they make poorly timed suggestions and plans for how the members should best live their lives. To their credit, reality therapists keep asking the members to evaluate for themselves whether what they are doing is getting them what they want. If the members concede that what they are doing is not working for them, their resistance is much more likely to melt, and they tend to be more open to trying different behaviors.

Once the members make some changes, reality therapy provides the structure for them to formulate specific plans for action and to evaluate their level of success. Feedback from the members and the leader can help individuals design realistic and attainable plans. Considerable time is devoted during the group sessions for developing and implementing plans. If people don't carry out a plan, it is important to discuss with them what stopped them. Perhaps they set their goals unrealistically high, or perhaps there is a discrepancy between what they *say* they want to change and the steps they are willing to take to bring about change.

I also like reality therapy's insistence that change will not come by insight alone; rather, members have to begin doing something different once they determine that their behavior is not working for them. I am skeptical about the value of catharsis as a therapeutic vehicle unless the release of pent-up emotions is eventually put into some kind of cognitive framework and is followed up with an action plan. In the groups that I facilitate, group members are challenged to look at the futility of waiting for others to change. I ask members to assume that the significant people in their life may never change, which means that they will have to take a more active stance in shaping their own destiny. I appreciate the emphasis of reality therapy on teaching clients that the only life they can control is their own and the focus placed on helping group members change their own patterns of acting and thinking.

For a more detailed discussion of reality therapy in groups, see Corey (2012, chap. 15).

REALITY THERAPY FROM A MULTICULTURAL PERSPECTIVE



Strengths From a Diversity Perspective

The core principles of choice theory and reality therapy have much to offer in the area of multicultural counseling. In cross-cultural therapy it is essential that counselors respect the differences in worldview between themselves and their clients.

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Counselors demonstrate their respect for the cultural values of their clients by helping them explore how satisfying their current behavior is both to themselves and to others. Once clients make this assessment, they can formulate realistic plans that are consistent with their cultural values. It is a further sign of respect that the counselor refrains from deciding what behavior should be changed. Through skillful questioning on the counselor's part, clients from diverse ethnic backgrounds can be helped to determine the degree to which they have become acculturated into the dominant society. Is it possible for them to find a balance, retaining their ethnic identity and values while integrating some of the values and practices of the dominant group? Again, the counselor does not determine this balance for clients, but works with them to arrive at their own answers. With this focus on thinking and acting rather than on exploring feelings, many clients are less likely to display resistance to counseling.

Glasser (1998) contends that reality therapy and choice theory can be applied both individually and in groups to anyone with any psychological problem in any cultural context. We are all members of the same species and have the same genetic structure; therefore, relationships are the problem in all cultures. Wubbolding (2007, 2011a, 2011b) asserts that the principles underlying choice theory are universal, which makes choice theory applicable to all people. All of us have internal needs, we all make choices, and we all seek to influence the world around us. Putting the principles of choice theory into action demands creativity, sensitivity to cultures and individuals, and flexibility in implementing the procedures of reality therapy. Reality therapy principles and procedures need to be applied differently in various cultures and must be adapted to the psychological and developmental levels presented by individuals (Wubbolding, 2011b).

Based on the assumption that reality therapy must be modified to fit the cultural context of people other than North Americans, Wubbolding (2000, 2011a) and Wubbolding and colleagues (1998, 2004) have expanded the practice of reality therapy to multicultural situations. Wubbolding's experience in conducting reality therapy workshops in Japan, Taiwan, Hong Kong, Singapore, Korea, India, Kuwait, Australia, Slovenia, Croatia, and countries in western Europe has taught him the difficulty of generalizing about other cultures. Growing out of these multicultural experiences, Wubbolding (2000) has adapted the cycle of counseling in working with Japanese clients. He points to some basic language differences between Japanese and Western cultures. North Americans are inclined to say what they mean and to be assertive. In Japanese culture, assertive language is not appropriate between a child and a parent or between an employee and a supervisor. Ways of communicating are more indirect. To ask some Japanese clients what they want may seem harsh and intrusive to them. Because of these style differences, adaptations such as those listed below are needed to make the practice of reality therapy relevant to Japanese clients:

- The reality therapist's tendency to ask direct questions may need to be softened, with questions being raised more elaborately and indirectly. It may be a mistake to ask individualistic questions built around whether specific behaviors meet the client's need. Confrontation should be done only after carefully considering the context.

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- There is no exact Japanese translation for the word "plan," nor is there an exact word for "accountability," yet both of these are key dimensions in the practice of reality therapy and are central to Japanese culture.
- In asking clients to make plans and commit to them, Western counselors do not settle for a response of "I'll try." Instead, they tend to push for an explicit pledge to follow through. In Japanese culture, however, the counselor is likely to accept "I'll try" as a firm commitment.

These are but a few illustrations of ways in which reality therapy might be adapted to non-Western clients. Although this approach assumes that all people have the same basic needs (survival, love and belonging, power, freedom, and fun), the way these needs are expressed depends largely on the cultural context. In working with culturally diverse clients, the therapist must allow latitude for a wide range of acceptable behaviors to satisfy these needs. As with other theories and the techniques that flow from them, flexibility is a foremost requirement.

A key strength of reality therapy is that it provides clients with tools to make the changes they desire. This is especially true during the planning phase, which is central to the process of reality therapy. The focus is on positive steps that can be taken, not on what cannot be done. Clients identify those problems that are causing them difficulty, and these problems become the targets for change. This type of specificity and the direction that is provided by an effective plan are certainly assets in working with diverse client groups.

Reality therapy needs to be used artfully and to be applied in different ways with a variety of clients. Many of its principles and concepts can be incorporated in a dynamic and personal way in the style of counselors, and there is a basis for integrating these concepts with most of the other therapeutic approaches covered in this book.

Shortcomings From a Diversity Perspective

One of the shortcomings of reality therapy in working with clients from certain ethics groups is that it may not take fully into account some very real environmental forces that operate against them in their everyday lives. Reality therapy gives only limited attention to helping people address environmental and social problems. Discrimination, racism, sexism, homophobia, heterosexism, ageism, negative attitudes toward disabilities, and other social injustices are unfortunate realities, and these forces do limit many individuals in getting what they want from life. It is important that therapists acknowledge that people do not choose to be the victims of various forms of discrimination and oppression. If therapists do not accept these environmental restrictions or are not interested in bringing about social justice as well as individual change, clients are likely to feel misunderstood. There is a danger that some reality therapists may overstress the ability of these clients to take charge of their lives and not pay enough attention to systemic and environmental factors that can limit the potential for choice.

Some reality therapists may make the mistake of too quickly or too forcefully stressing the ability of their clients to take charge of their lives. On this point,

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Wubbolding (2008) maintains that because of oppression and discrimination, some people have fewer choices available to them, yet they do have choices. Wubbolding sees reality therapy as helping clients to focus on those choices they *do* have. Although focusing on choices clients do have is useful, I believe clients may need to talk about the ways their choices are restricted by environmental circumstances. Therapists would do well to consider how both they and their clients could take even small steps toward bringing about societal changes, as do feminist therapists (see Chapter 12).

Another shortcoming associated with reality therapy is that some clients are very reluctant to directly verbally express what they need. Their cultural values and norms may not reinforce them in assertively asking for what they want. In fact, they may be socialized to think more of what is good for the social group than of their individual wants. In working with people with these values, counselors must "soften" reality therapy somewhat. If reality therapy is to be used effectively with clients from other cultures, the procedures must be adapted to the life experiences and values of members from various cultures (Wubbolding, 2000, 2011a; Wubbolding et al., 2004).

Reality Therapy Applied to the Case of Stan



As a reality therapist, I am guided by the key concepts of choice theory to identify Stan's behavioral dynamics, to provide a direction for him to work toward, and to teach him about better alternatives for achieving what he wants. Stan has not been effective in getting what he needs—a satisfying relationship.

Stan has fallen into a victim role, blaming others, and looking backward instead of forward. Initially, he wants to tell me about the negative aspects of his life, which he does by dwelling on his major symptoms: depression, anxiety, inability to sleep, and other psychosomatic symptoms. I listen carefully to his concerns, but I hope he will come to realize that he has many options for acting differently. I operate on the premise that therapy will offer the opportunity to explore with Stan what he can build on—successes, productive times, goals, and hopes for the future.

After creating a relationship with Stan, I am able to show him that he does not have to be a victim of his past unless he chooses to be, and I assure him that he has rehashed his past miseries enough. As counseling progresses, Stan learns that even though most of his problems did indeed begin in childhood, there is little he can now do to undo his childhood. He eventually recognizes that all of his symptoms and avoidances keep him from getting what he most wants. He eventually realizes that he has a great deal of control over what he can do for himself now.

I have Stan describe how his life would be different if he were symptom free. I am interested in knowing what he would be doing if he were meeting his needs for belonging, achievement, power, freedom, and fun. I explain to him that he has an ideal picture of what he wants his life to be, yet he does not possess effective behaviors for meeting his needs. I talk to him about all of his basic psychological needs and how this type of therapy will teach him to satisfy them in effective ways. I also explain that his *total behavior* is made up of acting, thinking, feeling, and physiology. Even though he says he hates feeling anxious most of the time, Stan learns that much of what he is doing and thinking is directly leading to his unwanted feelings and physiological reactions. When he complains of feeling depressed much of the time, anxious

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at night, and overcome by panic attacks, I let him know that I am more interested in what he is *doing* and *thinking* because these are the behavioral components that can be directly changed.

I help Stan understand that his *depressing* is the feeling part of his choice. Although he may think he has little control over how he feels, over his bodily sensations, and over his thoughts, I want him to understand that he can begin to take different action, which is likely to change his depressing experience. I frequently ask this question, "Is what you are choosing to do getting you what you want?" I lead Stan to begin to recognize that he does have *some,* indirect control over his feelings. This is best done after he has made some choices about doing something different from what he has been doing. At this point he is in a better place to see that the choice to take action has contributed to feeling better, which helps him realize that he has some power to change.

Stan tells me about the pictures in his head, a few of which are becoming a counselor, acting confident in meeting people, thinking of himself as a worthwhile person, and enjoying life. Through therapy he makes the evaluation that much of what he is doing is not getting him closer to these pictures or getting him what he wants. After he decides that he is willing to work on himself to be different, the majority of time in the sessions is devoted to making plans and discussing their implementation. We both focus on the specific steps he can take right now to begin the changes he would like.

As Stan continues to carry out plans in the real world, he gradually begins to experience success. When he does backslide, I do not put him down but help him refocus. We then develop a new plan that we both feel more confident about. I am not willing to give up on Stan even when he does not make major progress, and Stan lets me know that my support is a source of real inspiration for him to keep working on himself.

I teach Stan about choice theory, and, if he is willing to engage in some reading, I suggest that he read and reflect on the ideas in *Counseling With Choice Theory: The New Reality Therapy* (Glasser, 2001) and *A Set of Directions for Putting and Keeping Yourself Together* (Wubbolding & Brickell, 2001). Stan brings some of what he is learning from his reading into his sessions, and eventually he is able to achieve some of his goals. The combination of working with a reality therapist, his reading, and his willingness to put what he is learning into practice by engaging in new behaviors in the world assist him in replacing ineffective choices with life-affirming choices. Stan comes to accept that he is the only person who can control his own destiny.

Follow-Up: You Continue as Stan's Reality Therapist

Use these questions to help you think about how you would counsel Stan using reality therapy:

- If Stan complains of feeling depressed most of the time and wants you to "fix" him, how would you proceed?
- If Stan persists, telling you that his mood is getting the best of him and that he wants you to work with his physician in getting him on an antidepressant drug, what would you say or do?
- What are some of Stan's basic needs that are not being met? What action plans can you think of to help Stan find better ways of getting what he wants?
- Would you be inclined to do a checklist on alcoholism with Stan? Why or why not? If you determined that he was addicted to alcohol, would you insist that he attend a program such as Alcoholics Anonymous in conjunction with therapy with you? Why or why not?
- What interventions would you make to help Stan explore his total behavior?



See *DVD for Theory and Practice of Counseling and Psychotherapy: The Case of Stan and Lecturettes* (Session 9 on reality therapy) for a demonstration of my approach to counseling Stan from this perspective. This session deals with assisting Stan in forming an action plan.

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SUMMARY AND EVALUATION

Summary

The reality therapist functions as a teacher, a mentor, and a model, confronting clients in ways that help them evaluate what they are doing and whether their behavior is fulfilling their basic needs without harming themselves or others. The heart of reality therapy is learning how to make better and more effective choices and gain more effective control. People take charge of their lives rather than being the victims of circumstances beyond their control. Practitioners of reality therapy focus on what clients are *able and willing to do in the present* to change their behavior. Practitioners teach clients how to make significant connections with others. Therapists continue to ask clients to evaluate the effectiveness of what they are choosing to do to determine if better choices are possible.

The practice of reality therapy weaves together two components, the counseling environment and specific procedures that lead to changes in behavior. This therapeutic process enables clients to move in the direction of getting what they want. The goals of reality therapy include behavioral change, better decision making, improved significant relationships, enhanced living, and more effective satisfaction of all the psychological needs.

Contributions of Reality Therapy

Among the advantages of reality therapy are its relatively short-term focus and the fact that it deals with conscious behavioral problems. Insight and awareness are not enough; the client's self-evaluation, a plan of action, and a commitment to following through are the core of the therapeutic process. I like the focus on strongly encouraging clients to engage in self-evaluation, to decide if what they are doing is working or not, and to commit themselves to doing what is required to make changes. The existential underpinnings of choice theory are a major strength of this approach, which accentuates taking responsibility for what we are doing. People are not viewed as being hopelessly and helplessly depressed. Instead, people are viewed as doing the best they can, or making the choices they hope will result in fulfilling their needs. With the emphasis on responsibility and choice, individuals can acquire a sense of self-direction and empowerment.

Too often counseling fails because therapists have an agenda for clients. The reality therapist helps clients conduct a searching inventory of what they are doing. If clients determine that their present behavior is not working, they are then much more likely to consider acquiring a new behavioral repertoire. Many clients approach counseling with a great deal of skepticism. Reality therapy can be used effectively with individuals who manifest reluctance and who are often highly resistant. For example, in working with people with addictions, reality therapy strategies can be used to help clients evaluate where their behavior is leading them and to provide clients with options to bring about positive changes in their behavior. Reality therapy has been effectively used in addiction treatment and recovery programs for more than 30 years (Wubbolding & Brickell, 2005). In many situations with these populations, it would be inappropriate to embark on long-term therapy

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that delves into unconscious dynamics and an intensive exploration of one's past. Reality therapy focuses on making changes in the present and is an effective, short-term approach, often in 10 sessions or less.

Limitations and Criticisms of Reality Therapy

From my perspective, one of the main limitations of reality therapy is that it does not give adequate emphasis to the role of the following aspects of the counseling process: the role of insight, the unconscious, the power of the past and the effect of traumatic experiences in early childhood, the therapeutic value of dreams, and the place of transference. Because reality therapy focuses almost exclusively on consciousness, it does not take into account factors such as repressed conflicts and the power of the unconscious in influencing how we think, feel, behave, and choose.

Dealing with dreams is not part of the reality therapist's repertoire. According to Glasser (2001), it is not therapeutically useful to explore dreams. For him, spending time discussing dreams can be a defense used to avoid talking about one's behavior and, thus, is time wasted. From my perspective, dreams are powerful tools in helping people recognize their internal conflicts. I believe that there is richness in dreams, which can be a shorthand message of clients' central struggles, wants, hopes, and visions of the future. Asking clients to recall, report, share, and relive their dreams in the here and now of the therapeutic session can help unblock them and can pave the way for clients to take a different course of action.

Similarly, I have a difficult time accepting Glasser's view of transference as a misleading concept, for I find that clients are able to learn that significant people in their lives have a present influence on how they perceive and react to others. To rule out an exploration of transference that distorts accurate perception of others seems narrow in my view.

As you will recall, Glasser (2001, 2003) contends that chronic depression and profound psychosis are chosen behaviors. Apart from specific brain pathology, Glasser argues that mental illness is the result of an individual's unsatisfying present relationships or general unhappiness. I have trouble viewing all psychological disorders as behavioral choices. People suffering from chronic depression or schizophrenia are struggling to cope with a real illness. In reality therapy these people may have additional guilt to carry if they accept the premise that they are *choosing* their condition.

I believe reality therapy is vulnerable to the practitioner who assumes the role of an expert in deciding for others how life should be lived and what constitutes responsible behavior. Wubbolding (2008) admits that reality therapy can lend itself to fixing problems and imposing a therapist's values on clients, especially by inexperienced or inadequately trained counselors. Wubbolding adds that it is not the therapist's role to evaluate the behavior of clients. Generally, clients need to engage in a process of courageous self-evaluation to determine how well certain behaviors are working and what changes they may want to make. It is critical that therapists monitor any tendency to judge clients' behavior, but instead to do all that is possible to get clients to make their own evaluation of their behavior.

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Finally, reality therapy makes use of concrete language and simple concepts. This can erroneously be viewed as a simple approach that does not require a high level of competence. Because reality therapy is easily understood, it might appear to be easy to implement. However, the effective practice of reality therapy requires practice, supervision, and continuous learning (Wubbolding, 2007b, 2011a). Competent reality therapists have a thorough understanding of choice theory and have mastered the art of applying reality therapy procedures to working with diverse clients with a range of clinical problems.

WHERE TO GO FROM HERE

In the *DVD for Integrative Counseling: The Case of Ruth and Lecturettes,* Session 8 ("Behavioral Focus in Counseling"), you will note ways that I attempt to assist Ruth in specifying concrete behaviors that she will target for change. In this session I am drawing heavily from principles of reality therapy in assisting Ruth to develop an action plan to make the changes she desires.

Free Podcasts for ACA Members

You can download ACA Podcasts (prerecorded interviews) at www.counseling.org; click on the Resource button and then the Podcast Series. For Chapter 11, Reality Therapy, look for Podcast 18, "Reality Therapy, Choice Theory: What's the Difference?" by Dr. Robert Wubbolding.

Other Resources

DVDs offered by the American Psychological Association that are relevant to this chapter include the following:

Wubbolding, R. (2007). *Reality Therapy*

Psychotherapy.net is a comprehensive resource for students and professionals that offers videos and interviews on demonstrating reality therapy working with addictions, adults, and children. New video and editorial content is made available monthly. DVDs relevant to this chapter are available at www.psychotherapy.net and include the following:

Wubbolding, R. (2000). *Reality Therapy* (Psychotherapy With the Experts Series)
Wubbolding, R. (2000). *Reality Therapy for Addictions* (Brief Therapy for Addictions Series)
Wubbolding, R. (2002). *Reality Therapy With Children* (Child Therapy With the Experts Series)

The programs offered by the William Glasser Institute are designed to teach the concepts of choice theory and the practice of reality therapy. More than 7,800 therapists have completed the training in reality therapy and choice theory. The institute offers a certification process, which starts with a 3-day introductory course known as "basic training" in which participants are involved in discussions, demonstrations, and role playing. For those wishing to pursue more extensive training, the institute offers a five-part sequential course of study leading to certification in reality therapy, which includes basic training, a basic practicum, advanced training,

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an advanced practicum, and a certification week. This 18-month training program culminates in a Certificate of Completion. Complete information on this program can be obtained directly from the institute.

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The *International Journal of Choice Theory and Reality Therapy* (online journal) focuses on concepts of internal control psychology, with particular emphasis on research, development, and practical applications of choice theory and reality therapy principles in various settings. To subscribe, contact:

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Recommended Supplementary Readings

Counseling With Choice Theory: The New Reality Therapy (Glasser, 2001) represents the author's latest thinking about choice theory and develops the existential theme that we choose all of our total behaviors. Case examples demonstrate how choice theory principles can be applied in helping people establish better relationships.

Reality Therapy (Wubbolding, 2011a) updates and extends previous publications on choice theory and reality therapy. As a part of the APA theories of psychotherapy series, this is a well-written and comprehensive overview of reality therapy and choice theory.

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Case Approach to Counseling and Psychotherapy (Corey, 2013) illustrates how prominent reality therapists Drs. William Glasser and Robert Wubbolding would counsel Ruth from their different perspectives of choice theory and reality therapy.

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*Books and articles marked with an asterisk are suggested for further study.

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CHAPTER 16

Supportive Psychotherapy

ARNOLD WINSTON
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Although supportive psychotherapy techniques are more widely used than expressive or insight-oriented techniques, supportive psychotherapy has been under represented in the literature and in psychotherapy research. This has been particularly true in the application and study of supportive therapy in personality disorder. It has been widely held that supportive treatment is only suitable for chronically or severely ill patients and that it is inferior to expressive approaches.

In 1954 Wolberg described supportive therapy as a treatment for "those with good ego strengths who have broken down under the impact of excessively severe environmental pressures and stresses" (p. 101) and also for those "with weak ego structures whose capacities for real change are minimal and who are unable to endure the anxieties inevitably associated with deeper therapy" (p. 101). In 1979, Binstock sounded a stronger negative note:

> Unfortunately, the concept of "support" serves as a ready rationalization for gratifying countertransference wishes to play the role of parent inappropriately. Active efforts to be especially "warm," "giving," "real," "directive," or simply "instructive" convey to the patient that he is especially needful of such efforts. As a result, he feels patronized, infantilized—with the result that he is undermined in his self-confidence and self-esteem, rather than supported. (p. 608)

Franz Alexander (1953) was one of the first to attempt to correct this negative image, stating that *"it is widely but erroneously held that supportive psychotherapy methods require less technical and theoretical preparation than psychoanalysis"* (emphasis added; p. 117). Wallace (1983) summarized the argument that the competent practice of supportive therapy is more difficult than the practice of expressive or uncovering therapy. He wrote that generally, when supportive therapy is indicated,

> the patient's pathology is more severe, there is a wider range of possible responses by the therapist, and it is difficult to decide which response is correct. You cannot wait for the patient to make connections. . . . You must decide . . . now to come down on the side of expressiveness, now of restraint, now to confront his intellectualization or reaction formation, now to support it, now to analyze the transference, now to utilize it as a suggestive or reinforcing lever . . . now to ask him what goes into his question, now to answer it immediately and directly, now to gratify his request for coffee or advice, now to analyze it. (pp. 345–346)

Pinsker, Rosenthal, and McCullough (1991) characterized supportive therapy as "a dyadic treatment which uses direct measures to ameliorate symptoms and maintain, restore, or improve self-esteem, adaptive skills, and psychological function. To the extent necessary to

accomplish these objectives, treatment may utilize examination of relationships, real or transferential and both past and current patterns of emotional response or behavior" (pp. 221–222). Change is seen as stemming from learning and from identification with or introjection of an accepting, well-related therapist, not through resolution of unconscious conflicts. Change is not a product of self-understanding or analysis of transference, but rather it is a direct consequence of better self-esteem and improved adaptive skills. Distortions about self and other are corrected by education. Self-understanding, although not central to the treatment, may be pursued to the extent that is supports the accomplishment of patient goals and therapist objectives. By this conceptualization, intellectual insight, although often dismissed as a manifestation of suboptimal therapy, is a satisfactory end point.

RATIONALE

Using a dynamic or analytic framework, supportive psychotherapy can be conceptualized at one end of a continuum with expressive psychotherapy at the other end (Dewald, 1971). In the middle of the continuum is a supportive–expressive approach which combines elements of both supportive and expressive psychotherapy (Luborsky, 1984).

Traditionally, supportive psychotherapy has been used with severely impaired or chronically mentally ill patients, while expressive psychotherapy generally has been indicated for more structurally intact patients with stable and cohesive psychological or ego functions. However, we have treated patients in supportive psychotherapy who improved clinically despite meeting traditional research criteria for expressive treatment. The vast majority of these patients had personality disorder, primarily of the Cluster C type.

The psychopathology of personality disorder involves "an enduring pattern of inner experience and behavior" (American Psychiatric Association, 1994), as manifested in cognition, affectivity, interpersonal functioning, and impulse control. Because supportive psychotherapy is flexible and comprehensive it can be applied to all kinds of psychopathology, including the typical problems of personality disorder. Supportive therapy typically targets dysfunctional thinking, interpersonal conflict and functioning, affect modulation, and frustration tolerance.

THEORETICAL FOUNDATIONS

It is now widely recognized that the patient–therapist or therapeutic relationship is a key ingredient of psychotherapy (Frank & Frank, 1991). Greenson (1967) suggested that the therapeutic relationship consists of three components: a transference–countertransference configuration, a real relationship, and a working or therapeutic alliance. Whether these components can be clearly separated is open to debate (Greenberg, 1994; Hoffman, 1991), but these constructs are useful in conceptualizing many psychotherapies and in distinguishing supportive from expressive psychotherapy. The transferential relationship in latent or manifest form is the pattern of reflexive attitudes, thoughts, and emotional responses which may be currently maladaptive and directly related to intrapsychic processes from an earlier time in psychosocial development. In expressive psychotherapies, this relationship is deemed to be of paramount importance for revealing conflicts, and therapeutic gain is ascribed to the essentially noncognitive process of working through transferential relationships.

The real relationship is universally recognized and forms the context of all treatment, including expressive therapy. This relationship is manifested in the therapeutic alliance and coexists with, and is to some extent reflective of, the transference relationship. For example, what appears on the surface to be a positive therapeutic alliance may be bolstered, without the patient's awareness, by the fact that dependency needs are gratified in the transference relationship.

In expressive treatment, the real relationship becomes a background or context within which the therapist responds mostly to the transference nature of the interpersonal process. There is a conscious minimization of real information about the therapist in the therapist's statement to the patient. Thus, much of the real relationship and therapeutic alliance depends on the acceptance by the patient of the rules and agreements on the conduct of treatment and its relationship to the therapist.

Supportive psychotherapy, on the other hand, emphasizes the real relationship as reflected in the therapeutic alliance. This process is based

on overt mutuality in the conduct of therapy. The relationship between therapist and patient is a mirror of other current relationships. This relationship contrasts to the socially unusual and anxiety-provoking neutral stance of the therapist in expressive treatments. The therapeutic alliance in supportive therapy is supported through the use of accurate empathic responses, validation of feeling states, and attention to detail, but development of transference neurosis is avoided. To that end, there is minimization of focus on transferential material, and "regression in service of ego" is not fostered. The fact that transference is not discussed does not mean that it is not recognized. Negative transference can threaten the alliance and the treatment, so the therapist must be vigilant about recognizing it and dealing with it. With higher-functioning patients, clarification of evidences of negative feelings or thoughts may be productive. With lower-functioning patients, it may be necessary for the therapist to change his stance, as people usually do when talking with someone who is becoming angry or distant.

In brief expressive psychotherapy, development of the transference neurosis is avoided by actively clarifying and confronting defenses and interpreting the transference. Supportive therapy does not confront defenses unless they are maladaptive (primitive projection, splitting, etc.). Although supportive psychotherapy is dynamic in that the therapist pays attention to transference material and recognizes primitive defenses, the therapist's responses are most likely to be within the domain of the real relationship.

Expressive therapies have a primarily intrapsychic focus with respect to the therapist's attention to and interaction with patient material. The therapist and patient look at the conflicts between the patient's mental constructs of id, ego, and superego or, stated in developmental terms, conflict between the inner representations of self and others. The therapist's persistent attention to these conceptual frames during the process of treatment assists the patient in being aware of and then consciously contributing to material with this focus in mind.

In contrast, supportive psychotherapy has a predominately interpersonal focus in that adaptive strategies, coping skills, and anxiety reduction are attended to within a frame of reference that looks at patterns of interpersonal behavior. The relationship between the patient and therapist is used to teach the patient about difficulties in transactions with other people, with the intent of improving the quality of the patient's relationships and diminishing maladaptive personality traits.

Other psychotherapies that are not psychodynamically based use techniques that are largely interpersonal or supportive. For example, cognitive-behavior therapy employs assertiveness training, social skills training, cognitive restructuring, and so on, all of which fit comfortably into supportive psychotherapy. Based on this common use of supportive techniques, Pinsker (1994) suggested another way of conceptualizing supportive therapy using the computer term "shell program." A shell program is one that fits over another program and is easier to use. In the same way supportive therapy can be considered a "shell" that fits over most theoretical orientations. The theoretic constructs of, for example, dynamic or psychoanalytic, cognitive-behavioral, and experiential are different, but they all use supportive techniques. Using a shell approach therefore implies the employment of supportive techniques in a theoretical manner, or that different theoretical orientations can encompass a supportive approach.

In this chapter we are guided by psychoanalytic concepts, but we describe supportive techniques derived from other orientations.

A SUPPORTIVE MODEL OF CHANGE

What constitutes change in supportive psychotherapies has been articulated in a number of different ways. As we have described elsewhere (Hellerstein, Pinsker, Rosenthal, & Klee, 1994; Pinsker & Rosenthal, 1988; Winston, Pinsker, & McCullough, 1986), change in supportive therapy can be understood as learning new adaptive behaviors and skills while ameliorating symptoms and enhancing self-esteem. We have suggested that such change stems from identification with or introjection of an accepting, well-related therapist. Likewise, Wallerstein (1989) has stated that the major operative supportive mechanism is the evocation of a positive dependent transference attachment, which serves as the basis for an enduring "transference cure." Holmes (1995) has seen many borderline patients make great use of the commitment, attention, and concern of supportive technique in psychoanalytic treatment, and

he suggests that it is the consequent development of secure attachments that fosters more autonomous functioning in them. Similarly, Buckley (1994) has postulated that a Kohutian empathic–introspective stance is a critical component of effective supportive psychotherapy.

We have also described change in supportive therapy as lasting and substantive because it includes changes in the dynamic structures, in specific intrapsychic configurations, such as defensive operations, thought and affect organization, anxiety tolerance, and ego strength, assumed to underlie and motivate behavior (Rosenthal, Muran, Pinsker, Hellerstein, & Winston, 1999). In the Menninger Psychotherapy Research Project, a naturalistic clinical study, Wallerstein (1989) found lasting and significant structural changes in patients treated with a dynamically based treatment, which although not well defined, was classified as supportive therapy. In the Menninger project, structural change was defined "as changes in specific intrapsychic configurations, in the patterning of defenses, in thought and affect organization, in anxiety tolerance and in ego strength" (p. 203). De Jonghe, Rijnierse, and Janssen (1994) and colleagues have proposed a "post-classical" view of change in supportive psychotherapy, characterized as "growth by experience," which supplants the notion of structural change as solely due to "growth by insight." This perspective invokes a view of change originally sowed by Ferenczi (1932) and later cultivated by Balint (1968), Alexander (Alexander & French, 1946), and Winnicott (1965).

GENERAL PRINCIPLES OF CHANGE

In this section we present some general principles of change that are central to our model of supportive psychotherapy for personality disorders.

Ameliorate Symptoms

Supportive psychotherapy can be characterized by the use of direct measures to ameliorate symptoms, which are described in detail later. Unlike more expressive approaches, which primarily focus on unconscious conflicts and making the unconscious conscious, our supportive approach places primary emphasis on helping patients with their presenting symptoms and complaints.

Minimize Anxiety

Another general principle of supportive psychotherapy is minimizing patients' experience of anxiety and maximizing their sense of control. There are a number of ways that this principle can be fulfilled, and Pine (1984) has described many strategies toward this end. The essence of supportive psychotherapy is not specific supportive techniques but a continuous concern about patient anxiety and a deliberate effort by the therapist to avoid subtle actions that might increase anxiety.

Enhance Self-Esteem

Likewise, the essence of supportive psychotherapy is a continuous concern for patient self-esteem. As we have previously noted (Pinsker et al., 1991), it is interesting how little formal attention has been given to protecting and enhancing self-esteem in psychotherapy. One of the ways in which we encourage supportive therapists in this regard is to watch for questions that have a challenging or critical impact. For example, questions that begin with the word "why" often run the risk of suggesting the meaning, "You shouldn't have done that!"

Respect Defenses

Unlike in expressive forms of psychotherapy, which typically aim to get rid of character defenses so that the core neurosis is exposed, in supportive psychotherapy, adaptive defenses and patient's personal style are generally respected. For example, the individual whose defense is maintaining control over emotions should not necessarily be asked to relax this control, unless there is good reason to believe that lack of expression is a significant problem. Although defenses in general are to be supported, by this we usually mean mature defenses; this support stops when the defense is maladaptive or immature, such as regression, denial, and projection.

Facilitate Adaptive Skills and Psychological Functions

An important principle of supportive psychotherapy involves facilitating the patient's

adaptive skills and psychological functions. By adaptive skills, we simply mean almost anything a person does to function more effectively. By psychological functions, we mean ego functions, including reality orientation, defense formation, regulation of affect, and so on. The boundary between adaptive skill and psychological function is not sharply defined. The patient's construal of events reflects psychological function; the action he or she takes in response reflects adaptive skill.

STRATEGIES AND TECHNIQUES

Below is a list of specific techniques used in our model of supportive psychotherapy to fulfill the general principles described previously.

Establishing Definition and Agreement on Tasks and Goals

It is a clear mandate of supportive psychotherapy for the therapist to be explicit about the tasks and goals: that is, the way in which therapist and patient will work, providing a clear rationale, and the direction in which they are headed. Our supportive psychotherapy mandates a meeting of the minds between patient and therapist about the tasks and goals. This mandate echoes Bordin's (1979) definition of the working alliance. We find that it is often helpful, for example, to make explicit how the topic at hand is connected to self-esteem, to a specified adaptive skill or psychological function. This mandate goes a long way toward minimizing the anxiety in the therapeutic situation.

Style of Communication

One way anxiety may be minimized is by the use of a conversational style. We do not mean to suggest that therapy is ordinary conversation but that the interaction between therapist and patient is modeled on conversation rather than interrogation or silent listening. Such conversation differs from normal social conversation in that there is always a clearly defined purpose and focus.

Clarification, Confrontation, and Interpretation

These techniques are often used in supportive psychotherapy, but not with the requirement that the unconscious be made conscious or that full linkages be made with impulses or affects connected with genetic figures. Clarification is used extensively in terms of summarizing, paraphrasing, and organizing the patient's statements without elaboration or inference. Such clarification provides the patient with critical evidence that therapist is actively listening. Confrontation and interpretation, which bring to attention a pattern of behavior or something that the patient is avoiding or not attending to, can be usefully employed within supportive psychotherapy, but with the following considerations: the minimization of anxiety, the maintainance of self-esteem, and the maximization of adaptive functioning. One guiding principle that is often useful with respect to confrontation and interpretation in supportive psychotherapy is one put forth by Pine (1984): Strike when the iron is cold.

Responding to Ventilation

Ventilation may be useful to a patient when a traumatic event is experienced or when something important has been unexpressed. The fact that the therapist has heard the patient's story and is not rejecting may be the essence of support for some. The therapist's active responses may include tracking (indicating the he or she is following the patient), universalizing (making it clear that many people have similar feelings, wishes, or problems), or decatastrophizing (minimizing issues or problems that the patient has exaggerated).

Encouragement

Encouragement may include praise, reassurance, and empathic comments. Most therapists have learned to offer empathic comments as responses to particularly difficult situations (e.g., "That must have been real hard for you."). We try to go further, though, and find opportunities to add words that tell the patient something about him- or herself, but not in a disengenuous way (e.g., "That took a lot of courage!"). The driving force of this strategy is to support self-esteem.

Advice and Suggestion

Advice and suggestion must be factual, related to the therapist's expert knowledge, and limited to the topics of therapy. Advice may be relevant

if it is designed to help the patient act in a way that will enhance his or her self-esteem and improve adaptive skills or psychological function. The basis and rationale for the advice must always be stated. It is the therapist's expertise, not his or her authority, that is crucial.

Rationalization

Rationalization can be a legitimate technique if done knowingly and for a good reason. For example, the adult patient who dwells excessively on what her parents did wrong may benefit from a statement such as, "Your parents seem to have been very rigid and cold, but you know, they were doing what the experts taught was the most scientifically correct way to raise children in the 1930s; they may have been doing the best they could."

Reframing and Reattribution

These strategies have often been described within a cognitive-behavioral framework (e.g., Freeman & Simon, 1989). Reframing can be used to assist the patient in diffusing or sidestepping painful affects or negative references, thus enhancing self-esteem. Reattribution typically involves the therapist effecting a more reasonable distribution of responsibility when the patient places sole responsibility for his or her difficulty on self or others.

Modeling

By modeling we refer to the therapist's providing, intentionally or unintentionally, a model of behavior and responsiveness. In a sense, therapists lend their ego to the patient in the course of treatment, by applying their problem-solving skills, affective responsiveness, and knowledge of individual and social behavior to the patient's problem. Another technical issue that can be subsumed under modeling is self-disclosure. As a teacher and role model, the therapist may reveal certain attitudes, ideas, and values. Such disclosure enables patients with severe ego deficits to gradually build more stable and cohesive self and object representations.

Anticipatory Guidance and Rehearsal

Anticipatory guidance allows the patient to move through new situations hypothetically, considering the possible events and ways of responding to them. This allows the patient to become acquainted with the context of the future event, reducing some of the anticipatory anxiety associated with it. Rehearsal further allows the patient to work out more appropriate or novel ways to participate in future events, thus adding to his or her repertoire of adaptive skills.

Developing Alternatives and Problem Solving

These strategies have also been described by cognitive-behaviorial therapists (e.g., Freeman & Simon, 1989). When a patient has alternatives for thinking or behaving, he or she has greater freedom of choice. An anxious style of responding can sometimes be attributed to seeing only one choice rather than many. When an anxious patient can see other options, he or she often assumes a greater sense of control over thoughts and actions. Likewise, educating a patient on how to solve a problem (brainstorming, weighing advantages and disadvantages, etc.) can create a greater sense of mastery.

PROCESS ISSUES INCLUDING COUNTERTRANSFERENCE

Many of the process issues have been elaborated in other sections of this chapter. Thus this section is confined to just a few issues. Typically supportive therapy uses a conversational style, coupled with a high activity level on the part of the therapist. Silence is avoided and the therapist attempts to make the patient feel comfortable by minimizing anxiety. However, at other times especially with personality disorder patients, the therapist may assume an exploratory position, confront maladaptive defenses, engage in relaxation training, teach assertiveness skills, or work on dysfunctional thinking. Therefore, therapists engaging in supportive psychotherapy with patients who have significant personality problems must be flexible and comfortable moving across different technical approaches. Therapists who are rigid and unable to engage difficult patients will have problems with this population.

In this type of therapy, as in others, countertransference issues must be recognized and attended to. Countertransference issues in supportive psychotherapy can be divided into those that particularly relate to the nature of supportive work and common countertransference re-

actions to the characterological problems of patients with personality disorder. This second category occurs in most psychotherapies and we do not focus on them here.

The first category derives from the nature of supportive psychotherapy. The therapist feels pressure from both the treatment and the patient to "take over" the patient's life to some extent. This can lead to therapist reactions of two types. The first therapist countertransference reaction involves the development of grandiose or arrogant thoughts and is reflected in behaviors toward the patient. Therapists who move into the role of directing or taking over a patient's life might find themselves behaving in a condescending manner and be unwilling to relinquish control over the patient. This could close off or inhibit patient growth and individuation, making the therapist more inclined to hold on to the patient and to not see the indications for termination from treatment. The second therapist reaction would be to defend against the patient's need for direction by becoming frustrated and angry or by withdrawing and distancing from the patient. It is incumbent upon the therapist to consistently monitor his or her feelings and behaviors toward the patient to both prevent untoward behaviors toward the patient and to use countertransference feelings to better understand the character pathology and needs of the patient.

INDICATIONS, CONTRAINDICATIONS, AND EXPECTED RESULTS

The traditional indication for supportive psychotherapy has been the presence of chronic severe mental illness (Drake & Sederer, 1986; Kates & Rockland, 1994), a condition that generally contraindicates uncovering-type treatment. There are also other clinical indications for supportive psychotherapy based on coping with a diagnosis or situation: early sobriety from alcohol or drugs of abuse (Kaufman & Reoux, 1988; O'Malley et al., 1992), acute crisis (Dewald, 1994), acute bereavement in patients with poor ego strength (Horowitz, Marmar, Weiss, DeWitt, & Rosenbaum, 1984), and medical illnesses acute and chronic (Alter, 1996; Markowitz et al., 1995; Massie & Holland, 1990). Diagnostic or situation-specific indications for supportive theerapy are thus described, and there is agreement about the utility of supportive psychotherapy in these areas, yet there has been little experimental validation through well-controlled randomized clinical trials. There has been little in the way of clinical guidelines to either suggest or reject the use of supportive psychotherapy in patients with personality disorders, yet in everyday clinical practice, this is the type of treatment that most patients with more severe personality disorders have been given in mental health clinics.

As we elucidate later, we believe that there is a broader range of disorders that are amenable to supportive treatment, including personality disturbance. However, in our experience, among the various personality disturbances, help-rejecting complainers and liars and patients with typical problems of hostile dominance do as poorly in supportive psychotherapy as they do in other predominantly dynamic expressive treatments. As such, patients with core psychopathy as a subset of antisocial personality disorder are unlikely to make constructive use of supportive psychotherapy. The controlling and hostile elements of these patients' interpersonal styles most likely interferes with the formation of a solid working alliance and thus precludes the initiation of task oriented treatment.

CLINICAL ILLUSTRATIONS

The two cases presented here briefly illuminate, with historical and clinical data, a graphical representation of change mapped to the Inventory of Interpersonal Problems (IIP; Horowitz, Rosenberg, Baer, Ureno, & Villaseñor, 1988; MacKenzie, Chapter 14, this volume) circumplex model in Figures 16.1 and 16.2, and link the theoretical basis for making specific supportive interventions with specific changes in patients. Interpersonal behavior can be measured based on the circumplex model (Henry, Schacht, & Strupp, 1986, 1990; Kiesler & Watkins, 1989) originally formulated by Leary (1957) and developed subsequently by Benjamin (1974; see Benjamin & Pugh, Chapter 20, this volume), Wiggins (1979), and Kiesler (1983). In the circumplex model, interpersonal behavior and attitudes are mapped on a surface defined by a horizontal "affiliation" axis reflecting with poles of friendliness and hostility, and a vertical "control" axis with poles of dominance and submission. The eight regions between the axes are labeled in this IIP analysis: domineering, vindictive, overly cold, avoidant,

nonassertive, exploitable, overly nurturant, and intrusive (Alden, Wiggins, & Pincus, 1990).

Case 1

A 38-year-old separated white man had initial target complaints consisting of being bothered greatly by his silence and withdrawal in dealing with people, severe lack of confidence about decisions made, and chronically low self-esteem. He would become extremely anxious at the workplace, which interfered with his concentration and ability to perform his work as a cashier. He had a history of childhood physical abuse by his distant, domineering, critical, and unpredictable alcoholic father. He believed that his father disliked him. Only at the end of his father's life was the patient able to achieve some degree of closeness with him. He had considered his relationships with his mother and his aunt as warm and close while growing up. He currently felt that the relationship with his mother was cool and strained and that interactions often ended in arguments. He believed that his mother preferred his younger sister's company and was more concerned about her welfare than his. Throughout his life, the patient generally felt inferior to those around him and had always been rejection sensitive with difficulty tolerating criticism. Apparently as a result, he developed an obsequious and distant interpersonal style. However, during his short marriage (3 years), when pressed to the limit of his capacity to contain anger verbally, he would become physically aggressive toward his wife. He also had little interest in sex. He had few friends and was generally aloof in all of his relationships. On screening with the Structured Clinical Interview for DSM-III-R (SCID-I; SCID-II) he met criteria for diagnoses of primary dysthymic disorder and avoidant personality disorder and subthreshold criteria for self-defeating personality disorder. On the subscales of the IIP at intake, he had a prominent concentration of problems in the avoidant, nonassertive, exploitable, and overly nurturant octants, consistent with the initial clinical descriptors and diagnoses (see Figure 16.1).

Conduct of the Treatment

Enhancement of self-esteem, a core principle of supportive psychotherapy, was a focal issue with this patient. The therapist hypothesized that patterns of rejection, criticism, abuse, and impersonal treatment from early caretakers promoted the patient's current expectancy that he would be taken advantage of in interpersonal situations and could not risk advocating for his own needs. The therapist purposely looked for opportunities to support the patient's self-esteem in a way the patient was able to use. This meant reinforcing a sense of empowerment without being perceived by the patient as gratuitous or, as in the case of this rejection-sensitive patient, demeaning. Even approaching the issue with some patients may place the patients in an anxious, self-invalidating state. If it

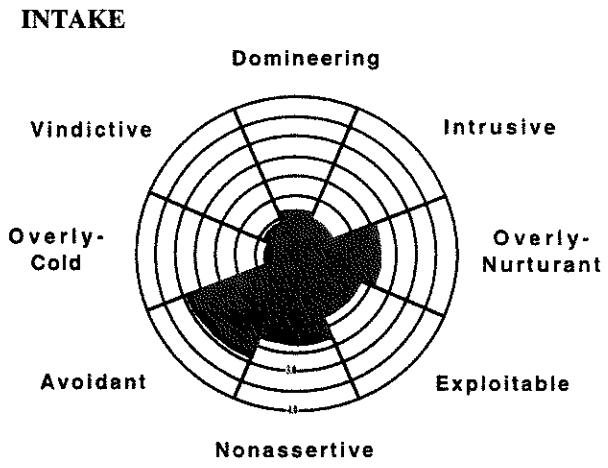


FIGURE 16.1. Scores at intake mapped on the eight circumplex subscales from the 64-item Inventory of Interpersonal Problems.

is hard for the patient to talk about something, consider asking, in a nonchallenging way, "How can we talk about this without increasing your anxiety, or without making you feel that you're being pressured?" (Pinsker & Rosenthal, 1988). The following is a therapy content and process example of how the therapist, using supportive techniques, carefully approached and focused on the issue of the patient's anxiety and experience of inadequacy at work and resulting avoidance of work.

P: I just feel overwhelmed all the time and I don't know how to get out of it, it's confusing . . . [Reiterates one of his chief target complaints.]

T: Maybe we can explore one of those target complaints, your being very anxious at work. [Focuses and clarifies the patient's statement.

P: Jeez, I don't know, it's kinda, you know, too much, and then I feel like an idiot. Like now. And you want an answer. [Becomes anxious, anticipates rejection, experiences decreased self-efficacy.]

T: Maybe there is some way we can discuss this where you don't feel overwhelmed, or pressured into giving an answer. [Gives an empathic suggestion which directly aims to empower the patient. Increasing his sense of control may also reduce or bind anxiety.]

P: Maybe . . . (*Sighs.*) Okay, let me think about it . . . [Takes on consideration of the problem from a less overwhelmed position.]

This patient's pathological avoidance was predicated on the patient's experience of being overwhelmed by self-doubt, anxiety, and fear of closeness.

P: I can hardly get to work. My supervisor knows I'm, . . . I'm not good at the job (*looks away*). [States two observations.]

T: So you want to avoid getting there because its so unpleasant for you? [Clarifies empathically.]

P: Yeah (*sheepishly*). [Agrees.]

T: I'm not clear on what you mean by not good at the job, is it okay with you to tell me how? [Asks if it is tolerable for the patient to clarify his statement.]

P: (*nodding*) I take too much time with the transactions because it's hard to concentrate. I get distracted. [Elaborates.]

T: Because you're anxious? [Demonstrates knowledge based on his real experience of the patient.]

P: Uh huh.

T: You stated that the supervisor has spoken to you about your performance. [Poses a clarification of the patient's earlier statement.]

P: Well, no, I mean, he hasn't actually said anything, (*shrugs*) but he must know. [Refutes his earlier observation, reality tests.]

T: People who are self-conscious and upset about a blemish or a pimple are often surprised to find out that they are the only ones who notice! (*Smiles.*) [Offers a humorous analogy that does not take the patient's actual performance into account.] Even if you were a little slower than some others, it would be unlikely that he knows that you are anxious inside. [Miminizes; supports adaptive defenses.]

P: He's not all that smart.

T: I wonder if you see any other situations where maybe you assume people know more than they really do?

P: Yeah, like, everything! (*Laughs.*)

The therapist continued to make anxiety-reducing interventions throughout the treatment. The clear focus on maintaining and improving self-esteem through the use of accurate empathic statements, clarifications and rationalizations aimed at increasing a sense of control, and support for adaptive defenses had beneficial results. At follow-up 6 months after the termination of therapy, the patient was bothered only a little by social withdrawal and low confidence in dealing with people. He had become more assertive, less avoidant, and less focused on having to please others. This is mirrored clearly in the diminution of the scores in the avoidant, nonassertive, and overly nurturant octants of the IIP. He was bothered very little by self-esteem problems. Although the concentration of interpersonal problems favored the submissive hemisphere, the entire set is markedly reduced in intensity. In addition, there is a relative shift toward the friendly nurturant side (see Figure 16.2).

Case 2

The second case is a 43-year-old single female freelance music teacher and musician with a

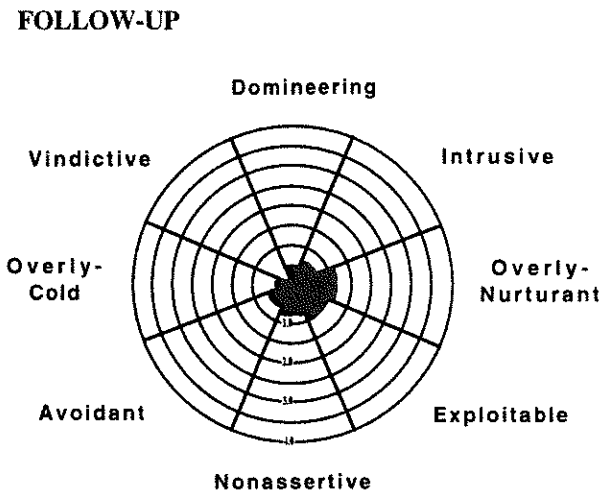


FIGURE 16.2. Scores at follow-up mapped on the eight circumplex subscales from the 64-item Inventory of Interpersonal Problems.

history of atypical depression characterized by symptoms of low energy, overeating and oversleeping, mood reactivity, and sensitivity to rejection and criticism. She was bothered a great deal by her difficulty controlling her weight and inability to complete tasks and to bring plans and ideas to fruition. She remembered feeling "fat" in kindergarten, which she would handle by starting new projects about which she could feel optimistic and good. Typically, the enthusiasm would fade after a time and she would not achieve her goals, a pattern that had persisted into adult life. She attributed her failure to follow through to a fear of exposing too much of herself, as she is sensitive to criticism. She also was quite disturbed by her self- and other-critical behavior. She attributed her being overly critical to parents who were overly critical of each other and her and to having studied music with a critical teacher. She was extremely concerned that she worried too much about her family's and other's reactions to her. She thought that she was not competitive enough with her peers. She believed that she subordinated her own needs in order to maintain attachments, and as a result, was not competitive, especially in her career. She fit SCID-II criteria for simple phobia, personality disorder not otherwise specified, and met subthreshold criteria for dysthymia. About 1 year before treatment began, a former lover had died and she had been grieving since that time. She had a recent 4-year relationship that she described as intense but very isolating. It ended badly, and the patient found out that he was a serious liar. She described her maternal grandparents as close and nurturing, but her father as distant, awkward, and unable to be intimate with her. She described herself as warm and gregarious in one-on-one situations or at social gatherings with friends but awkward and uncomfortable in work-related group settings with colleagues, worried that she would make a fool of herself. She considered herself a devoted friend. She enjoyed the teaching aspects of her work and the connections she built with her students. She attributed the genesis of her musical career to the development of a crush on her music teacher while learning the flute in junior high school. At intake, she demonstrated a concentration of problems on the IIP subscales in the nonassertive, exploitable, and overly nurturant areas, consistent with her experience of having to sacrifice her needs in order to maintain connections. She had difficulty expressing anger, holding to her position, and competing in a straightforward way, again, consistent with her own description (see Figure 16.3).

Conduct of the Treatment

As in case 1, enhancement of self-esteem is a focal issue here, with emphasis on empowerment and motivation for acting in her own interest. Two other foci are (1) increasing ego functioning, specifically cognitively reframing

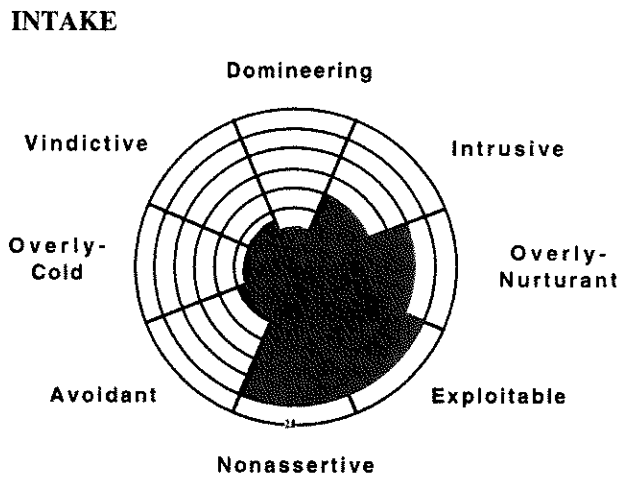


FIGURE 16.3. Scores at intake mapped on the eight circumplex subscales from the 64-item Inventory of Interpersonal Problems.

her perception/expectation that she is or will easily be the subject of ridicule; and (2) development of more adaptive skills, specifically using anticipatory guidance and rationalization to reduce *in situ* anxiety. The therapist hypothesized that an early temperamental vulnerability to dysphoria attendant to real or experienced separation set up interactive patterns which promote the patient's expectation that she dare not risk advocating for her own needs. The therapist looked for opportunities to support the patient's use of accurate estimation of the risk of rejection, while also supporting means by which the patient could reduce her anxiety and negative thoughts about herself. The therapist must support the patient's motivation to pursue her own goals in the face of possible rejection or loss. The real art in this case lay in empowering the patient to risk being more assertive and feeling good about it without the patient feeling "one down" with the therapist.

P: I have another staff meeting this week. I don't even want to show up. [Reiterates one of her chief target complaints.]

T: Maybe we can explore this more, since you've said that this is an example of where you want to be less passive. [Focuses and clarifies from the patient's own statements.]

P: Ughh. I usually just sit there, stewing in my own anxiety. I can hardly look at any one. [Anticipates rejection, experiences decreased self-efficacy.]

T: So, this is maybe where you feel they are looking at you critically? [Joins with her experience, asks an empathic question as a hypothesis.]

P: It sure seems that way—I don't know. I just get so embarrased, like I'm a fake, and they'll find out. [Allows that the criticism by others is anxiously anticipated, but not actually experienced in this case.]

T: . . . and meanwhile you experience yourself as embarrased and not strong. Sitting like that is not good for your self-esteem bacause it confirms your fears . . . [Suggests to the patient that she might be able to manage her her experience of anxiety.]

P: You mean maybe I could do something different in there? [Allows that perhaps they can explore alternatives, in spite of her previous experiences.]

T: One way people use to handle tricky or difficult situations is to rehearse it. Driving a car is complex and makes people anxious when they first get behind the wheel. [Proposes anticipatory guidance. Repetition serves mastery.]

P: But, I never know what someone is going to say, and I always expect the worst, so how can I prepare? [Reiterates an at-risk, anxious position, outwardly focused.]

T: But I'll bet you know pretty much how *you* think and feel, even when no one is talking directly to you (*empathic smile*). [Clarifies

and directly supports self-esteem: that he believes the patient has the capacity to change.]

P: (*Pause*) Oh, how I'll handle *ME!* (*Laughs.*) [Gives positive feedback to therapist that she understood the therapist and is willing to look at herself.

T: Can we look at what happens during the meeting? When does your anxiety build up, any particular time or topic? [Focuses patient on tracking her experience more specifically, but allows for her to back off if she feels not ready.]

P: When they start pitching new ideas for teaching, I'm afraid they'll criticize . . .

T: I'm not clear on something. May I ask you to clarify whether this is something you anticipate or something you have generally experienced from the group? [Asks patient in a supportive manner to clarify her experience.]

P: Sure, No, they don't ever get critical because I never offer my ideas. I actually have plenty (*smile*). [Clarifies her experience and reveals her creative side.]

T: So, maybe this is about learning to offer what you think, in spite of the fear? Once people learn to drive, it's far less threatening even though there are real risks . . . [Offers a learning model with a rationalization to help bind anxiety.]

Continued reiteration of the therapist's direct support for the patient's testing reality against anticipated rejection helped her to learn to negotiate tasks that were previously overwhelming for her. Based partly on the therapist's use of reframing and rationalization, and modeling of anticipatory strategies, the patient was able to develop a capacity for positively evaluating her attempts to address high-anxiety situations. She was better able to take some responsibility to reduce her experience of vulnerability as an act of "mental hygiene." At termination she was better able to compete at work and make reasonable demands of those around her, both at work and in her personal life. There was a clear decrease in most areas of interpersonal difficulty, including a reduction in her critical behavior. Brief supportive psychotherapy had a clear impact at follow-up on her overall position with respect to interpersonal space. Though still somewhat reticent and critical, she had become less overly friendly at the expense of autonomy and had become more assertive (see Figure 16.4).

EMPIRICAL EVALUATION OF SUPPORTIVE THERAPY FOR PERSONALITY DISORDERS

Although there has been some research demonstrating the treatment efficacy of supportive psychotherapies (e.g., Conte & Plutchik, 1986; Rockland, 1993; Wallerstein, 1989; Winston et al., 1986), very little has been published on supportive psychotherapy specifically for personality disorders. From our own brief psy-

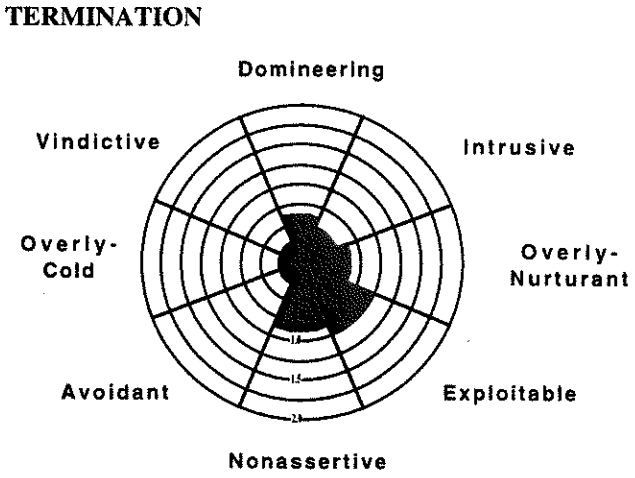


FIGURE 16.4. Scores at termination mapped on the eight circumplex subscales from the 64-item Inventory of Interpersonal Problems.

chotherapy research program, however, we have recently published some preliminary data regarding our supportive psychotherapy model that demonstrate (1) efficacy on various patient-rated measures of symptomatology, presenting complaints, and interpersonal functioning (Hellerstein et al., 1998); (2) change not only at termination of treatment but also sustained change at a 6-month follow-up, (Rosenthal et al., 1999), and (3) efficacy comparable to a short-term dynamic psychotherapy, which was more expressive in nature. The study involved 24 patients who presented with personality disorders that included obsessive–compulsive (5), avoidant (3), dependent (2), narcissistic (2), paranoid (2), self-defeating (2), histrionic (1), and personality disorder not otherwise specified (7), as defined by DSM-III-R (American Psychiatric Association, 1987) and diagnosed (reliably by trained graduate students) by the SCID-II (Spitzer, Williams, & Gibbon, 1987b). Seventeen of the patients also presented with depressive disorders on Axis I on DSM-III-R, four with anxiety disorders, and two with adjustment disorders, according to SCID-I (Spitzer, Williams, & Gibbon, 1987a). Seven patients (29%) dropped out of the supportive treatment, which is consistent with findings from other brief models that we have investigated (Winston, Muran, Safran, Samstag, & Bernbach, 1998). The treatment was generally based on the principles and techniques articulated in this chapter and was 40 sessions in length. Our research has also provided preliminary evidence that the therapeutic alliance as rated by both patient and therapist in our supportive psychotherapy showed stable and high levels across the course of treatment when contrasted with more expressive treatments; this was particularly true in good outcome cases (Hellerstein et al., 1998).

The only other notable exception of empirial evaluation of supportive treatment for personality disorders has been reported by William Piper and colleagues (Piper, Joyce, McCallum, & Azim, 1998). They compared the efficacy of their models of supportive and interpretitive psychotherapy in a sample of 144 outpatients, 60.4% of whom received an Axis II diagnosis: 29.2% avoidant, 24.3% obsesssive–compulsive, 22.2% borderline, and 21.5% paranoid. The results indicated no outcome differences for the two models. The only interesting differences were that quality of object relations had a significant interaction effect and there was a higher dropout rate for the interpretative model, which suggests that supportive psychotherapy may be the treatment of choice when considering a wide range of patient types.

CONCLUSION

We believe that supportive psychotherapy is an effective form of treatment for patients with personality disorder. It uses direct measures in a comprehensive and flexible manner to improve self-esteem, adaptive skills, and psychological or ego functions. Additional systematic research, including clinical trials and process investigations, is needed to refine the technique and help determine the most useful approaches for patients with personality disorder.

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CLINICAL HYPNOSIS - A Primer

Before we dive right into the "how to" of hypnosis and hypnotherapy, we should go over a little background information, from exactly what hypnosis and hypnotherapy are, to how hypnosis came to be a professionally practiced technique, and who recognizes it as a legitimate form of treatment, and why.

The Definition of Hypnosis

The definition or explanation for hypnosis is generally conclusive, but as with anything, different people explain things in different ways, or in greater or less detail. What follows are a sampling of "official" definitions.

- ***The APA's Division 30 Definition and Description of Hypnosis:***

  Hypnosis typically involves an introduction to the procedure during which the subject is told that suggestions for imaginative experiences will be presented. The hypnotic induction is an extended initial suggestion for using one's imagination, and may contain further elaborations of the introduction. A hypnotic procedure is used to encourage and evaluate responses to suggestions. When using hypnosis, one person (the subject) is guided by another (the hypnotist) to respond to suggestions for changes in subjective experience, alterations in perception, sensation, emotion, thought or behavior. Persons can also learn self-hypnosis, which is the act of administering hypnotic procedures on one's own. If the subject responds to hypnotic suggestions, it is generally inferred that hypnosis has been induced. Many believe that hypnotic responses and experiences are characteristic of a hypnotic state. While some think that it is not necessary to use the word "hypnosis" as part of the hypnotic induction, others view it as essential.

  Details of hypnotic procedures and suggestions will differ depending on the goals of the practitioner and the purposes of the clinical or research endeavor. Procedures traditionally involve suggestions to relax, though relaxation is not necessary for hypnosis and a wide variety of suggestions can be used including those to become more alert. Suggestions that permit the extent of hypnosis to be assessed by comparing responses to standardized scales can be used in both clinical and research settings. While the majority of individuals are responsive to at least some suggestions, scores on standardized scales range from high to negligible. Traditionally, scores are grouped into low, medium, and high categories. As is the case with other positively-scaled measures of psychological constructs such as attention and awareness, the salience of evidence for having achieved hypnosis increases with the individual's score.

  (This definition and description of hypnosis was prepared by the Executive Committee of the American Psychological Association, Division of Psychological Hypnosis. http://www.apa.org/divisions/div30/define_hypnosis.html)

- ***Standard 'Complementary and Alternative Medicine' (CAM) definition:***

  A form of cognitive information-processing in which a suspension of peripheral awareness and critical thinking can lead to apparently involuntary changes in perception, memory, mood, and physiology. The resulting relaxation of the mind and openness to positive suggestion helps in the diagnosis and treatment of medical and psychological disorders such as addictions and destructive behavior.

- ***The British Medical Association (BMA) definition:***

  A temporary condition of altered attention in the subject which may be induced by an-other person and in which a variety of phenomena may appear spontaneously or in response to verbal or other stimuli. These phenomena include alterations in consciousness and memory, increased susceptibility to suggestion, and the production in the subject of responses and ideas unfamiliar to him in his usual state of mind. Further, phenomena such as an aesthesia, paralysis and rigidity of muscles, and vasomotor changes can be produced and removed in the hypnotic state. (BMA, 1955)

- ***U.S. Dept. of Education, Human Services Division definition:***

  "The by-pass of the critical factor of the conscious mind (a persons analytical and judgmental ability) followed by the establishment of acceptable selective thinking."

So that this short statement by the US Government makes a little more sense, a brief explanation is likely in order. The mind has four different divisions: the Conscious Mind, the Critical Factor, the Subconscious Mind, and the Unconscious Mind.

The Unconscious Mind is where our instincts are - the things we intrinsically know from birth and throughout life - and protects us reflexively.

The Unconscious Mind controls the Autonomic Nervous system, respiration and heartbeat. Its learning is capacity limited, but Pavlov demonstrated that it can be impacted when he made dogs salivate to a ringing bell. This is known as classical conditioning.

The Subconscious Mind starts out empty, then, as we grow and learn, the unlimited capacity database stores our experiences and perceptions, which then create our personality, emotions, learned responses, beliefs and habits. These ideas and concepts become 'imbedded' and are then highly resistant to change.

The Subconscious Mind uses it's database to protect against known threats, and to motivate us to fulfill our needs. This is the concept of Tabula Rasa - experiences and learning are written upon us. Some have called this *temperament* (Jung). The subconscious mind resists conscious change, but hypnosis can have immediate effect on the subconscious (Banyan).

The Conscious Mind is analytical and logical, and is where we spend most of our time. The Conscious Mind protects us against immediate threats and is where we problem-solve - assess situations and implement resolutions; it evaluates the specific concern or issue that has its attention at the moment, then makes choices and decisions for day-to-day functioning.

In the Conscious Mind, thoughts considered to be more important will replace those of less importance. Conscious change is temporal, because the capacity is limited. This makes subconscious reprogramming more effective.

The Conscious Mind is also where "will power" comes into play. But as you may have noticed, quite often our will power is not powerful enough, and we often quickly revert back to our old habits and ways. This is due to the influence of the Critical Factor section of our mind.

The Critical Factor is the gate-keeper between the Subconscious Mind and the Conscious Mind. It functions as a filter for new information coming in from the Conscious Mind, and compares/contrasts it to the existing database of beliefs in the Subconscious Mind. When the newly introduced data does not jibe with the existing database, the Critical Factor rejects it, by not allowing changes in emotional or behavioral responses to occur. This is why it is so difficult to break habits or change our beliefs.

"The bypass of the critical factor" of the Government's definition refers to the hypnotic processes' release of the imposed limiting beliefs - essentially disarming the Critical Factor - which then allows emotional or behavioral hypnotic suggestions to pass through the Conscious Mind and into the Subconscious Mind.
"The establishment of acceptable selective thinking" refers to the actual creation of new beliefs in the Subconscious Mind through suggestibility - a sort of reprogramming, which allows you to break habits, change perceptions, and create new auto-responses to situations.

The Definition of Hypnotherapy

"Hypnotherapy" is the term used to describe the use of hypnosis in a therapeutic context - with the intention to solve problems or improve conditions of the mind or body - and many hypnotherapists refer to their work as "clinical hypnosis" in addition to using the term hypnotherapy.

Hypnotherapy may be practiced as an adjunct therapy to the work of licensed physicians, psychologists and mental health professionals, or it can be practiced in a stand-alone environment.

The Definition of a Hypnotherapist

Perhaps one of the simplest yet most conclusive definitions or job descriptions of a hypnotherapist comes from the US Government, as defined in *The Federal Dictionary of Occupational Titles, p*ublished by the United States Department of Labor, authored by Dr. John Kappas, 1977.

> 079.157.010 | Hypnotherapist
> Alternate Title: Master Hypnotist | Alternate Title: Hypnotist
> Hypnotherapist induces hypnotic state in client to increase motivation or alter behavior pattern through hypnosis. Consults with client to determine the nature of problem. Prepares client to enter hypnotic states by explaining how hypnosis works and what client will experience. Test subjects to determine degrees of physical and emotional suggestibility. Induces hypnotic techniques of hypnosis based on interpretation of test results and an analysis of clients problem. May train client in self-hypnosis conditioning.

The Definition of Clinical Hypnosis

People often use the term Clinical Hypnosis interchangeably with the term Hypnotherapy, and many would argue that there is no difference beyond semantics, while some would argue that there is a practical difference.

The 'difference' opinion is that clinical hypnosis is a term applied to a greater range of both physical and psychological ailments, rather than a more restricted area of purely psychological phenomena (which they would call 'hypnotherapy').

The Definition of Self-Hypnosis

Self-hypnosis or auto-suggestion hypnosis is when a person hypnotizes him or her self without the assistance of another person to serve as the hypnotist. People most often utilize this practice in effort to stay on a diet, quit smoking or some other addiction, or elevate the person's self-confidence.

Self-hypnosis is rarely suggested or used for more complex or controversial difficulties, such as those that may require a hypnotist to monitor the hypnotized person's reactions and responses, and then respond accordingly.

Most people who practice self-hypnosis require a focus in order to become fully hypnotized; this is where self-hypnosis CDs come in handy, as they provide the necessary induction.

HYPNOTHERAPY VS. TRADITIONAL THERAPY

Traditional Psychotherapy asks, "How can the client function to his greatest potential?"

Brief Therapy asks, "How can I return the client to an adequate level of functioning?"

Traditional Psychotherapy asks the client to develop understanding into the problematic behavior.

Brief Therapy asks the client to change now and develop understanding later.

Traditional Psychotherapy is driven by a multiplicity of client needs.

Brief Therapy focuses on specific client needs.

Traditional Psychotherapy is often driven by client desires.

Brief Therapy is often driven by client needs.

Traditional Psychotherapy is often insight oriented.

Brief Therapy is often cognitive-behavioral in approach.

Traditional Psychotherapy is often desired by both client and therapist.

Brief Therapy is often all third-party payers are willing to accept.

Traditional Psychotherapy presumes a continuum of care.

Brief Therapy is often limited in time and resources.

Why traditional approaches to therapy are limited:

1.) They fail to recognize the importance of the subconscious
2.) Limited time and resources
3.) Managed care constrains treatment
4.) They do not address deeper client needs and/or motivations in context of life experiences.

Brief therapy is rapidly becoming the new form of today's traditional therapy.

Hypnosis is the ultimate form of brief therapy.

HYPNOSIS FACT AND FICTION:

There are many misbeliefs or misconceptions about hypnosis, and they all stem primarily from both a lack of education on the subject and from myths perpetrated by entertainment venues and uneducated or biased persons.

- Over the years, fictional movies and novels have created a mystery about the use of hypnotism, a Svengali-like belief that one is under the control of the hypnotist. This couldn't be further from the truth.

  (Svengali is the name of a fictional hypnotist in George du Maurier's 1894 novel, *Trilby*. The sensational book created a classic stereotype of the evil, mind/behavior-controlling hypnotist that persists to this day.)

- The 1980's brought us Repressed Memory therapy, where hypnotic recollections were used in criminal cases on issues of child abuse and cult abductions, often with disastrous results for society.

- Stage Hypnotism can be a lot of fun to both witness and participate in. But while it demonstrates the power of hypnotic suggestion, it can also create an image of hypnosis that is less than professional, and may perhaps increase a person's fears of public humiliation.

The following are the answers to some common questions about hypnosis.

- **Is hypnosis an "altered state of consciousness"?**

No. An "altered state" implies an external factor contributing to a change, such as drunkenness or being stoned on drugs. Hypnosis, as previously discussed, is a natural state that we all enter a few times each day.
Researchers have divided our natural brain functions into four separate levels of cycles per second, or CPS.

(CPS) Activity Levels:

1) BETA
   Normal daytime consciousness, critical thought level. (18 - 40 CPS)

2) ALPHA
   Relaxation level - beginning to awaken in the morning and crossing over into sleep at night. Associated with imaginative thinking; corresponds to light and medium levels of Hypnosis. (8 - 17 CPS)

3) THETA
   Early stages of sleep; deep daydreaming state. Associated with creative thinking; corresponds to medium and deep levels of Hypnosis. (4 - 7 CPS)

4) DELTA
   Profound sleep; dream state. (1 - 3 CPS)

That's it; there's no place else for your brain to go, in hypnosis or otherwise.

For most people, hypnosis is a mid-Alpha range activity, and although you are under hypnosis, you remain fully conscious of everything that is going on.

Hypnosis is simply a matter of setting aside the conscious mind, to one degree or another, and selectively focusing one's attention on either a particular point or a whole range of experiences.

Because of the hypersuggestibility inherent in the Alpha and Theta levels, positive programming during hypnosis is extremely effective in helping to create positive life changes.

- **Is hypnosis the same as sleep?**

No. Hypnosis is generally a profound state of relaxation, but it is different from sleep. A sleeping person cannot usually respond to suggestion.

"HYPNOS" *is* derived from the Greek word for sleep - an historical mistake in naming terminology that we will discuss shortly - but a hypnotist who uses the word "sleep" during the hypnotic process should explain to his/her clients that hypnosis does not have the same connotation as night sleep.

- **Is a person who is hypnotized still in control?**

Absolutely and at all times.

The government, CIA, FBI and KGB spent years trying to find methods of 'mind control' that could turn a person into an obedient zombie. They failed. Remember the movie *The Manchurian Candidate* from 1962 with Frank Sinatra? It made for thrilling entertainment, but it could not have happened that way.

Comedic stage hypnotists often contribute to this 'mind-control' misconception by having volunteer spectators do all kinds of outlandish things, such as singing like a pop star or acting as

if they are having sex with a chair. But remember, the people up on stage are the 10 out of 100 in the audience who, having a good idea what to expect beforehand, have actually *volunteered* to get up and do this stuff. Moreover, although they may ultimately feel humiliated in retrospect after the show, at the moment they make the decision to participate, they have chosen to please their audience of peers by doing what the hypnotist suggests.

But even while hypnotized during the show they do still have a choice, and every now and then you will see someone on stage who, for whatever personal judgment call reason, simply refuses to comply with the hypnotist's suggestion.

- **Can a person be "stuck" in hypnosis?**

No. It is simply not possible; hypnosis does not last forever. If the hypnotist quits speaking (or if the power goes out while listening to a hypnosis CD) a person would simply choose to either wake-up, or fall into a natural sleep and take a nap.

- **Can anyone be hypnotized?**

Yes. In theory, everyone *can* be hypnotized, because everyone has the capacity to relax and to concentrate. There may be a few exceptions; I suppose drunk/drugged people or perhaps those whose mental deficits prevent concentration, relaxation or the following of directions might not be easily hypnotizable.

However, some people *will* not be hypnotized, by personal choice. Some will never overcome their fear of what they think hypnosis is and may be too guarded to respond. Others may fail to respond due to a lack of trust resulting from a personality clash with the hypnotist. But overall, most can easily be hypnotized, some very easily, and they are what we call natural somnambulists.

- **Is it easier to hypnotize those who are less intelligent?**

Absolutely not; the smarter you are the easier it is to be hypnotized. Abstract reasoning skills, self-confidence, and the ability to focus and concentrate are important traits for hypnosis.

- **Is the use of hypnosis considered a 'new age' or 'alternative' therapy?**

No. Hypnosis has been around for centuries; Egyptian hieroglyphics record use of hypnosis, and Greek sleep temples used hypnosis for healing.

Perhaps because the field is still not legally regulated, unlicensed practitioners - who may also be drawn to other unregulated or alternative healing methods - tend to learn and practice hypnosis at a greater rate than the rest of the population, but hypnosis in and of itself is not part of any new age movement or alternative therapy scheme.
Remember, the AMA, ADA, BMA and the APA are not exactly the most liberal and accepting groups when it comes to alternative therapies, yet all of these groups do formally accept hypnosis.

- **Is suggestibility real?**

Yes. Just ask any advertising executive; how many items have you bought "on impulse" at the check-out counter? Have you ever yawner afer seeing the visual cue of someone else yawning first.

History is replete with people responding to suggestion - even in masses - sometimes in healthy ways and sometimes in negative ways. Even clients in only a moderate level of trance depth can be effectively suggestible.

- **Is hypnosis the best way to resolve problems?**

No. Hypnosis is *a* way to solve some problems. There is more than one way to skin a cat, and there is more than one way to quit smoking. Hypnosis, as with any other form of treatment, works for those whom it works for.

Because of the unique positive physical and psychological responses many people have to hypnosis, research demonstrates that it is an effective treatment protocol in a variety of situations, ranging from addiction (smoking cessation), to managing symptoms of IBS (irritable bowel syndrome), to altering life-long destructive patterns of behavior (diet and exercise). Again, the APA is the champion of pro-hypnosis treatment for these matters.

- **Are audio CD's a good substitute for 'real' hypnosis?**

Yes. Just like live hypnosis, audio CDs can: a) train a person, and b) teach skills. Sometimes they are actually better than live hypnosis because: a) the client can listen whenever they want, and b) since they are professionally produced, the CDs are usually are free from any peripheral distractions or script errors common to live hypnosis presentations.

5 Reasons to Practice Hypnotherapy

There are many answers to this question. We will cover a few here briefly, and more in detail throughout the text, but the simple answer is: *Because it works*.

1.) Hypnosis is Safe.

Hypnosis is simply a state of being, quite similar to when you're just falling asleep or waking up; it is a condition of focused relaxation.

Under hypnosis, you are always conscious and you always maintain the innate ability to make behavioral and emotional decisions. If you do not like a hypnotic suggestion that has been presented to you, the built-in protectors of your mind will automatically kick in, allowing you to reject the suggestion, or to even emerge/awaken from the hypnotic state on your own.

Brain scans of those under hypnosis show normal brainwave functioning. Even abreaction - the re-experiencing of past emotional situations - is not physically harmful and is generally considered positively therapeutic.

2.) **Hypnosis is Effective.**

People have used hypnosis for hundreds, and perhaps even thousands of years, because when used appropriately, it works.

Research outcomes (many of which will be cited throughout this text) have caused hypnosis to be recognized by the American Medical Association (AMA) as a clinically proven adjunct treatment for many medical conditions such as Irritable Bowel Syndrome; by the AMA and the American Dental Association (ADA) as an anesthesia in dental work or other surgery; by the National Institute of Health (NIH) for pain management and PTSD treatment; by the American Association of Marriage and Family Therapists (AAMFT) for sexual dysfunctions and marital discord; by the American Psychiatric Association (APA) for treating pain and anxiety; and by the American Psychological Association (other APA) for its effective treatment of many psychological and behavioral problems such as weightloss management, stress management and smoking cessation, as well as to improve memory function and reduce test anxiety.

However, hypnosis is certainly not a 'cure-all' for anything and everything. While the list of approved or accepted uses for hypnosis is quite long and varied, despite some claims, hypnosis will not make your penis or breasts grow larger.

3.) Hypnosis is Natural.

Hypnosis is a natural state of being; people naturally experience hypnotic states several times a day: when going to sleep, when awakening from sleep, when driving a car or watching a movie.

Have you ever been driving down the road, then suddenly look up and notice your exit, and wonder how you got there, because the last thing you remember was about ten miles back? They call this experience "highway hypnosis" for a good reason. You were still driving, avoiding cars, even maintaining your lane and speed - but your mind was intently focused on something else, like a problem at work or where to have dinner later.

Have you ever become so engrossed in a TV show or a book that you tuned out most everything else around you, and were perhaps startled when someone spoke to you or touched you? You were "zoned out" in a state very similar to hypnosis.

The techniques used in hypnosis inductions are also natural. Therapeutic relaxation, from muscle control to controlled breathing and focusing on peaceful images, are all natural mind and body processes.

Hypnotherapy strategies, such as age-regression, are natural human functions. When you look at a photograph from the past, your mind will automatically take you back to the place and time in the photo, even if for only an instant. When you talk with friends about events that happened yesterday, a few months ago, or even twenty years ago, your thoughts

naturally go back in time and you essentially re-experience, from a distance, those specifically recalled events.

Hypnosis is also natural in that the process involves no chemicals or drugs.
Consider the benefits of conservative, effective, drug-free treatment for anxiety, children's A.D.D., or detoxification from nicotine, alcohol and other drugs. The ability to control addiction withdrawal symptoms without the need for psychotropics should be considered a landmark treatment breakthrough.

4.) Hypnosis Feels Good, and is Good for You.

Many people claim that one hour of hypnosis feels equivalent to getting eight hours of quality sleep.

To enter a state of hypnosis, we employ stress-reducing deep- breathing techniques, rather than taking short, stimulating breaths.
When under hypnosis, we are in a deep state of focused mental and physical relaxation (which, by definition, should be a pleasant experience).

Our brain naturally generates four different kinds of electrical waves in response to different kinds of mental and physical activities (alpha, beta, theta and delta). These waves can be detected by special sensors or an EEG machine, and they will change from frequency to frequency, depending on the amount and type of activity that occurs.

When we are relaxed - and/or in a state of hypnosis - the brain produces alpha waves. We are most productive while in alpha level, because we are completely relaxed yet fully awake, effortlessly alert. In this situation, we naturally have a better command over our physical and emotional functioning. We can think more clearly, better evaluate our responses and problem-solve, and can even make more creative decisions with greater long-term results.

Alpha waves are also shown to help to improve the body's natural immune functioning, and reduce stress and anxiety related disorders.

5.) Hypnosis Teaches Skills that can be Applied to a Variety of Problems.

Hypnosis is more than just relaxation. Through hypnotherapy, a person can be taught to manage physical symptoms and responses to pain, learn ways to cope with psychological difficulties, gain skills to improve their communication strategies, increase work and school performance, and even enhance their relationships.

The skills taught to control or improve one kind of situation can often easily be applied to other situations as well.

THREE LAWS OF HYPNOSIS:

1. The subject must have a clear image of what the result will feel like and be like.

2. When the subject's will and imagination (belief) are in conflict, imagination always wins.

3. A suggestion is more likely to be accepted when it is tied to a positive emotion or affect with which the subject can identify.

Hypnosis works because:

1.) We have total control over our body and the body's response to its environment at all times.
2.) We reprogram a mindset of sickness with a mindset of health.
3.) The body responds to relaxation.
4.) Belief is a healing force.
5.) Behvior that reinforces healing is congruent with belief.
6.) Recovery and regeneration are facilitated by hypnosis.

Hypnosis and Hypnotherapy are Not Difficult to Learn

Truly, the basic techniques of hypnosis are easy to learn since hypnosis is a natural process. Hypnotherapy is a bit more challenging to master, the difficulty being the individual differences and responses common to any therapeutic process. This is learned through experience, which is why you must not be shy in practicing the techniques with your peers and with clients, in order to gain knowledge and experience to refine your skills.

APPLICATIONS OF HYPNOSIS

When people ask me if hypnosis is effective, I tell them, "Hypnosis is effective for those whom hypnosis is effective for." I believe the research, and the research shows that people with medical problems, emotional problems and undesirable habits can overcome their difficulties using hypnosis - weightloss, smoking cessation, nail-biting, memory enhancement, anxiety management, pain management, and so forth.

I believe it is a preferred modality of treatment in many cases, and for most clients, because it is natural, and it is primarily a form of brief therapy. In today's world of limited time and resources, brief therapy is a preferred method.

But again, hypnosis is only effective for those whom it is effective for. Only quacks believe that only one modality of treatment always works and is effective for everything and everyone.

Different things work for different people, and research proves that hypnosis works for a lot of people. However, it is not something to apply indiscriminately to all clients for all conditions. Sometimes hypnosis is the better choice, and sometimes it is not. For those clients who are willing to learn new skills and seek brief intervention via a natural approach with demonstrated efficacy, hypnosis is effective and should be considered a first-line intervention, rather than relegated to the back of the clinician's repertoire.

LEGITIMATE USES OF HYPNOSIS

Below is a list of legitimate uses of Hypnosis, as defined by the ***American Society of Clinical Hypnosis (ASCH)***, an organization sanctioned by the APA.

- Allergies
- Anxiety & stress management
- Asthma
- Bed-wetting
- Concentration
- Bowel Syndrome
- Depression
- Sports & athletic performance
- Smoking cessation
- Raynaud's disease
- Test anxiety
- Colitis
- Sleep disorders
- Obesity & weight control
- High blood pressure
- Sexual dysfunctions
- Learning disorders
- Crohn's Disease
- Dermatologic Disorders (Eczema, Herpes, Neurodermatitis, Pruritus [itching], Psoriasis, Warts)
- Pain (back pain, cancer pain, dental anesthesia, headaches and migraines, arthritis or rheumatism)

SOME NON-LEGITIMATE USES

- Growing taller
- Psychic powers
- Getting lucky
- Remote influence
- Magical seduction powers
- Penis growth

The foundation for the success of hypnosis is the client-therapist relationship.

"The orientation towards hypnosis emphasizing skillful relating to the client minimizes the use of ritualistic or impersonal approaches to hypnosis, instead encouraging an assessment of and flexible responsiveness to individual client needs."

"Cookie cutter hypnosis sessions with prerecorded inductions with two clients scheduled at a time for 30 minutes sessions, may be efficient for the therapist but is ineffective for the client."

~ Michael Yapko, Trancework

Characteristics of a Good Therapist-Client Relationship

Just as with any traditional counseling relationship, hypnotherapy requires certain core elements to be in place for the therapeutic relationship to be effective.

1) Assessment of Client Strengths
Using existing strengths to solve provlems is much more efficient than trying to resolve deficits or create new strengths.

2) Rapport
Ability to create friendly dialogue that builds trusting relationships. This often involves "search talk" or asking "how do you feel?" and "why do you think that?"

3) Respect
Clients must be respected, which is not the same as unconditionally accepting their cognitive-errors.

4) Honesty
The hypnotherapist must be honest about the limits of hypnotherapy and all other matters.

5) Equality
Along with respect, we are the client's coach, working *with* them, rather than an infallible guru they must accept without question.

6) Openess
The hypnotherapist must answer questions and promote client learning on relevant issues.

7) Trust
Trust comes from a hypnotherapist/client relationship were the hypnotherapist is percieved as a genuinely concerned helper.

8) Listening
Hypnosis appears to be a one-sided communication; it is not. The hypnotherapist must listen to the client before, during and after sessions in order to provide effective hypnosis services.

9) Avaiablility
The hypnotherapist is available not only through the emotional support provided, but also simply by being accessible.

10) Consistency
The hypnotherapist must start and end on time, and provide quality services, every time.

11) Confidence
You must have confidence to achieve results with hypnosis, and this confidence must be communicated through both verbal and non-verbal presence.

Elements of Hypnosis

1.) Hypnotic voice / tone
If you listen to any self-hypnosis CDs or experience a live hypnosis session, you may notice that the hypnotist often speaks in a rather different and special tone of voice, one designed to convey confidence, calm, trust, and general serenity. This is important to the hypnotic process, and you will want to practice this voice to perfection.

You should speak clearly and loudly - not necessarily louder than normal conversation, but take care that you do not whisper to trail off at the end of words or sentences, or drop consonants (especially Ds and Ts). Also take care not to speak too quickly, nor too slowly.

It is good practice to regularly read aloud from a book or newspaper, and to open your mouth a little wider when you speak. Doing so, you may notice that the words are more clear and project a more confident sound.

You may wish to record yourself so that you know how your voice sounds to others (which is different than how it sounds to yourself), and practice using different tones until you are comfortable with both the quality and consistency.

2.) Hypnotic posture
Posture is important to both your overall professional appearance and your hypnotic voice. If you look down while reading aloud and your chin is close to your chest, it will change the tonal quality of your voice.

Try it both ways, and notice the difference (record it if possible). Read a passage aloud while looking down, then, with your head level (eyes looking forward) and your neck straight - read it aloud again. You will notice that the sound of your voice is immediately clearer with your head raised, because the larynx is able to work unhindered.
Always try to keep your head level and your neck straight when you speak, as this adds to the clarity and the innate sound of authority in your voice. Additionally, improved physical fitness not only helps develop posture and lungpower, but also tends to 'sweeten' vocal harmonics.

3.) Hypnotic setting
Hypnosis sessions can take place anywhere, but just as with traditional therapy, you want to

present an attitude of professionalism with both your own appearance and the surroundings of the session. A comfortable chair should be available for the client, and you should be comfortable as well, whether sitting or standing. The room should be free of sight and sound distractions during the induction process.

4.) Willing subject
For hypnosis to work, the subject being hypnotised must be willing to be hypnotized; they must want to listen and follow your suggestions. Unwilling subjects may only need a more detailed pre-talk explanation of the process to answer any lingering questions or concerns, or if they are adamantly resistant, they may simply need to be referred for more traditional forms of therapy.

5.) Confident therapist
For the client to trust you and be completely willing to follow your suggestions during the induction process and the suggestive hypnotherapy session, you must present yourself as being totally confident about what you are doing - even at first when you are new and inexperienced. This is both an attitude and a natural skill that will come from practice.

6.) Knowledge of approaches
You must be knowlegable about the various approaches to inducing and deepening hypnosis, and be able to determine (through assessment) which approaches will be more beneficial to the client. This knowledge comes from research and practice, along with a healthy dose of common sense.

Elements of Hypnotherapy

1.) Assessment
Just like with traditional therapy, the initial assessment is likely the most important part of the therapeutic process. Not only must you clearly understand the client's presenting problem or goal to be addressed, but you must also be able develop specific, targeted intervention (suggestion) strategies for helping the client reach the goal or solve the problem.

Areas to consider are intellectual functioning, educational level, social skills, job skills, communication skills, relationship skills and so forth. A low-functioning client awake will remain a low-functioning client under hypnosis, and you will want to frame your suggestions with the appropriate level of direction to achieve the greatest impact, just as you would with traditional therapeutic approaches.

You also need to assess that the client's goal is realistic. For example, if the client has come for hypnotherapy to lose weight, but they expect to lose 50 pounds in the next two weeks, you will need to work with them through education to understand that a more reasonable goal would be 50 pounds over several months, and so forth.

2.) Interviewing strategies
Initial client interviews for hypnotherapy sessions are similar to initial interviews for traditional therapy, however they are often more focused, as it is best to only address one primary subject per hypnosis session. If the client wishes to both curb anxiety and lose weight, you will likely want to schedule separate sessions to handle the individual goals.

Your interviewing technique should convey confidence and competance, as well as professional empathy. Pay attention to the client's personality and communication styles, and take notes on potentially effective hypnotherapeutic approaches to use in hypnosis sessions, based on the client's style of response to questions.

Make sure to ask questions that relate to the client's typical functioning levels, social supports and so forth, so that you can use this information to reinforce the intended goals within the hypnotic suggestions.

3.) Understanding hypnotic methods

There is more to hypnotherapy than simply offering suggestions to alter behavior. The word choices you use, combined with directive or indirective approaches all work together to help reprogram subconscious associations, which then help to alter consciously destructive behaviors.

For example, negatives (don't, not, can't) are not easily processed, and are generally ignored or deleted from sentences in the mind. Consequently, "don't smoke" becomes "smoke". Therefore it is far more effective to say, "you are a person who breathes clean air" than, "you do not smoke anymore".

There are many more hypnotherapeutic 'tricks' such as these that you will need to learn in order to increase the effectiveness of hypnotic sessions, which will come from education and practice.

4.) Referral skills

Clinical hypnosis is one method of treatment for many conditions, but it is not always the best method for all conditions or all clients. You must be able to both recognize when another approach would be more beneficial for a client, and be able to refer them to an appropriate source for alternative therapy. It is helpful to network within your community and become familiar and friendly with available clinicians and agencies who provide services that you cannot or are not intersted in providing, so that you are able to help your clients get the help they need, and so that they know you are out there and can refer hypnosis-ready clients to you.

How To Hypnotize Someone

1.) All hypnosis is self-hypnosis.

Hypnosis is not really something that you do *to* someone, but rather it is a state of being that you will assist the client in achieving, by guiding them through the hypnotic process.

2.) As a hypnotherapist, you will utilize the Hypnotic Process.
The Hypnotic Process has five stages:

1) The Pre-Talk
2) The Induction
3) The Deepener
4) The Suggestive Script
5) The Awakener

Anyone - a trained and licensed mental health professional or a comedy stage hypnotist - can follow this process to hypnotize someone. Again, however, performing Hypnotherapy requires deeper knowledge and skills training, and effective two-way communication strategies.

THE PRE-TALK

I have friends who are stage hypnotists, and I have done stage hypnosis work in the past. If you have never seen a stage hypnosis show, I recommend that you do so at your first opportunity, simply for the educational value (not to mention the entertainment value), as it provides an emmensely powerful example of what can be accomplished with hypnosis, quite easily. Even in a noisy and often chaotic environment, stage hypnotists can hypnotize large groups of willing people - in just a few minutes.

That may sound unbelievable, but truly, anyone *can* be hypnotized. Unlike the eager participants in a stage show, those who are not so sure that they want to be hypnotized, and even those who might actually fear hypnosis, can still be hypnotized - *if* the hypnotist takes the time to explain the hypnotic process to them.

This is why the first step in any hypnosis session is the “PRE-TALK.”

People who have never experienced hypnosis first hand can be quite wary, nervous and perhaps even resistant to the process, even while wanting to try it, and this can diminish suggestibility and efficacy. But explaining to the client exactly what hypnosis is, how it works, how it feels, what will happen, and how much control they will maintain or lose over what they say or do, can go a long way towards soothing the client’s nerves, advancing suggestibility, and building necessary client/hypnotist trust and raport.

Remember at the beginning of this text when I said you should have a little background information before we dove into how to hypnotize someone? The information that followed was the equivalent of the pre-talk portion of this course; its something rather like the concepts of ‘patient education’ and ‘informed consent’ rolled into one.

The pretalk accomplishes three things:

1.) Educates the client about hypnosis being a natural, safe state.
2.) Builds trust between the hypnotist and the client.

3.) Often includes ‘convincers’ that serve to both demonstrate the nature of the human mind and test client suggestibility.

Milton Erickson often began his sessions with general, natural conversation, weaving in and out his thoughts about the day, the process of hypnosis, and helping people to experience trance naturally. Though quite subtle, this strategy is still a pre-talk, woven into the ramblings preceeding the formal induction.

The following ideas are important to convey to the client during the pre-talk:

1.) It is essential for the client to understand that hypnosis is a natural state.

As we discussed earlier, you can explain that everyone experiences a natural form of hypnosis several times a day, and you can give examples such as when we are waking, going to sleep, driving or watching TV.

This puts the concept of hypnosis into a perspective they can relate to, and helps eliminate much of the ‘mystical mumbo-jumbo’ they may invision about the trance state.

2.) It is essential to overcome common objections or myths.

I always assure my clients of four things:

1.) They will feel better than ever when we are done.
2.) Hypnosis can assist them in making positive changes.
3.) They will remain fully in control of their mind and body at all times.
4.) They will never be asked to reveal any personal information outside the context of the pre-established therapeutic goals, or to do aything humiliating.

Also during the pre-talk, it may be useful to let the client know that they will still function normally - they can swallow, move around to become more comfortable and even speak when they so desire.

Convincers

“Convincers” are tests the client can do during the pre-talk that serve to both reinforce the concepts and capabilities of hypnosis to the client, and also give the hypnotist an idea of the client’s early suggestibility response level, their focused concentration level, and their ability and desire to follow directions.

For example, the “book/balloon” test is probably one of the better known convincers.
It works like this:

After just briefly having the client close his eyes and focus on relaxing, he is told to extend one arm straight out, palm up. He is then told to raise his other arm straight up above his head, and to make a fist with his thumb sticking up like he’s hitchhiking.

Then, the hypnotist tells the client that in his open palm is a heavy book that he must support, and will simulate this by pushing down a bit on the client’s open palm. The hypnotist also tells

the client that tied to his thumb is a large helium filled balloon, which is causing his arm to rise higher and higher, and the hypnotist may gently squeeze the client's thumb, for focal point emphasis.

As the hypnotist repeats and reinforces these suggested concepts over and over more and more intensely in the next minute, the client's extended arm will begin to fall from the perceived weight of the heavy book, and their raised arm will stretch higher and higher, as if he's almost being carried away by the balloon.

When the client is suddenly told to open his eyes and finds himself in this interesting position, he is usually far more convinced of the power of hypnosis.

If, however, the client does not respond as expected, then he is perhaps either intentionally resistant, or simply unable to focus or follow directions. The hypnotherapist should then discuss the situation with the client and determine the lack of response cause, and then to overcome the problem, if possible, with further education, or perhaps simply to refer the client for a different form of therapy.

Remember, for hypnosis to work, the client must be willing.

Other convincer tests (and their brief testing instruction examples) include:

- **Fingers Drawn Together test - as if they were magnetized**

  "Hold your arms straight out in front of you, with your fingers clasped together, and then extend the pointer finger of each hand.

  Now, pull both fingers apart as far as possible, and stare hard at the space between them.

  You will notice that no matter how hard you try to keep your fingers apart, they are being drawn closer and closer together. The harder you pull, the stronger the magnet gets that pulls them together.

  Focus on the space between your fingers. Pull harder. Can you feel the magnet? Your fingers are being drawn by the magnet, closer and closer together, until they touch."

- **The Eye-Lock test - the client's eyes close and become glued shut**

  "Take a deep breath - hold it - now, as you let it out, just close your eyes and let your body relax.

  Again, take a deep breath - hold it - now slowly let it out and relax even more.
  Relax your eyes, and relax all the muscles around your eyes, fully and completely.

  Your eyelids are relaxing and they are happy to be closed.

  You are now so comfortably relaxed that your eyes want to remain closed.

No matter how hard you try to open your eyes, they will remain closed.

Take another deep breath - hold it - now let it out and relax even more.

Now that you are completely relaxed, try to open your eyes, and satisfy yourself that they remain closed."
(Pause)

"Good. Now stop trying and relax them again."

Alternative (more direct) version:

"I'm going to count down from five to one. As I do, your eyelids will lock so tightly closed that the more you try to open them, the tighter they will lock closed.

Five. Close your eyes. Your eyes are pressing down tightly.

Four. Pressing down and sealing shut.

Three. Your eyes are sealing closed, as if they were glued.

Two. They are locked shut. The more you try to open them, the tighter they lock closed.

One. Your eyes are completely locked shut.

Now, try to open your eyelids. You will find they lock tighter and tighter.

Very good. You can stop trying now. Simply relax."

- **The Hand Clasp test - locking/gluing a client's hands together**

"Hold both hands out in front of you, palms up.

Good. Now I am going to put some glue on your palms."

(Brush your hand across both of theirs.)

"Now, press your palms together." (Demonstrate.)

"As you press harder and harder, the glue on your hands begins to set - to dry and harden - and your palms become stuck together.

The harder you press your hands together, the more stuck they become.
Press hard. Feel the glue hardening.

Your hands are now completely glued together, and even if you were to try to pull them apart, they would stay stuck together.

Your hands are stuck together. You can try to pull them apart, but they will remain glued shut.

Go ahead, try to pull them apart."

(Pause)

"Good. You can stop trying now. Your hands are no longer stuck together."

Alternative version:

"Now we're going to help you find out how strong your powers of imagination and concentration are.

Stand here, and look me directly in the eyes, and concentrate completely on the ideas behind my words.

Put your arms straight out in front of you. Good.

Clasp your palms together and interlock your fingers. Very good.

Keep looking at my eyes. Now, push your hands together as tightly as you can and concentrate on this idea: 'My hands are stuck together, my hands are stuck together.'

Very good. As you push your hands together, tighter and tighter, your arms grow stiff and rigid, stiff and rigid, as your hands lock more tightly together, as if they're made of one solid piece of steel."

(NOTE: This 'stiff and rigid, solid as steel' situation is called 'catalepsy' of the arms, as will be explained below.)

"Keep looking at my eyes. Now, as you fully and completely concentrate on the idea that your hands are one solid piece of steel, try in vain to pull them apart, and find that they are stuck together, stiff and rigid, stuck together."

(Let them try for two or three seconds - no more).

"Good. Now stop trying, and allow your arms and hands to relax, become loose and limp, and they will release easily."
(On occasion, some people may have a little problem getting their hands apart. Just gently shake them loose and say "It's okay, you can relax now and just let go.")

- **The Hot Object test - client holds a warming object until it is too hot**

"Hold your hand out, palm up. Good. Now look at me. In a minute, I'm going to put a small stone in your hand.

As the stone comes in contact with your skin, it will begin to get warmer and warmer.

As it heats up, I want you to hold on to it as long as you can, until it's just too hot for you to hold.

Are you ready? Good."

(Place the stone - or other small, smooth object) in the center of their palm, and close their fingers around it.)
"The stone in your hand is now reacting and growing warmer. The heat grows gradually, second by second, it gets warmer and warmer.

The longer you hold it, the warmer it gets.

It's getting warmer and warmer, hotter and hotter.

It's like a hot potato.

It's okay to open your hand, but it will still get hotter.

Very good. Just try to hold on to it as long as you can stand the heat.

It's now hotter, and hotter.

(Keep it up until the client drops the stone, or perhaps should, due to effort limits - "Very good. You can put it down now, so it will cool off.")

- **Right/Left Brain "6" test**

"Sit up straight in your chair with your feet flat on the floor.

Which hand do you write with?" (Left or right?)

"Good. Now, raise that arm and point up (at a slight angle, like to a far corner of the room - demonstrate), and extend and raise your (right/left) foot off the ground (foot from same side as arm - left/left or right/right).

Excellent. Now, I want you to do two things at the same time.

With your finger, you're going write the number 6 in the air, starting at the top (counter-clockwise, demonstrate) - and - with your foot, you're going to make a clockwise circle (demonstrate)."
NOTE: You will demonstrate these two actions separately. Why? Because as the client will instantly discover, it's darn near impossible to do them both at the same time, at least without highly concentrated practice.

After the inevitable laughter, continue:

"Now, try it again, but draw the 6 from the loop up, while moving your foot (both clockwise, demonstrate) - or from the top down, but change direction of your foot (both

counter-clockwise, demonstrate)."

-- And then you might briefly discuss left/right brain functioning and concentration, because it's interesting.

- **Lemon Drop test**

"The mind and body work together in amazing ways. When the mind concentrates fully and completely on a specific thought, the body will automatically act as if that thought is true. Let's try an example.

Close your eyes and hold out your palm."

(Very lightly and briefly, touch the center of their palm.)

"Now, imagine there is a candy lemon drop in your hand.

Visualize the color in your mind. See the way the light glints off the crystal coating.

Pretend you can feel the weight of it in your hand.

As you gently squeeze the lemon drop, listen for any sound; feel the shape and texture."

(Don't be too specific in your suggestive descriptions - their lemon drop might not look like the one in your mind.)

"In your mind's eye, notice all the lemon drop's details, as you bring it up closer to your mouth, and pop it inside...

Notice how at first the lemon drop is sweet - until the sugary coating begins to wear off - and now it's sour! Oh so sour!"

NOTE: You're looking for physical reactions - puckering face, eye scrunching, etc. They may swallow, mush their mouth around, and so forth, as they would when eating a real lemon drop.

When convincer tests are used during the awake state, they are sometimes called "waking hypnosis." At other times, variations of these tests are used as "deepeners" during the induction process.

Muscle 'catalepsy' is a term used to describe the induced phenomena of a person's muscles locking or becoming stiff and rigid, like a board. In this state, the cataleptic limb(s) can be placed in any position you tell them, and will remain there. You can pull on a person's cataleptic arm, and it will not move. When this happens, you know for certain that the person is in a hypnotic trance.

Stage hypnotists frequently use catalepsy tests, as it provides both certainty of trance in the subject, and also a amazing demonstration for the audience of the power of hypnosis.

Famous hypnotist Ormond McGill states that principles of effective convincers include the following:

1.) The client's mind must be locked around a specific idea.

2.) To have a person lock their mind around an idea, the suggestion must be given with confidence.

3.) Presentation must be made in a way that does not bring the critical factors of the mind into action. (This is where experience and hypnotic language are important.)

4.) Client must be free from objection, so they will want to experience the suggestion.

THE INDUCTION

The hypnotic induction is not magic. People can by hypnotized without induction or with very short inductions. For those without hypnotic experience, induction accomplishes several things:

1.) Guides them into a state of relaxation

2.) Tests for suggestibility
3.) Gives them a framework for practicing hypnosis
4.) Helps a person relax
5.) Focuses attention on something (example: specific imagery)
6.) Detaches them from external stressors/activity

A few sample Inductions:

- **Progressive Muscle Relaxation (PMR) - the client is guided to relax all muscles.**

  This is the most frequently used induction. It is very non-threatening for first time clients, highly effective at producing trance, and teaches skills that can be translated into non-hypnotic activities.

  This induction can be performed in various lengths - short, general and directive, or long, detailed and more indirect. Which version you choose will likely depend on the client's prior hypnosis experience level; those with little or no experience or who are still a little anxious may benefit more from the longer version, while those who are eager and more suggestible should likely do fine with an abbreviated version.

  1.) Always speak to the client slowly, in a calm, soothing voice. Encouragement is beneficial, so remember to pepper your language throughout the session with positive reinforcements such as - "That's right; very good; you're doing just fine, excellent."

  2.) Instruct the client to sit up in a comfortable position, feet flat on the floor, arms relaxed on their thighs.

  3.) Now begin to guide the client into relaxation, by following a script such as:

  "Keep your eyes focused on me. Take a deep breath, hold it, and as you let it out, feel yourself begin to relax.

  Take another deep breath. All the way in, all the way out. Good. Eyes focused on me.

  Again, breathe all the way in, all the way out. With every breath, let yourself relax more and more, deeper and deeper. All the way in, all the way out.

  Your eyes may be getting heavy. If so, it's okay to close them now. You're never alseep, just relaxed. All the way in, all the way out."

  Note: If they still have their eyes open after a few more breaths, go ahead and tell them to let their eyes close.

  4.) It's important for the inexperienced client to know that they will always be able to hear your voice and that you are there to help them and will always be there to take care of them.

  Example:

"Focus on my voice. I'm here to guide you. You will always be able to hear what I say, and my suggestions are designed to help you relax and feel more comfortable.

Take a deep breath. All the way in, all the way out. Pay attention to the sound of my voice and listen to the suggestions that I am about to give you."

5.) Now, have the client focus their awareness on different parts of their body, and choose to relax them.

Example:

"Become aware of the muscles in your neck and shoulders. Notice the tension there, and now choose to let your shoulders relax.

Take a deep breath and let your head grow heavy. Now exhale, and and let your chin drop down to your chest.

As you become more relaxed, you become more and more comfortable. Excellent. Let your arms become loose and limp.

Take a deep breath and as you exhale, let your chest and back become even more relaxed.

Deeper and deeper, never alseep, just relaxed."

NOTE: You will suggest this type of instruction for all areas of the body (head, shoulders, arms, chest, back, buttocks, legs…).

6.) How long it should take and how detailed you need to be will depend on the individual client.

Watch their responses, and give more attention to those areas where they appear to carry more stress. For most people, 25 or 30 minutes is quite long enough; those with more experience or who are highly suggestible can easily be fully relaxed (and therefore "inducted") in 5-10 minutes.

7.) That's all there is to it. Now you can deepen them further and/or implement the therapeutic suggestions.

8.) When you are ready to bring them up, you can say something simple like:

"Now, I'm going to count to five, and then your eyes will open and you will feel alert, relaxed and refreshed."

As you count up to five slowly, your voice can become a little less soothing and more natural sounding with each number.

When you reach four, if their eyes are not yet open, say, "On the next number your eyes will open and you'll feel completely alert, relaxed and refreshed. Five. Eyes

open, feeling alert, relaxed and refreshed."

- **Eye Fixation - the client stares at an object or a light source.**

This is a traditional, highly effective, rapid method of inducing hypnosis. It produces eye fatigue, a physical cue to which the mind readily responds. When combined with light, this induction style can create abstract visual images. They hypnotist may also combine eye fixation with Kinesthetics by having the client fixate on their extended arm.

1.) After having the client sit in a comfortable position with their feet flat on the floor and arms loose in their lap, ask them to direct their gaze at an object of fixation (a light, a spot on a wall, a specific item), and not to shift their focus.

2.) Instruct the client to:

“Stare at the light (or other specific fixation object). Lock your eyes on it.

Take a few deep breaths, all the way in, all the way out. Perfect. Just keep breathing deeply.

Focus on the (object) and listen to the sound of my voice.

You will find that your eyelids may begin to get heavy, almost as if they had a heavy weight attached to them.

The longer you stare at this (object), the heavier your eyelids become, and you blink.

Your eyes feel heavy, weighted, as if something is pulling them down, as if they want to slowly close. Keep breathing deeply. You’re doing just fine.

Your eyes are drowsier and sleepier and heavier, and you feel as if they are slowly closing, slowly closing; getting drowsier and more tired, and when they finally do close, how good you'll feel.

Drowsy, heavy, pulling down, down, down, slowly closing. It’s getting harder and harder to see, and you feel good. Excellent.

It is very, very hard to keep them open, and you feel that very soon they will close tightly; almost tightly closing, almost tightly closing, tightly closing.

Your eyes are tightly closed; you feel good; you feel comfortable; you are relaxed all over. Just let yourself drift and enjoy this peaceful, comfortable, relaxed state. Wonderful.

You will find that you head feels heavier; it tends to nod forward some, and this is comfortable. Just continue to let yourself drift in an easy, calm, relaxed state.”

3.) That's it. Believe it or not, induction really can be this simple. Now you can deepen them further and/or implement the therapeutic suggestions.

4.) As with all inductions, you can intensify this induction by carefully observing the client's reactions and timing your suggestions very closely with them. For example, you might make the remark, "Occasionally, your eyes are going to blink," immediately after you see the client blink. This reinforces that their behavior is acceptable, and encourages them to continue following your instructions.

- **Mental Imagery - the client invisions certain scenarios for relaxation**

Mental imagery is a common induction technique, and can also be used for deepening. The 'images' utilized can vary from the following examples, to an hourglass letting out sand, to a flower sprouting, growing and blooming, an ocean wave rolling out to sea, and so forth.

However, the hypnotherapist should take reasonable care to not utilize images or objects within those images that might be disturbing to the client in some way.

For example, in the following induction, the image of a 'fluffy pillow' is used. I had originally written this script to use a 'feather pillow' - but someone mentioned that they had bad allergies to feathers, and I decided that rather than risk unnecessary resistance in the induction based on an instinctive reflex against feathers, I would simply change the object's description to 'fluffy', as it would serve the same purpose.

If the client has an extreme fear of heights, using a staircase might not be such a good idea for the induction; if they are afraid of open water, an ocean wave rolling out to sea might not be so calming for them. These issues, however, might well be addressed for treatment in future sessions, if the client is agreeable.

You do not necessarily need to catalog each client's allergies, fears and phobias before you begin an induction, but if your induction or prescriptive script utilizes imagery that some people are known to have difficulty with, it would not hurt to ask a general question about the subject during the pre-talk.

Another consideration for imagery induction is the client's individual learning style: Auditory, Visual or Kinesthetic (tactile feelings).

To begin a 'visually' focused imagery induction:

1.) Have the client sit in a comfortable position with their feet flat on the floor and arms loose in their lap, take them quickly through a deep breathing relaxation period, until their eyes are closed.

2.) Instruct the client similar to the following:

"Outside distractions or sounds won't bother you; you are focused on relaxation and my voice.

Pick a point of light in the room such as a light bulb to stare at, and do not - do not close your eyes, until I tell you to.

Every fiber of you body is becoming relaxed. The day is done; problems are a thousand miles away.

Take a deep breath. All the way in, all the way out. Your shoulders, back and every muscle of your body are relaxed. Excellent.

Your find your eyelids are heavy and want to close, as if you were in a deep sleep. Soon you will be able to close them.

Let your head fall gently forward towards your chest. Close your eyes, and keep them closed.

A part of your mind knows you could open them if you wanted to, but you simply do not care to. You could fight this relaxation if you wanted to, and you know that you do not have to participate, but the smart ones always do best and are able to focus.

Easier to relax and let go. Take another deep breath, all the way in, all the way out. Never asleep; just relaxed.

Let your mind focus simply on relaxing, being reminded that a little B is just a backwards D, and an M is just a right-side E, while the 2 and the Z look the same.

Now, in your mind's eye, imagine you are in a meadow, under the clear blue sky. You see one single white puffy cloud floating high above you.

Imagine yourself lying down in the meadow, looking up. Watch the puffy cloud as it begins to float away.

The further away it gets, the smaller it gets, until it floats far, far away, and as it floats away you become even more and more relaxed.

The cloud is now far, far away in the sky, and you are twice as deep.

Now ten times as deep, and now one hundred times deeper into relaxation.

(SNAP)

Now the cloud is gone - leaving only the clear blue sky.

Even more relaxed. A thousand times deeper into relaxation.

All of your muscles feel as loose as a pile of rubber bands.

You have the ability to relax and concentrate. The smarter you are the easier it is to focus and relax. Don't try too hard, just make the decision to let go. Perfect.

In this state of relaxation, imagine yourself at the top of a heavenly flight of stairs.

There are ten stairs. At the bottom are a soft, fluffy bed and a comfortable pillow.

I am going to count down from ten, as you move down each stair.

As I say the number ten, you will relax. With each additional number, you will simply move down, deeper and deeper, one step at a time, relaxing more completely with each step, eventually resting comfortably on that fluffy pillow atop the comfortable bed.

Ten. At the top of the stairs, relaxing and letting go.

Nine. Becoming more loose, more limp and more calm.

Eight. Sinking into relaxation and serenity.

If I touch you or move you, this only relaxes you deeper.

Ten. Nine. Eight.

Seven. Way down. A thousand times deeper into relaxation.
Seven. Six. Five. That's fine, perfect, moving towards the fluffy bed.

Four. Three. Two. Relaxing and following my suggestions.

Five. Loosen the muscles in your shoulders and neck.

Four. Three. Two. Way down.

Three. Two. Way, way relaxed.
Three. Two. Deeper and deeper.

And finally, one. Let your body go limp. Simply sink into a more comfortable, calm and peaceful position.

Excellent. You're doing perfect….

3.) Now you can deepen them further and/or implement the therapeutic suggestions.

<u>It is important that you become effective at inducing trance with a wide range of people.</u>

Naturally, different people respond differently to different cues and suggestions; some people are more audio-focused, others more visual or more tactile. For this reason, it is important that you become proficient at determining what style of induction to use with different clients.

Through education and experience you will need to:

1. **Develop your skills with different styles of induction.**
   Practice relaxation, imagery, and other forms of rapid inductions, noticing the differences between the concepts/strategies they employ, and the responses they illicit.

2. **Become skilled at noticing signs of trance.**
   Pay close attention to the client's physical, emotional and verbal responses to your suggestions.

SIGNS OF TRANCE:

> Catalepsy during convincer tests
> Physical stillness
> General relaxation and loosening of the muscles
> Gradual postural slumping
> "Flattening" of the facial muscles (they look very relaxed).
> Waxy skin tone (the person may begin to look more like a mannequin - this is very subtle)
> Fluttering of the eyelids
> REM type eye movements
> Increased redness or wetness of the eyes, when open
> Elevated body temperature
> Feelings of warmth, coolness or tingling sensations
> Swallowing or gulping
> Changes in breathing rate and depth
> Fingers, arms or legs twitching slightly

3. **Positively reinforce when you see a sign of trance.**
   When the client responds positively to your induction suggestions, it is important to let them know that they are doing well. The client wants to please you, and hearing periodically that they are performing properly will encourage them to continue following your instructions, and help them go deeper into trance.

4. **Adjust your suggestions to the client's responses.**
   When you suggest something that obviously works for the client, you can do more of it or something similar. If you try something and it does not work, quickly suggest something else of a different orientation.

THE DEEPENER

Deepeners follow the induction. They are are verbal suggestions given by the hypnotist to help the client go deeper into trance, in order to continue the process of hypnosis.

Deepeners are used to:

1.) Help a client relax even further
2.) Help insure concentration and suggestability
3.) Help facilitate guided imagery
4.) Measure current trance depth

They may contain suggestions or tests, but generally not. They can tell a story that ties metaphorically to the prescriptive script, or they can be directive and visual like the staircase deepener.

They may be gradual, or swift, depending on the goal, and may be used as necessary at any time during the hypnosis session.

Some common deepening techniques:

- **Number Counting**

  They hypnotherapist counts down (10, 9, 8…), and instructs that with each next number spoken by the hypnotherapist, the client will relax more and more, and go deeper (into trance). This is a directive approach.

  NOTE: Generally, 'deepeners' count down (10, 9, 8) while 'awakeners' count up (1, 2, 3).

- **The Dave Elman Count**

  In Dave Elman's induction/deepener, "suggested amnesia" produces the deepening.

  To use this method, the hypnotherapist will suggest that the client begin counting in their mind, from 50 down to 1, and as they do, they will begin to forget the numbers that follow, and drift into deep trance.
  For example:

  "I am going to ask that you count backwards in your mind from Fifty to One.

  In your mind, slowly count the number Fifty, then Forty-Nine.

  As you count, you will begin to forget the numbers that follow, and with each number, you will double the level of relaxation you experience.

  Forty-eight. Forty-seven.

  Notice the numbers begin to fade away, and as they do, relaxation doubles again, and you feel good.

Forty-six.

Continue to count backwards in your mind until all of the numbers disappear and fade away, becoming more relaxed with each number. Good."

Or, a client may be told to recite the alphabet backwards, beginning with Z, then Y, then X, and that as they do, the letters will simply fade away, and they will drift into a deep relaxing trance.

- **Fractination**

Fractionation is the hypnotic principle which states that if a person is in a heightened state of trance and is interrupted, when they are allowed to return to the trance state, the depth of their trance will become even more powerful.

To use fractionation, the hypnotist acts like a snooze alarm, guiding the client in and out of trance: asleep - awake - talk/function - asleep - awake - talk/function - asleep. For example:

At 6:00am the alarm clock goes off. You wake up, and feel pretty good. But you know you still can get by with a little more sleep, so you hit the snooze button and roll over for 9 more minutes.

When the alarm goes off again, you hit snooze and relax.
In 9 more minutes, it goes off again. Now you are irritated; you were asleep again, so you quickly hit the snooze and go right back into sleep.

In 9 more minutes, it goes off again, this time waking you from a profound sleep and really grating on your nerves. So you hit snooze again.

When it finally goes off again in 9 more minutes, you realize you are out of time and scramble out of bed, now feeling more tired and more groggy and perhaps generally much worse than you originally felt at 6:00am.

Hypnosis fractionation works in the same manner: it confuses the mind and quickly increases and decreases trance depth.

This may be somewhat be irritating to the client, but the process produces profound trance; it forces the client to concentrate on the words being spoken, and to subconsciously tune out other distractions and thoughts.

- **Vogt's Fractionation**

The purpose behind the fractionation method of inducing hypnosis (sometimes known as Vogt's fractionation) is for the hypnotherapist to understand the individual experience of the client as they begin to enter trance, and then to funnel back this information, in order to take them deeper.

The client is relaxed into the early stages of trance, and then roused and questioned at each point for a verbal description about their current experience of hypnosis.

They hypnotherapist quickly analyzes this information (is what they have been doing working to produce the desired effect, or not?), and then uses it to help the subject to go deeper still. Using fractionation, the novice hypnotist can learn a great deal about the experience of relaxation and trance as it occurs in others.

Essentially, the client ends up describing how they personally can best be hypnotized. This type of induction/deepener is certainly not as quick as other methods, but its interactive nature does seem to lead to the deeper trance states, which may be beneficial for those who are experiencing hypnosis for the first time.

For example:

1.) Implement a short induction asking the client to visualize something with his eyes closed. Then, use a three-count (1, 2, 3) awakening.

2.) Now the client is asked, “Did you see colors? Shapes? Were objects clear to you? Did you recognize the mental images from your past, or were they new creations in the mind?

3.) After considering the client’s verbal responses, immediately implement a short progressive muscle relaxation (PMR) induction, followed by a three-count awaking.
4.) Now the client is asked, “How did that feel to you? Did you relax all of your muscles? Did you notice the difference between tension and relaxation? Were you able to hear my voice? Is there anything that would make you more comfortable?”

5.) Consider the client’s responses, then immediately implement another short induction, and a short deepener, followed by a three-count wake up.

6.) Ask the client, “Did that increase your relaxation? Did you feel in control? Was it pleasurable? What did you like about the experience?

7.) Consider their answers, then immediately re-induct, perform deepeners, and go on to the appropriate prescriptive script.

Vogt’s fractionation can be a lengthy process, but can teach the clinician a lot about the client’s personal styles and preferences.

- **Staircase Deepeners**

These combine visual imagery with a number countdown. The hypnotist mentally guides the client through climbing or decending a staircase towards a 'goal' such as a comfortable pillow or a peaceful meadow. As the client moves closer and closer to the goal, their state of trance deepens and deepens.

- **Visual Imagery / Metaphorical Deepeners**

  The hypnotist instructs the client to imagine strolling up a mountain, following a white, puffy cloud, or watching a flickering candle. As they climb higher, see the cloud float away or watch the candle burn down, they move deeper and deeper into trance.

  Metaphorical deepeners are interwoven with a story or theme designed to correllate the visuals with the session's current goals.

  For example:

  "As you climb higher and higher up the mountain, the cares of the day fall farther and farther away, and you become more and more relaxed."

  "As the candle burns farther and farther down, your muscles will relax more and more, melting into a puddle much like the wax of the candle as it drips down, down, down."

- **Deep Breathing**

  The hypnotist instructs the client to take slow, deep breaths, with the suggestion that their relaxation (trance) level will double with each breath.

  For example:

  "Take a slow, deep breath. All the way in, all the way out. Your breathing will be slow, smooth and rhythmic. With ever breath, your relaxation will double. All the way in, all the way out. You are now twice as deeply relaxed. Again. Breath in, and let it out. Now even more relaxed."

THE PRESCRIPTIVE SCRIPT (Therapeutic Suggestions)

There are two basic types of prescriptive scripts: Direct and Indirect (or Ericksonian). Some say the differences between the two are simply a matter of the hypnotherapist's personal style, while others claim there is a difference in the client's response, depending on the style used. You will determine which style works best for you and your clients through practice and experience.

- **Directive**

  Directive hypnosis, also known as "skill building" hypnosis, is designed to help the client improve a specific area of life, and is frequently utilized for advancing skills in athletics, musical proficiency, reading, studying, dance, and so forth.

  In this manner, the hypnotherapist gives the client specific suggestions, much like direct instructions, for future behaviors and situational perceptions. This is similar to many cognitive-behavioral therapy strategies.

  > Trance is natural phenomena
  > Induction facilitates trance
  > Healing occurs through experiencing the body in audio, visual and kinesthetic ways
  > The subconscious responds to direct suggestion
  > Less interactive; instead you direct the client with suggestions formulated through assessment

- **Indirect / Ericksonian**

  Ericksonian style hypnosis does not use directive commands. You won't hear an Ericksonian hypnotherapist say, "You are now falling into a deep sleep." Instead, they will make more subtle suggestions like, "You *could* close your eyes now" or "You *may* be feeling sleepy."

  In this way, the hypnotherapist acts only as a facilitator or guide. The suggestion is still implied, but the client remains in control of their trance. This is a slightly more Rogerian approach to hypnotherapy.
  > Trance is natural phenomena
  > Induction is not nescessary
  > People generate psychic healing apart from directive commands
  > The subconscious is responsible for most behavior
  > Vantagepoint is more interactive, oftening interviewing patient

6 Hallmarks of Prescriptive Scripts

1.) **Visual**

The client may be instructed to visualize the desired goal or outcome as if it were already achieved to perfection, and/or to visualize themselves working through the identified problem and reaching the desired conclusion.

For example, they may see themselves relaxed and reading a book on an airplane (instead of anxiously figeting), or speaking confidently in front of a large group (instead of sweating and stuttering).

2.) **Symptom Targeted**

As with cognitive-behavioral therapy, the suggestions given will focus on specific identified issues, problems or situations, and are designed to provide alternative relief strategies or goals.

3.) **Metaphorical**

Hypnotherapy often makes use of metaphors in effort to show how specific concepts, perceptions, actions, beliefs and goals may relate, as well as to convey concepts which can later be translated and applied to multiple situations. They are also frequently used as relief mechanizms.

For example:

“Your daily stress is like the leaves of a tree. It builds from the roots, then rises. See your stress rising up through the trunk, and travelling out along the branches, thinning and thinning as it flows upward and outward, getting smaller and smaller as it reaches the end of the thin branches, then even smaller as it turns into beautiful green leaves. See the green leaves turn golden, then brown, and now watch them detach, fall off and float to the ground, discarded and no longer necessary, no longer existing.”

4.) **Body Awareness**

Hypnotherapy is quite effective for treating numerous physical problems and symptoms, and therefore suggestions related to body awareness are commonly used.

Body awareness scripts are good for those who are nervous or have anxiety disorders, as well as those who lose focus due to body preoccupations. Suggestions may be given to control breathing or heart rate during stressful situations, to raise or lower body temperature for sports activities, and even to minimize or eliminate allergic reactions. Body awareness suggestions are also frequently used for pain management - becoming aware of the absense of pain.

For example: “You can feel the hurt leaving the pain. There is less intensity now.”

5.) **Post-Hypnotic Suggestions**

Post-hypntotic suggestions are new patterns of behavior that are given during hypnosis,

with the intention of being incorporated into the client's life following hypnosis.

For example:

"When you feel like smoking, you will take a deep breath or a short walk instead."

Or: "When you feel the urge to snack, you will reach for healthy, natural foods, instead of processed chips and dip or cookies."

Stage hypnotists often end their show with silly post-hypnotic suggestions, like:

"When you hear the word "California" you will return to the stage and believe that you are the pop star Madonna. When you hear the song *Like a Virgin*, you will sing it at the top of your lungs and dance just like Madonna!"

6.) **Suggestions Must be Understood by the Client**

Sure, those with abstract reasoning skills will likely understand that the mental picture created in thier mind of walking confidently through a dense brush in the mountains symbolizes that he can make it though difficult times by using his inner strength of perseverance -- but the more cloaked the suggestion is in abstraction, the more difficult it will be to understand.

REMEMBER: Whatever the functioing level of the client is when they are awake, will be the functioning level of the client when they are asleep.

You must tailor the therapeutic suggestions of your prescriptive scripts to meet the intellectual, educational, social, economic, physical and emotional functioning levels of the individual client. If they would not be able to grasp the metaphorical relationship between 'the view of a sunrise from atop a mountain and a positive outlook on their future marital relationship' while awake, they will not be able to do it under hypnosis.

THE AWAKENING

Awakeings are generally simple. For example:

"I am going to count from one to three. When I reach three, you will open your eyes, feel alert and refreshed, full of energy, and go about the tasks of the day with renewed hope. One. Two. Three. Alert, refreshed and full of energy."

They can also be more complex or drawn out, allowing the client to awaken more gradually. In some situations, clients may also be given the option of staying hypnotized and going right into a natural deep and refreshing sleep.

OVERCOMING CLIENT RESISTANCE

If a client fears hypnosis, it is usually due to their prior life experiences. Perhaps they saw a stage hypnotist in person or on television, and fear that if they were hypnotized, they might be made to act in foolish or humiliating ways. Alternatively, perhaps the fear is simply a misguided product of their imagination, because they really know nothing about hypnosis whatsoever. Whatever the reason, if they fear hypnosis, it will likely negatively influence or impede the hypnotic process.

Common reasons why a client may resist using hypnotherapy for self-improvement purposes:

- The professional experience of a prior hypnotherapist was not adequate, causing past hypnotic suggestions to be ineffective.
- The client tries too hard to become hypnotized; they overanalyze what is happening during the process, instead of just letting it happen.
- The client is simply curious, and has no real intention of entering the hypnotic state.
- Extreme stress or anxiety may prevent the client from going into hypnosis for the first time.
- The client consciously tries to hold onto reality, in effort to control their behavior, reactions, and responses to suggestions.
- The therapeutic suggestions may be aimed at a problem that the client is not yet ready to address.
- The client may fear that they will say or do something while under hypnosis that they would normally not say or do.
- The client may fear not awakening from or becoming stuck in hypnosis.
- The client may fear he/she will not perform as expected while under hypnosis.
- The client may assume that they will fail, fearing he/she will not be able to follow through with the suggestions after hypnosis.
- The client may fear that they will actually succeed and achieve their desired objective.
- The client may simply fear new experiences and/or change, in general.

HYPNOTIC SCRIPTS / Suggestive Therapy

I like calling hypnotherapy or clinical hypnosis “suggestive therapy,” as suggestion is really the heart of clinical hypnosis. Most clients arrive with the knowledge or hope that we are going to help them change something that is distressing about their life, and they expect that we will make suggestions for how to do this.

In fact, the chief difference between hypnotherapy and traditional therapy is that in hypnotherapy, suggestion, even advice, is given. In traditional approaches the therapy, especially Rogerian and psychoanalytic approaches, direct suggestion or advice is not given, even avoided.

Clients like to hear suggestions. People like advice, hence the popularity of Ann Landers and Dear Abby long after their deaths, and the perpetuation of their columns in reruns and by ghostwriters. Perhaps this innate desire to have an expert guide us is part of why people seek hypnotherapy as a solution, and part of why hypnotherapy is successful.

The mind responds to suggestive therapy. Suggesting to a client that they can create mental pictures, or concentrate, is the mind responding to hypnosis.

The body also responds to suggestive therapy. When hypnotherapy is applied as anesthesia or treatment of a medical disease (like IBS), the body has the ability to respond to this suggestion.

The mind and body are one and the same. This may be a difficult concept for some who are rooted in a disease-concept understanding of pathology, but we are in total control over our bodies at all times.

I have seen stage hypnotists suggest that the volunteers feel like they are freezing or that they are sweltering in the desert - and they will respond by actually shivering or sweating, because that is the body’s natural response to those types of environmental situations.

In medical settings, the use of hypnosis to reprogram the mind’s understanding of and the body’s reaction to pain, to the point where dental and surgical procedures can take place, occurs frequently.

Nothing about the human condition separates our physical state from our mental state at any time. We can choose to gain or lose weight, to feel the physical manifestations of stress or to relax. If we stub our toe, we can choose to magnify the pain and become unable to go to work, or to modulate the pain, decrease its importance, and complete our essential tasks despite the injury.

Suggestion not only influences behavior, but all aspects of an individual’s life.

Suggestion is powerful, even when not formally a part of hypnosis. The next time you are with a group of people at home, open up a new container of milk, take a sniff, and make a face as if it were putrid. Then, pass it around for everyone to smell. Even though the milk is fresh, those in the room will most likely agree that it is rancid.

People use suggestion to manage relationships, all the time. In couples counseling, the guilty adulterer may say, “I did not have sex with that woman!” - and the spouse may simply agree with a blatantly false statement, and dismiss her feelings of betrayal.

A magician friend of mine once said that, “Performing magic is easy; all you have to do is tell them what they see, and they will see it every time.” Another magician friend once said, “People will believe ninety-nine per cent of what you tell them, no matter what you tell them, without question.” I believe that both of these concepts are true.

A few years back, a cult of computer programmers were told they could ride on the Hale-Bopp comet, if they mixed barbiturates with yogurt, wore Nike tennis shoes, and brought quarters to make phone calls. Twenty-nine cult members died in a California mansion believing this suggestion.

Suggestions can certainly be abused, but seeing the effectiveness of these abuses does help us realize its powerful impact.

9 Principles of Effective Suggestive Therapy

1.) **Phrase suggestions in the positive.**

Positive: “You enjoy breathing fresh clean air.”
vs.
Negative: “Whenever you smoke, you will breathe in filthy air and hate it.”

The mind does not respond near as well to negative statements as it does to positive ones. We want to look up and forward towards a positive goal, not at negative roadblocks.

If positive statements do not yet roll naturally off your tongue, take the time to write out the suggestions you plan to use (create a script), making sure to phrase all suggestive statements in a positive light, then read the script during the session.

2.) **Metaphorical suggestions should not be so abstract or foreign to the client’s experiences that the client cannot extrapolate the meaning behind it or relate to it.**

You probably do not want to use the metaphor: “It tastes exactly like the salty water of the ocean” to someone who has never been to the ocean or tasted it.

During the intake/assessment process, you will be able to get a feel for the client’s intellectual processing level. This will help you determine the best approach for suggestions.

3.) **Focus suggestions on single or closely related topics, in a single session.**

This statement: “You now have the desire to eat healthy, stop smoking, get up early,

complete a daily meditation, and go to they gym each day" imparts four too many topics in one session.
"Becoming healthy" is a general concept that can easily be woven into most sessions, regardless of the focus topic. However, if a client wishes to both quit smoking and increase their confidence in public, those two topic goals will require different therapeutic approaches, and need to be split into different hypnotic sessions, so that the session suggestions are focused, and effective.

4.) **Phrase the suggestions in the present tense.**

It does not really matter if the client was sloppy and habitually tardy in the past; for this session's purpose, the past is not relative to the future, and the suggestions will focus on their new behaviors, now.

There is a place for past-tense language, but even then, you are still in the present, looking retrospectively to see what accomplishments have been made, as foundation and encouragement for the future, i.e.: "You were able to do all this, and this proves that you are able to do more now."

Post-hypnotic suggestion includes tasks to accomplish in the future, but for the most part, "You no longer smoke," is regarded as more effective than, "You will stop smoking." (Hammond)

Conceptually, this may seem to be only a matter of semantics, but the mind is quite literal. "In the future" is not definitive; the future could be in ten minutes or ten years. This kind of general terminology provides an escape route by not defining a specific time and place. Remember: Tomorrow never comes. Frame your suggestions in the now.

5.) **Base the suggestions on the client's existing strengths and resources.**

Every client will have different resources, different strengths, different intellectual processing levels, and so forth, and thus you must create and frame the suggestions you use to match each client individually.

If you frame the suggestions outside of the individual client's existing strengths, resources and abilities, they will not be effective. You do not want to suggest that they call a friend for support every time they feel like smoking, if they - or their friend - are without consistent easy access to a phone. You do not want to suggest that they take up jogging or high-impact aerobics, if they have a bum knee.

This is where thorough assessment comes in to play. As you complete the intake/assessment, utilize a form such as the NSRI: Nongard Client Strengths and Resources Inventory, and make notes on what concepts or strategies would make good suggestions for this specific client, as well as those to avoid that would prove futile.

6.) **Suggestions should be attainable.**

You must frame the suggestions - just like the goals you are striving for - in practical,

functional reality; they must be realistic.

Realistic: “Each day, you will see yourself as being thinner and losing weight.”
vs.
Unrealistic: “You will be as light as a feather when this session is over.”

One simply cannot become light as a feather, and so this post-hypnotic suggestion will not be effective.

While a person can quit smoking instantly, a person cannot drop a hundred pounds when they walk out the door. The suggestions offered must be practical and realistic, designed specifically to meet the client’s practical and realistic goal.

7.) **Suggestions should incorporate the five senses.**

Visual imagery (what the client sees), olfactory descriptions (what the client smells), tactile descriptions (what the client feels physically), sound (what the client hears), taste (what the client tastes) during the practice of the suggestions are all-important areas to address. The more senses you can incorporate into the suggestions used throughout the session, the greater the efficacy will be.

When they survey the crowd in front of them, they will see friendly, attentive faces. They will breathe in fresh, springtime air. They will hear a faint relaxing melody. The floor will feel solid under their feet, and they will taste success.

8.) **Suggestions should relate directly to the individual client’s goals and presenting problems, rather than come only from a prepackaged, inflexible script.**

The world of hypnotherapy is full of scripts - scripts you have written, scripts others have written, scripts that you can download from the internet or purchase in books. There are prepackaged scripts available on just about every subject you can imagine.

These scripts are useful; they can provide direction or guidelines for addressing specific issues, and most of the time, the author has taken the time to use specific language and phrasing deemed beneficial or effective. However, as mentioned before, every client is different, and has not only different strengths and resources, but also different needs. Not every client needs to lose fifty pounds; some only wish to drop a few pounds and tone up their muscles. These are different goals, and will require different suggestions.

Especially when you are less experienced, it is fine to use these scripts as a tool for both understanding the hypnotherapeutic process and implementing suggestions - but you also want to take the time to tailor the script to meet the specific needs of each individual client.

The scripts are not written in stone, and can be easily modified; you may substitute individual words (fluffy for feather) or entire concepts (climbing stairs instead of a mountain). You may take a section from one script and insert it in another.

As you become more experienced, you will likely rely less and less on complete scripts, and instead you may utilize a hastily sketched outline that you develop during the intake/assessment process, designed to meet the specific needs of the client.

9.) **Remember the law of dominant effect: A stronger emotion will always replace a weaker emotion. Attach significant emotions to change.**

The mind is an amazing machine; modern computers can hardly rival its ability to categorize and index thoughts, feelings, sensations, corollary relationships, entire experiences, and so forth. Each item in the mind's database is simultaneously organized chronologically, by intensity, by location, by time, by person, by emotion, by smell, by preference, by pain, and so on.

When you suggest that the mind retrieve an emotion, the mind will assess its meaning, function, relevance, relationship, and intensity to everything in its database. To facilitate positive change, the hypnotherapist needs to get the mind's attention in a big way. Using suggestions with stronger emotional impact will increase efficacy for change.

For example, "Now that you are a nonsmoker, you are enthusiastic about life, and feel as energetic as a person ten years younger," is far more attention getting to the mind than, "When you quit smoking, you will feel better."

Better, relative to what? Better is a generic term that the mind will pass over as being less important and intense than even 'good' - and "when" is not concrete or specific.

Better than you felt yesterday? That is more specific, but still not dramatic enough to take much notice.

Better than ever before? Well, that is better, but is still neither specific nor intense enough to cause the mind to take active interest.

Good -> Better -> Decent -> Great -> Wonderful -> Fantastic -> Amazing

Give the client's mind specific emotional cues, using terms associated with high intensity.

We have discussed directive and indirective suggestions. These two styles of hypnotherapeutic approach correlate with authoritative and permissive suggestion styles.

Directive / Authoritative Suggestion:
"You no longer feel tension and pain in the abdomen."

Indirective / Permissive Suggestion:
"Because you want to be pain free, you have the ability to let the pain leave your hurt."

Script length

Those with experience providing hypnotherapy will likely find short, concise prescriptive suggestion scripts to be satisfactory, especially with clients who are also experienced with hypnotherapy.

However, those new to hypnotherapy will likely find that becoming familiar with the process through using longer scripts filled with repetitive suggestions to be more beneficial to both the hypnotist and the client, until you both develop a comfort level.

Dehypnotizing (Awakening)

When you are done with the hypnotic suggestion session and are ready to wake the client up, you want to incorporate four suggestive concepts into the awakening process to:

1.) Easily move out of trance
2.) Empower the client
3.) Motivate client to take action on the prior suggestions
4.) Energize the client

Methods:

1.) Simple count, 1-5. (Opposite of the deepening, which is a count down, 5-1.)
2.) In some settings (hospital, home, or even on production CD's) client can be given option of waking up feeling refreshed or may choose to go into a deep and natural sleep.

Resistance to awakening:

1.) The client can be left alone; they will reawaken eventually. This is not generally useful in office/clinical setting.
2.) Work with the client's resistance: "This feeling of hypnosis is something you enjoy, so I will allow you some more time to relax." Give the client a minute or two. Then, "Now it is time to wake up, feeling refreshed and energized." Go into 1-5 count.
3.) You can clap on 5, and if the client remains in state of hypnosis, sharply blowing on their eyelids will energize them rapidly.

ABREACTION

Even though I am opposed to the practice of inducing abreaction just for the sake of abreaction in most situations, especially by those who are not both highly skilled and licensed as a psychologist or medical practitioner, it is a necessary part of hypnotherapy to discuss, if for no other reason than it does occasionally happen spontaneously.

We can define abreaction in a number of ways. In the broadest sense, since hypnosis is generally regarded as a relaxing experience that makes one feel good, abreaction then can be defined simply as a "bad trip' or unpleasant experience in hypnosis.

The hypnotist may intentionally direct abreaction, or it may occur spontaneously and for a variety of reasons. Rarely does this happen in entertainment (stage hypnosis) settings, but it is a risk for which every hypnotist must be prepared.

The ethical hypnotherapist always aims to avoid doing harm. Therefore, to intentionally cause abreaction with no therapeutic purpose in mind can be indicative of a poor assessment of the client's condition, or of attempting to treat a client with hypnosis when their psychological difficulties would best be addressed through traditional counseling or psychiatric approaches.

The narrower, clinical definition of abreaction is the situation when a client re-lives or re-experiences past traumatic life events, as if they were actually occurring now.

Painful memories are difficult for some people to examine and deal with. Re-living the pain — tasting, hearing, feeling, smelling and visualizing the trauma as if it were actually occurring at that moment — are hallmarks of clinical abreaction. Whether it occurs spontaneously or through hypnotherapist lead direction, the client has regressed in time to a point of trauma, and in his own mind is in that exact time and place, re-experiencing the trauma.

Years ago, I practiced substance abuse counseling in a drug and alcohol treatment center where on staff we had a therapist who specialized in Pyschodrama. I sat in on her groups on a regular basis and watched as she lead open-ended sessions, sometimes for several hours, where people acted out the traumas of their past. The emotional catharsis was intense, far more intense than traditional individual or group therapy. Sometimes people would break down and become catatonic for a while, and other times intense anger would manifest physically, and patients occasionally had to be restrained.

I have always wondered what the real purpose was behind these sessions. It has always seemed to me that if a client is lucky enough to have repressed the intensity of their emotions in regards to past trauma, why would we charge clients money to have them re-experience these traumas through either psychodrama or hypnotherapy?

Forgive the crude example, but I also think that the pressure put on clients to perform "emotional catharsis" in psychodrama or hypnotherapy is a lot like forcing someone to urinate, when they don't really need to go. After a few hours on the pot, they may be able to squeeze something out - but at what price? I think forcing abreaction has little therapeutic value, and instead meets the *therapist's* own narcissistic needs, and so my metaphorical comparison is probably appropriate.

Although I do believe it is important for clients to understand the past, I do not believe they must relive the past to get well. While understanding the past may make some changes easier, I

do not think it is essential for change to occur - because truly, understanding the past is not a part of the change process; it is more of a tool for relapse prevention.

Change is not something easily or quickly seen; tears, anger and intense emotions are, and thus provide a sense of instant gratification. In my opinion, this is the primary reason some therapists seek abreaction; they want to document something happening.

Another improper reason, in my view, that hypnotherapists seek abreaction is because it puts the therapist in a position of power over the client. The creation of an unequal relationship - where the therapist has power over the intensity of a client's emotions - is an abuse of power. Generally, this is rather narcissistic behavior on the part of the therapist, and can lead not only to bad therapy but also to liability - especially if the accuracy of the abreactive memories later proves to be false.

Abreaction may also be used improperly as a tool for financial profit. One of the great things about hypnosis is that it is the ultimate intervention in brief-therapy; it works. This, of course, means the client does not need to come back - which may cause some therapists financial difficulty. Abreaction is amazingly emotionally intense, and like crack cocaine, dependency occurs easily. Create abreaction dependency, and the pocketbook swells.

Notice, however, that I did not say 'create an addiction to hypnosis.' A person cannot become "addicted to hypnosis" -- but they can become emotionally dependent on a therapist.

That all said, there are some valid therapeutic uses for abreaction, but the validity of the details of the recovered event must be confirmed, as not all memories are a hundred per cent accurate, and clients' memories can be mislead.

Directed Abreaction
The hypnotherapist uses a regression technique designed to take the client back to a specific emotional situation or event of their past. This generally causes the client to abreact, which should be no surprise.

When used properly, a therapeutic abreaction has only two purposes: First, to discover what happened, and second, to discover who, if any, the players are. Once this information is determined, the hypnotherapist should terminate the abreaction and begin transformational therapy.

Non-Directed Abreaction
The hypnotherapist will instruct the client to go back in their mind as far as needed to arrive at the first situation or event that caused the specific issue he wants to resolve. This is somewhat technically a 'directed' regression, as you will send the client to a specific place, but since you do not know exactly when or where the place is, this process is termed 'non-directional.'

The Spontaneous Regression
When some clients are hypnotized, they will spontaneously fall into abreaction and start crying, yelling or flailing about, because they have regressed to a traumatic event on their own. Unfortunately, you cannot pre-determine which clients might do this by looking at them or talking to them. The subconscious mind sometimes protects a person from having to deal with traumatic events - such as rape or a terrible accident - by blocking that event from their

conscious memory; so while they may appear to be emotionally well-adjusted on the outside, they are actually subconsciously suffering from some deep emotional situation, which may spontaneously resurface under hypnosis.

The Recreational Abreaction
People are interested in being regressed back to a specific or general point in their past for various reasons. Some believe in the concept of past lives, and want to explore the possibility. I do not advocate this practice, period.

The hypnotherapist must remember that whenever he uses a regression technique - of any kind for any purpose - the possibility that the client will arrive at a highly emotional situation that causes them to suffer a strong - and perhaps quite painful - abreaction is always a factor.

How to handle abreaction:

1.) If the client did not come to you to resolve the problem that the abreaction exposes, stay away from doing any therapy for it. That is not the issue they are paying you to resolve.

2.) Never physically touch a client in any way during abreaction.

3.) During abreaction, the hypnotherapist must remain calm and not show fear of or for the client. Unconditional positive regard is essential at this point.

4.) To emerge the client from abreaction, use a script that brings them into the present, such as, “You can breath smoothly again now. The picture in your mind is fading and becoming more distant. Whatever you are seeing and experiencing is fading away, and has passed into yesterday. You are now in the present, in my office, and breathing smooth breaths of fresh air.”

5.) It is my contention the therapeutic value of abreaction is overstated, and that if you practice directed abreaction, you need a medical or psychology license, or you may be liable for practicing medicine or psychology without a license.

HYPNOTIC PHENOMENA

Just as every person is different and has different life experiences, every person will have slightly different experiences when under hypnosis, ranging from changes in reasoning, awareness and their creative imagination, to physical changes in blood pressure, heart rate, body temperature and other sensory perceptions.

We refer to these changes as hypnotic phenomena. Hypnotherapists may utilize certain phenomena for specific treatment purposes, while stage hypnotists may exploit certain phenomena for entertainment purposes.

I remember my first stage show as a hypnotist as if it happened yesterday. Because there is really no way to practice a stage show before you go live with it, you have to get it right the first time - but with no practical experience in stage hypnosis, it can be difficult to develop the confidence (some might call it narcissism) necessary to pull off that first show.

During the show, I suggested to a lady that she would forget the existence of the number "2". This is a frequent stage demonstration, because not only can be quite humorous, it is simply amazing to witness.

I told her to hold out her right hand, and count her fingers. She looked at me, looked at her hand, and counted: "1, 3, 4, 5, 6," then gave me a sheepish grin.

The audience was blown away - but so was I; I was shocked that it actually worked! I then had her hold out her other hand and count all of her fingers. She did and said, "1-3-4-5-6-7-8-9-10-11." With this, the crowd went wild, and I tried to conceal my own amazement.

After the show, still holding on to a little disbelief myself, I talked to her about her experience. She remembered little about the hour she spent on stage, and had clearly been in a deep trance state.

I knew beforehand that demonstrating these kinds of hypnotic phenomena is the bread and butter of stage hypnosis shows, because it is how the hypnotist impresses and amazes the audience and creates entertainment. However, witnessing it as a participant and being the one to make it happen are two different things.

After I got over my shock that it worked so easily, I learned something from my experiences on stage: hypnotic phenomena is easy to produce, because even though these phenomena may seem odd or strange when isolated as a direct command on stage, they are actually natural extensions of life experience; every manifestation of hypnotic phenomena occurs to one degree or another in real life.

For example, 'perceptual changes' are a frequent hypnotic phenomena. The stage hypnotist uses this when they suggest to participants that they are freezing cold - and the group then instinctively huddles together in comical ways to stay warm. Mothers use this same phenomenon when their child bruises a knee, by suggesting that Mom can kiss the 'owie' and make the pain go away.

Singing and dancing like a rock star is a powerful stage demonstration of disassociation, but in real life, most of us depart in our mind from the stressors of the day to our daydreams, and students are notorious for creating alternate realities in their heads during college

lectures. The athlete who hurts himself during the big game, but suppresses the pain so they can continue to do what is most important to them, essentially delaying the pain response until the final whistle, is practicing what can be considered hypnotic phenomena.

The clinical hypnotist can use hypnotic phenomena to help clients in a variety of ways. Obviously, in medical situations, manifesting analgesia, anesthesia and ideodynamic responses can be useful. Helping a client take physical control of their body for panic disorder, test anxiety or nicotine withdrawal through disassociation, sensory perceptions, and other phenomena can be useful. Helping test-takers, golfers and athletes create dual-realities can be of incredible performance value to them. Techniques of age-regression can help a person to resolve past difficulties by gaining new perspectives on their life events, and the phenomena of time distortion is a great stress management tool.

Now that I have more experience in hypnosis, I can understand the great precautions that a stage hypnotist must take in order to protect his participants, and I can also see the potential for abuse. However, unlike the AMA or the APA who despise stage hypnotism, I still think that responsible demonstrations of hypnotic phenomena can actually enhance the entire field of hypnosis, including clinical hypnotherapy. I regularly attend stage hypnosis shows in Las Vegas and elsewhere, and often take my friends who are curious about hypnosis, because they can see the power of hypnotic phenomena; they see people having a good time and realize that hypnotic phenomena are not something to fear.

Examples of Hypnotic Phenomena

Selective Amnesia

While in a hypnotic state, the subject may ‘forget’ certain facts or concepts as suggested by the hypnotist. Stage hypnotists often demonstrate this phenomenon by having the participant forget the number two, or even their name, sometimes only being able to recall it by a specific cue, such as singing “Happy Birthday to me, Happy Birthday dear (my name).”

Muscular Catalepsy

This is a physical condition characterized by muscular rigidity; the limbs become so stiff that they will remain in whatever position they are placed - or told to be placed - and they do not respond to external stimuli, be it verbal or physical. Muscular catalepsy occurs in a variety of physical and psychological disorders, such as epilepsy and schizophrenia, and can be intentionally induced by hypnosis.

Analgesia

Analgesia is the phenomenon of deadening or absence of the sense of pain, without loss of consciousness.

Anesthesia

This is the induced local or general insensibility to pain, with or without the loss of consciousness.

Disassociation or Dissociation

This is the state of being unconnected in memory or imagination, when some integrated part of a person's life becomes separated from the rest of the personality and functions independently.

Positive Hallucination

This is a suggestive phenomenon where the client experiences the perception of a visual, auditory, tactile, olfactory, or taste sensation - without any real external stimulus - but as if the sensation were real.

Negative Hallucination

This is opposite of a positive hallucination; a frame of reference is created so that the client does not perceive what is actually present. For example, through hypnotic suggestion, they do not smell the lemon under their nose, or see the color red.

Sensory Perception

It is amazing how you can turn the senses up and down or on and off through suggestion.

Time Distortion

Not only can you be suggested to feel as if you are freezing or sweating when under hypnosis, but you will likely have no true idea how long that sensation lasted, either. Most people naturally greatly underestimate how long they were under hypnosis. After a thirty-minute session, when asked, they may respond that they believe they were only “out” for about five or ten minutes.

Involuntary Movement

In a hypnotic state, clients will feel very relaxed and will hear everything you say. This state encourages the client to stop moving and pay attention, but occasionally involuntary movement will occur, such as fluttering of the eyelids or slight twitchy movement in the fingers or legs as the muscles relax and pulse and breathing rates become slower. Clients may also move involuntarily as a natural instinctive response to your suggestions, like a reflex.

Perceptual Changes

Thought [created through suggestion] can give rise to specific patterns of muscular tension and activity, particularly in those muscles that are symbolically represented in the thought in question (Weitzenhoffer, 1953, p. 246). There are also a number of experiments where it has been shown that various physiological and perceptual responses can be evoked by means of waking suggestion (Barber 1961, 1965). Among the responses we find such things as heart acceleration and deceleration, color blindness, deafness, autonomic changes, salivation, analgesia, and allergic dermatitis. (Heart acceleration, for example, could be produced by words associated with fear-producing stimuli; i.e., by suggesting something fearful.)

A DISCUSSION ON MEMORY AND HYPNOSIS

Vivid memory recall can be an important component of hypnosis to help us concentrate on specific parts of past events. Forensic hypnotists may use memory recollection strategies to help victims of crime recall certain aspects of a situation that may help investigators to solve crimes. Enhancing creative memory power for test taking is a growing discipline within the hypnotherapy industry. However…

****Caution**** Yes, hypnosis can help enhance the recall of memories - but memories are phenomena in and of themselves: they are fluid, suggestible and they can change. The concept of "repressed memories" (particularly of childhood sexual abuse or alien abduction) has plagued hypnotherapy and psychology since the late 1980's - early 1990's. While a person may at times fail to recall details of specific events, the brain does not work in a manner that represses all memory of trauma (Loftus, *Myth of Repressed Memory*, 1994). In fact, for most clients, their difficulty is not in failing to recall specific life events, but rather, being unable to let go of distressing visual and emotional recollections of past events.

Nobody ever went to hypnotherapy in Kansas in 2006 and suddenly recalled, "WOW! Until now, I forgot that I *was* in New York at the World Trade Center on September 11, 2001. It must have been so painful that I repressed it!" For the unfortunate victims of devastating traumas, their recollections are usually too close to the surface, real and vivid.

These days, therapists who practice "repressed memory therapy" are only a small minority, and the number of such practitioners will continue to dwindle as liability continues to force them from the profession. However, the damage done to the overall reputation of hypnotherapy and clinical counseling by the repressed memory movement of the 1990's will continue to negatively affect professional stature for decades to come.

> "The notion that traumatic events can be repressed and later recovered is the most pernicious bit of folklore ever to infect psychology and psychiatry." ~ Richard McNally, Harvard University

> "...it is an unsettling fact that we can manufacture, wholesale and out of pure nothingness, whole events and pasts that never occurred. This fantastical creative ability of our minds may be treasured when it produces King Lear or War and Peace, but it can sometimes destroy lives and families when applied to ordinary, daily life." ~ Elizabeth Loftus.

> "There is general agreement among memory researchers that memories of events which happened before the age of 24 months are never remembered into adulthood and cannot be recovered."
> ~ religioustolerance.org

I have actually met licensed mental health professionals - those with master's degrees and doctorates - who have helped clients "uncover" fantastic, fictional creations of the mind while under hypnosis, including alien abduction and visitations of spiritual beings. While in America you have a right to any religious belief you choose, there is no place in professional hypnotherapy for the practice of religious myth. These types of beliefs are as fantastical and fanatical as the Christian who fears hypnosis opens a doorway in the soul to the devil's foothold.

IDEODYNAMIC RESPONSES

Ideodynamic Responses are those automatic or conditioned responses that are generated on an unconscious level by the subject in response to stimuli either external or internal.

Unlike the other induced phenomena that we may experience while in trance, Ideodynamic Responses occur no matter what one does. There is literally no way the individual can prevent unconscious body movements, or keep from re-experiencing feelings and sensations.

However, responses to feelings, movements or sensations suggested by the hypnotist, can be enhanced, the greater the degree of dissociation present, since Ideodynamics are by definition Unconscious Responses. Therefore facilitating dissociation is a necessary first step prior to attempting procedures such as automatic writing, or finger signaling.

There are three kinds of Ideodynamic Responses:

- Ideoaffective
- Ideomotor
- Idesosensory

Ideoaffective - Emotional Manifestations

Emotional manifestations are the automatic responses attached to the experiences of each individual. It is virtually impossible to be entirely devoid of feelings about something. Every person exhibits and attaches a wide range of emotional responses to various events in their lives, and these responses may differ in temperament and in the degree of intensity from person to person, and situation to situation. While in trance, the individual's feelings associated with the ideas contained in the hypnotherapist's suggestions may come to the surface, such as buried negative feelings of hurt and despair, or positive feelings of joy and pleasure.

Ideomotor - Physical Manifestations

The body's unconscious reactions to one's thoughts are often referred to as Ideomotor Responses. This is also more commonly called 'body language'.

In therapeutic settings, the ideomotor response can be used to facilitate dissociation, to deepen the trance state, and as an indicator of responsiveness.

Idosensory - Sensory Manifestations

These are mental experiences, automatic experiences of sensation associated with the processing of hypnotic suggestions. The hypnotist will give a detailed description of the various sensory components associated with an experience, which allows the patient to re-experience those sensations to a degree determined by the amount and type of past personal experience with it.

Age Regression

- Revivification - re-experiencing an event as if it were real right now
- Hypermnesia - creating a vivid recollection as an observer of the past

This may sound weird, but it's really not. We all do this when looking at old photos, or when we hear a song and it takes us mentally back to a time in our past that has significance.

Therapists who dwell on the past will make little progress, but sometimes, understanding how our past has brought us to the present is essential.

One of the most effective interventions is assessing what we did in the past— but stopped doing —that could help solve today's problem. It is always easier to draw from existing but underutilized coping strategies, than to create new one. Age regression can help us do this.

Age Progression

> Age progression means projecting oneself into the future. Not astral projection or fortune telling, but seeing oneself as you would handle situations and manage challenges. For example, creating a mental picture of being able to breath easily during strenuous activity (for the smoker), or being able to see oneself as making healthy eating choices at a buffet or when shopping, or seeing oneself in old age, content because you have drawn upon your resources to problem solve.

HYPNOSIS WITH CHILDREN

One night my son, who must have been about six at the time, just could not get to sleep. He kept tossing and turning, and whining about this and that. The usual goodnight prayer and a kiss strategy had no effect, and getting him to sleep this particular night was taking over an hour.

At the time, I was working on a project and preparing to head out of town the next day to teach a professional education seminar. I had to finish my work, and my son's constant neediness was both distracting and frustrating me.

He pleaded with me to stay in his room. He wanted my presence. But I had already spent over an hour with him.

They say desperate necessity breeds invention, and suddenly, I had an idea. I went to my office and retrieved a cassette player (this happened a long time ago) and a Progressive Muscle Relaxation tape that I had produced for use with clients. I took it into his room, turned it on, and went back to my office.

After about five minutes, the silence was deafening, and so I went to check on him. My son was lying in bed with his eyes closed, breathing deeply and clenching and unclenching is fists. He was following along with the instructions - not just listening to the tape, but actually doing what was suggested!

This was my first experience with children and hypnosis. Since this time, I have encouraged parents, teachers and others to use PMR and hypnosis with children. There are no contraindications with hypnosis and children. In fact, children usually have an easier time with hypnosis than adults do, in part because they are not yet jaded by the real world and seem to be quite capable of using all five senses in their imagination. Any stage hypnotist will tell you that high school Grad Nights are their favorite performances, because kids do well at hypnosis.

Research indicates that pediatric hypnosis is effective:

Hypnosis in pediatrics: applications at a pediatric pulmonary center

By Ran D Anbar
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BMC Pediatrics 2002, 2:11 doi:10.1186/1471-2431-2-11

Background
This report describes the utility of hypnosis for patients who presented to a Pediatric Pulmonary Center over a 30-month period.

Methods
Hypnotherapy was offered to 303 patients from May 1, 1998 – October 31, 2000. Patients offered hypnotherapy included those thought to have pulmonary symptoms due to psychological issues, discomfort due to medications, or fear of procedures. Improvement in symptoms following hypnosis was observed by the pulmonologist for most patients with habit cough and conversion reaction. Improvement of other conditions for which hypnosis was used was gauged based on patients' subjective evaluations.

Results
Hypnotherapy was associated with improvement in 80% of patients with persistent asthma, chest pain/pressure, habit cough, hyperventilation, shortness of breath, sighing, and vocal cord dysfunction. When improvement was reported, in some cases symptoms resolved immediately after hypnotherapy was first employed. For the others improvement was achieved after hypnosis was used for a few weeks. No patients' symptoms worsened and no new symptoms emerged following hypnotherapy.

Conclusions
Patients described in this report were unlikely to have achieved rapid improvement in their symptoms without the use of hypnotherapy. Therefore, hypnotherapy can be an important complementary therapy for patients in a pediatric practice.

Complementary and Alternative Medicine: A Focus on Hypnosis in Children and the Integrative Treatment of Atopic Dermatitis
By Tammy C. Tempfer, MSN, RN-C, PNP

Indications for Hypnosis in Children
The indications for hypnosis include patient motivation and a problem that is potentially treatable with hypnosis. The patient needs to be motivated to remedy the problem, and a good rapport must exist between the patient and the clinician.

Numerous pediatric problems have been successfully treated with hypnosis, including anxiety associated with procedures or illness, asthma and allergies, enuresis and encopresis, and habit disorders like smoking, tics, nail biting, and thumb sucking. It has been used to enhance self-esteem and self-confidence, and for sports enhancement, sleep problems, and attention deficit disorders. Hypnosis has also been successfully applied to enhance surgical anesthesia and ease discomfort during dental procedures.

AGE REGRESSION

As previously discussed, age regression is a valid hypnotic method - for any time you think back to a time prior to the current time, you are technically regressing. And while it is one of the most commonly used hypnotic methods, it is also perhaps one of the most controversial. This primarily stems from the fantastical claims by untrained hypnotherapists that they can regress clients back to infantile states with memory recollection, or even go back past this lifetime and into prior lives.

Of course, neither of these things is possible. Although one can mimic infantile behaviors, this knowledge comes from learned behavior, from observing the functioning of infants, rather than accurate recollections of one's own infancy. Now, while it may be true that I cannot prove scientifically that past-lives do not exist, it is also true that people believe in all kids of things, ranging from the Easter Bunny and personal visits by aliens, to gaggles of virgins awaiting suicide bombers in heaven, which I also cannot prove or disprove.

Regardless of your beliefs or the beliefs of your clients, infantile and/or past-life regression has no place in clinical hypnotherapy. Even if it were possible, it serves no useful purpose in clinical practice. People simply have enough to worry about in this life, without having to manage past lives as well.

A secondary reason for age-regression's controversial status is simply its overuse. While it can serve a valid purpose, like other forms of psychotherapy it become a crutch.

I have always liked Gerard Egan's model of problem solving in therapy. His approach recognizes the importance of the past, but in his book, *The Skilled Helper,* he defines nine stages of problem solving, with only one of those nine devoted to the past.

Clinicians of all kinds, including hypnotherapists, can become caught up in overemphasizing the past. Why?

- It fills another billable hour.
- It is interesting.
- It is easy.
- They are lazy.
- It provokes strong emotions (which is a way for clinically immature therapists and clients to emotionally *feel* that progress is being accomplished.)
- It is perhaps a permeation and holdover from Freud's theories and influence in all branches of psychotherapy.

Clients like dwelling on the past because:

- It is easy.
- It produces catharsis.
- It avoids action.
- It creates the "illusion of being busy".
- It can be a source for blame-shifting.

Age regression is not mystical; it is a natural phenomena. It happens when we hear an old song on the elevator that transports us mentally to our wedding day. It happens when we look at old photos. It happens when we recall specific dates, like death anniversaries or holidays. There is not a Christmas that goes by that I don't re-experience emotionally and visually the images of being a child and sitting on my grandmothers brown Persian rug awaiting the gifts I was to receive, and even thirty-plus years later, and I can still remember specific details of playing with those toys. It happens spontaneously as I watch my own children open gifts or as I daydream while driving to visit relatives or reclining in my seat on American Airlines.

Now that these cautions have been thrown out, we do not want to throw the baby out with the bathwater. As Egan points out, framing the past is often a key to problem solving.

Value of the Past

1.) Knowledge Database

Most of us have the strategies for problem solving already within us, and bringing these to the forefront is highly effective. Take weight loss, for example. We already know what is healthy and unhealthy to eat. We already know we need to exercise. There is no need to buy a prepackaged diet of prepared food, but rather we can simply draw on the knowledge we already possess. Hypnosis can help bring this knowledge to the forefront of our consciousness.

Michael Johns called me one day after his show in Vegas and complained of being bored and "in a rut." Instead of telling him to develop new skits and routines, I suggested he look back to his first twenty shows and consider what made them so exciting, then find a way to re-implement the music, skits and communication patterns with his audience that led to his current status as one of the most successful stage performers in Las Vegas. The concept I conveyed was that if it worked before, it could work again.

When working with addicts on relapse prevention, I have them describe times they normally would have used tobacco, drugs or alcohol but did not, and to identify the coping strategies they used during these periods of abstinence.

2.) Emotional Catharsis

Experiencing grief, anger or other intense emotions can have value. When working with probationers in criminal justice counseling settings, most clients do not have any trouble expressing or experiencing their anger. Their problem is often in over-showing their emotions, and a ventilationist approach is not appropriate - even contraindicated - for these folks.

But, everyone is different. I also once had a client who was a 'people pleaser.' Although I despise the term, I cannot think of a better one to describe this particular client: she was "codependent". She had never once acknowledged her anger. When, eventually, she finally said that she was angry, I had her yelling it out as loud as she could. The ventilationist approach can be useful with some clients, some of the time.

3.) Regression can help a person desensitize.

While I now fly all over the world all the time, there was a period of about ten years when I would not fly at all - not from Dallas to Los Angeles, or even from Dallas to Austin. I had always flown before, but then I quit. I became the champion of driving. "You can't see all the neat little towns along the way when you fly," I would say in my defense. Or, "I have to carry so much stuff with me, driving is just easier," I'd claim, heading out the door for a twelve hour drive with an overnight bag and a box of books.

Why? Because I had become afraid to fly. I was on a three-hour flight, in a rainstorm, over the Appalachian Mountains, and it was nothing but horrendous turbulence from take-off to landing. (At least it seemed that way at the time.) It was so traumatic that I was determined never to fly again. There was no reason to; why fly when I could drive?

Obviously, I have since learned to manage my fears, but if I had utilized the power of age-regression in hypnosis, I would likely have been flying again years earlier.

Regressing to a specific point in time - that flight, for example - and re-experiencing the event from a safe distance as an observer rather than a participant, can desensitize the person to a point where they can better rationalize the event and lessen the intensity of their reactionary emotions.

This concept is a basic tenet of behavioral therapy, and is quite effective in hypnotherapy.

4.) Forensic Memory Recall

Regression can be used in forensic hypnotherapy, but you must be cautious, because memories are not like a tape recorder. As discussed, memories can be easily influenced, distorted and created.

It is also important to note that memories initially recalled in hypnosis are often NOT admissible in court. If a client believes that when under hypnosis they will recall events necessary to testify in a court of law, they must FIRST seek other ways to verify the memories. Hypnotherapists should take care to make this a condition of "informed consent" in forensic hypnosis.

APPLICATIONS OF HYPNOSIS

When people ask me if hypnosis is effective, I tell them, "Hypnosis is effective for those who hypnosis is effective for." I believe the research, and the research shows that people with medical problems, emotional problems and undesirable habits can overcome their difficulties using hypnosis. I believe it is a preferred modality of treatment in many cases, and for most clients, because it is natural, and it is primarily a form of brief therapy. In today's world of limited time and resources, brief therapy is a preferred method.

But again, hypnosis is only effective for those who it is effective for. Only quacks believe that only one modality of treatment always works and is effective for everything and everyone.

I know many chiropractors, some who even practice hypnosis. One of these friends recently had orthoscopic surgery on his injured knee. Obviously, he did not reject traditional surgical procedures to deal with his orthopedic condition. Most chiropractors are not quacks. They use chiropractic medicine as a conservative treatment to avoid invasive procedures, and most chiropractors will certainly refer to a surgeon or other medical doctor when necessary.

But I have also meet a few chiropractors who *are* quacks. They believe that ALL human disease, pain and suffering is in the spine, and they detest traditional medicine. They think adjustments can cure everything from ear infections to cancer and psychiatric difficulties, and they never make referrals.

(Out of fairness to my chiropractor friends, in my opinion, some MD's - who believe (or at least practice as if) medication is the cure to all human difficulties - are quacks too. We know, for example, that exercise three times a week is MORE effective than psychotropic medication for the treatment of major depression, yet many doctors will automatically write a prescription for pills instead of helping the client develop an appropriate exercise regimen. Why? The pill approach is simply faster and easier for the doctor. When I asked an anesthesiologist why he does not use hypnosis during childbirth, his reply was honest: "I can make $1400 in five minutes giving an epidural, or $100 doing a forty-minute hypnosis session. You do the math.")

Different things work for different people, and research proves that hypnosis works for a lot of people. However, it is not something to apply indiscriminately to all clients for all conditions. Sometimes hypnosis is the better choice, and sometimes it is not. For those clients who are willing to learn new skills and seek brief intervention via a natural approach with demonstrated efficacy, hypnosis is effective and should be considered a first-line intervention, rather than relegated to the back of the clinician's repertoire.

Hypnosis in Contemporary Medicine
By James H. Stewart, MD Mayo Clin Proc. 2005;80:511-524

Hypnosis became popular as a treatment for medical conditions in the late 1700s when effective pharmaceutical and surgical treatment options were limited. To determine whether hypnosis has a role in contemporary medicine, relevant trials and a few case reports are reviewed. Despite substantial variation in techniques among the numerous reports, patients treated with hypnosis experienced substantial benefits for many different medical conditions. An expanded role for hypnosis and a larger study of techniques appear to be indicated.

Hypnosis for Common Behavioral Issues

Smoking Cessation
Other than weight loss, smoking cessation is probably the most prevalent use for hypnosis.

Why it works:
Hypnosis is effective because helps a person manage the physical effects of withdrawal. Hypnosis in and of itself can be an alternative coping strategy.

Hypnosis also works because it changes the subconscious associations people have with cigarettes and replaces them with new associations. Many smokers report that after they quit, they still reach for cigarettes in their purse or front pocket. These people have quit without hypnosis. Appropriate hypnotherapy for smoking cessation will change automatic associations and behavior patterns. The smoker may have associated getting in the car with a cigarette, taking a break from work with a cigarette, and lighting up after an orgasm while relaxing in bed. Chances are, they have at one time or another lit one cigarette, set it down, and then lit another cigarette, unaware of the first one still burning in the ashtray.

The mind is interesting; it is like a computer, remembering everything that we do and creating automatic behaviors out of mundane life experiences. Without reprogramming these automatic associations, quitting smoking can be a difficult task. However, with hypnosis, changing these automatic behaviors is possible; the old associations are erased in the computer of our mind, and the new and healthy patterns are committed to the subconscious.

Evidence that it works:

Researchers at Texas A&M University System Health Science Center College of Medicine reported 81% of patients referred to hypnotherapy for smoking cessation reported they were smoke free at the conclusion of a three-session treatment program.
~ 48% reported they were smoke free at the end of a year.
~ 95% reported they were satisfied with the treatment they received.

The Dana Farber Cancer Institute in Boston found, "These results suggest that hypnotherapy may be an attractive alternative smoking cessation method, particularly when used in conjunction with a smoke-free worksite policy that offers added incentive for smokers to think about quitting."

In a study of the American Lung Association of Ohio Hypnotherapy Smoking Cessation Program, it was found that only 20% of clients prescribed nicotine replacement therapies were compliant and that "Hypnotherapy smoking cessation treatment offers an alternative cessation method, which may meet the unique needs of certain individuals."

In the Ohio study, a randomly selected sample of participants completed telephone interviews up to 15 months after attending a treatment session. Twenty-two percent of participants reported not smoking during the month prior to the interview.

Cautions:

Avoid aversion scripts and negative suggestions. Your clients will be more successful with positive live-changing suggestions.

Additionally, suggestions (both when awake and under hypnosis) to utilize adjunct relapse prevention supports (Nicotine Replacement Therapies, call a friend, breathe deeply, exercise, etc) are highly recommended for greatest efficacy.

- **Drug/Alcohol Withdrawl**

Clinical hypnotists who claim smoking cessation as their territory baffle me when they state that hypnosis should not be used for alcoholism and drug addiction treatment. I spent over ten years working as a drug and alcohol counselor in traditional 12-Step oriented treatment centers, and I cannot think of a more effective tool to help clients manage withdrawal symptoms than hypnosis. If hypnosis can alter the subconscious association people have with nicotine, surely the same principle can apply to alcohol and drug abuse.

Nevertheless, 97.3% of treatment programs exclusively use a 12-Step approach, and the political structure of these programs generally considers any intervention that does not endorse this method as heretical. Two factions dominate the industry: the 12-Step orientation and the new medical model that believes a pharma-co-therapeutic solution will end addiction within a few years. National conventions for addiction professionals do not even teach strategies such as methadone treatment, behavioral self-control training and hypnosis.

In the past, I was a frequent speaker at these national conventions on the subject of alcoholism and drug addiction, but they will likely not invite me back to speak on the subject of addiction again, because I no longer tote the party line; I do not believe in a one-size-fits-all solution. I love 12-Step groups, I think the 12-Steps are profound, and I actually believe the principles of recovery they offer are quite sound and easy to implement. However, I also believe that some people respond better to different approaches. Hypnotherapy is one approach that can meet the needs of many individuals - and one can certainly combine it with 12-Step treatment approaches to produce real serenity.

Evidence that it works:

Am J Clin Hypn. 2004 Apr;46(4):281-97

In a research study on self-hypnosis for relapse prevention training with chronic drug/alcohol users, individuals who played self-hypnosis audiotapes "at least 3 to 5 times a week," at 7-week follow-up, reported the highest levels of self-esteem and serenity, and the least anger/impulsivity, in comparison to the minimal-practice and control groups

Cautions:

Medical detoxification may be required to avoid severe withdrawal, even death. In addition, many addicts have numerous medical issues, and a physician should physically evaluate anyone receiving treatment for addiction.

Weight Loss

Hypnosis can help with weight loss and can produce profound results. Most of us have tried losing weight through traditional methods of diet and exercise, and these kinds of programs are scientifically effective. In the long run though, only one strategy will help a person to succeed at both losing weight and keeping it off, and that is to expend more calories than you take in each day. Fad diets, like eating nothing but grapefruit or cutting out carbs can help a person to reach a quick weight-loss goal, but without a plan for maintaining your motivation and weight once you achieve your initial goals, you will quickly find yourself right back where you started.

The real reason people fail at weight loss is not the ineffectiveness of the diets, but rather the failure that people on diets have with changing lifelong patters of thinking. There is no such thing as a "Hypnosis Diet." Hypnosis is an effective tool for weight-loss because it can do what diets cannot: It can help you reprogram the destructive associations you have with food and reframe your vantage point on exercise and physical fitness.

Conscious approaches to learning, like affirmations or cognitive therapy, can play a role in helping a person to achieve weight loss, and learning specific strategies for diet, nutrition and exercise are all valuable tools. But research now shows that the subconscious mind controls much more of our behaviors than previously thought.

The subconscious mind controls muscle movement, heart rate, breathing and the autonomic nervous system. It is powerful, and quickly commits to habit what it learns. We were not born knowing how to walk, but over time, we learned. Watch any toddler and you can see them trying to remember how to take the next step. By the time a child can run, they no longer think about how to take the next step - they just do it.

We have no doubt committed to memory unhealthy patterns of eating, such as unconsciously associating bread with butter and salad with dressing. If you are on a diet, you make conscious choices based on the recommendation of the diet. But when you are off the diet, the unconscious kicks in and tells you to put frosting on your brownie, without ever thinking about the impact on your diet.

Many of us grew up hearing our parents tell us to eat everything on our plate and not to be wasteful. Now, when we go into a restaurant that wants to convince us we have received a good value for our money by serving us portions that are three to four times larger than the recommended caloric intake, we subconsciously choose to eat everything they serve us, even if it is self-defeating and unhealthy.

There is much more to weight loss than simply dieting or exercise. The subconscious relationships we have with food are just as powerful as our conscious behaviors. In essence, you could say that it is possible to think yourself thin - because if we change these subconscious associations, we are able to choose a healthy manner of living automatically.

People are often surprised to learn that they can alleviate lifelong phobias and fears with neuro-linguistic programming, a modality of psychology that quickly transforms our subconscious perceptions of events through creative visualization. People are equally surprised to learn that clinical hypnosis is a highly effective way of achieving life goals. Both methods,

which often overlap in some ways, can help you to make significant life changes if you choose to learn the principles of change.

The following unconscious misbeliefs often keep people from trying to lose weight from achieving their goals:

1.) If it looks weird, green, or like a vegetable, it must taste bad.
2.) If it is good for me, it must taste terrible.
3.) If it is bad for me, it must taste good.
4.) Food brings me emotional satisfaction.

These kinds of misbeliefs are often ingrained into our subconscious. Sam-I-Am's friend did finally try the green eggs and ham, but it took a lot of coaxing. Hypnotic suggestion could have saved Sam a lot of distress and changed his little friend's subconscious associations in only one page! (But, then the story would have been too short…)

Evidence that it works:

In a meta-analysis of two outcome studies, patients using hypnosis lost almost twice the weight of patients who did not receive hypnosis during treatment for weight loss. Kirsch, Irving (1996). Hypnotic enhancement of cognitive-behavioral weight loss treatments--Another meta-reanalysis. Journal of Consulting and Clinical Psychology.

A study on the treatment of obesity (Bolocofsky and Coulthard-Morris. 1985) showed a superiority of the combination of behavioral therapy and hypnosis even with a catamnesis of 24 month.

Cautions:

Healthy weight loss is gradual. Clients who come to hypnosis - or begin any weightloss program - often have "magical thoughts" about results. Realistic expectations and goals should be discussed, and healthy levels of weight loss should be the target.

Habit Control (nail biting, tics, coughs, etc.)

Cigarette smoking is more than a habit; it is nicotine addiction. Habits are things like nail biting, hair chewing, knuckle cracking, throat noises, coughing, tics, and so forth. Habits differ from addiction because there is not a physiological dependency on the behavior. Withdrawal from habits is only emotional, rather than physical (although some habits may require physical healing). Habits are usually products of subconscious associations and learning patterns, although personality characteristic may come into play, including perhaps obsessive traits and poor anxiety management skills.

Evidence that it works:

Hypnosis in pediatrics: applications at a pediatric pulmonary center
by Ran D Anbar
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BMC Pediatrics 2002, **2:**11 doi:10.1186/1471-2431-2-11

Hypnotherapy was associated with improvement in 80% of patients with persistent asthma, chest pain/pressure, habit cough, hyperventilation, shortness of breath, sighing, and vocal cord dysfunction. When improvement was reported, in some cases symptoms resolved immediately after hypnotherapy was first employed. For the others, improvement was achieved after hypnosis was used for a few weeks. No patients' symptoms worsened and no new symptoms emerged following hypnotherapy.

Cautions:

Hypnotherapy for changing or eliminating habits often requires multiple sessions and the targeted development of alternative behaviors.

Emotional and Psychiatric Disorders

Anxiety and Panic

If you look up the DSM-IV criteria for panic attack, you will find the following symptoms listed:

- A discrete period of intense fear or discomfort, in which four (or more) of the following symptoms developed abruptly and reached a peak within 10 minutes:
- palpitations, pounding heart, or accelerated heart rate
- sweating
- trembling or shaking
- sensations of shortness of breath or smothering
- feeling of choking
- chest pain or discomfort
- nausea or abdominal distress

- feeling dizzy, unsteady, lightheaded, or faint
- derealization (feelings of unreality) or depersonalization (being detached from oneself)
- fear of losing control or going crazy
- fear of dying
- paresthesias (numbing or tingling sensations)
- chills or hot flushes

Panic and anxiety are highly physical in their manifestations, and you may notice that the first eight of these criteria are all physical manifestations of emotional response.

One of the reasons anxiety is so distressing is that the intense physical responses combine with the intense emotions to further increase the anxiety. A full-blown panic attack can paralyze a person in crisis, or even lead to behaviors of amok, perhaps with legal consequences (like on a plane).

Hypnosis, with an emphasis on teaching physical skills for managing anxiety, can help clients in a number of ways:

1.) It can reprogram the subconscious to automatically use healthy coping strategies.
2.) Teach a client how to calm themselves down physically.
3.) Help a person dissociate from unrealistic fears.

Evidence that it works:

Three-year follow-up and clinical implications of a mindfulness meditation-based stress reduction intervention in the treatment of anxiety disorders.

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Abstract
A previous study of 22 medical patients with DSM-III-R-defined anxiety disorders showed clinically and statistically significant improvements in subjective and objective symptoms of anxiety and panic following an 8-week outpatient physician-referred group stress reduction intervention based on mindfulness meditation. Twenty subjects demonstrated significant reductions in Hamilton and Beck Anxiety and Depression scores post-intervention and at 3-month follow-up.

Cautions:

Anxiety is a natural emotion, and just like depression and anger, it can produce both positive and negative results. Anger can serve as a warning instrument for self-protection. Clients should carefully evaluate their anxiety and discover what it is telling them, in this context.

Depression

Micheal Yapko, in his landmark book, *Treating Depression with Hypnosis*, comes down hard on the psychiatric and pharmaceutical industries that have sold the lie to Americans that depression is as simple as chemical imbalances that can be "rebalanced" with anti-depressants.

Yapko does concede that biology can play a role in depression, but points to the fact that most depressions are holistic in nature. His tome makes a point that I have often made: we are much more than the sum total of our dopamine and serotonin receptors.

Those who view medication as a first line treatment for depression are simplistic in their understanding of the human condition. In the past, hypnosis was viewed as contraindicated for

patients who were depressed. However, new research suggests that not only can hypnosis be utilized with the depressed person, it can also be highly effective in treating the depression.

Hypnosis draws on client strengths and existing coping strategies. It amplifies positive attributes that individuals already possess, to help them solve problems - and these are the characteristics of successful treatment protocols for depression.

It is true that biology can play a role in why some people are depressed, and anti-depressants can help some people function better - but they are no more effective than regular exercise and traditional cognitive-behavioral approaches to treating depression, and in fact are often *less* effective.

Medications rake in about ten billion dollars a year for the pharmaceutical companies and are over-prescribed by doctors and underutilized by patients. Hypnotherapy can easily be integrated into a rational CBT treatment modality and effectively used as a conservative approach, beneficial to clients, without side effects.

Evidence it works:

Limited research has been done in this area, but CBT has been demonstrated to be highly effective with depressed clients, and hypnosis is easily integrated with CBT.

J Consult Clin Psychol. 1995 Apr;63(2):214-20. Related Articles, Links
Hypnosis as an adjunct to cognitive-behavioral psychotherapy: a meta-analysis.

Kirsch I, Montgomery G, Sapirstein G.

Department of Psychology, University of Connecticut, Storrs 06269-1020, USA.

A meta-analysis was performed on 18 studies in which a cognitive-behavioral therapy was compared with the same therapy supplemented by hypnosis. The results indicated that the addition of hypnosis substantially enhanced treatment outcome, so that the average client receiving cognitive-behavioral hypnotherapy showed greater improvement than at least 70% of clients receiving nonhypnotic treatment.

Cautions:

Suicide assessments should always be completed with depressed clients. The ASIQ, available from P.A.R., is a great screening tool for counselors and hypnotherapists. Suicidal clients should be referred immediately without hesitation to crisis treatment centers.

Phobias (ie. fear of flying) and Post-Traumatic Stress Disorder (PTSD)

Fear of flying can stem from prior events, such as an emergency landing or becoming ill on a plane as a child, or from a generalized anxiety or specific fear such as mechanical failure or terrorism. Fear of flying can also originate as another anxiety disorder, such as a generalized anxiety that becomes more severe in the controlled situation of an aircraft, or as a manifestation of claustrophobia. I have selected fear of flying to discuss in this section, because I believe it occurs more frequently now than in the past, and it can debilities individuals,

leading to erratic behavior on flights with legal consequences, or derail certain career paths, and it can keep people from enjoying all that life has to offer including friends and family.

Specific phobias also include reptiles, insects, mice, clowns, or other specific fears that manifest in an otherwise healthy person's life. The fear can actually be of just about anything, and phobias to one degree or another are common in the general population (DSM-IV-TR).

Regression therapy is often used by hypnotherapists when clients can isolate a specific actuating event that triggered the fear. In this case, regression can help desensitize a person, much like the Neuro-lingustic strategy of visualization that allows a person to begin seeing themselves observing life difficulties in an effort to reduce the importance of the actuating event. There are probably more effective strategies than regression when a client is unable to remember the actuating event prior to hypnotic trance.

Combining the principles of NLP with hypnosis can be quit effective for specific phobias and PTSD.

Evidence that it works:

Travel Phobias
By Iain B. McIntosh1
Journal of Travel Medicine, Volume 2 Page 99 - June 1995
doi:10.1111/j.1708-8305.1995.tb00635.x

"Hypnotherapy can be very successful, especially in people who present within a few days of impending flight. Hypnotherapy incorporates trance induction, ego-strengthening, relaxation, desensitization, and autohypnosis in a therapeutic program."

New uses of hypnosis in the treatment of posttraumatic stress disorder.
by Spiegel D, Cardena E.
Department of Psychiatry and Behavioral Sciences, Stanford University School of Medicine.
J Clin Psychiatry. 1990 Oct;51 Suppl:39-43; discussion 44-6.

Hypnosis is associated with the treatment of posttraumatic stress disorder (PTSD) for two reasons: (1) the similarity between hypnotic phenomena and the symptoms of PTSD, and (2) the utility of hypnosis as a tool in treatment. Physical trauma produces a sudden discontinuity in cognitive and emotional experience that often persists after the trauma is over. This results in symptoms such as psychogenic amnesia, intrusive reliving of the event as if it were recurring, numbing of responsiveness, and hypersensitivity to stimuli.

Two studies have shown that Vietnam veterans with PTSD have higher than normal hypnotizability scores on standardized tests. Likewise, a history of physical abuse in childhood has been shown to be strongly associated with dissociative symptoms later in life. Furthermore, dissociative symptoms during and soon after traumatic experience predict later PTSD. Formal hypnotic procedures are especially helpful because this population is highly hypnotizable. Hypnosis provides controlled access to memories that may otherwise be kept out of consciousness. New uses of hypnosis in the psychotherapy

of PTSD victims involve coupling access to the dissociated traumatic memories with positive restructuring of those memories. Hypnosis can be used to help patients face and bear a traumatic experience by embedding it in a new context, acknowledging helplessness during the event, and yet linking that experience with remoralizing memories such as efforts at self-protection, shared affection with friends who were killed, or the ability to control the environment at other times. In this way, hypnosis can be used to provide controlled access to memories that are then placed into a broader perspective. Patients can be taught self-hypnosis techniques that allow them to work through traumatic memories and thereby reduce spontaneous unbidden intrusive recollections

The Additive Benefit of Hypnosis and Cognitive-Behavioral Therapy in Treating Acute Stress Disorder.
By Bryant, Richard A.; Moulds, Michelle L.; Guthrie, Rachel M.; Nixon, Reginald D. V.
Journal of Consulting and Clinical Psychology. 73(2), Apr 2005, 334-340

This research represents the first controlled treatment study of hypnosis and cognitive–behavioral therapy (CBT) of acute stress disorder (ASD). Civilian trauma survivors (N 87) who met criteria for ASD were randomly allocated to 6 sessions of CBT, CBT combined with hypnosis (CBT–hypnosis), or supportive counseling (SC). CBT comprised exposure, cognitive restructuring, and anxiety management.

CBT–hypnosis comprised the CBT components with each imaginal exposure preceded by a hypnotic induction and suggestions to engage fully in the exposure. In terms of treatment completers (n 69), fewer participants in the CBT and CBT–hypnosis groups met criteria for posttraumatic stress disorder at posttreatment and 6-month follow-up than those in the SC group. CBT–hypnosis resulted in greater reduction in reexperiencing symptoms at posttreatment than CBT. These findings suggest that hypnosis may have use in facilitating the treatment effects of CBT for posttraumatic stress.

Cautions:

Abreaction is a risk in regression to traumatic events. Be prepared.

Clients who define themselves by their traumas or a victim role need to redefine a new identity in order to actually make healthy, positive changes.

Complex PTSD can be a catastrophic psychiatric illness, its complexity going beyond hypnotherapy as a sole intervention in many cases, and necessitating referral to a qualified mental health professional (who practices solution-focused therapy) for long-term treatment.

Stress/Pain Management

Relaxation

I have been practicing progressive muscle relaxation with clients for almost 20 years, because it is one of the most effective ways to relax and to take physical control over your body. It is a wonderful way to put yourself to sleep at night, or at any time simply to reduce tension. PMR is commonly used as an induction into hypnosis - a state of profound relaxation - by most hypnotherapists.

As a side note, not drinking alcohol, avoiding caffeine and other drugs - including nicotine - can help you to feel rested at night and to feel stress free. Although in the short run, these substances seem to produce relaxation, they physically affect the body in the opposite manner. Finding a natural way to depart from the stressors of the day is essential, and hypnosis can be a positive way to do this.

Evidence that it works:

Hypnosis is in and of itself a relaxation experience, and it teaches strategies for relaxation that carry easily into many aspects of daily living.

> ***Psychosocial and immune effects of self-hypnosis training for stress management throughout the first semester of medical school***
>
> WG Whitehouse, DF Dinges, EC Orne, SE Keller, BL Bates, NK Bauer, P Morahan, BA Haupt, MM Carlin, PB Bloom, L Zaugg and MT Orne
> Institute of Pennsylvania Hospital, University of Pennsylvania Medical School, Philadelphia, USA.
>
> This study was a 19-week prospective conducted to determine the effectiveness of a self-hypnosis/relaxation intervention to relieve symptoms of psychological distress and moderate immune system reactivity to examination stress in 35 first-year medical students. Twenty-one subjects were randomly selected for training in the use of self-hypnosis as a coping skill and were encouraged to practice regularly and to maintain daily diary records related to mood, sleep, physical symptoms, and frequency of relaxation practice. An additional 14 subjects received no explicit training in stress-reduction strategies, but completed similar daily diaries. Self-report psychosocial and symptom measures, as well as blood draws, were obtained at four time points: orientation, late semester, examination period, and postsemester recovery.
>
> It was found that significant increases in stress and fatigue occurred during the examination period, paralleled by increases in counts of B lymphocytes and activated T lymphocytes, PHA-induced and PWM-induced blastogenesis, and natural killer cell (NK) cytotoxicity. No immune decreases were observed. Subjects in the self-hypnosis condition reported significantly less distress and anxiety than their nonintervention counterparts, but the two groups did not differ with respect to immune function. Nevertheless, within the self-hypnosis group, the quality of the exercises (ie, relaxation ratings) predicted both the number of NK cells and NK activity.

It was concluded that stress associated with academic demands affects immune function, but immune suppression is not inevitable. Practice of self-hypnosis reduces distress, without differential immune effects. However, individual responses to the self-hypnosis intervention appear to predict immune outcomes.

Cautions:

Hypnosis is safe, natural and easy. There are few cautions, other than perhaps to let clients engaged in PMR know that if they feel an isolated deep pain in any part of the body while doing the 'tension and relaxation' exercises, strength training at the gym might help, and consulting with a physician regarding usual pain can rule out bone and muscle injuries that have gone undiagnosed.

Hypnosis and Clinical Hypnotherapy in the Treatment of Psychological and Psychosomatic Ailments

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Review Article

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Abstract

This review on hypnosis and clinical hypnotherapy in the treatment of psychological and psychosomatic ailments comes to shed the light on a topic since its inception is neglected in our Arab world, though historically and scientifically it had been evolved as it is the case of other clinical procedures. Tracing of the most significant efforts in the history of hypnotism, the nature of the phenomenon, its scope and process was of great importance to be addressed. Also, earlier, late and current evaluations of the subject from a scientific outlook were highlighted, where it was shown that hypnosis and hypnotherapy is a valid and reliable clinical tool in the treatment of many health problems. From here, it leads us to conclude that hypnosis and hypnotherapy is a significant clinical tool in medical practice; in past and at present. With hope that one day hypnosis and hypnotherapy will become a therapeutic choice for all who may need it; individuals, families and groups; for a healthy living, happiness and much better promising quality of life.

Key words: hypnosis, clinical hypnotherapy, treatment, psychological and psychosomatic ailments.

الخلاصة

تأتي هذه المراجعة حول التنويم المغناطيسي والعلاج بالتنويم المغناطيسي الاكلينيكي في معالجة الامراض النفسية والنفس جسدية لإلقاء الضوء على موضوع منذ بداياته نراه متجاهل في عالمنا العربي، على الرغم من انطلاقته العلمية والتاريخية الطويلة كغيره من الطرق الإكلينيكية الأخرى. استعراض أهم الجهود ذات الدلالة في تاريخ التنويم المغناطيسي، وطبيعة تلك الظاهرة، مجالاتها وعملياتها، كان من أهم ما تطرق اليه. أيضا تم تسليط الضوء على تلك التقويمات العلمية للموضوع، المتقدمة منها والمتأخرة، والمعاصرة، ومن وجهة نظر علمية، والتي من خلالها تم الكشف عن حقيقة أن التنويم المغناطيسي والعلاج بالتنويم المغناطيسي كأداة اكلينيكية ذا مصداقية ووثوقيه معتبرة في معالجة العديد من المشكلات الصّحية. وفي نهاية الامر، خلصنا الى ان التنويم المغناطيسي والعلاج بالتنويم المغناطيسي أداة اكلينيكية ذات دلالة في المراس الطبية؛ في الماضي والحاضر. مع الامل في ان يكون التنويم المغناطيسي والعلاج بالتنويم المغناطيسي يوما ما خيارا علاجيا لكل من قد يحتاجون اليه؛ افرادا، واسرا وجماعات؛ وكله من اجل حياة صحية، وسعادة ونوعية حياة واعدة.

Introduction

Hypnosis as a technique for health issues that seems to have been originated with Budhism and other eastern meditative techniques of ancient India cultures, [1] as it is the story of sleep temples in ancient Egypt,[2] Greece and the Romans.[3] Literally speaking, hypnotism is a state where to place an individual in a sleep-like status, though as we will see it is a different state of sleep.

In the golden times of Islam and Muslims, Avicenna (Abu Ali Ibn Sina) (980-1037), the well known

philosopher and physician of his time, was the earliest medieval age to make a distinction between natural sleep and the state of hypnosis, where in "The Book of Healing" (Kitab-ul-Shifaa) (published in 1027), he referred to the phenomenon as "Al-Wahm Al-Amil" (the working illusion). From Avicenna's perspective, one could create conditions in another person to make him or her accept the reality of hypnosis [4].

Hypnosis as a modern science is documented in Paris as far back as the 18th century, where it was used as a medical tool in dealing with pain resolving troubles, and the Austrian physician Franz Anton Mesmer (1734-1815) was the first to become a famous in using a form of hypnosis referred to as "animal magnetism" and later as "Mesmerism"[5].

True that within the same period of time, we find the Swiss priest Johann Joseph Gassner (1729-1779), to challenge "Mesmerism" and to judg Mesmer's method as curing through "exorcism" and Mesmer to confront Glassner's religious beliefs in a controversial scientific session before the Munich Academy of Sciences [6]. Later, we find Pope Pius VI (1717-1799) of Rome, to criticize Gassner's views, where the later position becomes difficult; in the end to be forbidden to conduct "exorcisms" or see hypnotism as "exorcism"[7].

Later as an interested fellow in the phenomenon, the Scottish surgeon James Braid (1795-1860) was the first to publish a major book on hypnotism, named Neurypnology (1843), where he discussed hypnotism's historical precursors in a series of specialized varied articles [8]. Further, Braid was the first to draw analogies between his own practices of hypnotism and other various forms of Hindu yoga meditation and related ancient spiritual practices[9]. Braid was the one who coined the term "hypnotism" as an abbreviation for "neuro-hypnotism," meaning "sleep of the nerves,"[10] where from here the man was taken by many as the first genuine "hypnotherapist" and the "father of hypnotism."[11]

Contemporary to Braid, John Elliotson (1791-1868), the President of the Royal Medical and Surgical Society of London performed successfully about 1834 surgeries while patients were under hypnosis. Also, within the same period of time, it was reported that James Esdaille (1808-1859) of Scotland, who is a famous British surgeon serving in India, as conducted about 2,000 operations (including amputations) with patients under hypno-anaesthesia, without feeling any pain [12].

Post Braid contributions and other British reputed figures, interest in hypnotism was temporarily waned for a while, gradually to be shifted from Great Britain to neighboring countries, to France, Germany, Austria, and Switzerland, where clinical and medical research on the topic began to grow, reaching its peaks in 1880s on the hands of Auguste Ambrose Liebeault, Hippolyte Bernheim, Emile Coue de la Châtaigneraie, and Jean-Martin Charcot[13].

At the Nancy school, which is considered the most famous earlier French hypnosis-centered school of psychotherapy, Auguste Ambrose Liebeault (1823-1904) and Hyppolyte Bernheim (1840-1919) where the first to come out with the notion that hypnosis is a normal phenomenon, and as due their works to be documented in various languages[14].

Based on such scientific efforts, hypnosis become an accepted therapeutic medical tool in medical science, where over the course of four years, Bernheim alone utilized hypnotic inductions in the case of

about 5,000 patients, with a 75% observed success rate[15].

Émile Coué (1857-1926) who studied on the hands of masters Liébeault and Bernheim, entered the arena as a psychologist and introduced a popular method of psychotherapy well based on autosuggestion[16]. His appreciated contributions made him to be seen by many medical critics as a second Nancy School per se [17].

In 1886, Jean-Martin Charcot (1835-1893), who becomes later the father of modern neurology in France, was the first to present his findings on the topic of hypnosis before the French Academy of Sciences; and albeit his critical conclusions that hypnosis is a manifestation of hysteria not a cure, hypnosis continued to be accepted and practiced as a therapeutic medical tool [18].

From the 1880s the examination of hypnosis passed from surgical doctors to mental health professionals, and Charcot's works led the way for others, where his clinical observations and enlightening studies continued by his pupils, particularly, Pierre Janet (1859-1947). Janet becomes the first to describe the theory of dissociation; the splitting of mental aspects under hypnosis (or hysteria as it were seen) as well he is the one who provoked interest in the subconscious mind and laid the framework for reintegration therapy in the case of dissociation and dissociated personalities [19].

Another enlightened pupil from Vienna, Sigmund Freud (1856-1939), who learned at the hands of Charcot himself, side by side with Janet, with some other medical reputed figures, like close friend Josef Breuer (1842-1925), we find them through hypnotism, strongly contributed enough to psychology, psychotherapy, the science of mind and other related mental processes [20].

In succession of Freud and Breuer, Clark Hull (1884-1952) who was an eminent experimental psychologist at Yale University, was the first American to present his data and observations on hypnosis, with a full description of the phenomenon in one of his revolutionary books, titled "Hypnosis and Suggestibility"[21].

Hull attempts dispelled all those misconceptions about hypnosis, and he did enough in comparing subjects' capacities while in hypnotic states with those in the awake states, and much more [22].

Succeeding Hull, Milton Hyland Erickson (1901-1980) as a reputed American mental health professional (both psychologist and psychiatrist) becomes a figure in what is denoted later as medical and clinical hypnosis [23-26].

It is enough to know that Erickson was the founding president of the American Society for Clinical Hypnosis in addition of being an active fellow of the American Psychiatric Association, the American Psychological Association, and the American Psychopathological Association. He is well noted for his distinguished approach to the unconscious mind as creative and solution-generating. Further, as a clinical hypnotist, the man noted in influencing plenty of psychological therapeutic techniques and related approaches, such as brief therapy, strategic family therapy, family systems therapy, solution focused brief therapy, in addition to his excellent contributions to the establishment and emergence of Neuro-Linguistic Programming -NLP [27].

Not to proceed further beyond, hypnosis and hypnotherapy becomes a well established medical procedure all over, where it is thoroughly investigated, taught, learned and clinically practiced at psychology,

psychiatry, and neurology schools, faculties, departments and units of health care settings and medicine.

Hypnotherapy: Nature, Scope and Process

Primarily speaking, hypnotherapy is the use of hypnosis in psychotherapy by licensed psychotherapists [28]. Also, it is the use of the same by sergeants, obstetricians, pediatricians, general medicine practitioners, psychiatrists, and dentists [29 30].

Referring to the British Psychological Society, "the term 'hypnosis' denotes an interaction between one person, the 'hypnotist', and another person or people, the 'subject' or 'subjects.' In this interaction, the hypnotist attempts to influence the subjects' perceptions, feelings, thinking and behavior by asking them to concentrate on ideas and images that may evoke the intended effects. The verbal communications that the hypnotist uses to achieve these effects are termed 'suggestions.' Suggestions differ from everyday kinds of instructions in that they imply that a 'successful' response is experienced by the subject as having a quality of involuntariness or effortlessness"[31].

When it comes to hypnotherapy and related forms, we find that modern hypnotherapy in clinical practice has been used in a variety of therapeutic forms, such as the traditional hypnotherapy [32], self-hypnosis [33], cognitive-behavioral hypnotherapy or clinical hypnosis combined with some elements of cognitive-behavioral therapy [34], analytical hypnotherapy or what is know as "hypnoanalysis" [35], hypnosurgery form [36 37], Dave Elman hypno method,[38] and the Ericksonian hypnotherapy, including the well established NLP formulae [39 40].

When it comes to scope, hypnosis and hypnotherapy originally founded to treat and cure psychological, psychosomatic, psycho-neurological and psychosocial related symptoms and syndromes. Within that scope, building good habits (i.e., taking prompt action, exercising regularly, improving study habits), removing bad habits (i.e., stop smoking, nail biting, emotional overeating, setting goals and following a plan of action, eliminating negative self-talk and replacing it with helpful self communication, eliminating unnecessary fears and phobias (i.e., flying, public speaking, test taking, improving performance (i.e., sports, music, acting, speaking, sales, shifting perspective and attitude to help a client move ahead (i.e., from what's wrong to what's right, from problem to solution, from worry to action, from what can't be done to what can, teaching self-help tools and techniques (i.e., self-hypnosis, teaching corporate seminars (i.e., stress management, creating excellence, teaching personal growth seminars, healing emotional pain, enhancing physical healing, creative problem solving, locating and removing limiting decisions or negative beliefs, locating and releasing blocks that prevent any of the above), pain management, childbirth preparation, working with suicidal depression, and working with most psychologically labeled disorders[41].

As a form of psychotherapy and clinical therapeutic technique, hypnosis is used to create a kind of unconscious changes in the patient through creating and directing new thoughts, feelings and responses. As cited by Richard J D'Souza; who is a senior hypnotherapist at Clinical Hypnotherapy Cardiff: "hypnosis is sometimes referred to as an "altered state of consciousness" in which an individual(s) can experience an

increased receptiveness and responsiveness to their inner experiential perceptions. It is a state of heightened suggestibility in which a therapist can use suggestions to influence the patients' unconscious mind while their conscious mind remains relaxed [42]. And of course, all is undertaken within a subject inner world while he or she is under hypnosis.

When it comes to induction, the process should be undertaken by a well trained and licensed hypnotist to establish the needed state of unconsciousness or all those necessary conditions required for hypnosis to take place [43]. The formula in core, usually is a specific peculiar "sleep command or any related demand," though induction methods could vary. Also, when it comes to scripts, we find them varied from A to Z [44]. Even to reach such a hypno state , self-hypnosis is possible; a state in which a subject listens to either an audio or a video taped induction formula or plays the role of a hypnotist and a receptive subject successively [45 46].

Hypnotic induction is a necessary step to lead the subject to enter a state of increased suggestibility, a state during which his or her critical faculties are reduced and become more susceptible to accept the commands of the hypnotist [47, 48]. The state hereby is known as a "trance"[49]. When it comes to the necessary components to induce such a trance state, simply, we find it residing in motivation, concentration, imagination and suggestion as the figure 1 shows.

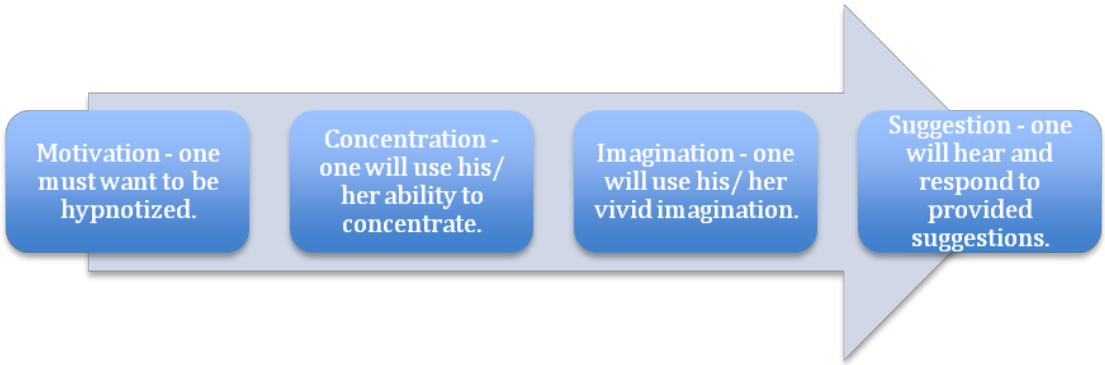


Figure 1 shows the necessary needed components for inducing a hypnotic trance within subjects.

In early hypnotic literatures, hypnosis induction was a gradual drawn-out process, where, methods such as progressive muscle relaxation were designed to relax the hypnotic subjects into a state of inner focus and the hypnotist would be better able to influence them, and as a consequence, create effects and changes at their subconscious level [50]. Modern alternatives include Dave Elman's induction techniques, where he uses many different handshake methods to hypnotize subjects in a short period of time (i.e., 1-4 minutes) [51, 52]. Of course, not to forget the Ericksonian styles and patterns [53].

As it is shown in figure 2 below, hypnosis takes place usually in four main stages, starting with initiation stage (i.e., the readiness, relaxation and sleep command stage), trance (i.e., the dizziness, heaviness, irresistibility and receptive stage), suggestibility stage (i.e., full suggestion upon subjects), and ending up with intended therapeutic consequences stage (i.e., the prospected therapeutic changes and solutions).

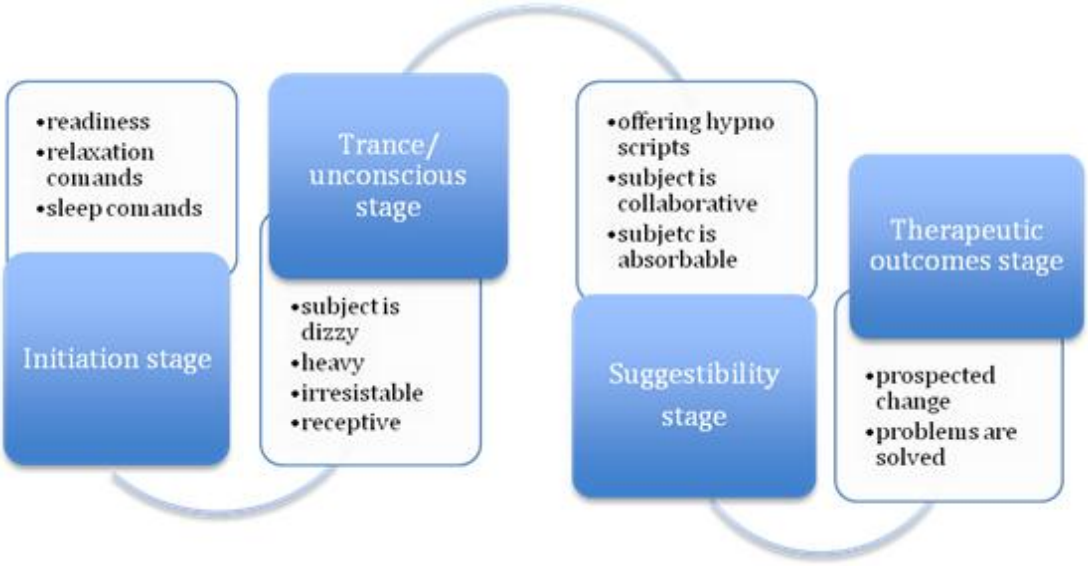


Figure 2 shows the process of inducing hypnosis in its major four stages.

Earlier Evaluations and Related Findings

In 1892, the British Medical Association (BMA) commissioned a team of doctors to undertake and evaluate the nature as well as the medical effects of hypnosis and hypnotherapy. Completing such historical medical investigation of hypnotism and related outcomes, the committee reported a full satisfaction of the genuineness of the hypnotic state. Furthermore, the committee members were of the opinion that as a therapeutic agent, hypnotism is frequently effective in relieving pain, procuring sleep, and alleviating many functional ailments [54]

From here then after, it could be said that medical practice of hypnosis and hypnotherapy had subsequently been approved by the BMA, where such a recognition was first established in 1892, to be strongly reinforced later in 1955 [55].

In 1955 efforts, the Psychological Medicine Group of the BMA commissioned a subcommittee, led by Prof. T. Ferguson Rodger, to deliver a second, and more comprehensive report on hypnosis. The subcommittee consulted several experts on the subject from various fields, including the eminent neurologist Prof. W. Russell Brain, the 1st Baron Brain, and the psychoanalyst Wilfred Bion. After two years of thorough investigation, its final report was published, with full emphasis to consider uses of hypnotism, its relation to medical practice in the present day, the advisability of giving encouragement to research into its nature and application, and the lines upon which such research might be organized [56].

On the basis of its study and investigative research findings, the subcommittee was satisfied that hypnotism is of value and may be the treatment of choice in some cases of so called psychosomatic disorders and psychoneurosis. Also, it may be of value for revealing unrecognized motives and conflicts in such conditions. And as a treatment, in the opinion of the investigating subcommittee it has proved its ability to remove symptoms and to alter morbid habits of thought and behavior, (…).

In addition to the treatment of psychiatric disabilities, it was found

that there is a place for hypnotism in the production of anesthesia or analgesia for surgical and dental operations, and in suitable subjects it is an effective method of relieving pain in childbirth without altering the normal course of labor. As Alexander Kennedy put it, it was concluded: "that the use of hypnosis has a recognized place in the medical armamentarium and is a useful technique in the treatment of certain illnesses when employed by qualified medical and dental personnel."[57]

All such reputed findings, crossed the Atlantic Ocean to be followed by the American Medical Association (AMA), where AMA fully acknowledged hypnosis as a valuable tool in medical treatments, particularly, in 1958, where the AMA commissioned a similar step on hypnosis and hypnotherapy in the US, which fully led to the endorsement of the 1955 BMA report.[58]

Later on, particularly in 1995, the US National Institutes of Health (NIH), initiated a Technology Assessment Conference that compiled an official statement entitled: "Integration of behavioral & relaxation approaches into the treatment of chronic pain and insomnia," which is truly an extensive report that includes a statement on the existing research in relation to hypnosis and hypnotherapy for chronic pain.

It was concluded that: "the evidence supporting the effectiveness of hypnosis in alleviating chronic pain associated with cancer seems strong." In addition, the panel was presented with other data suggesting the effectiveness of hypnosis in other chronic pain conditions, which include irritable bowel syndrome, oral mucositis, temporomandibular disorders and tension headaches[59].

Further distinguished works, the British Medical Journal (BMJ) in 1999, published a clinical review on the current medical research findings of hypnosis, hypnotherapy and relaxation therapies. Concisely, to summarize some of their findings, it was concluded that they are:

1. effective for panic disorders and insomnia, particularly when integrated into a package of cognitive therapy (including, for example, sleep hygiene),
2. effective enough, mainly with cognitive behavioral therapy for conditions such as phobia, obesity, and anxiety,
3. enough supporting the use of many relaxation techniques for treating both acute and chronic pain,
4. of great value in asthma and in irritable bowel syndrome, and
5. enough effective for the treatment of cancer related anxiety, pain, nausea, and vomiting, particularly in children [60].

Further Research Works and Further Findings

In 2001, the Professional Affairs Board of the British Psychological Society (BPS) commissioned a working group of expert psychologists to publish a report entitled "The Nature of Hypnosis," with the purpose to provide a considerable statement about hypnosis and important issues concerning its application and practice in a range of contexts, notably for clinical purposes, forensic investigations, academic research works, entertainment and training. At the end, the report came out and provided a brief summary of the nature of hypnosis and current scientific research findings on the subject. The most significant issue was that: "hypnosis is a valid subject for scientific study and research and a proven therapeutic medium," and regarding hypnosis therapeutic uses and related research outcomes, the report came out with the a fact that

"enough studies have now accumulated to suggest that the inclusion of hypnotic procedures may be beneficial in the management and treatment of a wide range of conditions and problems encountered in the practice of medicine, psychiatry and psychotherapy."[61]

Then, the working group (within the same report) provided an overview of some of the most significant contemporary research findings on the efficacy of clinical hypnotherapy, and they were as it follows:

1. there is convincing evidence that hypnotic procedures are effective in the management and relief of both acute and chronic pain and in assisting in the alleviation of pain, discomfort and distress due to medical and dental procedures and childbirth,
2. hypnosis and the practice of self-hypnosis may significantly reduce general anxiety, tension and stress in a manner similar to other relaxation and self-regulation procedures,
3. likewise, hypnotic treatment may assist in insomnia in the same way as other relaxation methods,
4. there is encouraging evidence demonstrating the beneficial effects of hypnotherapeutic procedures in alleviating the symptoms of a range of complaints that fall under the heading psychosomatic illness, including: tension headaches and migraine; asthma; gastrointestinal complaints such as irritable bowel syndrome; warts; and possibly other skin complaints such as eczema, psoriasis and urticaria, and
5. there is evidence from several studies that inclusion of clinical hypnosis in a weight reduction program(s) may significantly enhance positive outcome(s).

In 2002 a study was set to investigate the effectiveness of Erickson's hypnosis style(s) and Jacobson relaxation technique(s) for the reduction of osteoarthritis pain, where participants reporting pain from hip or knee osteoarthritis were randomly assigned to one of the following conditions: (a) hypnosis (i.e. standardized eight-session hypnosis treatment); (b) relaxation (i.e. standardized eight sessions of Jacobson's relaxation treatment); (c) control (i.e. waiting list). Overall, results show that the two experimental groups had a lower level of subjective pain comparatively to the control group and that the level of subjective pain decreased within time. Furthermore, an interaction effect between group treatment and time measurement was also observed in which beneficial effects of treatment appeared more rapidly for the hypnosis group. Results also show that hypnosis and relaxation are effective in reducing the amount of analgesic medication taken by participants [62].

In 2003, a meta-analysis study on the efficacy of clinical hypnotherapy was published by Flammer and Bongartz (two eminent German researchers from the university of Konstanz - Germany), where they examined a huge body of data related to psychosomatic illnesses, test anxiety, smoking cessation and pain control during orthodox medical treatments.

In their investigation, Flammer and Bongartz considered a total of 444 studies on hypnotherapy published prior to 2002, by selecting the most suitable research designs for meta-analysis through narrowing their focus merely on 57 controlled trials. The surprising outcomes though the procedures were rigorous, showed that on average hypnotherapy achieved at least 64% success compared to 37% improvement among untreated control groups. Conclusively, it means that hypnotherapy is enough effective.

Expansion of their meta-analysis to include the other non-randomized trials, also, produced enough reliable results. When all other 133 studies were included and were analyzed on the basis of previous outcomes (a step in which providing extra data for over 6,000 patients), the findings suggested an average improvement in 27% of untreated patients over the term of the studies compared with a 74% success rate among those who received hypnotherapy treatments [63]

In 2004, on the basis of a literature search of scientific articles cataloged in CINAHL, PUBMED, the Cochrane Library, and AMED databases relating to the effectiveness of 13 non-pharmacologic methods (hypnosis is one of them) used to relieve pain and reduce suffering in labor among women, it was found that there is adequate evidence of benefit in reducing pain for continuous labor support, baths, intradermal water blocks, and maternal movement and positioning. Hypnotherapy in addition to acupuncture, massage, and transcutaneous electrical nerve stimulation were promising, though further investigations are needed. All the methods studied had evidence of widespread satisfaction among a majority of users [64].

In contrary, in 2005, in another meta analysis study conducted by the Cochrane Collaboration, to find no evidence that hypnotherapy as more successful than other therapies, though the study was restricted to following the issue of smoking cessation, it was found that no hypnotic treatment in achieving cessation of smoking for at least six months [65].

When it comes to irritable bowel syndrome, Wilson, Maddison, Roberts, Greenfield, & Singh (2006) published one of their systematic reviews suggesting that hypnotherapy is effective in the management such a case, where over half of the trials (i.e., 10 of 18) showed a significant benefits [66]

With the adoption of the concept of Neuroplasticity in medical sciences, hypnotherapy is being acknowledged as a method to affect and enhance the brain's neural pathways[67].

In 2007, another meta-analysis by Cochrane Collaboration group found that the therapeutic effect of hypnotherapy was "superior to that of a waiting list control or usual medical management, for abdominal pain and composite primary irritable bowel syndrome and related symptoms, in the short term in patients who fail standard medical therapy;" with no harmful side-effects[68].

For further research findings, searching in many databases, such as the Pubmed, Medline, Dynamed and the PsycINFO and many others, the status of hypnosis and hypnotherapy as a significant clinical procedure found to be in favor of the treatment of many psychological and psychosomatic ailments.

In many of such research works, hypnosis and hypnotherapy found to be useful in the treatment of many conditions, such as pains in children and adolescents,[69, 70] depressions[71], sexuality problems [72], post traumatic stress disorders [73,74], asthma [75], obstetrics, labor and delivery and preterm labor issues [76], cardiovascular responses, [77] fibromyalgia pains [78], dental pains [79], psychosomatic disorders, [80] sleep disorders [81], pain and pain management [82, 83], chronic pains, headaches and migraine [84,85], postoperative pains,86 bowel diseases and gastrointestinal related disorders [87-90], conversion disorders [91], and few to mention.

To end up with pointing to one of those thorough promising research findings, Jensen and Patterson (February-March 2014) in one of their most recent works emphasized on the fact that empirical support for hypnosis for chronic pain management and related stuff has flourished over the past two decades, where many clinical trials showed hypnosis as an effective technique for reducing chronic pain and related psychological and psychosocial factors, although outcomes vary between individuals. In brief, they showed that findings of such clinical trials simply indicate that hypnotic treatments have a number of positive effects beyond any pain control procedures; where such scientific research results as many researchers see it, have an important implications for how clinicians can help their clients experience maximum benefits from hypnosis and those treatments that include hypnotic components [92].

Summary and Conclusions

From the previous tracing history of hypnosis and hypnotic works, the precise outlook on its nature, scope and process with reference to well reputed published works on the subject, it becomes clear that hypnosis and clinical hypnotherapy is not a myth rather a natural phenomenon which goes back to ancient times and well established scientific endeavors. It is a phenomenon that was observed and used cross millennia and centuries until investigated thoroughly in modern times and validated by concerned and authorized medical bodies, like the BMA, AMA, NIH and the BPA. When it comes to its efficacy, it was found through many researches that hypnosis and hypnotherapy is a working clinical tool in the treatment of many psychological and psychosomatic syndromes and related signs and symptoms. Hence, we may conclude that hypnosis and hypnotherapy is an effective valid and reliable clinical tool to be used as a natural medical procedure in health settings; in tackling, managing and curing of many health problems.

Recommendations

On the basis of such findings, strongly the first recommendation that jumps in mind is that time had come for many clinicians in our Arab World to learn all necessary skills vis-à-vis hypnosis and hypnotherapy. Medical staff across the Arab World, particularly, psychologists and psychiatrists are in front of those to be introduced to hypnosis and hypnotherapy. In addition, physicians of all specialties and paramedical health professionals are not to be excluded.

To add more, teaching of hypnotism should be incorporated in medical and health science curricula in all university medical schools, faculties of allied medical and health sciences, concerned departments and units. Though hypnosis and hypnotherapy is a well established choice in Western medicine since more than 200 years, we may say to our medical officials, to start introducing, endorsing and incorporating such medical endeavor though late, much better than not to.

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Play Therapy

Garry Landreth & Sue Bratton

In the process of growing up, children's problems are often compounded by the inability of adults in their lives to understand or to respond effectively to what children are feeling and attempting to communicate. This "communication gap" is widened as a result of adults' insistence that children adopt that means of expression commonly used by adults. Efforts to communicate with children on an exclusive verbal level assume the presence of a well-developed facility for expression through speech and thus confine children to a medium that is often awkward and unnecessarily restrictive. Play is to the child what verbalization is to the adult. It is a medium for expressing feelings, exploring relationships, describing experiences, disclosing wishes, and self-fulfillment. The problems children experience do not exist apart from the persons they are. Therefore, play therapy matches the dynamic inner structure of the child with an equally dynamic approach.

Rationale for Play Therapy

Because children's language development lags behind their cognitive development, they communicate their awareness of what is happening in their world through their play. In play therapy toys are viewed as the child's words and play as the child's language–a language of activity. Play therapy, then, is to children what counseling or psychotherapy is to adults. In play therapy the symbolic function of play is what is so important, providing children with a means of expressing their inner world. Emotionally significant experiences can be expressed more comfortably and safely through the symbolic representation the toys provide. The use of toys enables children to transfer anxieties, fears, fantasies, and guilt to objects rather than people. In the process, children are safe from their own feelings and reactions because play enables children to distance themselves from traumatic events and experiences. Therefore, children are not overwhelmed by their own actions because the act takes place in fantasy. By acting out through play a frightening or traumatic experience or situation symbolically, and perhaps changing or reversing the outcome in the play activity, children move toward an inner resolution, and then they are better able to cope with or adjust to problems.

In a relationship characterized by understanding and acceptance, the play process also allows children to consider new possibilities not possible in reality, thus greatly expanding the expression of self. In the safety of the play therapy experience, children explore the unfamiliar and develop a knowing that is both experiential-feeling and cognitive. It can then be said that through the process of play therapy, the unfamiliar becomes familiar, and children express outwardly through play what has taken place inwardly. A major function of play in play therapy is the changing of what may be unmanageable in reality to manageable situations through symbolic representation, which provides children opportunities for learning to cope.

The Process of Play Therapy

Given the opportunity, children will play out their feelings and needs in a manner or process of expression that is similar to that for adults. Although the dynamics of expression and the vehicle for communication are different for children, the expressions (fear, satisfaction, anger, happiness, frustration, contentment) are similar to those of adults. Children may have considerable difficulty trying to tell what they feel or how their experiences have affected them. If permitted, however, in the presence of a caring, sensitive, and empathetic adult, they will reveal inner feelingsthrough the toys and materials they choose, what they do with and to the materials, and the stories they act out. The play therapy process can be viewed as a relationship between the therapist and the child in which the child utilizes play to explore his or her personal world and also to make contact with the therapist in a way that is safe for the child. Play therapy provides an opportunity for children to live out, during play, experiences and associated feelings. This process allows the therapist to experience, in a personal and interactive way, the inner dimensions of the child's world. This therapeutic relationship is what provides dynamic growth and healing for the child.

Because the child's world is a world of action and activity, play therapy provides the therapist with an opportunity to enter the child's world. The child is not restricted to discussing what happened; rather, the child lives out at the moment of play the past experience and associated feelings. If the reason the child was referred to the therapist is aggressive behavior, the medium of play gives the therapist an opportunity to experience the aggression firsthand as the child bangs on the Bobo or attempts to shoot the therapist with a gun and also to help the child learn self-control by responding with appropriate therapeutic limit-setting procedures.

Without the presence of play materials, the therapist could only talk with the child about the aggressive behavior the child exhibited yesterday or last week. In play therapy, whatever the reason for referral, the therapist has the opportunity to experience and actively deal with that problem in the immediacy of the child's experiencing. Axline (1947) viewed this process as one in which the child plays out feelings, bringing them to the surface, getting them out in the open, facing them, and either learning to control them or abandon them.

Toys and Materials

Although desirable, a fully equipped playroom is not essential for children to express themselves. What is important is that children have ready access to play materials selected for the purpose of encouraging expression. All toys and materials do not automatically encourage children's expression or exploration of their needs, feelings, and experiences. Therefore, toys should be selected, not collected. Play therapy is not used as a way to pass the time or to get ready to do something else. The purpose is not to engage the child's hands while trying to elicit some verbal expression from the child's mouth. Consequently, careful attention should be given to selecting play materials that aid in the following:

- Exploration of real life experiences
- Expression of a wide range of feelings
- Testing of limits
- Expressive and exploratory play
- Exploration and expression without verbalization
- Success without prescribed structure

Mechanical or complex toys would not fit these objectives and so are avoided. Play materials that require the counselor's assistance to manipulate are inappropriate. Many children in need of play therapy have poor self-concepts and are overly dependent. Play materials should not reinforce such problems. Landreth (1991) has provided a list of specific toys and materials.

Setting Limits in Play Therapy

Limit setting is a necessary and vital part of the play therapy therapeutic process. Although the procedures for setting limits may vary, the setting of therapeutic limits is part of all theoretical approaches to play therapy. The structure of therapeutic limits is what helps to make the experience a real-life relationship. Limits in play therapy have both therapeutic and practical benefits in that they preserve the therapeutic relationship, facilitate the child's opportunities to learn self-responsibility and self-control, among many other dimensions, and provide the child and the therapist with a feeling of emotional security and physical safety. This feeling of emotionalsecurity enables a child to explore and express inner emotional dimensions that perhaps have remained hidden in other relationships.

Play therapy is not a completely permissive relationship because children do no feel safe, valued, or accepted in a relationship without boundaries. Boundaries provide predictability. Therefore, children are not allowed to do anything they want to do. A prescribed structure provides boundaries for the relationship that the play therapist has already determined are necessary. The play therapy relationship has minimal limits. Messiness is accepted, exploration is encouraged, neatness or doing something in a prescribed way is not required, and persistent patience is the guiding principle. The child's desire to break the limit is always of greater importance than actually breaking a limit. Because play therapy is a learning experience for children, limits are not set until they are needed. The child cannot learn self-control until an opportunity to exercise self-control arises. Therefore, placing a limitation on a child pouring paint on the floor is unnecessary unless the child attempts such an activity. Limits are worded in a way that allows the child to bring himself or herself under control. The objective is to respond in such a way that the child is allowed to say "No" to self. "You would like to pour paint on the floor, but the floor is not for pouring paint on; the pan on the table is for pouring paint into" recognizes the child's feeling, communicates what the floor is not for, and provides an acceptable alternative. The child thereby is allowed to stop himself or herself.

Play Therapy Research and Results

Play therapy is not an approach based on guess, trial and error, or whims of the play therapist at the moment. Play therapy is a well-thought-out, philosophically conceived, developmentally based, and research-supported approach to helping children cope with and overcome the problems they experience in the process of living their lives. Play therapy has been demonstrated to be an effective therapeutic approach for a variety of children's problems including, but not limited to, the following areas:

- abuse and neglect
- aggression and acting out
- attachment difficulties
- autism
- burn victims
- chronic illness
- deaf and physically challenged children
- dissociation and schizophrenia
- emotionally disturbed children
- enuresis and encopresis problems

The popular myth that play therapy requires a long-term commitment for many months is unfounded as is shown in case studies and research reports reported by Landreth, Homeyer, Glover, and Sweeney (1996) in their book, *Play Therapy Interventions with Children's Problems*.

Professional Training

Prospective play therapists have to be adequately trained. Most play therapists have a master's degree in counseling, psychology, or social work, although other disciplines also are represented in the field. A master's degree representing some area of the helping profession with emphasis on the clinical or counseling aspects of therapeutic relationships is a general prerequisite. Within or in addition to such a program, training should incorporate the areas of child development and basic counseling skills including acquisition of a theoretical approach incorporating a rationale for behavior change consistent with the play therapy approach utilized. The program of study should include extensive training in the area of play therapy and a supervised practicum experience with children in play therapy.

Future Trends in Play Therapy.

The field of play therapy is growing and is now represented by the Association for Play Therapy, an international professional organization. A national Center for Play Therapy has been established at the University of North Texas. Increasing numbers of elementary school counselors and therapists in private practice and agencies are incorporating play therapy into their work with children. There is a trend in family therapy to address social and emotional values of developmental as well as family group session issues. In filial therapy, parents are trained to use play therapy procedures with their children. This method is well researched and has proven to be effective in ameliorating children's problems through enhancing the parent-child relationship.

Conclusion

Play therapy is based on developmental principles and, thus, provides, through play, developmentally appropriate means of expression and communication for children. Therefore, skill in using play therapy is an essential tool for mental health professionals who work with children. Therapeutic play allows children the opportunity to express themselves fully and at their own pace with the assurance that they will be understood and accepted.

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“Many things can wait; children cannot.

Today, their bones are being formed,

their blood is being made, their senses are being developed.

To them, we cannot say ‘tomorrow.’

Their name is today.”

—Gabriela Mistral, Nobel Laureate (1945)

Introduction

Angel did not know how to play. She scanned all the toys on the shelves or laid out on the mat. She stood still, shoulders stiff, with a vigilant gaze. The lady with some glitter in her hair and paint on her clothes said Angel could play with any of the toys that she liked, but Angel sat still and silent on the couch for the first three sessions.

In session four, Angel went over to home corner and threw some dishes down onto the floor. Then she noticed the baby dolls. Some were in prams and some were in cots. They had blankets and bottles nearby. Angel took two of the baby dolls out of their cots and began hitting them, calling them stupid, changing their clothes very roughly, and throwing them around the room.

Angel was 5, and her father was diagnosed with cancer a year ago. Angel could not speak much to anyone. At home when she tried to speak she was usually ignored or shouted at. Mum said that she needed to be quiet ever since her father got sick. “We don’t want to upset him again. Who knows what he might do to us this time!” Angel had lots of nightmares, but she did not know who to go to for cuddles when she was afraid. At school she was scared that she was going to do the wrong thing, so she tried to keep still during free time, and she never volunteered an answer in class. She tried to write her name on some colouring-in once, but only knew how to do the *A*. The other children spoke in sentences, so Angel’s toddler-like vocabulary didn’t seem to be enough for the teacher. At lunchtime there were balls and skipping ropes around, and the other children seemed to know how they worked. The other children laughed and talked to each other while they played.

Angel's older brother Tim had not fared much better. He got into fights at school and was often in trouble. When Tim said he wished he was dead, Mum started crying more than usual. She told Angel and Tim that she was sorry.

Mum telephoned the lady with some glitter in her hair and paint on her clothes. "It's not their fault. But I have noticed that Angel has changed. She's not the happy girl she used to be. She's quiet and sad and sometimes gets angry. She's not doing very well at school and is getting in trouble a lot. I didn't think this would affect her or Tim quite so much. I can't change their father. Can play therapy help? What does play therapy do?"

What is Play?

Commentary on play dates back to the time of Plato (429 - 347 BC), who is quoted as saying: "You can discover more about a person in an hour of play than in a year of conversation." Albert Einstein noted that "the highest form of research is play" (cited in Henniger, 1987), and Friedrich Froebel, the creator of the first kindergarten supposed that "Play…. is the free expression of what is in a child's soul … play is not mere sport but full of meaning and import" (1903).

Play is popularly considered to be spontaneous pleasurable behaviour, solely for amusement, yet it is far more than that. Play is central to a child's healthy cognitive, motor, linguistic, emotional and social development (Ginsberg, 2007). Play is crucial for children's creativity, exploration, physical skills, self-regulation, interpersonal development and social learning. The necessity of play for a child's growth is undeniable.

Jean Piaget (1962) proposed that play bridges the gap between concrete experience and abstract thought. That is, in play, a child deals in a sensory-motor way with objects as symbols of something else abstract in the child's experience. Children, who are developmentally not yet able to engage fully in abstract reasoning until approximately age eleven (Landreth, 2002), express themselves more freely and directly through self-initiated, spontaneous play. A child may scribble furiously with sharp quick movements to express anger or frustration, or sing quietly while sitting with a doll to express contentment. A fearful child is likely to refrain from play, and a confused child may not be able to settle into one play activity for an extended time.

Landreth (2002) further outlined that play provides a means through which feelings can be communicated. Instead of verbalising particular situations, thoughts or emotions, a child may use toys to communicate – e.g. shooting a dragon, locking up the bad guys, or punishing the doll that represents a younger sibling. For many, this type of play is an example of how children attempt to organise their experiences at a developmentally appropriate level, and gain a sense of mastery and control in their life.

Stagnitti and Cooper (2009) explained that elements of play which include storytelling and imagination are known precursors to language development and other functional skills such as understanding context, narrative, predicting and planning. Further, play is a child's introduction to formal education, as children familiarise themselves with shapes, colours, numbers, letters, and words.

The United Nations (1991) proclaim play as a "universal and inalienable right of childhood."

The History of Play Therapy

Historically, play therapy has emerged from the conceptual understandings found in psychoanalytic and humanistic psychology, psychotherapy and child development literature. In Europe in the early 1900s, Melanie Klein and Anna Freud (the daughter of Sigmund Freud) included play in their psychoanalytic treatment of children (Cattanach, 2008; Donaldson, 2002; Landreth, 2012). In 1935, Margaret Lowenfeld first published *Play in Childhood* documenting her in-depth observations of children whilst working at the 'Clinic for nervous and difficult children' in London (Urwin & Hood-Williams, 1988). In the United States, Virginia Axline focused her work on conceptualising and documenting Non Directive Play Therapy (NDPT) by drawing from the humanistic and relational perspective of her teacher and colleague Carl Rogers (Axline, 1969). Rogers (1951) was instrumental in establishing treatment plans based on necessary and sufficient conditions for growth. These included therapeutic congruence, unconditional positive regard, empathic understanding and acceptance.

With a focus on child populations, Axline (1969) identified her eight principles for NDPT where the therapist:

- Must develop a warm and friendly relationship with the child.

- Accepts the child as she or he is.
- Establishes a feeling of permission in the relationship so that the child feels free to express their feelings completely.
- Is alert to recognise the feelings the child is expressing and reflects these feelings back in such a manner that the child gains insight into their behaviour.
- Maintains a deep respect for the child's ability to solve their problems and gives the child the opportunity to do so. The child is empowered to decide and to make choices.
- Does not attempt to direct the child's actions or conversations in any manner. The child leads the way, the therapist follows.
- Does not hurry the therapy along.
- Only establishes those limitations necessary to anchor the therapy to the world of reality and to make the child aware of their responsibility in the relationship.

From these beginnings, several different theoretical models of play therapy have emerged. Based on Axline's principles, the most well-known is Non-Directive or Child-Centred Play Therapy (CCPT) which has been further developed by Garry Landreth and colleagues (Ginott, 1937; Guerney, 1994; Homeyer & Landreth, 1998; Landreth, 2012; Landreth & Bratton, 1998; Landreth, 1978, 2001; Landreth & Hendricks, 1977; Landreth & Wright, 1997; VanFleet, Sywulak, & Sniscak, 2010; West, 1996) and integrated into teaching parents or guardians the principles found in CCPT, known as Filial Play Therapy (Guerney, 2000; Guerney & Guerney, 1987, 1989; Smith & Landreth, 2003; VanFleet, 2005; VanFleet, Ryan, & Smith, 2005).

What is Play Therapy?

Play therapy is a therapeutic modality wherein play is the principle therapeutic factor. Because play is a child's natural medium of communication, play therapy is an ideal modality in which to allow children to express their feelings and deal with their emotional problems. In essence, play therapy is for children what 'talk therapy' or counselling is for adults.

Landreth (2012, p. 11) defines play therapy as:

> *… a dynamic interpersonal relationship between a child (or person of any age) and a therapist…who provides selected play materials and facilitates the development of a safe relationship for the child ... to fully express and explore self (feelings, thoughts,*

experiences, and behaviours) through play, the child's natural medium of communication, for optimal growth and development.

Play therapy provides the child with a consistent and predicable therapeutic relationship and environment in which to explore their fears, difficulties, struggles and pain, as well as hopes, dreams and fantasies. The underpinning assumption of play therapy is that, given this relationship and environment, the child has the inner resources to bring about growth and change in their own lives.

Play therapists utilise a range of theoretical models which are founded on varying therapeutic disciplines including: Child-Centred, Gestalt, Adlerian, Psychoanalytic, Jungian, Object relations, Family Therapy, Eriksonian, and Cognitive Behavioural. The play therapy model utilised by a play therapist will vary depending on the therapist's specific training and theoretical orientation.

The more traditional approaches, such as CCPT, move at the child's pace. Other approaches view the therapist as the expert and utilise more directive approaches, such as Cognitive Behavioural play therapy (Drewes & Cavett, 2012) and Prescriptive Play Therapy (Goodyear-Brown, 2010). This differentiation between directive and non-directive approaches has led to the development of other models that incorporate both styles of engaging with a child.

An example of an emerging model is the Play Therapy Dimensions Model (PTDM; Yasenik & Gardner, 2012). This PTDM model is an integrative model, as is considers how the therapist responds to the specific needs of the client by examining the therapeutic needs as they occur on two dimensions: the conscious/unconscious dimension and the directive/non-directive dimension. This creates four quadrants (See Figure 1):

- *Non-intrusive responding* (unconscious/non-directive)
- *Co-facilitation* (unconscious/directive)
- *Active utilization* (conscious/non-directive)
- *Open discussion and exploration* (conscious/directive)

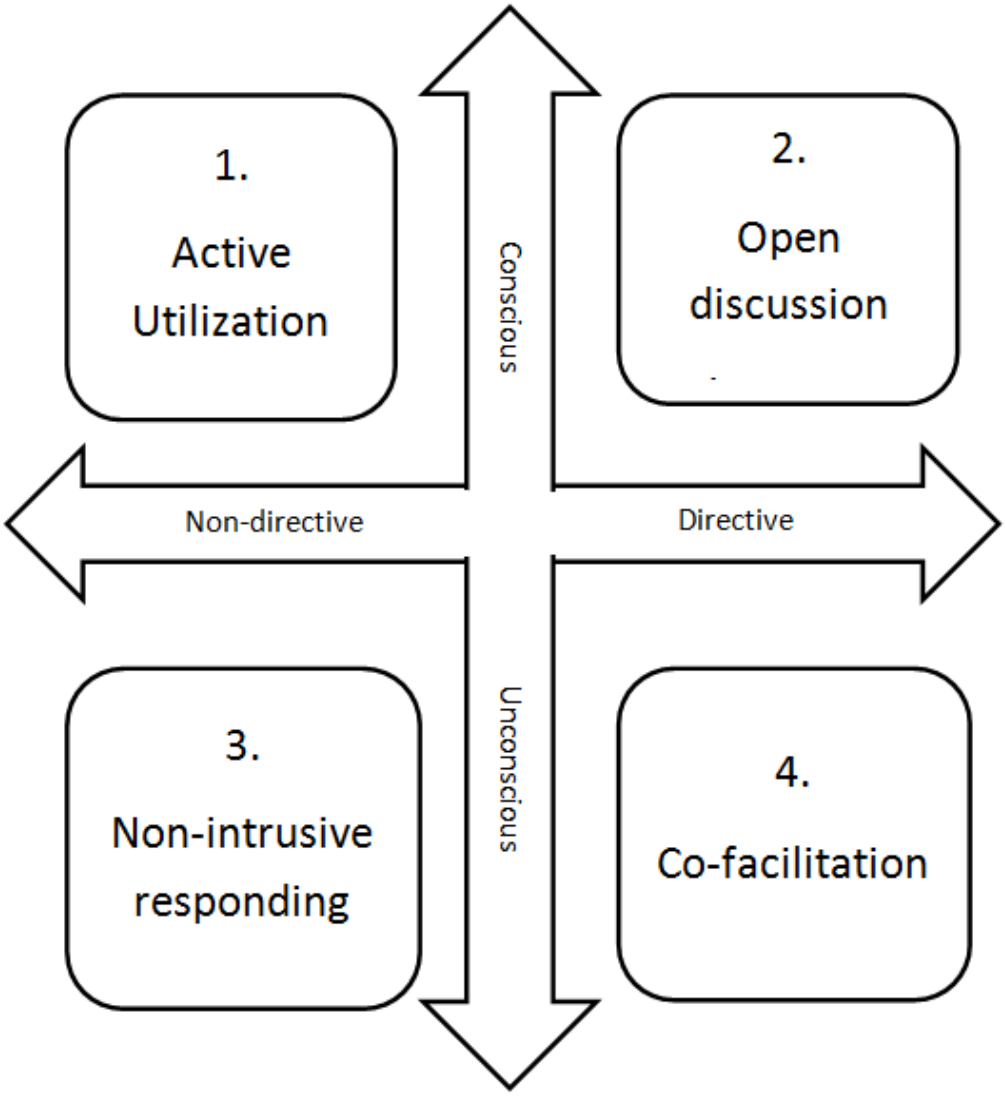


Figure 1. Yasenik and Gardener's Play Therapy Dimensions Model.

The Play Room

The necessary elements within the play room are: the child, the therapist, the relationship formed by the child and therapist, the play therapy room and the play room contents. The play therapy space is usually a designated room, set up in a particular and predictable way. Within the room, there are a wide range of expressive tool and toys. These may include art and craft materials, dress-ups and masks, a home corner/kitchen, musical instruments, puppets, figurines, toy animals, toy weapons and military characters, a toy medical kit, superheroes, books, vehicles, building blocks, a dollhouse and dolls, balloons and balls, a sand tray, toys for dance and movement, and a table with clay. Some describe the play

therapy room as needing to contain something to represent everything in the child's world. Some play rooms contain model court houses or prisons, if these settings are poignant for the child. Guerney (2001) outlined that a range of toys allow a child to understand that a range of behaviours are permitted in the playroom. Toys that support issues of aggression, regression, independence and mastery are as important as toys that foster nurturing, acceptance and contentment.

The Play Therapy Process

In the play therapy process, the therapist first undertakes a thorough parent/carer consultation, obtaining a concise history in relation to the child's development, functioning, and present or past concerns. Regular reviews are conducted with parents, teachers and others involved in family support. Most children participating in play therapy are between 2 and 12 years of age, however, this may vary. For example, a child who has a cognitive or developmental delay may participate in play therapy at an older chronological age.

Generally, the therapist and child meet weekly for sessions of between thirty and fifty minutes. As with all forms of therapy, the number of sessions undertaken will depend on the child's presenting issues, the particular therapist's approach, and available funding. Some children with mild issues may see improvement after 6 or 12 sessions, where children with complex issues may require up to forty-eight sessions, or more.

In most play therapy modalities, the therapist gives limit-setting statements, designed to provide safety and boundaries. Norton and Norton (2002) outline that there are 'absolute limits' (the child will not hurt themself or the therapist), 'clinical limits' (all the toys remain in the playroom, the child is encouraged to remain in the play room for their allocated time, when the session ends, the child leaves the room) and 'reactionary limits' (the child does not deliberately break the toys). The play therapist conveys the limits to the child in a firm and consistent way. Many children desire to test these boundaries. In response to this occurring, Guerney (2001) outlined that a combination of the acceptance of the child's wish or determination to defy the rules, along with the authoritative stance of the therapist that the rules are adhered to, is one of the most powerful therapeutic phenomena in child-centred play therapy.

Therapeutic Stages in Play Therapy

A number of play therapists have described a discernible pattern in the therapeutic process of children's play in therapy (see Landreth, 2002; Nordling & Guerney, 1999; Moustakas, 1997). Norton and Norton (2002) illustrate the therapeutic stages of play therapy as follows (see Figure 2):

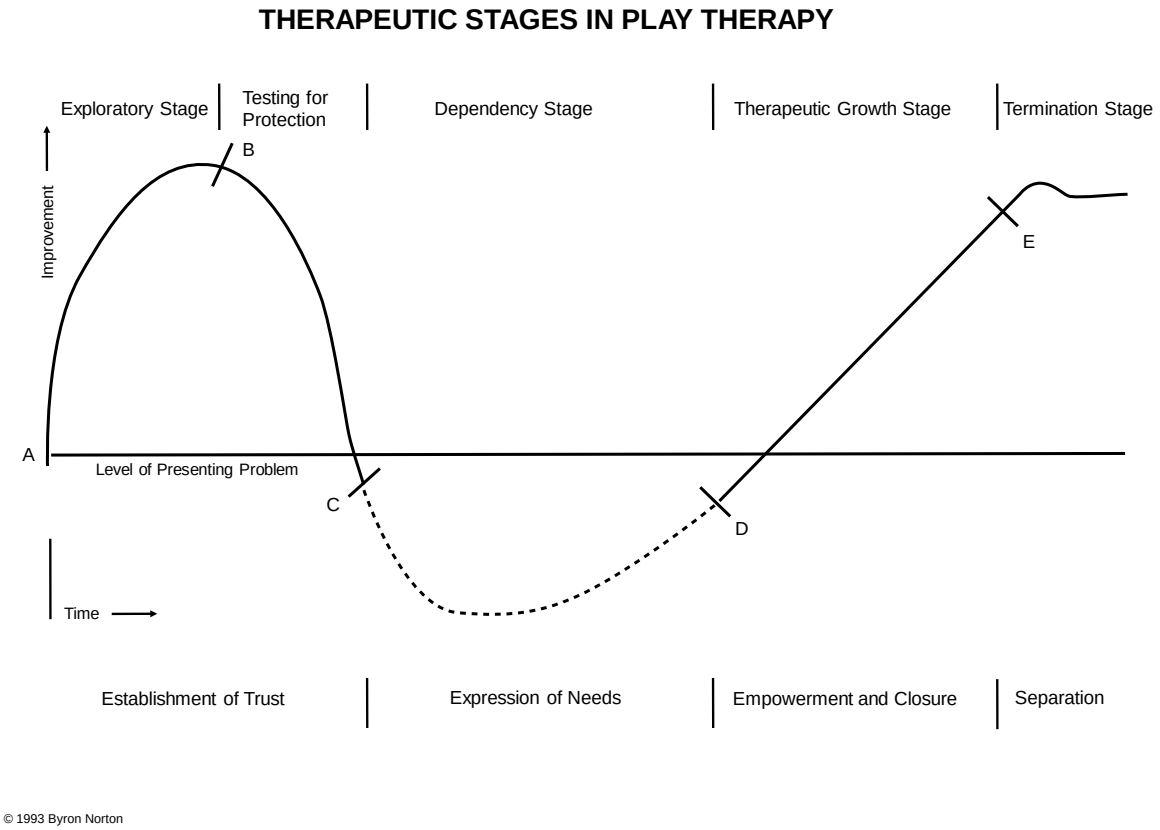


Figure 2. Norton and Norton's Therapeutic Stages in Play Therapy.

- The *Exploratory Stage* involves the establishment of trust. The child becomes familiar with the room by exploring the environment, and the therapist begins to build a relationship with the child, consistently responding in a particular way.

- The *Testing for Protection Stage* is characterised by expression of the child's needs. In this stage, the child must know that the therapist will accept them and the feelings that they share. The child may be tentative in sharing emotions. As trust grows, progress continues.

- The *Dependency Stage* involves greater expression of needs. The child works through a number of emotional themes that are personally meaningful. The child may engage

in regressive play or dramatic play and use fantasy play to disguise the content. The therapist continues to respond or participate in a particular way.

- The *Therapeutic Growth Stage* is characterised by empowerment and closure. The nature of the play changes as the child gains mastery and greater emotional understanding. The play becomes more interactive and cooperative and shifts progressively further away from dependency towards mastery.
- The *Termination/Closure Stage* involves separation and cessation of the relationship.

Stage duration varies from child to child – sometimes lasting only long enough to be recognisable, while others last for many sessions. Consistent with all stage theories, regression is seen shortly after establishment of trust. Parents and carers may notice deteriorating behaviours at home or in other environments shortly after play therapy commences, due to the child uncovering, exposing and examining their own hurts or difficulties. Experienced play therapists will provide assurance that empowerment and improvement will follow.

Neurobiology and Attachment

Contemporary neuroscience and psychotherapy theories suggest that health and well-being is related to increasing levels of growth, complexity and integration within the human brain. Neurologically, this means that well-functioning neural networks should be continually communicating and forming new connections. In daily life, the result of an integrated brain is greater ability to self-regulate, love, relate, play and learn (Cozolino, 2010). The goal of scientifically informed psychotherapy is therefore to foster integration in the brain. In keeping with best practice in adult therapy (Hubble, 1999), the primary way this integration occurs during play therapy is in the building of a working alliance between child and therapist. In addressing childhood trauma, leading researchers and clinicians Perry and Szalavitz (2006) stress the importance of this relationship: “Humans are inescapably social beings. . . Relationships are the agents of change…” (pp. 230-231). Similarly, Siegel and Hartzell (2004) add:

> *It is not just inheritance that determines human development. Genes determine much of how neurons link up with each other, but equally important is that experience*

> *activates genes to influence this linkage process. How we treat our children changes who they are and how they develop . . . Nature needs nurture* (p. 34).

During play therapy, the therapist remains attuned to the child's needs, and 'mirrors' the child as they play; physically copying the child's play actions, in such a manner as to reflect the child's play actions back to them. In this way, the child gains insight into their behaviour, and experiences the natural consequences of their actions. Further, social bonding neural networks, such as the mirror neuron system are activated in a relationship in which this type of reciprocity occurs. In the child's brain, neuropeptides and opioids are released, leading to the child feeling a sense of well-being and contentment (Golding & Hughes, 2012). This reduces feelings of anxiety or anger, and improves the child's immune system (Badenock, 2008). This act of mirrored play allows the child to safely operate at reduced levels of stress while in the therapeutic space, activating neural growth hormones supportive of new learning (Cowan & Kandel, 2001).

Conversely, chronic stress prevents meaningful play. A child who has experienced trauma, severe loss or grief, witnessed violence, endured chronic stress, or demonstrates insecure attachment is largely unable to play. This in turn means the child feels reduced connection to others, is less able to form meaningful relationships and therefore remains in a state of self-preservation. Thus the lower brain, which is responsible for regulating basic safety and survival processes, such as breathing, blood circulation and arousal regulation, impairs higher order brain functions such as speech and language and socio-emotional communication (Perry, 2009).

Treatment of this state of chronic stress is achieved via bottom-up sensory-motor regulatory approaches through child-centred play therapy (Gaskill, 2010). In addition, filial therapy, or the inclusion of a parent-child treatment, is recommended to target disrupted attachment, often having most impact on these lower brain centres (Barfield, Dobson, Gaskill & Perry, 2012).

Play itself is a creative process and enhances every domain of a child's development (Perry, 2001). Recent research suggests that the neural networks for creativity are located in multiple regions of the brain which are all working together, truly making creative play a whole of brain process (Kaufman, 2013). Activating a child's creative networks leads to desired therapeutic outcomes of growth, complexity and brain integration. Relaxed and engaged play

physiologically interrupts the stress cycle (Vayle, 2013) and reduces chronic stress and heightened arousal.

In play therapy, the therapist acts as a secure attachment figure for the child, providing necessary predictability and safety, allowing the child to rest from allocating attention to safety and survival. Although this experience only occurs in the playroom, the child neurologically develops an understanding of safety and predictability. Capacity for growth and development is therefore increased, and becomes transferable to other spheres of life.

Measurement

Measuring progress through play therapy is as important as it is in any therapy modality. Goodman's Strengths and Difficulties Questionnaire (SDQ; Goodman, 1997) is internationally used by play therapists to measure the child's observable behaviours pre and post-treatment. Typically, the SDQ is completed by both the child's parent or carer and the child's teacher in order to gather comprehensive data from the home and school or pre-school environments. The SDQ incorporates scales of:

1) Emotional symptoms (5 items) e.g. *often unhappy, downhearted or tearful*
2) Conduct problems (5 items) e.g. *often fights with other children or bullies them*
3) Hyperactivity/inattention (5 items) e.g. *easily distracted, concentration wanders*
4) Peer relationship problems (5 items) e.g. *rather solitary, tends to play alone*
5) Pro-social behaviour (5 items) e.g. *considerate of other people's feelings*

As well as providing insight into a child's score on each scale, adding scales 1-4 together generates a total difficulties score (the pro-social behaviour scale is excluded from this total difficulties score). From the total difficulties score, an indication of the overall severity/clinical significance of presenting problems can be obtained. A score of 16-25 indicates some risk, 25-29 indicates considerable risk, and 30+ indicates extreme risk. Generally, the higher the initial scores, the more therapy sessions the child will need in order to return to normal levels of functioning.

A number of other scales and assessments are used play therapists, including the Spence Anxiety Assessment (Spence, 1998), the Symbolic Play Test (Lowe & Costello, 1988) and the Test of Pretend Play (Lewis & Boucher, 1997). Another assessment tool, the Child-Initiated Pretend Play Assessment (ChIPPA; Stagnitti 2007), assesses both conventional-

imaginative play and symbolic play in the same play assessment. It considers the quality of a child's ability to spontaneously self-initiate pretend play and measures the child's ability to sequence their play actions and use symbols in play. Children who are struggling in their play may not use all the toys, may demonstrate poor ability to start a play idea, or may have moved a lot of the toys but have no overall linking narrative. The ChIPPA has been used for children with cerebral palsy (Pfeifer et al., 2012), children with acquired brain injury (Fink, Stagnitti & Galvin, 2012), children with pre-academic problems (Stagnitti, Unsworth & Roger, 2000) and children with autism (O'Connor & Stagnitti, 2011; Stagnitti, 2010).

Play Therapy is Effective for Which Presenting Problems?

Play therapy has an established and growing evidence-base and has been proven to be effective in addressing myriad of presenting problems in children. These presenting problems include:

- *psychosocial issues*, such as shyness, anxiety, stress, poor communication, grief and loss;
- *behavioural problems* such as aggression, poor motor co-ordination, self-harming, and attention deficit hyperactivity disorder;
- *responses to family and relationship problems*, such as family violence, parental separation, attachment disorders, trauma and abuse;
- *educational issues* such as poor organisational skills, poor planning and execution of tasks, poor story comprehension, and
- *disability* including autism, psychosis, sensory impairment and intellectual impairment.

Results

A summary of the effectiveness of play therapy, presented by Play Therapy International and Play Therapy UK in 2011 showed that, based on data from 8026 cases, between 74% and 83% of children receiving play therapy showed a positive change (Thomas, 2011). The percentage of girls who showed improvement (79%) was slightly higher than boys (73%). It was also found that younger children showed a greater percentage of positive change than their older counterparts: 80% at age 6 compared with 71% at age 12. Children who presented with more severe problems showed greater positive change; for those considered *abnormal/at*

risk, 76% showed a positive change, for those at *considerable risk*, 82% showed a positive change, and for those at *extreme risk*, 88% showed a positive change. Of the children who do not show positive change, some experience an unplanned exit from play therapy, due to funding restrictions or family circumstances, before the therapeutic growth stage had taken place.

Applications of Play Therapy in Specific Settings

Further applications of play therapy are regularly emerging in situations where children can be assisted to work through a range of difficulties.

Government Departments and Family Intervention Services

Play therapy is already operating with great success in government departments such as Family and Community Services (FaCS), and Aging Disability and Home Care (ADHC). Within ADHC, a play therapy pilot was established to address the psychiatric and severe emotional and behavioural problems experienced by children suffering Intellectual Disability. Of the 31 children in the program, 74% had a secondary diagnosis, the two most prevalent being Autism (35%) and ADHD (29%). It was hypothesised that a non-directive approach would be most appropriate for children with Intellectual Disability as they many have limited choice and control in their world (Brown & Brown, 2005).

Pre and post therapy scores showed significant improvement in the children's presenting issues after an average of 16 sessions. Parents also reported many qualitative changes in the children's emotional, behavioural and social presentation, including increases in use of general and emotion-specific language at home and articulating words more clearly and correctly. There were extensive changes in children's capacity to self-regulate, such as: initiating a calm physiological state, expressing feelings, and seeking affection and comfort from parents. Socially, the children attuned to others, took turns, developed problem solving strategies, sought out interaction and began to develop friendships. These findings are consistent with existing literature (Demanchick, Cochran & Cochran, 2003; McMahon, 1992; Swan, 2011), and suggest profound change for a group of previously isolated and vulnerable children.

Of the subgroup with autism, many parents commented on the social and behavioural differences in their children. For example, some children began to make independent choices in daily life, others made eye contact, some started to hug their parents and others started communicating verbally with their peers and parents rather than using aggression to meet their needs. These findings are congruent with other research on autism and play therapy (Josefi & Ryan, 2004; Levin, 2005; Parker & O'Brien, 2011, Scanlan, 2007) and suggest play therapy is beneficial for children with autism.

Within Non Government Organisations (NGO's), play therapy successfully operates in Brighter Futures Family Support and Intensive Family Support services, assisting to provide positive outcomes for vulnerable families. Further to this, play therapy has an effective service to offer children at risk within the juvenile justice system. Partnerships between play therapy services and health, education and disability agencies are recommended.

Play Therapy in Special Educational Settings

Play therapy has also been demonstrated to be an effective intervention in special school educational settings. Stagnitti, O'Connor and Sheppard (2012) found that children in a specialist play program increased in their social competence and language ability compared to children who were not involved in the program. A comparison of a play based curriculum versus a traditional curriculum showed that the play based children improved significantly more in narrative language, complex play ability and social cohesion than children in the traditional curriculum (Reynolds, Stagnitti, & Kidd, 2011).

Within 7 sessions in individual therapy, children showing signs of autism were shown to have improved in the areas of representational thinking, using symbols in play, social turn taking, understanding use of the toys, and emotional involvement in the play (Stagnitti & Casey, 2011). These skills promote greater skills in self-regulation and social competence in later life. Play based intervention in school would be beneficial for target groups that included children with autism, pre-academic problems and developmental delay.

Critical Incident Intervention

Play therapy has also been effectively used as a critical incident intervention for children. Critical incident intervention may occur in prisons, domestic violence shelters, or in shelters

set up immediately following natural disasters. Play therapy is usually used in modified form when applied to short-term community crisis intervention, such as was the case after the Victorian bushfires of February 2009. The Australasia Pacific Play Therapy Association (APPTA) was one of many groups who offered practical support following the bushfires, sending a number of play therapists to attend some of the worst affected areas. Therapeutic play spaces were set up in school halls, libraries and classrooms, and play tools including fire trucks and other emergency vehicles were provided. Bratton, Ray, Rhine and Jones (2005) reported an inverse relationship between number of sessions and treatment effect size in critical incident play therapy. These results suggest that, in keeping with early intervention principles, children in crisis respond more readily to treatment provided at the time of crisis, rather than delaying treatment onset.

Asylum Seeker, Refugee and Linguistically Diverse Populations

Providing effective therapy is essential for asylum seeker and refugee populations who are known to regularly suffer from post-traumatic stress (Vincent, Jenkins, Larkin, & Clohessy, 2013). However, verbal therapy can be problematic when the client and therapist do not share a common language. Play therapy is effective in this regard, as it is a primarily non-verbal approach, and it has demonstrated effectiveness for children who experience trauma and abuse (Drewes, 2001; Homeyer & Landreth, 1998; Ogawa, 2004). It is therefore suggested that play therapy be recognised, considered and researched as a suitable therapy modality for asylum seeker and refugee populations, and non-English speaking populations.

Hospitalised Children

It is well documented that when children require hospitalisation and medical procedures that it can be stressful for the child and the child's family (Hagglof, 1999; Parson, 2004, 2008, 2009). In Sweden, in order to mitigate and relieve the child's stress, every child admitted to hospital since 1977 has been allocated a play therapist (Hall & Cleary, 1988). However, in Australia, hospital based play therapists are extremely limited, particularly in rural and regional areas (Parson, 2009). Play therapy could assist in preparing children for potentially painful medical procedures, and be used as a complementary psychosocial health care service.

The Value of Early Intervention

Early intervention services, such as play therapy, are regularly estimated to provide a far greater return than the initial investment. While large scale studies are yet to be conducted regarding the cost-benefit ratio of play therapy in Australia, The University of New South Wales Social Policy Research Centre estimates that across a broad range of early intervention strategies, the cost-benefit ratio ranges from between $2.36 to $19.10 for every $1 spent (Valentine & Katz, 2007). Parents for Children's Mental Health in Ontario Canada (2013) surmise that for every $1 spent on early intervention and treatment of mental illness in children and youth, an estimated $7 will be saved to the economy.

Mental illness is the single largest contributor to the disability burden in Victoria, Australia, and accounts for 70 per cent of the disease burden in young people (The Boston Consulting Group, 2006). Since it is estimated that 70 per cent of childhood mental health issues can be solved with early intervention and therapy (Parents for Children's Mental Health, 2013), can society afford to delay investment in early intervention services?

The benefits of early intervention extend well beyond the financial arena. Families report greater stability, improved satisfaction in family relationships, and reduced child behaviour management problems. This in turn promotes child safety, as reduced levels of household stress contribute to increased safe parenting practices, and reduced notifications of child abuse and neglect (Productivity Commission, p. 77). Appropriate intervention for children not only benefits the child and family, but also has broader community benefits. Large scale early intervention programs report societal benefits such as "longer term savings from improved educational outcomes, better labour market participation, reduced dependence on public assistance and lower levels of criminal activity" (Karoly, 2001, p. 2). The growing body of evidence from post-play therapy assessments are consistent with these findings, as most children demonstrate increased ability to learn, improved mental health, and improved pro-social behaviour.

While early intervention services remain out of reach for many children who need them, the economic, social and criminal cost to the Australian economy grows as demand on services increases at the tertiary end of the service continuum. In New Zealand, in 2001, the Department of Corrections estimated the costs and benefits for a range of preventive interventions, designed to reduce the chance of imprisonment. These estimates revealed that identifying a behaviour disorder and providing an early intervention program for a child in

the primary school years would cost $5000 per child, as compared with $255 000 per offender later in life, at a cost benefit ratio of 1:51 (Jacobsen, 2002).

Training and professional recognition of Play therapy in Australia

Worldwide, play therapy is gaining recognition as an effective psychosocial and mental health treatment modality. In some countries, Play Therapy has already become a protected title, for example in USA, Canada and South Africa. Play therapy is not yet a recognised as a distinct professional title or allied health service in Australia.

Currently, play therapists registered under the Access to Allied Psychological Services (ATAPS) or Better Outcomes programs are able to offer play therapy services to their clients under this scheme. The number of families who will benefit from play therapy will further increase if play therapy is given professional recognition as an approved therapeutic intervention by government funding programs.

The undergraduate training of play therapists are commonly nursing, occupational therapy, counselling, psychology and education. In 2015, a Master of Child Play Therapy will become available via Deakin University, in Geelong, Victoria. The course will be offered off campus with intensive on-campus units and will include a range of play therapy techniques using humanistic, systemic and emerging models of play therapy. This approach gives this degree a clear theoretical and evidence-based foundation which is required for further progression of play therapy in Australia. Play therapy research projects are currently being undertaken with eleven research students across at least five states.

Currently, the three play therapy associations in Australia provide various and some overlapping services: clinical registration, educational opportunities and accreditation, and professional networks. The Australasia Pacific Play Therapy Association (APPTA), the first not-for-profit organisation, was formed in 2007, then Play Therapy Australasia (PTAU) a branch of Play Therapy International (PTI) and more recently the Australian Play Therapists Association (APTA). The three associations are forming networks and will campaign with a united voice to showcase the benefits of play therapy within Australia. Further public and political awareness of play therapy is necessary to support the professionalisation of play therapy in its own right. Members of the public can access play therapy by contacting one of the above associations.

Conclusion

Angel saw Beth, the lady with the glitter in her hair and paint on her clothes, about twenty-four times. After twelve sessions Angel seemed to feel safer in the play room, and she let her guard down a bit. She checked that Beth would help her with what she wanted to do in the sand tray. She found the pack of bear cards and methodically pulled out the ones she wanted. She put the rest of the cards standing up around the edges of the sand tray, just as she wanted them. She asked Beth to help her spell out the words “Dad” and “me” in big writing on cardboard, then she sat down for 10 minutes and concentrated with the scissors. She placed the letters in the sand, and arranged the cards, just as she wanted them. Then she asked Beth to take a picture.



Beth said that after 24 sessions, Angel’s total difficulties score came down from twenty-eight to sixteen. Angel’s language has improved and she has started stringing sentences together. Her dramatic play includes nurturing and caring for others, she laughs, and smiles, and her

aggression has decreased. It seems as though the weight of the adult world is off her shoulders.

Angel's kindergarten teacher recently telephoned Beth and said: "I can't get over it! It's like she's a different girl! She looks me in the eye and says good morning, she's more settled in class and seems more relaxed, which of course means she's free to learn more effectively. She told me that in free time she likes spending time in reading corner. She's really progressing with her schoolwork, and is catching up to the rest of the class. She says yes sometimes now when other children ask her to play, and yesterday she even invited another child to play with her and they made up a game together. They were giggling and running around for most of lunchtime!"

Angel's mother said that at home Angel is sleeping better and doesn't have so many nightmares. She's eating a bit more and even laughs sometimes at the dinner table. She recently received her first invitation to another child's birthday party. Tim is doing well too, he still gets angry, but he isn't getting into so many fights. Angel even seems to have calmed him down a bit too. All in all the family are doing better, Angel especially. She's more like a child now, with brighter eyes, more spring in her step, and a growing circle of friends. She laughs, and learns, and plays.

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PSYCHODRAMA AND DRAMA THERAPY: A COMPARISON

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In therapy, as in all human activity, drama is both inevitable and necessary. It is inevitable because, during the human life-cycle, people are constantly confronted with dramatic changes and it is necessary because all transitions occur as a result of more or less dramatic experiences-in-action. Thus, it is not surprising that drama has been used for centuries both within the theatre and in various healing rituals to reflect on life. Today, drama is the common source of inspiration for both psychodrama and drama therapy. Basing their philosophies on the fact that life itself is dramatic and that the artistic use of drama within the theatre makes much psychological sense, these modern approaches to therapy have made use of techniques such as role playing, impersonation, enactment and improvisation for the purpose of helping people to deal with various aspects of their lives.

However, though psychodrama and drama therapy are based on a common source, they are not identical. Precisely because of their great similarities, they are frequently confused with one another and with similar creative action methods. The purpose of the present paper is to clearly delineate the actual differences between psychodrama and drama therapy. Such a delineation has become increasingly important not only because of the recent growth in scope and in number of practitioners in both approaches, but also because of the simple fact that presumptive employers, academic investigators, students, teachers and clients need to have at least a preliminary idea of the actual discrepancies between psychodrama and drama therapy before they choose one instead of the other. Furthermore, within the present-day atmosphere of psychotherapy integration, commonly agreed upon boundaries of theory and technique would facilitate non-dogmatic discussion among eclectic practitioners around agreed-upon basic concepts regarding the areas or patient populations in which each can contribute to a multidimensional approach to psychotherapy.

After a brief review of history and a discussion of the various definitions, psychodrama and drama therapy will be compared from the point of view of theory, practice, target population and therapist functions, with conclusions summarized in a comparative overview. The comparison is based on a careful review of the literature, extensive personal experience in both approaches and interviews carried out with a small but representative sample of practitioners from both approaches.

History

Psychodrama was founded by Jacob Levy Moreno in the early 1920s as a theatre experiment based on spontaneous improvisations (Blatner & Blatner, 1988; Marineau, 1989). Having observed how professional actors and children who were engaged in role playing exercises felt remarkably revealed by these, Moreno became intrigued by the therapeutic potentials and social implications of a completely spontaneous theatre—one without a written manuscript and without a separation between actors and audience. However, as psychodrama became a more clinical form of group psychotherapy, it slowly turned away from experimental theatre and, though initially used in a general sense to refer to a variety of role playing activities, it

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became a more specific and structured psychotherapy method, with its first professional society founded in 1942.

During the 1960s, around the same time as psychodrama became a more structured form of group psychotherapy, drama therapy rediscovered the therapeutic potentials of improvisational and spontaneous theatre. Focusing again on the aesthetic qualities of drama and on the various influences of Brecht, Stanislavsky, Grotowski and Artaud, the early drama therapists remained within the frameworks of experimental theatre. Many started out by helping hospitalized mental patients, prisoners and students to put on conventional plays that depicted relevant emotional and/or social issues. Recognizing the sometimes subtle but dramatic changes that occurred in the participants as a result of this work, they attempted to apply their techniques to new populations and transfer them to other settings in which the techniques were further modified and expanded to suit various special developmental (Johnson, 1982) and expressive needs. The influences of the British approach to ''remedial drama'' (Jennings, 1973), various forms of creative dramatics (Spolin, 1973; Way, 1969), the human potential movement and educational theatre schools in the United States finally combined to build a more specific drama therapy approach (Emunah, 1994; Jennings, 1987; Landy, 1994a; Petitti, 1992; Schattner & Courtney, 1981), which emerged as a new profession with its own national society in 1979, almost 40 years after its psychodramatic forerunner and counterpart.

Both societies are today members of the same National Coalition of Arts Therapies Associations (NCATA) in the U.S. and there are several common international events in which practitioners from both meet and exchange experiences.

Definition

The term ''psychodrama,'' from the Greek ''psyche'' (soul/spirit) and ''drama'' (action), means presenting the soul in action. Classical, protagonist-centered psychodrama is today seen as

> a method of psychotherapy in which clients are encouraged to continue and complete their actions through dramatization, role playing and dramatic self-presentation. Both verbal and nonverbal communications are utilized. A number of scenes are enacted depicting, for example, memories of specific happenings in the past, unfinished situations, inner dramas, fantasies, dreams, preparations for future risk-taking situations or unrehearsed expressions of mental states in the here and now. These scenes either approximate real-life situations or are externalizations of more or less imaginary inner mental processes. If required, other roles may be taken by group members or by inanimate objects (e.g., the ''empty chair''). Many techniques are employed, such as role reversal, doubling, mirroring, concretizing, maximizing and soliloquy. Usually the phases of warm-up, action, working through, closure and sharing can be identified, with a post-session processing session following (Kellermann, 1992, p. 20)

Although psychodrama is usually practiced in a group setting, its techniques can be used also within individual, family, couple, network or milieu therapy and, with various modifications, as a method for exploring social conflicts (''sociodrama''). As so defined, psychodrama should be clearly differentiated from general role playing, sociometry, group psychotherapy, encounter groups and other related forms of action approaches, including drama therapy.

Drama therapy, or ''dramatherapy'' as it is written in the UK, is more difficult to define in a concise manner, succinctly expressed by McNiff (1986) who noted, ''I know what is not psychodrama, but sometimes I do not know what is drama therapy'' (cited in Petitti, 1992, p. 42). An obvious reason for this difficulty in defining drama therapy is its emphasis on spontaneity, creativity and play which, by necessity, leaves a lot of freedom for experimentation and change. However, it seems that drama therapy lately also has evolved into a more systematic and carefully controlled approach for exploring emotional issues through dramatic action (Emunah, 1994). Vaguely describing drama therapy as an extension of the natural play of children (Langley, 1983), Johnson (1984) stated that the term ''drama therapy'' should be more specifically used for ''those approaches which stress the appreciation of creative theater as a medium for self-expression and playful group interaction and which base their techniques on improvisation and theater exercises'' (p. 105). Most practitioners probably agree that drama therapy refers to the utilization of dramatic methods in group situations, usually for the general purposes of promoting healing intrinsic to theatre art, developing skills of improvisation and cre-

ative thinking, expanding the repertoire of roles with the inclusion of body movement and other aesthetic dimensions. From a technical point of view, drama therapists use a wide range of exercises built on music, movement, sound, mime, physical relaxation, narratives, guided daydreaming, imagery and play. Often, various stage props, such as dolls, masks, costumes, make-up and inanimate objects, are used as imaginary stimulation for dramatization of stories and myths, detailed improvisation of situations or the enactment and exploration of classical (e.g., Greek or Shakespearan) texts. Role playing sessions may become imaginary journeys on themes that are preconstructed or created on the spot by the participants. Much emphasis is put on the ritual realm of healing ceremonies and on various cultural modes of expression. Throughout, drama therapy is process- rather than outcome-oriented, progressing through various stages. But there is generally no final play performed in front of an audience.

A main controversy within and between both psychodrama and drama therapy concerns the delineation of art and of psychotherapy. Though prominent practitioners of both camps firmly state that their approach is a form of art and not a method of psychotherapy, others maintain the opposite view. Bentley's (1977) discussion of the connections (and distinctions) between drama as therapy and drama as entertainment is still highly relevant. This controversy is most outspoken within drama therapy, in which Jennings (1990) and Langley (1983) seemed to pull toward the artistic side of the dichotomy, stating that drama therapy is an ''art form'' (albeit with therapeutic potential). Jennings (1990, p. 9) and Johnson (1984) pulled toward the other side, stating that ''drama therapy, like the other creative arts therapies (art, music and dance), is the application of a creative medium to psychotherapy'' (p. 105). Jennings' (1986) categorization of three different approaches or ''modes'' of drama therapy—creative/expressive, task centered and psychotherapeutic/insight-oriented—is another way of looking at this controversy. Similarly, Landy's (1994b) recent prediction of three possible scenarios for the future of drama therapy—one as a part of theatre, the second as a part of psychology and the third as part of the expressive/creative arts therapists—indicates the ambivalence of drama therapists regarding which club to join. Within psychodrama, Moreno (1972) refused to separate art and therapy from the very beginning, characterizing psychodrama variously as a theology, a political system, a science and/or as a way of life, thus making it impossible for anyone to compartmentalize psychodrama into a specific field.

Without a clear definition of what we mean by the ambiguous terms art and psychotherapy, the above semantic discussion becomes meaningless. Obviously, art does not convey simply aesthetics, and psychotherapy is certainly not just psychological treatment (Szasz, 1974). As the Jungian psychodramatist Barz (1994) pointed out, ''Good therapy must always—among other things—also be good theatre. And good theatre is always archetypical, liberating both the individual and the social components of the person'' (p. 12). Therefore, instead of characterizing psychodrama and drama therapy simply as art and/or as psychotherapy, it would, of course, be more constructive to try to delineate their respective aims, purposes and underlying basic philosophies.

Viewed from this perspective, we have found that there is a fundamental difference between psychodrama and drama therapy. It seems that whereas in psychodrama the ''soul'' (psyche) is the aim and the ''action'' (drama) is the means, the opposite is true for drama therapy in which drama itself (as pure art) is the aim and the psyche is the means (of expression). This is much more than a purely semantic difference; it is a difference in basic philosophy.

Theory

Most psychodramatists refer to the classical formulations of J. L. Moreno (1972) when asked to provide a rationale for their work. ''Psychodrama's scientific roots are buried deep in Moreno's philosophies of spontaneity, creativity, the moment, and theories of role and interaction'' (Yablonsky & Enneis, 1956, p. 149). Moreno's theories on role taking and role playing, spontaneity-creativity, sociometry, social atom, tele and catharsis are clearly indispensable for any understanding of psychodrama. However, though these theories may explain many clinical situations, some practitioners feel that they fail to provide a sufficiently uniform and comprehensive theoretical structure for psychodrama. They prefer, therefore, to justify their practice with the help of theories adapted from psychodynamic, social, behavioral or humanistic psychology. Others feel most comfortable within an integrative framework that tries to join together the best of two or more separate approaches into one broad multimodal conceptual framework. There has lately been a number of important contributions to the theory of psychodrama as clinical role playing (Kip-

per, 1986), strategic family therapy (Williams, 1989) and inspirational technique (Holmes & Karp, 1991), and from the point of view of its therapeutic aspects (Kellermann, 1992), object relations theory (Holmes, 1992) and innovations in theory and practice (Holmes, Karp & Watson, 1994), to mention just a few of the recent books written in English.

Drama therapy presently lacks a systematic, coherent theory of its own and most practitioners seem to use techniques without any firm theoretical basis. Unlike psychodrama, "drama therapy does not refer to a specific theory or technique generated by one person" (Johnson, 1984, p. 107). Moreover, when talking to drama therapists, many seem to be decidedly antitheoretical, having a clear preference for spontaneous action, play and the expression of feelings at the expense of critical questioning and theory building. This attitude is in great contrast to some of the founders of drama therapy (e.g., Sue Jennings) who are prolific writers contributing much to its theoretical development. Apart from the obvious early literature by Artaud, Brecht and Stanislavsky, contemporary handbooks of drama therapy have been written by Chesner (1994), Emunah (1994) and by Grainger (1990) who explained the roots of drama therapy from the point of view of rituals (Scheff, 1979) and personal construct theory (Kelly, 1955). Landy (1994a) explained both the roots of drama therapy and offered the theoretical positions in the field. Landy's (1993) book on the meaning of role in drama therapy and in everyday life stands out as the main recent contribution to this growing knowledge. Although much work remains to be done in the field of theory building, there has lately been an encouraging development of quality textbooks in drama therapy theory (e.g., Chesner, 1995; Gersie, 1995; Mitchell, 1995).

Practice

From a practical perspective, psychodrama and drama therapy may be compared with one another from the point of view of their different employment of (a) imagination and reality, (b) cognitive integration and processing, (c) individual focusing and (d) the use of specific techniques.

First, although both approaches deliberately activate the imagination of participants through the employment of various as-if maneuvers, drama therapy remains largely in this realm whereas psychodrama touches upon both reality and surplus reality during the course of one session. Drama itself is, of course, metaphorical action and, indeed, most material presented in both approaches have symbolic meaning. The use of imagination helps people disclose private parts of themselves that they would not confront directly. Thus, dramatic distancing (Jennings, 1990; Landy, 1983) and "as-if" (Kellermann, 1992) paradoxically give a feeling of safety because it is only a game while at the same time bringing people closer to themselves and revealing unconscious material spontaneously in action. In the words of Emunah (1994), "The scenes in drama therapy are not necessarily directly related to people's real life experiences. Rather, drama therapy utilizes far more improvisation of fictional scenes, capitalizing on the notion that to play and to pretend enables a sense of freedom and permission, and promotes expression and self revelation, albeit obliquely" (p. 18).

Whereas in psychodrama, such a focus on imaginary material, presented in a freely associative manner, is either used in the beginning phase of the classical process or in the separate forms of surrealistic, symbolic, or "dream-reenactment" procedures, in drama therapy it is the actual substance of action. Subsequently, participants in psychodrama are encouraged to reenact a scene from their actual lives that they suddenly remembered as a result of their imaginary experience whereas drama therapists often discourage such identification with a metaphor. A reason for this reluctance to connect imagination with reality was explained in the following manner, "We believe that the metaphor is the treatment itself and we do not think that we can find out what is hiding behind the metaphor in only one session, a process matter to be done by the participants themselves." According to Jennings (1990, p. 20), this difference is due to the fact that psychodrama generally emphasizes personal emotional involvement whereas drama therapy emphasizes dramatical distancing (Landy, 1983)—a polarity resembling the opposing viewpoints of Stanislavsky who emphasized involvement and Brecht who emphasized distance. Though such a polarization may have a certain heuristic value, we feel that it is highly simplified and suggest that any dramatic approach to therapy must include both involvement and distance, both imaginary and real phenomena, and that the aim should be not to choose one instead of the other, but to find a proper balance between them.

Second, though both approaches put a lot of emphasis on emotional experience, psychodramatists seem to encourage much more cognitive integration than drama therapists do. This may be done, for ex-

ample, through action-insight, verbalization, processing and direct or indirect *analysis* of the material expressed. Contrarywise, some drama therapists minimize cognitive reflection as much as possible. When asked about this sometimes complete absence of emphasis on understanding, one drama therapist simply stated that ''there is no need for it! All required understanding comes from the dramatization itself.'' Although this might not be standard practice for all drama therapists (cf., Landy's [1993] phase of ''reflection upon the role play, integrating roles to create a functional life system, and social modeling''), it seems that whereas in drama therapy the expression has value in itself, psychodrama emphasizes the importance of connecting experiences with awareness. Perhaps this was exaggerated in Chesner's (1994) observation that ''dramatherapy may take place under cover of relative darkness while psychodrama tends to shine a torch of conscious awareness into the dark recesses of the psyche'' (p. 129).

Third, though both approaches deal with issues that concern the entire group, individual issues are less emphasized in drama therapy than in psychodrama. Individual issues are pursued in psychodrama through the choice and re-enactments of one *protagonist,* the central figure who is usually absent in drama therapy. In drama therapy, all group members associate with the presented issue and transform it into a common story, play or myth that they can all participate in.

Drama therapy techniques are viewed by psychodramatists as initiations to sessions, as stimulating warm-ups for the entire group to catalyze creative processes (Blatner & Blatner, 1988; Chesner, 1994). ''The graded series of exercises extends the 'warm-up' through many sessions and at the same time strengthens protagonists' egos, which help prepare them for deeper, more insight-oriented work (i.e., psychodrama)'' (Blatner in Emunah, 1994, p. vii). Indeed, drama therapy exercises have been included in the vast collection of warm-up exercises that contemporary psychodramatists use in their groups (most notably Blatner, Sternberg, Garcia, Fox & Leveton, according to Emunah, 1994, p. 19). If an entire session deals with the exploration of one of these exercises, without focusing on an individual protagonist, it is called ''group-centered'' or ''theme-centered'' psychodrama, sometimes developed into a sociodrama (Sternberg & Garcia, 1989), the exploration of common social roles and conflicts. Drama therapy remains in this universal realm for the entire session, searching for archetypal experiences. For example, when Jennings (1990) brought a Greek play, such as *Antigone,* to her drama therapy group for exploration, she intentionally did not focus on any individual real-life situation, but on the universal father-daughter and sister relations in a distanced scenario and the participants agreed not to interpret anything in relation to their own families. This would, of course, be very unlikely within classical, protagonist-centered psychodrama.

Another example is their different uses of masks. In drama therapy, masks are usually explored from a more aesthetic and non-psychological perspective; participants learn to prepare them, try them out, play with them in various roles and finally talk about how they felt doing all this. In psychodrama, participants may start out doing the same, but, at one point, or another, the psychodramatist will start asking highly personal, individual questions of the participants about their masks and the ''persona'' they put on in their daily lives. They may ask, ''Who are *you* behind this mask?'' And, ''Who is behind that one? What is the most private part that you cannot share with anyone? Who are you most ashamed of? Why? Can you tell him or her? Why not? What would happen if you did? Would you like to try? Let's do it now! Show us! . . . '' And later, ''Would you consider taking off your mask—have a different one?'' Some psychodramatists would also take the opportunity to explore some of the interpersonal aspects of such a group-masquerade, asking the participants to explore for whom they put on their masks, how they want other people to see them and how they actually feel toward one another, leading the group into a sociometric exploration. Thus, while in psychodrama questions are often asked in a direct, confrontative but hopefully sensitive manner, drama therapy lets the participants deal with the same issues in a more subtle and indirect manner, leaving much of the actual individual processing to the participants themselves.

Fourth, psychodrama and drama therapy use specific dramatic techniques very differently. There is not only a general difference in the use of such instruments as scene setting, putting actors in role, enactment and sharing, but also in the therapeutic employments of role reversal, doubling, soliloquy, mirroring and concretization. Although in psychodrama, these techniques are used to advance some kind of intra- and/or interpersonal working through of issues and problems raised during the session through catharsis, action-insight, interpersonal or behavioral

learning, drama therapy is much less focused and structured in the uses of such specific techniques, generally emphasizing expression in itself as the main medium.

A suitable illustration of this difference is the technique of the "empty chair," which is used frequently in both approaches. In drama therapy, a mother who reveals that she has difficulties with her adolescent son may be asked to imagine that her son is sitting in front of her and that she is talking to him. She may express and reveal whatever pent-up feelings she has kept in toward her son when talking to the empty chair and that would complete the work of the drama therapist. A psychodramatist would probably continue the session, putting an auxiliary (representing the absent person) on the empty chair, suggest role reversal and doubling to work through and resolve the often complicated relations between adolescents and their parents, thus hoping to clarify and untangle some of the inner representations and actual perceptions one has of the other.

Target Population

One possible consequence of the above differences is that drama therapy and psychodrama may be suitable for different target populations. Some practitioners from both camps claim that their method is the treatment of choice for all mental disorders whereas others state that their method can be helpful only for specific populations, most of which cannot even be labeled with psychiatric diagnoses. As empirical outcome research has been consistently neglected in both psychodrama and drama therapy, there is yet no conclusive evidence behind any of the above claims. Because of limited scope and reliability, the more than 200 empirical research reports that have been published on actional role play methods (Schramski & Feldman, 1984) are insufficient to objectively substantiate their therapeutic effects.

It is our experience that drama therapy and psychodrama can be suitable only for people who are able to enter into the exhausting psychic rituals of a dramatic setting. The ability, for example, to participate in the imaginary process of role playing without losing touch with outer reality seems to be a minimal requirement in both approaches. For example, people who are too mentally rigid, introverted and unspontaneous, usually will have great difficulties in such groups. This may be somewhat surprising as they are the very people who would have most to gain from drama and who are often referred to nonverbal approaches because of their difficulties to make progress in verbal therapy.

Both psychodrama and drama therapy have shown potential applications in certain client populations and within various settings, either by themselves or as adjuncts to the more traditional approaches to therapy. Though it would be impossible to mention all settings where these approaches could be applied, the most common are probably psychiatric hospitals (Emunah, 1983; Polansky & Harkins, 1969), outpatient clinics, prisons, schools, universities, old age homes and in personnel management. Drama therapists have recently documented their work with clinical studies of a great variety of patients, including acute or chronic inpatients, various groups of outpatients, children and adolescents, addicts, the eating disordered, post traumatic stress disordered, personality disordered and survivors of sexual abuse (Gersie, 1995; Jennings, 1995; Mitchell, 1995; Winn, 1994).

Clearly, most of the main target populations are similar, but some groups seem to be more suitable to one approach than the other. For example, drama therapy may be the treatment of choice for certain disorders first evident in infancy, childhood and adolescence, including some developmental disorders, mental retardation, autism and conduct disorders in which communication is more nonverbal. Drama therapy also seems suitable for those with learning disabilities (Chesner, 1995) and with physically handicapped people (Irwin, 1979) within a rehabilitation and occupational therapy framework. With some of these populations, drama therapy can be more flexibly adjusted than psychodrama to suit various levels of communication and awareness with the possible use of simple drama exercises such as movement and play. On the other hand, psychodrama is probably indicated for alcoholics and drug addicts who need a more direct and confrontational approach to psychotherapy, apart from the expressive focus.

Paradoxically, psychodrama may be viewed as more suitable for people who are both more healthy and more ill than participants in drama therapy. From the point of view of psychopathology, protagonists may be more severely ill in various psychiatric disorders, but more healthy in certain mental functions including ego strength and ordinary sensory perception. For example, the use of "representational" role

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reversal (Kellermann, 1994), doubling and mirroring is impossible with a group of chronic psychiatric inpatients and mentally retarded children. The ability to participate in psychodrama is not only dependent on a certain degree of intellectual, imaginary, emotional and interpersonal functioning, but also on role taking and role playing skills, which are insufficiently developed in many persons. Furthermore, protagonists must be able to experience surges of feelings without a loss of impulse control, have at least some capacity to establish interpersonal relations, have a minimal tolerance for anxiety and frustration, some psychological-mindedness and a capacity for adaptive regression in the service of the ego (Kellermann, 1992, p. 23). In the final analysis, psychodrama seems to be especially suitable for some of the conditions that are not normally attributable to a mental disorder, but that are nevertheless a focus of treatment, to speak in the language of DSM-IV. Such conditions would include various relational problems within and outside the family, phase of life circumstances and uncomplicated bereavements that may be the results of developmental, traumatic or transitional crises rather than a developmental deficiency.

Most non-clinical psychodramatists and drama therapists shun diagnoses. Psychiatric disorders, they say, are a product of social forces that operate upon people in a self-fulfilling manner, and people who are labeled and treated as if they were disturbed, increasingly become more disturbed and later permanently adopt the role of mentally ill. Such practitioners do not have a conception of health, normality or pathology; diagnosis is therefore irrelevant and unnecessary. Their kind of work is not ''therapy'' in the medical sense of the word, but an emotional experience within the framework of developmental play. This experience may or may not make people more balanced, more happy, less neurotic or more aware of themselves. In any case, the goal is not to produce a ''cure,'' but simply to become as creative, spontaneous and expressive as possible within the boundaries of each individual's personal limitations. By definition, the discrepancy between this activity (whether we call it drama therapy or psychodrama) and play in general is almost nonexistent.

The goals of clinical psychodramatists and drama therapists are generally more specific. Participants in these approaches want to get rid of symptoms, handle difficult situations better, get through their mourning, let out pent-up anger, remember and work through forgotten traumatic experiences from the past and/or gain in personal self-esteem. Discussion and disagreement between clinical and non-clinical uses of drama prevail across camps.

Therapist Functions

Though some of the roles and functions of psychodramatists and drama therapists overlap, others are slightly different or incompatible. Both approaches demand extensive personal and professional experience and usually attract people with great extraversion, spontaneous enthusiasm and histrionic inventiveness. Clearly, anyone working within a dramatic approach must have sufficient flexibility to permit rapid changes of mode to meet variable individual and group needs on the spur of the moment.

Psychodramatists fulfil four interrelated and highly complex tasks. First, as analysts, they are responsible for making themselves fully aware of the protagonist's condition. This includes understanding both personal and interpersonal phenomena in order to attribute meaning to emotional experiences. Second, as producers, psychodramatists are theatre directors translating the material presented into action that is emotionally stimulating and aesthetically pleasant. Third, as therapists, they are agents of change who influence their protagonists in ways that facilitate healing. Fourth, as group leaders, they foster a constructive work group climate that facilitates the development of a supportive social network. The overlapping and interlacing of these various roles form the basis of the psychodramatist's professional identity (Kellermann, 1992).

Drama therapists function mostly as theatre producers, including the roles of dramaturg, artist, leaders of ritual and teachers of drama. Many practitioners bring with them unique experiences from the fields of art, acting, occupational and expressive therapy, social work, anthropology, nursing, special education, psychology and creative drama to put a very individualized touch to their (varied) drama therapist role-perception. They are usually familiarized with artistic media of expression and put a lot of emphasis on aesthetic qualities. Emunah (1989) observed that ''the fact that drama therapy students enter the program with a strong background in theatre further contributes to the high aesthetic level of the scenes'' (p. 30).

Both psychodramatists and drama therapists refuse to be lumped together with others of their kind. Many

of them are essentially individualists, non-joiners and charismatic figures with a personalized style of their own. But their kinship with other practitioners is real enough. Like psychotherapists, they try to understand and help people who suffer from emotional distress and, like dramatists, they share a fascination with action and have developed an aesthetic, romantic and sometimes escapist approach to life and nature.

Concluding Comparison

Jenning's (1973) comparison of psychodrama and drama therapy along two continuous lines depicting both of them in terms of more or less therapeutic depth and symbolic distance (Davies, 1975) seems still to be largely valid. Putting both approaches on the same continuum, we would add that drama therapy, as it is practiced today, is oriented specifically toward creative-expressive learning of roles whereas psychodrama is oriented more toward experiential learning, including specific working through of emotional, cognitive, interpersonal, behavioral and nonspecific issues. Some of the other differences are presented in the comparative overview in Table 1.

It seems to us that because of the explicit focus on distancing and the frequent use of metaphors, drama therapy stays more on the surface of material (which does not necessarily mean that it is more ''superficial'') and makes it ''safer'' than the psychodramatic approach of deep penetration of the soul. As a result, both cuisines are more easily digested by different diners. Blatner and Blatner (1988) correctly pointed out that ''in some settings the ''psycho-'' or the ''-drama'' have unpleasant or misleading connotations (p. 7), and ''drama therapy complements psychodrama for those who are not ready to directly address the emotionally loaded issues in their real lives'' (Blatner in Emunah, 1994, p. vii). Thus, although drama therapy may be perceived as more stimulating, entertaining and ''fun'' in some educational settings, psychodrama is not so easily accepted in such settings because of the personal self-disclosure required.

Table 1
Comparative Overview of Psychodrama and Drama Therapy

	Psychodrama	Drama Therapy
Definition	group psychotherapy psyche (aim) drama (means)	expressive art therapy drama (aim) psyche (means)
Theory	J. L. Moreno and others spontaneity-creativity role; sociometry social psychology object relation theory behavioral learning	no one ''founder'' theatre theory anthropology and ritual role and play theory Expression Jungian psychology
Aims	therapeutic self-awareness involvement	aesthetic expression distance
Therapeutic factors	catharsis tele action-insight as-if magic	play improvisation distancing rituals group work
Practice	clear structure imagination and reality cognitive integration focus on individual specific techniques	unclear structure imagination, myth no processing focus on group no specific techniques
Target population	conflicts life crises psychological-minded	developmental deficiencies handicapped, retarded
Therapist functions	analyst, producer therapist, group leader	dramaturg, teacher artist, shaman

Discussion

Naturally, any comparison of methods that are continually changing is a difficult task. Being based on spontaneity-creativity, psychodrama and drama therapy defy clear boundaries and operational definitions. As a result, any comparison quickly becomes erratic and/or obsolete, as eloquently pointed out by Jennings (1990) who said that ''no dramatherapy treatise can be definitive for more than a blink of a gazelle's eye'' (p. 26). This is, of course, true also of the present work. The fact that practitioners who call themselves psychodramatists or drama therapists cannot accept others who claim the same titles, does not make the situation easier. Clearly, as both approaches are practiced differently in various places around the world, a comparison such as the present one is at best a plausible view of the moment.

One characteristic indication of this ambiguity is that drama therapists view psychodrama as a part of drama therapy whereas the opposite is true for psychodramatists. As many drama therapists use psychodramatic techniques as a follow-up when indicated and many psychodramatists use drama therapy techniques as a warm-up, the question of which approach is a part of the other becomes meaningless. Moreover,

"group-centered" psychodrama, an approach that is more or less similar to drama therapy, has been used for years with people who are unsuitable or unmotivated to participate in protagonist-centered psychodrama. Finally, there are few drama therapy methods that were not experimentally used within the Moreno Institute under a variety of such names as "bibliodrama," "axiodrama," or spontaneity training. Prominent drama therapists, such as Landy (1994a), recognize that psychodrama provided drama therapists with both a theoretical source and a series of techniques. "That psychodrama has been a fundamental part of the work of most drama therapists goes without saying" (Emunah, 1994, p. 19). Thus, both methods may be viewed as different branches of one and the same tree; both developed from the works of J. L. Moreno, "the grandfather of all action therapies" (Johnson, 1991, p. 1).

Instead of discussing which approach is a part of the other, it is more important to highlight some of the characteristic frustrations that some practitioners feel regarding the shortcomings of their own approach. For example, some drama therapists feel that by staying only within the symbolic realm, though momentarily exciting, will keep participants from "anchoring" their experiences in actual (outer or inner) reality. On the other hand, some psychodramatists feel that by using specific cognitive distance techniques (such as mirroring) too frequently, though momentarily thought-provoking, will keep participants from acting spontaneously, improvising freely and exploring unknown territory. Thus, it appears that each approach may have something to offer the other in terms of complementarity, as Blatner suggested in his Foreword to Emunah (1994). It is our view, however, that practitioners working within such an integrative model of "psycho-drama-therapy" should be able to clearly differentiate between one and the other from the point of view of theory, practice, therapist functions and therapeutic factors and to be able to specify what works best for whom within what setting. It is our hope that the present work will make this job of differentiation a little bit easier.

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Introduction to Psychodrama

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Workshop leaders: Chip Chimera and Clark Baim

WHAT IS PSYCHODRAMA?

Developed by Psychiatrist Dr. J. L. Moreno (1889-1974) from the 1920s onwards, psychodrama was the first recognised method of group psychotherapy and is practised in more than 100 countries. Psychodrama is fully accredited as a psychotherapy by the United Kingdom Council for Psychotherapy (UKCP). It has an extensive literature of more than 6,000 publications, plus many national and regional journals and associations around the world and has contributed ideas and techniques used in many other forms of psychotherapy. (See www.pdbib.org for a comprehensive international bibliography of psychodrama).

Psychodrama is a holistic, strengths-based method of psychotherapy in which people are helped to enact and explore situations from their own life - past, present and future. The scenes enacted may be based on specific events in a person's life, their current or past relationships, unresolved situations, desired roles or inner thoughts and conflicts. The method is typically used in group settings, with group members taking on the various roles in the drama as needed. Witnessing and participating in each others' personal stories can generate feelings of deep understanding and trust amongst group members.

In its so-called 'classical' form, a psychodrama will often begin with a current problem or difficulty and trace it back to earlier life situations. Here, the participant may have the chance to experience what was missing but needed at that time. The enactment then returns to the present, where new learning can be integrated and put into practice.

Each psychodrama addresses the concerns of the person who is in focus. The range of issues may be wide. The person who shares their work is chosen sociometrically by the group, highlighting the group concern. Hence all members of the group also share in the work in a personal way.

Psychodrama can, for example, help people to better understand themselves and their history, resolve loss and trauma, overcome fears, improve their intimate and social relationships, express and integrate blocked thoughts and emotions, practise new skills or prepare for the future.

Psychodrama allows for the safe expression of strong feelings and, for those who need it, the practice of containing emotions. As participants move from 'talking about' into action, opportunities arise to heal the past, clarify the present and imagine the future. Psychodrama can offer a wider perspective on individual and social problems and an opportunity to try out new behaviours.

Each psychodrama includes:

a. ***The protagonist:*** The person whose story or issue is presented through guided dramatic action.
b. ***The auxiliary egos:*** Group members who assume the roles of significant others in the drama. This may include significant people, objects or even aspects of the self or a person's internal world, e.g. 'my optimistic self' or 'my internal critic.'
c. ***The audience:*** Group members who witness the drama and who may become involved in auxiliary roles.
d. ***The stage:*** The physical space in which the drama is conducted.
e. ***The director:*** The trained psychodramatist who guides participants through each phase of the session.

Some of the core techniques in psychodrama include role reversal, role taking and role play, the double, the mirror technique, surplus reality, the empty chair, scene-setting and enactment.

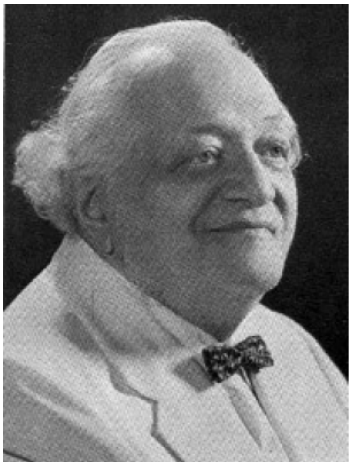
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SOME HISTORICAL NOTES ABOUT PSYCHODRAMA

The method of Psychodrama was created by Psychiatrist Dr. **Jacob Levy Moreno** (1889-1974). Psychodrama developed from Moreno's interest in philosophy, mysticism and theatre, and his observations of group interactions and relationships. Moreno was among the first to recognise the healing power of a group, including the importance of self-help groups, where each person becomes the healing agent of the other without any special training or knowledge other than his or her own experience.



Jacob Levy Moreno

About 1912, while walking the streets of Vienna, Moreno noticed the arrests of prostitutes in the city's red light district. A young medical student, he organised an informal group where the women could talk about their problems. They gradually moved from talking about their problems with the police to more personal issues, providing emotional support to each other. This experience contributed to Moreno's interest in working with groups rather than individuals, as was the style set by Sigmund Freud, an older contemporary. Moreno later coined the phrase "group psychotherapy" to recognise this particular form of treatment.

In 1925, Moreno left Europe and emigrated to the United States. He brought to the USA his interest in working with children, spontaneous theatre and psychotherapy treatment with the use of theatre principles. He also was very interested in the study of relationships in groups, dating back to his work at a First World War refugee camp in Mitterndorf. He continued to refine his sociometric studies at the Hudson State Training School for Girls, Hudson, N.Y. He devised methods of matching delinquent girls according to living and work task preferences, which increased cooperation among the girls and decreased the number of runaways.

Moreno established Beacon Hospital, an alternative hospital with a therapeutic theatre, in the late 1930s, and shortly after met **Zerka Toeman.** She first became his assistant, then student, then trained auxiliary and finally his wife and collaborator, writing with him "Psychodrama Volume II" and "Psychodrama Volume III" as well as numerous articles. She travelled with him throughout the world to demonstrate and teach psychodrama and sociometry and helped write "The First Psychodramatic Family" to detail use of psychodrama in the family setting. In 1942, he founded the American Society of Group Psychotherapy and Psychodrama, the first group therapy organisation, which continues today.



Zerka Toeman Moreno

J.L. Moreno died in 1974, and group psychotherapy, psychodrama and sociometry have continued to grow and expand as he predicted. Those interested in J.L. Moreno's life may seek out his psychodrama volumes and other works. Two excellent biographies include "Jacob Levy Moreno, 1889-1974" by Rene Marineau and "J.L. Moreno" by Paul Hare and June Hare.

After his death, Zerka continued to travel throughout the world to teach and demonstrate psychodrama. She has been especially noted for bringing psychodrama to interested clinicians and educators in Asia, including China, Taiwan and Japan. Zerka, now in her 90's, continues to write and conduct training sessions from her home in Virginia. Her collected writings have recently been edited by Horvatin et al, listed in the bibliography.

(Historical notes excerpted, with permission, from unpublished work by Karen Carnabucci)

WHO CAN BENEFIT FROM PSYCHODRAMA?

Because psychodrama is highly versatile and places equal emphasis on thoughts, emotion, body and action, it can be useful in helping people with difficulties in relationships, self-management and social and emotional functioning. Psychodrama is also particularly useful for helping people experiencing unresolved loss, trauma or mood disorders such as anxiety or depression. Psychodrama has also been used to good effect with people who have been diagnosed with personality disorders and a wide range of other mental health conditions, including psychosis.

WHERE IS PSYCHODRAMA USED?

Psychodrama and its related methods - including sociodrama, sociometry and role play - are applied in diverse settings such as mental health, trauma and abuse recovery, counselling, medical training, residential children's homes, social work, schools and universities, prisons and probation, youth and community work, addictions programmes, public and voluntary sector agencies, refugee centres, retirement homes, personal development workshops, relationship and marital counselling, community-building, professional training and development, human resources, and business and industry. Psychodrama is often used in combination with other therapies such as cognitive-behavioural therapy, creative arts therapies and group analysis, depending on the training of the therapist and the nature of the problem.

FUNDAMENTAL TECHNIQUES

The Double

Doubling occurs when a member of the group takes on the physical stance of the protagonist and attempts to enter their internal world by speaking their inner thoughts and feelings. Developmentally, the Double is meant to capture the essence of attuned interactions between a parent / caregiver and an infant.

Functions of the Double

- To support the protagonist.
- To deepen the protagonist's feeling.
- To notice the non-verbal clues from the protagonist.
- To mirror back/reflect to the protagonist.
- To emphasise feelings, etc.
- To interpret thoughts, feelings and behaviour.
- To broaden the protagonist's understanding and awareness of self and others.
- To guide the protagonist in their work.

Other benefits of doubling:

- An empathy building technique.
- A way of gently challenging, e.g. through paradoxical doubling.
- It is one of a number of techniques that keep the group members active and involved in protagonist-centred work.

The Mirror Technique

Adaline Starr (1977, p. 178) describes the mirror technique thus:

"the moment when [the child] sees his/her image in the mirror and discovers that the image is of him/herself….he/she may touch it, kiss it, or hit it. This action, immortalised in the myth of Narcissus, is translated into an action technique in which the looking-glass reflects the self as seen by another."

The mirror technique can be used in many ways:

1. **To distance the protagonist from the emotions -** this is particularly important if the scene is traumatic or if the protagonist is often overwhelmed by negative feelings. The person is helped to feel that they have control over the action in the way they were unable to have before.

2. **To connect the protagonist to emotions -** it may be that the person has split off their feelings. Connected to this, it can also enable them to recognise the locus of control is with them and not with others.

3. **Observation of behaviour/response in a situation -** it may be that the protagonist has little or no awareness or insight into how they may have reacted in a situation.

4. **Role training -** the protagonist can see their reactions in a given situation, and then observe how others may respond. It is always important that the protagonist reflects on the input of others and then re-does the scene themselves.

5. **To observe the dynamics within a system -** this allows the protagonist to observe not only their own responses, but also the others within the scene. They are then more able to gain some understanding of what the different dynamics are within the system.

It is very important that this technique is used carefully, as in certain circumstances it may be experienced as critical or humiliating. The facilitator must also encourage the protagonist to be 'as if' they are an observing ego (i.e. an external assessor) when commenting on what they see. This helps to prevent the protagonist from becoming very self-critical and/or self-effacing.

Role Reversal

Zerka Moreno describes role reversal as the 'engine that drives the psychodrama'. In Psychodrama Volume III (Moreno and Moreno 1969/1975) she lists some 'rules' of psychodrama. Rule 14 (p. 238) states:

"The protagonist must learn to take the role of all those with whom he is meaningfully related, to experience those persons in his social atom, their relationship to him and to one another."

She goes on to say: "Taking this a step further still, the [person] must learn to 'become' in psychodrama that which he sees, feels, hears, smells, dreams, loves, hates, fears, rejects, is rejected by, is attracted to, is wanted by, wants to avoid, wants to become, fears to become, fears not to become, etc.

The person has 'taken unto himself' with greater or lesser success, those persons, situations, experiences and perceptions from which he is now suffering. In order to overcome the distortions and manifestations of imbalance, he has to re-integrate them on a new level. Role reversal is one of the methods par excellence in achieving this, so that he/she can re-integrate, redigest and grow beyond those experiences which are of negative impact, free himself and become more spontaneous along positive lines."

There are two kinds of role reversal:

- **Reciprocal -** between self and another person.
- **Representational -** between roles/objects/ parts of the self.

Functions of Role Reversal

To gain information: Once the auxiliary has been roled in, the facilitator and group can gain information concerning the significant other's perception and view of the protagonist. This may be distorted due to the protagonist's projection/transference onto this person, and so help the director in understanding the dynamics of the relationship. Often, the protagonist is more able to speak from another person's/object's role than from their own.

To understand the role of the other: This is important, particularly if there seems to be some distortion of the role in terms of what may be projected onto the other by the protagonist. The director needs to challenge in an appropriate way, and enable the protagonist to go fully into the role of the other to gain a deeper and more empathic view of the person. Various techniques may be used to enable this process to take place - the honesty chair; the internal voice, the insight chair etc.

To heighten spontaneity: When the protagonist's energy seems to drop, it may be useful to have them reverse roles with the person, object, feeling etc, that has held the most energy. It may be needed if the auxiliary is not portraying the role correctly, causing the protagonist's energy to drop. It can also be used to increase the intensity of the interaction.

To see themselves/increase empathy: It can be very powerful for a protagonist to experience him or herself from the other person's role. This in itself can correct a distorted view of the other person and the relationship between. It also develops empathy for the other person, and this is particularly important when working with people who have patterns of abusive behaviour.

OTHER TECHNIQUES

There are scores of psychodramatic techniques; these are some of the more commonly used techniques. A more comprehensive list can be found in *Foundations of Psychodrama* by Adam Blatner.

Aside During an interaction, the protagonist makes a comment to the audience that the other people in the scene 'cannot hear.' This allows hidden thoughts and feelings to be expressed when they cannot be said to the other people.

Auxiliary The supporting people in an enactment. Usually, an auxiliary will have a particular role, and this may include the role of *double.*

The Empty Chair A chair is used to represent another person, or an abstract concept, e.g. 'my past', 'my critical self' or 'my smoking habit.' This is commonly used in one-to-one work. Using an empty chair to represent another person can make it more tolerable for the protagonist. It is also useful when doing *monodrama.*

Future projection A future scene is examined and / or enacted. This may be for the purposes of rehearsal, confronting fears, practising a hoped-for outcome, testing expectations, etc.

Monodrama When the protagonist plays all of the roles in the psychodrama, and is the only person 'on stage.' A common technique in one-to-one working.

Role training Role training is typically used to practise new skills and responses. It is particularly useful as a method for helping the protagonist to expand their role repertoire – the range of skills and responses they can call on to meet life's challenges.

Scene setting / Enactment The creation and enactment of scenes from the protagonist's life or inner world, usually with auxiliaries. The realism of the enactment promotes maximum opportunity for warming up; for expression of actions, thoughts and feelings; assessment of the protagonist's response; working through issues; and developing and integrating new responses.

Sharing The phase of the group following an enactment, in which group participants share with the protagonist their own personal connections with the psychodrama.

Soliloquy The protagonist shares with the audience their inner thoughts and feelings. Sometimes this is done in dialogue with a *double.* Soliloquy is similar to the use of character voiceovers in film.

Spontaneity The quality of one's response to a situation. With sufficient spontaneity, we can provide an adequate response to new situations and fresh responses to old situations.

Surplus reality A key concept in Psychodrama. Surplus reality recognises that subjective experience is an important part of actual experience. Examples of surplus reality scenes include: A 'goodbye scene'; An encounter with a future imagined partner / child / friend; A conversation that in reality 'could never happen'; A conversation with someone who is no longer living; An encounter with a spiritual entity such as Buddha, God, etc; A scene where one is rescued and protected from a childhood abuser; A scene from the future looking back over one's life; A forgiveness scene, or a scene in which one can make reparations to others for one's own wrongdoing.

Warm up The early phase of a group or session. This can include a wide range of techniques and exercises aimed at creating group cohesion, a safe and containing atmosphere, and a focus for the session.

RELEVANCE OF PSYCHODRAMA TO ATTACHMENT THEORY AND THE DYNAMIC-MATURATIONAL MODEL

Psychodrama is a just as much a method for encouraging emotional containment as it is a method for emotional expression. It is often misinterpreted as a method that focuses on emotional catharsis – which may be very useful for people with an insecure 'A' strategy - neglecting the very important function of emotional containment – which may be useful for people with an insecure 'C' strategy. Crittenden (2005) and others have supported the notion of 'purposeful eclecticism' for psychotherapy; that is, the purposeful adaptation and combination of psychotherapeutic interventions to best meet the needs and learning style of each client. With this in mind, we can make a number of observations about the purposeful application of psychodrama techniques for people with 'A' and 'C' strategies of self-protection.

Therapeutic implications for 'A'

When implementing psychodrama-based interventions with clients who have a prominent 'A' strategy for self-protection, the psychodramatist would emphasise:

- Building trust to overcome suspicion.
- Contracting to 'stay in it for the long haul,' as therapy may be prolonged.
- Being aware of the client's tendency to want a 'quick fix'.
- Encouraging 'I' statements, i.e. to own feelings. The technique of **Doubling** may be very useful here.
- Expression of true negative affect, e.g. fear, sadness, disgust, shame, guilt, anger.
- Accurate distribution of responsibility, especially in relation to experiences of abuse from attachment figures.
- Honouring the story whilst eliciting more balanced stories, including painful and difficult emotions.
- Not 'attacking' the idealised attachment figure, but instead allowing gradual reappraisal of episodes and relationships.
- Unpicking assumptions and cognitive errors / 'rules'.
- Developing intimacy skills such as communication, talking about emotions, asking for care / comfort.
- Developing understanding and skills of reciprocity in relationships.
- Identifying strengths and building self esteem.
- Encouraging client to appraise self from own rather than parents' perspective.
- Giving permission for 'shadow' side to be given a voice and developed, e.g. 'forbidden' or 'unacceptable' emotions or roles.

Therapeutic implications for 'C'

When implementing psychodrama-based interventions with clients who have a prominent 'C' strategy for self-protection, the psychodramatist would emphasise:

- Creating a coherent story from uncontained emotion and unstructured narrative.
- Separating one's own feeling from those of other people.
- Making and accurate link between one's affect and the event provoking the response. This is to address fundamental questions such as 'Why do I feel like this?' and 'Do I still want to respond this way to an event that happened years ago?'
- Creating structures and clear boundaries.
- Developing skills of accurate perspective taking. **Role reversal** and the **Mirror** technique may be very useful here.
- Revisiting avoided areas of life or 'unspeakable' emotions. The technique of **Doubling** may be useful here.
- Providing an accurate and full factual account of episodes from the past, including accurate temporal order, cause and effect, person, place, time, who did what in what sequence, who was responsible for what, etc.
- Completed and clear episodes which provide an antidote to the C strategy.
- Accurate distribution of responsibility, including self-responsibility where it may be all too easily avoided with the C strategy.
- Externalisation: Using objects, chairs, writing, drawing and any of a variety of methods to concretise and 'make real' what can often seem chaotic and overwhelming when the C strategy is functioning.

For both A and C

- Role training and repetition/rehearsal of new strategies helps to anchor them. The DMM shows the importance of repetition in neurobiological growth. Role training can help participants move towards B strategies in appropriate situations.
- Sharing helps with integration and reflective functioning.
- Careful summarising to help increase the likelihood that new learning is understood and taken in.
- Identifying exceptions, for example when mother was caring / uncaring.
- Teaching problem-solving skills.
- Developing a balanced view of people and events. For example, most people are not 'all bad' or 'all good.'

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- Birmingham Institute for Psychodrama: www.birminghampsychodrama.co.uk
- Comprehensive international bibliography of psychodrama: www.pdbib.org
- Dr. Adam Blatner's website (a very valuable and extensive website with many further links and reading on the subject of psychodrama) www.blatner.com/adam/

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Integrating Yoga with Psychotherapy: A Complementary Treatment for Anxiety and Depression
Intégrer le yoga avec la psychotherapie : Un traitement complémentaire pour l'anxiété et la dépression

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ABSTRACT

This article addresses the empirical research on yoga as an effective, complementary, clinical intervention for anxiety and depression based on an examination of studies published from 2003 to 2010. There is a discussion of study findings and research limitations and suggestions for researchers and future research. Yoga appears to be an effective clinical intervention for anxiety and depression. However, more rigorous research needs to be completed to further support using yoga in mental health treatment planning. This article concludes with practical suggestions and implications for mental health professionals interested in using yoga.

RÉSUMÉ

Cet article traite la recherche empirique sur le yoga comme intervention clinique complémentaire effective pour l'anxiété et la dépression fondée sur une enquête des études publiées de 2003 à 2010. Une discussion des résultats des études et des limites de la recherche ainsi que des suggestions pour les chercheurs et les recherches futures sont présentées. Le yoga semble être une intervention clinique effective pour l'anxiété et la dépression. Cependant, la recherche plus rigoureuse doit être effectuée afin de soutenir l'usage du yoga dans la planification du traitement de la santé mentale. Cet article termine avec des suggestions pratiques et des implications de l'utilisation du yoga pour les professionnels de la santé mentale.

Approximately 50% of American adults suffer from a mental health disorder at some point in their lives, the most prevalent being depressive and anxiety disorders (Kessler et al., 2005). Intriguingly, people with mental health disorders, such as depression, often engage in self-help treatments before seeking treatment from a medical or mental health professional (Oliphant, 2009). Conceivably, they may choose to engage in self-help treatments in an effort to maximize financial savings, to refrain from consuming synthetic medications, or avoid negative stigmatization. People may also desire a more holistic treatment than allopathic care or psychotherapy can offer; perhaps this is why the use of complementary and alternative medicine (CAM) has surged. CAM is perceived as more wholesome, with fewer negative side effects than conventional treatments (Oliphant, 2009).

Thus, individuals are using CAM to manage chronic pain, to improve health or prevent negative health issues (Faass, 2006), and to treat mental health issues (Barnes, Bloom, & Nahin, 2008).

CAM is "a group of diverse medical and health care systems, practices, and products that are not generally considered part of conventional medicine" (National Center for Complementary and Alternative Medicine [NCCAM], 2010, para. 2). Approximately two out of every five adults in the United States has utilized CAM (Barnes et al., 2008). Specifically, anxiety and depression are two of the mental health issues most commonly treated with CAM (Barnes et al., 2008; Pilkington, Kirkwood, Rampes, & Richardson, 2005). In fact, those with anxiety, depression, or a comorbidity of these disorders are twice as likely to use complementary and alternative treatments when compared to others with mental health disorders and four times as likely when compared to those without any mental health disorders (Wahlstrom et al., 2008).

One of the five domain categories of CAM treatments is mind-body interventions, defined as "interactions among the brain, mind, body, and behavior, with the intent to use the mind to affect physical functioning and promote health" (NCCAM, 2010, para. 9). The use of CAM mind-body interventions has increased, with yoga ranking as a commonly used CAM mind-body intervention (Barnes et al., 2008). Saper, Eisenberg, Davis, Culpepper, and Phillips (2004) found that yoga is commonly used to treat anxiety and depression. Perhaps people are attracted to yoga because they desire to integrate the body and the mind in healing. Many mental health professionals agree that the body and mind are not separate, especially those professionals who subscribe to theoretical orientations such as behavioural therapies or transpersonal, humanistic, or existential psychologies (Leijssen, 2006). However, finding mental health professionals who integrate mind-body dualism in their practice can be difficult; psychotherapeutic approaches are conceived as existing on a continuum of verbal interactions and an integration of awareness of the body (Leijssen, 2006).

Yoga is meant to supplement conventional treatments for clients with various mental health disorders, including anxiety and depression (Gerbarg & Brown, 2007). The practice of yoga alone cannot correct mental illness (Douglass, 2009); professional help is necessary to treat the primary cause of the disorder (Torkos, 2008). Interestingly, as Douglass (2009) found in her research, mental health professionals recognize that yoga may be a beneficial therapeutic supplement to psychotherapy, but they are not entirely informed about why or how yoga is effective. Thus, mental health professionals may benefit from learning what makes yoga effective and why and how to incorporate their learning into their practice with clients. By understanding the relationship between yoga and physiological functioning, mental health professionals will likely appreciate the benefits of yoga and, therefore, possibly offer a more comprehensive and desired mind-body therapeutic treatment.

PHILOSOPHICAL TENETS OF YOGA

Definition

Yoga is an approximately 5,000-year-old ancient "science, art, and philosophy" derived from East Indian culture (Austin & Laeng, 2003, p. 282). The father of yoga, Patanjali, devised yoga to be one of six philosophy systems to unite the body and mind and to also unite the personal spirit with the Universal spirit (Iyengar, 1966). To fully educate the reader about yoga within the scope of this article would be impossible. People can study and practice yoga for a lifetime yet still remain in the infancy stages of understanding yoga (Austin & Laeng, 2003). Thus, my intention in writing this article is to provide an introductory understanding of yoga and its relevance for mental health professionals.

The word *yoga* translates from Sanskrit to English as "'to yoke,' reflecting its purpose in joining the mind and body in harmonious relaxation" (Dey, Barrett, & Yuan, 2003, p. 172). Since yoga first began to be practiced in North America, most people practicing yoga place less emphasis on the philosophical practice and more emphasis on the physical postures, breathing techniques, and forms of meditation (Austin & Laeng, 2003).

Hatha yoga is the most commonly practiced style of yoga in North America. It is intended to create physical and emotional balance between body and mind (Dey et al., 2003). Iyengar is known for developing Iyengar yoga, a popular type of Hatha yoga that focuses on "technical alignment" of the body (Riley, 2004, p. 21). He is also revered for promoting the practice of yoga "as a form of health care throughout the world" (Austin & Laeng, 2003, p. 284). Ashtanga and Viniyoga are also popular types of Hatha yoga and focus on movement between the postures (Riley, 2004). Regardless of the style of Hatha yoga, a contemporary practice involves concentrated breath work through a variety of standing, seated, and balancing postures followed by forms of twists and backbends or inversions, and ends with a relaxation or meditation posture (Austin & Laeng, 2003).

Breath work. The Sanskrit word *prana* signifies the spirit and has various English meanings including "breath … life … [or] energy" (Iyengar, 1966, p. 44). Yogic breathing techniques may be stimulating and create energy, such as Ujjayi breathing, while other techniques may be calming and create balance, such as Bhastrika breathing (Gerbarg & Brown, 2007). Accordingly, the ability to rhythmically control the breath through inhalation, exhalation, and retention in an effort to reach a pure mind is important; mastering the breath squelches cravings and distractions in the mind and fosters control over the senses, leading to a state of concentration (Iyengar, 1966). A mind that is clear and free from thoughts and cravings may be considered pure: carefree, motionless, and mindless; this mind is ready for concentration, meditation, and self-examination (Iyengar, 1966). Reaching this state is important because concentration restores tranquility in the mind (Iyengar, 1966).

Physical postures. The physical postures, known in Sanskrit as *asanas*, positively alter "flexibility, strength, coordination, balance, and circulation" of the musculo-

skeletal system (Austin & Laeng, 2003, p. 285). Each posture was created to serve a purpose: exercise of a distinct body part (Iyengar, 1966). Other physical training tools (e.g., weights) are unnecessary, as the body and its limbs provide the required tools (Iyengar, 1966). As a result, postures can be weight-bearing, stabilizing, or mobilizing (Austin & Laeng, 2003).

While the postures benefit the body physically, they also benefit the mind (Iyengar, 2002). For example, physical postures can stimulate psychological and emotional responses and can change energy levels (Austin & Laeng, 2003). Postures "can be sequenced to … induce a variety of effects, such as to ground, soothe, stimulate, or revitalize one's energy level" (Austin & Laeng, 2003, p. 287). Specifically, forward bends have a calming or soothing effect (Austin & Laeng, 2003); backbends and inversions are invigorating (Austin & Laeng, 2003); and balancing postures "help develop psychoemotional poise and strength" (Austin & Laeng, 2003, p. 288).

Meditation. Once the body, the mind, and the senses have been stilled through the physical postures and the concentrated breath work, meditation can begin (Iyengar, 1966). Meditation is an advanced state of contemplation reached through sustained concentration over a period of time (Iyengar, 1966). In the deepest state of meditation, the body and the senses are relaxed, and the mind is alert but not distracted (Iyengar, 1966).

LITERATURE REVIEW

There have been an abundance of studies examining yoga as an effective clinical treatment intervention for psychological and physiological concerns. Khalsa (2004) completed an extensive review of the literature examining all of the clinical studies published that used yoga as a therapeutic intervention for psychological and physiological concerns. He found that there is some evidence to support the use of yoga as a treatment intervention for popular physical health concerns, such as cardiovascular and respiratory conditions, as well as for mental health conditions such as depression, anxiety, and addiction (Khalsa, 2004). Since Khalsa's review, additional researchers have found similar results; yoga has shown to be an effective physical intervention for the management of cardiovascular conditions (Donesky-Cuenco, Nguyen, Paul, & Carrieri-Kohlman, 2009), respiratory conditions (Vempati, Bijlani, & Deepak, 2009), chronic pain (Kolasinski et al., 2005), neurological conditions (John, Sharma, Sharma, & Kankane, 2007), as well as mental health disorders such as depression (Butler et al., 2008; Chen et al., 2009; Franzblau, Echevarria, Smith, & Van Cantfort, 2008; Krishnamurthy & Telles, 2007; Shapiro et al., 2007; Woolery, Myers, Sternlieb, & Zeltzer, 2004), anxiety (Descilo et al., 2010; Javnbakht, Kenari, & Ghasemi, 2009; Khalsa, Shorter, Cope, Wyshak, & Sklar, 2009; Kozasa et al., 2008; Lavey et al., 2005; Smith, Hancock, Blake-Mortimer, & Eckert, 2007; Subramanya & Telles, 2009; Telles, Guar, & Balkrishna, 2009; Telles, Naveen, & Dash, 2007; Telles, Singh, Joshi, & Balkrishna, 2010), and addiction (Khalsa, Khalsa, Khalsa, & Khalsa, 2008).

To fully review the efficacy of yoga as a treatment intervention for all physical and mental health disorders is outside the scope of this article. Given that anxiety and depression are the two most common mental health disorders being treated with CAM (particularly yoga), a review of the research findings for these two specific mental health disorders and plausible psychophysiological reasons to explain the research findings will be given. Below is a discussion of the relevant research studies conducted on yoga and anxiety, as well as yoga and depression, from 2003 to 2010. The discussion is organized for both anxiety and depression using the three major components of yoga: breath work, physical postures, and meditation.

Clinical Findings: Anxiety

BREATH WORK

Researchers speculated that devoting a large portion of time to focusing on the concentrated breath work aspects of a yoga practice would substantially aid in the reduction of symptoms of anxiety (Descilo et al., 2010; Khalsa et al., 2009; Kozasa et al., 2008; Telles et al., 2010). Kozasa et al. (2008) concluded that after one month of practicing breath work and meditation, both state and trait anxiety scores significantly reduced. After practicing yoga for 2 months, participants in the Khalsa et al. (2009) study reported an increase in self-confidence and clarity, as well as a reduction in fatigue and musical performance tension and anxiety. The physical and cognitive symptoms related to musical performance anxiety decreased due to the practice of yoga, particularly the breathing techniques (Khalsa et al., 2009). Telles et al. (2010) concluded in their study on the effects of yoga breathing practices on anxiety that there was an increase in feelings of anxiety in those participants who did not participate in the yoga practice. In the same year, Descilo et al. (2010) found that yoga breath work was effective in reducing symptoms of depression and anxiety, particularly posttraumatic stress disorder (PTSD).

PHYSICAL POSTURES

Khalsa et al. (2009) found that practicing yoga over a 2-month period improved physical strength, increased flexibility, and reduced physical tension in young musicians with performance anxiety. Attending three classes a week for 8 weeks, regardless of the physical intensity of the practice, created these physical changes in the participants (Khalsa et al., 2009). In the study conducted by Javnbakht et al. (2009), state and trait anxiety scores decreased significantly in women 30 to 40 years old after practicing yoga twice a week for 2 months. These researchers credited the decrease in anxiety scores to yoga's ability to increase body awareness (including body tension) and foster a sense of confidence and control over the body (Javnbakht et al., 2009).

MEDITATION

Subramanya and Telles (2009) compared the effects of cyclic meditation (yoga postures followed by supine rest) and supine rest (a relaxing yoga posture such as

shavasana) on levels of anxiety and memory scores. A short yoga practice of cyclic meditation (22:30 minutes) was significantly more effective in increasing memory scores and almost four times as effective in decreasing anxiety as the same amount of supine rest (Subramanya & Telles, 2009).

YOGA THEORY AND PHILOSOPHY

Researchers concluded that both practicing yoga and learning yoga theory are effective in reducing stress and symptoms of anxiety (Khalsa et al., 2009; Telles et al., 2009). For example, Khalsa et al.'s (2009) study contained two groups: one only practiced yoga and the other practiced and learned yoga theory. Both groups experienced decreased stress and performance anxiety (Khalsa et al., 2009). Telles et al. (2009) also found similar results in their study with two groups: one group practiced yoga and the other group watched a video to learn yoga theory. All participants reported a decrease in anxiety; however, participants who completed the 2-hour yoga practice reported a more significant decrease in anxiety symptoms (14.7%) than those participants who watched the video (3.4%; Telles et al., 2009).

Interestingly, when yoga was compared to conventional relaxation techniques designed to aid in stress management and decreasing anxiety (e.g., progressive muscle relaxation), yoga was found to be equally "as effective as relaxation with reducing stress and anxiety" (Smith et al., 2007, p. 81). Smith et al. (2007) concluded that yoga might be a safe way for people to assist themselves in managing symptoms related to anxiety and stress.

Clinical Findings: Depression

BREATH WORK

Practicing yoga breathing techniques for 45 minutes for 4 days was helpful in decreasing battered women's feelings of depression (Franzblau et al., 2008). Franzblau et al. (2008) found a significant decrease in depression scores in participants who received training in yoga breath work; specifically, participants who learned yoga breath work practices and also gave testimony experienced the most significant decrease in depression scores.

PHYSICAL POSTURES

Woolery et al. (2004) studied the effects of yoga on reducing symptoms of depression in young adults with mild depression. Researchers over a 5-week period examined a 1-hour Iyengar yoga class that focused on physical postures that "are supposed to alleviate depression, particularly back bends, [vigorous] standing poses, and inversions" (Woolery et al., 2004, p. 61). As expected, the inversions, standing physical postures of Iyengar yoga, as well as those postures that open and lift the chest, were effective in reducing depression scores by almost one-third and significantly improving mood (Woolery et al., 2004). Woolery et al. (2004) proposed that the backbends, inversions, and standing postures "enhanced feelings

of mastery … [and] may have countered the slumped body posture associated with depression" (p. 62).

Shapiro et al. (2007) also examined Iyengar yoga postures as an additional treatment for people experiencing symptoms of remission but taking antidepressant medications for unipolar major depression (Shapiro et al., 2007). After the study participants took 20 sessions of yoga practice three times weekly, Shapiro et al. found that an Iyengar yoga practice with inversions, standing postures, and postures that expand the chest were helpful in diminishing mood-related symptoms of depression. The physical postures positively increased mood state experiences of people with depression and also positively reduced other mental health areas such as anxiety; participants reported increases in positive mood characteristics (e.g., confidence and happiness) and energy levels (e.g., attentiveness), as well as decreases in negative mood characteristics (e.g., frustration and pessimism) (Shapiro et al., 2007). Interestingly, researchers concluded that improvements in mood after a yoga practice are not dependent on gender or having a mood disorder diagnosis (Lavey et al., 2005).

meditation

Krishnamurthy and Telles (2007) studied changes in depression scores in severely depressed seniors after practicing yoga versus receiving ayurveda medicine (herbal preparations). After 6 months of practicing yoga 6 days a week for 75 minutes a day, only the participants who practiced yoga showed significant decreases in depression (Krishnamurthy & Telles, 2007). Krishnamurthy and Telles reported that participants most enjoyed the last 15 minutes of the practice spent on devotional spiritual meditation songs; the songs "may have been able to give participants a group experience of expressing emotions, as well as a better acceptance of themselves" (2007, p. 22). Butler et al. (2008) also examined the effects of yoga, including meditation, on symptoms of chronic mild depression. Participants who received yoga and meditation training experienced the largest remission rate at 77% and did not develop any new depressive episodes regardless of using medications or receiving psychotherapy (Butler et al., 2008). Butler et al. concluded that meditation in yoga "may be used to let go of thoughts that maintain the depressive affect" (p. 816).

yoga theory and philosophy

No published studies were found in this exhaustive literature review on depression and how learning yoga theory and philosophy affects levels of depression.

Explanations of Yoga's Potency

After practicing yoga, people often report many "beneficial emotional, psychological, behavioral, and biological effects" (Shapiro et al., 2007, p. 501); an increased feeling of well-being is one important example (Dey et al., 2003). People experience beneficial changes to both the mind and body because "yoga creates change in the neurophysiology of the body" (Douglass, 2009, p. 127). The con-

nection between yoga and psychophysiology is gaining interest from researchers, and several hypotheses have been put forth to explain why yoga creates cognitive and physical changes. Researchers strongly suspect that yoga is effective because it positively alters brain neurochemistry (Streeter et al., 2007), and it "counteract[s] stress and reduce[s] autonomic arousal" (Khalsa, 2004, p. 277) of the autonomic nervous system (ANS), including the sympathetic nervous system (Ross & Thomas, 2010).

The causes of anxiety and depression can be "genetic, environmental, psychological, emotional, and social" (Forbes et al., 2008, p. 88), all of which change neurobiology (Forbes et al., 2008). Recently, researchers examined the effect of yoga on several neurotransmitters known to be involved with anxiety and depression; cortisol, gamma-aminobutyric acid (GABA), serotonin, and melatonin are strongly related to the regulation of stress and mood (Douglass, 2009).

BREATH WORK

Researchers examined ANS functioning and noted that yoga, especially the breath work, increases heart rate variability (Khattab, Khattab, Ortak, Richardt, & Bonnemeier, 2007), decreases blood pressure levels (Harinath et al., 2004), and decreases respiratory rates (Raghuraj & Telles, 2008). For people with mild to moderate depression, practicing Sudarshan Kriya Yoga (SKY) can improve depressive symptoms (Brown & Gerbarg, 2005b). For people with anxiety, practicing Ujjayi breath work as part of SKY "can restore a sense of control" (Brown & Gerbarg, 2005b, p. 713). Thus, using yoga breath work to counteract the ANS causes the sympathetic and the parasympathetic nervous system to function effectively in response to stress instead of "becoming hypo-reactive or hyper-reactive" (Brown & Gerbarg, 2005a, p. 195).

PHYSICAL POSTURES

People anecdotally report feeling more at ease after a yoga practice (Douglass, 2009). Researchers attributed these feelings of relaxation to decreased cortisol levels; "cortisol is referred to as the 'stress hormone'" (Douglass, 2009, p. 128) and is released during stress to prepare the body for a stress response (Ross & Thomas, 2010). A stress response is a physical or cognitive reaction "resulting from various disturbing physical, emotional, or chemical factors" (Sarbadhikari & Saha, 2006, Background, para. 2). Interestingly, Granath, Ingvarsson, von Thiele, and Lundberg (2006) found that after 10 sessions of a yoga practice focused on physical postures, an increased level in cortisol occurred in participants. Woolery et al. (2004) also found that after 10 yoga classes emphasizing Iyengar style postures, those who completed a regular 1-hour practice, twice a week, had higher levels of morning cortisol by the end of the 5 weeks than the control group who did not participate in yoga activities. Higher levels of cortisol are usually associated with a response to stress. However, in those people who experience above-average levels of a stress response, normal homeostasis of neurobiological systems may be disrupted (Lydiard, 2003). Thus, researchers

suggested that finding increased levels of morning cortisol may be a positive measure (Granath et al., 2006; Woolery et al., 2004), especially for those people with anxiety or depression who may have a lowered or altered homeostasis of neurobiology.

Lower GABA levels have been found in people with depression (Brambilla, Perez, Barale, Schettini, & Soares, 2003) and anxiety (Lydiard, 2003). Streeter et al. (2007) speculated that the physical postures of yoga would increase GABA activity levels in the brain, thus decreasing anxious and depressive symptoms. After the participants in the study completed a 1-hour session of yoga postures, Streeter et al. (2007) indeed found that GABA levels increased by approximately one-quarter when compared with baseline measurements, regardless of the type of yoga practiced.

Serotonin is also believed to improve mood states (Douglass, 2009). Davies et al. (2006) found that serotonin regulates both psychological and cardiovascular responses to stress in people with anxiety. To this author's knowledge, there have been no studies published on yoga's effect on serotonin levels. However, Young (2007) noted that there are many non-pharmacological methods for increasing serotonin, including exercise. Interestingly, Netz and Lidor (2003) examined the effects of mindful exercise (e.g., yoga) versus aerobic exercise (e.g., swimming) on anxiety and depressive mood. These researchers concluded that after just one session, mindful exercise more effectively reduced anxiety and increased positive mood than aerobic exercise (Netz & Lidor, 2003). Netz and Lidor proposed that this finding may be due to the cognitive focus found in mindful activities.

MEDITATION

Melatonin is believed to regulate mood and sleep patterns (Douglass, 2009). In Harinath et al.'s (2004) study, researchers found that participants had increased melatonin levels after practicing yoga, particularly meditation. Additionally, researchers found that people with depression who practiced yoga reported feeling more positive about themselves and their health and also reported sleeping better (Chen et al., 2009). Harinath et al. (2004) hypothesize that the participants' positive reports may be due to increased levels of melatonin.

Summary

Yoga reduced anxiety for people with state, trait, and performance anxiety as well as for people with PTSD. The breath work practice, meditation practice, and learning about yoga practices decreased feelings of anxiety, whereas the physical postures decreased the physical symptoms of tension and anxiety. Given that yoga was found to be as effective as conventional relaxation techniques in reducing stress and anxiety, people with anxiety may wish to also consider the unconventional relaxation techniques of yoga.

Yoga reduced depression for people with many forms of depression, including those who were taking antidepressant medications or participating in conven-

tional psychotherapy. Particularly, the physical postures of Iyengar yoga had many physical and cognitive benefits for people with depression. Focusing on postures that open and lift the chest counteracted feelings of depression, increased positive mood, offset the collapsed body posture, and helped people with depression. Additionally, the breath work practices gave people with depression a sense of control over their bodies, while the meditation practices allowed people with depression to release patterns of rumination on depressive thoughts.

Studies showed that yoga improved participants' psychophysiology. For example, the autonomic nervous system response became regulated, and neurotransmitter and hormone levels positively changed. After practicing yoga, a form of homeostasis was created in the body similar to the effects of conventional medications. Thus, yoga may be an appropriate non-pharmacological clinical intervention for treating anxiety and depression (Javnbakht et al., 2009).

Research Critique

Several issues with the methodology of the studies reviewed may limit the validity of the results supporting the use of yoga as a clinical intervention for anxiety and depression. The methodological issues preventing researchers from concluding that yoga is an effective treatment intervention for anxiety or depression are fourfold. First, more rigorous research needs to be done, especially randomized, controlled trials (RCTs). Approximately half of the studies reviewed were RCTs. While the aforementioned studies show promising results, researchers are in the infant stages of drawing conclusions about the efficacy of yoga as a treatment intervention for anxiety and depression.

A second methodological concern is that researchers have not determined which type of yoga, which aspect of yoga, or what amount of yoga creates the most psychological benefit for those with anxiety or depression. Researchers used a range of yoga interventions from gentle practices such as Hatha yoga to invigorating practices such as Ashtanga yoga, which altered the types of breathing techniques, physical postures, and forms of meditation. Also, the length of time, the amount of time, and the intensity of the yoga intervention varied; some participants in the studies received one yoga intervention while other participants received days, weeks, or months of practice. Thus, researchers may wish to focus on a style of yoga and an aspect of yoga to narrow the scope of research. Additionally, researchers could examine the benefits derived from a regular yoga practice over a substantial period of time.

A third concern is that it is difficult to generalize the findings regarding yoga's potential effectiveness to all people with anxiety or depression because the participants in each study varied in ethnicity, age, sex, yoga experience, motivation, and severity of anxious or depressive symptoms. Participants in some studies were East Indian while participants in other studies were not, which may potentially have created biases in the results, given that yoga may be a familiar form of health practice in East Indian culture (Descilo et al., 2010).

Some studies had participants similar in age while others had a range of ages. Additionally, some studies included both male and female participants but others included either female or male participants. Further, participants in some of the studies were unfamiliar with yoga or yoga techniques while others were familiar and were current yoga practitioners. The participants' motivation and interest in participating in yoga practice were also incongruent between studies. Finally, the severity of anxious or depressive symptoms ranged from anticipated symptoms to formal diagnoses of disorders. As well, there was no consistency in the assessment tools used to evaluate the anxiety and depression. Thus, it will be important for researchers to conduct studies that have a larger sample size, as well as a representative sample population, in order to generalize findings.

Fourth, there is a paucity of information on safety or contraindications with respect to yoga. Information on any adverse effects related to the practice of yoga was not explored in any of the studies reviewed. However, it is important that people understand the techniques of yoga to avoid incorrect practices leading to harmful consequences. For example, people with a pre-existing medical condition such as pregnancy should avoid practicing certain breathing techniques (Sharma, 2007). Thus, it will be important for researchers to examine any contraindications and determine which type of yoga is appropriate for people who do have pre-existing health conditions or comorbid health conditions.

Implications for Researchers

How researchers measure the effectiveness of yoga is complicated. There is a lack of congruence between Western quantitative research methods and the Eastern philosophy of complementary and alternative medicine; the individualized, comprehensive, and multidimensional nature of these interventions is difficult to measure quantitatively with RCTs (Spencer, 2003). Applying Western scientific scrutiny to the holistic practice of yoga may jeopardize the sacredness and holistic unity of yoga. Conceivably, both qualitative and quantitative methods may be useful in validating the effectiveness of yoga. Quantitative evidence "shows effects that are statistically significant according to predefined criteria [but] do not address why an intervention is effective, meaningful, or beneficial" (Lake, 2007, p. 71) whereas qualitative data assess "the impact of a treatment on quality of life" (Lake, 2007, p. 71).

Interestingly, Indian researchers have conducted most of the research on the effectiveness of yoga (Khalsa, 2004). In comparison, Western researchers have published fewer studies, but they have conducted more RCTs than Indian researchers (Khalsa, 2004). Khalsa (2004) posited that researchers, particularly Indian researchers, should implement research designs that are more rigorous to continue to validate the effectiveness of yoga as a clinical treatment intervention. The development of a standardized yoga practice manual may be helpful for effectively studying yoga in a respectful yet rigorous way (Uebelacker, Epstein-Lubow, et al., 2010; Uebelacker, Tremont, et al., 2010).

Suggestions for Practice for Mental Health Professionals

Despite the lack of conclusive research evidence to support integrating yoga as an effective clinical treatment, yoga continues to be a popular complementary and alternative treatment. Integrating yoga with psychotherapy may allow mental health professionals to work more collaboratively with clients. Uebelacker, Tremont, et al. (2010) argued that yoga and psychotherapy can be easily married, as there "are many similarities between psychotherapy and yoga" (p. 259), both are multidimensional, and each can be implemented uniquely (Uebelacker, Tremont, et al., 2010). However, most mental health professionals do not have training in CAM (Bassman & Uellendahl, 2003). Consequently, integrating yoga with psychotherapy encourages mental health professionals to refer clients to suitable yoga classes, collaborate with a yoga therapist to co-manage clients, and/or become yoga therapists themselves to deliver both psychotherapy and yoga in one session.

For clients with anxiety or depression who are willing to participate in community-based yoga classes, yoga may be used as a supplement to psychotherapy. Accordingly, it is important that clients be accurately matched to a suitable type of yoga (Boudette, 2006). Thus, a large onus is placed on the mental health professional to be sufficiently informed about the practice of yoga and about suitable types of yoga to help the client select an appropriate class. Additionally, a large onus is placed on the client to be willing to participate in a community-based yoga class; clients need to feel safe doing yoga in order to benefit from the practice.

For clients who may resist joining a community-based yoga class, mental health professionals may alternatively refer clients with a diagnosis of anxiety and/or depression to a yoga therapist as "an adjunct to psychotherapy or medication management" (Forbes et al., 2008, p. 90). A yoga therapist is certified in the practice of yoga and is responsible for designing an individual yoga practice to treat a person's mental and physical health (Khalsa et al., 2009). In such a scenario, the client may complete a session with the mental health professional and then join the yoga therapist for a private, supervised, and uniquely designed yoga practice. Both the referring mental health professional and the yoga therapist may work together to co-manage clients and deliver a suitable mind-body treatment.

Given that the licensing procedures greatly vary for CAM practitioners (White, 2000), it is helpful for mental health professionals to be aware of competent CAM practitioners to guide the referral process for the client. If clients are left on their own to find a CAM practitioner, they may find a practitioner who is licensed but "less than fully competent and trustworthy" (Bassman & Uellendahl, 2003, p. 267). Currently, there is a lack of national standards regulating the licensing process for yoga therapists (Douglass, 2009). Thus, it would be beneficial for mental health professionals interested in using yoga with psychotherapy to familiarize themselves with local yoga therapists suitably qualified to co-manage clients with anxiety and depression.

For mental health professionals who wish to offer yoga and psychotherapy as a mind-body treatment in a single cohesive session, seeking dual accreditation

to also become a licensed yoga therapist is necessary. Interestingly, White (2000) pioneered the idea of an integrative medical psychology that combines psychology with CAM interventions to provide more holistic care. White is a dually trained practitioner: she is a psychologist, and she is also a CAM practitioner trained "in Oriental medicine and homeopathy" (p. 678). White acknowledged that being a dually trained practitioner does create many questions about boundaries and may elicit uncertainty for clients. However, she noted that in many CAM practices, the offering of a "body-mind treatment" by one practitioner is extremely beneficial (White, 2000, p. 679).

Implications for Practice

Several implications arise when combining yoga and psychotherapy; mental health professionals can offer a larger continuum of care, but their legal and ethical responsibilities will also expand.

continuum of care

For mental health professionals, integrating yoga with psychotherapy enables them to offer an expanded continuum of care beyond psychotherapeutic and pharmacological interventions. Therapy can be individualized for the needs of each client, rather than making all clients fit one psychotherapeutic approach (Strozier, 2008). Expanding the continuum of care to include yoga may help mental health professionals consider new ways of understanding mental health, illness, and healing. It is possible that mental health professionals have failed to notice the value in providing wholesome solutions "in addition to or in lieu of psychotherapy and prescription medications" to address mental health concerns (White, 2009, p. 633).

legal responsibilities

Mental health professionals assume a far greater burden of liability, including greater legal accountability for malpractice, by incorporating CAM with psychotherapy even when they co-manage clients with CAM practitioners or use CAM themselves within psychotherapeutic sessions (Cohen & Schouten, 2007). Therefore, mental health professionals providing CAM mind-body treatments could collect clients' detailed medical history to avoid recommending a complementary and alternative treatment that may negatively harm pre-existing medical conditions (Cohen & Schouten, 2007). For example, mental health practitioners should avoid recommending certain yoga breath work techniques to clients who hyperventilate, faint, or have high blood pressure because it may exacerbate clients' anxiety (Sharma, 2007).

Additionally, mental health professionals should provide treatment that has research evidence to support its effectiveness (Cohen & Schouten, 2007). Cohen and Schouten (2007) recommended that conventional treatments should be the initial form of care, especially if the client's "condition can be readily cured by conventional care" (p. 24). Lake (2007) maintained that it is safe to combine

conventional treatments with CAM when there is sufficient evidence from RCTs regarding the efficacy of each treatment; case studies may be considered for CAM when few RCTs have been completed. Thus, for people with anxiety or depression who are interested in combining yoga with treatment, it may be advantageous for mental health professionals to recommend conventional psychotherapeutic and pharmacological treatments first before the incorporation of yoga is considered. However, given that the conventional treatment for anxiety and depression is not always effective, that people may not want to take medications, that people may desire a more holistic treatment, and that there are minimal risks associated with an informed yoga practice, yoga is a viable treatment option to consider alongside conventional treatments in the decision-making process.

ethical responsibilities

Mental health professionals have an increased ethical responsibility to get informed consent before integrating any treatments with clients (Canadian Association of Social Workers [CASW], 2005; Canadian Counselling and Psychotherapy Association [CCPA], 2007; Canadian Psychological Association [CPA], 2000). Ensuring that clients are sufficiently educated about treatment options in decision-making is necessary. It is optimal to collaborate with clients to assess and examine the risks and the benefits of combining treatment options, including the safety of combining conventional and complementary treatments (Lake, 2007). Mental health professionals also have an increased ethical responsibility to be accurate and honest about their areas of competence, knowledge, and practice, and need to receive adequate training and supervision before declaring competency in various treatments (CASW, 2005; CCPA, 2007; CPA, 2000). White (2009) recommended 15 hours of training as the minimum for psychologists to sufficiently learn about CAM. Additionally, mental health professionals have an increased ethical responsibility to adhere to ethical principles of confidentiality, especially when referring and/or co-managing clients with other professionals (CASW, 2005; CCPA, 2007; CPA, 2000). Finally, mental health professionals have an increased responsibility to manage boundaries (CASW, 2005) and dual relationships (CCPA, 2007; CPA, 2000), especially if they are accredited as both mental health professionals and CAM practitioners (they will also have to manage two sets of codes of ethics, scopes of practice, and licensing procedures).

Thus, it is extremely important that mental health professionals do not recommend or force yoga on clients who are uninformed about the risks and benefits of a yoga practice or who are ambivalent or uncomfortable with the intervention; integrating yoga works best for clients interested in participating in and committing to a regular yoga practice. If a mental health professional is not adequately trained as a yoga therapist, the professional should be honest about his or her lack of training and competency with the client and not implement yoga practices within sessions but should instead refer the client to a qualified yoga therapist. Mental health professionals should also respect client confidentiality when collaborating with yoga therapists. Some clients may share

the same information with both a yoga therapist and a mental health professional, while others may choose to share only certain information with either professional.

Finally, if trained in both disciplines, the professional has to carefully establish appropriate boundaries as both a mental health professional and a yoga therapist in the relationship with the client. For example, in most talk therapies, the mental health professional does not touch the client, whereas in a yoga session, the yoga therapist may touch the client to adjust the posture or support a position. Being cognizant of the differing boundaries of the two professional roles should help navigate the dual relationship with the client.

CONCLUSION

People often seek alternative ways to manage their anxiety and depression; self-help treatments such as yoga seem to offer desirable physical and mental benefits, especially when conventional treatments for anxiety and depression may not always be desirable or effective. Thus, addressing anxiety and depression through the concentrated breath work practices, physical postures, and meditation of yoga may be an additional way for mental health professionals to treat anxiety and depression. Accordingly, it is important that mental health professionals encourage more rigorous research to further support the efficacy of yoga as a clinical treatment intervention for anxiety and depression, as there is a paucity in the current research. Those mental health professionals who choose to use yoga as a complementary treatment with psychotherapy may work more collaboratively with clients, deliver a more individualized therapeutic treatment, and implement a more holistic therapy.

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SBV AHEAD

YOGA THERAPY: AN OVERVIEW

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"Yoga Chikitsa is virtually as old as Yoga itself, indeed, the 'return of mind that feels separated from the Universe in which it exists' represents the first Yoga therapy. Yoga Chikitsa could be termed as "man's first attempt at unitive understanding of mind-emotions-physical distress and is the oldest wholistic concept and therapy in the world."

- Yogamaharishi Dr. Swami Gitananda Giri, ICYER at Ananda Ashram, Pondicherry.

Yoga may be said to be as ancient as the universe itself, since it is said to have been originated by *Hiranyagarba*, the causal germ plasm itself. This timeless art and science of humanity sprouted from the fertile soil of *Sanathana Dharma*, the traditional pan-Indian culture that continues to flourish into modern times. [1,2,3,4]

Today, Yoga has become popular as a therapy, and most people come to it seeking to alleviate their physical, mental and emotional imbalances. We must understand, however, that the use of Yoga as a therapy is a much more recent happening in the wonderful long history of Yoga–which has historically served to promote spiritual evolution. Yoga helps unify all aspects of our very being: the physical body, in which we live our daily life; the energy body, without which we will not have the capacity to do what we do; the mind body, which enables us to do our tasks with mindfulness; the higher intellect, which gives us clarity; and, finally, the universal body, which gives us limitless bliss. [5,6]

All aspects of our life--physical, energetic, mental, intellectual and universal--are unified through the practice of Yoga, which may also be described as the science of right-use-ness, that is, of using our body, emotions, and mind responsibly and in the most appropriate manner. One of the best definitions of Yoga given by Swami Gitananda Giri is that it is a 'way of life'.[7,8] It is not something you do for 5 minutes a day or 20 minutes a day. It is indeed a 24 × 7 × 365. lifestyle.

Illness, disease and disorders are so common in this world,and people everywhere are desperately seeking relief from their suffering. Yoga helps us to think better and to live better; indeed, it helps us improve ourselves in everything we do. Hence it holds out the promise of health, well-being and harmony. According to the Bhagavad Gita, an ancient text which can be said to be a *Yoga Shastra* (seminal textual source of Yoga), Lord Krishna the Master of Yoga (Yogeshwar) defines Yoga as "*dukkhasamyogaviyogam yoga samjnitham*" meaning thereby that Yoga is the disassociation from the union with suffering. Pain, suffering, disease - Yoga offers a way out of all of these. [5,6,9,10]

One of the foremost concepts of Yoga therapy is that the mind, which is called adhi, influences the body, thus creating vyadhi, the disease. (Fig 1) This is known as the adhi *vyadhi* or *adhija vyadhi,* where the mind brings about the production of disease in the physical body. In modern language, this is called psychosomatic illness. [6] Virtually every health problem that we face today either has its origin in psychosomatics or is worsened by the psychosomatic aspect of the disease. The mind and the body seem to be continuously fighting each other. What the mind wants, the body won't do, and what the body wants, the mind won't do. This creates a dichotomy, a disharmony, in other words, a disease. Yoga helps restore balance and equilibriumby virtue

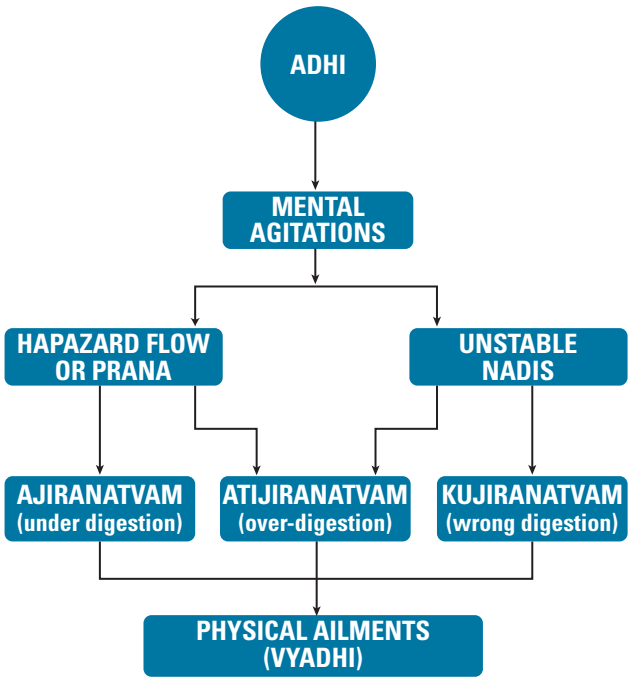


Fig 1. Causation of disease, the Yogic perspective

of the internal process of unifying mind, body and emotions. The psychosomatic stress disorders that are so prevalent in today's world can be prevented, controlled and possibly even cured via the sincere and dedicated application of Yoga as a therapy.

Psychosomatic disorders go through four major phases. The first is the psychic phase, in which the stress is located essentially in the mind. There is jitteriness, a sense of unnatural tension, a sense of not being 'at ease'. If the stress continues, the psychic stage then evolves into the psychosomatic stage. At this point,the mind and body are troubling each other and fluctuations, such as a dramatic rise in blood pressure, blood sugar or heart rate, begin to manifest intermittently. If this is allowed to continue, one reaches the somatic stage, where the disease settles down in the body and manifests permanently. At this stage, it has become a condition that requires treatment and therapy. In the fourth, organic stage, the disease settles permanently into the target organs. This represents the end stage of the disease.

Yoga as a therapy works very well at both the psychic and psychosomatic stages. Once the disease enters the somatic stage, Yoga therapy as an adjunct to other therapies may improve the condition. In the organic stage, Yoga therapy's role is more of a palliative, pain relieving and rehabilitative nature. Of course the major role of Yoga is as a preventive therapy, preventing that which is to come. Maharishi Patanjali tells us in his *Yoga Darshan*, "*heyamdukkhamanagatham*"-prevent those miseries that are yet to come". [1,11]

MODALITIES OF YOGA AS A THERAPY[6]

PHYSICAL THERAPIES
- Asanas, Kriyas, Mudras and Bandhas

EMOTIONAL THERAPIES
- Swadhyaya, Pranayama, Pratyahara, Dharana, Dhyana and Bhajans
- Development of +ve attitudes
  - Vairagya
  - Chitta Prasadan
  - Maitri, Karuna, Mudita, Upekshanam

MENTAL THERAPIES
- Relaxation & visualization, Trataka, Pranayama, Pratyahara, Dharana & Dhyana

SPIRITUAL THERAPIES
- Swadhyaya, Satsangha, Bhajans and Yogic counseling

PREVENTIVE THERAPIES
- Start early in childhood
- Prevention of accidents
- Improved immunity
- Knows the technique so that can do it if needed

REHABILITATIVE THERAPIES
- Prevention of disability & improving QOL

PAIN RELIEF THERAPIES
- Increases pain tolerance - improved quality of life

If the practice of Yoga is taken up during childhood, we can prevent so many conditions from occurring later on in life. This is primary prevention. Once the condition occurs, once the disease has set in, we have secondary prevention, which is more in the nature of controlling the condition to whatever extent we can. Tertiary prevention is done once the condition has occurred, as we try to prevent the complications, those that affect the quality, and even the quantity, of a patient's life. [7]

When we use Yoga as a therapy, we need to consider both the nature of the person–his or her age, gender and physical condition–and the nature and stage of the disorder. A step-by-step approach must include a detailed look at all aspects of diet, necessary lifestyle modifications, attitude reconditioning through Yogic counseling, as well as the appropriate practices. All of these are integral components of holistic, or rather, wholesome Yoga therapy. When such an approach is

adopted, tremendous changes will manifest in the lives of the patients and their families. The quality of life improves drastically and, in many cases, so does th quantity.

As human beings, we fulfill ourselves best when we help others. Yoga is the best way for us to consciously evolve out of our lower, sub-human nature, into our elevated human and humane nature. Ultimately, this life giving, life enhancing and life sustaining science of humanity allows us to achieve in full measure the Divinity that resides within each of us.

I would like to conclude this overview of Yoga therapy with a word of caution. Yoga therapy is not a magic therapy! It is not a 'one pill for all ills'. There should be no false claims or unsubstantiated tall claims made in this field. Yoga therapy is also a science and must therefore be approached in a scientific, step-by-step manner. It should be administered primarily as a 'one on one' therapy that allows the therapist to modify the practices to meet the needs of the individual. It is not a "one size fits all" or "one therapy fits all" approach!

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The effect of yoga in psychotherapy

Dr. Balaji Deekshitulu PV

Abstract
The review article states that yoga has been used throughout the world for thousands of years and their benefits have been repeatedly documented by physiological and psychological problems. Yoga can formulate a positive change in people suffering from these distressing states of mind and behaviour it is proved can help the reducing their anxiety, stress, depression, phobia etc… levels and thus capable of dealing with mental ill health situations this astanga yoga techniques work towards balancing the excitatory neurotransmitter levels in the brain.

Key words: Psycho illness, yoga, meditation, mudras, pranayama, asanas

Introduction
Yoga is an ancient art that is defined as the union of the soul with God. The practice is at least 3000 years, It is "a path of personal spiritual development that utilizes meditation to bring enlightenment, self-realization, and, ultimately, the attainment of God and bliss". Originally, the ultimate goal of yoga was called Samadhi, or self-realization. *Patanjal defined*: *Yogas Citta Vrtti Nirodhah*, "Yoga is the restraint of the agitation of thoughts"(Iyangar, 2001) [1]. Patanjali is father of yoga around the sixth century B.C. appeared in the massive epic The Mahabharata written by sage Vyasa and containing The Bhagavad Gita. Krishna explains to Arjuna about the essence of Yoga as practiced in daily lives ('Song of the Lord'), uses the term "yoga" extensively in a variety of ways. *Bhagavad-Gita* yoga is also defined as "skill in action" and "moderation in everything" (Verma *et al*. 2014) [2], In addition to an entire chapter dedicated to traditional yoga practice, including meditation, it introduces three prominent types of yoga:

- Karma yoga: The yoga of action
- Bhakti yoga: The yoga of devotion, note Krishna had also specified devotion itself was action similar to above.
- Jnana yoga: The yoga of knowledge.

Yoga in psychotherapeutic use: The various yoga techniques are treatment of psychiatric patients. They found the yogic treatment to be more efficient than psychoanalysis or psychotherapy and behaviour therapy Khalsa SB *et al*. (2012) [13]. They later put forth a new approach termed psycho-physiologic therapy based on the concepts and techniques of Patanjali like Pranayama (breathing), Asana (postures),Yama (restraint), Niyama (healthy observances), Pratyahara (sensory withdrawal), Dharana (concentration),, Dhyana (meditation), Samadhi (higher consciousness). Psychoanalysis and Yoga differ not only in method but also in their aims. The aim of psychoanalysis is to resolve the conflicts and strengthen the ego so that the individual is better adjusted to the normal demands of situations, but the aim of Yoga is not only to remove mental strains but also to transcend the ego-consciousness so that spiritual consciousness may dawn.

What is mental illness? A mental illness is a medical condition that disrupts a person's thinking, feeling, mood, ability to relate to others and daily functioning. Serious mental illnesses include major depression, schizophrenia, bipolar disorder, obsessive compulsive disorder (OCD), panic disorder, posttraumatic stress disorder (PTSD) and borderline personality disorder.

A mental health problem also affects how a person thinks, feels, and behaves, but to a lesser extent than a mental illness.These illnesses may also be referred to as a mental disorder, mental impairment or psychiatric disability.

Causes of Mental Illness: Mental illness results from complex interactions between the mind, body and environment. Factors which can contribute to mental illness are:

- long-term and acute stress
- biological factors such as genetics, chemistry and hormones
- use of alcohol, drugs and other substances
- cognitive patterns such as constant negative thoughts and low self esteem
- social factors such as isolation, financial problems, family breakdown or violence

These factors can be minimised by a strong and supportive community environment.

Signs for Metal Illness

- Eating or sleeping too much or too little
- Pulling away from people and usual activities
- Having low or no energy
- Feeling numb or like nothing matters
- Having unexplained aches and pains
- Feeling helpless or hopeless
- Smoking, drinking, or using drugs more than usual
- Feeling unusually confused, forgetful, on edge, angry, upset, worried, or scared
- Yelling or fighting with family and friends
- Experiencing severe mood swings that cause problems in relationships
- Having persistent thoughts and memories you can't get out of your head
- Hearing voices or believing things that are not true
- Thinking of harming yourself or others
- Inability to perform daily tasks like taking care of your kids or getting to work or school

Yoga and mental health: Yoga is a mind and body practice with historical origins in ancient Indian philosophy. Like other meditative movement practices used for health purposes, various styles of yoga typically combine physical postures, breathing techniques, and meditation or relaxation. Yoga in its full form combines physical postures, breathing exercises, meditation, and a distinct philosophy. There are numerous styles of yoga. Hatha yoga, commonly practiced in the United States and Europe, emphasizes postures, breathing exercises, and meditation. Hatha yoga styles include Ananda, Anusara, Ashtanga, Bikram, Iyengar, Kripalu, Kundalini, Viniyoga, and others. However, the psychiatric illness outcome with Arun Kumar Nayak (2016) [3], Kusuma (2016) [4]. The yoga is physiological benefits of the practice, such as increased flexibility and decreased muscle
stiffness. However, not enough is mentioned about the mental, psychological, and cognitive benefits of yoga—and there are plenty. There have been several studies done to assess the effectiveness of yoga for brain and mental health

- Yoga improves your psychological/mental well-being.
- Yoga helps with anxiety and depression.
- Yoga boosts memory and improves concentration.
- Yoga prevents the onset of mental health conditions, which are prevalent during adolescence.
- Yoga reduces the effects of traumatic experiences (Goyeche JR (1979) [5], Syed Kareemulla (2010) [6], Derebail Gururaja, *et al*. (2011) [7], Ingunn Hagen and Usha S. Nayar (2014) [8], Shruti Srivastava *et al*. (2015), Raja Selvaraj (2016) [9], Camila Ferreira Vorkapic (2016) [10].

Asana for Psychotherapy

In today's hectic world it has become quite obvious for people to lose their mental health balance. This loss of balance of mental health is depicted in the form of anxiety and depression. It is a proven fact that yoga really helps to stabilize the mental health of human beings. Let us see how yoga can really help you enhance. Hatha Yoga describes Shatkarmas (six processes) in details for Body purification and mind purification. Various asanas (Yoga Positions), six shatkarmas, mudras & bandhas (psychic knots or psycho-physiological energy release techniques) and Pranayama are described in old ancient Sanskrit texts of Hatha Yoga (Hatha Yoga Pradipika and Gherandasamhita). They work at the chitta (subtle aspect of consciousness) levelthat eliminates the physical and mental tensions. They are trained insupine and prone position of the body respectively (Arndt Büssing, *et al*. (2012) [11], Hilary B. Meyer *et al*. (2012) [12], Khalsa SB *et al*. (2012) [13], Kazufumi Yoshihara (2014) [15], Singh and Amarendra N. (2015) [14], Shivarama Varambally and B. N. Gangadhar (2016), Farahnaz Davar *et al*. (2016) [17], Meena Ramanathan *et al*. (2017) [18] Shavasana and Vijrasana, Bhujangasana, Trikonasana, Halasana, Sarvangasana etc…important relaxative asanas (Figure 1).

Vajrasana is best asana of Stress & Tension and Anxiety it makes the body exceptionally strong and healthy. Vajra means Diamondin Sanskrit. During this asana, feel the sensations in the body as youmove them (Vikrant Thakur and Vikash Bhatnagar, (2016) [19] Do the Movement slowly and in a relaxed manner. Inhale and exhale slowly. While drawing the abdominal region inwards and expanding the chest, focus the attention on these body parts. The way the muscles move must be felt Relax before, during and after the yoga posture. 1. With knees, ankles and big toes touching the ground, take a kneeling position. 2. One should sit on the heels and place palms on the knees. 3. The
Spine should be erect and breath should be deep. 4. Draw the abdominal region inside and expand the chest (Figure2).

The triangle pose: Trikonasana The legs are spread apart and the body stretches to the left, moving from the pelvis and extending over the left leg. Both arms are perpendicular to the floor, the left hand on the floor, or resting on the calf or the outer ankle of the left foot, and the right hand reaching up straight. The spine is straight, chest open, the body facing to the front (Figure 3).

The cobra pose: Bhujangasana The pose starts from a downward facing position with hands flat on the floor below the shoulders. The spine is lengthened and the buttocks firmed as the head and chest is slowly lifted. The elbows stay close to the body and the eyes look up. The neck is kept neutral (Figure 4).

Halasana

1. Take supine position hands straight by the side of thighs, palm resting on the ground

2. Slowly raise your legs together without bending at knees by pressing your hands and stop at 30° angle
3. After few seconds raise your legs further up to 60° angle and maintain
4. Now slowly bring the legs at 90° angle
5. Pressing both the hands bring the legs little towards the head.
6. Continue the bending of legs till toes touch the ground and then stretch your legs backward as far as possible.
7. Now place both the palms on the head making finger lock. Bring the elbows on the ground.
8. While returning back to the original position first release the finger-lock. Stretch the hands straight and place them on the ground by the side of the body.
9. Lower the waist and raise the legs from the ground. Slowly let the waist rest on the ground and stop bringing the legs at 90° angle.
10. Slowly come back to the original position. (Figure 5)

Sarvangasana: 1.Lie on your back and lift your legs up into air. 2. Place your hands on your lower back for support, resting your elbows and lower arms on the ground. 3. Make sure your weight is on your shoulders and mid to upper back -- not your neck. 4. Breathe deeply and hold for at the posture for at least 5-10 breaths, increasing the hold over time. 5. To come down, slowly lower your legs, keeping them very straight -- a little workout for your abdominal muscles. (Figure 6).

Mudras in Pranayama and Meditation

Meditation and Pranayama is an ancient technique to revive your weary soul and help you deal effectively with stress, anxiety, depression, phobias etc…, Meditation one of the eight limbs of yoga this is completely shutting out all kinds of mental fluctuations (Mishra *et al*. (2012) [20], Lavretsky *et al*. (2013) [21], Edo Shonin *et al*. (2013) [22], Meera Balasubramaniam *et al*. (2013) [23], Goyal M *et al*.(2014) [24], Tim Lomas *et al*. (2014) [25].

*Mudra*s are particular positions of the hands and the body in order to establish powerful and special energy patterns, similar to electrical circuits. Mudras are helps active and flexibly of body and mind (Kuladeep singh (2015,) [26], Balaji P.V Deekshitulu (2016) [27]. Mudra a symbolic representation of particular desired psychological or spiritual qualities they are…

Gyana Mudra: Touch the tip of the thumb to the tip of the index finger, with the other three fingers stretched out. (Figure 7).

Prana Mudra: Bend ring finger and little finger and touch the tip of thumb with their tips keeping the remaining two fingers stretched. (Figure 8)

Pranayama Padmasana Sit on the ground by spreading the legs forward. 2. Place the right foot on the left thigh and the left foot on theright thigh. 3. Place the hands on the knee joints. 4. Keep the body, back and head erect. 5. Eyes should be closed. 6. One can do Pranayama in this asana (Figure 9).

Siddhasana Place the left heel at the anus, the terminal opening of the alimentary canal or digestive tube. 2. Keep the right heel on the root of the generative organ. 3. It should be in a way that the knees and the heels, both lie upon the other. 4. The hands must be placed on either sole of the feet. Keep the eyes half closed (Figure 10).

Breathing exercise Pranayama, the breathing exercise, may rejuvenate you, to carry on your work with extra energy Bhole (1982) [28], G.V.Lathadevi *et al*(2012) [29].

- Put your right thumb on your right nostril
- Deeply inhale air using your left nostril
- Close your left nostril with your right index finger and hold breath for few seconds
- Exhale through left nostril
- Do it similarly with left nostril closing right nostril
- Now inhale through left nostril, hold breath and exhale through right nostril and do the other way (Figure 11).

Conclusion: Taken together, while several reviews suggest positive benefits of Yoga, can helps to improve the mental health of all of them by reducing stress and improve Prevention of physical and mental health to Quality of Life in all age groups. It is quite likely that yoga may help to improve self-efficacy, self-competence, physical fitness, and group support, and may well be effective as a supportive adjunct to mitigate mental medical conditions,

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MEDITATION FROM A MENTAL HEALTH PROSPECTIVE[1]

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SUMMARY

The term "Meditation" is defined and the aims of meditation-practice areo utlined. The physiological and psychological correlates of meditation, as found in different studies, are described. The need for an integrated approach to the study of consciousness is emphasized. The clinical applications of meditation practice are spelt out. Areas of further research are indicated.

Definition, Process and the Aims:

The word 'Meditation' means different things to different people. In the Christian tradition, for example, meditation is most often understood as a dwelling upon certain ideas or engaging in a directed, intellectual course of activity, while the Eastern methods of meditation understand the term to mean dwelling on anything but ideas and with the goal of attaining an a-conceptual state of mind that excludes intellectual activity, normally associated with the waking state. For our purposes, meditation may be defined as a group of practices that train attention to enhance awareness, through mental discipline, with the ultimate aim of development of insight into the nature of mental processes, consciousness, identity, reality and spiritual values.

Though the practice of meditation can be traced back to 3000 years, there has been an upsurge in interest in meditation in the recent past. In the U.S.A. and Canada alone, there are no less than 300 meditation centres, and Transcendental meditation or T. M. today, has a world-wide following. Along with the increased interest in meditation by the laity, psychologists and psychiatrists have also shown a greater interest in the scientific study of the subject and its possible clinical applications. A spate of books have also appeared on the subject. While there may be a number of factors responsible for the recent increase in interest in meditation, some of the major ones would appear to be: increasing disenchantment with the materialistic approach to the attainment of happiness, increasing alienation from oneself and one's environment, a quest for increased knowledge about one's true nature and one's relationship to the universe and the intrinsic urge to transcend oneself.

In terms of the psychology of consciousness, there are two general varieties of meditation (Ornstein, 1972); those exercises which involve restriction of awareness, focussing of attention on the object of meditation or on the repetition of a word or "mantra" (concentrative meditation) and those which involve a deliberate attempt to "open up awareness of the external environment". Many models are a blend of the two.

Th ultimate aim of meditation is to realise, through personal experience, the True Self or Transpersonal self but in addition to this ultimate aim, there are several intermediate aims like achieving a

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sense of well-being, harmony with nature, creativity, and promoting spiritual growth, by spiritual, we mean possessing values higher than average in the ethical, aesthetic, humanitarian and the artistic fields.

The transpersonal experience or realisation of the True Self has been well documented by Bucke (1923), Underhill (1911). These experiences, described by people in different periods and different cultures have some fundamental aspects in common like amazement, rightness, knowledge, unity, universality and social relevance.

The experience has an intrinsic existence of its own and is not a product of one's imagination. The 'Peak-experiences' described by Maslow (1962), belong to the same category. We have all had experiences, which may be considered rudimentary transpersonal experiences, where we have lost ourselves, may be, when we sat on a river-bank and the sound of the river flowed through us. At such moments, we are the experience itself, the flow, the moment in all its fulness. There is peace, harmony tranquility, the joy of transcending our individual selves and being part of the process. These moments bring a sense of perfection, of being one with the universe, of clarity, of being free of the tension, self-conscious thinking. These may be euphemeral moments, but they constitute the essence of meditation, and give us a glimpse of the transpersonal world. If we don't have access to these moments of awareness, more often, it is because the road is blocked by a traffic-jem of discursive thoughts. Still, the experience occurs more frequently than we imagine, in the general population (Wuthnow, 1978 and Hay, 1982). It has been claimed that those who meditate regularly have a greater number of transpersonal experiences than non-meditators.

Physiological and Psychological correlates of Meditation:

There seems to be general agreement that meditation does, in fact, induce a state of relaxation (Benson et al., 1974). During the act of meditation itself, certain physiological changes have been reported consistently, like reduced heart-rate (Wallace et al., 1971; Anand et al., 1961 and Wegner and Bagchi, 1961), decreased oxygen consumption (Wallace et al., 1971, Wegner and Bagchi, 1961), decreased Blood Pressure (Wallace et al., 1971; Wegner and Bagchi, 1961), increased skin resistance (Wallace et al., 1971; Wegner and Bagchi, 1961) and increased regularity and amplitude of alpha activity (Wallace et al., 1971; Wallace, 1970 and Anand et al., 1961). It has been hypothesised that these physiological changes during meditation produce a "hypometablic state" (Wallace et al., 1971). Goleman and Schqartz (1976) feel that mindful meditation helps in deconditioning of habitual response-patterns and altering the ergotropic-trophotropic ratio in the tropho-tropic direction. This diminution of sympathetic reactivity is believed to constitute the relaxation-response. The dehabitation effect of meditation seems to be confirmed by EEG studies on seasoned meditators. Kasamatsu and Hirat (1966) studies the habituation of the orienting response to a repeated click, both in ordinary people and zen masters. The subjects, in this experiment, sat in a sound-proof room and listened to a click repeated each 15 seconds, while EEG was being taken. The non-meditating subjects showed the customary phenomenon of habituation. The response of the brain's electrical activity to the clicks began to decrease after the 3rd or

4th click; the click had been tuned out of awareness. When the zen masters were meditating and were exposed to the same repetitive click over a period of 5 minutes, they did not show the customary habituation; they responded to the last click just as strongly as they did to the first. Meditation has the primary after-effect of "de automatisation", an opening up of awareness, a reduction of the normal selectivity of input. Deikman's (1966) experimental subjects, who gazed at a blue vase for half-an-hour at a time, for several sessions, reported that the vase appeared "more vivid" and "more luminous". That meditation is an altered state of consciousness is supported by the finding that seasoned meditators can influence the EEG wave-pattern associated with meditation-practice and it would seem that the evolution of higher states of consciousness, brings about a state of diminished brain-arousal, as suggested by the progressive appearance of beta, alpha and theta waves, and in some cases, even delta waves, as the depth of meditative state increases.

Several studies have looked at behavioural indices of attitude and perceptual change. Davidson et al. (1976) and Pirot (1975) have noted that meditators seemed to have greater auditory receptivity and perceptual discrimination that controls, as well as improved reaction-times and increased capacity to attend. There is reason to believe that meditation enhances intuitive cognition, associated as it is with a greater frequency of occurrence of Transpersonal experiences, as compared to the non-meditative state. Studies have also been directed to documenting perceptual and/or behavioural changes that occur at times other than during meditation, using Shostrom's personal orientation Inventory (Seeman et al., 1974; Venkatesh et al., 1989), Otis Descriptive Personality list and Otis Physical and Behavioural Inventory (Otis, 1974). All these studies report that meditators change more and "self-actualisation". These changes include such things as self-perceived increase in the capacity for intimacy, increased spontaniety, increased self-regard, increased acceptance of aggression; and increased inner-directedness. Goleman and Schqartz (1976) postulates that meditation's effectiveness in reduction of anxiety is due to "global desensitization". The individual, who learns to relax in meditation, and learns to witness in this relaxed state, the random flow of thought inluding anxiety-producing ones, learn to reciprocally inhibit the anxiety normally elicited by such thoughts.

Back at home, Desiraju (1987) and his team at the National Institute of Mental Health and Neuro Sciences, Bangalore, have been working deligently for the past ten years on the biochemical neurophysiological and psychological correlates of different types of meditation. There observations were :

(1) The changes observed in a given subject differed among different sessions, even though the meditational practice was of the same kind.

(2) Different individuals practising a given type of meditation, showed differences in the pattern of effect.

(3) These observations applied to all the parameters studied. Their impression was "that" a given meditative procedure probably helps not by producing a certain fixed type of change in a certain area like increase of alpha activity or decrease in sympathetic activity, but produces a pattern of changes, resulting out of optimising the balance of the homeostatic physiological controls, which would be to the best advantage of the particular individual".

The need for integrating the Western and the Estoeric Psychologies in understandeng consciousness and reality

If we have not been able to contribute more substantially to the body of knowledge in the fields of consciousness and meditation, it is probably because of our scientific conservatism, a tendency to adhere rigidly to our paradigms, and our reluctance to permit new inputs. We have not, for example, explored the possibility of integrating some parts of the estoeric psychologies of the East into our knowledge-base. There is evidence to suggest that of the main functions in two major modes of consciousness; one is analytic and the other intuitive or holistic and is complimentary to the other, but we tend to de-emphasize the a-rational, intuitive, non-verbal mode of consciousness. Again, we are easily led by our assumptions about reality, rather than consider that there might be alternative ways of conceptualising reality and several levels of consciousness. Instead of believing what we see, we are disposed to see what we believe. This has been well-illustrated by Bruner and Postman (1949) and his associates with their tachistocopic experiments. One of our shared social assumptions concerns the suit of playing cards. Through years of observation, we have learned that spades and clubs are black, while hearts and diamonds are red. Bruner asked his subjects to look at some cards through a tachistoscope, a device which allows visual materials to be flashed on a screen for an exact short time. Intermixed with the ordinary cards were several anomalous ones-a red ace of spades and a block four of hearts, for instance. Many of the experimental subjects did not "see" the visual cards, as they really were. Rather, they correctd the anomally, based on their assumptions and reported seeing a black spade of six and a red four of hearts! What applied to perception, applies also to our beliefs. Our traditional psychological models post a limited number of states of consciousness, and our usual waking state is believed to be optimal. No consideration is given to the possibility that states of consciousness may exist, that are more optimal than our waking one. On the other hand, meditation theories view our usual waking state as sub-optimal. The mind is seen as largely outside voluntary control, and as continuously creating a largely unrecognised stream of thoughts, emotions, images, fantasies and associations. These are held to distort our awareness, perceptual processes and our sense of identity to an unrecognised degree.

It is however, encouraging to note that psychologists of late, like modern physicists, are increasingly feeling that, to understand psychological phenomena, we have to give up our sectarian approach and adopt a general systems or bootstrap approach, according to which, there may not be any one theory capable of explaining the entire spectrum of psychological phenomena. We may have to be content with a network of interlocking models, using different languages to describe different aspects and levels of reality. One of the most comprehensive systems to integrate different psychological schools is the Spectrum Psychology of Wilber (1977). It unifies numerous approaches, both Eastern and Western, into a spectrum of psychological models and theories, that reflects the spectrum of human consciousness. Each of the levels or bands of the spectrum is characterised by a different sense of identity, ranging from the supreme identity of cosmic consciousness to the drastically narrowed identity of the personal self or ego.

Clinical applications of meditation:

At the therapeutic level, there has been a greater degree of interest and enthusiasm to explore the potential of meditation as a therapeutic tool or as an adjuvant to the established modalities of psychiatric treatment like psychotherapy. The attitude today is radically different from what it was in the psychoanalytic circle earlier. Freud (1962), for example, dismissed the oceanic experience in meditation as a reaction formation of omnipotence to infantile helplessness. Jung inspite of his sympathetic interest in eastern philosophies, did not see a therapeutic role for meditation. Alexander (1931) interpreted meditation as self-induced catatonia. The task force on Meditation, of the American Psychiatric Association (1977) felt that meditation can be considered as a method of inducing relaxation, and that it may be useful in psychotherapy and in reducing the need for psychotropic meditation.

Many studies have reported that meditation reduces anxiety both in Generalised Anxiety Disorder (Girodo, 1974; Shapiro, 1910; Vahia et al., 1972) and specific phobias, such as of enclosed spaces, examinations, being alone or heart attack (Bondreu, 1972; French and Tupin, 1974). Studies have also indication that drug and alcohol abuse may be reduced (Shafi et al., 1975; Shapiro and Zifferblaff, 1976; Benson and Wallace, 1972).

Meditation has been employed successfully in rehabilitation after Myocardial Infarction (Tulpule, 1971), in the treatment of bronchial asthma and insomnia and in the reduction of blood cholesterol levels and high blood pressure (Datey et al., 1969; Benson and Wallace, 1972; Stone and Deleo, 1976).

Kutz et al. (1985) have reported that meditation can be a useful adjuvant to psychodynamic psychotherapy, on account of the effects of meditation on mental processes, viz., ideas, images and objects being experienced in greater vividness, and carrying new meanings and different perspectives, plasticity of cognition due to non-linear and multidimensional handling of data, loosening of defenses, and emergence of repressed material including Primary Process material. Asagioli's Psychosynthesis (1985) employs many techniques which are basic in meditation-practice.

Meditation would appear to have preventive potentials, through its relaxing effect in stress-induced psychiatric disorders and in what Frankl calls "Noogenic Neuroses" characterised by dis-satisfaction with living inspite of having all the material satisfaction, a sense of meaninglessness in living a sense of alienation from oneself and one's environment. Many cases of "Middle age crisis" probably constitute sub-clinical varieties of this clinical entity. Meditation would also appear to have potential for promoting positive mental health, especially if we include the spiritual dimension in our concept of mental health. It is likely to promote the "Being" mode of living, postulated by Fromm (1976), as contrasted with the "Having" mode.

Employing the transpersonal model of a person, a new form of Psychotherapy, termed Transpersonal Psychotherapy has emerged in recent years (Walsh and Gaughan, 1980). In this form of Psychotherapy, consciousness itself rather than its contents, is made the focus of study. It assumes that our waking consciousness is a defensively contracted state, filled as it is, with a continuous flow of largely uncontrollable thoughts and fantasies, which exert a powerful, though unappreciated influence

on perception, cognition and behaviour. It also assumes that there are strata of consciousness, more optimal than our waking consciousness and these can be experienced in meditative practice and in transpersonal experiencess. The goals of Transpersonal Psychotherapy include the traditional ones like conflict-resolution and behaviour change and also work at the transpersonal level. This may include the provision of an adequate conceptual framework for handling transpersonal experiences, information on psychological potential and the importance of assuming responsibility, not only for one's behaviour, but also one's experiences. In addition to working through psychodynamic processes, the therapist aims to assist the client in disidentifying from and transcending psychodynamic issues and employs for this purpose meditational techniques.

Adverse effects of meditation:

Some meditators report hallucinatory experiences and/or derealisation depersonalisation experiences during meditation. These are usually benign and resolve spontaneously, and do not affect the normal functioning of the individual. Stray cases of psychiatric breakdown have been reported following meditation practice, and it would appear that these persons have had a fragile ego-structure or were Border-line schizophrenics or severe schizoid personalities. Hence it would be wise to undertake meditation practice under the guidance and supervision of an experienced meditator.

There is a danger that people with psychiatric problems are unable to cope with the stresses of day-to day living might take to meditation-practice, as a retreat from the problems of living and coping.

Those who undergo Transpersonal experiences may become victims of self-exaltation, self-glorification and develop delusional thinking or depression, when the normal waking state is resumed.

It should, however, be stated, that considering the larg-scale practice of meditation the number of adverse reactions reported in infinitesmally small the advantages would seem to outweigh the infrequent adverse effects.

Further research in the area of meditation-practice should address the following issues among other:—

1. Phenomenological analysis of meditation experience and transpersonal experience.
2. Determinants and significance of transpersonal experience, its frequency of occurrence in meditators and non-meditators.
3. Frequency of occurrence of disturbances of perception, orientation, and body-image during meditation, their determinants and significance; follow-up of such cases to determine what percentage go on to psychiatric breakdown and need psychiatric treatment.
4. A comparative study of creativity, spiritual growth, maturity, social adjustment and self-actualisation in meditators and matched non-meditating control groups.

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Psychology of Meditation and Health: Present Status and Future Directions

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Abstract

Past four decades has witnessed substantial scientific research on meditation as an alternative mind-body therapy. This paper is an attempt to provide a comprehensive view of the present state of the research in meditation and health. It reviews major findings related to meditation and its effects on various disorders. Two major types of meditation practices dominating presently (concentration and mindfulness) are introduced. Effects of meditation on human physiology such as heart beat, blood pressure, cortical activity, metabolism, respiration, and skin resistance are discussed. Impact of meditation on human perception and cognition is also addressed. Possible pathways or mechanisms through which meditation impacts health such as, relaxation, systematic desensitization, release of repressed memories, un-stressing and so on are also discussed. Finally, major conceptual and methodological issues that need serious attention from researchers in this area for future research is addressed.
Key words: Meditation, health, concentration, mindfulness.

Resumen

Los últimos cuatro decenios han presenciado importantes investigaciones científicas sobre la meditación como una alternativa terapéutica. Este artículo intenta brindar una visión global del estado actual de la investigación sobre meditación y salud. Examina los principales hallazgos relacionados con la meditación y sus efectos sobre diversos trastornos. Se presenta una introducción a los tipos principales de prácticas de meditación dominan actualmente (la concentración y el *mindfulness*). Se tratan los efectos de la meditación sobre aspectos de la fisiología humana como el ritmo cardiaco, la presión arterial, la actividad cortical, el metabolismo, la respiración, y la resistencia eléctrica de la piel, y se trata el impacto de la meditación sobre percepción y cognición humanas. Se discuten las posibles vías o mecanismos mediante los cuales la meditación impacta en la salud como son la relajación, la desensibilización sistemática, la liberación de recuerdos reprimidos y la disminución del estrés. Por último, se mencionan las principales cuestiones conceptuales y metodológicas que necesitan la atención de los investigadores para futuros estudios en esta área.
Palabras clave: meditación, salud, concentración, *mindfulness*.

The use of meditation for healing and enlightenment is not new. The practice of meditation has been prevailing throughout the human history among diverse cultures. In fact, all religious traditions practice some forms of meditation. It is generally

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associated with healing, spiritual growth, and enlightenment. After its introduction to the western world by Indian spiritualist Paramahansa Yogananda in 1920, the nature of scientific investigation of spiritual beliefs and practices underwent a drastic change. However, it was only during the 1960s that scientific studies started focusing on the clinical effects of meditation on health after the reports of extraordinary feats of bodily control and altered states of consciousness by eastern yogis reached the west. These reports captured the interest of many western behavioural scientists. With the scientific advancement and refinement in instrumentation, scientific study of effects of meditative practices became possible. A formal acknowledgement of the academic curiosity within psychology came in 1977, when the American Psychological Association issued a statement on meditation stating that-"meditation may facilitate the psychotherapeutic process." They also encouraged research "to evaluate its possible usefulness" (Kutz, Borysenko, & Benson, 1985, p.1). As a result, both health care professionals and lay people embraced meditation as a valuable tool for stress reduction and a device for healing both mental and physical disorders.

The word "meditation" is derived from the Latin *meditari*, which means "to engage in contemplation or reflection." The word meditation comes from the same Greek and Latin root as the word medicine. Manocha (2000) described meditation as a discrete and well-defined experience of a state of "thoughtless awareness" or mental silence, in which the activity of the mind is minimized without reducing the level of alertness. Walsh and Shapiro (2006) defined meditation from cognitive and psychological perspective, as a family of self-regulation practices that aim to bring mental processes under voluntary control through focusing attention and awareness. Other major descriptions of meditation emphasize components such as relaxation, concentration, an altered state of awareness, suspension of logical thought processes, and maintenance of self-observing attitude (Craven, 1989). Thus, meditation has been conceptualized in many ways and there exists no consensus definition. It is very difficult to capture its essence in one definition. However, Cardoso *et al*. (2004) developed an operational definition encompassing both traditional and clinical parameters. They defined any practice as meditation if it (1) utilizes a specific and clearly defined technique, (2) involves muscle relaxation somewhere during the process, (3) involves logic relaxation (i.e., not "to intend" to analyze the possible psychophysical effects, not "to intend" to judge the possible results, not "to intend" to create any type of expectation regarding the process), (4) a self induced state, and (5) the use of a self-focus skill or "anchor" for attention.

Types of meditation

Presently many meditation techniques are being practiced. However, all of them can be grouped into two basic approaches- concentrative meditations and mindfulness/ insight meditations. *Concentration meditation* aims at single pointed focus on some sound, image or sensation to still the mind and achieve greater awareness. Most popular form of this meditation is "transcendental meditation"(TM) developed by Maharshi Mahesh Yogi in 1958. TM is generally done by focusing the mind on some *mantra* (sound) to achieve transcendental state of consciousness. *Mindfulness meditation* on the other

hand involves opening up or becoming more alert to the continuous passing stream of thoughts, images, emotions and sensations without identifying oneself with them. Such practice helps in developing non-reactive state of mind, which is the foundation for calm and peaceful state of consciousness. Here instead of narrowing the focus (concentration) practitioner becomes alert to the entire field of consciousness. Vipassana and Zen meditations belong to this category.

Mikulas (1990) propounded the classification of meditative practice into four components- form, object, attitude, and behaviours of the mind. *Form* refers to the setting of meditation and the activity of body during the meditation, whereas *object* refers to object of one's attention during the meditation. *Attitude* is the mental set with which one approaches meditation. *Behaviours of mind* connotes whether the meditation is based on concentration or mindfulness.

Effects of Meditation: Physiological

Even though meditation is a mental activity, its effects on human physiology has received much attention. Few Indian studies have attempted to examine and establish the neurological correlates of yogic practices, including meditation, (Bhushan, 2002, 2004; Ramamurthi, 1977; Varma, 1979) the development of brain imaging technique has thrown the ball in the Western court. The unavailability of adequate infrastructure and needed scientific temperament within the country has made very few centers move parallel to the Western labs. Some of physiological effects are summarized here.

Heart rate

Studies have indicated that heart rate slows down during quite meditation and quickens in the moments of ecstasy during meditation (Tamini, 1975). Meditations like TM, Zen, relaxation response and other calming forms of meditation generally decrease the rate of heart beat (Bono, 1984; Delmonte, 1984a). However, very pronounced decrease in heart rate is found among long term practitioners only.

Blood pressure and hypertension

Blood pressure is one of the easiest measurable physiological variables. There is strong evidence that meditation lowers blood pressure for the people who are normal or moderate hypertensive (Sears & Raeburn, 1980; Swami Karmananda Saraswati, 1982; Wallace *et al*., 1983). However, most studies indicate that the benefit disappears once practice is discontinued (Patel, 1976).

Cortical activity

Evidence from many studies indicates that during meditation alpha activity increases significantly (Delmonte, 1984a; Daniels & Fernhall, 1984). Alpha waves are slow and high amplitude brain waves with frequency ranging from eight to thirteen cycles

per second. Alpha activity is generally indicator of deep relaxed state of mind. Long term meditation practitioners also exhibit theta brain wave activity (five to seven cycle per second) during which they report peaceful and pleasant experience with intact self awareness (Jacobs & Luber, 1989; Delmonte, 1984a). Research also indicates that during meditation right brain activity increases (Pagano & Frumkin, 1977). Delmonte (1984b) reported that meditation practice may begin with left-hemisphere activity, which then shifts towards the right hemisphere, while in advanced meditation both left- and right-hemisphere activity are largely inhibited or suspended. Schwartz (1975) pointed that meditation practices can lead to heightened cortical arousability and decreased limbic arousability, which lead to heightened perception and reduction of emotional activity. Goleman (1976) reported that meditators showed a significantly increased cortical excitation during meditation and a simultaneous limbic inhibition.

One of the recent research performed on the monks of *Dharamsala*, Himanchal Pradesh, have proved that their prefrontal lobes is lit even when they are not meditating (Davidson *et al*., 2003). This area is responsible for positive emotions. Lou *et al*. (1999) found significant changes in the blood-flow in the cortex during *yoga nidra*. Using H2O-PET they found a decreased flow in the executive system of the brain, i.e., dorsolateral prefrontal, anterior cingulate, orbital frontal cortices, striatum, thalamus, brain stem and cerebellum. Lazar, Bush, Gollub, Fricchione, Khalsa, and Benson (2000) studied 5 subjects who had practiced *Kundalini* meditation daily for at least four years. A fMRI record was made while the participants passively observed their breath and silently repeated Sanskrit phrases during inhalations and exhalations during the session. During the meditation process an increased fMRI signal was recorded in the putamen, midbrain, pregenual anterior cingulated cortex and the hippocampal/ parahippocampal formation suggesting that the neural activity evolve during meditation practice and are dynamic.

The profound effect of Transcendental Meditation on the brain function is reflected in the form of increasing degrees of orderliness, integration and coherence. Several studies in the recent past confirm a unique style of brain functioning in enlightened people (Travis, 2001; Travis & Orme-Johnson, 1989; Travis & Pearson, 2000; Travis, Tecce, & Guttman, 2000). While a relative excitement is continuously present in the brains of non-enlightened subjects, the enlightened people maintain a low level of excitation until s/he confronts the very moment when it is appropriate to make a decision. This exactly matches with their subjectively felt and narrated experience of persistent immovable inner calmness, even while engaged in dynamic outer activity. Even though such empirical findings are coming now, it has always been advocated in the Indian religious/ philosophical texts and narrations. Attempts have also been made for MEG recording during transcendental meditation practice.

Litscher, Wenzel, Niederwieser, and Schwarz (2001) used TCD (Transcranial Doppler Sonography) and nearinfrared spectroscopy to measure oxygenation levels in the cerebral tissues of two QiGong (a Chinese meditation exercise) experts. During meditation the mean blood flow increased in the right posterior cerebral artery and decreased in the left middle cerebral artery. A simultaneous increase in oxyhemoglobin and total hemoglobin was also recorded. During QiGong predominant EEG activity was witnessed in the anterior half of the brain while it silently occurred in the posterior

half. This can be considered the cerebral *ying* and *yang*. Attempts are also being made to understand the relationship between neurotransmitters and meditation. The dopaminergic system seems to play an important role in the suppression of executive system during relaxation meditation.

Metabolism and respiration

Many studies have shown that during meditation oxygen consumption is reduced (sometime up to 50%), carbon dioxide elimination is reduced (sometime up to 50%) and respiration rate is lessened (Sudsuang, Chentanez, & Veluyan, 1991; Kesterson, 1986).

Skin resistance

Low skin resistance (measured in terms of galvanic skin response) is a good indicator of stress. As expected high skin resistance has been documented by many researchers especially among TM practitioners (Bono, 1984; Bagga & Gandhi, 1983).

Effects of meditation: Psychological

Many perceptual and cognitive abilities are associated with meditation practices. These abilities may range from normal to paranormal. Many scientific investigations have been conducted to measure various psychological and behavioral effects of meditation.

Perceptual ability

Brown, Forte, and Dysart (1984a, 1984b) conducted experiments on visual sensitivity among Buddhist meditation practitioners using before-after and control group design. Post test was conducted after three months of rigorous meditation practice. Visual sensitivity was measured by detection threshold and discrimination threshold using simple light flashes. They reported significant improvement in visual sensitivity after the meditation retreat. Other studies also reported similar decrease in visual threshold and increased auditory acuity after the meditation (McEvoy, Frumking, and Harkins, 1980; Keithler 1981). Other studies reported increased visual imagery abilities (Heil, 1983), enhanced attentive ability (Linden, 1973), reduction of perceptual noise (Walsh, 1978), increased reaction time (Robertson, 1983), and enhanced perceptual motor speed (Jedrczak, Toomey, & Clements, 1986).

Memory and intelligence

Jedrczak *et al*. (1986) reported that number of months of TM practice predicted the higher performance on nonverbal intelligence test. Other researchers also reported similar results of improvements in cognitive abilities (Verma, Jayashan, & Palani, 1982). Studies on TM practitioners generally reported to have positive impact on intelligence, school grades, learning ability, short and long term memory (Cranson *et al*., 1991).

Creativity and self actualization

Mixed results have been reported regarding creativity and meditation. TM researchers in particular reported enhanced creativity with TM practice (Ball, 1980; Orme-Johnson & Granieri, 1977). However, other researchers could not find any relationship between meditation and creativity (O'Haire & Marcia, 1980; Domino, 1977).

Self actualization is thought to be the major goal of dedicated meditation practitioner. Various studies have measured several aspects of self actualization and experiences in meditation. Alexander, Rainforth, and Gelderloos (1991) performed a meta-analysis on 42 studies on the effects of TM and other forms of relaxation on self actualization. Their measure of self actualization included three independent factors: affective maturity, integrative perspective on the self and world, and resilient sense of self. They found effect size of TM on self actualization is approximately three times larger than other forms of meditation and relaxation practices. Other studies also reported increase in various dimensions of self actualization with meditation (Gelderloos, Walton, Orme-Johnson, and Alexander, 1990).

Mechanisms for the therapeutic effects of Meditation

Helminiak (1981) described six possible mechanisms through which meditation works. These six mechanisms capture most of the explanations found in the existing literature. They are:

(1) *Relaxation:* Relaxation is one of the primary components of all kinds of meditation which induce a pleasant and deep relaxed state of body and mind. Herbert Benson (1976) developed a therapeutic technique called "relaxation response" which is a form of meditation. His whole concern with meditation was to reduce stress and hypertension by inducing a state of deep relaxation. Benson (1976) measured series of physiological parameters in response to relaxation response. Various effects include-decrease in the rate of metabolism, decrease in the rate of heart beat, muscle relaxation, slow and rhythmic breathing, decrease in blood pressure, and so on. All this effects help in balancing physiological abnormalities and promotes healing.

(2) Systematic desensitization: Joseph Wolpe's (1961) behavioural therapy is especially instrumental in reducing anxiety. This therapy involves three steps. First, the client is thought to induce a deep state of muscle relaxation. This is followed by preparing a hierarchical list of stimuli inducing anxiety. Finally, in a deep relaxed state client confronts (either by imagination or by presentation of actual stimuli) each of the anxiety producing stimuli progressing in hierarchy. This therapy is based on the principle of reciprocal inhibition. Since anxiety and relaxation are incompatible to each other, the stimuli loose their anxiety provoking quality. Client continues this process until he is desensitized to highest item in the hierarchy.

In meditation also a practitioner undergoes similar steps. Every meditation involves induction of a relaxed state. In meditation, the practitioner first enters in deep relaxation and suspends conscious thoughts by either detached observation or concentration. As a result, many anxiety provoking repressed memories, thoughts, and feelings arise in the mind. When one confronts them in a deeply relaxed state, these factors loose their power to induce anxiety and finally get eliminated.

(3) Release of repressed psychic material: This is related to systematic desensitization. With regular practice of meditation, most of the practitioners encounter release of repressed unconscious thoughts, emotions, and images (Schwartz, 1974). This is very similar to the release of unconscious phenomenon during free association in psychoanalysis. This could be initially disturbing, but with constant practice unconscious mind gets cleaned of such memories and healthy mind is achieved. During meditation, the practitioner remains under low arousal and sensory deprivation for a long time and under such condition repressed feelings and thoughts arises (Benson, 1976).

(4) Unstressing: Parallel to release of repressed memories, many practitioner reports many physical reactions during intense meditation. This could be involuntary muscular-skeletal movements such as repeated twitches, spasms, gasps, tingling, tics, jerking, swaying, pains, shaking, aches, internal pressures, headaches, weeping, and laughter. The experience covers the range from extreme pleasure to acute distress (Goleman, 1971). TM practitioner calls this as *"unstressing"*. Goleman (1971) interprets this phenomenon on the basis of psycho-physiological principle contemplating that all psychic and emotional phenomena have parallel physiological processes.

(5) Dissolution of habitual patterns of perception: Human beings are mostly governed by rigid and fixed patterns of thinking, feeling, and reactions. Many of these patterns are unhealthy and cause neurotic and psychotic problems. Most of the unhealthy habitual patterns are due to our identification with emotions that we are not able to control and regulate. With detached observation, emotions and thoughts loose their power and practitioner is able to identify the unhealthy patterns of behavior and remove them with healthy ones.

(6) Cosmic consciousness: Attainment of cosmic consciousness is a mystical concept and not available for scientific investigation. Many terms are used to represent cosmic consciousness such as *samadhi*, *nirvana*, *satori*, and *moksha*. It is the highest goal achieved by meditation in esoteric traditions where a person transcends his personal ego. In the state of cosmic consciousness a person realizes that he/ she is one with the whole cosmos and is not separate from others. As a result, a tremendous sense of love and compassion arises in him and it is the highest state a human can achieve.

Meditation and Health: Review of present status

Effects of meditation on health are based on the principle of mind-body connection. Meditation practices are generally accepted as mind-body treatments for health related problems and overall well-being. There is a growing body of literature showing the efficacy of meditation on various health related problems. Meditation is reported to be effective in pain management and enhancing immune system (Kabat-Zinn, 1990). Studies on the long-term mental benefits of meditation show that meditation reduces stress and increases reported levels of happiness, self-confidence, and general effectiveness (Fergusson, Bonshek, & Boudigues, 1995; Hawks, Hull, & Thalman, 1995). Meditative interventions have been found to be beneficial in treating various clinical conditions. These include- hypertension (Barnes *et al*., 1997); cardiovascular disorders (King, Carr, & D'Cruz, 2002); pain syndromes and musculoskeletal diseases (Astin, 2004); respiratory disorders such as asthma, congestive obstructive pulmonary disease (Wang, Collet, & Lau, 2004); dermatological problems such as psoriasis, allergies (Bilkis & Mark, 1998); immunological disorders (Astin *et al*., 2003) and treatment-related symptoms of breast

and prostate cancer (Coker, 1999). Several Indian researchers have also made significant contribution. Their study range from anxiety (Jangid, Vyas, & Shukla, 1988; Sharma & Agnihotri, 1982), psychosomatic disorders (Divekar, 1982), neurotic disorders (Naug, 1975; Nagarathna & Nagendra, 1980) and stress (Sethi, Trivedi, & Anand, 1981). Studies suggest that intervention program using meditation is helpful in reducing headache as well as pain in neck, extremities and joints (Brendstrup & Launso, 1993). Decline in the use of tranquilizers, antirheumatics, and gastrointestinal agents have also been reported after meditation practices (DeBerry, Davis, & Reinhard, 1989; Ganguli, 1985; Singh, 1992). Further, Sethi (1989) has argued that meditation is a key coping mechanism for the problem of burnout in organizations. Various studies also reported the benefits of TM meditation in de-addiction from chemical substances (Gelderloos *et al*. 1991), and smoking (Royer-Bounouar, 1989).

Classic distinction suggests as many as 14 different forms of mediation (Goleman, 1978). However recently, University of Alberta Evidence-based Practice Center, Canada, conducted an exhaustive review of studies on meditation and its effects on health. This review was conducted on studies up to 2005. They identified five categories of meditation practices:

(1) *Mantra meditation* (Comprising TM, relaxation response, and clinically standardized meditation),
(2) *Mindfulness meditation* (comprising Vipassana, Zen Buddhist meditation, mindfulness based stress reduction, and mindfulness based cognitive therapy),
(3) *Yoga* (This is based on Indian Yogic tradition developed by Patanjali. It incorporates various techniques like body postures, breath control and meditation.)
(4) *Tai Chi* (This is a Chinese martial art that incorporates various slow rhythmic movements that emphasize force and complete relaxation. It has been also called "meditation in motion.")
(5) *Qi Gong* (This is an ancient Chinese practice that combines breathing patterns with various physical postures, bodily movements, and meditation.)

This review was conducted on 813 studies. However, they reported most of the studies in this area are poor in quality. Most commonly studied disorders were hypertension, cardiovascular diseases, and substance abuse. Results of meta-analysis conducted on hypertensive participants showed that TM, Qi Gong and Zen Buddhist meditation significantly reduced blood pressure. Other findings indicated that Yoga contributed in stress reduction. However, Yoga was no better than Mindfulness-based stress reduction at reducing anxiety in patients with cardiovascular diseases. No clear picture from the studies on substance abuse was obtained. Meta-analyses on 55 studies indicate that some meditation practices produced significant changes in healthy participants. What is alarming is that the role of effect modifiers such as age, gender, duration of practice and so on has been largely neglected in the scientific literature. It is clearly evident from the current literature that therapeutic effects of meditation is uncertain and inconclusive and more rigorous studies are needed in future to get firm conclusions. Earlier also Delmonte (1986) had concluded that meditation as an intervention strategy was successful with anxiety and hypertension, but its effectiveness is doubtful in the treatment of other disorders.

Major Issues and Future Directions

Reports of the review study on meditation and health by University of Alberta Evidence-based Practice Center clearly indicates that the present state of research does not give clear evidence regarding the efficacy of meditation as a therapeutic tool, primarily because of lack of quality studies. Till date there is no consensus operational definition of meditation that can be employed by diverse studies. Design of the studies is also a major issue in this area. Most studies employ before-and-after design which does not provide clear causal inference. So, using control group is imperative for drawing causal inference. Apart from that, consideration of effects of various confounding variables (effect modifiers) has to be taken into account. Various confounding variables such as age and gender of the practitioner, duration of the meditation, and so on can have profound effect on the results. These effect modifiers have generally been neglected in studies. Employment of large sample should also be an important consideration. Most of the present studies have utilized small sample which limits their generalization.

Beside these methodological considerations, there are great possibilities of biasness in this area. Researchers have been talking consistently about the benefits of meditation but one should be open to the possibility of negative impact of meditation also. Meditation is an important tool for the release of repressed emotional contents and this could be very disturbing to initial practitioners. Encountering such disturbing emotional contents could have adverse impact on the health of the practitioner, unless proper guidance is provided. For example, Walsh (1979) reported a number of disturbing experiences encountered during meditation, such as anxiety, tension, and anger. Researchers also posited that meditation may precipitate a psychotic episode in individuals with a history of schizophrenia (Walsh & Rauche, 1979). So, there is possibility for researcher's biasness, especially when one has preconceived notions of benefits of meditation.

Meditation is not only a technique, but also an art. Some people are predisposed towards it while others are not. Some can delve deep into the meditation and acquire all benefits while others may come out more disturbed. Such individual differences should also be considered while making any conclusion regarding the benefits of meditation. It appears that meditation may have therapeutic value, but limited to those who are psychologically healthy, well integrated and may have mild neurosis or psychosomatic disorders. These issues need serious attention from researchers in future to get firm conclusion regarding the efficacy of meditation as an adjunct to mind-body therapy.

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Shavasana

The human body and its functions are regulated through nervous system consisting of brain, spinal cord and nerves. The nervous system has two distinct parts : one part is under voluntary control and most of the ordinary functions of the body like walking, doing work manually, etc. fall under the category of voluntary control. Vital functions of the body like respiration, circulation, digestion, etc. are involuntary.

The human body has developed some self-regulatory mechanisms for its survival. In case of fear, the sympathetic nervous system is activated which prepares the Individual either to fight or run away from the fear (flight). As the danger disappears the parasympathetic nervous system comes into action. This calms down the individual. In real emergency situations, stimulation of sympathetic activity is helpful to "fight". Hence it is known as sympathetic. However, when it remains stimulated during chronic stress and in an imaginary fearful situations, it is a drain on the energy of the individual and proves harmful. Shavasana and meditation activate the parasympathetic nervous system which makes secretion of hormones like adrenaline, noradrenaline and corticosteroid normal. As a

result, the heart rate, blood pressure and blood sugar become normal. Blood becomes thin and the arteries dilate. The formation of lactic acid is also reduced which results in retardation of the aging process. Endorphins and neuropeptides are secreted from the brain. This brings peace and joy. It is noted that when an individual achieves this relaxation, slow rhythmic alpha activity is recorded from the frontal lobe of the brain. Various chemical changes occurring in the body promote healing by increasing the immune status of the individual. Thus all the degenerative processes of the body are retarded and regenerative processes get accelerated.

Shavasana has been practised from the ancient times in different ways. It has been considered the king of asanas. Physical and mental relaxation achieved by shavasana has no equal. During shavasana slow rhythmic alpha activity is recorded from the frontal lobe of the brain, and various chemical changes occurring in the body promote healing by increasing the immune status of the individual.

The purpose of this asana, as the name suggests, is to assume a dead-body posture. There are various techniques to perform this asana. The one adopted in the program of Dr. Dean Ornish has been evolved by Swami Satchidanandaji settled in Virginia, USA. It relies on the premise that for optimum relaxation of muscles of the body, prior stretching of muscles is necessary. Experience has shown that if relaxation is preceded by stretching,

it yields optimum results. A very significant point to remember is that while stretching a particular group of muscles, it is not correct to believe, the more the stretching the better the relaxation. The stretching must be just enough to create a pleasant sensation. The awareness of an individual doing this exercise must be focused on the process of stretching and relaxing. The simple observation that the awareness must be brought on to the part of the body being stretched is the cardinal feature of the technique, without which the benefit of shavasana is not complete.

The world abounds in material wealth. But that does not make you prosperous. Shavasana and meditation explain scientifically how you could exercise your optimum capacity for efficient and productive action. Your present actions are not tuned for maximum output. That is because of lack of mental concentration. As you are acting in the present, your mind rambles into the past and fantasises about the future. It broods over memories of what has been done earlier or over expectations of results to come. Your mind hardly remains in the present. This is the quality of human mind. Shavasana and meditation provide you with the ways and means of controlling the mind. They help you to concentrate, to channelise your mind towards the present activity. Shavasana and meditation prevent the dissipation of your mental energy in unproductive channels of the past and the future. Your activities are thereby rendered

more efficient, more fruitful, more productive. They provide a blue-print for efficiency, productivity and prosperity.

The practice of shavasana prepares the individual for meditation. Shavasana, as we know, stems from the progressive deep relaxation. It is achieved by prior stretching and relaxation of the group of muscles one by one as shown by the various stretching and relaxation exercises. Indeed, bringing of awareness to the present, that is, to the process of stretching and relaxing is of crucial importance. As the Father of medicine Hippocrates has observed, the mind is present in all the cells of the body and so when the body is relaxed during shavasana, the mind too automatically relaxes. This is the essence of shavasana.

Science has established that our awareness is, for most of the time, either in the past or in the future. Out of 16 hours of our waking time, it is hardly for an hour and a half or so that we live actually in the present. It is well-known that stress is minimum when awareness is in the present. In shavasana, by various stretching and relaxation exercises of the body and keeping the awareness on the process, progressive deep relaxation is achieved.

Moderate stretching and relaxation exercises followed by shavasana and meditation bring about the desired relaxation with all the helpful changes, viz. increased secretion of endorphins, decreased formation of lactic acid and reduction in secretion of noradrenalin and adrenalin. Balance between

parasympathetic and sympathetic or right and left brain will promote health. The right brain stands for intuitive instinct and the left brain for hard logic. Briefly, in relaxation mode parasympathetic tone is more than sympathetic, The right brain is more active than the left, while in the stress mode the reverse occurs.

When a complete relaxation of the muscles of the body is achieved, the individual brings his awareness to the natural process of breathing. He now becomes a witness to his own breathing process without any special effort to breathe in a particular manner. It is only the natural process of inhalation and exhalation that is witnessed. This brings about relaxation and the peace of mind. It is precisely at this stage, when the individual's awareness gets focused on the natural process of breathing, that the suggestions are given that all the muscles of the body are now relaxed, and hence the coronary arteries which are made up of muscles are also relaxed: This leads to dilatation of the arteries and increase of blood flow to the heart muscles. It improves the functioning of the heart, which in turn increases the perfusion of all the cells of the body with the result that the individual feels very energetic and well.

All the advice in management of CHD (coronary heart disease) regarding hygienic measures like regular walk, giving up tobacco, moderation in use of alcohol and reduction in dietary fat and animal protein are all physical

measures. They do not deal with the root causes of stress like sense of isolation, anger, hostility and fear of death.

The yogic disciplines, abdominal breathing, shavasana and meditation increase the inner strength of the individual and foster the feeling of connectedness bringing about reduction in stress.

Shavasana helps you develop spiritually. It also prescribes the ways and means to gain material prosperity. Puts you right on top of the world.

In essence shavasana and meditation go hand in hand. The physical aspect of shavasana is only superficially physical. It actually serves the purpose of bringing about richer and more successful meditation.

Moreover recent research has shown that shavasana and meditation increase the heart rate variability.

Heart-Rate Variability

HRV is not cardiac arrythmia or irregular heart beats; rather it is a variation in the sinus rhythm itself as it changes from beat to beat. This beat-to-beat time variation is in milliseconds and cannot be appreciated in a routine electro-cardiogram. In a normal person the HRV is rhythmic with a value of more than 50 milliseconds on an average between beats. It can be captured only by advanced sensitive computers. Negative emotions like anger,

frustration, envy, worry, etc. cause erratic and nonharmonious HRV. Positive emotions like caring-love, appreciation and compassion allow smooth, even and harmonious HRV. The disturbance in HRV is a sensitive index of mind and body malfunction. Erratic HRV puts extra wear and tear on blood vessels and heart. HRV has corresponding changes in cellular DNA and immune system also. Increased sympathetic activity with dampening of parasympathetic pathways diminish heart rate variability and also make it erratic and nonharmonious. A Multicentre Post Infarction Research Group Study found that low HRV, less than 50 milliseconds, has 5.3 times higher chances of sudden death in the patients recovering from heart attack. The yogic disciplines such as abdominal breathing, shavasana and meditation admirably bring about a reduction in the sympathetic activity and have beneficial effects on heart rate variability.

Heart Resonance Imaging

Heart Resonance lmaging is a technique to bring beneficial effect on Heart Rate Variability. With Heart Resonance Imaging, you replace the conditioned reflex of anger with the reflex of caring-love. This cannot be done as a statement, declaration, or pep talk. It has to be developed as a biological reflex. If you diligently practise over 24 hours, you will be able to develop the new reflex.

If you stay with it everyday for a week, you will master the new reflex. You will enjoy the unconditional caring love state for the second time in your life – the first being when you were a child. **The Heart Resonance Imaging steps are simple.** During meditation :

- Focus awareness on your anatomical heart.
- Experience caring-love towards a particular person.
- Stay anchored to your heart.
- Observe your breathing in your belly as you stay anchored to your heart.
- Repeat these drills until you are able to shift your centering from your mind to heart and to belly effortlessly.

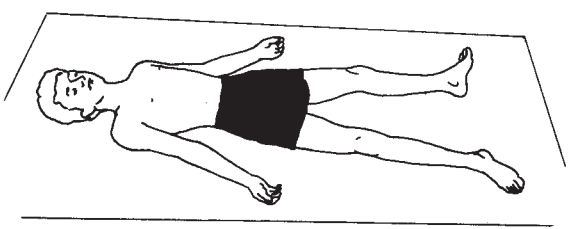
Benefits of HRI (Heart Resonance Imaging)

- Your heart heals as shown by increase in heart rate variability.
- Your immune system becomes stronger.
- You resolve conflicts in your mind and you feel better.
- You feel less stressed and more energized.
- You develop biologically effective altruism. This is healing by your internal pharmacy. You upgrade your emotions from apathy to empathy and from anger to caring-love. HRI may be practised whenever one interacts with another person.

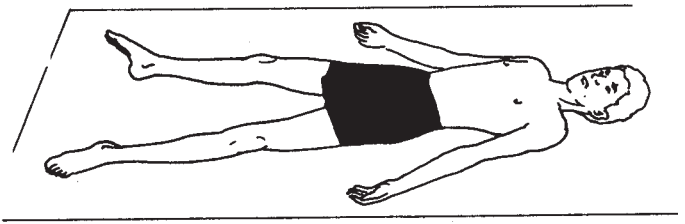
Technique of Shavasana

Normally to practise shavasana one lies down flat on his back on a mat on a floor or on a mattress on a cot. However, shavasana can be practised even in a sitting posture. In our program, quite a few have practised it while sitting in a wheel chair. Practice of shavasana begins with bringing one's awareness onto one's body. Normally our mind is wandering. Bringing the awareness to the body and keeping it there, is the first vital step to control the wandering mind. With practice one can bring the awareness onto the body very gently and if the mind wanders, without undue force gently bring it back to the body. The benefits derived from shavasana depend upon the adherence to the simple guidelines explained here.

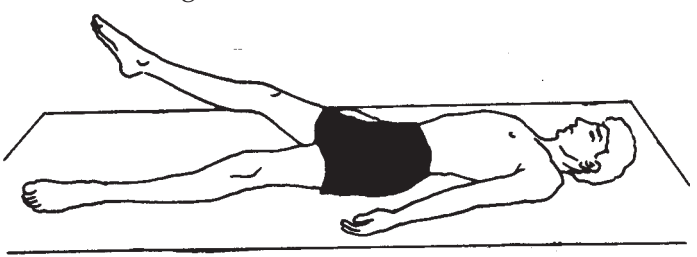
Lie on your back. Keep your feet 12" apart. Let each foot lie on its side in a comfortable position. Keep your arms 6" away from the body with your palms facing the sky and fingers half bent.



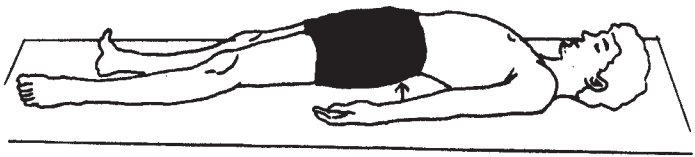
Bring your awareness to the feet and bend the toes towards the ground, feel the stretch and then relax. Now forget the toes. Do this exercise again.



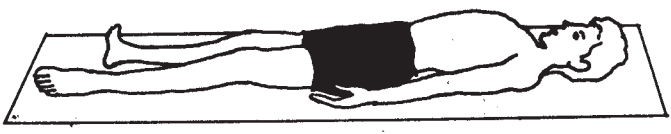
Bring your awareness on your right leg. Stretch the muscles of the right leg from toes to hips, lifting the leg a little off the ground. Feel the stretching of the muscles. Then relax. The leg will automatically fall to the ground. Now forget the right leg and repeat the same with the left leg. Do this exercise again.



Bring your awareness on your back. Contract the muscles of the lower back and pelvic region (around the hip joints). Body will rise slightly off the ground. Feel the stretch, then relax. Body will come down automatically. Do this exercise again.



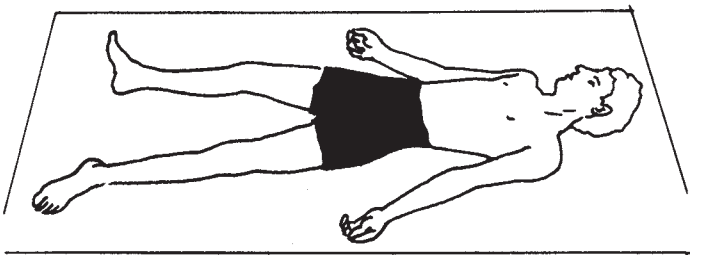
Bring your awareness on your chest. Contract the muscles of the chest while inhaling and raising the chest a little off the ground. Feel the stretch and then relax. Chest will come down automatically. Do this exercise again.



Bring your awareness on your arms. Close the fists of the arms. Contract the muscles of the whole arms up to the shoulder lifting the arms a little above the ground. Feel the stretch and then relax. The arms will come down automatically to the ground. Repeat the same exercise.

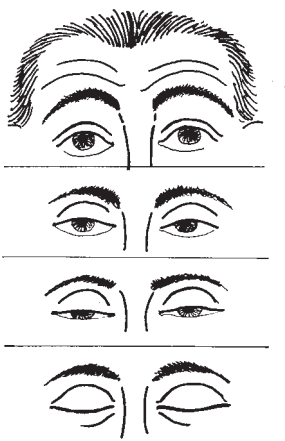


Rotate the neck from one side to another side gently as if somebody is holding the neck and moving it. Feel the stretch and then relax keeping the neck in a comfortable position. Do this exercise again.



Bring your awareness on your jaws. Open the lower jaw towards the chest and experience the stretching of the muscles of the jaws and the cheeks. Feel the stretch and then relax, bringing the jaw back to the normal position. Repeat the exercise.





Open the eyes gently and fully, stretching the muscles of the eyes, brows and the forehead. Feel the stretch. Then relax these muscles, letting the eyes close effortlessly. Do this exercise again.

Having relaxed the whole body, the practitioner remains in the relaxed position forgetting his whole body, and brings his awareness to the natural process of breathing. Resting in the comfortable position he gradually becomes one with the breath, and remains in that state for a few minutes.

While he is one with his breath, he is meditating. **So in a nut-shell, shavasana starts with meditation on the body and ends with meditation on the breath.**

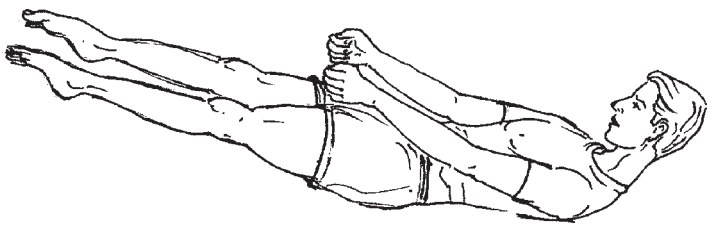
Quick Shavasana

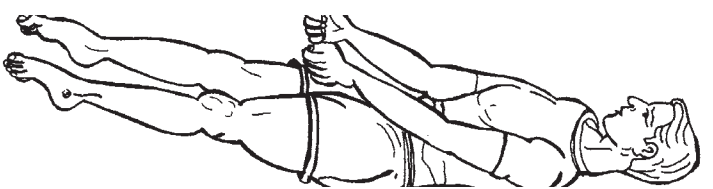
Swami Satchidanandaji of Virginia, USA has shown a technique to perform shavasana quickly in a short time.

The practitioner lies flat on his back in a relaxed position. Having brought his awareness to his body and gently keeping it there, he contracts the muscles of the whole body, raising his head a little off the ground and his arms and the lower trunk with both the legs also raised a little off the ground. He experiences the stretch in all the muscles for a few seconds and then relaxes them bringing the body back to position and experiences the relaxation in the whole body. The whole exercise is repeated once again.

In this relaxed posture, forgetting the body, the practitioner focuses his awareness to the natural process of breathing. Without making any effort to change breathing, he becomes a witness to every single breath that goes in through the nose during inhalation and every single breath that comes out during exhalation. This way he becomes one with his breath. He remains in this blissful state for two or three minutes. Then he takes in a deep breath and exhales slowly. He gently opens his eyes, makes some movement of his hands and feet, turns on his side and assumes sitting position.

The whole exercise would take less than five minutes but it refreshes the practitioner for a long time. It is easy to see that a complaint of lack of time to do this simple asana is irrelevant. Those who have complaint of back pain may not contract the muscles of the whole body. They may contract only the muscles of the hands and lower trunk, raising only the hands and the legs off the ground, keeping the upper trunk and head in resting flat position.





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EFFECTIVENESS OF SHAVASANA ON DEPRESSION AMONG UNIVERSITY STUDENTS

S. S. Khumar[1], *Paramjit Kaur*[2] *& Ms. Sarabjit Kaur*[3]

ABSTRACT

Present study was undertaken to examine the effectiveness of 'Shavasana' as a therapeutic technique to alleviate depression. Fifty female university students were identified as cases of severe depression with the help of two diagnostic tools (1) Amritsar Depressive Inventory, (2) Zung-Depression Scale. They were subjected to 30 sessions of 'Shavasana'. A control group design with pre, mid and post-assessments was used. Resuits revealed that (1) Shavasana was a useful technique even when it was used independently from other yogasanas, kiryas and pranayama, (2) continuation of the treatment for a longer period resulted in a significantly increased positive change in the patients.

Modern age is the age of stresses and strains. Due to the technological development the spiritual values have gone behind the value matrix and the material values have come to the fore. There is a change in the life styles making the people to run beyond their capacities, with the result that the diseases of life styles such as hypertension, liver cirrhosis, ulcers of stomach and heart attacks have become as common in developing and third wold countries as in the west.

Depression is an affective disorder which is used to describe a wide variety of clinical states varying from mild sadness of mood to psychotic depressive illness. It is also viewed as the total reaction of human being in the face of a traumatic psychological experience with or without some biological predisposition. Sadness is only one outward manifestation of wide underlying change in the individual's psychophysiological process (Singh et. al., 1974). Costello (1970) has reported that women are more prone to depression than men. The number of depressed women has been found to be almost double than men in urban areas.

Though drugs are effective in reducing or removing depression temporarily in most of the patients yet these should be replaced by non-drug therapeutic techniques as they are not advisable for patients with mild depression. These may have their side effects on some individuals due to their anticholinergic properties, such as, dry mouth, constipation, reduced gastric motility and secretions, cardiovascular side effects including tachycardia, hypotension, blurred vision and frequent tremors (Davis, 1980). Non-drug therapeutic tecnniques are, therefore, suggested for the treatment of depression. Some of these are transcendental meditation, biofeedback training, nada yoga (music) and yogasanas.

Yoga is a way of life which offers a

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released outlook on life. It provides mental and physical steadiness, good health and feeling of lightness. The release of muscle tension plays a significant role in the physiological responsiveness to stress by altering the level of general arousal (Grossman, 1973). As the asana is performed and maintained there is no increase in fatigue resulting from exercise (Kuvalyananda and Vinekar, 1968). There is an improved muscular fitness i. e. enhanced strength, tone, flexibility and work output (Moorthy, 1982-83; Gharote 1971a, 1973 and 1976b and Nayar et. al., 1975). Besides the benefits of yogasanas to the physical process the cognitive and affective functioning is also influenced positively by such practices.

Shavasana is a very important and highly powerful asana for curing anxiety and depression besides many other physical diseases. It removes mental tension, stress, physical fatigue, heart aliments, insomnia and many other nervous disorders. The practitioner lies on his back on the floor and is taught to have slow rhythmic diaphragmatic breathing with a long pause (two seconds) after each inspiration and a shorter pause (one second) at the end of each expiration. This procedure keeps the patient alert yet relaxed and also keeps his thoughts concentrated. The practitioner is required to relax all his body muscles and nerves and to leave himself as a dead man. This yogasana is performed for 30 minutes once or twice a day.

It has been observed that 25 to 30% of college undergraduates suffer from depression in America (Rosenhan and Sehginan, 1984). The situation in educational institutions in Inida is still worse. Therefore, finding out ways for its management has become very important. This is particularly true of the girls of very sensitive age group studying in the universities. Since depression creates problems in studies and the normal development of students, it becomes all the more essential to take up studies of this kind.

The main objectives of the present study were to see whether 'Shavasana' had curative value for cases of severe depression and whether duration of the treatment had any relationship with therapeutic gains.

METHOD

Subjects

The sample consisted of 50 cases of of severe depression identified with the help of two diagnostic tests : Amritsar Depressive Inventory (1974) and Zung Depression Self Rating Scale (1965), and personal interviews. These subjects were drawn out from unmarried postgraduate girl students living in the hostels of Punjabi University, Patiala. Their age ranged from 20-25 years. The cases diagnosed were without any organic ailment. The duration of depression was two to three months. It was ascertained that no subject was taking therapy from elsewhere in the form of drugs, counselling or group lectures.

Material Used

1. Amritsar Depressive Inventory (Singh et. al., 1974).
2. Zung Depression Self-Rating Scale (Zung, 1965).
3. Personal Interview Schedule.

Procedure

The subjects were randomly divided into two groups of 25 each. One of these

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groups was treated with yoga exercise, namely Shavasana which is considered to be the most effective method of relaxation that helps depressive cases to get rid of their suffering. The other group did not get any treatment and acted only as a control group. Shavasana was performed by each subject for half an hour daily for a period of 30 days in the morning at 7 O' clock. Each subject was to lie down flat on the floor with legs little apart and eyes and mouth closed. She was asked to relax all body muscles and to feel that her legs, arms and hips were getting heavier. She was further instructed to take the breath inside and release it after two seconds, feeling that all the body tension was being released with breath. The control group was added in order to see if the depression of the subjects was more or less stable and whether they needed some effective treatment for becoming normal. Some people believe that the time heals depressive symptoms. The authenticity of this vi ew was to be investigated by camparing the experimental and control groups. Both groups were evaluated three times during 30 days period: before commencement of the treatment, at the end of 15 days and after 30 days of treatment. The instructions were imparted by a trained yoga expert. The place for performance was the same for all individuals. The evaluation of the subjects after 15th and 30th day was made with Zung Depression Scale only.

RESULTS AND DISCUSSION

The Hartley test indicated that the assumption of homogeneity of variance of the data was upheld. Table 1 shows mean difference of depression scores on Zung Depresson Self Rating Scale at various stages of treatment in both experimental and control groups. The differences in the mean scores between pre and mid treatment, mid and post treatment, and pre and post treatment evaluations are significant at 0.01 level ('t'=6.51, 8.29 and 3.66 respectively). This indicates a significant decrease in depression scores on each stage of treatment in the experimental group. But non-significant differences were obtained in the control group.

Table 1. Mean Difference of Depression Scores at Various Stages of Treatment (N=50)

Group	Stages of Evaluation	Mean	SD	't'	P
Experimental	Pre Treatment	80.89	3.06	6.51	0.01
	Mid Treatment	69.30	8.18		
	Mid Treatment	69.30	8.15	8.29	0.01
	Post Treatment	57.52	13.49		
	Pre Treatment	80.89	3.06	3.66	0.01
	Post Treatment	57.52	13.49		

(Contd.)

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Control	Pre	Treatment	80.02	2.12	1.59	n. s.
	Mid	Treatment	78.70	3.47		
	Mid	Treatment	78.70	3.47	1.60	n. s.
	Post	Treatment	77.92	6.04		
	Pre	Treatment	80.02	2.12	0 55	n. s.
	Post	Treatment	77.92	6.04		

The two groups do not differ significantly in their pre-treatment scores (Table 2). But the intergroup differences found at mid treatment as well as post treatment level are significant ($p<0.01$) indicating decrease in depression scores in the experimental group.

Table 2. Intergroup Difference of Depression Scores at Various Stages of Treatment (N=50)

Stages of Evaluatian	Group	Mean	SD	't'	P
Pre Treatment	Experimental	80.89	3.06	1.14	n. s.
	Control	80.02	2.12		
Mid Treatment	Experimental	69.30	8.15	5.19	0.01
	Control	78.70	3.47		
Post Treatment	Experimental	57.52	13.49	6.75	0.01
	Control	77.92	6.04		

Table 3 shows qualitative analysis at various stages of treatment. The first 15 days of treatment seem to be very effective; out of 25 subjects, one became normal, 2 mild, 7 moderate, and 15 were not affected at all. At the end of 30 days treatment the effect of Shavasana appears to be more marked with 11 becoming normal, 4 mild and one moderate. The remaining 9 didn't show any improvement. The improvement in the control group was almost nil because of no treatment. Only 2 out 25 patients became mild and none became totally free from the symptoms of depression.

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Table 3. Qualitative Analysis at Various Stages of Treatment

Stages of Treatment	No. of severly Depressive Cases		No. of Moderate Cases		No. of Mild Cases		No. of Normal Cases	
	E	C	E	C	E	C	E	C
Pre Treatment	25	25						
Mid Treatment (After 15 days)	15 (60%)	25 (100%)	7 (28%)	Zero	2 (8%)	Zero	1 (4%)	Zero
Post Treatment (After 30 days)	9 (36%)	23 (92%)	1 (4%)	2 (8%)	4 (16%)	Zero	11 (44%)	Zero

It is evident from the above results that Shavasana is an effective and useful techique for totally alleviating or reducing depression to almost normalcy level in a significantly large number of cases. Any one technique of behavioural medicine may not be equally effective on all patients due to individual differences, but some techiques are generally more effective than the others. Shavasana seems to be one like that. The results also reveal that the duration of treatment has a positive relationship with therapeutic gains; longer treatment, gives better results. But in the absence of adequate follow up studies it is not possible to ensure whether Shavasana technique remains effective even without becoming the life style of the patient. The technique seems to have immediate effects no doubt and it also makes the patient to learn how to relax in tense situations, but if the individual has to prevent the sccurrence of stressful reactions he will have to make yogasanas, particularly Shavasana, a way of life.

The trend of results in the present study is in line with the results of other studies conducted in this area. Walia et al. (1989) in a multidisciplinary study on the effect of yoga techniques of Kriyas, asanas and pranayama found significantly positive effect in mitigating depression among nurses of the PGI, Chandigarh. The studies conducted by Datey et. al. (1969) on the therapeutic effect of Shavasana for improving subjects with renal essential and arteriosclerotic hypertension also showed positive results. Udupa (1972) also confirmed the usefulness of yogic practices on physiological endocrine and metabolic responses.

It is suggested that further investigations should be conducted to study the effect of Shavasana not only on depressives but also on the patients suffering from other psychological and psychophysical disorders adopting follow up designs.

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Healing

What is Pranic Healing?

Overview

Pranic Healing is a highly developed and tested system of energy based healing technique that utilizes life force or prana to balance, harmonize and transform the body's energy processes.

Pranic Healing is a simple to learn, yet a powerful and effective no-touch energy healing. It is based on the fundamental principle that the body is a "self-repairing" living entity that possesses the innate ability to heal itself.

Pranic Healing works on the principle that increasing the life force or vital energy on the affected part of the physical body accelerates the healing process. Pranic healing also influences the proper functioning of chakras within the energy body, which in turn are responsible for proper regulation of life force in the bioplasmic body, which manifests in the upkeep of good health.

Pranic Healing is not intended to replace orthodox medicine, but rather to complement it.

Disclaimer

- Pranic Healing is not intended to replace orthodox medicine, but rather to complement it.
- For severe ailments and persistent symptoms, the affected persons are advised to consult a medical practitioner / doctor to get proper medical treatment.
- Pranic Healing is a system of balancing energy for the purpose of stress reduction, relaxation and empowerment only and is not intended to replace the prevalent medical or physiological diagnosis and treatment. It is only meant to complement them and to be used as a preventive system.
- Pranic Healing practitioners do not physically touch the recipient's body, diagnose diseases, prescribe any drugs/substances or make any health claims or promises.
- A Pranic Healing Practitioner is not a licensed physician, surgeon or psychotherapist. They are neither authorised nor competent to make medical diagnosis or give medical prescription and are strictly prohibited from giving medical advice or interfering with the medical advice given by a competent medical practitioner/doctor.

Applications of Pranic Healing

Pranic Healing can be applied to bring about a state of physical well-being. Parents can bring down the temperature of their children in just a few hours. Cough and cold can usually be cured in a day or two. Pain due to mechanical injury can be reduced almost instantly. Pranic healing protocols also exist for major illnesses such as eye, liver, kidney, and heart problems, which can be alleviated over a period of time.

Advanced Pranic healing contains protocols and methodologies for healing ailments of the different systems of the body:

- Immunity and Defence System
- Disorders of the Eye, Ear and Throat
- Skin Disorders
- Heart and Circulatory Ailments
- Respiratory Ailments
- Gastrointestinal Ailments
- Urinary Ailments
- Reproductive Ailments
- Endocrine Ailments
- Skeletal and Muscular Disorders
- Blood Disorders
- Disorders of the Brain and Nervous System
- Tumours and Cancer

Please note that Pranic Healing is not intended to replace orthodox medicine, but rather to complement it. If symptoms persist or the ailment is severe, please consult immediately a Medical Doctor and a Certified Pranic Healer.

Pranic Psychotherapy on the other hand uses Pranic Healing principles for the treatment of psychological ailments. Pranic Psychotherapy contains protocols and methodologies for healing the following conditions and ailments:

- Healing Stress, Irritability, Anxiety, Grief and Hysteria
- Healing Phobias, Trauma, Obsessions and Compulsions
- Healing Addiction: Smoking, Alcohol and Drug Addiction
- Healing Depression
- Healing Violent and Paranoid Patients
- Healing Mentally Retarded Patients

There is a lot more to Pranic Healing than just healing. Pranic Healing also teaches one to harness the possibility of using positive energy to bring in bring in harmony and productivity to every aspect of our lives. Pranic Healers around the world have started using Pranic Healing for their businesses, homes, relationships, projects and studies with fantastic results.

Pranic Healing also teaches techniques such as Pranic Breathing, a practice by which you literally breathe in the difference into your physical, emotional and energy levels by taking them to a more relaxed state of being and dynamic meditation techniques such as the Meditation on Twin Hearts for Peace and Illumination which enhances connection with the soul (the divine nature), and sharpens intuition and spiritual growth.

Pranic Healing practitioners across the world have reported the following benefits from the

- Improved health and increased stamina
- Inner peace and happiness
- Better memory and concentration
- Rapid spiritual growth
- Reduced stress
- Better interpersonal skills
- Greater self-esteem
- Attain the ability to attract good luck and become more prosperous

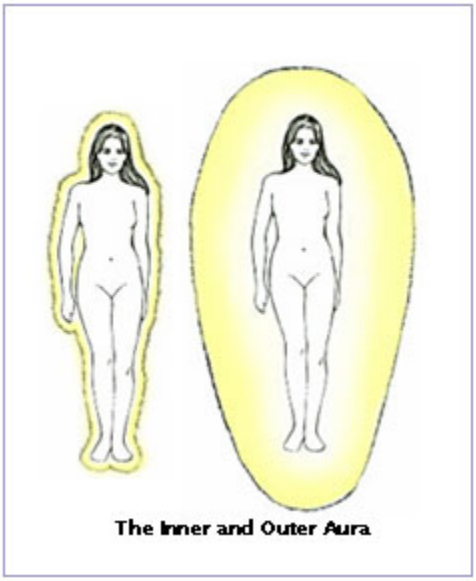
We urge you to experience these benefits for yourself.

Genesis

The MCKS Pranic Healing system has its roots within the ancient Indian system of yoga and the Chinese External Medical Chi Kung School. Pranic Healing in its existing form has been painstakingly, systematically, and scientifically developed, revealed and taught globally by Master Choa Kok Sui. Along with committed participants he spent several years experimenting to determine the effectiveness and the mechanism of the healing techniques commonly known and used by healers and students of esoteric sciences. Hundred and thousands of students have learned to do Pranic Healing, and millions of patients have been healed or relieved.

Basic Principles

"Prana" is a Sanskrit word that means "life-force". This invisible bio-energy or vital energy keeps the body alive and maintains a state of good health. In acupuncture, the Chinese refer to this subtle energy as "Chi". It is also called "Ruah" or the "Breath of Life" in the Old Testament.


The Inner and Outer Aura

Pranic Healing is based on the concept of prana and the overall structure of the human body. Human's whole physical body is actually composed of two parts, the visible physical body and the invisible energy body, called the biosplamic body or the energy body.

This bioplasmic body is the invisible, luminous energy body which interpenetrates the visible physical body, and extends beyond it by tour or five inches. Traditionally this energy body has been called etheric body or Aura. This bioplasmic body contains the mould and blueprint of the physical body and absorbs life energy and distributes it to the organs and glands. The bioplasmic body also contains energy centres that regulate the flow of prana to various parts of the body. These energy centres are called chakras. Proper regulation of prana in the bioplasmic body manifests in the upkeep of good health and vitality. Diseases first appear as energetic disruptions in the energy field before manifesting as ailments in the physical body .

Scientific evidence provides tangible proof of the existence of the energy body and its relation to the health and well being of the physical body. Scientific experiments conducted by the eminent Russian scientist, Semyon Kirlian, using ultra sensitive photographic process showed a colourful, radiant energy field surrounding the physical bodies of humans, animals and plants. This energy field, or aura, interpenetrates the visible physical body, extending about 4 to 5 inches from the skin's surface. Experiments in Kirlian photography have also revealed that diseased energies appear first in the energy body before manifesting as a physical ailment.

Pranic Healing utilizes "prana" –also known as Ki or life energy, to promote physical, emotional, and mental health and well-being. It has also been called “Medical Qigong, vitalic healing, therapeutic touch, laying of the hands, magnetic healing, and charismatic healing .

Pranic healing is based on two functional principles:

- Principle of Self-Recovery – The body has an innate ability to heal itself at a certain rate. Medicines, Doctors, Therapists and Practitioners of the healing arts just assist in this process by accelerating this innate ability.
- Principle of Energy Force– for Life to exist, the body must have prana, chi or energy. Prana or energy is to a body what gasoline is to a car. This prana can be transfer and manipulated in order to accelerate the healing process.

Pranic Healing influences this natural life force and the bioplasmic body to bring about a healthier physical body. In chemistry, electrical energy is sometimes used as a catalyst to increase the rate of chemical reaction. Light can affect chemical reaction. In the same way, in pranic healing, prana serves as the catalyst to accelerate certain reactions involved in the natural healing process of the human being.

There is nothing paranormal about MCKS Pranic Healing. Just about anyone can learn to use these simple but powerful techniques to bring about physical, emotional, mental and spiritual well-being.

Levels in MCKS Pranic Healing

Pranic healing has several levels graduating from simple to more complicated concepts and from easy to difficult techniques.

- **Level One - Elementary Pranic Healing:** At this level, the concepts and techniques are easy to learn. Tactile concentration is required. It takes about three to five sessions to learn the basic principles and techniques and to be able to do simple pranic healing. About a few days, a few weeks or a few months of regular practice and application and required to become proficient, depending on the attitude of the student.
- **Level Two – Intermediate Pranic Healing:** This level is still easy. Pranic breathing is used at this level. Visual concentration is optional but still not required. Just as in level one, this takes about three to five sessions to learn the basic principles and techniques to be able to start healing more difficult cases. To become proficient, takes about a few weeks to about two months or regular practice and application, depending on the attitude of the student.
- **Level Three – Distant Pranic Healing:** This level may take several weeks or months to a year or two of regular practice and application to become very accurate with diagnosis and to produce specific, accurate, predetermined results.
- **Level Four – Advanced Pranic Healing:** The use of visualization techniques is definitely required

and a more through knowledge of the disease and knowledge of different types of prana are necessary. To become proficient with Advanced Pranic Healing takes about one to two years of practice.
- **Level Five – Pranic Psychotherapy:** Pranic psychotherapy is Advanced Pranic Healing applied in the field of psychological ailments. The practitioner should be at least proficient in intermediate pranic healing.
- **Level Six – Pranic Crystal Healing:** Pranic Crystal Healing is using crystals as instruments to facilitate pranic treatment.

MCKS Pranic Healing Certification Program

The Pranic Healing Healer's Certification Program was established in response to the need to provide a matrix for both formal coursework and the continued practical hands-on experience in Pranic Healing for serious healers who would like to enhance their healing skills and become a member of an internationally recognized and accepted group of certified Pranic Healers.

In order to do this, the Institute for Inner Studies is guided by the following principles:

- The Pranic Healing Healer's Certification Program certification program combines a high standard of professional, disciplined training within a flexible structure and schedule
- The development of individual healing abilities is an essential part of the program.
- Opportunities for professional development can be provided.

Through the guidance of Grandmaster Choa Kok Sui, the foremost expert in Pranic Healing, and the professional abilities and healing accomplishments of assigned Pranic Healing coordinators, mentors and instructors, the Institute for Inner Studies ultimately provides the program in all its aspects of Pranic Healing.

The programme has the following levels of certification:

- **Certified Associate Pranic Healer:** Proficient in elementary and intermediate pranic healing. In the process of becoming proficient in distant healing, advanced pranic healing and pranic psychotherapy. An associate pranic healer is just like a medical intern or an apprentice.
- **Certified Pranic Healer:** Proficient in advanced pranic healing, pranic psychotherapy, and pranic crystal healing.
- **Certified Senior Pranic Healer:** Very experienced and proficient in advanced pranic healing, pranic psychotherapy, and pranic crystal healing. For the moment, relatively rare.
- **Certified Assistant Master Pranic Healer:** Proficient in using powerful and very advanced pranic healing techniques not revealed for the moment to the public. A certified assistant master pranic healer has been trained under the guidance of a master or grand mater pranic healer. Also, should have a track record of producing many rapid miraculous healing that other pranic healers in general will find difficult to duplicate. For the moment, rare.
- **Certified Master Pranic Healer:** Much more skilful and powerful than an assistant master pranic healer. Has trained several students to become a certified assistant master pranic healer. For the moment, rare.
- **Certified Grand Master Pranic Healer:** Can heal very difficult cases in one or two sessions. The high level of healing skill achieved is extremely rare.

Spirituality

Manifest your Greatness

The Latin root of the word meditation, *mederi*, means "to heal."The word meditation has had different connotations in different parts of the world. I the West, the word meditation means a concentrated state of mind in serious reflection. In the East meditation has been linked more to a spiritual ideal. The Hebrew word for meditation means a very intense spiritual battle. According to Zen, meditation is an awareness of inner silence. Meditation can be used to mean the practice of prolonged awareness. It could mean spiritual practice or spiritual cultivation .

Meditations and spiritual development speeds up evolutionary processes and are responsible for development of the higher faculties. Human beings have infinite potential. Spiritual practices are the missing link that can enable one to realize his or her unrealised potential, and achieve just about anything.

Many powerful techniques for spiritual development and advancement were revealed to Master Choa Kok Sui by his Teacher Mahaguruji Mei Ling to help humanity achieve an empowered, safe and accelerated spiritual life in the shortest time possible. Master Choa Kok Sui has shared these highly distilled and simplified techniques with many around the world enabling them to lead a more fulfilling and enriching life.

Meditation on the Twin Hearts

Meditation on Twin Hearts is a meditation technique aimed at achieving illumination or universal consciousness. It is also supposed to be an instrument of world and community service that would eventually harmonize the earth by blessing it with loving-kindness, peace, joy and goodwill.



Grand Master Choa Kok Sui, the modern founder of Pranic healing and Arhatic Yoga first introduced this meditation as a tool for spiritual development in his book, The Ancient Science and Art of Pranic Healing (1987). Presently it is being practiced globally in more than 58 countries throughout the world by tens of thousands of people of different religions and backgrounds.

Twin Hearts Meditation, however, is not just for the spiritual development of the individual. A part of this meditation includes the practitioner using his/her Divine Energy to bless the whole world and all living beings. This is a form of world-service that helps to heal the world and can help usher in the Golden Age of World Peace.

Twin Hearts Meditation is an integral part of the Pranic Healing system, but can also be learnt independently.

Benefits of the Meditation on Twin Hearts

Practitioners of the Meditation on Twin Hearts around have reported the following benefits from the meditation technique

- A more magnetic personality
- Sharper and clearer thinking
- Healthier physical bodies
- Reduced stress
- Inner peace
- Self Confidence
- Increased spirituality

Meditation on the Twin Hearts can also be used for effective team building. In the Meditation on Twin Hearts, the meditator experiences feelings of kindness and love. When in this state of parasympathetic arousal - the physiological opposite of 'fight or flight' - the set of reactions generate a feeling of calmness and confidence facilitating co-operation. Instead of hostility, anger and suspicion the interaction with the group and others is from a feeling of well being and contentment.

Meditation on Twin Hearts also helps one improve ones Emotional Quotient. Emotional Intelligence is being aware of and handling emotions competently. Today more importance is given to Emotional Quotient than to Intelligence Quotient. Awareness of feelings of self and others and the ability to handles one's feeling effectively are a great asset. Clever people become stupid and are unable to think when under the grip of strong emotions. The Meditation on Twin Hearts is as much 'Them' oriented as it is 'I' oriented. The awareness and sensitivity which develop through regular practice help enormously in relationships. This coupled with the technique of blessing others with good things in life that one personally desires, increases one's competence in relating effectively. Emotional equilibrium is a great asset which benefits people in all walks of life.

Research on the Meditation on Twin Hearts

Research and Scientific studies have been completed on the neurophysiological, psychological, and sociological and psychospiritual effects of the Meditation on Twin Hearts. These studies have been conducted in the most part by Glenn Mendoza, M.D. in New York City, New York and by Dr. Vrunda and Supriya Ghorpadkar and Dr. Sundaram in Bangalore, India.

- Meditation on Twin Hearts showed measurable changes on the pattern of electrical activity through the brain by electroencephalograph (EEG). Comparing EEG studies before and after meditation, there was a dramatic transition from beta waves to alpha waves and delta-theta waves in 14 meditators and non-meditators.
- There was an observable increased in brain synchrony (alpha-delta-theta waves) between the two hemispheres of the brain after the Meditation on Twin Hearts. There was synchronization of brain waves initially between the same hemisphere and followed by synchrony between the frontal, middle and rear portions of the brain.
- There was a decrease in heart rate and respiratory rate of up to 15-20% and an increase in oxygen saturation after the Meditation on Twin Hearts.
- A positive relationship of skin conductance before and after the Meditation was observed in all subjects.
- The Meditation on Twin Hearts was found to show a dramatic muscle potential measurement through electromyogram (EMG) of the scalene and extensor muscles of the arms.
- There was significant increase in neurohormones serum Serotonin and plasma Melatonin (up to 300%) following Meditation on Twin Hearts in a pilot study of 17 subjects and a follow-up study of 35 subjects.

Planetary Peace Movement Trust, India

The Planetary Peace Movement Trust was founded by Grand Master Choa Kok Sui in Bangalore in 1999. It aims to promote world peace by spreading the Meditation on Twin Hearts. The organization is working actively to promote meditation among schools and organizations around India.

Till November 2005, close to 125,000 school children in the country have been introduced to the meditation.

Arhatic Yoga

"Arhat" is a Pali term, which literally means "He who is Worthy", or "He who is worthy of Praise". Arhatic Yoga, also called The Synthesis of Yogas or, in the higher levels, The Yoga of Synthesis, is designed to help the practitioner develop to the level of an Arhat.



The word "yoga" is from the Sanskrit root "yug" which means to yoke or unite. Yoga is thus the term given to any technique that culminates in uniting us with the divine.

There are many paths of yoga, bhakti yoga aims to achieve this union through love and devotion, karma yoga does it through service, jnana yoga aims at union through knowledge while raja yoga works at union through meditation. Grand Master Choa Kok Sui has successfully combined all these methods of yoga along with kundalini yoga to form one of the fastest and most powerful methods for spiritual development and evolution. Arhatic Yoga was revealed to Master Choa Kok Sui by his Teacher Mahaguruji Mei Ling, to help humanity achieve an empowered, safe and accelerated spiritual life in the shortest time possible.

Arhatic Yoga is not a religion. You are not asked to change your religious beliefs. Arhatic Yoga has been specifically designed to allow the practitioner to maintain their initial religious or philosophic practice. Practitioners may even improve or become more dedicated to their faith through this system of personal and spiritual development. One would become a better person through the practice of the techniques discussed. Your life will become calm, not chaotic.

Arhatic Yoga to enable people of various formal religious beliefs or philosophies, to have access to inner teachings that make it easier for them to pursue spiritual development while at the same time maintaining a ’normal’ life. This means that in Arhatic Yoga, the individual who wishes to pursue the spiritual path need not retreat from the world, their career or their current relationships. In fact, these pursuits are used to help the individual to further develop, through the daily challenges and interactions, as well as difficulties that may occur within the family, in the office, or in the contemporary lifestyle.

Arhatic Yoga is further distinguished by its multi-leveled approach and its focus on purification, character-building and service. All Arhatic Yoga practitioners begin at the "Preparatory Level," where they learn the basics of spiritual development, as well as the fundamental meditations and breathing exercises, or pranayama, that get their physical and energy bodies ready for the great amount of energy that their practice will generate at the higher levels of Arhatic. Arhatic Yoga places a lot of emphasis on service, character building, developing a clean and more refined energy body.

International Arhatic Yoga Ashram



GMCKS Arhatic Yoga Ashram is the first Ashram in the world for Pranic Healers and Arhatic Yogis. Situated in the midst of nature, surrounded by valleys, mountains and the forest, preserved from the encroachment of humanity by a deep gorge and lake on one side, and by verdant valleys and hills on the other, the Ashram is the ideal getaway from the stress and bustle of urban living. In this pristine atmosphere, an individual is inspired to evolve.

Said to be near a mythological cave where the Pandavas meditated, the Ashram is situated on a site embedded with natural crystals.

In this lush environment, Grand Master Choa Kok Sui's vision is coming to fruit – the 1st phase of the Ashram is almost complete. This includes meditation halls, teaching rooms, a kitchen and dining area, an administrative hall and studio units for accommodation. Classes and retreats have already begun at the Ashram and starting 2006, monthly activities are planned there.

Attending a Master Choa Kok Sui course is like learning driving. The basics are fairly simple to grasp, but the mastery of the techniques takes repeated practice and study.

All the workshops of Master Choa Kok Sui are experimental in nature. During the workshop the principles within the scope of the course would be explained thoroughly and the techniques practiced exhaustively so that you will be confident in your ability to produce positive results when you finish the course.

- **Foundation Courses**
  - Basic Pranic Healing
  - Advance Pranic Healing
  - Pranic Psychotherapy
- **Intermediate Courses**
  - Pranic Crystal healing
  - Psychic Self Defence
- **Spirituality and Meditation**
  - Achieving Oneness with the Higher Soul
  - Arhatic Yoga
- **MCKS Higher Courses**
  - Kriyashakti
  - Pranic Feng Shui
  - Higher Clairvoyance
  - MCKS Pranic Healing Certification Programme
  - Teachers Training Course
  - Other Courses:
    - Inner Teachings of Hinduism Revealed
    - Inner Teachings of Buddhism Revealed
    - Om Mani Padme Hum

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REIKI: OLD INDIAN THERAPY AS NEW METHODOLOGY.

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ABSTRACT:

Reiki is an ancient healing therapy, which was utilized at the time of the 19th century in India which helps to maintain the physical, mental, emotional, spiritual balances within a person's body. Reiki is an effective healing delivery system that addresses wonderful glowing radiances which passes through or around the biological systems. Reiki shows benefits also in reducing the side effect of chemotherapy and radiation therapy which are commonly utilized for the treatment of cancer. Reiki corrects some diseases by maintaining enzyme activity within the body like neurodegenerative disorders, anxiety, depression also show benefits into CVS minimizing all type of heart disease as like Hypertension, angina.

The principle of the reiki depends on the stimulation of chakra and aura within-person. We studied this topic about the role of this therapy on the body. Hormonal balance results due to stimulation of system shows benefit with control on mental and physical consequences. In one of the study, we found that therapy is very cost-effective and manage the disease state better than placebo effect commonly utilized for correction in neurodegenerative disorders. It minimizes exacerbation of diseases like cancer which can produce fatality and thus there is a need to utilize the therapy in the present era.

It is easy to learn and transfer. Completing a two-stage course from any reiki education center and practicing the therapy over the world affords low-cost consumption and increases the chances to minimize the deadly disease rapidly changing their form in India.

KEYWORDS: Reiki, healing, chakra, Aura, Neurodegenerative disorders, Cancer.

INTRODUCTION:

History And Origin Of Reiki:

Reiki is an old therapy which is based on both Japanese and Indian culture. The origin of reiki was done in Japan by Dr. Mikio Usai, who completed training in psychology and spiritual therapy. He was interested to find new therapy that isn't based on any religion. After completion of his training, he went for meditation purposes in caves located at Mount Kurama. With his twenty-first training, he found some miraculous symbols having Sanskrit origin which was discussed by the ancient saint of India.

From the study of symbols, he got information about chakra and aura phenomenon within the human body. He performed the study of symbols and postulated the new therapy which maintains the mental, physical, and spiritual well-being of an individual with the stimulation of symbols. In Tokyo in 1922, Dr. Usai started the clinic for providing such a wonderful healing therapy to other persons for spiritual and mental status well-being.

After some time another person named Dr. Hayashi took interest in a study that therapy. He had improved therapy providing the importance of hand position to cover all over the body. He was postulated that utilization of hand the spiritual energy should be transferred appropriately for stimulation and activation of chakra within-person further all over world therapy practiced on these guidelines [3,4]

Factors Evolving Reiki Therapy*:*

Chakra and Aura are the main factors involving reiki therapy which are postulated by ancient saints of India. These are symbols that lie within humans adequately regulate all body function and by its proper stimulation or activation, any problem over the biological system will correct adequately. These are energy tools of the body which regulate all secretary and hormonal function. Reiki is based on energy transport that chakra and aura within humans and regulate the biological function within him [8,16]

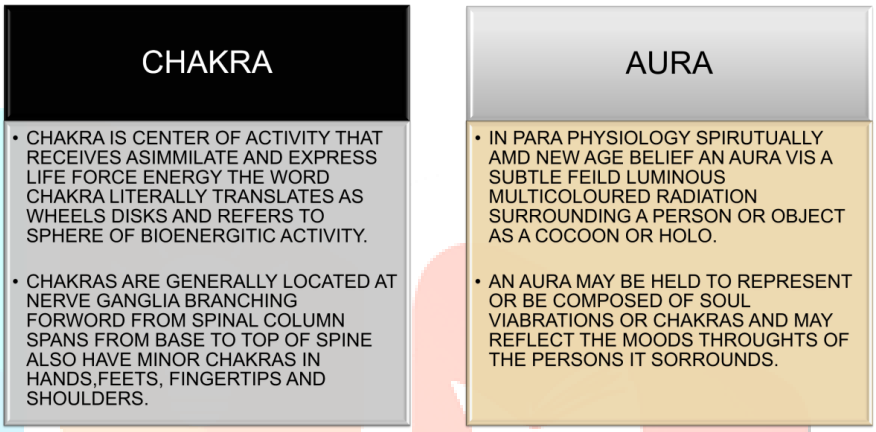


Fig. No. 1: Activity of Chakra and Aura.

WHAT IS REIKI THERAPY:

Reiki is energy healing therapy with the utilization of universal energy within the person which is it lie. Reiki is one of physiotherapy in which the stimulation of Chakra and Aura lie within humans is performed by a practitioner who has done the qualified course. Primary energy healing is transferred to the person whom it requires. The energy healing results in a balance of hormonal changes and secretary defects within persons and disease exacerbation will be managed.

As per procedure, the practitioner follows the steps to give proper healing energy to the person it requires. It is effective physiotherapy having some miraculous results for minimization of death-causing disease. The mental, physical, spiritual well-being is the main objective behind reiki [1,2]

- **Some Reiki techniques:**
  1. centering.
  2. clearing.
  3. beaming.
  4. extracting harmful energies infusing.
  5. Smoothing and raking the aura.

The above list is of the techniques utilized for transferring reiki to another person. The practitioner will have to do a qualified course of reiki which is given in 1, 2 and master level format. The technique is discussed in deep undergoes reiki course and all practitioner are follows the steps which are discussed within [5]

- **Basic Factors Of Reiki:**
  - ***Chakra Within Body*:**

Chakras are the center of activity of human mental, physical, spiritual balances. 7 type of chakra are lie within a person's having different relative roles as given below:



Fig. No.2: 7 Chakras Within Body.

- ***Flow Of Reiki within Body:***

The main principle behind reiki depends on endocrine stimulation which results in hormonal balance. The Chakra center of activity carries the transport of primary energy obtained from the practitioner. Based on reiki touch, 1st 4 steps involve the crown, brow chakra which relates to the stimulation of the pineal gland, hypothalamus, and pituitary gland respectively. Further 4th hand position relates to throat chakra act by stimulation and regulation in the thymus, thyroid and parathyroid glands also have some role production in much more amount of white blood cells which prevent infection within the body. The adrenal gland is present at the lower back of a person which obtain healing from hand position 10 and position number 7 is relates to glycemic balance because it heals the pancreatic area. Reproductive systems possess various glands which are relatively healed by the reiki position number 8 and 10 given by practitioners [21

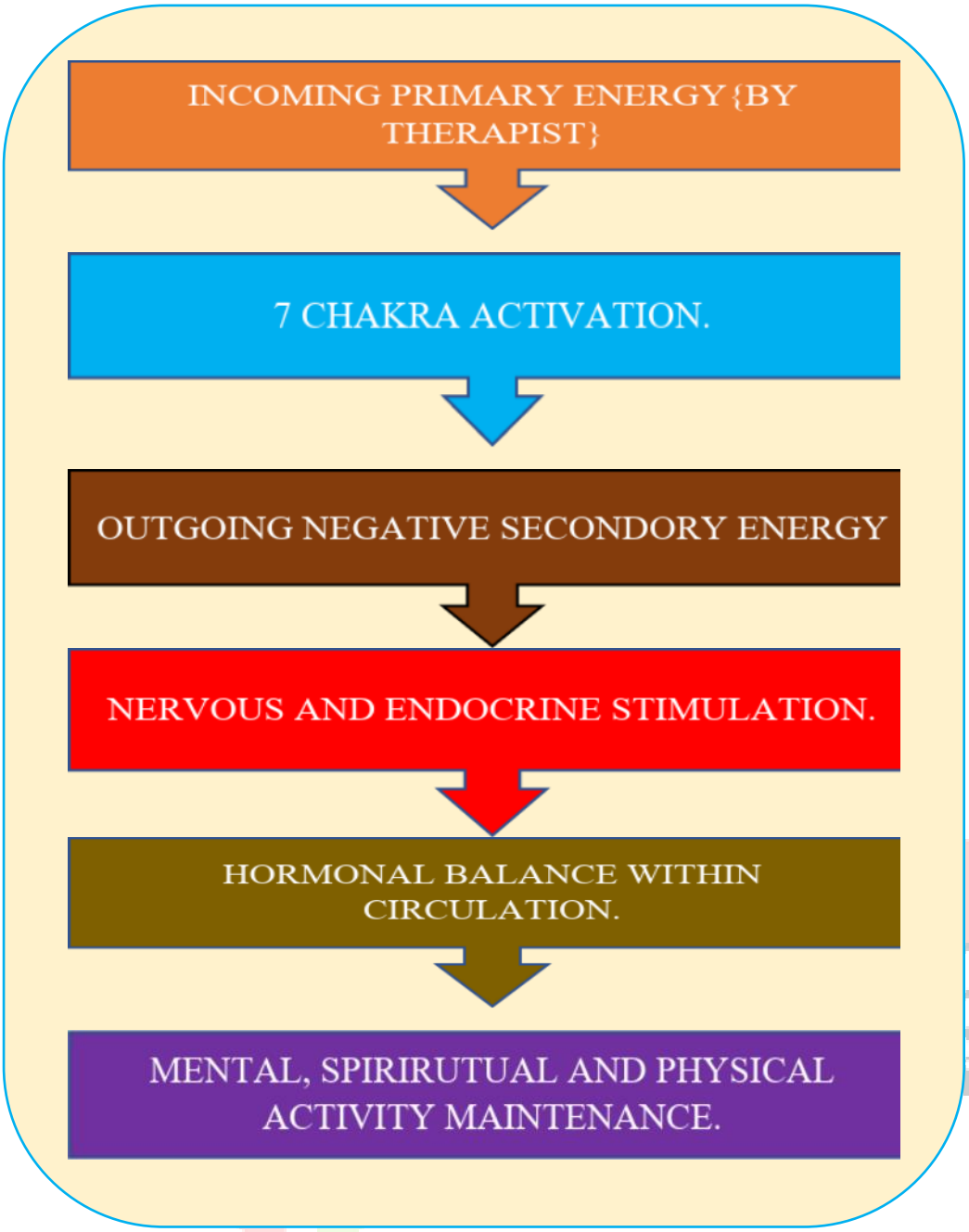


Fig.no.3: The flow of reiki within body.

- ***The Systemic effect of reiki on the Human body;***



Fig. no. 4 Systemic effects of reiki therapy.

PROCEDURE FOR REIKI IMPLIMENTATION:

Reiki is transferred by a qualified practitioner to patients who have done the reiki therapy qualification course which is essential for transfer the healing to the patient.

Session of reiki healing is of a minimum of 30 min. in which one hand position is last for 2-3 min. as per the necessity of the patient.

There is no restriction to the consumer of this therapy only he has to relax deeply to acquire this therapy and feels the healing given by the practitioner.

Step 1: Place your hands together in the middle of the chest follows below to the chin. Breathing should be under control focus should be slow and deep in nature step is mainly done for the relation of mind. [1-2 min.]

Step 2: Place both hands at the top of the head breathing of patients should be deep and relax in nature tissue relaxation taken place by these processes so that circulation and the nervous supply should be maintained using this type of method. [2 min.]

Step 3: Follow the hand position on the eyes as well as breathing by the patient should be continued a focus of patient will be on eyes which are covering the patient's eye by the practitioner hands and the relaxation of eye muscles regulate the secretion and the circulation regulation will be done. [2 min.]

Step 4: Place the right hand gently on the throat and left hand on the heart associated with maintaining the deep breathing without applying any force on both hands so that both throat chakra and the heart chakra are healed and the relaxation of all surrounding muscles of thyroid glands and parathyroid glands enhance the secretion are done by this step. [2 min.]

Step 5: Move the hands 1 width below up to breast line middle fingers touches to each other with continue breathing focus of patient will be steadily on the hand position and heart area and which regulate the heart blood supply along with relaxing the cardiac muscles and circulation will be maintained. [2 min.]

Step 6: Move hands in below direction on the upper stomach position which is the area of the solar plexus chakra associated with the metabolic gland i.e. liver and regulate the secretion by the stomach which regulates the PH and ions excessive production which causes nausea, vomiting, dyspepsia occurs when the treatment of cancer chemotherapy given to the patient. [2 min.]

Step 7: Place the hands on the navel area which are the area of the pancreatic glands and adrenal glands which present at the top the kidney production of the hormones and neurotransmitter as, like adrenaline, noradrenalin are maintained. [2 min.]

Step 8: Put the hands-on on both shoulders which transfer healing to the whole spinal cord area nervous regulation will be maintaining the phenomenon of relaxation and contraction by the certain release of proper neurotransmitters. [2 min.]

Step 9: Gently place your hand on the waist area which mainly the kidney area relaxes all muscles associated with it and the release of the rennin which regulates the cardiovascular condition and adrenaline and noradrenaline release which is an excitatory neurotransmitter associated with the autonomic nervous systems. [2 min.]

Step 10: One hand on the top of the feet and the one hand below the feet which results in the lower circulation regulation by transferring the healing but the breathing of the patient are always deep, slow and relaxed focus on the one fact to regulate the stress shows mental well-being. [2 min.][6]

SYSTEMIC BENEFITS OF REIKI PHYSIOTHERAPY:

Reiki physiotherapy has its beneficial role in the cure of the following disease which is well explained as follow:

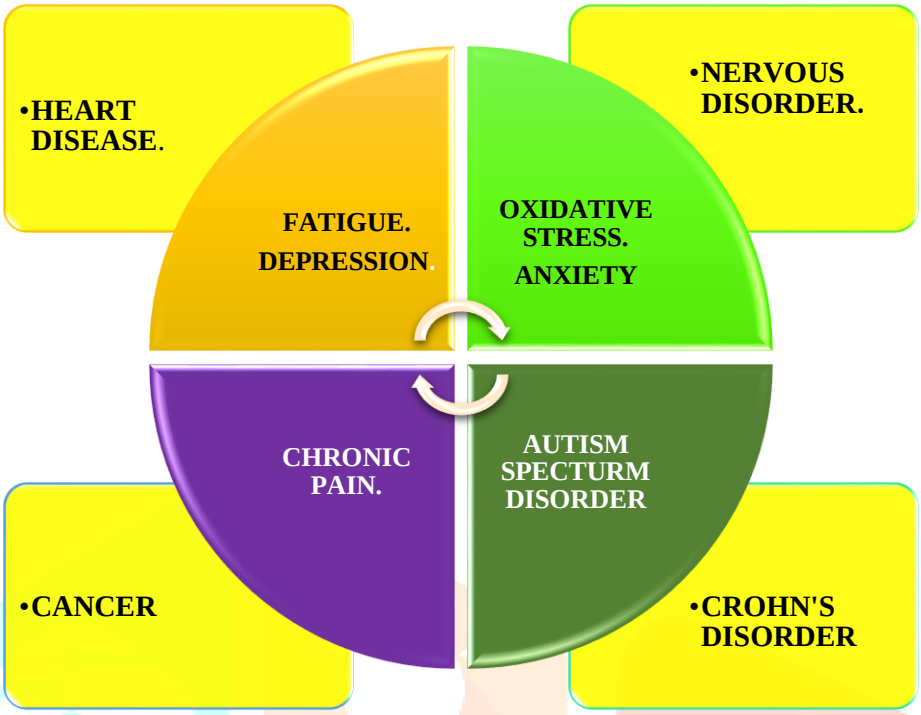


Fig.no.5 Systemic benefits of reiki therapy.

Oncogenesis: Initiation of cancer.

Process of initiation of cancer or tumor formation due to aggregation of malignant tumor cells induced due to the effect of the radiation, chemical agents, and certain viruses. Oncogenesis is the depend on two factors i.e. molecular pathogenesis and the other is the chemical carcinogens both occur due to genetic factors or cellular changes respectively. The main therapy for correction of cancer is named as chemotherapy and it associated with the various adverse effects which can cause systemic disturbances. Condition like hair loss, nausea, vomiting, fatigue, anxiety is most commonly occurs during chemotherapy causes mental and physiological disturbances within the person. Reiki physiotherapy doesn't have any evidence of DNA correction and to cure structural change that occurred due to carcinogens. It can control the adverse events associated with chemotherapy which are very hazardous to patient's health occur in each and every patient under the chemotherapy. As a complementary medicine, it enhances the biological activity if utilized in the early stages of cancer correct the disease condition also. Most commonly chronic pain is associated with oncogenesis so physiotherapy has the significance of correcting chronic pain along with the relaxation of the muscles.[22,23,24]

Cardiovascular disorders:

Hypertension:

Persistent elevation into blood vessels is known as hypertension. Reiki significant to act on the autonomic nervous and regulate the cardiac tone. Except for genetic factors most of the time stress, anxiety are reasons to induce hypertension. as per the study, reiki heals the area of the adrenal and regulates the production of mineral corticoids, adrenaline, noradrenalin which play an important role in cardiac tone. Most common fatigue, stress occurs during hypertension which can be controlled by the utilization of this therapy. Heart chakra healing is opposite to psyche i.e. the emotion of the person is regulated so that the chances of the correction are increased and vagal tone would be controlled by the energy healing. [25, 26]

Ischemic heart disease:

IHD is a cardiovascular disorder characterized by the insufficient blood supply to the heart results in chronic pain due to structural and functional changes. Reiki acts as complementary medicines in IHD by showing the action on the patient had undergone the CABG and primary or secondary PCI. On coughing or breathing the feeling of the cracking or crushing in the heart is occurs most commonly in such type of patient. As we know that reiki acts on chronic pain and it can also control the symptoms like anxiety, nausea, vomiting, sweating that occurred due to medications also cured. In IHD most commonly nitrates are used to perform vasodilatation have the common side effect of hyper vasodilatation in the brain area results the severe headache would be cured if the reiki is preferred associated with the medications of IHD.[27]

Central nervous system disorders:

Mental well-being and spiritual enhancement are the main motos behind reiki therapy. Feel on the concentrating on breath and transfer of the healing regulates all the neurotransmitter regulation within the synaptic cleft. Maintaining the release of excitatory and inhibitory neurotransmitter various nervous activity will be controlled. Promoting mood conditions like depression anxiety would be minimized and all physiological activity is maintained. Energy healing is specially used for the correction of neurodegenerative disorders like Alzheimer's and dementia due to its activity on nervous injury and tissue damage. Conditions like fibromyalgia are most occurring nowadays caused due to depletion of serotonin levels which can be only cured utilizing SSRI's it is possible to correct it by reiki physiotherapy. [28,29]

Musculoskeletal system disorders:

Abnormal diet results in insufficient food nutrition to the bone and results in conditions like osteoporosis early. Elder patients are previously distressed due to chronic pain because of their age so the reiki has proven that it can correct the severe pain patients minimizing the inflammation and cure the deadly pain by relaxing the muscles associated with the particular organ. [30]

Endocrine system disorders:

Transfer of reiki therapy by the qualified practitioner heals the 7 chakras within our body and corrects all the regulation of the hormones by their respective glands. As we studied position number 1-4 heals the glands present on the upper side of the body as like pituitary, thyroid, parathyroid, liver which is the largest gland as well as the metabolic center of the human. Position 4-7 heals the lower glands adrenal, pancreatic, and sexual glands. Regulation of hormones results in the proper body physiology and their regulation to each and every part of the body. [21]

CONCLUSION:

Reiki is an old Indian therapy having an ancient background of Indian culture which is fluently used in the abroad countries. It is a need to utilize it in India nowadays to remembering the moderation in disease states. This physiotherapy is the low cost consuming and proven that have wondrous curing properties on transferring the healing. To get the qualification complete a two-stage course anywhere over the world and practice it. There are much better chances to correct the deadly diseases like cancer, IHD, Neurodegenerative disorders frequency of such disease occurred more nowadays and Session is also very small up to 30 min only which can be conducted easily by any patient. Most commonly placebo therapy is used in some patients to correct their disease by a physician but it is proved reiki is better than that of placebo so needs to utilize as alternative complementary medicine.

DISCUSSION:

The reason behind the selection of this topic to introduce the physiotherapy in India having special Indian cultural background things like chakra and aura are introduced by ancient sage or saints who are practicing the medical profession in their time. The procedure to transfer it is simple but it is mandatory to complete the 2 stages course to practice it therapy is fluently practiced in abroad countries so it needs to utilize India. Special background in of India the ancient saints and sage are introduced those persons daily work is totally maintained by the phenomenon of chakra and Aura.

As per the study of the journal of alternative and complementary medicines their researches have proven that reiki has spiritual and physiological enhancement which are shows benefits in the correction of diseases like oncogenesis and cardiovascular disorder. As a complementary medicine, it improves the quality of patients' life and supports to minimize the symptoms of the deadly disease.

Reiki physiotherapy is needed in India to controlling the mortality and morbidity of disease state and if will be practiced fluently like abroad countries so the death rate due to disease condition definitely minimized and India will prove itself a healthy and wealthy country.

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Tai Chi and Qigong for the Treatment and Prevention of Mental Disorders

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OVERVIEW

Tai Chi and Qigong

Tai Chi and Qigong are traditional Chinese exercises that are widely practiced for their health benefits and as martial arts. Developed over hundreds and thousands of years, respectively, Tai Chi and Qigong are practiced worldwide in a variety of modern and traditional forms. In 2002, there were more than 2.5 million Tai Chi users and 500,000 Qigong users in the United States.[1] Both Tai Chi and Qigong involve sequences of flowing movements coupled with changes in mental focus, breathing, coordination, and relaxation.[2] There is significant overlap between the 2 practices in terms of movements and in the shared focus on breathing and mindfulness. Both practices are low-impact, moderate-intensity aerobic exercises that are suitable for a diverse patient population with regards to gender, age, and health status.[3] Tai Chi and Qigong have been characterized as mind-body interventions and as "meditative movements."[4] They are relatively safe, nonpharmacologic practices, which can be used for treatment and prevention of psychosomatic disorders, with few adverse events reported in the literature.[5]

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Putative Physiologic Mechanism of Response

Tai Chi and Qigong have been shown to promote relaxation and decrease sympathetic output.[6–9] Relaxation interventions are known to reduce clinical somatic symptoms and to benefit anxiety, depression, blood pressure, and recovery from immune-mediated diseases.[10] Tai Chi and Qigong have been shown to improve immune function and vaccine-response,[11] to increase blood levels of endorphins[12] and baroreflex sensitivity,[13] as well as to reduce levels of inflammatory markers (C-reactive protein [CRP]),[14] adrenocorticotropic hormone,[12] and cortisol.[15,16]

Electroencephalography (EEG) studies of participants undergoing Tai Chi and Qigong exercise have found increased frontal EEG α, β, and θ wave activity, suggesting increased relaxation and attentiveness.[17–19] These changes have not been present in exercise controls.[20,21]

Clinical Research Applications of the Use of Tai Chi and Qigong

A growing body of clinical research has begun to evaluate the efficacy and safety of Tai Chi and Qigong. A systematic review of Tai Chi interventions published in 2011 found 31 Tai Chi randomized controlled trials (RCTs) published from 2002 to 2007, and 11 for 1992 to 2001.[22] That study found suboptimal quality of reporting of Tai Chi intervention trials, with only 23% of RCTs providing adequate details of the Tai Chi intervention used in the trials. Another review of Tai Chi from 1993 to 2007 found 77 RCTs and concluded “research has demonstrated consistent, significant results for many health benefits in RCTs, evidencing progress toward recognizing the similarity and equivalence of Qigong and Tai Chi.” The study found 6410 participants included across these reported studies.

Problematic research issues within the literature on Tai Chi and Qigong are usually related to small sample size, use of different styles of Tai Chi and Qigong, significant variance in practice duration and frequency, and differences in study durations. Because of the similarity of Tai Chi and Qigong and because clinical research has largely failed to differentiate between the 2 exercises, this review considers the benefits of both practices.[23]

Health Outcomes of Tai Chi and Qigong Interventions

RCTs have shown that Tai Chi and Qigong may improve bone density, cardiopulmonary health, arthritis, fibromyalgia, tension headaches, and other medical conditions.[24–36]

Given the relationship between physical and mental health, general improvements in physical health or reductions of chronic disease symptoms may help to improve mental health. Chronic physical health problems are associated with stress, anxiety, depression, and poor mood.[37,38]

Health-Related Quality of Life

Health-related quality of life (HRQOL) serves as a comprehensive measure of patient well-being, and it reflects patient perceptions of personal health and life satisfaction over a period of time. Individuals suffering from mental health conditions are particularly likely to report poor HRQOL. A study comparing patients with common medical disorders to those with mental health conditions found significant differences in HRQOL between the 2 groups. Individuals with mental health conditions had significantly greater impairment of HRQOL.[39] The ability of Tai Chi and Qigong to improve HRQOL[31,40,41] is an important consideration for treating patients with mental disorders. Although there is no evidence that Tai Chi and Qigong may be effective for a particular condition, they may still provide some benefit by improving HRQOL.

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Tai Chi and Qigong as Exercise

Studies have tried to understand the effects of Tai Chi and Qigong as aerobic versus mindful exercise. Independent from any special benefits Tai Chi and Qigong may confer as meditative movements, they also benefit patients as general, low-impact, moderate-intensity aerobic exercise. There is extensive evidence on general exercise interventions with regards to mental disorders.[42] Systematic reviews have found that exercise results in significant reductions in depression symptoms comparable with cognitive-behavioral therapy.[43,44] Two studies found that exercise is comparable with sertraline (Zoloft) in terms of efficacy for treatment of major depressive disorder. In studies comparing the benefits of Tai Chi and Qigong with general exercise, both interventions have been shown to have comparable effects at reducing anxiety.[45,46] In general, evidence from clinical trials supports a positive association between physical activity and physical and psychological health.[47–50]

Tai Chi and Qigong as Meditative Practices

Tai Chi and Qigong practices include a mindfulness component, which may explain why some patients experience greater benefits from Tai Chi or Qigong than from general aerobic exercises.[16,46] A Cochrane collaboration review of meditation therapy for anxiety disorders found only a few studies that permitted firm conclusions on efficacy. The review identified 50 studies of meditation on anxiety, but only 2 that were randomized, controlled and that met criteria for *Diagnostic and Statistical Manual of Mental Disorders* or *International Classification of Diseases* classification of a psychiatric disorder. These 2 studies were of moderate quality with active control comparisons (alternative meditation, biofeedback, or relaxation). One of these studies, which used transcendental meditation, showed a reduction in anxiety and electromyography score comparable with biofeedback and relaxation therapy.[51] The other study compared Kundalini yoga with relaxation/mindfulness meditation and found no significant difference between groups. A separate review concluded that several studies of exercise and yoga have shown benefits comparable with established depression and anxiety treatments. A third review compared 12 RCTs of mindfulness exercises versus nonmindfulness exercises and found that both were effective in causing short-term reductions in depression levels and symptoms.[49]

An RCT of brief daily yogic meditation (Kirtan Kriya) for family dementia caregivers with mild depressive symptoms found that meditation resulted in lower levels of depressive symptoms as well as improvements in mental health and cognitive functioning. Participants in the yogic meditation group showed a 43% improvement in telomerase activity after 12 minutes of daily practice for 8 weeks, compared with 3.7% in relaxation music control participants. This finding suggests that brief daily meditation practices may lead to improved mental and cognitive functioning and may also benefit stress-induced cellular aging. Another report found that Kirtan Kriya reversed the pattern of increased nuclear factor κB (NF-κB)-related transcription of proinflammatory cytokines and decreased interferon regulatory factor 1–related transcription of innate antiviral response genes in distressed dementia caregivers. This finding reinforces the relationship between stress reduction and beneficial immune response.[52]

RESEARCH APPLICATIONS TO MENTAL HEALTH

Psychosocial Well-Being

The evidence base for Tai Chi on psychosocial well-being was evaluated in a meta-analysis published in 2010[53] and a systematic review published in 2009.[54] The meta-analysis identified 40 studies (17 RCTs, 16 nonrandomized comparison studies [NRSs], and 7 observational studies) with a total of 3817 individuals reporting at least 1 psychological health outcome from a search of 11 English and Chinese databases. Twenty-one of the 33

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RCTs and NRSs found that in community-dwelling participants between 1 hour up to 1 year of regular Tai Chi significantly increased psychological well-being, reduced stress (effect size [ES], 0.66; 95% confidence interval [CI], 0.23–1.09), anxiety (ES, 0.66; 95% CI, 0.29–1.03), and depression (ES, 0.56; 95% CI, 0.31–0.80), and enhanced mood (ES, 0.45; 95% CI, 0.20–0.69).

The review concluded that "Tai Chi appears to be associated with improvements in psychological well-being including reduced stress, anxiety, depression and mood disturbance, and increased self-esteem. Definitive conclusions were limited due to variation in designs, comparisons, heterogeneous outcomes, and inadequate controls. High-quality, well-controlled, longer randomized trials are needed to better inform clinical decisions."[53] This systematic review limited analysis to 15 RCTs published in English because of concerns about study quality in the non-English literature. The reviewers identified a subset of 8 high-quality trials that together included evaluations of anxiety, depression, mood, stress, general mental health, anger, positive and negative effect, self-esteem, life satisfaction, social interaction, and self-rated health. Tai Chi was found to have a significant positive effect in 13 of the 15 studies, and in 6 of the 8 high-quality trials. Earlier reviews have concluded that Tai Chi seems to improve psychosocial well-being.[55–57]

The effects of Tai Chi on self-esteem have been evaluated in 3 RCTs.[58] All of these studies found an increase in self-esteem compared with control groups, but only 1 produced statistically significant between-group results.[58] That study randomized 21 women diagnosed with breast cancer who had completed treatment within the last 30 months to receive 12 weeks of Tai Chi or psychosocial support 3 times a week. A review of 51 studies of general exercise found that aerobic exercise is effective in improving self-esteem.[59]

Stress Management

The most logical clinical application of mind-body techniques is for stress reduction. Out of 5 RCTs, 4 found a significant association between Tai Chi and Qigong and positive effects on stress.

1. Beneficial effects were found in a study in 2008[60] that evaluated a population infected with the human immunodeficiency virus (mean age 42 years, n = 252) randomized to practice Tai Chi for 90 minutes once weekly for 10 weeks or to a cognitive-behavioral relaxation group, spiritual growth group, or a wait-list control group.
2. A study in 2001[61] that used a healthy geriatric population (mean age 73 years, n = 72) randomized to practice Tai Chi for 1 hour twice weekly for 24 weeks versus a wait-list control group also found that Tai Chi reduced stress.
3. Tai Chi was shown to reduce stress in a study in 1996[62] that evaluated health among older adults (mean age 67 years, n = 20) who practiced Tai Chi for 2 hours, once weekly for 10 sessions compared with a routine physical activity group.
4. Positive effects were found in a study in 1992[45] that looked at healthy adults (mean age 36 years, n = 96) who underwent a single, 1-hour session of Tai Chi versus meditation, brisk walking, or neutral reading.
5. One RCT did not find positive results; a study in 2007[63] that evaluated individuals with hip or knee osteoarthritis (mean age 70 years, n = 152) who practiced Tai Chi for an hour twice weekly for 12 weeks found no significant difference compared with a hydrotherapy or wait-list control group.

All studies used subjective stress measures, 1 measured body temperature,[62] and 2 measured salivary cortisol levels,[60] which decreased as a result of Tai Chi practice.

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Mood, Anxiety, and Depression

Seven RCTs have found that Tai Chi significantly improves mood, including:

- A 2011 study of 100 outpatients (mean age 67 years) with systolic heart failure who received either a 12-week Tai Chi exercise program or time-matched education.
- A study published in 2009 randomized 21 obese women to either a 2-hour weekly session of Tai Chi or a conventional structured exercise program and found that only the Tai Chi group experienced improvements in mood.
- Benefits to mood were also found in a 2005 study of 38 adults (20–60 years old) with advanced HIV/AIDS who participated in 8 weeks of twice weekly, 1-hour-long Tai Chi practice versus aerobic exercise or usual activity.[64]
- A 1995 study of 135 healthy, sedentary adults (mean age 53 years) who practiced Tai Chi for 45 minutes, 3 times a week, for 16 weeks reported improved mood compared with exercise and relaxation control groups.[65]
- Three of the trials described earlier[45,60,61] also found benefits to mood.

An RCT[66] found no significant impact of Tai Chi on mood. That trial randomized 22 community-dwelling participants (mean age 68 years) with lower extremity osteoarthritis to 12 weeks of twice-weekly, 1-hour-long Tai Chi sessions or to a control group. Tai Chi was found to improve pain, physical function, and other arthritis symptoms (measured using the Arthritis Self-Efficacy Scale) as well as satisfaction with general health status, but it did not result in a statistically significant difference in mood.

Ten RCTs have investigated the effects of Tai Chi on anxiety, 9 of which showed significant positive effects.

- A Japanese trial in 2010[67] evaluated 34 community-dwelling elderly participants with cerebral vascular disorder who were randomized to receive either Tai Chi or standard rehabilitation in group sessions once weekly for 12 weeks. Participants in the Tai Chi group experienced improvements in sleep quality, anxiety/insomnia, and depression.
- A study in 2008[33] randomized 20 patients with rheumatoid arthritis to 12 weeks of twice-weekly sessions of Tai Chi or attention control. Participants in the Tai Chi group experienced greater improvements in anxiety and depression than the control group.
- A study in 2007[35] of 65 patients (mean age 70 years) with chronic heart failure received 16 weeks of either Tai Chi practice twice weekly or standard medical care without exercise rehabilitation. They reported that both groups had a significant reduction in anxiety scores, found no between-group differences in anxiety, and found that depression was reduced only in the Tai Chi group.
- A study in 2007[68] of 84 sedentary older people (mean age 70 years) contrasted Tai Chi with low-impact exercise for 12 weeks and found that both groups of patients experienced improvements in anxiety.
- A study in 2003[69] of 76 healthy individuals (mean age 52 years) found improvements in anxiety for participants who received 12 weeks of 50-minute Tai Chi sessions 3 times a week compared with a sedentary life control group.
- Improvements to anxiety were also found in 3 RCTs described earlier.[45]
- One RCT described earlier[63] did not find a significant effect on anxiety.

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Fourteen RCTs have evaluated the effects of Tai Chi and Qigong on depressive symptoms, 13 of which found positive results. Several of these RCTs have already been described.[67]

- A single-blind, 12-week study[70] of participants with fibromyalgia (mean age 50 years, n = 66) published in 2009 found that Tai Chi produced greater improvements in depression than a stretching and wellness education group.
- Another single-blind, 12-week trial in 2009[71] randomized 40 patients (mean age 65 years) with knee osteoarthritis to receive either Tai Chi or wellness education and stretching and found that patients in the Tai Chi group experienced greater improvement in depression.
- An RCT in 2008 evaluated 14 community-dwelling older patients from a psychogeriatric outpatient clinic who were randomized to receive a 3-month Tai Chi intervention or to a wait-list control. Only the Tai Chi group experienced improvements in depressive symptoms.
- A trial in 2007 randomized 112 healthy older adults (aged 59–86 years) to 25 weeks of either Tai Chi or health education. Participants in both groups experienced improvements in depressive symptoms.
- An RCT in 2005[72] of 291 women and 20 men (aged 70–97 years) recruited from 10 matched pairs of congregate living facilities found that 48 weeks of Tai Chi led to a significantly greater reduction in depression than wellness education (P <0.001).
- A Chinese trial in 2004[21] randomized 14 elderly persons (mean age 73 years) with depression to 12 weeks of Tai Chi 3 times a week for 45 minutes or to a wait-list control group. Only the Tai Chi group experienced improvements in depressive symptoms.
- A trial in 1998[73] of 51 patients aged 18 to 60 years with chronic low back pain reported improvements in depressive symptoms compared with a control group after 6 weeks practicing Tai Chi once a week for 90 minutes.
- Only 1 RCT did not find evidence that Tai Chi and Qigong are effective in reducing depressive symptoms or had any effect on anxiety or stress management.[63]

Most of these studies were conducted in patient populations without known mental disorders. Only 2 studies involved participants with clinically diagnosed depression.[72] A review in 2009[74] of Tai Chi and Qigong in older adults found 36 clinical trials with 3799 participants and concluded that Tai Chi and Qigong practice causes significant improvement in depression and anxiety. Tai Chi has been particularly recommended as a first-line treatment of mild depression in geriatric populations given its known benefits in improving balance and reducing falls.[75] Depression and falls are associated through a complex bidirectional relationship.[76] Antidepressant use has also been associated with falls,[77] especially selective serotonin reuptake inhibitors, which are associated with fragility fractures to a higher degree than other classes of psychotropic medications.[78,79]

On the other hand, more recent research has produced mixed results on the effectiveness of Tai Chi and Qigong for prevention of falls. One Cochrane collaboration meta-analysis[80] found that Tai Chi had a moderate effect on reducing falls in community-based geriatric populations, and a second meta-analysis[81] found insufficient evidence to support the use of Tai Chi for prevention of falls. One of the most recent RCTs of Tai Chi as a community-based falls prevention intervention was an 11-site multicenter study conducted in New Zealand.[82] A total of 684 community-residing older adults with at least 1 risk factor for falls were randomized to receive 20 weeks of either Tai Chi once a week, Tai Chi twice a week,

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or general exercise once a week. All groups experienced a reduction in rate of falls; however, there was no statistically significant difference between groups over the 17-month follow-up period.

Sleep Disturbance

Tai Chi and Qigong may also be able to improve sleep quality, with corresponding impact on mental health.

- An RCT[6] of Tai Chi for improving sleep quality in older adults with moderate sleep complaints randomized[67] 112 participants to receive 25 weeks of either Tai Chi or health education. The study found that participants in the Tai Chi group were more likely to achieve a treatment response and to show global improvements in sleep quality.
- A second RCT[83] in a geriatric (aged 60–92 years) population (n = 118) found that Tai Chi, practiced for an hour, 3 times weekly for 24 weeks, was more effective than general exercise at improving sleep quality and daytime sleepiness.
- A third RCT,[67] described earlier, found that 12 weeks of Tai Chi practice once a week was more effective at improving sleep quality than a rehabilitation control group. Chronic sleep problems are associated with impaired health status and depressive symptoms.[84]
- A fourth RCT[85] assigned 102 community-dwelling participants (mean age 68.9 years) in Vietnam to receive 6 months of Tai Chi training or to maintain their routine daily activities. Compared with the control group, individuals in the Tai Chi group experienced significant improvements in sleep quality, balance, and cognitive performance.

Substance Abuse

A Chinese RCT[86] of 86 patients randomized to a Qigong treatment group, medication group, or no-treatment control group reported that participants in the Qigong group experienced comparatively fewer withdrawal symptoms. Qigong was also credited with a lower relapse rate and improved anxiety scores. A nonrandomized controlled trial of 248 patients in a short-term residential treatment program who self-selected participation in either a Qigong meditation program or stress management plus relaxation program reported that Qigong participants experienced a higher treatment completion rate and greater reduction in cravings.[87] Participants were offered Qigong meditation twice daily, 5 or more days a week, for a total of 2 weeks. The study noted that female Qigong participants reported significantly more reduction in anxiety and withdrawal than any other group.

Cognitive Functioning

Published interim results from a year-long Chinese RCT[88] suggest that Tai Chi may provide a cognitive benefit. The study randomized 389 geriatric participants with dementia or amnestic mild cognitive impairment to either a Tai Chi group or a strengthening and toning exercise group. After 5 months of triweekly practice sessions, both groups showed improvements in global cognitive function, delayed recall, and subjective complaints. Only the Tai Chi group maintained a stable clinical dementia rating and showed improvements in visual spans.

Another RCT of healthy community-dwelling older adults (mean age 69 years, n = 132) found that Tai Chi produced greater improvements in a cognitive function measure than a Western exercise or attention control group. The improvement in cognitive functioning was

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maintained throughout the 12-month follow-up period. An RCT described earlier found that Tai Chi improves motor speed and visual attention in elderly individuals.

Parkinson Disease

An RCT in 2012[89] of 195 patients found that 24 weeks of Tai Chi was more effective than resistance training or stretching at improving primary balance outcomes (maximum excursion and directional control). The Tai Chi group also performed better than the stretching group in all secondary balance measures, including strength, functional reach, timed up-and-go tests, motor scores, and number of falls. The Tai Chi group performed better than the resistance group in stride length and functional reach. The effects of Tai Chi training were maintained 3 months after the end of the intervention, and no serious adverse events were observed. The study concluded that Tai Chi seems to reduce balance impairments in patients with mild-to-moderate Parkinson disease, with the additional benefits of improving functional capacity and reducing falls.

Other trials of Tai Chi and Qigong in populations with Parkinson disease have found similar results. An RCT in 2008 (n = 33) found that 20, 1-hour sessions of Tai Chi were effective at improving several balance measures, and at improving well-being compared with a no-intervention control group.[90] Another RCT (n = 30) found that a 12-week Tai Chi program was effective in reducing falls and slowing functional decline.

Traumatic Brain Injury

With the increased interest in traumatic brain injury (TBI), an RCT of 20 patients with TBI found that participation in Qigong improved mood and self-esteem relative to a nonexercise control group, but it found no difference in physical functioning between groups. Participants in that study attended a Qigong exercise session for 1 hour per week over 8 weeks, whereas control participants engaged in non–exercise-based social and leisure activities. A second RCT[91] evaluated 18 participants with TBI assigned to either a wait-list control or Tai Chi group and found that Tai Chi provided short-term benefits after TBI. The participants in the 6-week Tai Chi course had improved outcomes in HRQOL, self-esteem, and mood. Patients with TBI often suffer from cognitive, emotional, and mental challenges.[92]

See Table 1 for a summary of RCTs of Tai Chi and Qigong for mental disorders.

SUMMARY/DISCUSSION

Complementary Approaches to Pharmacologic Strategies

Tai Chi and Qigong are nonpharmacologic treatments that can be used in conjunction with pharmacologic treatments. Nonpharmacologic approaches to mental disorders are particularly important given that many patients fail to achieve symptomatic remission and functional recovery with first-line pharmacotherapy.[99,100] Treatments are needed to complement pharmacotherapies to help patients achieve remission, experience reductions in mental disorder symptoms, and enjoy improved social and health functioning.[101]

Limited research has specifically evaluated Tai Chi and Qigong as a combined treatment with pharmaceutical intervention. A study of older adults with major depression found that the complementary use of Tai Chi augments the use of escitalopram (Lexapro). The study randomized 73 partial responders to escitalopram, who continued to use escitalopram daily, to a 10-week course of either Tai Chi or health education. Individuals in the Tai Chi group were more likely to report greater reduction in depressive symptoms and to achieve a depression remission ($F_{[5, 285]} = 2.26$; $P<.05$). Those individuals also showed greater

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improvements in HRQOL physical functioning (group × time interaction: $F_{[1, 66]} = 5.73$; $P = .02$) and cognition (ie, memory; group × time interaction: $F_{[1, 65]} = 5.29$; $P<.05$), and a decline in an inflammatory marker, CRP (time effect: $F_{[2, 78]} = 3.14$, $P<.05$ and group × time trend in posttreatment period: $F_{[1, 39]} = 2.91$; $P = .10$).[14]

Clinical Recommendations

There is not strong evidence that Tai Chi and Qigong are effective as either primary or complementary treatments for mental disorders. Only a few mental and neurologic disorders have been specifically evaluated in the literature. As the evidence for Tai Chi and Qigong continues to develop, promising results from multiple RCTs suggest that these are potentially effective treatments for reducing stress, anxiety, depression, and low mood, as well as for improving self-esteem and general psychosocial well-being. Results from the RCTs evaluating Tai Chi and Qigong for specific mental diseases suggest that they may be effective for improving symptoms of Parkinson disease, TBI, sleep disturbance, substance abuse, and cognitive impairment. There remains a pressing need for methodologically robust studies of Tai Chi and Qigong for mental disorders. Multiple RCTs may have produced mixed results on the efficacy of Tai Chi and Qigong for a particular indication because of the variations in designs, comparisons, patient populations, and interventions.[54] Few studies evaluated patient populations with diagnosed mental disorders.

Given that Tai Chi and Qigong are nonpharmacologic and noninvasive treatments, recommending these exercises to patients with mental disorders generally seems an appropriate option for clinicians, particularly for conditions that have been studied in RCTs. Whether or not Tai Chi and Qigong can improve disease-specific outcomes, significant evidence supports the assertion that Tai Chi and Qigong can improve HRQOL and mental health. Tai Chi and Qigong may be particularly appropriate for patients who have physical comorbidity known to be responsive to Tai Chi and Qigong practice, in geriatric populations, who are more susceptible to adverse effects from pharmacologic therapies, or in patients who choose to use exercise or mind-body practices.

Instructor quality may affect patient outcomes, but this has not been addressed in clinical research. There is no licensing body that regulates Tai Chi teachers and no certifying body that clearly distinguishes higher-quality instructors. Consumers in this market rely largely on word of mouth and local reputation. Although Tai Chi has traditionally been taught 1-on-1 or in class settings, a variety of home exercise programs are publically available for patients who are logistically or financially unable to study in person or who prefer to study independently. For example, Beachbody, LLC (Santa Monica, California), makers of the popular home exercise program *P90X*, sell a home DVD exercise Tai Chi program, *Tai Cheng*.

Practice style, frequency, and duration have been variable. The predominant style of Tai Chi used in the RCTs evaluated in this review was the Yang style or Yang style short-form. Most styles are similar in practice although it is frequently claimed by practitioners that one style is superior. It is unclear that there is any benefit from one style versus another at this point. Regardless of the form of Tai Chi being evaluated, most studies had participants practicing 30 minutes to 2 hours, 1 to 3 times per week. Clinicians prescribing Tai Chi should consider recommending that patients practice for a minimum of 30 minutes, 3 times a week on an ongoing basis. Alternatively, those recommendations may be left to an experienced instructor. Practice durations in the literature are generally in the range of a few weeks to a few months, although studies found both short-term benefits from as little as 1 practice session and long-term benefits in multiyear practitioners. Comparative effectiveness research has not yet addressed the optimal duration of the exercise.

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KEY POINTS

- Tai Chi and Qigong are evidence-based approaches to improve health-related quality of life, and they may be effective for a range of physical health conditions.
- Evidence from randomized controlled trials suggests that Tai Chi and Qigong may be effective in reducing depressive symptoms, stress, anxiety, and mood disturbances.
- Minimal research supports Tai Chi and Qigong as promising treatments for Parkinson disease, traumatic brain injury, insomnia, substance abuse, and cognitive impairment.
- Indications for Tai Chi and Qigong include inadequate response to other evidence-based treatments, physical comorbidities known to be responsive to Tai Chi and Qigong, patients interested in exercise-based or mindfulness-based interventions, and in geriatric patients who are more susceptible to adverse effects from pharmacologic therapies.

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Table 1

The summary of the RCTs of Tai Chi and Qigong for mental disorders

	Positive Findings	Negative Findings
Depression	Thirteen studies with significant positive findings,[11,21,33,34,61,63,65–73,93]; only 2 with clinically diagnosed depression populations[21,72]	One study did not find effect on depressive symptoms[63]
Stress	Four studies with significant positive findings,[60] subjective stress measures,[45,60–63] body temperature,[62] and salivary cortisol levels[45,60]	One study did not find effect on subjective stress measures[63]
Anxiety	Eight studies with significant positive effects[67]	One study had negative findings on anxiety[35,63]
Mood and psychological well-being	Seven studies with significant positive effects[93,94]	One study did not find positive effect on mood[66]
Self-esteem	One study with significant positive effects[58]	Two without positive effect[65,95]
Parkinson disease	Three studies with significant positive effects[89,90,96]	
TBI	Two studies with significant positive effects[91,97]	None reported
Sleep disturbance	Three studies with significant positive effects[67,83,91]	None reported
Substance abuse	One study with significant positive effects[86]	None reported
Cognitive functioning	Two studies with significant positive effects[88,98]	None reported

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PERSPECTIVE
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Treating Depression With Tai Chi: State of the Art and Future Perspectives

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Major depressive disorder (MDD) is one of the most prevalent mental illnesses in America. Current treatments for MDD are unsatisfactory given high non-response rates, high relapse rates, and undesirable side effects. Accumulating evidence suggests that Tai Chi, a popular mind–body intervention that originated as a martial art, can significantly regulate emotion and relieve the symptoms of mood disorders. In addition, the availability of instructional videos and the development of more simplified and less structured Tai Chi has made it a promising low-intensity mind-body exercise. In this article, we first examine a number of clinical trials that implemented Tai Chi as a treatment for depression. Then, we explore several mechanisms by which Tai Chi may alleviate depressive symptoms, hypothesizing that the intervention may modulate the activity and connectivity of key brain regions involved in mood regulation, reduce neuro-inflammatory sensitization, modulate the autonomic nervous system, and regulate hippocampal neurogenesis. Finally, we discuss common challenges of the intervention and possible ways to address them. Specifically, we pose developing a simplified and tailored Tai Chi protocol for patients with depression, comparatively investigating Tai Chi with other mind–body interventions such as yoga and Baduanjin, and developing new mind–body interventions that merge the advantages of multiple mind–body exercises.

Keywords: Tai Chi, mind–body intervention, major depressive disorder, depression, brain network, anti-inflammation

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INTRODUCTION

Major depressive disorder (MDD) is a highly prevalent mental illness in the United States (1). Psychotherapy and pharmacotherapy are the mainstay treatments for MDD (2). However, treatment of the disorder is associated with substantial direct and indirect costs, and the clinical efficacy of the treatments has been criticized (3–5). Psychotherapy poses significant time costs on both health professionals and patients, and many patients are troubled by medication side effects such as "sexual dysfunction, weight gain, and sleep disturbance" (6). A significant proportion of patients respond only partially to antidepressants and may require augmentation with other agents in order to enhance the limited effect of the medication (7). Given the disadvantages of first-line treatments, some researchers have begun to explore the effectiveness of alternative therapies.

Tai Chi, a popular mind–body intervention, has recently drawn the attention of the public and researchers alike. Traditional Tai Chi training requires direct supervision from mentors following strict postures. With modern technology and video-sharing sites like Youtube, Tai Chi is becoming increasingly accessible to the general public. We feel it qualifies as a low-intensity exercise, for 1) it can be practiced with video alone and does not require the assistance of a formally trained Tai Chi

instructor, and 2) more simplified and less structured Tai Chi has been developed in recent years to accommodate different populations.

Tai Chi employs slow, gentle movements, breathing techniques, and cognitive tools (i.e., attention, imagery) to strengthen, integrate, and relax the body and mind (8). It can be practiced by people of all age groups with varying physical conditions and requires very little physical space. The benefits of Tai Chi on MDD have been supported by a number of well-designed studies (9–11). An advantage of using Tai Chi to treat or augment treatment of MDD is that it is safe and not associated with the adverse events commonly seen with pharmacological agents.

Although promising, Tai Chi's therapeutic effect needs to be further explored, as the underlying mechanism of the intervention remains unclear. Although different styles of Tai Chi exist, none of these styles have been developed to specifically target patients with depression. The complexity of Tai Chi exercise further inhibits its application in patients with depression. Thus, there is an urgent need to develop a simplified Tai Chi protocol that is tailored for depression.

In this manuscript, we first summarize the findings of clinical studies on Tai Chi treatment of depression. Then, we attempt to summarize the potential mechanisms by which the intervention treats depressive symptoms. Finally, we propose a new direction of Tai Chi research, including a new Tai Chi protocol based on these putative mechanisms. Please also see several recently published review papers on the beneficial effects of Tai Chi for individuals with depression and mood disorders (8, 12, 13).

Potential of Tai Chi Treatment of Depression—Results From Clinical Studies

In an earlier study, Chou (14) investigated the effects of Tai Chi on the depressive symptoms of 14 older Chinese patients. Researchers found that 3 months of Tai Chi intervention can significantly reduce scores on the Center for Epidemiological Studies Depression Scale (CES-D) and all of its subscales (including symptoms related to somatic, negative affect, interpersonal relations, and well-being) as compared to a waitlist control. These decreased scores remained significant after controlling for age, gender, and education but not after controlling for social support changes, as measured by the Lubben Social Network Scale (LSNS). This finding suggests that social support may contribute to the effect of Tai Chi on depressive symptoms. This was one of the earliest studies investigating the effect of Tai Chi on depression with positive and notable findings. However, it was limited by a small sample size and the use of a passive control.

In a follow-up study, Lavretsky et al. (15) explored if SSRI (escitalopram) treatment supplemented with 10 weeks of Tai Chi, as compared to health education (HE), would enhance depression treatment in 73 older adults. They found that patients in the Tai Chi-supplemented condition were more likely to 1) experience a greater improvement in depressive symptoms or achieve depression remission and 2) have greater improvements in C-reactive protein levels and the 36-Item Short Form Health Survey physical functioning and cognitive tests compared to the control group. These findings suggest that supplementing pharmacologic treatment with Tai Chi may yield greater clinical improvement for individuals with geriatric depression. This study had a large sample size and obtained positive findings in both patients' subjective ratings and in inflammatory marker levels. It demonstrates the benefits of adding Tai Chi to an antidepressant regimen but does not examine the specific effect of Tai Chi on depression.

Field et al. (16) investigated the effects of combined Tai Chi/yoga in 92 prenatally depressed pregnant women. They found that women practicing Tai Chi/yoga (20 min per week for 12 weeks) had lower depression, anxiety, and sleep disturbance scores compared to a waitlist control group (**Table 1**). This study had a large sample size and provided important evidence on the effects of Tai Chi on depressed pregnant women, who generally would avoid pharmacologic treatment. However, combining Tai Chi and yoga is uncommon in the real world, and a waitlist is considered a weak control.

In another study, Yeung and colleagues (10) examined the practicality and outcome of using Tai Chi to treat depressive symptoms in 39 Chinese Americans with MDD. They found that 73% of patients in the Tai Chi group completed the intervention, and no adverse events were reported. This was a proof-of-concept study with a small sample size to investigate the feasibility and safety of Tai Chi for depressed Chinese Americans, a population that tends to avoid conventional mental health services due to their high levels of stigma against having mental illnesses. In a following study with a larger sample size ($n = 67$) (11), researchers found that following a 12-week intervention, "response rates were 25%, 21%, and 56%, and remission rates were 10%, 21%, and 50% for the waitlisted, education, and Tai Chi intervention groups, respectively." Participants randomized to the Tai Chi group experienced a greater response to treatment than did individuals randomized to the waitlist and education groups. Further, participants in the Tai Chi group experienced a significantly greater remission rate than the waitlist group and "a trend of improved remission compared to the education group" (**Table 1**). This study provided preliminary evidence on the effectiveness of Tai Chi on depressed Chinese Americans, using both passive and active control groups. When the Tai Chi group was compared to the passive control group, improvements in both response rate and remission rate showed statistical significance. Yet, when the Tai Chi group was compared to the active control group, improvement in response rate showed statistical significance but not for improvement in remission rate. The relatively small sample in each of the study groups may explain this negative finding. Studies with larger sample sizes will be needed to provide a more definitive conclusion.

Depression is also a common disorder among elderly adults, and many studies have explored depression treatment for this population (17, 18). Brown et al. (19), for instance, compared psychological changes associated with 16 weeks of moderate-intensity walking (MW), low-intensity walking (LW), low-intensity walking plus relaxation response (LWR), and Tai Chi in healthy, sedentary adults. They found that women in the Tai Chi group experienced decreased mood disturbance (tension, depression, anger, confusion, and total mood disturbance) and

TABLE 1 | Tai Chi studies applied on patients with depression and primary outcomes.

Study	Patient population demographics/sample size	Setting	Treatment (*n*)/ control (*n*)	Treatment-related information	Primary outcome measures	Major results
Yeung et al. (10)	39 Chinese Americans with MDD; 77% women; mean (SD) age = 55 (10)	Group Tai Chi class in Boston, MA; taught in Chinese (Cantonese, Mandarin)	Tai Chi: *n* = 26 Waitlist: *n* = 13	Tai Chi: 1-h class twice a week, 12 weeks Waitlist: 12 weeks	Depression severity following treatment, as measured by the HAM-D_{17}	Response and remission rates were better in the Tai Chi group versus the waitlist group. However, these differences were not significant (p = 0.15 and p = 0.30 for response and remission rates, respectively). Tai Chi was proven safe and feasible for Chinese Americans.
Chou (14)	14 Chinese patients with MDD, ages 60 and older	Classes led by Tai Chi instructor in group setting	Tai Chi: *n* = 7 Waitlist *n* = 7	Tai Chi: 3 × 45 min/week, 12 weeks Waitlist: 12 weeks	Depression severity following treatment, as measured by the CES-D	Tai Chi can yield a reduction in depressive symptoms compared to a waitlist control (main effect of group assignment: 0.82, $p < 0.01$). Controlling for social support between groups removes any benefit of Tai Chi on CES-D scores. Thus, social support may contribute to the effects of Tai Chi on depressive symptoms.
Lavretsky et al. (15)	73 adults over 60 with MDD; Escitalopram with Tai Chi group: 64% women, mean (SD) age = 69.1 (7.0); Escitalopram with health education (HE) group: 60% women, mean (SD) age = 72.0 (7.4)	Both Tai Chi and HE classes were conducted by study staff. Tai Chi classes included a warm up and cool down. HE classes included lectures and discussion.	Escitalopram with Tai Chi: *n* = 36 Escitalopram with HE: *n* = 37	Escitalopram with Tai Chi: 2 h/week, 10 weeks Escitalopram with HE: 2 h/ week, 10 weeks	Depression severity following treatment, as measured by the HAM-D_{24}; subjects were classified as "remission," "response," and "nonresponse" according to score	A higher percentage of participants achieving remission and response was observed in the escitalopram with Tai Chi group than in the escitalopram with HE group ($p < 0.05$). Compared to HE, Tai Chi may better augment the effects of SSRI medication in the treatment of major depression.
Field et al. (16)	92 pregnant women with MDD; mean (SD) age = 26.6 (5.5); range = 18–37	Participants recruited from medical clinic; classes taught in group setting	Tai Chi with Yoga: *n* = 46 Waitlist: *n* = 46	Tai Chi with yoga: 20 min/ week, 12 weeks Waitlist: 12 weeks (received Tai Chi/yoga intervention after initial 12 weeks)	Depression severity following treatment, as measured by the CES-D	Tai Chi with yoga intervention yielded a greater reduction in depressive symptoms than the waitlist control (p = 0.001).
Yeung et al. (11)	67 Chinese Americans with MDD; ages 18 to 70	Group Tai Chi class in Boston, MA; taught in Chinese (Cantonese, Mandarin)	Tai Chi with yoga: *n* = 23 Waitlist: *n* = 22 Healthy education: *n* = 22	Tai Chi: 1-h class twice a week, 12 weeks Waitlist: 12 weeks Healthy education: 1-h class twice a week, 12 weeks	Response and remission rates following treatment, as measured by $HDRS_{17}$	Tai Chi intervention yielded greater response and remission rates than the waitlist group (odds ratio for response = 2.11, 95% CI; odds ratio for remission = 3.01, 95% CI). Tai Chi intervention yielded a significantly greater response rate (odds ratio = 8.90, 95% CI) but an insignificantly greater remission rate (odds ratio = 4.40, 95% CI) compared to HE group.

MDD, major depressive disorder; HE, health education; CES-D, Center for Epidemiological Studies–Depression Scale; HAM-D_{17}/$HDRS_{17}$, Hamilton Rating Scale for Depression (17 items); HAM-D_{24}, Hamilton Rating Scale for Depression (24 items); CI, confidence interval.

an improvement in general mood. Women randomized to the MW group also reported increased satisfaction with physical attributes (body cathexis), while men in the same group reported increased positive affect. These results suggest that mind–body interventions such as Tai Chi may have more psychological benefits than exercises without a cognitive component, thus demonstrating Tai Chi's value in promoting mental health.

Finally, there is accumulating evidence that Tai Chi can relieve depressive symptoms in patients with fibromyalgia (20, 21), arthritis (22–24), multiple sclerosis (MS) (25), heart failure (26–28), mild dementia (29), and cerebrovascular disorder (30).

Mechanisms of Tai Chi Treatment of Depression

Tai Chi Can Modulate the Brain Regions/Networks Associated With Depression

Studies have shown that depression is linked to structural and functional abnormalities in brain regions that are associated with emotion processing, self-representation, reward, and external stimulus (i.e., stress, distress) interactions (31–39). Among these brain regions are the hippocampus, amygdala, anterior cingulate, ventromedial prefrontal cortex, and dorsomedial prefrontal cortex.

Studies also suggest that core components of mind–body interventions such as Tai Chi may include attentional control, emotion regulation, and self-awareness (40). Although no brain-imaging study has directly investigated the modulation effect of Tai Chi in patients with depression, brain-imaging studies in healthy subjects and other patient populations have endorsed the potential pathways of Tai Chi modulation. Furthermore, results from intervention studies support that the target brain regions outlined in this manuscript can indeed be altered by various interventions in patients with depression (**Figure 1**). For instance, a systematic review by Gudayol-Ferre et al. notes that "antidepressive treatment is capable of normalizing brain activations in depressed patients during affective tasks in areas such as the DLPFC," and treatments for depression are associated with alterations in the default mode network (DMN) (41).

One potential mechanism of Tai Chi on MDD may occur *via* the cognitive (attentional) control network (40). During Tai Chi, attention is focused on body posture/movement and breathing regulation and is shifted away from the stressor. Thus, repeated Tai Chi exercise (movement in a mindful way) can modulate the attentional control network and relieve depressive symptoms, and noradrenaline may be heavily involved in attention regulation (42).

In support of this hypothesis, we found that Tai Chi can significantly modulate resting state functional connectivity (rsFC) of the dorsolateral prefrontal cortex (DLPFC), a key region of the cognitive (attentional) control network, with the medial prefrontal cortex and anterior cingulate cortex (MPFC/ACC),

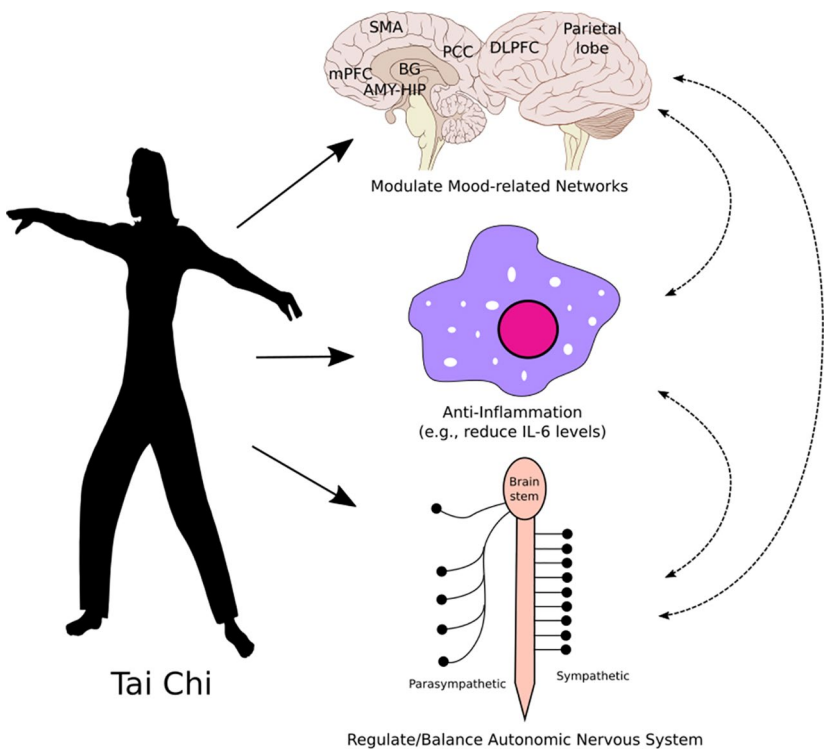


FIGURE 1 | Hypothesized mechanisms of Tai Chi treatment of depression: directly and indirectly modulating the activity and connectivity of key brain regions involved in depression and mood regulation, reducing neuro-inflammatory sensitization, and modulating the autonomic nervous system. Abbreviations: AMY, amygdala; BG, basal ganglia; DLPFC, dorsolateral prefrontal cortex; HIP, hippocampus; mPFC, medial prefrontal cortex; PCC, posterior cingulate cortex; SMA, supplementary motor area.

key regions of the DMN and limbic system, in healthy elderly adults (43) and patients with fibromyalgia (44). In another study, Wei et al. found that Tai Chi may decrease fractional amplitude of low-frequency fluctuations (fALFF) in the bilateral frontoparietal network (executive/attentional control network). Researchers also identified an association between cognitive control performance and fALFF of the frontoparietal network (45). In a more recent study (46), investigators found that Tai Chi can significantly decrease the functional connectivity between the DLPFC and the thalamus, ventral striatum, and right middle frontal gyrus compared to the control group.

Studies suggest that Tai Chi can also modulate the DMN, a brain network involved in self-referential processing, affective cognition, and emotion regulation (47–54). For instance, results from a previous study revealed significantly decreased fALFF in the Tai Chi condition compared to a control, as well as a significant association between mind–body practice experience and fALFF in the DMN (45). As mentioned above, we also found that Tai Chi can modulate functional connectivity between the cognitive control network and key regions of the DMN (MPFC/ACC), indicating that Tai Chi can modulate the interaction of the two networks. One of the most reliable brain-imaging findings on MDD is the disruption of the DMN (55–61). Thus, Tai Chi may also relieve depressive symptoms by modulating the DMN. In addition, studies have shown that Tai Chi can significantly modulate brain structures in areas related to mood regulation, such as the insula, putamen, and medial temporal pole (62, 63). Taken together, the above findings suggest that the central nervous system may play an important role in the modulation effect of Tai Chi.

Tai Chi may Relieve Symptoms of Depression by Reducing Stress and Modulating the Inflammation System

Stress is produced as the brain and body respond to demands. Literature suggests that stress, particularly long-term stress, may initiate cognitive, affective, and biological processes that increase one's risk for depression (64, 65) and that inflammation may be involved in this process. Specifically, stress-provoked neuro-inflammatory sensitization can lead to significant behavioral changes and the onset of common depressive symptoms, such as sad mood, anhedonia, fatigue, psychomotor retardation, and social–behavioral withdrawal (64–67). The hypothalamus, anterior insula, and ACC may be involved in this process (64).

Recent studies have suggested that mind–body interventions like Tai Chi may reduce stress and modulate the inflammation process (68, 69) (**Figure 1**). For instance, Jin (70) found that Tai Chi can raise heart rate, increase noradrenaline excretion in urine, and decrease salivary cortisol concentrations. Compared to baseline levels, subjects reported less tension, depression, anger, fatigue, confusion, and anxiety and felt more vigorous. Irwin and Olmstead (71) evaluated the effects of Tai Chi on circulating markers of inflammation in older adults and found that, among those with elevated Interleukin 6 (IL-6) at entry, Tai Chi yielded a reduction in IL-6 levels comparable to those found in Tai Chi and HE subgroups who had low levels of IL-6 at entry. Meanwhile, IL-6 in HE subgroups remained higher than the Tai Chi and HE subgroups with low IL-6 at entry. Depressive symptom decreases in the two treatment groups were correlated with IL-6 decreases.

Finally, Tai Chi is a mild to moderate exercise, and many studies (72, 73) have suggested that physical exercise itself can modulate the immune and inflammation systems. Such findings endorse the role of Tai Chi in reducing stress and producing anti-inflammatory effects.

Other Potential Mechanisms

Previous studies have suggested that MDD is associated with decreased activity of the parasympathetic nervous system (74–77). Heart rate variability (HRV) is a noninvasive index for monitoring the dynamic equilibrium between sympathetic and parasympathetic nervous system activity. Studies have shown that the high-frequency (HF)–HRV component is inversely correlated with depression severity and a marker of treatment response (75, 78–80).

Lu and Kuo (81) found that Tai Chi can yield increased vagal modulation and shift the sympathovagal balance towards decreased sympathetic modulation in elderly individuals. Audette and colleagues (82) compared the effects of a shortened Tai Chi regimen and a brisk walking training program on metrics such as aerobic capacity, HRV, and strength in elderly sedentary women, and researchers found a significant improvement in estimated VO2 max in the Tai Chi group. In the Tai Chi group only, the mean within-person change of HF power in normalized units was increased, representing increased parasympathetic activity. Meanwhile, low-frequency (LF) power in normalized units (nu) decreased, representing decreased sympathetic activity. In a recent meta-analysis on 17 randomized controlled trials (RCTs), investigators (83) found that combined Tai Chi/yoga produced significantly beneficial effects on HRV parameters (normalized LF, normalized HF, and LF-to-HF ratio) and stress level. These results suggest that the autonomic nervous system may also be involved in the modulation effects of Tai Chi on depressive symptoms (**Figure 1**).

According to the neurogenic theory (84), depression results from impaired adult hippocampal neurogenesis, the restoration of which leads to recovery. Studies have shown that Tai Chi can significantly modulate the gray matter volume of the hippocampus (63) and rsFC of the hippocampus with the medial prefrontal cortex, a key region of the DMN (85). These findings imply that Tai Chi may alter hippocampal neurogenesis to reduce depressive symptoms (86). The various mechanisms by which Tai Chi may improve depressive symptoms is illustrated in **Figure 1**.

Challenges and Future Directions

Simplification of Tai Chi Protocol

The complexity of some Tai Chi movements has significantly limited its application as a clinical intervention. Also, not all elements of Tai Chi are relevant to mood regulation and the maintenance of health. Thus, there is a pressing need to develop simplified Tai Chi protocols.

The simplification of Tai Chi has recently begun in China, its country of origin. An example of such simplification is the eight-style Tai Chi that is considered a part of the Chinese martial arts system. Eight-style Tai Chi consists of 10 total postures,

including the beginning, the end, and eight main actions from Yang-style Tai Chi.

One attempt to simplify Tai Chi in the Western world is the 12-week Tai Chi protocol developed by Dr. Peter Wayne. The first section introduces several traditional Tai Chi warm-up exercises that can last 15–30 min. The second section focuses on five core Tai Chi movements following the Cheng Ma Ching Yang-style short form. Participants progressively add these movements over the course of the 12 weeks. The program concludes with 5 min of simple cool-down exercises. The total exercise lasts about 45–60 min (87).

Given the ease of the simplified Tai Chi protocols, novice students may practice Tai Chi by following the instructors' movements in class or at home using videotaped clips. It is worth noting that although these simplified protocols are encouraging, how they differ from traditional Tai Chi remains unknown. Comparative studies are needed to compare the beneficial effects of traditional Tai Chi and simplified Tai Chi.

How Tai Chi Differs From Other Mind–Body Exercises

Mind–body exercise encompasses a family of complex practices such as Tai Chi, yoga, and Baduanjin, each with different characteristics and foci. Although the exact mechanisms of these interventions are still under investigation, studies have found that the underlying mechanisms of different mindfulness movements may not be identical (88, 89).

In previous studies, we compared the modulation effect of Tai Chi and Baduanjin, another mind–body exercise. Baduanjin consists of only eight postures; thus, it is much simpler than other mind–body exercises and can be easily practiced by older adults, specifically those with cognitive decline, when guided by video or audio at home. As a result, we found that Baduanjin can produce greater and more extensive improvements in Wechsler Memory Scale (WMS) subscores and gray matter brain volume changes than Tai Chi in healthy older adults (63).

In another study (90), we compared the modulation effect of Tai Chi with Baduanjin on the DMN. We found that compared to the control group, Tai Chi increased posterior cingulate cortex rsFC with the right putamen/caudate, while Baduanjin decreased rsFC between the mPFC and orbital prefrontal gyrus/putamen. Direct comparison between the two mind–body interventions revealed that Tai Chi was significantly associated with increased rsFC between the mPFC and right putamen/caudate compared to Baduanjin, suggesting that these interventions may be associated with different mechanisms and treatment effects.

Developing a Tailored Tai Chi Protocol for Different Disorders

One important direction of Tai Chi research may be to create a tailored Tai Chi regimen for different disorders and individuals. Specifying a Tai Chi protocol for a particular population may significantly enhance the benefits of Tai Chi and reduce potential adverse effects.

In one such attempt, Tai Chi practitioner and researcher Dr. Albert Yeung developed a Tai Chi protocol specifically for individuals with mood disorders: Tai Chi for Mood (Tai Chi^{M}). The Tai Chi^{M} protocol consists of 12 separate Tai Chi moves along with mental focus and deep, paced breathing. Tai Chi^{M} excludes the complex transitions between Tai Chi movements, making it easier to learn and potentially a stronger intervention. A unique characteristic of Tai Chi^{M} is that it focuses more on meditative movements, relaxation, and paced breathing and less on physical strength training. Such characteristics make it an optimal choice for those with MDD and other mood disorders.

While furthering the development of Tai Chi may be enhanced with tailored protocols, this area of research also demands more fully powered RCTs. The studies summarized in this manuscript support the efficacy of Tai Chi in improving depressive symptoms, but future studies should strive to increase their sample sizes. Conducting studies with larger and more diverse samples will allow us to test the effects of Tai Chi on different subgroups of the MDD patient population. In addition, most of the current studies have been performed on an Asian population, and future studies targeting other populations are needed. Finally, future research should examine the differences between traditional Tai Chi and simplified, low-intensity Tai Chi to identify the most crucial components of this promising mind–body intervention. For instance, it is worth investigating which elements of traditional Tai Chi can be excluded without sacrificing the efficacy of the practice. We believe that the results obtained will facilitate the acceptance of Tai Chi and lower the cost of the practice, thereby making it a more economical and accessible alternative to pharmacologic and other treatments for depression.

We believe that Tai Chi^{M} could be a potential solution to the shortage of mental health providers and well-trained Tai Chi instructors worldwide. Further, it could reduce disparities in mental health treatment among ethnic minority populations who tend to avoid conventional psychiatric treatment due to stigmas surrounding mental illness. To enhance its availability, a video demonstrating Tai Chi for mood is publicly available on YouTube with Chinese, English, and Spanish subtitles (https://youtu.be/08IFKiXb3bA).

In summary, although Tai Chi has demonstrated its potential in mood regulation and relieving depressive symptoms, its underlying mechanism of action remains to be discovered. The instructional videos and the development of less structured Tai Chi has made it a promising low-intensity mind-body therapy. A fully powered RCT would largely benefit the development of Tai Chi, as would an exploration of ways to lower the cost of the intervention. Most importantly, a simplified and tailored Tai Chi—or a new intervention that combines Tai Chi and other mind–body exercises to enhance modulation effects on a specific patient population or individual—may represent a direction for further development of modern Tai Chi.

AUTHOR CONTRIBUTIONS

JK and AY conceived of the ideas presented in this manuscript. All authors contributed to manuscript preparation.

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Conflict of Interest Statement: JK holds equity in a startup company (MNT) and a pending patent to develop a new neuromodulation device.

The remaining authors declare that the research was conducted in the absence of any commercial or financial relationships that could be construed as a potential conflict of interest.

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Tai Chi on psychological well-being: systematic review and meta-analysis

Chenchen Wang*[1], Raveendhara Bannuru[1], Judith Ramel[1], Bruce Kupelnick[1], Tammy Scott[2] and Christopher H Schmid[2]

Abstract

Background: Physical activity and exercise appear to improve psychological health. However, the quantitative effects of Tai Chi on psychological well-being have rarely been examined. We systematically reviewed the effects of Tai Chi on stress, anxiety, depression and mood disturbance in eastern and western populations.

Methods: Eight English and 3 Chinese databases were searched through March 2009. Randomized controlled trials, non-randomized controlled studies and observational studies reporting at least 1 psychological health outcome were examined. Data were extracted and verified by 2 reviewers. The randomized trials in each subcategory of health outcomes were meta-analyzed using a random-effects model. The quality of each study was assessed.

Results: Forty studies totaling 3817 subjects were identified. Approximately 29 psychological measurements were assessed. Twenty-one of 33 randomized and nonrandomized trials reported that 1 hour to 1 year of regular Tai Chi significantly increased psychological well-being including reduction of stress (effect size [ES], 0.66; 95% confidence interval [CI], 0.23 to 1.09), anxiety (ES, 0.66; 95% CI, 0.29 to 1.03), and depression (ES, 0.56; 95% CI, 0.31 to 0.80), and enhanced mood (ES, 0.45; 95% CI, 0.20 to 0.69) in community-dwelling healthy participants and in patients with chronic conditions. Seven observational studies with relatively large sample sizes reinforced the beneficial association between Tai Chi practice and psychological health.

Conclusions: Tai Chi appears to be associated with improvements in psychological well-being including reduced stress, anxiety, depression and mood disturbance, and increased self-esteem. Definitive conclusions were limited due to variation in designs, comparisons, heterogeneous outcomes and inadequate controls. High-quality, well-controlled, longer randomized trials are needed to better inform clinical decisions.

Background

Mental illness affects 450 million people worldwide with 25% of the population affected in their lifetimes[1]. It is a leading cause of disability for people aged 15-44[2]. A growing list of psychological states including stress, anxiety, depression and mood disturbance have been linked to many chronic disorders such as coronary heart disease, cancer, diabetes and mental disorders as well as to accidents [3,4]. Mental illness poses significant economic burdens to those involved, reduces productivity and increases health care costs[5]. Thus, there is an urgent need for inexpensive and effective strategies to promote psychological well-being and improve general heath status, especially for people with chronic conditions.

Over the past decade, evidence from epidemiological studies and clinical trials has demonstrated a positive association between physical fitness and psychological health. Numerous studies have shown that physical activity and exercise as well as mind-body practice reduce morbidity and mortality for coronary heart disease, hypertension, obesity, diabetes and osteoporosis, and improve the psychological status of the general population [6-10].

Tai Chi, a form of Chinese low impact mind-body exercise, has been practiced for centuries for health and fitness in the East and is currently gaining popularity in the West. Our previous investigations have shown that Tai Chi has potential benefits in treating a variety of chronic

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conditions [11-13]. Significant improvement has been reported in balance, strength, flexibility, cardiovascular and respiratory function, as well as pain reduction and improved quality of life [11]. Several recent reviews have suggested that Tai Chi appears to improve mood and enhance overall psychological well-being [11,14,15]. However, convincing quantitative evidence to estimate treatment effects has been lacking. No meta-analysis addressing any psychological outcomes with Tai Chi has ever been published. To better inform patients and physicians, we systematically reviewed the quantitative and qualitative relationship between Tai Chi and psychological health outcomes (stress, anxiety, depression, mood and self-esteem) by critically appraising and synthesizing the evidence from all published studies of healthy and chronically ill populations in the East and West.

Methods

Data sources and searches

We conducted a comprehensive computerized search of the medical literature using 8 English databases: MEDLINE (from 1950), PsycINFO (from 1806), CAB (from 1910), Health Star (from 1966), Cochrane Database of Systematic Reviews (from 1991), CINAHL (from 1982), Global Health (from 1910) and Alt HealthWatch (from 1969). We also searched 3 major Chinese databases recommended by domain experts in evidence-based medicine in China. These included: China Hospital Knowledge Database (from 1994), China National Knowledge Infrastructure (from 1915) and WanFang Data (from 1980) through March 2009. We also searched reference lists of selected articles and reviews. The search terms for our review included "Tai Chi", "Tai Chi Chuan", "Tai Chi Chih", "ta'i chi," "tai ji," "Tai Ji Quan", and "taijiquan".

Study selection

Published articles that reported original data of randomized controlled trials (RCT), non-randomized comparison studies (NRS) and observational studies (OBS)[11] were eligible if they clearly defined a Tai Chi intervention [16]. We considered English and Chinese publications with at least 10 human subjects and evaluation of at least 1 of the following psychological health outcomes: (1) ***Psychological stress***--an imbalance between perceived capabilities and situational demands with manifestations in emotional states, as well as physiological, psychological and behavioral responses; (2) ***Anxiety***--an emotional state, characterized by a cognitive component (e.g. worry, self-doubt and apprehension) and a somatic component (e.g. heightened awareness of physiological responses such as heart rate, sweaty palms and tension); (3) ***Depression***--a depressive state diagnosed with standard instruments and/or clinical interviews; (4) ***Mood***--a pervasive and sustained emotion that colors the perception of the world; (5) ***Self-esteem***--a awareness of good processed by an individual and a representation of how positive one feels about oneself in general [17-19]. We excluded articles such as reviews, case reports, and conference proceedings that did not provide primary data.

Data extraction and quality assessment

We assessed the characteristics of the original research and extracted data based on study design; demographics; type and duration of Tai Chi exercise and controls; the psychological measures of stress, depression, anxiety, mood and self-esteem; results and/or the authors' main conclusions. When data were not provided in publications, we contacted the authors for information. Two reviewers extracted data and assessed trial quality of each study independently. Interrater reliability was satisfactory ($r \geq 90$). The methodological quality for the RCTs was evaluated based on the Jadad instrument [20], which takes into account whether a study described randomization, blinding, and withdrawals/dropouts.

Assessment of effect sizes and statistical analysis

When data were reported, we computed effect sizes (ES) in each study separately for stress, anxiety, depression and mood. ES was determined by calculating the standardized mean difference between groups. Overall outcome was assessed by pooling the ES of each study.

We calculated Hedges' g score for each study as a measure of ES. To correct for small sample size bias we computed the bias-corrected Hedges' g score for each measure. The magnitude of the ES (clinical effects) indicates: 0-0.19 = negligible effect, 0.20-0.49 = small effect, 0.50-0.79 = moderate effect, 0.80(+) = large effect. RCTs used the difference between the treatment and control group means. NRS used within-group difference between pretreatment and post-treatment means. In studies that involved more than one active intervention, we restricted our analyses to the Tai Chi and control groups. In view of significant heterogeneity, random-effect models were used for pooling. Heterogeneity was estimated with the I^2 statistic for both RCTs and NRS. All analyses were conducted using Meta-Analyst 3.13 statistical software (Tufts Medical Center, Boston, MA) [21].

Results

We reviewed 2579 English and Chinese articles and retrieved 61 full-text articles for detailed evaluation (Figure 1). Twenty-one studies were eliminated for not reporting original or relevant psychological outcome data. Ultimately, forty studies were identified for data abstraction and critical appraisal. Our search did not identify any unpublished literature.

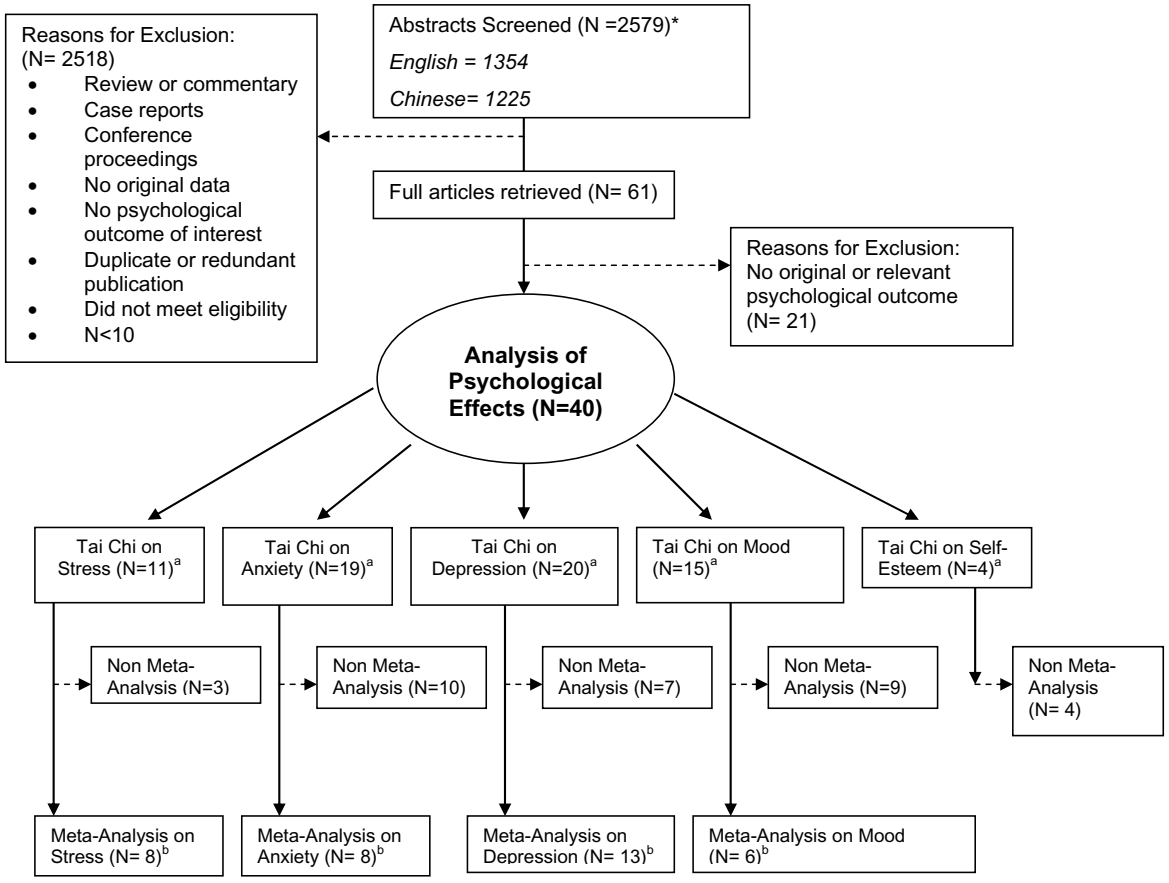


Figure 1 Flow diagram of selection of articles for inclusion* *N = number of studies. [a]These numbers add up to more than 40 because most studies assessed more than one psychological well-being state. [b]These numbers add up to more than 21 because most studies assessed more than one psychological well-being state.

Table 1 lists the study design and number of studies and participants for each psychological domain. Table 2 describes the 40 studies including 17 RCTs, sixteen NRS and 7 OBS published between 1980 and 2009. They were conducted in 6 countries (USA, China, France, Germany, UK and Australia). There were 3072 healthy individuals (25 studies) and 745 patients with chronic conditions (14 studies); no data were available in 1 study [22]. Mean age ranged from 11 to 92 years, and 62% of participants were female. Various controls were compared among the 17 RCTs, with 6 studies having multiple types of controls. Fourteen of the 16 NRS were self-comparisons while 2 used routine activity as controls. Three of the 7 OBS used routine activity, two used aerobic activity, one was a self-comparison and 1 used the general population as comparisons.

Nine RCTs reported on randomization; eight of these described an appropriate method, one an inappropriate one. Ten RCTs reported on blinding; in all 10, outcome assessors were blinded. Twelve studies described withdrawals and dropouts. Dropout rates were high with 4 studies reporting dropouts ≥ 25%, and 3 others that reported attrition of 32%, 35%, and 47%. Eleven NRS and 6 OBS reported dropouts, with high rates listed for 3 NRS, reporting 57%, 43% and 33% (Table 2).

In all, we included 21 trials (12 RCTs and 9 NRS) of 33 that provided data on psychological quantitative measures in the meta-analysis. The 7 OBS were not included in the meta-analysis. Figure 2 displays the overall effects of Tai Chi on stress, anxiety, depression and mood. Table 3 qualitatively assesses the 19 studies that were excluded from the meta-analysis, showing study characteristics, methodology quality, and psychological outcomes. We describe below results for each outcome separately for studies that provided data for meta-analysis and those excluded from meta-analysis.

Table 1: Summary of reviewed 40 Tai Chi studies (a)

Psychological Profiles	Study Design and Number of Studies and Participantsb			
	RCT	NRS	OBS	Participants
Stress	5	5	1	870
Anxiety	5	9	5	1,869
Depression	10	6	4	2008
Mood	4	8	3	1613
Self-Esteem	3	1	0	425

Abbreviations:
N = number of studies. RCT = randomized controlled trial; NRS = nonrandomized comparison study;
OBS = observational study,
[a]*Publication year:*
1980-2009. Language: 8 in Chinese and 32 in English.
Study site: United States (n = 20); China (n = 11); Australia (n = 6); United Kingdom (n = 1); Germany (n = 1); France (n = 1). Most studies assessed more than one psychological well-being state (the numbers add up to more than 40.)
[b]*Study design:*
RCT (N = 17): The investigator manipulates use of the procedure by offering it to one group of people (the exposed group), and offering nothing, a placebo, or some other procedure (s) to another group (the control group) by random allocation. Both groups are then followed to observe the rates of various outcomes.
NRS (N = 16 before and after trials and 2 external comparisons): The investigator causes some subjects to be exposed to the procedure, but does not use random allocation to determine the exposure that includes both an "internal comparison (before and after)" and an "external comparison (unexposed group)."
OBS (N = 7, cross-sectional nature): The investigator does not manipulate use of the procedure, but merely observes and interprets the outcomes.

Tai Chi and Stress

Five RCTs, five NRS and 1 OBS, conducted in 4 countries (USA, Australia, Germany and China) reported the effects of Tai Chi on stress in 870 participants with ages ranging from 16 to 85 years (Table 2). Most studies employed subjective stress measures, including the Depression, Anxiety, Stress Scale [23], Exercise Experiences Scale [24], Self-Perceived Stress score [25], Perceived Mental Stress score, Functional Assessment of HIV Infection [26], Impact of Event Scale [27], Perceived Stress Scale [28], and the Chinese Psychological Stress Scores. Two objective measures collected were body temperature[29] and salivary cortisol levels [30,31].

Meta-analysis results

Four RCTs and 4 NRS with 444 participants assessed the effects of Tai Chi on stress in individuals with HIV [32,33], elderly with symptomatic hip or knee osteoarthritis [34], healthy participants [29,35-37], and elderly with cardiovascular disease risk factors [38]. Tai Chi was performed between 10 and 24 weeks (60 to 120 minutes, 1 to 4 times per week). We found statistically significant improvements in stress management and psychological distress (ES, 0.66; 95% CI, 0.23 to 1.09) (Figure 2a), with an I^2 = 82%. This result remained significant upon dropping the study with the largest effect [29].

Studies not in meta-analysis

Three studies on stress were not included in the meta-analysis because 1 RCT and 1 NRS treated participants with Tai Chi practice for only one hour [39,40], and the third was an OBS[41] (Table 3). The RCT with 96 healthy adults showed significantly decreased levels of stress in all groups after one hour of intervention (Tai Chi, meditation, brisk walking and neutral reading) [39]. The NRS also reported a single one-hour Tai Chi intervention that significantly reduced stress in healthy adults [40]. The OBS using a Chinese psychological stress questionnaire with 76 healthy Chinese elderly reported that 5 years of regular Tai Chi experience (>30 minutes and >3 times per week) significantly improved stress compared with less physical activity (<30 minutes and <3 times per week). This study, however, found no statistically significant difference between Tai Chi and regular activities of the same duration and frequency [41].

Summary

Overall, Tai Chi was positively associated with improved in stress levels in healthy adults, patients with HIV-related distress and elderly Chinese with cardiovascular disease risk factors [29,32,33,35-41]. However, the overall study quality was modest with inadequate or no controls in the majority of studies.

Table 2: Characteristics of 40 articles published in 1980-2009 on Tai Chi and psychological status

Referencea	Mean age, yr	Population, n	Duration	Intervention Frequency, style		Psychological Status Measured	Methodological Quality		
				Tai Chi	Control(s)		Randomizationb	Outcome Assessors Blindedc	(% dropout)d
17 Randomized Controlled Trials									
Wang et al, 2009, USA	50	Individuals with FM, 60	12 wks	1 hr, 2×/wk (classical Yang style)	Attention control	Depression (CES-D)	Y	Y	10.6%
Dechamps et al, 2009, France	44	Sedentary obese women, 21	10 wks	2 hr, 1×/wk (Yang style)	Exercise Program	Depression (BDI)	Y	Y	0%
Wang, 2008, USA	50	Functional class I or II RA, 20	12 wks	1 hr, 2×/wk (classical Yang style)	Attention control	Depression (CES-D)	Y	Y	0%
Wang et al, 2008, USA	65	Elderly with symptomatic KOA, 40	12 wks	1 hr, 2×/wk (classical Yang style)	Attention control	Depression (CES-D)	Y	Y	0%
McCain et al, 2008, USA	42	Individuals with HIV infection, 252	10 wks	90 min, 1×/wk (focused short form of Tai Chi with 8 movements)	1.Cognitive -behavioral relaxation training 2.Spriritual growth 3.Wait Liste	Stress (IES, Salivary Cortisol Level)f, Mood and Emotion (FAHI)	Y	Y	47%
Fransen et al, 2007, Australia	70	Elderly with symptomatic hip or KOA,152	12 wks	1 hr, 2×/wk (modified 24 forms Sun style)	1.Hydrotherapy 2.Waiting Liste	Stress, Anxiety, Depression (DASS 21)	Y	Y	12.5%
Irwin et al, 2007, USA	70	Healthy elderly adults, 112	16 wks	40 min, 3×/wk (unspecified style)	Health education	Depression (BDI)	Y	ND	8.9%
Sattin et al, 2005, USA Wolf et al, 2003, USA	81	Elderly transitioning to frailty, 311	48 wks	60-90 min progression, 2×/wk (6 of the 24 simplified TC forms)	Wellness education	Depression (CES-D)	ND	Y	29.9%
Galantino et al, 2005, USA	(20-60)	Advanced HIV/AIDS, 38	8 wks	1 hr, 2×/wk (unspecified style)	1.Aerobic exercise 2.Usual activity	Anxiety (POMS)	Y	ND	25.5%
Chou et al, 2004, China	73	Elderly with depression,14	12 wks	45 min, 3×/wk (18 form of Yang style)	Waiting list	Depression (CES-D-Chinese Version)	ND	Y	ND
Mustian et al, 2004, USA	52	Breast cancer survivors, 21	12 wks	1 hr, 3×/wk (15 short form Yang style)	Psychosocial support therapy	Self-Esteem (RSE)	ND	ND	32.3%

Table 2: Characteristics of 40 articles published in 1980-2009 on Tai Chi and psychological status (Continued)

Tsai et al, 2003, China	52	Healthy elderly, 76	12 wks	50 min, 3×/wk (108 postures Yang style)	Sedentary life control	Anxiety (STAI)	IA	Y	13.6%
Li et al, 2001, USA	73	Healthy elderly, 72	24 wks	1 hr, 2×/wk (classical Yang style)	Waiting list	Stress (SEES) Depression (CES-D) Mood and Emotion (PANAS)	ND	ND	26.5%
Kutner et al, 1997, USA	76	Healthy elderly 130	15 wks	1 hr, 2×/wk (10 modified forms Yang style)	1. education control 2. balance training	Self-Esteem (RSE)	ND	ND	35%
Sun et al, 1996, USA	(60-79)	Healthy elderly Hmong immigrants, 20	12 wks	2 hrs, 1×/wk for 10 sessions (unspecified style)	Routine physical activity	Stress (SPS, Body Temperature[f])	ND	ND	0%
Brown et al, 1995, USA	53	Healthy adults, 135	16 wks	45 min, 3×/wk (unspecified style--"mindful exercise")	1.moderate intensity walk 2.low intensity walk 3.low intensity walk & relaxation 4.usual lifestyle behaviors	Anxiety, Depression, Mood and Emotion (STAI, POMS, Tukey multiple comparison test, LSES, PANAS) Self-Esteem (RSE, SPES and BCS)	ND	ND	25%
Jin, 1992, Australia	36	Healthy adults, 96	1 hr	Single 1 hr session (long form, Yang style or Wu variation of Yang style)	1.TC meditation 2.brisk walking 3.neutral reading	Stress (Salivary Cortisol Level[f]) Anxiety (STAI Y-1) Mood and Emotion (POMS)	ND	Y	ND
16 Non-Randomized Comparison Studies									
Lee et al, 2007, Hong Kong, China	83	Healthy elderly, 139	26 wks	1 hr, 3×/wk (unspecified style)	Usual activity	Self-Esteem (SSES-Chinese Version)	N	ND	ND
Esch et al, 2007, Germany	28	Healthy young adults, 9	14 wks	90 min, 1×/wk for 12 sessions (Yang style)	Self-comparison	Stress (Perceived Mental Stress (VAS), Salivary Cortisol level) [f]	N	ND	57.1%
Robins et al, 2006, USA	42	HIV disease, 59	10 wks	1 hr, 1×/wk (8 movements short form, unspecified style)	Self-comparison	Stress (FAHI, IES)	N	ND	ND
Taylor-Piliae et al, 2006, USA	66	Elderly Chinese with CVD risk factors, 38	12 wks	1 hr, 3×/wk (24-posture short form, Yang style)	Self-comparison	Stress (PSS) Anxiety, Depression, Mood and Emotion (POMS)	N	ND	2.6%
Chen et al, 2005, China	54	Healthy elderly, 30	24 wks	1 hr, 4×/wk (simplified 24 forms Yang style & 42 forms TC sword)	Self-comparison	Stress, Anxiety (STAI, POMS) Depression, Mood and Emotion (POMS)	N	ND	ND

Table 2: Characteristics of 40 articles published in 1980-2009 on Tai Chi and psychological status (Continued)

Wall, 2005, USA	(11-13)	Healthy children, 11	5 wks	1 hr, 1×/wk (traditional segments of Yang style & MBSR)	Self-comparison	Anxiety (Written subjective statements)	N	ND	ND
Li, 2004, China	20	Healthy college students, 66	24 wks	1 hr, 3×/wk (unspecified style)	Self-comparison	Anxiety, Depression (SCL-90)	N	ND	ND
Taggart et al, 2003, USA	56	Individuals with FM, 21	6 wks	1 hr, 2×/wk (Yang style short form)	Self-comparison	Anxiety, Depression (FIQ)	N	ND	43.2%
Hernandez-Reif et al, 2001, USA	15	Adolescents with ADHD, 13	5 wks	30 min, 2×/wk (unspecified style)	Self-comparison	Anxiety, Mood and Emotion (CTRS-R)	N	ND	ND
Mills et al, 2000, UK	48	Adults with MS, 8	8 wks	6 individual sessions (unspecified style or duration)	Self-comparison	Anxiety, Depression (POMS)	N	ND	33.3%
Ross et al, 1999, USA	(68-92)	Healthy elderly, 13	8 wks	1 hr, 3×/wk (unspecified style)	Self-comparison	Mood and Emotion (MAACL-R-composite score)	N	ND	23.5%
Chen & Sun, 1997, USA	(50-74)	Healthy adults, 28	16 wks 4th mo to 9th mo follow-up	1 hr, 2×/wk, (simplified 24 forms) 1×/mo during follow-up	Routine activity	Anxiety (TMAS and SAI)	N	ND	22.2%
Gibb et al, 1997, Australia	80	Elderly with dementia (56%) or AD patients (44%), 9	7 wks	13 sessions 2×/wk (unspecified style)	Self-comparison	Mood and Emotion (Biographical Self Reflection Program)	N	ND	10%
Fu et al, 1996, Australia	34	Healthy students & teachers, 90	1 yr	30 min, 6×/wk (simplified 24 forms)	Self-comparison	Mood and Emotion (Records from self- report program)	N	ND	ND
Jin, 1989, Australia	(16-75)	Healthy adults with TC experience, 66	1 hr	Single 1 hr session (Long Form Yang style or Wu variation of Yang style)	Self-comparison	Stress (Salivary Cortisol Level[f]) Depression (POMS) Anxiety (STAI) Mood and Emotion (POMS)	N	ND	ND
Mack, 1980, Australia	ND	Afro-American males, ND	24 wks	ND	Self-comparison	Mood and Emotion (no test reported)	N	ND	ND

Table 2: Characteristics of 40 articles published in 1980-2009 on Tai Chi and psychological status (Continued)

7 Observational Studies									
Chen et al, 2006, China	(20-35)	Healthy students & teachers, 480	0.5-8 yrs	Over 2 hrs/day (24, 28, 58, 88 forms of Chen, Wu, Yang style & TC sword)	Self-comparison	Mood and Emotion Self-design (10 items)	N	Y	ND
Wang & Wang, 2004, China	62	Healthy elderly, 76	>5 yrs	>30 min, >3×/wk (regular TC & TC sword)	1. some activity, <30 min, <3×/wk 2. other activity, >30 min, >3×/wk	Stress (Chinese Psychological Stress Scores)	N	ND	ND
Yang et al, 2004, China	62	Healthy elderly, and middle-aged, 373	>1 yr	Regular TC (unspecified style)	Routine activity	Anxiety (Zung SAS) Depression (Zung SDS)	N	ND	ND
Bond et al, 2002, USA	37	Healthy adults, 249	>6 mos	Regular TC at least 20 min, 3×/wk (unspecified style)	1.sedentary 2.moderate aerobic activity	Anxiety (STAI)	N	ND	0.8%
Chen et al, 2001, Taiwan, China	74	Healthy elderly, 80	≥1 yr	Regular TC at least 2×/wk & 30 min/session (unspecified style)	No exercise control group	Anxiety, Depression, Mood and Emotion (POMS-SF)	N	ND	ND
Liu & Zhang, 2000, China	(18-20)	Healthy individuals, 150	<1 yr >1 yr >2 yrs	Regular TC (unspecified frequency & style)	General population	Anxiety, Depression (Self-Rating Scale-90)	N	ND	ND
Long et al, 2000, China	62	Healthy adults, 239	1-14 yrs	Regular TC (Yang style)	Routine activity	Anxiety, Depression, Mood and Emotion (POMS)	N	ND	ND

Abbreviations: yr = year; n = only evaluated participants included; FM = Fibromyalgia; wk(s) = week(s); hr = hour; CES-D = Center for Epidemiology Studies Depression index; RA = Rheumatoid Arthritis; KOA = Knee Osteoarthritis; DASS 21 = Depression, Anxiety, Stress Scales 21 item questionnaire; min(s) = minute(s); BDI = Beck Depression Inventory; TC = Tai Chi; HIV = Human Immunodeficiency Virus; AIDS = Autoimmune Deficiency Syndrome; POMS = Profile of Mood States; ND = no data; RSE = Rosenberg Self-Esteem scale; STAI = State-Trait Anxiety Inventory; SEES = Subjective Exercise Experience Scale; PANAS = Positive and Negative Affect Schedule; SPS = Self-Perceived Stress score; LSES = Life Satisfaction in the Elderly Scale; SPES = Sonstroem Physical Examination Scale; BCS = Body Cathexis Score; SSES = State Self-Esteem Scale; VAS = Visual Analogue Scale; FAHI = Functional Assessment of HIV Infection; IES = Impact of Events Scale; CVD = Cardiovascular Disease; PSS = Perceived Stress Scale; MBSR = Mindfulness-Based Stress Reduction; SCL-90 = Symptom Checklist-90; FIQ = Fibromyalgia Impact Questionnaire; ADHD = Attention Deficit Hyperactive Disorder; CTRS-R = Conners' Teacher Rating Scale-Revised; MS = Multiple Sclerosis; MAACL-R = Multiple Affect Adjective Checklist-Revised; mo(s) = month(s); TMAS = Taylor Manifest Anxiety Scale; SAI = State Anxiety Inventory; AD = Advanced Dementia; SAS = Self-Rating Anxiety Scale; SDS = Self-Rating Depression Scale.

[a]Studies in **bold** are meta-analyzed

[b]Randomization: Was the study described as randomized (this includes the use of words such as randomly, random, and randomization)? Y = The method to generate the sequence of randomization was described and it was appropriate (table of random numbers, computer generated, etc.); ND = randomization not described; IA = The method to generate the sequence of randomization was described and it was inappropriate (patients were allocated alternately, or according to date of birth, hospital number, etc.); N = non-randomized trial.

[c]Blinding: Was the study described as blind? (Double-blinding is impractical in Tai Chi studies, our modification gave 1 point for proper single blinding of the outcome assessor.) Y = The method of double blinding was described and it was appropriate (identical placebo, active placebo, dummy, etc.); ND = blinding not described.

[d]Dropouts and Withdrawals: Was there a description of withdrawals/dropouts? ND = withdrawals/dropouts not described.

[e]Treatment group compared to Tai Chi in meta-analysis.

[f]Objective measure.

Table 3: Effects of Tai Chi on psychological health (19 non-meta-analyzed studies)

Reference	Control Group(s)	Psychological Status Measured	P-value	Main Conclusions
5 Randomized Controlled Trials				
Galantino et al, 2005	1.Aerobic exercise 2.Usual activity	Anxiety (POMS)	0.005	Tai Chi decreased tension-anxiety
Mustian et al, 2004	Psychosocial support therapy	Self-Esteem (RSE)	0.01	Tai Chi improved self-esteem
Kutner et al, 1997	1. Education control 2.Balance training	Self-Esteem (RSE)	NS	Tai Chi improved self-esteem vs. exercise control and vs. education & balance training
Brown et al, 1995	1.Moderate intensity walk 2.Low intensity walk 3.Low intensity walk & relaxation 4.Usual lifestyle behaviors	Anxiety, Depression, Mood and Emotion (STAI, POMS, Tukey multiple comparison test, LSES, PANAS) Self-Esteem (RSE, SPES and BCS)	<0.05	Tai Chi improved anxiety, depression, general mood and mood disturbance and negative affect (women)
			NS	Tai Chi improved self-esteem in physical competence (men and women)
Jin, 1992	1.TC meditation 2.Brisk walking 3.Neutral reading	Stress (Salivary Cortisol Level[a]) Anxiety (STAI Y-1) Mood and Emotion (POMS)	<0.005	Tai Chi reduced anxiety vs. neutral reading group
			<0.001	All groups reduced stress level and improved total mood disturbance
7 Non-Randomized Comparison Studies				
Lee et al, 2007	Usual activity	Self-Esteem (SSES-Chinese Version)	<0.001	Tai Chi improved self-esteem
Wall, 2005	Self-comparison	Anxiety (Written subjective statements)	ND	Students reported feeling calmer, peaceful, relaxed & experienced well-being & improved sleep
Mills et al, 2000	Self-comparison	Anxiety, Depression (POMS)	NS	Tai Chi decreased tension-anxiety
			<0.01	Tai Chi decrease depression-dejection
Gibb et al, 1997	Self-comparison	Mood and Emotion (Biographical Self Reflection Program)	ND	Tai Chi improved structured reminiscence with facilitated focused & insightful thinking
Fu et al, 1996	Self-comparison	Mood and Emotion (Records from self- report program)	ND	Tai Chi decreased total mood disturbance
Jin, 1989	Self-comparison	Stress (Salivary Cortisol Level) [a] Depression (POMS) Anxiety (STAI) Mood and Emotion (POMS)	<0.01	Tai Chi decreased stress during and after Tai Chi

Table 3: Effects of Tai Chi on psychological health (19 non-meta-analyzed studies) (Continued)

			<0.001	Tai Chi decreased anxiety and total mood disturbance during and after TC
			<0.05	Tai Chi decreased depression during and after Tai Chi
Mack, 1980	Self-comparison	Mood and Emotion (no test reported)	ND	Tai Chi improved mood status
7 Observational Studies				
Chen et al, 2006	Self-comparison	Mood and Emotion (Self-design, 10 items)	ND	Tai Chi improved mood status & psychological health
Wang & Wang, 2004	1. >30 min, >3×/wk (regular Tai Chi & Tai Chi sword) 2. some activity, <30 min, <3×/wk 3. other activity, >30 min, >3×/wk	Stress (Chinese Psychological Stress Scores)	<0.01	Tai Chi decreased psychological stress vs. some activity (<30 min, <3×/wk)
			NS	Tai Chi decreased psychological stress vs. other activity (>30 min, >3×/wk)
Yang et al, 2004	Routine activity	Anxiety (Zung SAS) Depression (Zung SDS)	<0.01	Tai Chi decreased anxiety and depression scores
Bond et al, 2002	1.Sedentary 2.Moderate aerobic activity	Anxiety (STAI)	<0.05	Tai Chi and moderate aerobic activity decreased state anxiety vs. sedentary group
Chen et al, 2001	No exercise control group	Anxiety, Depression, Mood and Emotion (POMS-SF)	0.000	Tai Chi decreased tension-anxiety, depression-dejection and total mood disturbance
Liu & Zhang, 2000	General population	Anxiety, Depression (Self-Rating Scale-90)	<0.01	Tai Chi decreased anxiety and depression scales
Long et al, 2000	Routine activity	Anxiety, Depression, Mood and Emotion (POMS)	<0.01	Tai Chi decreased anxiety, depression scales and total mood disturbance

Abbreviations: ND = no data; NS = not statistically significant; DASS 21 = Depression, Anxiety, Stress Scales 21 item questionnaire; POMS = Profile of Mood States; RSE = Rosenberg Self-Esteem scale; STAI = State-Trait Anxiety Inventory; LSES = Life Satisfaction in the Elderly Scale; PANAS = Positive and Negative Affect Schedule; SPS = Self-Perceived Stress score; SPES = Sonstroem Physical Examination Scale; BCS = Body Cathexis Score; SSES = State Self-Esteem Scale; FAHI = Functional Assessment of Human Immunodeficiency Virus Infection; SAS = Self-Rating Anxiety Scale; SDS = Self-Rating Depression Scale. [a] Objective measure.

Tai Chi and Anxiety

Five RCTs, 9 NRS and 5 OBS investigated the anxiety-reducing effect of Tai Chi in 1869 people from 4 countries (USA, UK, Australia and China) (Table 2). Seven studies used the Profile of Mood States Anxiety subscale[42] and 6 employed the State-Trait Anxiety Inventory [43]. The remainder used: the Depression, Anxiety, Stress Scale [23], Connors' Teacher Rating Scale-Revised [44], Taylor Manifest Anxiety Scale [45], State Anxiety Inventory[46] and Zung Self-Rating Anxiety Scale [47]. Two disease-specific anxiety measures were used: the Symptom Checklist-90[48] and the Fibromyalgia Impact Questionnaire [49].

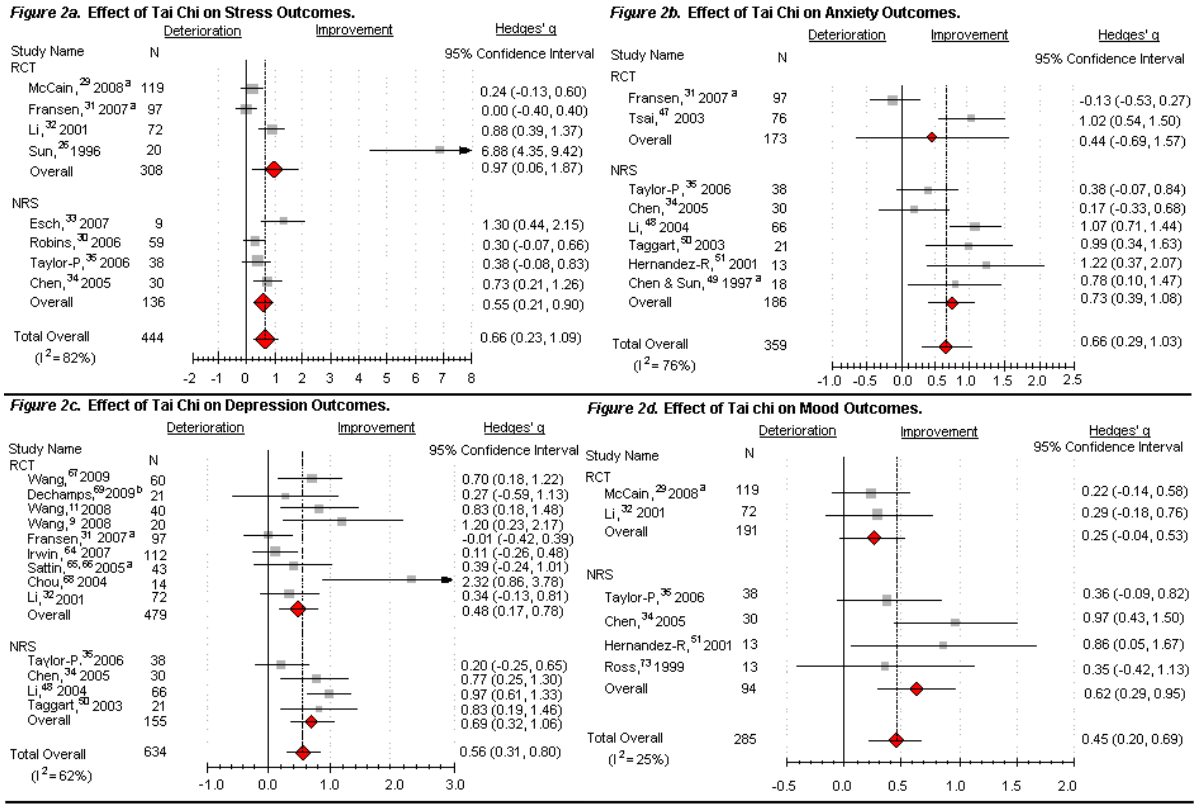


Figure 2 Effects of Tai Chi on stress, anxiety, depression and mood*. RCT = randomized controlled trial; NRS = nonrandomized comparison study (all the meta-analyzed NRS are self-comparison studies). N = number of participants. [a] McCain, 2008, included only Tai Chi versus wait list control (n = 119); Fransen 2007, included only Tai Chi versus control group (n = 97); Chen & Sun 1997, included only participants in Tai Chi group as pretreatment, posttreatment (n = 18); Sattin 2005, included only clinically depressed participants in Tai Chi and control arms (n = 43). [b] Dechamps, 2009, used an active control compared to Tai Chi. *The magnitude of the effect size (clinical effects) indicates: 0-0.19 = negligible effect, 0.20-0.49 = small effect, 0.50-0.79 = moderate effect, 0.80+ = large effect.

Meta-analysis results

Two RCTs and 6 NRS with 359 participants including patients with symptomatic osteoarthritis [34], healthy adults [37,50-52], elderly with cardiovascular disease risk factors [38], individuals with fibromyalgia [53], and adolescents with ADHD [54] found that Tai Chi practiced 2 to 4 times a week (30 to 60 minutes/time) for 5 to 24 weeks was associated with a significant reduction in anxiety (ES, 0.66; 95% CI, 0.29 to 1.03) (Figure 2b) with I^2 = 76%.

Studies not in meta-analysis

Three RCTs, 2 NRS and 5 OBS were not included in the meta-analysis due to lack of sufficient quantitative data (Table 3). The duration of Tai Chi interventions ranged from 1 hour to 14 years (20 minutes to 1 hour, 1 to 3 times a week). Results from 2 RCTs with 134 subjects reported that Tai Chi practice for a single 1 hour session [39], or twice a week for 60 minutes/time over 8 weeks was associated with a significant reduction in anxiety scores both among HIV patients[55] and healthy adults [39,40,56]. Of note, the third RCT by Brown et al, who randomized 135 healthy adults to a Tai Chi-type activity, moderate or low intensity walking, walking with relaxation, or control reported results by gender [56]. The authors observed a significant decline in anxiety for women, but an insignificant decline in anxiety for men. Wall et al conducted an NRS with 11 6th and 8th graders who performed Yang style Tai Chi and Mindfulness-Based Stress Reduction training (1 hour once a week, for 5 weeks) and found that the participants were "calmer, more peaceful and enjoyed improved sleep relaxation" [57]. The other NRS by Mills et al reported a nonsignificant decrease in tension-anxiety in 8 adults with Multiple Sclerosis [58]. Five large OBS of 1091 healthy adult participants, who practiced Tai Chi between 6 months and 14 years, showed significantly reduced anxiety measures compared to the general population [59], sedentary controls and people engaged in routine or moderate aerobic activity [60-63].

Summary

Overall, Tai Chi was positively associated with reduced anxiety using one or more anxiety measures, but overall study quality was modest.

Tai Chi and Depression

Ten RCTs, 6 NRS and 4 OBS examined the effects of Tai Chi on depression in 2008 subjects. Tai Chi intervention ranged from a single 1-hour session to 14 years. Six studies used the Center for Epidemiology Studies Depression Scale [64]. Seven used the Profile of Mood States Depression subscale [42], and the rest tests were the Beck Depression Inventory [65], Fibromyalgia Impact Questionnaire [49], Zung Self-Rating Depression Scale[66] and Self-Rating Scale-90.

Meta-analysis results

Nine RCTs and 4 NRS in 634 people examined the effects of Tai Chi on depression versus education, routine activity, waiting list and other forms of exercise as well as self-comparison among healthy adults [35,37,51,67-69], individuals with rheumatoid arthritis [12], osteoarthritis [13,34], fibromyalgia [53,70], depression disorders [71], sedentary obese women [72], and elderly Chinese with cardiovascular disease risk factors [38]. Of note, only two studies involved participants with clinically diagnosed depression [68,69,71].

Overall, our analysis suggests that 6 to 48 weeks (40 minutes to 2 hours, 1 to 4 times a week) of Tai Chi practice resulted in significant depression-reduction effects compared to various controls (ES, 0.56; 95% CI, 0.31 to 0.80) (Figure 2c) with I^2 = 62%. This result remained significant upon dropping the study with the large effect [71].

Studies not in meta-analysis

One RCT, 2 NRS and 4 OBS lacked sufficient quantitative detail for analysis (Table 3). Brown et al found a significant decrease in depression for women [56]. The studies by Mills and Jin also reported significant decreases in depression after 8 weeks [58] and an hour of Tai Chi practice, respectively [40]. Four OBS in 842 healthy Chinese subjects showed that regular Tai Chi practice up to 14 years demonstrated statistically significant reductions in depressive symptoms compared with routine activity [59,60,62,63].

Summary

Overall, evidence from most studies showed that Tai Chi tended to reduce depression. This result was associated with improvement in symptoms and physical function in patients with rheumatoid arthritis and multiple sclerosis, as well as improvement in the immune response of healthy elderly participants. However, the vast majority of the studies suffer from less rigorous designs and were conducted on "healthy" populations with only two studies reporting results on participants diagnosed with clinical depression [68,71].

Tai Chi and Mood

Evidence on the effects of Tai Chi on mood was examined from 4 RCTs, eight NRS and 3 OBS in 1613 subjects. Duration of Tai Chi practice ranged from 1 hour to 14 years (1 to 7 times a week). The majority of studies reported the total score of the Profile of Mood States scale [42]; other measures were the Functional Assessment of HIV Infection [26], Positive and Negative Affect Schedule [73], Life Satisfaction in the Elderly Scale [74], Symptom Checklist-90 [48], Conners' Teacher Rating Scale-Revised [44], and Multiple Affect Adjective Checklist-Revised [75].

Meta-analysis results

Two RCTs and 4 NRS assessed the effects of Tai Chi on mood in healthy elderly [35,76], individuals with HIV [32], elderly Chinese with cardiovascular disease risk factors [38], and adolescents with attention-deficit hyperactivity disorder[54]. Tai Chi was performed between 5 and 24 weeks (30 to 90 minutes, 1 to 4 times per week). Tai Chi significantly improved mood compared to various controls with overall ES of (0.45; 95% CI, 0.20 to 0.69), and the I^2 = 25% (Figure 2d).

Studies not in meta-analysis

Two RCTs, four NRS and 3 OBS lacked sufficient quantitative detail for analysis. Brown et al reported that 16 weeks of Tai Chi showed significantly elevated mood for women [56]. Significant improvement in mood was also reported in a RCT following short-term Tai Chi training [39]. Results from 3 NRS reported non-significant improvements in mood following 7 weeks to 1 year of Tai Chi [22,77,78] and one NRS reported a significant decrease in mood disturbance after an hour of Tai Chi practice [40]. Similarly, 3 OBS of 799 subjects reported that 0.5 to 14 years of Tai Chi practice significantly improved mood disturbance in healthy participants [62,63].

Summary

Overall, evidence suggested short and long-term Tai Chi practice had favorable effects on mood among healthy adults [35,37,39,40,56,62,63,76], elderly with cardiovascular risk factors [38], obese women [32], and adolescents with attention deficit hyperactivity disorder [54]. However, the overall study quality was poor with inadequate or no controls in the majority of studies.

Tai Chi and Self-esteem

Meta-analysis results

Only 3 RCTs and 1 NRS evaluated the effects of Tai Chi on self-esteem in 425 subjects and there are no sufficient quantitative data for meta-analysis.

Studies not in meta-analysis

In these 4 trials, Tai Chi practice lasted from 12 to 26 weeks (45 to 60 minutes, 2 to 3 times per week). Among the measurements employed were: Rosenberg's 10-item Global Self-Esteem Scale [79], the Chinese version of the State Self-Esteem Scale [80], Sonstroem Physical Examination Scale [81] and the Body Cathexis Score [82]. Results from 3 RCTs involving 286 subjects reported that 12 to 16 weeks of Tai Chi was associated with increases in self-esteem scores compared with control groups [56,83,84]. Two RCTs, however, reported nonsignificant results [56,84]. A significant improvement in self-esteem was reported in a recent 26 week NRS that compared Tai Chi to routine activity among 139 Chinese healthy elderly [85].

Summary

Overall, Tai Chi was positively associated with improvement in self-esteem although no meta-analysis result was provided.

Discussion

Tai Chi, a form of low impact mind-body exercise, has spread worldwide over the past two decades. This systematic review and meta-analysis summarizes and updates results of the effects of Tai Chi exercise on health outcomes[11] in terms of psychological effects in various populations.

Evidence accrued from clinical trials and observational studies indicates that Tai Chi-- both short and long-term--appears to have mental health benefits in promoting psychological well-being, self-esteem and life satisfaction among healthy subjects and patients with chronic conditions. Specifically, twenty-three of the 33 RCTs and NRS from our quantitative meta-analysis and qualitative evidence synthesis reported that 1 hour to 1 year of regular Tai Chi activity significantly reduced stress, anxiety and depression, and enhanced mood in healthy adults and patients with chronic conditions. The 7 OBS with relatively large sample sizes reinforce the beneficial effects of Tai Chi on psychological health, although bias may be inherent in these observational data.

Our review is congruent with other recent epidemiological reports, experimental trials and literature reviews supporting the fact that physical activity and exercise are associated with better psychological health [8,14,15,86]. Biddle et al recently reviewed evidence on physical activity and exercise in relation to different aspects of mental health. They found that exercise is associated with the strongest anxiety-reduction effects and emphasized the causal link between physical activities and reduction in clinically-defined depression [18]. In particular, evidence from meta-analyses and narrative reviews demonstrates that physical activity and exercise as well as mind-body practice have consistently been associated with positive mood and affect [10,14,15,87-89].

There is insufficient evidence to find any dose-response effect of Tai Chi for psychological outcomes. The studies included in this review exhibit a wide variety of Tai Chi styles, frequency, duration and follow up. The Yang style was used in 17 studies. The majority of studies featured Tai Chi 2 to 3 times per week (frequency), for at least 20 minutes and for an average of 40 to 60 minutes per session (duration). The length of exercise programs ranged from 5 weeks to 1 year, or a single hour for two studies. In the OBS, Tai Chi duration ranged from 6 months to 14 years, and all studies found positive psychological benefits. Few studies reported the intensity and relationship between adherence to Tai Chi and positive psychological effects. Further studies are needed to optimize effective evidence-based dose-response effects and should strictly demand descriptions of intensity, frequency, duration and adherence of the Tai Chi exercise.

Tai Chi appears to be an effective therapeutic modality to improve psychological well-being among various populations. However, it is still difficult to draw firm conclusions. *First*, we did not include any unpublished studies. The overall methodological quality of previous studies is unsatisfactory, consisting mostly of small sized or non-randomized comparisons. Given the few high quality RCTs available for investigation, our review is limited by wide variations in methodological rigor of clinical trials and observational studies. *Second*, the heterogeneous amalgamation of instruments used to collect clinical psychological health data restricts our ability to evaluate differences in these outcomes. *Third*, it remains unclear whether Tai Chi mind-body exercise provides equal or superior psychological benefits compared to moderate-intensity aerobic exercises. *Fourth*, most studies failed to provide objective measures of stress and anxiety such as salivary cortisol level, blood pressure or heart rate, and some studies only reported a subset of psychological outcomes. Due to the limited physiological variables in our analyses, we were unable to analyze the effect of Tai Chi on physiological effects. *Fifth*, the studies included in the meta-analyses demonstrated a relatively high degree of heterogeneity. Various patient populations were used, and most studies involved healthy people. There were also many variations between the included studies with regard to methodological quality (eg, problems of randomization, allocation concealment, or reporting results), which prohibited us from analyzing the quantita-

tive evidence. However, it is difficult to compare results across studies because they were assessed at different time points. Additionally, all the studies published in mainland China, Hong Kong and Taiwan reported unanimously positive results. Differences in methodological rigor between eastern and western studies may be potential sources of heterogeneity, and publication bias may vary across countries and cultures.

Few published studies have specifically investigated the underlying mechanism of action of Tai Chi's effects on psychological health. Many intermediate, but unidentified, variables may lie along the pathway from Tai Chi to improved psychological well-being. Measures of psychological variables and a multitude of other outcome measures are empirically inter-related, and treatment of each outcome can reciprocally and exponentially improve the other. Improvement of psychological status is also associated with improvement in other clinical outcomes such as arthritic pain as well as health status [90]. The possible mechanisms for enhanced psychological health resulting from Tai Chi mind-body exercise may therefore act through its beneficial influence on biological, physiological, cardiovascular, neurological, and immunological effects as well as overall well-being [11,67,89,91].

Conclusions

In conclusion, the results of these studies suggest that Tai Chi may be associated with improvements in psychological well-being including reduced stress, anxiety, depression and mood disturbance, and increased self-esteem. High-quality, rigorous, prospective, well-controlled randomized trials with appropriate comparison groups and validated outcome measures are needed to further understand the effects of Tai Chi as an intervention for specific psychological conditions in different populations. Knowledge about the physiological and psychological effects of Tai Chi exercise may lead to new complementary and alternative medical approaches to promote health, treat chronic medical conditions, better inform clinical decisions and further explicate the mechanisms of successful mind-body medicine.

Competing interests

The authors declare that they have no competing interests.

Authors' contributions

CW obtained funding for the study. CW, BK and CS designed the study. CW, RB, JR, BK and CS conducted the research. RB and CS conducted the meta-analysis. CW wrote the first draft of the manuscript. CW, CS, RB, JR, BK and TS participated in the revision of subsequent draft. All authors read and approved the final manuscript.

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